

Auszug aus der UAW-Datenbank des BfArM

Stand der Datenbank: 18.03.2013

Einzelfallausdrucke

Kriterien: Diclofenac, Deutschland ab 1993, SMQ Hepatic disorders

Hinweise zur Interpretation der Daten:

Es handelt sich bei den Berichten sowohl um Spontanmeldungen als auch um Berichte aus systematisierten Untersuchungen, aus Projekten oder aus der Literatur. Die Meldungen sind dem Bundesinstitut für Arzneimittel und Medizinprodukte durch pharmazeutische Unternehmer, durch die Arzneimittelkommissionen der Heilberufe, direkt durch niedergelassene oder in Krankenhäusern tätige Ärzte sowie durch sonstige Meldequellen, z. B. von Patienten, berichtet worden. Es muss beachtet werden, dass es sich um Verdachtsfälle unerwünschter Arzneimittelwirkungen handelt, dass also ein Kausalzusammenhang im Einzelfall nicht sicher belegt ist. Ferner lassen sich auf der Grundlage von Spontanberichten keine Aussagen darüber machen, wie häufig eine bestimmte unerwünschte Wirkung unter der Anwendung eines Arzneimittels vorkommt, und auch vergleichende Angaben darüber, wie viel mal häufiger eine bestimmte unerwünschte Wirkung bei einem Arzneimittel im Verhältnis zu einem anderen Medikament auftritt, sind anhand solcher Berichte kaum möglich.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92205826

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19050312	Age: 86 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased	19920211				Unknown
2 Prothrombin increased	19920210				Unknown
3 Phosphatase alkaline increased	19920211				Unknown
4 Thrombocythaemia	19920210				Unknown
5 Nausea	19920210				not recovered/not resolved
6 Melaena	199202				Unknown
7 Haematemesis	19920210				not recovered/not resolved
8 Anaemia	19920210				Unknown
9 Dyspepsia	19920206				Unknown
10 GI haemorrhage	19920210				not recovered/not resolved
11 Duodenal ulcer	19920210				not recovered/not resolved
12 BUN increased	19920210				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren 50 diclofenac sodium	Oral	175 Mg milligram(s)/Week	1989-199202	Backache, unspecified
			-	

Drugs (concomitant)

2 arelix piretanide	Unknown	/	-	
3 bayotensin nitrendipine	Oral	/	-199202	Hypertension arterial
4 lanitop metildigoxin	Unknown	/	-	
5 ossofortin calcium gluconate; calcium phosphate; colecalciferol	Oral	/	-199202	Osteoporosis
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n. schenkelhalsfraktur
Glaucoma, unspecified			k.A.	concomitant disease
Disorders of sacrum			k.A.	concomitant disease
Other specified forms of osteochondropathy			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92205829

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19200407	Age: 71 Year	weight (kg): 82	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oliguria	199202				Unknown
2 Eosinophilia	199202				Unknown
3 Itchy skin	19920204				Unknown
4 NPN increased	19920203				Unknown
5 Rash maculo-papular	19920204				Unknown
6 Lymphopenia	19920205				Unknown
7 BUN increased	19920203				Unknown
8 LDH increased	19920205				Unknown
9 Gamma-GT increased	19920205				Unknown
10 Albuminuria	19920205				Unknown
11 Urine abnormal	19920205				Unknown
12 Anaemia	19920205				Unknown
13 Nephritis interstitial	19920203				recovering/resolving
14 Renal failure acute	19920203				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 baypen 5 g mezlocillin sodium	Intravenous (not otherwis	015 G gram(s)/Day	19920129-19920205	Postoperative infection, not elsewhere classified
2 voltaren diclofenac sodium	Rectal	150 Mg milligram(s)/Day	19920127-19920204	Other specified complications of procedures, not elsewhere

Drugs (concomitant)

3 adalat t 10 nifedipine	Oral	30 Mg milligram(s)/	19920115-19920119	-
4 adumbran oxazepam	Unknown	8 Df dosage form/	19920119-19920214	-
5 aniflazym serrapeptase	Oral	010 Mg milligram(s)/Day	19920128-19920204	-
6 ben-u-ron paracetamol	Rectal	1 G gram(s)/	19920116-19920116	-
7 liquemin n 5000 heparin sodium	Subcutaneous	/	19920114-19920205	-
8 multibionta forte ascorbic acid; biotin; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Oral	002 Df dosage form/Day	19920128-19920203	-
9 nolvadex 30 tamoxifen citrate	Oral	001 Df dosage form/Day	19920121-	-
10 omsat forte sulfamethoxazole; trimethoprim	Oral	003 Df dosage form/Day	19920117-19920120	Postoperative infection, not elsewhere classified
11 staphylex flucloxacillin sodium	Intravenous (not otherwis	006 G gram(s)/Day	19920116-19920120	Postoperative infection, not elsewhere classified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cyst of kidney, acquired			k.A.	concomitant disease
Malignant neoplasm of female breast, unspecified			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n. basaliom-op

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93008468

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19751110 Age: 16 Year weight (kg): 56 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	19921012		7 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	900 Mg milligram(s)/Day	19921009-19921021	-
2 sobelin 300 clindamycin hydrochloride	Oral	150 Mg milligram(s)/Day	19921006-19921021	Chronic osteomyelitis
3 thrombophob-5000 heparin sodium	Subcutaneous	015 Kiu iu (1000s)/Day	19920918-19921021	Embolism and thrombosis of unspecified site

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Neoplasm of unspecified nature of bone, soft tissue, and sk			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004621

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth:	Age: 78 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19920126	deathcause: Anaphylactic shock
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting	199107				Unknown
2 Diarrhoea	199107				Unknown
3 Asthenia	199107				Unknown
4 Jaundice	199107				Unknown
5 Urine abnormal	199107				Unknown
6 Anaphylactic shock	199107				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Intramuscular	/	19910729-19920126	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93018845

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: 19231103 Age: 69 Year weight (kg): 54 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	19930428		7 Day		recovered/resolved
2 Bilirubin increased	19930428		7 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 50 stada diclofenac sodium	Oral	100 Mg milligram(s)/Day	19930422-19930428	Pain in joint

Drugs (concomitant)

2 diazepam woelm 2 diazepam	Oral	006 Mg milligram(s)/Day	19930422-	Pain in joint
3 folsan folic acid	Oral	015 Mg milligram(s)/Day	19930421-19930429	
4 liquemin 5.000 heparin sodium	Subcutaneous	010 Kiu iu (1000s)/Day	19930412-	
5 paracetamol-ratiopharm I paracetamol	Oral	1.8 G gram(s)/Day	19930420-19930430	Pain in joint

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hemiplegia, unspecified			k.A.	concomitant disease
Embolism and thrombosis of unspecified artery			k.A.	concomitant disease
Cachexia			k.A.	concomitant disease
Emphysema			k.A.	concomitant disease
Hypertension arterial			k.A.	concomitant disease
Dysphagia			k.A.	concomitant disease
Aphasia			k.A.	concomitant disease
Acute, but ill-defined, cerebrovascular disease			k.A.	concomitant disease
Spastic hemiplegia			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93018856

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19120304	Age: 80 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19930207	deathcause: Dyspnea
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency	19930125				Unknown
2 Petechiae	19930125				Unknown
3 Water excessive loss of	19930125				Unknown
4 Semi-coma	19930125				Unknown
5 Thrombocytopenia	19930125				Unknown
6 Hepatic function abnormal	19930125				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 500 stada acetylsalicylic acid	Oral	/	-	-
2 diclophlogont diclofenac sodium	Oral	100 Mg milligram(s)/Day	-19930125	-
3 triamteren comp.-ratiopharm hydrochlorothiazide; triamterene	Oral	001 Df dosage form/Day	19920208-19930125	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Heart failure, unspecified			k.A.	concomitant disease
Cerebral atherosclerosis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93014237

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19420602 Age: 51 Year weight (kg): 63 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis cholestatic	19930823				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Rectal	150 Mg milligram(s)/Day	19930731-19930823	Lumbago

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93016155

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19240612 Age: 69 Year weight (kg): 61 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	19931011		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	100 Mg milligram(s)/Day	- 2 -	Osteoarthritis, localized, not specified whether primary or

Drugs (concomitant)

2 coversum perindopril erbumine	Oral	001 Df dosage form/Day	- 2 -	Heart failure, unspecified
3 glutril glibornuride	Oral	001 Df dosage form/Day	- 6 -	Diabetes mellitus

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93016240

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 72 Year weight (kg): 74 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased	199307				recovered/resolved
2 SGOT increased	199307				recovered/resolved
3 SGPT increased	199307				recovered/resolved
4 Phosphatase alkaline increased	199307				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo 50 von ct diclofenac sodium	Oral	/	-	Osteoarthritis, localized, not specified whether primary or
2 indomet-ratiopharm 25 indometacin	Oral	/	-	
3 indomet-ratiopharm 50 indometacin	Oral	/	-	Osteoarthritis, localized, not specified whether primary or

Drugs (concomitant)

4 clexane 40 enoxaparin	Subcutaneous	/	-
5 heparin-natrium heparin sodium	Subcutaneous	/	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93016241

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 72 Year weight (kg): 65 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	199307				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clexane 40 enoxaparin	Subcutaneous	/Day	199307- -	
2 diclofenac-ratiopharm 50 diclofenac sodium	Oral	/Day	199307- -	Osteoarthritis, localized, not specified whether primary or
3 indomet-ratiopharm 50 indometacin	Oral	/Day	199307- -	Osteoarthritis, localized, not specified whether primary or

Drugs (concomitant)

4 heparin-natrium heparin sodium	Subcutaneous	/Day	199307- -
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Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94014126

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19310404 Age: 62 Year weight (kg): 93 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxic effect	19931012		13 Day		recovering/resolving
2 Stomatitis	19931012		13 Day		not recovered/not resolved
3 SGPT increased	19931012		13 Day		recovering/resolving
4 SGOT increased	19931012		13 Day		recovering/resolving
5 Erythroderma	19931012		13 Day		recovering/resolving
6 Gamma-glutamyltransferase incr.	19931012		13 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 50 heumann diclofenac sodium	Oral	150 Mg milligram(s)/Day	19930921-19930927	Gout acute
			-	

Drugs (concomitant)

2 allopurinol 300 heumann allopurinol	Oral	300 Mg milligram(s)/Day	19930921-19931012	Gout acute
			-	
3 colchicum-dispert colchicine	Oral	006 Df dosage form/Day	19930921-19930923	Gout acute
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcoholic liver damage, unspecified			k.A.	concomitant disease
Other and unspecified hyperlipidaemia			k.A.	concomitant disease
Hepatitis, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94900361

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19231109	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	19940324		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac heumann diclofenac sodium	Oral	100 Mg milligram(s)/Day	19940309-19940324	Low back pain
			-	

Drugs (concomitant)

2 calcium calcium	Subcutaneous	100 lu international unit(s)	19940309-19940319	Osteoporosis
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont. k.A.	Comment
Other and unspecified hyperlipidaemia				concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94003884

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19460701 Age: 47 Year weight (kg): 43 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	19940204				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren 50 diclofenac sodium	Oral	150 Mg milligram(s)/Day	19940118-19940129	Unspecified arthropathy
2 voltaren 50 diclofenac sodium	Rectal	150 Mg milligram(s)/Day	19940129-19940214	Unspecified arthropathy

Drugs (concomitant)

3 imurek azathioprine	Oral	075 Mg milligram(s)/Day	199312-	Tropical sprue
4 presomen estrogens conjugated	Oral	001 Df dosage form/Day	1991-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other chronic nonalcoholic liver disease			k.A.	concomitant disease
Unspecified intestinal malabsorption			k.A.	concomitant disease
Osteoporosis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

case: DE-BFARM-94003919

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19430910 Age: 50 Year weight (kg): 86 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice	19940201				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	100 Mg milligram(s)/Day	199401-	Other specified complications of
diclofenac			-	procedures, not elsewhere

Drugs (concomitant)

2	enflurane enflurane	Intravenous (not otherwis /	19940118-19940118	
3	lansoprazol lansoprazole	Oral	030 Mg milligram(s)/Day	Oesophagitis NOS

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94007120

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: 19791021	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels					recovered/resolved
2 Alveolitis allergic					recovered/resolved
3 Exanthema					recovered/resolved
4 Hepatosi					recovered/resolved
5 Vasculitis allergic					recovered/resolved
6 Thrombocytopenia					recovered/resolved
7 Fever					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 azulfidine sulfasalazine	Oral	001 G gram(s)/Day	19940826-19940915	Juvenile chronic polyarthritis
2 diclofenac 50 mg diclofenac sodium	Oral	075 Mg milligram(s)/Day	19940903-19940915	Juvenile chronic polyarthritis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other viral infection in conditions classified elsewhere and of			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94006009

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 35 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGOT increased	19940826		8 Week		recovered/resolved
2 SGPT increased	19940826		8 Week		recovered/resolved
3 LDH increased	19940826		8 Week		recovered/resolved
4 Gamma-GT increased	19940826		8 Week		recovered/resolved
5 Hepatitis toxic	199408		8 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	100 Mg milligram(s)/	19940824-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Obesity			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

case: DE-BFARM-94901890

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Male Birth: 19140714 Age: 80 Year weight (kg): BMI int: Age fetus:

Reactions

	MedDRA LLT	Begin	End	duration	TtO	outcome
1	Allergic reaction	1994	1002			recovered/resolved
2	Oedema NOS	1994	1012			recovered/resolved
3	Presyncope	1994	1012			recovered/resolved
4	Raised liver enzymes	1994	1012			recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allvoran	Intramuscular	7*1 Df dosage form/	-	Other unspecified back
diclofenac sodium			-	disorders

Drugs (concomitant)

2	nifedipine	Oral	030 Mg milligram(s)/Day	-
	nifedipine			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: nieren-insuffizienz kompensiert im stadi
Nephrolithiasis			k.A.	concomitant disease
Neoplasm of unspecified nature of other genitourinary organ			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003255

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGOT increased					Unknown
2 Hepatitis cholestatic					Unknown
3 SGPT increased					Unknown
4 Icterus					Unknown
5 Hepatitis toxic					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Unknown	/	-	-
2 diclofenac diclofenac	Unknown	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95001046

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic dysfunction nonicteric					Unknown
2 Nausea					Unknown
3 Vomiting					Unknown
4 Pain epigastric					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fluctin	Oral	/	-	
fluoxetine hydrochloride			-	
2 ludiomil 75	Oral	/	-	
maprotiline hydrochloride			-	
3 tavor	Oral	/	-	
lorazepam			-	
4 voltaren 100	Oral	/	-	
diclofenac sodium			-	

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95900316

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19391105 Age: 54 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGOT increased	199311				Unknown
2 SGPT increased	199311				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 capozide 25 captopril; hydrochlorothiazide	Oral	/	-	-
2 diclofenac diclofenac	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95001648

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Eosinophilia					Unknown
2 Liver function tests abnormal NOS					Unknown
3 Fever					Unknown
4 Allergic reaction					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren injektionsloesung diclofenac sodium	Intramuscular	/	-	-

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003777

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19321118 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic disease NOS					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005061

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19560506 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time prolonged					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 marcumar phenprocoumon	Oral	/	-	-
2 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005911

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19360912 Age: 56 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased					Unknown
2 SGPT increased					Unknown
3 Toxic epidermal necrolysis	19921122				Unknown
4 Stevens Johnson syndrome	19921122				Unknown
5 Lymphopenia	19921126				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin plus c acetylsalicylic acid; ascorbic acid acetylsalicylic acid	Oral	/	-	-
2 aspirin 100	Oral	/	-	-
3 limptar aminophylline; quinine sulfate	Oral	/	-	-
4 muskel trancopal chlormezanone	Oral	/	-	-
5 normoc bromazepam	Oral	/	-	-
6 reparil for injection escin sodium	Intravenous (not otherwis	/	-	-
7 tramal tramadol hydrochloride	Intravenous (not otherwis	/	-	-
8 tridin calcium citrate; calcium gluconate; sodium fluorophosphate	Oral	/	-	-
9 valium diazepam	Intravenous (not otherwis	/	-	-
10 vitamin-b calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Intravenous (not otherwis	/	-	-
11 voltaren diclofenac sodium	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95007023

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19270718 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema					Unknown
2 Elevated liver enzyme levels					Unknown
3 Phosphatase alkaline increased					Unknown
4 Cholestasis					Unknown
5 Hepatitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 azulfidine ra	Oral	/	-	
sulfasalazine			-	
2 diclofenac 100 mg supp.	Rectal	/	199511-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96900455

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96004031

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19370507 Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestasis	19960125				Unknown
2 Hepatocellular damage NOS	19960125				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96015546

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Circulatory failure	1996				Unknown
2 Renal failure acute	1996				Unknown
3 Hypersensitivity	1996				Unknown
4 Oedema of extremities	1996				Unknown
5 Rhabdomyolysis	1996				Unknown
6 Hepatic function abnormal	1996				Unknown
7 Vascular disorder	1996				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bactrim sulfamethoxazole; trimethoprim	Unknown	/	-	-
2 diclofenac diclofenac	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96016351

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 74 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	199607				Unknown
2 Bilirubin increased	199607				Unknown
3 Haemolytic autoimmune anaemia DCN	199607				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002136

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19250220 Age: 71 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	19960725				Unknown
2 Hepatic enzymes increased	19960725				Unknown
3 Stevens Johnson syndrome	19960725				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 briserin n	Oral	/	-	
clopamide; reserpine			-	
2 voltaren emulgel	Topical	/	-	
diclofenac diethylamine			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97003877

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19631228 Age: 33 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	19970602				Unknown
2 Ill feeling	19970602				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97004458

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth:	Age: 30 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-glutamyltransferase incr.	19970423				recovered/resolved
2 Serum glutamic-pyruvic ta incr	19970423				recovered/resolved
3 Serum glutamic-oxaloacetic ta inc	19970423				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 skid minocycline hydrochloride	Oral	100 Mg milligram(s)/Day	19970423-19970502	-
2 voltaren diclofenac sodium	Oral	/	-	-
3 zyprexa olanzapine	Oral	020 Mg milligram(s)/Day	-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97009212

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19310829 Age: 65 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis cholestatic	19961031				Unknown
2 Icterus	19961031				Unknown
3 Upper abdominal pain	199610				Unknown
4 Itching	199610				Unknown
5 Urine discolouration	19961026				Unknown
6 Faeces discoloured	19961026				Unknown
7 Weight loss					Unknown
8 Elevated liver enzyme levels					Unknown
9 Bilirubin increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/	-	
2 voltaren diclofenac sodium	Rectal	/	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97009456

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	199711				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 delix ramipril	Oral	/	-	
2 diclophlogont diclofenac sodium	Unknown	/	-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97009891

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: 19391006	Age: 55 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19950821				Unknown
2 Vomiting	19950821				Unknown
3 Anaemia haemolytic	19950821				Unknown
4 Jaundice	19950821				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophlogont ampullen	Unknown	/	-	
diclofenac sodium			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98002558

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19040820	Age: 92 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cardiac failure	19970603				Unknown
2 Circulatory failure	19970603				Unknown
3 Creatinine increased	199706				Unknown
4 Haemoglobin decreased	199706				Unknown
5 Prothrombin time increased	199706				Unknown
6 C-reactive protein positive	199706				Unknown
7 Renal failure acute	19970603				Unknown
8 Cholinesterase decreased	199706				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren 100 zaepfchen	Rectal	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98003361

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
YES

Patient

Initials: Sex: Female Birth: Age: 34 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hyperbilirubinaemia	19901231				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 azulfidine	Oral	/	-	
sulfasalazine			-	
2 diclofenac	Oral	/	-	
diclofenac			-	
3 haldol-janssen	Oral	/	-	
haloperidol			-	
4 zantic	Oral	/	-	
ranitidine hydrochloride			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98004671

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Electrolyte abnormality	19980207				Unknown
2 Creatinine blood increased	19980207				Unknown
3 Serum glutamic-pyruvic ta incr	19980207				Unknown
4 Serum glutamic-oxaloacetic ta inc	19980207				Unknown
5 Amylase increased	19980207				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina 50	Oral	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98004436

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	199806				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Intravenous (not otherwis /		-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98004998

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19530121 Age: 44 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Inflammatory oedema reaction	199801				Unknown
2 Pancreas enzymes increased	199801				Unknown
3 Raised liver enzymes	199801				Unknown
4 Skin vasculitis NOS	199801				Unknown
5 Abdominal discomfort	199801				Unknown
6 Pancreatitis	19980108				Unknown
7 Pustular rash	199801				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	/	-	
diclofenac			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006038

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: 27 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19980324				Unknown
2 Hepatic enzymes increased	19980324				Unknown
3 Hepatitis cholestatic	19980324				Unknown
4 Nausea	19980324				Unknown
5 Abdominal pain	19980324				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006058

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19630827 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Creatinine increased					Unknown
2 Gamma GT increased					Unknown
3 Serum glutamic-pyruvic ta incr					Unknown
4 Serum glutamic-oxaloacetic ta inc					Unknown
5 Exanthema					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	
diclofenac			-	
2 suprax	Unknown	/	-	
cefixime trihydrate			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007225

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19230603 Age: 75 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Ascites	199808				Unknown
2 Anaemia	199808				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren	Oral	/	-	
diclofenac sodium			-	
2 voltaren zaepfchen	Rectal	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98009739

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19120906 Age: 86 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sugar urinary					Unknown
2 Blood sugar increased					Unknown
3 Diabetes mellitus					Unknown
4 Gastritis					Unknown
5 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat	Oral	/	-	
diclofenac resinate			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98009041

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: 84 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	19980408				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 moduretik amiloride hydrochloride; hydrochlorothiazide	Oral	/	-	-
2 sostril ranitidine hydrochloride	Unknown	/	-	-
3 voltaren diclofenac sodium	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002211

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19590823 Age: 39 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hyperbilirubinaemia	199901				Unknown
2 Icterus	199901				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000279

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19190506	Age: 79 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Thrombopenia	199901				Unknown
2 Sepsis	19990120				Unknown
3 Clotting time lengthened	19990118				Unknown
4 Creatinine increased	19990118				Unknown
5 Slurred speech	19990118				Unknown
6 LDH increased	19990118				Unknown
7 Serum glutamic-oxaloacetic ta inc	19990118				Unknown
8 Diarrhea	19990118				Unknown
9 Creatine phosphokinase increased	19990118				Unknown
10 DIC	19990118				Unknown
11 Arrhythmia	19990119				Unknown
12 Tachycardia	19990119				Unknown
13 Oliguria	19990119				Unknown
14 Leucopenia	199901				Unknown
15 Embolism cerebral	19990118				Unknown
16 Pulmonary embolism	19990118				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 didronel-kit etidronate disodium	Oral	/	-	-
2 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000802

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19510305 Age: 47 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased	19981213				Unknown
2 SGPT increased	19981213				Unknown
3 Epidermal necrolysis	19981212				Unknown
4 Stevens Johnson syndrome	19981212				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allobeta 300	Oral	/	-	
allopurinol			-	
2 diclofenbeta uno	Oral	/	-	
diclofenac sodium			-	
3 herz ass-ratiopharm	Oral	/	-	
acetylsalicylic acid			-	
4 meditonsin h	Oral	/	-	
homeopatics nos			-	
5 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000714

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19500129 Age: 49 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19990311				Unknown
2 Renal failure NOS	19990311				Unknown
3 Icterus	19990311				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	
diclofenac			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99901293

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: 19330509	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased	19970808				Unknown
2 Alkaline phosphatase serum incr	19970808				Unknown
3 SGPT increased	19970808				Unknown
4 SGOT increased	19970808				Unknown
5 Multiple organ failure	19970808				Unknown
6 Abscess					Unknown
7 Sepsis	19970808				Unknown
8 Bilirubin increased	19970808				Unknown
9 Thrombocytopenia	19970804				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo-divido long	Oral	/	-	
diclofenac sodium			-	
2 voltaren	Oral	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99005284

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19381005 Age: 59 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fasciitis necrotising	19980611				Unknown
2 Leukocytosis	19980611				Unknown
3 Gamma GT increased	19980611				Unknown
4 Bilirubin increased	19980611				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm 75 sl diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001898

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19680101	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19941111	deathcause: Hepatic coma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sepsis	19941101				Unknown
2 Stomatitis	19941101				Unknown
3 Coma hepatic	19941101				Unknown
4 Blistering	19941101				Unknown
5 Exanthema	19941101				Unknown
6 Stevens Johnson syndrome	19941101				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atosil tropfen promethazine hydrochloride	Oral	/	19941025-19941026	Unspecified functional disorder of stomach
2 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	19941023-19941029	Unspecified infectious and parasitic diseases
3 ben-u-ron paracetamol	Rectal	/	19941021-19941023	Pyrexia of unknown origin
4 bifiteral lactulose	Oral	/	19941023-19941031	Unspecified chronic liver disease without mention of
5 dopamin dopamine hydrochloride	Intravenous (not otherwis	/	19941027-	Renal failure, unspecified
6 furorese furosemide	Oral	/	19941028-19941029	Oedema
7 humatin paromomycin sulfate	Oral	/	19941023-19941031	Encephalopathy, unspecified
8 konakion phytomenadione	Unknown	/	-	Deficiency of vitamin K
9 lasix furosemide	Intravenous (not otherwis	/	19941024-19941031	Oedema
10 novalgin injektionsloesung metamizole	Intravenous (not otherwis	/	19941030-19941030	Pyrexia of unknown origin
11 quantalan colestyramine	Oral	/	19941031-	Cirrhosis of liver without mention of alcohol
12 rocephin ceftriaxone sodium	Intravenous (not otherwis	/	19941030-	Unspecified infectious and parasitic diseases
13 voltaren diclofenac sodium	Rectal	/	19941101-19941101	Other general symptoms

Drugs (concomitant)

14 l-polamidon methadone hydrochloride	Unknown	/	-	
15 maalox aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Unknown	/	19941005-19941006	Unspecified functional disorder of stomach

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug dependence of morphine type			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001908

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19130526 Age: 81 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	19941212				Unknown
2 LDH increased					Unknown
3 Erythema multiforme severe	19941212				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	
diclofenac			-	
2 esidrix	Oral	/	-	
hydrochlorothiazide			-	
3 meaverin	Intracardiac	/	-	
mepivacaine hydrochloride			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001912

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19561020 Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukocytosis	19940729				Unknown
2 Leucopenia	19940729				Unknown
3 Stevens Johnson syndrome	19940727				Unknown
4 LDH increased	19940729				Unknown
5 SGPT increased	19940729				Unknown
6 SGOT increased	19940729				Unknown
7 Gamma GT increased	19940729				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin	Unknown	/	-	
amoxicillin			-	
2 diclofenac-ratiopharm sf	Intramuscular	/	-	
diclofenac sodium			-	
3 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002655

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19390413 Age: 60 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver enlargement	199904				Unknown
2 Peliosis hepatis	199904				Unknown
3 Hepatic failure	199904				Unknown
4 Angiosarcoma	199904				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 captopril	Oral	/	-	
captopril			-	
2 voltaren	Oral	/	-	
diclofenac sodium			-	

Drugs (concomitant)

3 dysto-loges	Oral	/	-	
homeopatics nos			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99005321

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth:	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain					Unknown
2 Pleuritis					Unknown
3 Bronchopneumonia					Unknown
4 Hepatic disease					Unknown
5 Mediastinitis					Unknown
6 Disease of spleen, unspecified					Unknown
7 Pericarditis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm sf	Unknown	/	-	
diclofenac sodium			-	

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99901526

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic cirrhosis	199907				Unknown
2 Hepatic enzymes increased	199907				Unknown
3 Gamma-GT increased	199907				Unknown
4 Bilirubin increased	199907				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexamethasone dexamethasone	Unknown	/	-	-
2 voltaren diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99901604

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19531007 Age: 45 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased	1999				Unknown
2 Phosphatase alkaline increased	1999				Unknown
3 Elevated liver enzyme levels	1999				Unknown
4 Cholestasis	19990902				Unknown
5 Bilirubin increased	1999				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Unknown	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99003707

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19240401	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain	19930118				Unknown
2 Ill feeling	19930118				Unknown
3 Pleurisy	19930118				Unknown
4 Disease of spleen, unspecified	19930118				Unknown
5 Hepatitis	19930118				Unknown
6 Pneumonia	19930118				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	/	-	
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99007116

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19481116 Age: 50 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rigors	19990223				Unknown
2 Nausea aggravated	19990223				Unknown
3 Hepatic damage	19990228				Unknown
4 Abscess	19990228				Unknown
5 Raised liver enzymes	19990228				Unknown
6 Fever	19990223				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 monoflam	Unknown	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004067

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19300607 Age: 68 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	19990227				Unknown
2 Creatinine increased	19990227				Unknown
3 Stevens Johnson syndrome	19990227				Unknown
4 Lymphopenia	19990227				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc 200 acetylcysteine	Oral	/	-	-
2 adumbran oxazepam	Oral	/	-	-
3 arlevert cinnarizine; dimenhydrinate	Oral	/	-	-
4 bepanthen dexpanthenol	Oral	/	-	-
5 furorese furosemide	Oral	/	-	-
6 gastrosil metoclopramide hydrochloride	Oral	/	-	-
7 globocef cefetamet pivoxil hydrochloride	Oral	/	-	-
8 godamed acetylsalicylic acid	Oral	/	-	-
9 voltaren dispers diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99007653

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis C					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Unknown	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00000324

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19570218 Age: 41 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	19980310				Unknown
2 SGOT increased	19980310				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001502

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19200113 Age: 79 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19991104				Unknown
2 Toxic epidermal necrolysis	19991104				Unknown
3 SGOT increased	19991106				Unknown
4 Lymphocytes raised	19991108				Unknown
5 Conjunctivitis					Unknown
6 Blisters					Unknown
7 SGPT increased	19991106				Unknown
8 Gamma GT increased	19991106				Unknown
9 LDH increased	19991108				Unknown
10 Fever	19991030				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arelix ace piretanide; ramipril	Oral	/	-	-
2 carbostesin bupivacaine hydrochloride	Intravenous (not otherwis	/	-	-
3 claforan cefotaxime sodium	Intravenous (not otherwis	/	-	-
4 dytide h hydrochlorothiazide; triamterene	Oral	/	-	-
5 mst 10 morphine sulfate	Oral	/	-	-
6 novaminsulfon metamizole	Oral	/	-	-
7 orelox cefopodoxime proxetil	Oral	/	-	-
8 pk-merz amantadine sulfate	Intravenous (not otherwis	/	-	-
9 tegretal carbamazepine	Oral	/	-	-
10 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00900102

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic function abnormal	20000327				Unknown
2 Porphyria	20000327				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00006090

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: 19631012 Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	2000				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 tetrazepam tetrazepam	Unknown	/	-	
2 voltaren resinat diclofenac resinate	Oral	/	-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00003909

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19410214 Age: 59 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	20000512				Unknown
2 Icterus	200005				Unknown
3 Nausea aggravated	200004				Unknown
4 Hepatic failure	20000516				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	
diclofenac			-	
2 vioxx 12,5 mg tabletten	Oral	/	-	
rofecoxib			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00004663

Serious Yes	Death Yes	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19410220	Age: 59 Year	weight (kg): 35	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epigastric pain	20000417				Unknown
2 Pulmonary embolism	20000512		2 Day		Unknown
3 Vomiting	20000417				Unknown
4 Appetite suppressed	20000417				Unknown
5 Nausea	20000417				Unknown
6 Liver failure	20000417				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arava leflunomide	Oral	010 Mg milligram(s)/Day	20000120-20000508	Rheumatoid arthritis
2 falithrom phenprocoumon	Oral	/	20000406-20000501	Phlebitis and thrombophlebitis of deep vessels of lower
3 lantarel methotrexate sodium	Oral	015 Mg milligram(s)/Day	199809-20000510	Rheumatoid arthritis
4 rewodina diclofenac sodium	Oral	150 Mg milligram(s)/Day	199910-20000508	

Drugs (concomitant)

5 antra 40 omeprazole	Oral	040 Mg milligram(s)/Day	199806-20000510	
6 calcimagon-d3 calcium carbonate; colecalciferol	Oral	/	200003-20000510	Osteoporosis
7 didronel etidronate disodium	Oral	/	199806-20000510	Osteoporosis
8 valoron n retard naloxone hydrochloride; tilidine hydrochloride	Oral	100 Mg milligram(s)/Day	199903-20000510	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n. Oberschenkelamputation rechts
unbekannt			k.A.	concomitant disease: z.n. rezidivierenden Lungenembolien
unbekannt			k.A.	concomitant disease: z.n. rezidivierenden tiefen Beinvenenthrombosen
unbekannt			k.A.	concomitant disease: z.n. Unterschenkelamputation links
Peripheral vascular disease, unspecified			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biological product			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00003842

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19480417	Age: 51 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Weakness generalized	20000410				recovered/resolved
2 Itching	20000410				recovered/resolved
3 Cholestasis	20000410				recovered/resolved
4 Icterus	20000410				recovered/resolved
5 Hepatitis	20000410				recovered/resolved
6 Elevated liver enzyme levels	20000410				recovered/resolved
7 Nausea	20000410				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 beloc zok metoprolol succinate	Oral	048 Mg milligram(s)/Day	1990- -	Hypertension arterial
2 falithrom phenprocoumon	Oral	1.5 Mg milligram(s)/	1981- -	Aortic valve disorders
3 rewodina diclofenac sodium	Oral	/	- -	Angina pectoris
4 viox 25 mg rofecoxib	Oral	025 Mg milligram(s)/Day	20000321-20000410 -	Osteoarthritis, unspecified whether generalized or

Drugs (concomitant)

5 estrone sodium sulfate estrone sodium sulfate	Oral	0.3 Mg milligram(s)/Day	1993- -	
6 Isosorbidmononitrat isosorbide mononitrate	Oral	060 Mg milligram(s)/Day	1994-200003 -	Angina pectoris
7 medrogestone medrogestone	Oral	005 Mg milligram(s)/Day	1993- -	
8 quinapril quinapril	Oral	/	1995-200004 -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified otitis media			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00008061

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19050115	Age: 95 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal failure acute	20001124		5 Day		recovered/resolved
2 Hepatic damage	20001124				recovered/resolved
3 Anuria	20001124				recovered/resolved
4 Vomiting	20001124				recovered/resolved
5 Malaise	20001124				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 isoptin verapamil hydrochloride	Oral	009 Df dosage form/Day	20001122-20001124	Tachycardia, unspecified
2 voltaren diclofenac sodium	Unknown	25 Mg milligram(s)/	20001122-20001122	Low back pain

Drugs (concomitant)

3 aldactone-a spironolactone	Unknown	/	-	
4 diart azosemide	Unknown	/	-	
5 myonal eperisone hydrochloride	Oral	50 Mg milligram(s)/	20001124-20001124	
6 naboal - slow release diclofenac sodium	Unknown	/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Heart failure, unspecified			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: formen der leberzirrhose ohne alkohol,so
Unspecified intestinal obstruction			k.A.	concomitant disease

Primary source

Qualification:	observestudytype:
Literature:	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01001224

Serious Yes	Death Yes	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19371121	Age: 63 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20010304	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gastritis acute					recovered/resolved with sequel
2 Pleural effusion					recovered/resolved with sequel
3 Pneumonia					recovered/resolved with sequel
4 Multiple organ failure					recovered/resolved with sequel
5 Renal failure acute					recovered/resolved with sequel
6 Sepsis					recovered/resolved with sequel
7 Leucopenia	20010215				recovered/resolved with sequel
8 Hepatic failure					recovered/resolved with sequel
9 Fasciitis necrotising	20010215				recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexaflam n lichtenstein dexamethasone sodium phosphate	Intramuscular	4 Mg milligram(s)/	20010212-20010212	Unspecified musculoskeletal disorders and symptoms
2 monoflam sf diclofenac sodium	Intramuscular	75 Mg milligram(s)/	20010212-20010212	Unspecified musculoskeletal disorders and symptoms

Drugs (concomitant)

3 aciclovir aciclovir	Oral	/Day	20010213-	Herpes zoster
4 virunguent p idoxuridine; prednisolone	Topical	/Day	20010213-	Herpes zoster

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			k.A.	concomitant disease
Coronary atherosclerosis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01001974

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19300518 Age: 70 Year weight (kg): 85 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20010308				recovered/resolved
2 Purpura	20010308				recovered/resolved
3 Petechiae	20010308				recovered/resolved
4 Angioneurotic oedema	20010308				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	150 Mg milligram(s)/Day	20010307-20010308	Arthralgia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other noninfective gastroenteritis and colitis			k.A.	concomitant disease
Rotator cuff syndrome of shoulder and allied disorders			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01002206

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19400823	Age: 60 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Antibodies drug specific					Unknown
2 Hb decreased	20010310		10 Day		recovered/resolved
3 Coombs direct test positive					Unknown
4 Icterus	20010310		10 Day		recovered/resolved
5 Haemolytic autoimmune anaemia DCP	20010310		10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac "ratiopharm" diclofenac sodium	Oral	004 Df dosage form/Day	200103- -	Pain in joint

Drugs (concomitant)

2 concor 5mg bisoprolol fumarate	Oral	005 Mg milligram(s)/Day	- -	
3 togonal acetylsalicylic acid; lithium citrate; quinine citrate	Oral	1 Df dosage form/ -	- -	Headache
4 verapamil-ratiopharm verapamil	Oral	001 Df dosage form/Day	- -	

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01005522

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: 19700609	Age: 30 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20010502				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	002 Df dosage form/Day	-	Pain in joint
diclofenac			-	
2 fumaderm	Oral	001 Df dosage form/Day	-	Other psoriasis
dimethyl fumarate; ethyl fumarate calcium; ethyl fumarate magnesium; ethyl fumarate zinc			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Paralysis, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-01005924

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Male Birth: 19531119 Age: 47 Year weight (kg): 88 BMI int: Age fetus:

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased	20010720				not recovered/not resolved
2 Autoimmune disorder NOS	20010720				not recovered/not resolved
3 Haemoglobin decreased	20010720				not recovered/not resolved
4 Anaemia haemolytic	20010720				not recovered/not resolved

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	140 Mg milligram(s)/Day	20010706-20010720	Backache, unspecified

MedDRA-term	Begin	End	Cont.	Comment
Sleep apnea syndrome			k.A.	concomitant disease

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01006143

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19200722	Age: 80 Year	weight (kg): 76	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia					recovered/resolved
2 Gamma GT increased					recovered/resolved
3 Exanthema	20010330		6 Hour		recovered/resolved
4 Blood pressure dropped	20010330		6 Hour		recovered/resolved
5 Collapse circulatory	20010330		6 Hour		recovered/resolved
6 Anaphylactic shock	20010330		6 Hour		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexahexal 4 dexamethasone sodium phosphate	Intramuscular	/	20010330- -	Unspecified polyarthropathy or polyarthritis
2 diclac diclofenac sodium	Intramuscular	/	20010330- -	Other unspecified back disorders

Drugs (concomitant)

3 azur compositum caffeine; codeine phosphate; paracetamol	Rectal	001 Df dosage form/Day	-	Other unspecified back disorders
4 endak 5 carteolol hydrochloride	Oral	001 Df dosage form/Day	-	Hypertension arterial
5 valoron n retard 50 naloxone hydrochloride; tilidine hydrochloride	Oral	001 Df dosage form/Day	-	Other unspecified back disorders

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Osteoporosis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01007631

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19870616	Age: 14 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20010818				recovering/resolving
2 Icterus	20010818				recovering/resolving
3 Hepatitis	20010818				recovering/resolving
4 Water excessive loss of	20010818				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprobay ciprofloxacin hydrochloride	Unknown	/	20010816-20010818	Colitis, enteritis, and gastroenteritis of presumed
2 lansoprazole lansoprazole	Unknown	/	-	-
3 talcid hydrotalcite	Unknown	/	-	-
4 trimethobenzamide trimethobenzamide	Unknown	/	-	-
5 voltaren diclofenac sodium	Unknown	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified disorder of liver			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01009760

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: 19530713	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20011014				not recovered/not resolved
2 Icterus	20011014				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 75 mg	Oral	075 Mg milligram(s)/Day	20010919-20010927	
diclofenac sodium			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02000646

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: 19791119 Age: 22 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic function abnormal	20020105				not recovered/not resolved
2 Fibrillation cardiac	20020105				not recovered/not resolved
3 QT prolonged	20020105				not recovered/not resolved
4 Motor unrest compulsive	20020105				not recovered/not resolved
5 LDH increased	20020105				not recovered/not resolved
6 CRP positive	20020105				not recovered/not resolved
7 Creatine kinase increased	20020105				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	/	-	Other general symptoms

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified hypothyroidism			k.A.	concomitant disease
Unspecified disorder of thyroid			k.A.	concomitant disease
Pure hypercholesterolaemia			k.A.	concomitant disease
Renal failure, unspecified			k.A.	concomitant disease
Hyperuricaemia			k.A.	concomitant disease
Abnormal glucose tolerance test			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02002034

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: 19190808 Age: 80 Year weight (kg): 70 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Haematemesis	20000706				Unknown
2 Weakness generalized	20000706				Unknown
3 Prothrombin time increased	20000706				Unknown
4 Esophagitis	20000706		12 Day		Unknown
5 Pain	20000706				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	/	200004-20000707 -	Other general symptoms

Drugs (concomitant)

2 accupro 10 quinapril hydrochloride	Oral	001 Df dosage form/Day	1999-20000712 -	Essential hypertension, unspecified
3 aredia pamidronate disodium	Intravenous (not otherwis	1 Df dosage form/	20000113- -	Secondary malignant neoplasm of bone and bone marrow
4 atacand 16 mg candesartan cilexetil	Oral	001 Df dosage form/Day	-20000712 -	Essential hypertension, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z. n. blasendarmkatheter 11/1998
Other, mixed, or unspecified drug abuse			k.A.	concomitant disease
Sudden hearing loss, unspecified			k.A.	concomitant disease
Bone pain			k.A.	concomitant disease
Renal failure, unspecified			k.A.	concomitant disease
Hyperplasia of prostate			k.A.	concomitant disease
Malignant neoplasm of prostate			k.A.	concomitant disease
Hydronephrosis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02003761

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19220403	Age: 78 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time increased	20000913				Unknown
2 Hyperpotassaemia	20000913				Unknown
3 Anuria	20000913				Unknown
4 Creatinine blood increased	20000913				Unknown
5 Renal insufficiency	20000913				Unknown
6 Nausea	20000913				Unknown
7 Rectal bleeding	20000913				Unknown
8 Haematoma NOS	20000913				Unknown
9 Gastrointestinal tract bleed NOS	20000913				Unknown
10 Vomiting blood	20000913				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 claversal 250mg zaepfchen mesalazine	Rectal	250 Mg milligram(s)/Day	20000904-20000911	Colitis ulcerative
2 decaprednil prednisolone	Oral	030 Mg milligram(s)/Day	20000911-20000914	Colitis ulcerative
3 falithrom phenprocoumon	Oral	15 Mg milligram(s)/Week	20000403-20000911	Thrombosis
4 rewodina retard diclofenac sodium	Oral	002 Df dosage form/Day	200004-20000914	Osteoarthritis, unspecified whether generalized or

Drugs (concomitant)

5 accuzide 20 hydrochlorothiazide; quinapril hydrochloride	Oral	001 Df dosage form/Day	200004-	Hypertension arterial
6 allo 300 von ct allopurinol	Oral	001 Df dosage form/Day	1998-	Gout, unspecified
7 corvaton retard molsidomine	Oral	/Day	1996-	Coronary atherosclerosis
8 cynt 0,4 moxonidine	Oral	001 Df dosage form/Day	200004-	Hypertension arterial
9 isdn 20 retard riker isosorbide dinitrate	Oral	003 Df dosage form/Day	1996-	Coronary atherosclerosis
10 nitregamma 10 nitrendipine	Oral	003 Df dosage form/Day	200004-	Hypertension arterial

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02004061

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19411124	Age: 60 Year	weight (kg): 80	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Potassium deficiency	20020330		3 Day		recovered/resolved
2 Convulsive seizure	20020330		3 Day		recovered/resolved
3 Acidosis metabolic	20020330		3 Day		recovered/resolved
4 Bradycardia	20020330		3 Day		recovered/resolved
5 Blood pressure decreased	20020330		3 Day		recovered/resolved
6 Palpitations	20020330		3 Day		recovered/resolved
7 Prothrombin time increased	20020330		3 Day		recovered/resolved
8 Collapse circulatory	20020330		3 Day		recovered/resolved
9 Leukocytosis	20020330		3 Day		recovered/resolved
10 Blood sugar increased	20020330		3 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren	Oral	50 Mg milligram(s)/	20020330-20020330	Ischias
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005294

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth:	Age: 12 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubinaemia					recovered/resolved
2 Dizziness					recovered/resolved
3 General malaise					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	6 G gram(s)/	-	
paracetamol			-	
2 rewodina 50	Oral	4 Df dosage form/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02004955

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: 19360707 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests abnormal NOS					Unknown
2 Abnormality of red blood cells					Unknown
3 Exanthema					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	3 Df dosage form/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02006469

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: Age: 59 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests abnormal NOS	20020524				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Unknown	001 Df dosage form/Day	-200205	Carpal tunnel syndrome
2 voltaren dispers diclofenac sodium	Oral	003 Df dosage form/Day	200203-	Carpal tunnel syndrome

Drugs (concomitant)

3 calcium calcium	Unknown	/	-
4 ibuprofen ibuprofen	Unknown	/	-
5 vitamin d ergocalciferol	Unknown	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Rheumatoid arthritis			k.A.	concomitant disease

Primary source

Qualification: Consumer or other non health professional observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02006739

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19540408	Age: 47 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic cirrhosis	200106				Unknown
2 Dyspnoea	200106				Unknown
3 Weight increase	200106				Unknown
4 Icterus	200106				Unknown
5 Bleeding gastric ulcer	2001				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	50 Mg milligram(s)/	2000-20020430 -	Rheumatoid arthritis
2 gold gold	Unknown	/	1976-1980 -	Rheumatoid arthritis
3 imurek azathioprine	Oral	/	1980-1988 -	Rheumatoid arthritis
4 metoprolol metoprolol	Oral	050 Mg milligram(s)/Day	-20020430 -	Hypertension arterial
5 prednisolon prednisolone	Oral	010 Mg milligram(s)/Day	200110- -	
6 sulfasalazin sulfasalazine	Oral	/	1976-1990 -	Rheumatoid arthritis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ulcus ventriculi			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02007360

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19090927	Age: 92 Year	weight (kg): 66	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20020405		10 Day		recovered/resolved
2 Gamma GT increased	20020405		10 Day		recovered/resolved
3 Phosphatase alkaline increased	20020405		10 Day		recovered/resolved
4 Gastroenteritis	20020405		10 Day		recovered/resolved
5 Nausea	20020405		10 Day		recovered/resolved
6 Appetite suppressed	20020405		10 Day		recovered/resolved
7 Diarrhoea	20020405		10 Day		recovered/resolved
8 Hepatitis	20020405		10 Day		recovered/resolved
9 Bilirubin increased	20020405		10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 katadolon kapseln flupirtine maleate	Oral	002 Df dosage form/Day	20020404-20020405	Arthralgia
2 voltaren resinat diclofenac resinate	Oral	002 Df dosage form/Day	20020404-20020405	Osteoarthritis, generalized

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			k.A.	concomitant disease
Myeloid leukaemia, chronic			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02007648

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19490624 Age: 52 Year weight (kg): 89 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased	20020129		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 75 id diclofenac sodium	Oral	002 Df dosage form/Day	20020121-20020130	Gout acute

Drugs (concomitant)

2 allopurinol 100 allopurinol	Oral	001 Df dosage form/Day	-	Hyperuricaemia
3 enalapril al 5 enalapril maleate	Oral	1.5 Df dosage form/Day	-	Essential hypertension, unspecified
4 norvasc 5 mg amlodipine besilate	Oral	001 Df dosage form/Day	-	Essential hypertension, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Obesity			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z. n. intrazerebr. massenblutung

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02011811

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests abnormal NOS	200210				Unknown
2 Hepatic enzymes increased	200210				Unknown
3 Pain abdominal	200210				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fragmin dalteparin sodium	Unknown	/	-	
2 voltaren resinat diclofenac resinate	Unknown	002 Df dosage form/Day	-	Other general symptoms

Drugs (concomitant)

3 beloc zok metoprolol succinate	Unknown	001 Df dosage form/Day	-	
4 fosinorm fosinopril sodium	Unknown	/	-	Essential hypertension, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n.hueftgelenksprothese 09/2002
Viral hepatitis A without mention of hepatic coma			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02012411

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19370111 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels					not recovered/not resolved
2 Hepatitis nonspecific					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophlogont diclofenac sodium	Oral	050 Mg milligram(s)/Day	1972-2002 -	Psoriatic arthropathy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02012826

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19301205	Age: 71 Year	weight (kg): 51	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Somnolence	20020921		17 Day		recovered/resolved
2 Hepatotoxic effect	20020921		17 Day		recovered/resolved
3 Raised liver enzymes	20020921		17 Day		recovered/resolved
4 Feeling bad	20020921		17 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	002 Df dosage form/Day	20020907-20020921	Pain in limb

Drugs (concomitant)

2 acc acetylcysteine	Oral	/	-	
3 berlinsulin h 30/70 pen insulin human	Subcutaneous	012 Df dosage form/Day	-	Diabetes mellitus
4 bifiteral lactulose	Unknown	/	-	
5 dalmadorm flurazepam hydrochloride	Unknown	/	-	
6 euthyrox 75 levothyroxine sodium	Unknown	075 Mg milligram(s)/Day	-	
7 fosamax 10 mg alendronate sodium	Unknown	/	-	
8 ranitic 300 ranitidine hydrochloride	Unknown	300 Mg milligram(s)/	-	
9 temgesic buprenorphine	Unknown	/	-	
10 valoron n loesung zum einnehmen naloxone hydrochloride; tilidine hydrochloride	Oral	/	-	
11 zocor 5 mg simvastatin	Unknown	/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease
Osteoporosis			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n. subkapitale oberarmfraktur links
unbekannt			k.A.	concomitant disease: novalgin-allergie
Heart failure, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02900673

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19411211	Age: 60 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pneumonia	200211				not recovered/not resolved
2 Fever	20021029		4 Month		Unknown
3 Cholecystitis	20021029		4 Month		Unknown
4 Vomiting	20021029		4 Month		Unknown
5 Epigastric pain	20021029		4 Month		Unknown
6 Liver failure	20021029		4 Month		Unknown
7 Lung oedema	200211				not recovered/not resolved
8 Medication error	20021029				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	2 Df dosage form/	200211-	
2 voltaren dispers diclofenac sodium	Unknown	100 Mg milligram(s)/Day	20000331-20011009	Arthralgia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pulmonary congestion and hypostasis			k.A.	concomitant disease
Pneumonia, organism unspecified			k.A.	concomitant disease
Pure hypercholesterolaemia			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03000446

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19350718	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic	200212				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Unknown	/	-	Polymyalgia rheumatica
2 ibuhexal ibuprofen	Unknown	/	-	Polymyalgia rheumatica
3 novalgin metamizole	Unknown	/	-	Polymyalgia rheumatica
4 taxilan perazine	Oral	100 Mg milligram(s)/Day	1991-200301	Unspecified psychosis
5 tramal tramadol hydrochloride	Unknown	/	-	Polymyalgia rheumatica

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03003602

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19580624	Age: 44 Year	weight (kg): 92	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pancreatitis acute	20030218		17 Day		recovered/resolved
2 Lipase increased	20030218		17 Day		recovered/resolved
3 Pain epigastric	20030218		17 Day		recovered/resolved
4 CRP positive	20030218		17 Day		recovered/resolved
5 Elevated liver enzyme levels	200302				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	100 Mg milligram(s)/Day	200212-200302	Arthralgia
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n.pankreatitis 1999

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-03900219

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 83 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	20021105				Unknown
2 Gamma-GT increased	20021105				Unknown
3 Elevated liver enzyme levels	20021105				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ranitic ranitidine hydrochloride	Oral	450 Mg milligram(s)/Day	20021028-	-
2 remergil mirtazapine	Oral	015 Mg milligram(s)/Day	20021028-20021029	-
3 remergil mirtazapine	Oral	045 Mg milligram(s)/Day	20021101-	-
4 remergil mirtazapine	Oral	030 Mg milligram(s)/Day	20021030-20021031	-
5 voltaren diclofenac sodium	Oral	150 Mg milligram(s)/Day	20021028-	-

Drugs (concomitant)

6 captopril captopril	Oral	025 Mg milligram(s)/Day	-	-
7 l-thyroxin levothyroxine sodium	Oral	075 µg microgram(s)/Day	-	-
8 ossofortin calcium gluconate; calcium phosphate; colecalciferol	Oral	250 Mg milligram(s)/Day	-	-
9 valoron tilidine hydrochloride	Oral	200 Mg milligram(s)/Day	20021030-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Essential hypertension, unspecified			k.A.	concomitant disease
Osteoporosis			k.A.	concomitant disease
Depression			k.A.	concomitant disease
Jaundice, unspecified, not of newborn			k.A.	concomitant disease
Unspecified disorder of liver			k.A.	concomitant disease
Unspecified hypothyroidism			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03005132

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Female	Birth: 19460912	Age: 56 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20030311	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Tachycardia	20030311				Unknown
2 Purpura	20030311				Unknown
3 Shock	20030311		12 Hour		Unknown
4 Low blood pressure	20030311				Unknown
5 Sepsis	20030311				Unknown
6 Leucopenia	20030311				Unknown
7 CRP positive	20030311				Unknown
8 Thrombocytopenia	20030311				Unknown
9 Renal failure NOS	20030311				Unknown
10 Adrenal suppression	20030311				Unknown
11 Pneumonia	20030311				Unknown
12 Hb decreased	20030311				Unknown
13 Bilirubin increased	20030311				Unknown
14 LDH increased serum	20030311				Unknown
15 Gamma GT increased	20030311				Unknown
16 Alkaline phosphatase increased	20030311				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Intramuscular	1 Df dosage form/	20030310-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03007918

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: 19470331 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 estragest tts estradiol; norethisterone acetate	Unknown	/	200304- -	Unspecified menopausal and postmenopausal disorder
2 voltaren resinat diclofenac resinate	Unknown	/	200304- -	Osteoarthritis, unspecified whether generalized or

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont. k.A.	Comment
Nondependent abuse of alcohol				concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03008510

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Male	Birth: 19340328	Age: 68 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaemia sideroblastic	20020612				recovered/resolved
2 Renal insufficiency					Unknown
3 Creatinine blood increased	20020612				Unknown
4 Coagulation disorder	20020612				Unknown
5 Hematopoiesis depressed	2002				Unknown
6 Arrhythmia					Unknown
7 Dyspnoea					Unknown
8 Feeling bad	20020517				Unknown
9 Respiratory disorder NOS	20020517				Unknown
10 Pleural fibrosis	20020517				Unknown
11 Gastritis	20020527				Unknown
12 Gamma GT increased	20020531				Unknown
13 SGOT increased	20020517				Unknown
14 TSH decreased					Unknown
15 Hyperpotassaemia	20020517				Unknown
16 Hyponatremia	20020517				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	1.5 Df dosage form/Day	200303-200304 -	Osteoarthritis, localized, not specified whether primary or
2 voltaren resinat diclofenac resinate	Oral	2.5 Df dosage form/Day	200110-200203 -	Arthralgia

Drugs (concomitant)

3 allopurinol allopurinol	Oral	100 Mg milligram(s)/Day	200111-200210 -	Hyperuricaemia
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diaphragmatic hernia without mention of obstruction or gan			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.helicobacter-befall magen
Unspecified inflammatory spondylopathy			k.A.	concomitant disease
Calculus of gallbladder without mention of cholecystitis			k.A.	concomitant disease
Congestive heart failure			k.A.	concomitant disease
Unspecified osteochondropathy			k.A.	concomitant disease
Atrioventricular block, other and unspecified			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.bluttransfusion
unbekannt			k.A.	concomitant disease: z.n.schrittmacherimplantation
Essential hypertension, unspecified			k.A.	concomitant disease
Gout, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician
Literature:

observestudytype:

case: DE-BFARM-03009119

Patient

Reactions

Drugs (suspected or interacting)**Drugs (concomitant)**

Medical History and concurrent conditions

Primary source

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FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03009616

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19610226	Age: 42 Year	weight (kg): 77	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea	20030418				not recovered/not resolved
2 Vomiting	20030418				not recovered/not resolved
3 Pain	20030418				not recovered/not resolved
4 Appetite lost	20030418				not recovered/not resolved
5 Icterus	20030418				not recovered/not resolved
6 Sweating attack	20030418				not recovered/not resolved
7 Hepatitis acute	20030418				not recovered/not resolved
8 Hepatotoxic effect	20030418				not recovered/not resolved
9 Weight decrease	20030418				not recovered/not resolved
10 Urine discolouration	20030418				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	075 Mg milligram(s)/Day	20030410-20030410	Pain in joint
diclofenac			-	

Drugs (concomitant)

2 dexamethason 4 mg	Intramuscular	004 Mg milligram(s)/Day	20030410-20030410	Pain in joint
dexamethasone sodium phosphate			-	
3 l-thyroxin	Oral	100 Mg milligram(s)/Day	2002-	Goitre, unspecified
levothyroxine sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Nondependent abuse of alcohol			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z. n. hysterektomie 1991
Unspecified disorder of uterus			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n. strumektomie 2002
Hepatomegaly			k.A.	concomitant disease
Chronic hepatitis			k.A.	concomitant disease
Nondependent abuse of tobacco			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03009936

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: 19531023	Age: 49 Year	weight (kg): 68	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic	200301				not recovered/not resolved
2 Raised liver enzymes	200301				not recovered/not resolved
3 Hepatic necrosis	200301				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	200 Mg milligram(s)/Day	200301-20030220	Pain in joint
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Rheumatoid arthritis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03010565

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19420924	Age: 60 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20030510		12 Day		recovered/resolved
2 Hepatic disease NOS	20030510		12 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Other general symptoms
diclofenac			-	

Drugs (concomitant)

2 atorvastatin	Unknown	/	-	Coronary atherosclerosis
atorvastatin			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n.hepatitis a
Unspecified disorder of metabolism			k.A.	concomitant disease
Essential hypertension, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-03011094

Patient

Reactions

Drugs (suspected or interacting)

Drugs (concomitant)

Primary source

107 / 594

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03900721

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19491013 Age: 53 Year weight (kg): 73 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	200306				not recovered/not resolved
2 Elevated liver enzyme levels	200306				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin "bayer" acetylsalicylic acid	Oral	500 Mg milligram(s)/Week	-	Headache
2 voltaren resinat diclofenac resinate	Oral	1 Df dosage form/	2000-200309	Arthralgia

Drugs (concomitant)

3 decortin prednisone	Oral	020 Mg milligram(s)/Day	-	
4 novothyral levothyroxine sodium; liothyronine sodium	Oral	100 Mg milligram(s)/Day	1981-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Autoimmune disease, not elsewhere classified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

case: DE-BFARM-03014643

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Male Birth: 19570605 Age: 46 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Phosphatase alkaline increased	20030718		40 Day		recovered/resolved
2 Gamma-GT increased	20030718		40 Day		recovered/resolved
3 ALT increased	20030718		40 Day		recovered/resolved
4 AST increased	20030718		40 Day		recovered/resolved
5 Hyperhidrosis	20030718		40 Day		recovered/resolved
6 Thrombocytopenia	20030718		40 Day		recovered/resolved
7 Elevated liver enzyme levels	20030718		40 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Torticollis, unspecified
diclofenac			-	
2 rosuvastatin	Oral	010 Mg milligram(s)/Day	20020724-20030718	Pure hypercholesterolaemia
rosuvastatin			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			k.A.	concomitant disease

Primary source

Qualification:	Consumer or other non health professional	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03016056

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19550619 Age: 47 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20030506				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	2001- -	Unspecified polyarthropathy or polyarthritis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Myalgia and myositis, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03900911

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	20030925				not recovered/not resolved
2 SGOT increased	20030925				not recovered/not resolved
3 Creatine phosphokinase increased	20030925				not recovered/not resolved
4 Proteinuria	20030923				not recovered/not resolved
5 Creatinine blood increased	20030923				not recovered/not resolved
6 Renal failure NOS	20030923				not recovered/not resolved
7 Haematuria	20030923				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	12 Df dosage form/	20030921-20030921	Backache, unspecified
2 voltaren diclofenac sodium	Oral	/	-20030920	Backache, unspecified

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03016383

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: 19711003	Age: 31 Year	weight (kg): 85	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic necrosis	200306				not recovered/not resolved
2 Bilirubin increased	200306				not recovered/not resolved
3 Elevated liver enzyme levels	200306				not recovered/not resolved
4 LDH increased	200306				not recovered/not resolved
5 Gamma-GT increased	200306				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Unknown	/	200306- -	Other tear of cartilage or meniscus of knee, current

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03017227

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19491107	Age: 53 Year	weight (kg): 45	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20030422		7 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 50 mg diclofenac sodium	Oral	002 Df dosage form/Day	20030415-20030425	Other general symptoms
2 fraxiparin 0,4 nadroparin calcium	Subcutaneous	001 Df dosage form/Day	20030415-20030505	-

Primary source

Qualification: Physician

observestudytype:

Literature:

case: DE-BFARM-04000158

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Male Birth: 19590722 Age: 44 Year weight (kg): 80 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20030906		11 Day		recovered/resolved
2 Pain epigastric	20030906				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 50 diclofenac sodium	Oral	001 Df dosage form/Day	200305-20030908	Backache, unspecified
2 insidon opipramol hydrochloride	Oral	050 Mg milligram(s)/Day	200305-20030908	Depression
3 viox 25 mg rofecoxib	Oral	001 Df dosage form/Day	200305-20030908	Backache, unspecified

Drugs (concomitant)

4	locol 40 fluvastatin sodium	Oral	001 Df dosage form/Day	2002- -	Other and unspecified hyperlipidaemia
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MedDRA-term	Begin	End	Cont.	Comment
Other unspecified back disorders			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000304

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Male Birth: 19350501 Age: 68 Year weight (kg): 80 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	20030505				Unknown
2 Gamma-glutamyltransferase incr.	20030505				Unknown
3 Phosphatase alkaline increased	20030505				Unknown
4 Bilirubin increased	20030505				Unknown
5 Hepatitis acute	20030505				recovering/resolving
6 SGOT increased	20030505				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aggrenox acetylsalicylic acid; dipyridamole	Oral	002 Df dosage form/Day	200209-20030505	-
2 diclofenac 50 mg diclofenac sodium	Oral	050 Mg milligram(s)/Day	200210-20030505	-
3 fosinorm 20mg fosinopril sodium	Oral	020 Mg milligram(s)/Day	200205-20030505	-
4 mevalotin 40mg pravastatin sodium	Oral	040 Mg milligram(s)/Day	200210-20030505	-

Drugs (concomitant)

5 actraphane hm insulin human injection, isophane	Subcutaneous	040 Iu international unit(s)	200101-20030514	Diabetes mellitus without mention of complication
6 amaryl 3 mg glimepiride	Oral	1.5 Mg milligram(s)/Day	200201-20030505	-
7 hct-beta 25 hydrochlorothiazide	Oral	001 Df dosage form/Day	200206-20030505	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute, but ill-defined, cerebrovascular disease			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: dyslipidaemie
Oesophagitis NOS			k.A.	concomitant disease
Other chronic nonalcoholic liver disease			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

case: DE-BFARM-04000702

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19381027 Age: 65 Year weight (kg): 70 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus					recovered/resolved
2 Elevated liver enzyme levels					recovered/resolved
3 Overdose effect					recovered/resolved
4 Delirium	20040102		4 Day		recovered/resolved
5 Confusional state	20040102		4 Day		recovered/resolved
6 Fits NOS	20040102		2 Minute		recovered/resolved
7 Hepatitis					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	550 Mg milligram(s)/Day	-	
diclofenac			-	
2 voltaren	Oral	550 Mg milligram(s)/Day	-	Unspecified arthropathy
diclofenac sodium			-	

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04002206

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Male Birth: 19670218 Age: 36 Year weight (kg): 78 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased	20040114				recovered/resolved
2 Gamma GT increased	20040114				recovered/resolved
3 Platelets abnormal	20040115				recovered/resolved
4 WBC abnormal NOS	20040115				recovered/resolved
5 Hb decreased	20040115				recovered/resolved
6 Nausea	20040115				recovered/resolved
7 Epigastric ache	20040115				recovered/resolved
8 Ulcus ventriculi	20040115		10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat	Oral	150 Mg milligram(s)/Day	200306-20040114	Osteoarthritis, localized, not
diclofenac resinate			-	specified whether primary or

MedDRA-term	Begin	End	Cont.	Comment
Osteoarthritis, unspecified whether generalized or localized			k.A.	concomitant disease

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04001905

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Male Birth: 19830823 Age: 20 Year weight (kg): 67 BMI int: Age fetus:

Reactions

	MedDRA LLT	Begin	End	duration	TtO	outcome
1	Tiredness	20040211				recovered/resolved
2	Rhabdomyolysis	20040212		8 Day		recovered/resolved
3	SGOT increased	20040212		8 Day		recovered/resolved
4	Creatine phosphokinase increased	20040212		8 Day		recovered/resolved
5	Back pain	20040211				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amitriptyline	Oral	/	200202-200402	12
amitriptyline			-	
2 fluoxetine	Oral	020 Mg milligram(s)/Day	200203-200404	12
fluoxetine			-	
3 voltaren emulgel	Topical	4 Df dosage form/Week	20040101-200402	12
diclofenac diethylamine			-	
4 voltaren resinat	Oral	2 Df dosage form/Week	200401-200402	12
diclofenac resinate			-	
5 voltaren resinat	Oral	150 Mg milligram(s)/Day	2003-2003	
diclofenac resinate			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: ess-stoerung
unbekannt			k.A.	concomitant disease: sehnen-erkrankung

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04002715

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes No

Patient

Initials: Sex: Female Birth: 19430126 Age: 60 Year weight (kg): 59 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Serum glutamic-oxaloacetic ta inc	20040103				recovering/resolving
2 LDH increased	20040103				recovering/resolving
3 Gamma-GT increased	20040103				recovering/resolving
4 Raised liver enzymes	20040103				recovering/resolving
5 Serum glutamic-pyruvic ta incr	20040103				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina 50 diclofenac sodium	Oral	002 Df dosage form/Day	20030912- -	Arthralgia

Drugs (concomitant)

2 vitamin e tocopherol	Oral	002 Df dosage form/Day	200310- -	Arthralgia
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Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04003543

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19411214	Age: 62 Year	weight (kg): 94	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Heartburn					not recovered/not resolved
2 Flu-like symptoms	20040213			1 Day	not recovered/not resolved
3 Fever	20040220			8 Day	not recovered/not resolved
4 Eye redness	20040302			19 Day	not recovered/not resolved
5 Conjunctivitis					not recovered/not resolved
6 Chills	20040220			8 Day	not recovered/not resolved
7 Transaminases increased	20040301			18 Day	not recovered/not resolved
8 Creatinine increased	20040301			18 Day	not recovered/not resolved
9 Pruritus	20040221			9 Day	not recovered/not resolved
10 Allergic exanthema	20040215	20040218	4 Day	3 Day	not recovered/not resolved
11 ESR increased	20040305			22 Day	Unknown
12 CRP increased	20040301			18 Day	not recovered/not resolved
13 Stevens Johnson syndrome	20040221	20040312	21 Day	11 Day	not recovered/not resolved
14 Lip swelling	20040229			17 Day	not recovered/not resolved
15 Mucosal erosion	20040229			17 Day	not recovered/not resolved
16 Vesicle	20040228			16 Day	not recovered/not resolved
17 Skin reaction	20040221			9 Day	not recovered/not resolved
18 Skin disorder	20040225			13 Day	not recovered/not resolved
19 Rash pruritic	200402				not recovered/not resolved
20 Pain of skin	20040221			9 Day	not recovered/not resolved
21 Marginal keratitis					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fenistil dimetindene maleate	Intravenous (not otherwise)	1 Df dosage form/Day Solution for injection	20040216-20040216	Itching 1440 Minute
2 PARACETAMOL paracetamol	Oral	/	20040212-20040212	Fever 1440 Minute
3 PARACETAMOL paracetamol	Oral	/	20040216-20040218	Fever 3 Day
4 prednisolon prednisolone	Oral	/	20040216-20040227	Allergic exanthema 12 Day
5 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwise)	250 Mg milligram(s)/Day Solution for injection	20040216-20040216	Exanthema 1440 Minute
6 VOLTAREN RESINAT diclofenac resinate	Oral	2 Df dosage form/Day Capsule	20040213-20040213	Fever 1440 Minute

Drugs (concomitant)

7 concor plus bisoprolol fumarate; hydrochlorothiazide	Unknown	/	-	Hypertension
8 KETEK telithromycin	Oral	1 Df dosage form/Day	20040224-20040224	Purulent bronchitis 1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Unknown	concomitant disease
Upper respiratory tract infection			Unknown	
Dermatitis			Unknown	due to drugs and medicaments taken internally

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 06.02.2008 DE-002147023-PHNU2004DE01207

Sender diagnosis: not reported

Narrative

Initial report received on 12 Mar 2004. Physician reported about patient who took one Voltaren (diclofenac) dose, about 10-12 days ago, for flu symptoms. Shortly thereafter, the patient showed an "extreme" skin reaction, and antibiotic treatment with Ketek (telithromycin) was started. The patient was hospitalized on 1 Mar 2004, and high dose prednisolone over 3 days (starting with 250mg/d, then tapering) was given. Due to affection of the conjunctiva, a dermatological consultation was requested, and Ketek (telithromycin) was stopped. Histology confirmed the diagnosis of Stevens Johnson syndrome. The patient remained on topical treatment, and the condition greatly improved during the following days. The patient's discharge was foreseen for 12 Mar 2004. Both Voltaren and Ketek were named as suspected drugs.

Follow up received on 24 Mar 2004 (written documentation from internal specialist): The patient took Voltaren on 22 Feb 2004, for rheumatic pains due to upper respiratory tract infection. Stevens Johnson syndrome started on ("about") 23 Feb 2004 and lasted for "more than 14 days". According to reporter, pruritic rash started after Voltaren use, and the picture of Stevens Johnson syndrome occurred one day after starting Ketek therapy. On an attached lab sheet, the following "lab data relevant for the AE" were reported: increased creatinine, increased transaminases, CRP increase and ESR increase. The condition did not abate after stopping therapy (no rechallenge since). Outcome was indicated as "not yet recovered". The reporting physician rated causality for Voltaren as probable (Ketek was not explicitly named as suspect drug).

Follow up received on 4 Feb 2008 from RegiSCAR project (internal case number 9150136). The Voltaren drug was specified as Voltaren Resinat (diclofenac, colestyramin); and according to this information (in contrast to previous data), Voltaren Resinat was used on 13 Feb 2004, for fever. No previous Voltaren drug use was recorded in this follow up. Fenistil (dimetinden maleate) was used on 16 Feb 2004, due to pruritus. On 13 Feb 2004, flu like symptoms occurred; and allergic exanthema was present from 15 until 18 Feb 2004. Chills and fever occurred on 20 Feb, and skin pains, pruritus, generalized macules and spots were visible on 21 Feb 2004. Purulent bronchitis was seen on 23 Feb, and skin changes further spread on 25 Feb 2004. The patient later also presented heartburn, marginal keratitis, eye redness, lip swellings and mucosal erosions. The first vesicle was seen on 28 Feb 2004. The patient was hospitalised on 1 Mar 2004. Outcome was reported as "not yet recovered". By the expert from the reporting center, the following drugs were mentioned as suspected (i.e. with a causality of at least possible): Voltaren Resinat, Fenistil inj., paracetamol, Solu-Decortin (prednisolone-21-hydrogensuccinate), and prednisolone; Ketek was not regarded as suspected. As other contributing or causal factor, flu infection was mentioned.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04003924

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: 19500624	Age: 53 Year	weight (kg): 74	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-glutamyltransferase incr.	20031029				Unknown
2 Elevated liver enzyme levels	20031029				Unknown
3 Hepatitis acute	20031029				Unknown
4 Bilirubin increased	20031029				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 vitamin e tocopherol	Oral	001 Df dosage form/Day	20030920-20031020	-
2 voltaren 50 diclofenac sodium	Oral	/	1987-20031029	Backache, unspecified

Drugs (concomitant)

3 analgetikum	Intramuscular	/	1987-2001	-
4 aspirin acetylsalicylic acid	Oral	/	1987-20031029	-
5 Homoeopathika	Oral	001 Df dosage form/Day	200212-200304	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic rhinitis due to other allergen			k.A.	concomitant disease
Low back pain			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z. n. myogelose 1987
unbekannt			k.A.	concomitant disease: z. n. knie-op 2001
Diverticula of colon			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04004302

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19430725 Age: 60 Year weight (kg): 74 BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Iron metabolism disorder	200307				recovered/resolved
2 Leukocytosis	200307				recovered/resolved
3 Blood sedimentation increased	200307				recovered/resolved
4 Cholelithiasis	200307				recovered/resolved
5 Ascites	200307				recovered/resolved
6 Monocytosis	200307				recovered/resolved
7 Hepatitis cholestatic	200307				recovered/resolved
8 Elevated liver enzyme levels	200307				recovered/resolved
9 Exhaustion	200307				recovered/resolved
10 Feeling bad	200307				recovered/resolved
11 Lymph nodes enlarged	200309				recovered/resolved
12 Icterus	200307				recovered/resolved
13 Bilirubinaemia	200307				recovered/resolved
14 Gamma-GT increased	200307				recovered/resolved
15 Phosphatase alkaline increased	200307				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren	Unknown	/	-	
diclofenac sodium			-	

Drugs (concomitant)

2	halcion triazolam	Unknown	/	-
3	hepa-merz granulat ornithine aspartate	Unknown	003 Df dosage form/Day	-
4	keltican n cytidine phosphate sodium; uridine triphosphate sodium	Unknown	/	-
5	lipotalon dexamethasone palmitate	Unknown	/	-
6	meaverin mepivacaine hydrochloride	Unknown	/	-
7	nexium esomeprazole	Unknown	020 Mg milligram(s)/Day	-
8	nexium mups esomeprazole	Unknown	020 Mg milligram(s)/Day	-
9	osyrol-lasix furosemide; spironolactone	Unknown	001 Df dosage form/Day	-
10	zeel tablet homeopatics nos	Unknown	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pure hypercholesterolaemia			k.A.	concomitant disease
Essential hypertension, unspecified			k.A.	concomitant disease
Other and unspecified hyperlipidaemia			k.A.	concomitant disease
Spondylosis of unspecified site			k.A.	concomitant disease
Oesophagitis NOS			k.A.	concomitant disease
Acquired deformity of chest and rib			k.A.	concomitant disease
Asthma, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04004463

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19390711	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	200403				Unknown
2 Gamma-GT increased	200403				Unknown
3 Phosphatase alkaline increased	200403				Unknown
4 Elevated liver enzyme levels	200403				Unknown
5 Bilirubinaemia	200403				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 sinei cimicifuga racemosa root	Unknown	/	-	-
2 voltaren diclofenac sodium	Oral	050 Mg milligram(s)/Day	200402-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04900261

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Male	Birth: 19260521	Age: 77 Year	weight (kg): 78	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rhabdomyolysis	20040329				recovering/resolving
2 Myalgia	20040329				recovering/resolving
3 Asthenia	20040329				recovering/resolving
4 Oedema peripheral	20040329				recovering/resolving
5 Creatine phosphokinase increased	20040329				recovering/resolving
6 Lyell syndrome	20040405				Unknown
7 Blisters	20040405				Unknown
8 Tiredness	20040329				recovering/resolving
9 LDH increased	20040329				recovering/resolving
10 SGOT increased	20040329				recovering/resolving
11 Confusion	20040429				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 novalgin metamizole	Oral	/	-20040416 -	Closed fracture of lumbar vertebra without mention of
2 voltaren resinat diclofenac resinate	Oral	/	-20040331 -	Other general symptoms

Drugs (concomitant)

3 Acetylsalizylsaeure acetylsalicylic acid	Unknown	100 Mg milligram(s)/Day	- -	
4 actonel risedronic acid	Unknown	/	- -	
5 Biphosphonat	Unknown	/	- -	
6 kuerbisoel cucurbita pepo seed oil	Unknown	/	- -	
7 rofecoxib rofecoxib	Unknown	025 Mg milligram(s)/Day	20040416- -	Other general symptoms
8 tramal tramadol hydrochloride	Oral	200 Mg milligram(s)/Day	20040429-20040430 -	Closed fracture of lumbar vertebra without mention of

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Closed fracture of lumbar vertebra without mention of spina			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: schmerzsyndrom
Abdominal pain			k.A.	concomitant disease
Osteoporosis			k.A.	concomitant disease
Hypertension arterial			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04006051

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No	YES	Yes

Patient

Initials:	Sex: Female	Birth:	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Immune hemolytic anemia drug-induced	2003		7 Week		recovered/resolved
2 Renal failure					recovered/resolved
3 Nausea					recovered/resolved
4 Jaundice					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC	Intramuscular	075 Mg milligram(s)/Day	-	Musculoskeletal pain
diclofenac			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cholecystectomy			No	
Back pain			Unknown	
Penicillin allergy			Unknown	
Essential hypertension, unspecified			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	AHRENS N, SCHEWIOR L, GARBE E, KIESEWETTER H, SALAMA A. MASSIVE HAEMOLYSIS AFTER INTRAMUSCULAR DICLOFENAC IN A PATIENT WHO APPARENTLY TOLERATED ORAL MEDICATION. VOX SANGUINIS 2004;86(1):71-74.	

Narratives (nur bei elektronischem Eingang)

Generics [UK] Ltd, 10.04.2008 DE-MYLANLABS-2008S1005302

Sender diagnosis: not reported

Narrative

Information from abstract of published case report:

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AHRENS N, SCHEWIOR L, GARBE E, KIESEWETTER H, SALAMA A. MASSIVE HAEMOLYSIS AFTER INTRAMUSCULAR DICLOFENAC IN A PATIENT WHO APPARENTLY TOLERATED ORAL MEDICATION. VOX SANGUINIS 2004;86(1):71-74.

*

Here, we describe a patient who apparently tolerated oral diclofenac but developed severe IHA following intramuscular injection of the drug. PATIENTS AND METHODS: A 66-year-old-female was admitted to hospital because of jaundice and nausea, which were initially presumed to be manifestations of a postcholecystectomy syndrome. The patient soon developed haemolysis and renal failure. Although the symptoms and signs were suggestive of autoimmune haemolytic anaemia (AIHA), the patient had diclofenac-induced IHA. RESULTS: Serological testing, including detection of drug-dependent antibodies, was performed using standard techniques. The patient's serum was found to contain a highly reactive diclofenac-dependent red cell antibody of the immune complex type (titre 256 000). She recovered after 7 weeks of treatment with prednisolone, blood transfusions, haemodialysis and plasma exchange. CONCLUSIONS: Diclofenac-induced IHA should always be considered when a patient on diclofenac develops haemolysis.

case: DE-BFARM-04008140

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19470302 Age: 57 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased	20040618				Unknown
2 Cholestasis	20040618				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	/	20040601-20040602	Hallux valgus
2 staphylex flucloxacillin sodium	Oral	/	20040608-20040617	

Drugs (concomitant)

3	anaesthetikum	Unknown	/	20040601-20040601	Hallux valgus
				-	
4	rofecoxib	Unknown	/	20040531-20040601	
	rofecoxib			-	
5	tilidine hydrochloride	Unknown	/	20040603-20040619	
	tilidine hydrochloride			-	

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04008460

Patient

Reactions

Drugs (suspected or interacting)

Medical History and concurrent conditions

Primary source

129 / 594

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04009906

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19530330	Age: 51 Year	weight (kg): 58	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pancreas enzymes increased	20040419				not recovered/not resolved
2 Abdominal pain	20040419				not recovered/not resolved
3 Raised liver enzymes	20040419				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dermestril 50 estradiol	Transdermal	/	1993-20040712 -	Menopausal or female climacteric states
2 diclac 5% gel diclofenac sodium	Transdermal	/	20021217-200405 -	Pain in joint

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04011027

Patient

Reactions

Drugs (suspected or interacting)**Drugs (concomitant)**

Medical History and concurrent conditions

Primary source

131 / 594

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04011191

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: 19201114 Age: 83 Year weight (kg): 81 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time increased	20040618		5 Day		recovered/resolved
2 Stool tarry	200406				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 75 id diclofenac sodium	Oral	075 Mg milligram(s)/Day	20040512-20040618	Lumbago
2 falithrom 1,5 phenprocoumon	Oral	1.5 Mg milligram(s)/Day	19840101-20040616	

Drugs (concomitant)

3 concor 10 bisoprolol fumarate	Oral	010 Mg milligram(s)/Day	19920101-20040618	Essential hypertension, unspecified
4 digitoxin digitoxin	Oral	0.4 Mg milligram(s)/Day	19840101-20040618	Congestive heart failure
5 Hydrochlorothiazid hydrochlorothiazide	Oral	025 Mg milligram(s)/Day	20020101-20040618	Congestive heart failure
6 lisino loratadine	Oral	020 Mg milligram(s)/Day	20020101-	Heart failure, unspecified
7 magnetrans forte magnesium oxide	Oral	150 Mg milligram(s)/Day	20030101-	Other musculoskeletal symptoms referable to limbs
8 nitro glyceryl trinitrate	Oral	1 Df dosage form/	19940101-	Coronary atherosclerosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: nieren-insuffizienz kompensiert im stadi
Varicose veins of lower extremities without mention of ulcer			k.A.	concomitant disease
Thyrotoxicosis without mention of goitre or other cause			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.appendektomie,cholezystektomie,te
Atrial fibrillation and flutter			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04011920

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: 19430208	Age: 61 Year	weight (kg): 62	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Serum glutamic-pyruvic ta incr	20040805		14 Day		recovering/resolving
2 Gamma-GT increased	20040805		14 Day		recovering/resolving
3 Phosphatase alkaline increased	20040805		14 Day		recovering/resolving
4 Serum glutamic-oxaloacetic ta inc	20040805		14 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	050 Mg milligram(s)/Day	20040613-20040805	Rheumatoid arthritis
diclofenac			-	
2 sulfasalazine	Oral	003 Df dosage form/Day	20040613-20040805	Rheumatoid arthritis
sulfasalazine			-	

Drugs (concomitant)

3 nexium mups 20 mg	Oral	001 Df dosage form/Day	20040701-	Nausea and vomiting
esomeprazole			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04012248

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Male Birth: Age: 45 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Phosphatase alkaline increased	20000905		1 Month		recovering/resolving
2 SGOT increased	20000905		1 Month		recovering/resolving
3 SGPT increased	20000905		1 Month		recovering/resolving
4 Hepatic function abnormal	20000905		1 Month		recovering/resolving
5 Gamma-GT increased	20000905		1 Month		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atosil promethazine hydrochloride	Oral	150 Mg milligram(s)/Day	20000808-20000905	-
2 risperdal risperidone	Oral	003 Mg milligram(s)/Day	20000811-20000905	-
3 voltaren diclofenac sodium	Oral	100 Mg milligram(s)/Day	20000821-20000902	-

Drugs (concomitant)

4 hypnorex lithium carbonate	Oral	1.2 G gram(s)/Day	20000808-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Manic-depressive psychosis, manic type			k.A.	concomitant disease
Diabetes mellitus			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04013287

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: 19440329 Age: 59 Year weight (kg): 103 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	200303				not recovered/not resolved
2 Aggravation of existing disorder	200309				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aequamen betahistine mesilate	Unknown	024 Mg milligram(s)/Day	-	Tinnitus
2 bisoprolol bisoprolol	Unknown	/	-	Essential hypertension, unspecified
3 calcitrat calcium citrate	Unknown	001 Df dosage form/Day	-	
4 diclac diclofenac sodium	Unknown	050 Mg milligram(s)/Day	-	Rheumatoid arthritis
5 enbrel etanercept	Subcutaneous	50 Mg milligram(s)/Week	200204-	Spondylitis ankylosing
6 felodipine felodipine	Unknown	010 Mg milligram(s)/Day	-	Essential hypertension, unspecified
7 fosamax alendronate sodium	Unknown	70 Mg milligram(s)/Week	-	Osteoporosis
8 methotrexate methotrexate	Unknown	/	-200407	
9 omeprazole omeprazole	Unknown	020 Mg milligram(s)/Day	-	
10 prednisolone prednisolone	Unknown	005 Mg milligram(s)/Day	-	Rheumatoid arthritis
11 torasemid torasemide	Unknown	020 Mg milligram(s)/Day	-	Essential hypertension, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Obesity			k.A.	concomitant disease
Dizziness and giddiness			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: syndrom, metabolisches

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04013603

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19660206	Age: 38 Year	weight (kg): 77	BMI_int: 30.08	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Upper abdominal pain	20040910				recovered/resolved
2 Vomiting	20040910				recovered/resolved
3 Nausea	20040910				recovered/resolved
4 Cholestasis	20040910				recovered/resolved
5 Hepatocellular damage	20040910				recovered/resolved
6 Transaminases increased	20040910				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	1 Df dosage form/Day	20040908-20040909	
diclofenac			2 Day	
2 imap	Intramuscular	/	20040908-	
fluspirilene			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pyelonephritis			Yes	
Nephrolithiasis			Yes	
Hypokalaemia			Yes	
Migraine NOS			Yes	concomitant disease
Infectious hepatitis			Unknown	
Drug hypersensitivity			Unknown	
Post vaccination syndrome			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04016445

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: 19230322	Age: 81 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pulmonary infiltration	200411				Unknown
2 International normalised ratio increased	200411				Unknown
3 Respiratory failure	200411				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cordarex amiodarone hydrochloride	Oral	1800 Mg milligram(s)/Wee	2000-20041124	Atrial fibrillation
2 diclofenac diclofenac	Oral	/	2004-	Thoracic vertebral fracture T12
3 ibuprofen ibuprofen	Oral	/	2004-	Thoracic vertebral fracture T12
4 marcumar phenprocoumon	Oral	3 Mg milligram(s)/	-	Atrial fibrillation
5 valdecoxib valdecoxib	Oral	/	2004-	-

Drugs (concomitant)

6 beloc zok metoprolol succinate	Oral	190 Mg milligram(s)/Day	2000-20041124	Cardiac insufficiency
7 dytide h hydrochlorothiazide; triamterene	Oral	075 Mg milligram(s)/Day	-20041124	Cardiac insufficiency

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diverticula of colon			Unknown	
Osteoporosis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05002076

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19170916	Age: 87 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaphylactic shock	20041224	20050125	33 Day	1 Day	recovered/resolved
2 Cardiogenic shock	20041231		26 Day		recovered/resolved
3 Red blood cell count decreased					recovered/resolved
4 White blood cell count increased					recovered/resolved
5 CRP increased					recovered/resolved
6 Protein total abnormal NOS					recovered/resolved
7 Arrhythmia absoluta	20041231		26 Day		recovered/resolved
8 Atrial fibrillation	20041231		26 Day		recovered/resolved
9 Hypersensitivity NOS	20041224				recovered/resolved
10 Bronchial infection					recovered/resolved
11 COPD exacerbation	20041231		26 Day		recovered/resolved
12 Aortic aneurysm					recovered/resolved
13 Diverticulum duodenal					recovered/resolved
14 Transient psychosis					recovered/resolved
15 Localised osteoarthritis					recovered/resolved
16 Neurogenic bladder					recovered/resolved
17 Blood alkaline phosphatase increased					recovered/resolved
18 GGT increased					recovered/resolved
19 GOT increased					recovered/resolved
20 GPT increased					recovered/resolved
21 LDH increased					recovered/resolved
22 Diabetes mellitus NOS					recovered/resolved
23 Blood creatinine increased					recovered/resolved
24 Blood urea increased					recovered/resolved
25 Blood sodium decreased					recovered/resolved
26 Blood potassium increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 indometacin		/	-	
indometacin			-	
2 tramal		/	-	
tramadol hydrochloride			-	
3 voltaren	Oral	050 Mg milligram(s)/	20041224-20041224	Back pain
diclofenac sodium			-	

Drugs (concomitant)

4	decortin	5 Mg milligram(s)/Day	-
	prednisone		-
5	digimerck minor	1 Df dosage form/Day	-
	digitoxin		-
6	eunerpan	25 Mg milligram(s)/Day	-
	melperone hydrochloride		-
7	isoptin	240 Mg milligram(s)/Day	-
	verapamil hydrochloride		-
8	kalitrans	1 Df dosage form/Day	-
	citric acid; potassium bicarbonate		-
9	lasix	40 Mg milligram(s)/Day	-
	furosemide		-
10	nexium	40 Mg milligram(s)/Day	-
	esomeprazole		-
11	symbicort turbohaler	4 Df dosage form/Day	-
	budesonide; formoterol		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Urinary tract infection			Yes	concomitant disease
Rales			Yes	
Heart insufficiency			Yes	concomitant disease
Oedema peripheral			Yes	
Respiration abnormal			Yes	
Eye operation			No	
Hemiparesis (right)			Yes	
Hypertensive heart disease			Yes	
Dyspnoea			Yes	
Face oedema			Yes	
Ventricular hypertrophy			Yes	
Pulmonary congestion			Yes	
Pleural effusion			Yes	
Atelectasis			Yes	
Chronic obstructive airways disease			Yes	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05002712

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19311207	Age: 72 Year	weight (kg): 62	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Diarrhoea	200406				not recovered/not resolved
2 Vomiting	200406				not recovered/not resolved
3 Nausea	200406				not recovered/not resolved
4 Hepatitis toxic	200406				not recovered/not resolved
5 Sweating attack	200406				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 captopril	Unknown	/	-	Hypertension arterial
captopril			-	
2 diclofenac	Intramuscular	/	-	Backache, unspecified
diclofenac			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Oesophagitis NOS			k.A.	concomitant disease
Ulcus ventriculi			k.A.	concomitant disease
Herpes simplex without mention of complication			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-05004654

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19400910 Age: 64 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic drug-induced	20050126				not recovered/not resolved
2 Transaminases increased	20050126				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	200501-	Back discomfort
diclofenac			-	

Drugs (concomitant)

2	amlodipine	Oral	/	-
	amlodipine			-
3	Bisobeta 5	Oral	/	-
	bisoprolol fumarate			-
4	Fosinorm 10mg	Oral	/	-
	fosinopril sodium			-
5	Simvahexal 10 mg Filmtabletten	Oral	/	-
	simvastatin			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Yes	
Hyperlipidaemia NOS			Yes	
Ovarian operation			No	
Hysterectomy			No	
Aortic valve disease			Yes	
Spinal deformity			Yes	
Headache NOS			Yes	
Carotid artery occlusion			No	
Diabetes mellitus			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006761

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic damage					Unknown
2 Overdose					Unknown
3 Renal insufficiency			8 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	8 G gram(s)/Day	-	
2 diclofenac diclofenac		1 G gram(s)/Day	-	Medication error
3 ibuprofen ibuprofen	Unknown	3.6 G gram(s)/Day	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Toxicol Clin Toxicol (Phila); 2005; 43(1) :31-37 Author: von Mach, et al Title: Experiences of a Poison Center Network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		ueber 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05007986

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy YES	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 32 Year	weight (kg):	BMI_int:	Age fetus: 29 Week
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	20050427			1 Day	recovered/resolved
2 Dizziness	20050427			1 Day	recovered/resolved
3 Somnolence	20050427			1 Day	recovered/resolved
4 Ductus arteriosus premature closure					recovered/resolved
5 Transaminase NOS increased	20050427			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	26 Df dosage form/	20050427-20050427	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05008807

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes		Yes				

Patient

Initials:	Sex: Female	Birth: 19470217	Age: 54 Year	weight (kg): 77	BMI_int: 24.58	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blood bilirubin increased	20021128				Unknown
2 Lymphadenopathy	20021128				Unknown
3 General physical health deterioration	200211				Unknown
4 Thyroid adenoma	20021128				Unknown
5 Blood alkaline phosphatase increased	20021128				Unknown
6 Gamma GT increased	20021128				Unknown
7 Antinuclear antibody positive					Unknown
8 Fatigue	20021128				Unknown
9 Hepatic failure	2002				Unknown
10 Transaminases increased	20021128				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac sodium	Intramuscular	/	20020922-	Intervertebral disc protrusion
diclofenac sodium		Solution for injection	-	
2 Ofen	Unknown	/	-	Intervertebral disc protrusion
diclofenac sodium		Gel	-	
3 Voltaren D50	Oral	/	20020922-	Intervertebral disc protrusion
diclofenac sodium		Dispersible tablet	-	

Drugs (concomitant)

4 Decortin	Unknown	40 Mg milligram(s)/Day	-
prednisone			-
5 Dociton	Unknown	15 Mg milligram(s)/Day	-
propranolol hydrochloride			-
6 Konakion	Unknown	3 Df dosage form/Day	-
phytomenadione			-
7 Pantozol	Unknown	20 Mg milligram(s)/Day	-
pantoprazole sodium			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Goitre			Yes	
Hyperthyroidism			Yes	
Colitis ulcerative			Yes	

Primary source

Qualification:	Lawyer	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05009400

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: 19400730 Age: 64 Year weight (kg): 54 BMI_int: 19.13 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic	20050509	20050607	30 Day	26 Day	recovered/resolved
2 Cholestasis	20050509	20050607	30 Day	26 Day	recovered/resolved
3 Jaundice	20050509			26 Day	recovered/resolved
4 Faeces discoloured	20050509			26 Day	recovered/resolved
5 Malaise	20050509			26 Day	recovered/resolved
6 Dizziness	20050509			26 Day	recovered/resolved
7 Blood alkaline phosphatase increased	20050509			26 Day	recovered/resolved
8 Transaminases increased	20050509			26 Day	recovered/resolved
9 Blood bilirubin increased	20050509			26 Day	recovered/resolved
10 Chromaturia	20050509			26 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren 50 diclofenac sodium	Oral	150 Mg milligram(s)/Day	20050414-20050509 26 Day	Shoulder operation

Drugs (concomitant)

2 tramal tramadol hydrochloride	/	-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Shoulder operation	20050414		No	
Pancreatitis NOS	200212		No	
Cholecystectomy			No	about 15 years ago

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05013211

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Peritonitis					recovered/resolved
2 Abdominal pain NOS					recovered/resolved
3 Prothrombin time prolonged					recovered/resolved
4 Pyrexia					recovered/resolved
5 Leukocytosis					recovered/resolved
6 Anaemia NOS					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clindamycin clindamycin	Oral	900 Mg milligram(s)/Day	- 10 Day	Antibacterial prophylaxis
2 diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	- 5 Day	Analgesic effect

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Wisdom teeth removal			Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature: Rostock University
ANZ Journal of Surgery
A: Schiffmann L et.al.
T: Colon perforation in an adolescent after short-term
diclofenac intake
Vol 75, 2005, P: 726-277

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05012409

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No				Yes		

Patient

Initials:	Sex: Male	Birth: 19500823	Age: 54 Year	weight (kg): 92	BMI_int: 27.17	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20050110		7 Day	4 Day	recovered/resolved
2 CRP increased	20050110		7 Day	4 Day	recovered/resolved
3 Pain abdominal	20050110		7 Day	4 Day	recovered/resolved
4 Urine discolouration	20050110			4 Day	Unknown
5 Faeces pale	20050110			4 Day	Unknown
6 Bilirubin total increased	20050110		7 Day	4 Day	recovered/resolved
7 Leukocyte count increased	20050110		7 Day	4 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	100 Mg milligram(s)/Day	20050107-20050110	Rupture of tendon,
diclofenac			4 Day	non-traumatic

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05013494

Serious Yes	Death Yes	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19181112	Age: 86 Year	weight (kg): 63	BMI_int: 21.3	Age fetus:
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Patientdeath

deathdate: 20050927	deathcause: Pneumonia
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal discomfort					Unknown
2 Nausea	200505				Unknown
3 Transaminases increased	200505				Unknown
4 Pneumonia	20050927				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	2 Df dosage form/Week	200503-	Pain in extremity

Drugs (concomitant)

2 ahp oxaceprol	Oral	/	-	
3 delix ramipril	Oral	10 Mg milligram(s)/Day	-	Coronary artery disease NOS
4 insulin insulin		/	-	Diabetes mellitus
5 paracetamol paracetamol		/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Spinal osteoarthritis			Yes	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05013780

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19540824	Age: 50 Year	weight (kg): 00070	BMI_int: 24.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzymes					not recovered/not resolved
2 Diarrhea					not recovered/not resolved
3 Overdose					Unknown
4 Gastrointestinal discomfort					not recovered/not resolved
5 Urine discoloration					not recovered/not resolved
6 Faeces pale					not recovered/not resolved
7 Lack of motivation					not recovered/not resolved
8 Tiredness					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CEFAZOLIN cefazolin	Intravenous (not otherwis	00000002 G gram(s)/	20041021-	Total hip replacement
2 PERFALGAN IV paracetamol	Intravenous (not otherwis	/	20041021-20041022	Total hip replacement
3 SEVORANE sevoflurane		/	20041021-	Total hip replacement
4 voltaren resinat diclofenac resinate		280 Mg milligram(s)/Day	20041021-20041111	Total hip replacement

Drugs (concomitant)

5 acc acetylcysteine	/		20041021-	
6 dipidolor piritramide	/		20041021-	Total hip replacement
7 DISOPRIVAN propofol	/		20041021-	Total hip replacement
8 dolantin pethidine hydrochloride	/		20041021-	Total hip replacement
9 MONO-EMBOLEX heparin	/		20041021-20041210	Total hip replacement
10 nimbex cisatracurium besilate	/		20041021-	Total hip replacement
11 remifentanil hydrochloride remifentanil hydrochloride	/		20041021-	Total hip replacement
12 tranxilium clorazepate dipotassium	/		20041021-	Total hip replacement

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Penicillin allergy	1982		Yes	
Alcohol use		200410	No	prior to Oct-2004.
Hepatitis A virus IgG antibody present	1982		Yes	

Primary source

Qualification:	Physician	observestudytype:
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Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit weitere Erläuterung
C-reactive protein	20051019	2.0	NA
C-reactive protein	2004	0.7	NA
Cholesterol	20051019	238: High	MG/DL
Gamma-glutamyltransferase	20051019	200: High	U/L
Glutamic-pyruvate transaminase	20051019	42.7: High	U/L
International normalised ratio	20041022	0.96	NA
International normalised ratio	20041028	0.88	NA
International normalised ratio	20041028	0.88	NA
Liver function tests		: High	NA
Mean corpuscular haemoglobin	20051019	32.6: High	PG
Mean corpuscular volume	20051019	95.4: High	FL
Serum glutamic-oxaloacetic transferase	20051019	11.5: Normal	U/L
Hepatitis viral test NOS		: Negative	NA
Autoantibody NOS		: Negative	NA
Quick's test	20041108	>130: High	%
Quick's test	20041104	>130: High	%
Quick's test	20041022	116: Normal	%
Quick's test	20041028	>130: High	%
LDL/HDL ratio	20051019	3.4	NA

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 09.02.2006 13100169

Sender diagnosis: not reported

Narrative

A male consumer reported that he experienced increased liver values after receiving an overdose of acetaminophen. The event had persisted for about 10 months on the date of this report. The event outcome was not reported. This report was considered medically important.

Supplemental information received on 27-Jan-2006 reported that a 50-year-old male consumer developed increased hepatic enzymes, chromaturia, pale feces, abdominal discomfort, diarrhea, fatigue, and apathy, after 2 days of therapy with a possible overdose of acetaminophen solution for infusion (dosage not provided) following a total hip replacement. Additional suspect therapy included cephalosporin, sevoflurane, and diclofenac + cholestyramine. Concomitant therapy included heparin, potassium clorazepate, propofol, remifentanyl, cisatracurium, meperidine, pirinitramide, and acetylcysteine. Medical history was significant for positive Hepatitis A antibody, penicillin hypersensitivity, and alcohol use prior to the month of surgery. At the time of reporting, the events had not yet resolved.

Of note, the form that this information was reported on was completely filled out by the consumer and signed by his physician.

As of 07 February 2006, this was the 1st report that contained the MedDRA PTs Chromaturia, Faeces pale, Abdominal discomfort, Fatigue, and/or Apathy, and the 3rd report of Diarrhoea, entered into the CARES safety database coincident with the use of acetaminophen solution for infusion.

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: Under an overdose increase of hepatic enzymes and abdominal discomfort are expected according the current SPC of PERFALGAN. The events 6-8 are non-serious.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05014014

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes						

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20040122	deathcause: Liver failure
deathdate: 20040122	deathcause: Renal failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	200306				fatal
2 Renal failure aggravated	200306				fatal
3 Stomatitis aphthous	200306				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arava leflunomide	Oral	10 Mg milligram(s)/	200304-200306 62 Day	Psoriatic arthritis
2 arava leflunomide	Oral	20 Mg milligram(s)/	200306-20031016 -	Psoriatic arthritis
3 celebrex celecoxib	Oral	200 Mg milligram(s)/	200211-200310 11 Month	Psoriatic arthritis
4 diclo diclofenac	Oral	50 Mg milligram(s)/	- -	Psoriatic arthritis
5 prednisolone prednisolone	Oral	15 Mg milligram(s)/	- -	Psoriatic arthritis

Drugs (concomitant)

6 methotrexate methotrexate		30 Mg milligram(s)/	- -	Psoriatic arthritis
7 methotrexate methotrexate		10 Mg milligram(s)/	200306-200309 3 Month	
8 methotrexate methotrexate		20 Mg milligram(s)/	200302- -	Psoriatic arthritis
9 Morphin morphine		360 Mg milligram(s)/	- -	
10 Morphin morphine		240 Mg milligram(s)/	- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug allergy			Yes	against clindamycine
Injection of knee joint			Yes	emphema
Tachyarrhythmia absoluta			Yes	
Pleural disorder			Yes	round lesion in pulmonary segment
Mucous diarrhea			Yes	
Decubitus			Yes	
Staphylococcal sepsis			Yes	
Obesity			Yes	
Anemia normocytic			Yes	
Psoriatic arthritis			Yes	
unbekannt			Yes	state after fever and chills under infliximab in July 2002
unbekannt			Yes	purulent bronchitis, MCP 3 emphyema in March 2003
unbekannt			No	state after collapse
Cushings syndrome			Yes	under cortisone
Urination impaired			Yes	

Primary source

Qualification: Physician observestudytype:

Literature:

case: DE-BFARM-05014820

Patient

Reactions

Drugs (suspected or interacting)

Drugs (concomitant)

154 / 594

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Podagra	1995		No	
Fever			Unknown	
Appendectomy			No	
Axial hiatal hernia			Unknown	
Gastritis erosive			Unknown	
Helicobacter pylori infection			Unknown	
Smoker			No	
Hip total replacement			Unknown	
Hypertension arterial			Unknown	
Wheezy bronchitis	200502		Unknown	

Primary source

Qualification: Physician observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Coombs direct test	20050319	positiv/positiv	IgG	
Antibodies drug specific		positiv/positive	Diclofenac	
Coombs direct test	20050319	positiv/positive	C3d	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05015869

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19331108	Age: 71 Year	weight (kg): 64	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminase NOS increased	20051102				recovered/resolved
2 Polyarthritits	20051027				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	20051028-20051102	Polyarthritits
diclofenac			6 Day	
2 Marcumar	Oral	1 Df dosage form/Day	200409-20051102	TIA
phenprocoumon		Tablet	-	

Drugs (concomitant)

3 Influenza Virus Vaccine	Intramuscular	/	20051025-20051025	Flu prophylaxis
influenza virus vaccine polyvalent			-	
4 kepinol	Oral	1 Df dosage form/Day	20051020-20051103	UTI
sulfamethoxazole; trimethoprim			15 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tachyarrhythmia absoluta			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	20051105	Anstieg/increase	77.4 U/L	
SGOT	20051105	Anstieg/increase	56.3 U/L	
GGT	20051107	Anstieg/increase	143 U/L	
LDH	20051105	Anstieg/increase	802 U/L	
Investigation	20051105	Anstieg/increase	44.6	
Leukocyte count NOS	20051105	Abfall/decrease	2.7	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05016255

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes			Yes		

Patient

Initials:	Sex: Male	Birth: 19540824	Age: 49 Year	weight (kg): 70	BMI_int: 24.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic	2004				not recovered/not resolved
2 Chromaturia	2004				Unknown
3 Diarrhea NOS	2004				Unknown
4 Dysgeusia	2004				Unknown
5 Gastrointestinal disorder NOS	200410				not recovered/not resolved
6 Transaminases increased	200411				not recovered/not resolved
7 Liver function test abnormal	2004				not recovered/not resolved
8 Ascites	200410				not recovered/not resolved
9 Urine output decreased	200410				Unknown
10 Hypersomnia	200410				Unknown
11 Prothrombin time shortened	20041022		48 Day		Unknown
12 International normalised ratio increased	20041022		48 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mono-embolex certoparin sodium		/	-20041210	-
2 perfalgan paracetamol	Parenteral	2 Df dosage form/	20041022-	-
3 Voltaren Resinat diclofenac resinate		2 Df dosage form/Day	20041021-20041111	Hip surgery 22 Day

Drugs (concomitant)

4 acc long acetylcysteine		/	-20041022	-
5 cephalosporin c cephalosporin c	Parenteral	2 G gram(s)/	20041021-20041021	-
6 dipidolor piritramide		/	-	-
7 nimbex cisatracurium besilate		/	-	-
8 pethidine pethidine		/	-	-
9 remifentanil remifentanil		/	-	-
10 sevorane sevoflurane	Respiratory (inhalation)	/	20041021-20041021	-
11 tranxilium clorazepate dipotassium		/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug hypersensitivity			Yes	
Blood immunoglobulin G increased			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05016969

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19230217	Age: 82 Year	weight (kg): 69	BMI_int: 29.48	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Quick's value decreased	20050630	20050707	8 Day	78 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac diclofenac sodium	Oral	/	20050413-20050620 69 Day	Gonarthrosis
2 diclac diclofenac sodium	Oral	/	20050621-20050630 10 Day	Gonarthrosis
3 MADOPAR benserazide; levodopa	Oral	/ Tablet	20050413-20050630 79 Day	Restless legs

Drugs (concomitant)

4 MAGNESIUM magnesium	Oral	/ Tablet	20050413-20050630 79 Day	Muscle cramps
5 metohexal metoprolol tartrate	Oral	/	20050413- -	Hypertension

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	
Gonarthrosis			Unknown	
Renal cyst			Unknown	
Restless legs			Unknown	
Total knee replacement	20050413		Unknown	
Cramps			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hemoglobin	20050630	8.2	NORMAL RANGE	RANGES NOT PROVIDED.
Quick's test	20050630	12	UNIT REPORTED AS %.	NORMAL RANGE NOT PROVIDED.

Narratives (nur bei elektronischem Eingang)

ROCHE, 30.01.2006 411814

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS AN 82 YEAR OLD FEMALE PATIENT WHO EXPERIENCED DECREASED QUICK VALUE DURING THE USE OF BENSERAZIDE/LEVODOPA, (MADOPAR) FOR RESTLESS LEGS AND DICLOFENAC (DICLAC) FOR GONARTHROSIS.

CONCOMITANT MEDICATION INCLUDED ORAL METOPROLOL (METOHEXAL SUCC) FOR HYPERTENSION. MEDICAL HISTORY INCLUDED BILATERAL PARAHILAR RENAL CYST AND STATUS POST TOTAL KNEE REPLACEMENT.

THE PATIENT COMMENCED ORAL BENSERAZIDE/LEVODOPA, 2.5 TABLETS A DAY (375 MG A DAY) AND ORAL DICLOFENAC, 100 MG A DAY. EIGHT DAYS LATER THE DOSE OF DICLOFENAC WAS REDUCED TO 50 MG A DAY. NINE DAYS LATER THE PATIENT DEVELOPED MACROHAEMATURIA (HEMOGLOBIN DECREASED TO 8.2 G/DL, FROM A BASELINE OF 11 G/DL) CAUSED BY A DECREASED QUICK VALUE OF 12%, THE PATIENT REQUIRED HOSPITALISATION AND THERAPY WITH BENSERAZIDE/LEVODOPA AND DICLOFENAC WAS DISCONTINUED. THE PATIENT WAS TREATED WITH PHYTOMENADIONE, (KONAKION) IMPLANTS (UNSPECIFIED) AND PHYSIOTHERAPY. THE EVENT OF DECREASED QUICK VALUE RESOLVED ON APPROXIMATELY 7 JULY 2005. SUBSEQUENT INVESTIGATIONS OF ONCOLOGY AND HAEMATOLOGY FOUND NOTHING ABNORMAL.

THE REPORTER CONSIDERED THE DECREASED QUICK VALUE TO BE POSSIBLY RELATED TO FOR BENSERAZIDE/LEVODOPA AND DICLOFENAC. NO FURTHER INFORMATION WAS AVAILABLE.

UPDATE INFORMATION WAS RECEIVED ON 24 NOVEMBER 2005; THE FOLLOWING INFORMATION WAS ADDED TO THE CASE: FURTHER PATIENT AND REPORTER DETAILS, THERAPY DETAILS, TREATMENT DETAILS AND MEDICAL HISTORY. THE EVENT OF DECREASED HAEMOGLOBIN WAS DELETED AS IT HAS BEEN REPLACED BY MACROHAEMATURIA, A COMANIFESTATION FOR DECREASED QUICK VALUE.

*** DATA ELEMENTS THAT WERE TRUNCATED NOW FOLLOW ***

<ichicsr.safetyreport.patient.test(testname=10053076)>

<testresult>

NOTHING ABNORMAL WAS FOUND AFTER RECOVERY OF THE PATIENT.

</testresult>

<ichicsr.safetyreport.patient.test(testname=10049506)>

<testresult>

AN ONCOLOGY INVESTIGATION SHOWED NOTHING ABNORMAL AFTER RECOVERY OF THE PATIENT.

</testresult>

case: DE-BFARM-05017499

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes					Yes		

Initials: Sex: Male Birth: 19860923 Age: 19 Year weight (kg): 77 BMI int: 22.74 Age fetus:

Reactions

MedDRA LLT		Begin	End	duration	TtO	outcome
1	Lipase increased	20051123			1 Day	recovered/resolved
2	Abdominal pain upper	20051123			1 Day	recovered/resolved
3	Ascites	20051123				recovered/resolved
4	Pancreatitis NOS	20051123		5 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ethanol		/	-	
ethanol			-	
2 voltaren resinat	Oral	75 Mg milligram(s)/Day	20051123-20051125	Intercostal neuralgia
diclofenac resinate			3 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06005884

Serious Yes	Death Yes	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multi-organ failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Multi-organ failure					fatal
2 Intestinal infarction					fatal
3 Haemoglobin decreased					Unknown
4 Ileus paralytic					fatal
5 Jaundice					Unknown
6 Back pain					Unknown
7 Shock circulatory					recovered/resolved
8 Anaemia haemolytic autoimmune					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	American journal of hematology, title: misdiagnosis in patients with diclofenac-induced hemolysis, vol.81,year:2006,pages:128-131	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06006863

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaemia haemolytic autoimmune					recovered/resolved
2 Jaundice NOS					recovered/resolved
3 Haemoglobin decreased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	american journal of hematology- title:misdiagnosis in patients with diclofenac-induced hemolysis- vol.81,year:2006,pages:128-131	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06006864

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Haemoglobin decreased					recovered/resolved
2 Jaundice					recovered/resolved
3 Anaemia haemolytic autoimmune					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Charité-Universitätsmedizin Berlin American Journal of Hematology A: Ahrens N et al T: Misdiagnosis in patients with diclofenac-induced hemolysis: New cases and a concise review Vol. 81, 2006, P: 128-131	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06006876

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice NOS					recovered/resolved
2 Hb decreased					recovered/resolved
3 Anaemia haemolytic autoimmune					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	american journal of hematology, misdiagnosis in patients with diclofenac-induced hemolysis- vol.81,year 2006,pages 128-131	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06006915

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice NOS					recovered/resolved
2 Hb decreased					recovered/resolved
3 Anaemia haemolytic autoimmune					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	american journal of hematology- Title: misdiagnosis in patients with diclofenac-induced hemolysis- vol.81,year:2006,pages:128-131	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06006917

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 52 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin disorder					recovered/resolved
2 Eosinophil count increased	200608				Unknown
3 Herpes zoster					recovered/resolved
4 Acute generalised exanthematous pustulosis	200608				recovered/resolved
5 Pyrexia					recovered/resolved
6 Streptococcus serology positive					recovered/resolved
7 C-reactive protein increased					recovered/resolved
8 Leucocytosis NOS					recovered/resolved
9 Swelling face					recovered/resolved
10 Malaise					recovered/resolved
11 GPT increased					recovered/resolved
12 GOT increased					recovered/resolved
13 GGT increased					recovered/resolved
14 Trichophyton infection					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	Arthropathy
diclofenac			-	
2 phenoxymethylpenicillin potassium	Oral	/	-	Nasopharyngitis
phenoxymethylpenicillin potassium			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Birte Prange, Alessandra Marini, et al. "Acute localized exanthematous pustulosis (ALEP)" J Dtsch Dermatol Ges 2005: (Vol. 3): 210-212	

Narratives (nur bei elektronischem Eingang)

Merck KGaA, 24.05.2006 DE-Merck KGaA-6022315

Sender diagnosis: not reported

Narrative

Literature report received on 28 Apr 2006.

*

A 52-year-old female patient developed acute localised exanthematous pustulosis after intake of penicillin and diclofenac. She presented with a common cold with sore throat, and one day after penicillin therapy, she complained of generalised malaise, redness and swelling in front of the right ear and on the right cheek as well as in the area of the right lower eyelid, and fever. At the time of presentation, she showed a large, painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localised exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with penicillin and diclofenac was negative. The reporters state that the event was probably caused by penicillin or diclofenac.

Note: The authors do not suspect an interaction between the both drugs taken by the patient. It is not clear which one of either drug was causative in the end.

The events are known as possible side effects of diclofenac.

The suspected "penicillin" was not further specified, e.g. if penicillin G or penicillin V or another derivate was concerned.

Reporter's comment:ratiopharm GmbH, 14.07.2006 DE-RATIOPH-2006R072034

Sender diagnosis: not reported

Narrative

A 52-year-old female patient developed acute localised exanthematous pustulosis after intake of Penicillin and Diclofenac. She presented with a common cold with sore throat, and one day after Penicillin therapy, she complained of generalised malaise, redness and swelling in front of the right ear and on the right cheek as well as in the area of the right lower eyelid, and fever. At the time of presentation, she showed a large, painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localised exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with Penicillin and Diclofenac was negative. The reporters state that the event was probably caused by Penicillin or Diclofenac. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No spectrum of possible side effects whereas ALEP is a new diagnosis, known mainly in dermatology specialty.

Reporter's comment:ratiopharm GmbH, 14.07.2006 DE-RATIOPH-2006R072034

Sender diagnosis: not reported

Narrative

A 52-year-old female patient developed acute localised exanthematous pustulosis after intake of Penicillin and Diclofenac. She presented with a common cold with sore throat, and one day after Penicillin therapy, she complained of generalised malaise, redness and swelling in front of the right ear and on the right cheek as well as in the area of the right lower eyelid, and fever. At the time of presentation, she showed a large, painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localised exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with Penicillin and Diclofenac was negative. The reporters state that the event was probably caused by Penicillin or Diclofenac. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No spectrum of possible side effects whereas ALEP is a new diagnosis, known mainly in dermatology specialty.

Reporter's comment:ratiopharm GmbH, 14.07.2006 DE-RATIOPH-2006R072034

Sender diagnosis: not reported

Narrative

A 52-year-old female patient developed acute localised exanthematous pustulosis after intake of Penicillin and Diclofenac. She presented with a common cold with sore throat, and one day after Penicillin therapy, she complained of generalised malaise, redness and swelling in front of the right ear and on the right cheek as well as in the area of the right lower eyelid, and fever. At the time of presentation, she showed a large, painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localised exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with Penicillin and Diclofenac was negative. The reporters state that the event was probably caused by Penicillin or Diclofenac.

CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No spectrum of possible side effects whereas ALEP is a new diagnosis, known mainly in dermatology speciality.

Reporter's comment:

ratiopharm GmbH, 14.07.2006 DE-RATIOPH-2006R072034

Sender diagnosis: not reported

Narrative

A 52-year-old female patient developed acute localised exanthematous pustulosis after intake of Penicillin and Diclofenac. She presented with a common cold with sore throat, and one day after Penicillin therapy, she complained of generalised malaise, redness and swelling in front of the right ear and on the right cheek as well as in the area of the right lower eyelid, and fever. At the time of presentation, she showed a large, painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localised exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with Penicillin and Diclofenac was negative. The reporters state that the event was probably caused by Penicillin or Diclofenac. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No spectrum of possible side effects whereas ALEP is a new diagnosis, known mainly in dermatology speciality.

Reporter's comment:

SANOFI-SYNTHELABO, 12.09.2006 DE-SANOFI-SYNTHELABO-A04200600858

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthema pustulosis
LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: increase of eosinophils
LLT: EOSINOPHIL COUNT INCREASED / 10014945
PT: EOSINOPHIL COUNT INCREASED / 10014945

Narrative:

Initial report received 30-Aug-2006

A hospital physician reported via phone about a 69-year-old female patient who was treated with terbinafine HCL (Octosan) at a dose of 250 mg. Indication not known. After several days of treatment (onset: unknown), she presented acute generalised exanthema (pustulosis) with increase of eosinophils. No medical history, underlying diseases or concomitant medication reported.

At the time of report no outcome was mentioned.

Further information will be requested.

SANOFI-SYNTHELABO, 04.10.2006 DE-SANOFI-SYNTHELABO-A04200600858

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthema pustulosis (AGEP)
LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: increase of eosinophils
LLT: EOSINOPHIL COUNT INCREASED / 10014945
PT: EOSINOPHIL COUNT INCREASED / 10014945

Narrative:

Initial report received 30-Aug-2006

A hospital physician reported via phone about a 69-year-old female patient who was treated with terbinafine HCL (Octosan) at a dose of 250 mg. Indication not known. After several days of treatment (onset: unknown), she presented acute generalised exanthema (pustulosis) with increase of eosinophils. No medical history, underlying diseases or concomitant medication reported.

At the time of report no outcome was mentioned.

Further information will be requested.

Addendum received on 20-Sep-2006

The patient was hospitalised in August 2006 because of acute generalised exanthema pustulosis (AGEP). Octosan (terbinafine HCL) was discontinued. The patient was treated with prednisone at a dose of 80 mg daily without success. Again occurrence of pustules on the whole body mainly on extremities. In the 3rd week dose was increased to 90 mg. No change of patients situation. Dosage was reduced to maintenance dose of 10 mg in week five of hospitalisation.

Local treatment with betamethasone hydrogenphosphate-dinatrium ointment. In week six the patient is still hospitalised. Outcome: ongoing.

Results of smear of pustules, skin and foot showed massive staphylococcus aureus. X-ray of the thorax was negative. Sonography of upper abdomen was negative. Inguinal sonography showed reactive lymph nodes. Microscopic result of skin of neck: showed suspicion of drug induced exanthema.

Configured erythema on the whole inguiment.

The reporting physician assessed the case as "highly probable".

No further information is awaited.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06006554

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials: Sex: Male Birth: 19540824 Age: 50 Year weight (kg): 00070 BMI_int: 24.22 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic drug-induced					not recovered/not resolved
2 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin w/clavulanic acid		/	-	
amoxicillin; clavulanate potassium			-	
2 CEFAZOLIN	Intravenous (not otherwis	00000002 G gram(s)/	20041021-	Total hip replacement
cefazolin			-	
3 HEPARINE		/	-	
heparin sodium			-	
4 perfalgan IV		00000001 G gram(s)/	20041021-20041022	Total hip replacement
paracetamol		124	2 Day	
5 SEVORANE		/	20041021-	Total hip replacement
sevoflurane			-	
6 voltaren	Oral	00000075 Mg milligram(s)	20041021-20041022	
diclofenac sodium			2 Day	
7 Voltaren Resinat		280 Mg milligram(s)/Day	20041021-20041111	Total hip replacement
diclofenac resinate			22 Day	

Drugs (concomitant)

8 acc		/	20041021-	Total hip replacement
acetylcysteine			-	
9 dipidolor	Intravenous (not otherwis	00000011 Mg milligram(s)	20041021-	Total hip replacement
piritramide			-	
10 DISOPRIVAN		/	20041021-	Total hip replacement
propofol			-	
11 dolantin		00000020 Mg milligram(s)	20041021-	Total hip replacement
pethidine hydrochloride			-	
12 MONO-EMBOLEX		/	20041021-20041210	Total hip replacement
heparin			51 Day	
13 nimbex		/	20041021-	Total hip replacement
cisatracurium besilate			-	
14 remifentanil hydrochloride		/	20041021-	Total hip replacement
remifentanil hydrochloride			-	
15 tranxilium		/	20041021-	Total hip replacement
clorazepate dipotassium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Penicillin allergy	1982		Unknown	
Alcohol use		200410	Unknown	prior to Oct-2004.
Unspecified disorder of liver	1981		Unknown	
Transaminases increased	1981		Unknown	after receiving mebendazol.
Infection NOS	1981		Unknown	
Chromosomal mutation			Unknown	Heterocygous mutation C282Y and H63 D without HFE-manifestation
Hip dysplasia			Unknown	bilateral
Hepatitis A virus IgG antibody present	1982		Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function tests		: High	NA	
Quick's test	20041022	116: Normal	%	
Quick's test	20041028	>130: High	%	
Quick's test	20041104	>130: High	%	
Quick's test	20041108	>130: High	%	
International normalised ratio	20041022	0.96	NA	
International normalised ratio	20041028	0.88	NA	
International normalised ratio	20041028	0.88	NA	
Serum glutamic-oxaloacetic transferase	20051019	11.5: Normal	U/L	
Glutamic-pyruvate transaminase	20051019	42.7: High	U/L	
Gamma-glutamyltransferase	20051019	200: High	U/L	
Cholesterol	20051019	238: High	MG/DL	
LDL/HDL ratio	20051019	3.4	NA	
Mean corpuscular volume	20051019	95.4: High	FL	
Mean corpuscular haemoglobin	20051019	32.6: High	PG	
C-reactive protein	2004	0.7	NA	
Hepatitis viral test NOS		: Negative	NA	
Autoantibody NOS		: Negative	NA	
C-reactive protein	20051019	2.0	NA	
Glutamic-oxaloacetic transferase	20041130	61: High	U/L	
Glutamic-pyruvate transaminase	20041130	206: High	U/L	
Gamma GT	20041130	705: High	U/L	
Gamma GT	200507	158: High	U/L	
Glutamic-oxaloacetic transferase	200507	20: Normal	U/L	
Glutamic-pyruvate transaminase	200507	36: Normal	U/L	
Alkaline phosphatase	20041130	188: High	U/L	
Alkaline phosphatase	20050421	103: Normal	U/L	
Ultrasound abdomen		: Normal	NA	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 20.02.2006 DE-Bristol-Myers Squibb Company-13100169

Sender diagnosis: not reported

Narrative

A male consumer reported that he experienced increased liver values after receiving an overdose of acetaminophen. The event had persisted for about 10 months on the date of this report. The event outcome was not reported. This report was considered medically important.

Supplemental information received on 27-Jan-2006 reported that a 50-year-old male consumer developed increased hepatic enzymes, chromaturia, pale feces, abdominal discomfort, diarrhea, fatigue, and apathy, after 2 days of therapy with a possible overdose of acetaminophen solution for infusion (dosage not provided) following a total hip replacement. Additional suspect therapy included cephalosporin, sevoflurane, and diclofenac + cholestyramine. Concomitant therapy included heparin, potassium clorazepate, propofol, remifentanyl, cisatracurium, meperidine, pirinitramide, and acetylcysteine. Medical history was significant for positive Hepatitis A antibody, penicillin hypersensitivity, and alcohol use prior to the month of surgery. At the time of reporting, the events had not yet resolved.

Of note, the form that this information was reported on was completely filled out by the consumer and signed by his physician.

As of 07 February 2006, this was the 1st report that contained the MedDRA PTs Chromaturia, Faeces pale, Abdominal discomfort, Fatigue, and/or Apathy, and the 3rd report of Diarrhoea, entered into the CARES safety database coincident with the use of acetaminophen solution for infusion.

Follow-up information was received from the physician on 07-Feb-2006 and 10-Feb-2006. The patient received 1 gram of acetaminophen on 21-Oct-2004 at 0930 hours. The patient had been diagnosed with acute drug toxic hepatitis after the administration of non-steroidal anti-rheumatics, heparine and amoxicillin + clavulanic acid. Therefore, heparine and amoxicillin + clavulanic acid were added as suspect drugs. The acute drug toxic hepatitis was characterized by dark urine, nearly colorless, pale stool, severe gastrointestinal discomfort with diarrhea, tiredness, lack of drive, and pathological liver values and pathological INR and Quick values. Following the administration of the acetaminophen IV, the patient had a marked increased in gamma-GT of up to 705 U/L (nl: <55 U/L). His GOT increased to 61 (nl: <35), GPT increased to 206 (nl: <45U/L), and alkaline phosphatase increased to 188 (nl: 56-119 U/L). Other tests included an ultrasound of the abdomen that was without pathological findings. The patient used no permanent medication and he had no alcohol consumption. On an unspecified date after 21-Jul-2005 the gamma-gt value was improved to 158, the GOT and GPT values were normal. The events were considered probably related to the acetaminophen therapy.

Medical history also included liver disease with increased transaminases, unclear infection after a stay in a foreign country, increased in transaminases after receiving mebendazole, heterocygous mutation C282Y and H63 D without HFE manifestation, and bilateral hip dysplasia.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06023231

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth:	Age: 31 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anuria					not recovered/not resolved
2 Application site pain					not recovered/not resolved
3 Ascites					not recovered/not resolved
4 Brain disorder NOS					fatal
5 Chills					not recovered/not resolved
6 Circulatory failure					fatal
7 CK increased					not recovered/not resolved
8 Coagulation disorder					not recovered/not resolved
9 Condition aggravated					fatal
10 CRP increased					not recovered/not resolved
11 Fever					not recovered/not resolved
12 Hypoglycaemia					not recovered/not resolved
13 Infection					not recovered/not resolved
14 Intracerebral hemorrhage					fatal
15 Leukopenia					not recovered/not resolved
16 Liver failure					not recovered/not resolved
17 Multiorgan failure					fatal
18 Muscle atrophy					fatal
19 Neurological disorder NOS					fatal
20 Pain					not recovered/not resolved
21 Phlegmon					fatal
22 Phlegmon					not recovered/not resolved
23 Rhabdomyolysis					not recovered/not resolved
24 Sepsis					fatal
25 Septic shock					not recovered/not resolved
26 Somnolence					fatal
27 Streptococcal toxic shock syndrome					not recovered/not resolved
28 Thrombocytopenia					not recovered/not resolved
29 Corneal ulcer					fatal
30 Application site swelling					not recovered/not resolved
31 Inflammation					not recovered/not resolved
32 Corneal infection					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac sodium	Intramuscular	/	-	
diclofenac sodium			-	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 09.05.2007 DE-002147023-PHNU2006DE01031

Sender diagnosis: not reported

Narrative

Initial report received on 15 Feb 2006. Case received from German Health Authority, originating source German drug safety publication called "arzneitelegramm" (case with individual case number). Scarce information concerning a patient who suffered from shock while under therapy with diclofenac sodium (drug not further specified), for unstated reasons.

Initial report received on 9 Mar 2006 (case from database of German drug safety publication called "arzneitelegramm", case number 10239). This patient received an i.m injection with a diclofenac drug of an unknown manufacturer (no dates provided). Patient showed severe chills, severe pain in the thumb area and increasing pain at the application site area (gluteal right-sided). Two days after injection, patient was hospitalised. Laboratory as follows: CRP 56mg/dl, leukopenia, onset of thrombocytopenia, fever up to 41 degrees Celsius. Patient presented extended shock symptomatology, and soft part tissue around injection site was extremely swollen. The diagnosis of necrotising soft part infection was done via MR diagnostics, and patient underwent immediate surgery. On the evening of the operation day, patient showed full signs of fulminant septic shock. Leukocyte counts were 1200/microl, inflammation parameters were high. Patient presented derailed thrombocytic and plasmatic coagulation and severe tendency for hypoglycaemia as sign of liver failure and anuria. On the following day, patient underwent hemofiltration, received artificial respiration, and was treated with antibiotics (Zienam, vancomycin). Due to an extended retroperitoneal phlegmon and ascites, patient underwent a further operation, including cholecystectomy. Operation smears and blood cultures gave evidence of beta-hemolysing streptococci group A, and the diagnosis of streptococcal toxic shock syndrome was made. Patient showed significant rhabdomyolysis (CK 83.400U/l) and a corneal infection of the right eye. After switch to high dose penicillin, clindamycin, and gentamicin and treatment with immunoglobulin M during the following weeks, patient developed persisting multi organ failure. Patient showed from several critic exacerbations with chronic sepsis and several operations. The patient's condition worsened 6.5 weeks after the injection, with somnolence. A right intracerebral mass haemorrhage, probably starting from basal ganglia, had to be surgically removed. During the following 4 weeks, persisting septic circulatory failure, impending brain stem incarceration, extended phlegmonous infiltrations in retroperitoneal, gluteal, and thigh regions, ulcerations of corneal infection, neurological deficit with persisting somnolence, and increasing muscle atrophy developed. Patient finally died as a consequence of multiorgan failure.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06010437

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth:	Age: 79 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hemolytic anemia drug-induced					recovered/resolved
2 Shock					recovered/resolved
3 Jaundice					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac sodium	Unknown	/	-	Drug use for unknown indication
diclofenac sodium			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	Ahrens N et al. Misdiagnosis in Patients With Diclofenac-Induced Hemolysis: New Cases and a Concise Review Am J Hema 2006; 81:128-131.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 20.03.2006 DE-GlaxoSmithKline-B0416252A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug-induced immune hemolytic anemia (IHA) in a 79-year-old female patient who received Diclofenac sodium for an unknown drug indication.

On an unknown date, the patient started Diclofenac sodium at an unknown dosing regime. At an unknown time after starting Diclofenac sodium, the patient experienced hemolytic anemia drug-induced, shock and jaundice. The patient was hospitalised.

It was noted that Diclofenac-dependent IHA was not suspected on admission. The patient developed jaundice and was initially thought to have autoimmune hemolytic anemia (AIHA) due to warm-reactive autoantibodies. Diclofenac- and/or ex-vivo-antigen-dependent red cell antibodies were detected. The serologic test results at the laboratory suggested drug-induced IHA. Treatment with Diclofenac sodium was discontinued. At the time of reporting, the events were resolved. The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "misdiagnosis of diclofenac-induced IHA may be due to limited awareness of drug-dependent hemolysis and/or to oversimplification of serologic investigations."

CARINO, 24.05.2006 DE-CARINO-20060523-01

Sender diagnosis: not reported

Narrative

A 79-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced shock and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 6.9 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that diclofenac-induced immune haemolytic anaemia is far more common than previously assumed.

CAUSALITY ASSESSMENT FOR diclofenac:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: unknown

History of Observed Event: unknown

Other Risk Factors: unknown

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

Reporter's comment:

Merck KGaA, 14.06.2006 DE-Merck KGaA-6023111

Sender diagnosis: not reported

Narrative

Case report from literature received on 06-Jun-2006.

*

Ahrens N, Genth R, Kiesewetter H, Salama A.

Misdiagnosis in patients with diclofenac-induced hemolysis: new cases and a concise review. Am J Hematol 2006; 81: 128-131

*

A 79-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced shock and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 6.9 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that "diclofenac-induced immune haemolytic anaemia is far more common than previously assumed".

See also cases # 6023108, # 6023113, # 6023115, #6023116, # 6023117, # 6023118, # 6023120, # 6023121 belonging to the same publication.

Overall company causality assessment of relationship drug / adverse reaction: possible. Haemolytic anaemia and shock are known as possible adverse reactions to diclofenac.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06010445

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 60 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hemolytic anemia drug-induced					recovered/resolved
2 Jaundice					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac sodium			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Ahrens N et al. Misdiagnosis in Patients With Diclofenac-Induced Hemolysis: New Cases and a Concise Review Am J Hema 2006; 81:128-131.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 20.03.2006 DE-GlaxoSmithKline-B0416267A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug-induced immune hemolytic anemia (IHA) in a 60-year-old male patient who received Diclofenac sodium for an unknown drug indication.

On an unknown date, the patient started Diclofenac sodium at an unknown dosing regime. At an unknown time after starting Diclofenac sodium, the patient experienced hemolytic anemia drug-induced and jaundice. The patient was hospitalised and the author considered the events were clinically significant (or requiring intervention). The patient was treated with glucocorticoids, IgG and red blood cells.

It was noted that Diclofenac-dependent IHA was not suspected on admission. The patient developed jaundice and was initially thought to have autoimmune hemolytic anemia (AIHA) due to warm-reactive autoantibodies. Diclofenac-and/or ex-vivo-antigen-dependent red cell antibodies were detected. The serologic test results at the laboratory suggested drug-induced IHA. Treatment with Diclofenac sodium was discontinued. At the time of reporting, the events were resolved. The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "misdiagnosis of diclofenac-induced IHA may be due to limited awareness of drug-dependent hemolysis and/or to oversimplification of serologic investigations."

CARINO, 24.05.2006 DE-CARINO-20060523-02

Sender diagnosis: not reported

Narrative

A 60-year-old male patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 5.0 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that diclofenac-induced immune haemolytic anaemia is far more common than previously assumed.

CAUSALITY ASSESSMENT FOR diclofenac:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: unknown

History of Observed Event: unknown

Other Risk Factors: unknown

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

Reporter's comment:

Merck KGaA, 14.06.2006 DE-Merck KGaA-6023113

Sender diagnosis: not reported

Narrative

Case report from literature received on 06-Jun-2006.

*

Ahrens N, Genth R, Kiesewetter H, Salama A.

Misdiagnosis in patients with diclofenac-induced hemolysis: new cases and a concise review.

Am J Hematol 2006; 81: 128-131

*

A 60-year-old male patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 5.0 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that "diclofenac-induced immune haemolytic anaemia is far more common than previously assumed".

*

See also cases # 6023108, # 6023111, # 6023115, # 6023116, # 6023117, # 6023118, # 6023120, # 6023121 belonging to the same publication.

Overall company causality assessment or relationship drug / adverse reaction: possible. Haemolytic anaemia is known as possible adverse reaction to diclofenac.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06010711

Serious Yes	Death Yes	Lifethr.	Hospital. Yes	Disabling	Congenital	Other No	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 82 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting					Unknown
2 Multi-organ failure					fatal
3 Hemolytic anemia drug-induced					Unknown
4 Lumbar pain					Unknown
5 Jaundice					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown
diclofenac sodium			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	Ahrens N et al. Misdiagnosis in Patients With Diclofenac-Induced Hemolysis: New Cases and a Concise Review Am J Hema 2006; 81:128-131.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 21.03.2006 DE-GlaxoSmithKline-B0416269A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug-induced immune hemolytic anemia (IHA) in a 82-year-old female patient who received Diclofenac sodium for an unknown drug indication.

On an unknown date, the patient started Diclofenac sodium at an unknown dosing regime. At an unknown time after starting Diclofenac sodium, the patient experienced hemolytic anemia drug-induced, lumbar pain, jaundice, vomiting and multiorgan failure. The patient was hospitalised and the authors considered the events were clinically significant (or requiring intervention). The patient was treated with glucocorticoids.

It was noted that Diclofenac-dependent IHA was not suspected on admission. Diclofenac- and/or ex-vivo-antigen-dependent red cell antibodies were detected. The serologic test results at the laboratory suggested drug-induced IHA. It was noted that the patient died 20 hours after the last diclofenac

application. The cause of death is unknown. It is unknown whether an autopsy was performed. The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "misdiagnosis of diclofenac-induced IHA may be due to limited awareness of drug-dependent hemolysis and/or to oversimplification of serologic investigations."

CARINO, 24.05.2006 DE-CARINO-20060523-04

Sender diagnosis: not reported

Narrative

An 82-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced lumbar pain, jaundice and vomiting and was erroneously diagnosed with mesenteric infarction and paralytic ileus on admission. Haemoglobin was 9.1 g/dl. Multi-organ failure occurred from which the patient died. Drug-induced antibodies against diclofenac and/or its metabolites were identified. The reporters state that a diclofenac-induced immune haemolytic anaemia is far more common than previously assumed.

CAUSALITY ASSESSMENT FOR diclofenac:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: unknown

History of Observed Event: unknown

Other Risk Factors: unknown

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

Reporter's comment:

Merck KGaA, 14.06.2006 DE-Merck KGaA-6023120

Sender diagnosis: not reported

Narrative

Case report from literature received on 06-Jun-2006.

*

Ahrens N, Genth R, Kiesewetter H, Salama A.

Misdiagnosis in patients with diclofenac-induced hemolysis: new cases and a concise review.

Am J Hematol 2006; 81: 128-131

*

An 82-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced lumbar pain, jaundice and vomiting and was erroneously diagnosed with mesenteric infarction and paralytic ileus on admission. Haemoglobin was 9.1 g/dl. Multi-organ failure occurred from which the patient died. Drug-induced antibodies against diclofenac and/or its metabolites were identified. The reporters state that "diclofenac-induced immune haemolytic anaemia is far more common than previously assumed".

*

See also cases # 6023108, # 6023111, # 6023113, # 6023115, # 6023116, # 6023117, # 6023118, # 6023121 belonging to the same publication.

Overall company causality assessment or relationship drug / adverse reaction: possible.

Haemolytic anaemia is known as possible adverse reaction to diclofenac. Multi-organ failure s unlisted.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06010712

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes			No		

Patient

Initials:	Sex: Female	Birth:	Age: 63 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hemolytic anemia drug-induced					recovered/resolved
2 Jaundice					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown
diclofenac sodium			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	Ahrens N et al. Misdiagnosis in Patients With Diclofenac-Induced Hemolysis: New Cases and a Concise Review Am J Hema 2006; 81:128-131.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 21.03.2006 DE-GlaxoSmithKline-B0416270A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug-induced immune hemolytic anemia (IHA) in a 63-year-old female patient who received Diclofenac sodium for an unknown drug indication.

On an unknown date, the patient started Diclofenac sodium at an unknown dosing regime. At an unknown time after starting Diclofenac sodium, the patient experienced hemolytic anemia drug-induced and jaundice. The patient was hospitalised.

It was noted that Diclofenac-dependent IHA was not suspected on admission. The patient developed jaundice and was initially thought to have autoimmune hemolytic anemia (AIHA) due to warm-reactive autoantibodies. Diclofenac- and/or ex-vivo-antigen-dependent red cell antibodies were detected. The serologic test results at the laboratory suggested drug-induced IHA. Treatment with Diclofenac sodium was discontinued. At the time of reporting, the events were resolved. The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "misdiagnosis of diclofenac-induced IHA may be due to limited awareness of drug-dependent hemolysis and/or to oversimplification of serologic investigations."

CARINO, 24.05.2006 DE-CARINO-20060523-05

Sender diagnosis: not reported

Narrative

A 63-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.3 g/dl. Diclofenac was discontinued. It was unknown if the patient experienced any complications but she recovered completely. Drug-induced antibodies against diclofenac and/or its anti-bodies were identified. The reporters state that a diclofenac-induced immune haemolytic anaemia is far more common than previously assumed.

CAUSALITY ASSESSMENT FOR diclofenac:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: unknown

History of Observed Event: unknown

Other Risk Factors: unknown

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

Reporter's comment:

Merck KGaA, 14.06.2006 DE-Merck KGaA-6023117

Sender diagnosis: not reported

Narrative

Case report from literature received on 06-Jun-2006.

*

Ahrens N, Genth R, Kieseewetter H, Salama A.

Misdiagnosis in patients with diclofenac-induced hemolysis: new cases and a concise review.

Am J Hematol 2006; 81: 128-131

*

A 63-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.3 g/dl. Diclofenac was discontinued. It was unknown if the patient experienced any complications but she recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that "diclofenac-induced immune haemolytic anaemia is far more common than previously assumed".

*

See also cases # 6023108, # 6023111, # 6023113, # 6023115, # 6023116, # 6023118, # 6023120, # 6023121 belonging to the same publication.

Overall company causality assessment or relationship drug / adverse reaction: possible. Haemolytic anaemia is known as possible adverse reaction to diclofenac.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06010715

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes			Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice					recovered/resolved
2 Hemolytic anemia drug-induced					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac sodium			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	Ahrens N et al. M Misdiagnosis in Patients With Diclofenac-Induced Hemolysis: New Cases and a Concise Review Am J Hema 2006; 81:128-131.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 21.03.2006 DE-GlaxoSmithKline-B0416271A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug-induced immune hemolytic anemia (IHA) in a 75-year-old male patient who received Diclofenac sodium for an unknown drug indication.

On an unknown date, the patient started Diclofenac sodium at an unknown dosing regime. At an unknown time after starting Diclofenac sodium, the patient experienced hemolytic anemia drug-induced and jaundice. The patient was hospitalised and the authors considered the events were clinically significant (or requiring intervention). The patient was treated with red blood cells.

It was noted that Diclofenac-dependent IHA was not suspected on admission. The patient developed jaundice and was initially thought to have autoimmune hemolytic anemia (AIHA) due to warm-reactive autoantibodies. Diclofenac- and/or ex-vivo-antigen-dependent red cell antibodies were detected. The serologic test results at the laboratory suggested drug-induced IHA. Treatment with Diclofenac sodium was discontinued. At the time of reporting, the events were resolved. The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "misdiagnosis of diclofenac-induced IHA may be due to limited awareness of drug-dependent hemolysis and/or to oversimplification of serologic investigations."

CARINO, 24.05.2006 DE-CARINO-20060523-06

Sender diagnosis: not reported

Narrative

A 75-year-old male patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.1 g/dl. Diclofenac was discontinued and the patient recovered completely. The reporters state that a diclofenac-induced immune haemolytic anaemia is far more common than previously assumed.

Sender's comment:

CAUSALITY ASSESSMENT FOR diclofenac:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: unknown

History of Observed Event: unknown

Other Risk Factors: unknown

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

Reporter's comment:

Merck KGaA, 14.06.2006 DE-Merck KGaA-6023118

Sender diagnosis: not reported

Narrative

Case report from literature received on 06-Jun-2006.

*

Ahrens N, Genth R, Kiesewetter H, Salama A.

Misdiagnosis in patients with diclofenac-induced hemolysis: new cases and a concise review.

Am J Hematol 2006; 81: 128-131

*

A 75-year-old male patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.1 g/dl. Diclofenac was discontinued and the patient recovered completely. The reporters state that "diclofenac-induced immune haemolytic anaemia is far more common than previously assumed".

*

See also cases # 6023108, # 6023111, # 6023113, # 6023115, # 6023116, # 6023117, # 6023120, # 6023121 belonging to the same publication.

Overall company causality assessment or relationship drug / adverse reaction: possible. Haemolytic anaemia is known as possible adverse reaction to diclofenac.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06010719

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes			Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 74 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Shock					Unknown
2 Hemolytic anemia drug-induced					recovered/resolved
3 Jaundice					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac sodium			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	Ahrens N et al. Misdiagnosis in Patients With Diclofenac-Induced Hemolysis: New Cases and a Concise Review Am J Hema 2006; 81:128-131.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 21.03.2006 DE-GlaxoSmithKline-B0416272A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug-induced immune hemolytic anemia (IHA) in a 74-year-old male patient who received Diclofenac sodium for an unknown drug indication.

On an unknown date, the patient started Diclofenac sodium at an unknown dosing regime. At an unknown time after starting Diclofenac sodium, the patient experienced hemolytic anemia drug-induced and jaundice. The patient was hospitalised and the authors considered the events were clinically significant (or requiring intervention). The patient was treated with glucocorticoids.

It was noted that Diclofenac-dependent IHA was not suspected on admission. The patient developed jaundice and was initially thought to have autoimmune hemolytic anemia (AIHA) due to warm-reactive autoantibodies. Diclofenac- and/or ex-vivo-antigen-dependent red cell antibodies were detected. The serologic test results at the laboratory suggested drug-induced IHA. Treatment with Diclofenac sodium was discontinued. At the time of reporting, the events were resolved. The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "misdiagnosis of diclofenac-induced IHA may be due to limited awareness of drug-dependent hemolysis and/or to oversimplification of serologic investigations."

CARINO, 24.05.2006 DE-CARINO-20060523-07

Sender diagnosis: not reported

Narrative

A 74-year-old male patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.9 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that a diclofenac-induced immune haemolytic anaemia is far more common than previously assumed.

CAUSALITY ASSESSMENT FOR diclofenac:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: unknown

History of Observed Event: unknown

Other Risk Factors: unknown

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

Reporter's comment:

Merck KGaA, 14.06.2006 DE-Merck KGaA-6023108

Sender diagnosis: not reported

Narrative

Case report from literature received on 06-Jun-2006.

*

Ahrens N, Genth R, Kieseewetter H, Salama A.

Misdiagnosis in patients with diclofenac-induced hemolysis: new cases and a concise review.

Am J Hematol 2006; 81: 128-131

*

A 74-year-old male patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced jaundice and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.9 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that "diclofenac-induced immune haemolytic anaemia is far more common than previously assumed".

*

See also cases # 6023111, # 6023113, # 6023115, # 6023116, # 6023117, # 6023118, # 6023120, # 6023121 belonging to the same publication.

Overall company causality assessment or relationship drug / adverse reaction: possible. Haemolytic anaemia is known as possible adverse reaction to diclofenac.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06010722

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth:	Age: 78 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hemolytic anemia drug-induced					recovered/resolved
2 Weakness					recovered/resolved
3 Jaundice					recovered/resolved
4 Pallor					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac sodium	Unknown	/	-	Drug use for unknown indication
diclofenac sodium			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	Ahrens N et al. Misdiagnosis in Patients With Diclofenac-Induced Hemolysis: New Cases and a Concise Review Am J Hema 2006; 81:128-131.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 21.03.2006 DE-GlaxoSmithKline-B0416273A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug-induced immune hemolytic anemia (IHA) in a 78-year-old female patient who received Diclofenac sodium for an unknown drug indication.

On an unknown date, the patient started Diclofenac sodium at an unknown dosing regime. At an unknown time after starting Diclofenac sodium, the patient experienced hemolytic anemia drug-induced, weakness, palor and jaundice. The patient was hospitalised.

It was noted that Diclofenac-dependent IHA was not suspected on admission. The patient developed jaundice and was initially thought to have autoimmune hemolytic anemia (AIHA) due to warm-reactive autoantibodies. Diclofenac- and/or ex-vivo-antigen-dependent red cell antibodies were detected. The serologic test results at the laboratory suggested drug-induced IHA. Treatment with Diclofenac sodium was discontinued. At the time of reporting, the events were resolved. The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "misdiagnosis of diclofenac-induced IHA may be due to limited awareness of drug-dependent hemolysis and/or to oversimplification of serologic investigations."

CARINO, 24.05.2006 DE-CARINO-20060523-08

Sender diagnosis: not reported

Narrative

A 78-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced weakness and palor and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.0 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that a diclofenac-induced immune haemolytic anaemia is far more common than previously assumed.

CAUSALITY ASSESSMENT FOR diclofenac:

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: unknown

History of Observed Event: unknown

Other Risk Factors: unknown

Sufficiency of Documentation: sufficient

OVERALL CAUSALITY ASSESSMENT: possible

Reporter's comment:

Merck KGaA, 14.06.2006 DE-Merck KGaA-6023115

Sender diagnosis: not reported

Narrative

Literature case was received on 06 Jun 2006

Ahrens N, Genth R, Kiesewetter H, Salama A. Misdiagnosis in patients with diclofenac-induced hemolysis: new cases and a concise review. Am J Hematol 2006; 81: 128-31

*

A 78-year-old female patient experienced diclofenac-induced immune haemolytic anaemia. The patient experienced weakness and palor and was erroneously diagnosed with autoimmune haemolytic anaemia on admission. Haemoglobin was 7.0 g/dl. Diclofenac was discontinued and the patient recovered completely. Drug-induced antibodies against diclofenac and/or its antibodies were identified. The reporters state that "diclofenac-induced immune haemolytic anaemia is far more common than previously assumed".

*

See also cases # 6023108, # 6023111, # 6023113, # 6023116, # 6023117, # 6023118, #6023120, # 6023121 belonging to the same publication.

Overall company causality assessment of relationship drug / adverse reaction: possible.

Haemolytic anaemia is known as possible adverse reaction to diclofenac.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06011198

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 72 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Hemolytic anemia drug-induced
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Urine discoloration					Unknown
2 Weakness					Unknown
3 Shock					Unknown
4 Multi-organ failure					Unknown
5 Hemolytic anemia drug-induced					Unknown
6 Abdominal pain					Unknown
7 Disseminated intravascular coagulation					Unknown
8 Respiratory insufficiency					Unknown
9 Renal failure					Unknown
10 Metabolic acidosis					Unknown
11 Pallor					Unknown
12 Jaundice					Unknown
13 Vomiting					Unknown
14 Diarrhea					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac sodium	Unknown	100 Mg milligram(s)/Day	-	Drug use for unknown indication
diclofenac sodium			-	indication

Drugs (concomitant)

2 piroxicam	/	-
piroxicam		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic back pain			Unknown	
Cardiac insufficiency			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Jurgensen JS et al. Fatal acute diclofenac-induced immune hemolytic anemia Ann Hematol 2001; 80: 440-442	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin direct		6.6	IU/l	
Bilirubin total		11.2	IU/l	
Blood pressure		105/70	mmHg	
Body temperature		37.5	oC	
Creatinine		1.7	g/dl	
Free hemoglobin		0.3	g/dl	
Hemoglobin		4.6	g/dl	

	DE-BFARM-06011198	
Lactate dehydrogenase	7080	IU/l
Pulse rate	112	
Respiratory rate	23	
Reticulocyte count	25	%
White blood cell count	12.2	/nl

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 22.03.2006 DE-GlaxoSmithKline-B0416534A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of Fatal acute diclofenac-induced immune hemolytic anemia in a 72-year-old female patient who received Diclofenac sodium.

The patient's medical history was significant for Her previous medical problems included heart insufficiency (NYHA III) and chronic low back pain that was treated with different nonsteroidal anti-inflammatory drugs including piroxicam.

On an unknown date, the patient started Diclofenac sodium (unknown) at 50 mg twice per day. At an unspecified time after starting diclofenac, it was noted that the patient complained of increasing weakness, dark brown urine, and abdominal pain of two days duration. Her daughter also reported that the patient had suffered from diarrhea and vomiting. The patient was admitted to hospital.

On physical examination the patient appeared pale and jaundiced and slightly obtunded. Her temperature was 37.5°C, pulse 112, respiration 23, and blood pressure 105/70 mmHg. The abdomen was noted to be tender and painful to deep palpation. Bowel sounds were decreased. Hematological laboratory tests on admission

showed a hemoglobin concentration of 4.6 g/dl, a reticulocyte count of 25‰, white blood cell count of 12.2/nl, and normal platelet count. Normoblasts were identified in the peripheral blood smear. No schistocytes could be detected. Free hemoglobin was elevated (0.3 g/dl) and hemoglobinuria was present. The lactate dehydrogenase level was 7080 IU/l, bilirubin was 11.2/6.6 (total/ direct) mg/dl, and creatinine 1.7 g/dl. Arterial blood gas analysis revealed a metabolic acidosis (pH 7.0, pO₂ 70.4 mmHg, pCO₂ 22.9 mmHg, HCO₃ 5.4 mmol/l). Because of a strongly positive direct antiglobulin test (DAT) and the clinical features, the diagnosis of AIHA was suspected. The patient was initially treated with 200 mg prednisolone i.v. per day and received four units of packed red cells. Because of rapidly developing multisystem organ failure with acute nonoliguric renal failure, respiratory insufficiency, and disseminated intravascular coagulation the patient was intubated and transferred to our tertiary care university center on the

second day. The diagnosis of drug-induced IHA due to diclofenac-dependent antibodies was established by immunohematological investigations. Corticosteroid therapy was continued and hemodialysis was initiated. Plasmapheresis plus plasma exchange with 3.5 l fresh frozen plasma was performed the same day. Laboratory indices of hemolysis stabilized, but a steady increase of aminotransferase levels, lactate concentration, and procalcitonin as well as a progressively tender abdominal wall prompted a CT scan that was obtained after administration of contrast medium. Signs of regional intestinal and hepatic hypoperfusion were seen. Because of a further deterioration of the clinical condition during the following two days, the abdomen was explored laparoscopically. This procedure revealed extensive necrosis of the colon and large parts of the small bowel. After careful consideration of the very limited prognosis, no further aggressive therapeutic intervention seemed justifiable and the patient died.

The authors considered the events were related to treatment with Diclofenac sodium.

The authors commented that "In conclusion, our observations illustrate the potential

severity of diclofenac-induced IHA in the occasional patient and emphasize the need for increased awareness of this complication of drug treatment."

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06012448

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes			Yes		

Patient

Initials:	Sex: Female	Birth: 19830518	Age: 67 Year	weight (kg): 58	BMI_int: 23.23	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dyspnoea	20060123		13 Day	11 Day	recovered/resolved
2 Renal failure acute	20060123		13 Day	11 Day	recovered/resolved
3 Transaminases increased	20060123		13 Day	11 Day	recovered/resolved
4 Tissue fluid increased	20060123		13 Day	11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	20060113- -	Rheumatism (joints only)

Drugs (concomitant)

2 bisoprolol bisoprolol	Oral	5 Mg milligram(s)/Day	20051220- -	Hypertension arterial
3 Hydrochlorothiazid hydrochlorothiazide	Oral	12.5 Mg milligram(s)/Day	20051220- -	Hypertension arterial

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Penicillin allergy			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06012681

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: 19510219 Age: 54 Year weight (kg): 87 BMI_int: 33.56 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased	20050727			10 Day	Unknown
2 Hemoglobin decreased	20050726			9 Day	recovering/resolving
3 Bilirubin increased	20050727			10 Day	Unknown
4 Autoimmune hemolytic anemia	20050726			9 Day	Unknown
5 Reticulocyte count decreased	20050728			11 Day	Unknown
6 Erythrocytes decreased	20050727			10 Day	Unknown
7 Thrombocyte count increased	20050727			10 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo diclofenac	Oral	2 Df dosage form/Day	20050725-20050727	Pain in extremity 4320 Minute
2 penicillin g benzylpenicillin	Intravenous (not otherwis	/Day	20050725-20050727	Thrombophlebitis 3 Day
3 voltaren resinat diclofenac resinate	Oral	2 Df dosage form/Day	20050718-20050722	Leg pain 5 Day

Drugs (concomitant)

4 BERBERIL berberine hydrochloride; tetrazoline hydrochloride	Oral	1 Df dosage form/Day	1985- -
5 ELOTRANS glucose; potassium chloride; sodium bicarbonate; sodium chloride	Oral	1 Df dosage form/Day	20050707-20050709 4320 Minute
6 ibuprofen ibuprofen	Oral	2 Df dosage form/Day	20050710-20050718 12960 Minute
7 IMODIUM "JANSSEN" loperamide hydrochloride	Oral	4 Df dosage form/Day	20050705-20050707 4320 Minute
8 innohep tinzaparin sodium	Subcutaneous	0.35 ml millilitre(s)/Day	20050725-20050725 1440 Minute
9 NEXIUM esomeprazole magnesium	Oral	20 Mg milligram(s)/Day	20050725-20050725 1440 Minute
10 NEXIUM esomeprazole magnesium	Oral	40 Mg milligram(s)/Day	20050726-20050726 1440 Minute
11 novalglin metamizole	Intravenous (not otherwis	500 ml millilitre(s)/Day	20050725-20050725 1440 Minute
12 TOGAL TABLET acetylsalicylic acid; lithium citrate; quinine citrate		/	20050719-20050720 2 Day
13 Vagifem estradiol		/	1998-20050720 -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hodgkin's disease recurrent	1996		No	
Splenectomy	1981		No	
Hodgkin's disease	1981		Unknown	M. Hodgkin 1981, recidive 1996, complete remission at present
Seasonal allergic rhinitis			Unknown	
Chemotherapy	199601	199606	No	
Erysipelas			Unknown	Onset of erysipelas
Thrombophlebitis			Unknown	Acute thrombophlebitis with onset of erysipelas
Hypothyroidism			Unknown	Latent hypothyroidism

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 09.05.2007 DE-002147023-PHNU2006DE01594

Sender diagnosis: not reported

Narrative

Initial report received on 5 Apr 2006. Report received from German Health Authority, originating source hospital unit FAKOS. This patient was treated with Voltaren Resinat (diclofenac, colestyramin) from 18 Jul 2005 until 22 Jul 2005, for leg pain. From 25 Jul 2006 until 27 Jul 2005, patient took Diclo 50 (manufacturer unstated), and penicillin G mega was also given i.v. over the same period, for acute thrombophlebitis with onset of erysipelas. Comedication with many other drugs was documented. On 26 Jul 2005, patient developed immune hemolytic anemia. Laboratory were added as follows: hemoglobin decreased (lowest level 6.6g/dl), erythrocytes 0.88T/l, reticulocytes 0.2%, leukocytes 9.5T/l, thrombocytes 609G/l, LDH increased (715IU/l), bilirubin increased (2.7mg/dl). Direct Coombs test on 27 Jul 2005 was positive. IgG positive, IgA and IgM negative. No testing of drug-induced antibodies was done. Voltaren Resinat, Diclo 50, penicillin G and Nexium were stopped, and hemoglobin increased thereafter. Outcome was indicated as "unknown". Causality was rated as possible, and Voltaren resinat, Diclo 50 and penicillin G mega were ticked as suspected.

Following internal review on 12 Apr 2006 of data received on 05 Apr 2006, Diclo (diclofenac sodium) was coded as Novartis Drug.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06014511

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19611104	Age: 41 Year	weight (kg): 77	BMI_int: 23.77	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20031230			83 Day	Unknown
2 Tachycardia	200311			1 Month	Unknown
3 Pancytopenia	200312			2 Month	Unknown
4 Aplastic anaemia	20031127			50 Day	Unknown
5 Leukopenia	20040102			65 Day	Unknown
6 Thrombocytopenia	20040102			65 Day	Unknown
7 Erythrocytes decreased	20040102			65 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 klacid pro clarithromycin	Oral	/	20031008-20031102	Arthritis 26 Day
2 roxithromycin roxithromycin	Oral	/	20031020-20031028	Fever 9 Day
3 voltaren diclofenac sodium	Oral	/	20031030-20031120	Arthritis 22 Day

Drugs (concomitant)

4 Abtei meersalz Nasenspray		/	1992-20040111	-
5 aspirin acetylsalicylic acid	Oral	3 Df dosage form/Day	20031010-20031020	11 Day
6 cialis tadalafil	Oral	/	20030715-20030915	Erectile dysfunction 63 Day
7 frubiase sport calcium carbonate	Oral	1 Df dosage form/Day	200305-20040111	-
8 metoprolol metoprolol	Oral	1 Df dosage form/Day	20031101-20031231	61 Day
9 paracetamol paracetamol	Oral	3 Df dosage form/Day	20031020-20031030	11 Day
10 prednisolone prednisolone	Oral	/	20031120-20040111	53 Day

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hemoglobin	20040102	10	G/DL	
Neutrophils	20040102	60	%	
Body height		180	CM	

Narratives (nur bei elektronischem Eingang)

ABBOTT, 21.04.2006 DE-ABBOTT-06P-062-0330394-00

Sender diagnosis: not reported

Narrative

Report from Germany of aplastic anemia coincident with CLARITHROMYCIN (KLACID PRO) therapy. On 08 Oct 2003, patient began KLACID PRO for arthritis, fever, and pain. On 20 Oct 2003, the patient began ROXITHROMYCIN for fever. On 28 Oct 2003, ROXITHROMYCIN was discontinued. On 30 Oct 2003, the patient began VOLTAREN TABLETS for arthritis. On 02 Nov 2003, KLACID PRO was discontinued. On 20 Nov 2003, VOLTAREN TABLETS was discontinued. On 27 Nov 2003, the patient experienced aplastic anemia. The outcome of aplastic anemia was unknown. KLACID PRO, VOLTAREN TABLETS, and ROXITHROMYCIN were possibly related to the event of aplastic anemia. The event of aplastic anemia was considered medically significant. VOLTAREN TABLETS and ROXITHROMYCIN were also considered suspect.

LABORATORY/COMMENTS:

30 Dec 2003 Bone marrow aspiration biopsy: Compatible with aplastic anemia; differential diagnosis of myelodysplastic syndrome
 30 Dec 2003 Bone marrow biopsy: Diagnosed with possible aplastic anemia; differential diagnosis of myelodysplastic syndrome
 02 Jan 2004 Lab tests: Signs of mild aplastic anemia (patient had no transfusion before blood sample was taken)
 Unknown date Chromosomal analysis: Normal, no anomalies in metaphases

ABBOTT, 15.05.2006 DE-ABBOTT-06P-062-0330394-00

Sender diagnosis: not reported

Narrative

Report from Germany of aplastic anemia, pancytopenia, increased transaminases, and tachycardia coincident with CLARITHROMYCIN (KLACID PRO) therapy. On 08 Oct 2003, patient began KLACID PRO for arthritis, fever, and pain. On 20 Oct 2003, the patient began ROXITHROMYCIN for fever. On 28 Oct 2003, ROXITHROMYCIN was discontinued. On 30 Oct 2003, the patient began VOLTAREN TABLETS for arthritis. On 02 Nov 2003, KLACID PRO was discontinued. On 20 Nov 2003, VOLTAREN TABLETS was discontinued. On 27 Nov 2003, the patient experienced aplastic anemia. The outcome of aplastic anemia was unknown. KLACID PRO, VOLTAREN TABLETS, and ROXITHROMYCIN were possibly related to the event of aplastic anemia. The event of aplastic anemia was considered medically significant. VOLTAREN TABLETS and ROXITHROMYCIN were also considered suspect.

Follow-up information received on 04 May 2006: Event information was added or revised. In the beginning of Nov 2003, the patient was hospitalized in a cardiological clinic due to a fever. No focus was found. Due to tachycardia, therapy with METOPROLOL was started. Also in Nov 2003, the patient was hospitalized again, this time in rheumatological clinic due to inflammatory-rheumatologic clinical picture with fever, signs of inflammation, and arthralgia. Therapy with PREDNISOLONE was started. Increasing thrombopenia was observed and a bone marrow biopsy showed hypocellular bone marrow and decreased megakaryopoiesis. On 03 Dec 2003, the patient was again hospitalized, this time due to increasing tricytopenia with increased transaminases. Physical examination was normal. Hospitalization was due to increasing pancytopenia of unclear etiology. On 02 Jan 2004, the patient was discharged with a diagnosis of aplastic anemia and was treated with oral DECORTIN H 12.5 mg per day, oral NEXIUM 20 mg per day, and oral BELOK ZOK MITE 47.5 mg per day. Further information was not available.

LABORATORY/COMMENTS:

30 Dec 2003 Bone marrow aspiration biopsy: Compatible with aplastic anemia; differential diagnosis of myelodysplastic syndrome
 30 Dec 2003 Bone marrow biopsy: Diagnosed with possible aplastic anemia; differential diagnosis of myelodysplastic syndrome
 02 Jan 2004 Lab tests: Signs of mild aplastic anemia (patient had no transfusion before blood sample was taken)
 Unknown date Chromosomal analysis: Normal, no anomalies in metaphases

Follow-up information received on 04 May 2006:

Unreported date Differential blood count: 35% segmented cells, 1 stab cell, 10% lymphocytic-monocytic cells, 4% monocytes, neutrophils with toxic granulations, partly Pseudo-Pelger form

30 Dec 2003 Cytological examination of the bone marrow: Hypocellular; decreased megakaryopoiesis- in one smear, just two morphological normal megakaryocytes seen; erythropoiesis: morphological normal, maturing erythropoiesis; granulopoiesis: with deviation to the left; counting of 127 cells; promyelocytes 75%; metamyelocytes 3.9%; stab cells 8.7%; segmented cells 3.1%; proerythroblasts 2.3%; erythroblasts 5.5%; normoblasts 13.3%; lymphocytes 24.4%; plasma cells 1.5%; monocytes 7.9%; rating of hypocellular bone marrow with megakaryocytopenia, skeocytosis in granulopoiesis, no increasing of blasts and no signs of morphological dysplasia; compatible with aplastic anemia.

30 Dec 2003 Histological examination of the bone marrow: Important, relative macroblastic, clearly ineffective hyperplasia of the erythropoiesis with siderosis of the reticular cells, granulocytopenic skeocytosis and megakaryocytic hypoplasia; two possible conclusions: an aplastic anemia in

regeneration or a myelodysplasia; at the time a pancytopenia is reported: leukocytes 1 400; thrombocytes 17 000; hemoglobin 11.4, and MCV 87
NOVARTIS, 09.05.2007 DE-002147023-PHNU2006DE01644

Sender diagnosis: not reported

Narrative

Initial physician report received on 11 April 2006 (via German health authority). The patient was treated with Voltaren tablet (diclofenac) from 30 Oct to 20 Nov 2003 due to arthritis. Concomitant medication was Klacid (clarithromycin) from 08 Oct 2003 to 02 Nov 2003 and roxithromycin from 20 Oct 2003 to 28 Oct 2003. The patient developed aplastic anemia on 27 Nov 2003. The lab values showed erythrocyte decrease on 02 Jan 2004 $3.13 \times 10^{12}/L$, leukopenia $1.2 \times 10^9/L$ and thrombocytopenia of $9 \times 10^9/L$ on 02 Jan 2004 (normal values not identifiable). Outcome was unknown.

Relationship to Voltaren, Klacid and roxithromycin was assessed as possible.

Serious spontaneous report, aplastic anaemia (medically significant), already listed in the Basic Prescribing Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06016435

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19550310	Age: 51 Year	weight (kg): 60	BMI_int: 21.01	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukocytosis	20060402				not recovered/not resolved
2 Hypokalemia	20060402				not recovered/not resolved
3 Oral pain	20060402				not recovered/not resolved
4 Oxygen saturation decreased	20060402				not recovered/not resolved
5 Anemia	20060402				not recovered/not resolved
6 Anxiety					not recovered/not resolved
7 Arterial embolism NOS	20060402				not recovered/not resolved
8 Atelectasis	20060402				not recovered/not resolved
9 Blood bicarbonate decreased	20060402				not recovered/not resolved
10 Bradycardia					not recovered/not resolved
11 Chills	20060401				not recovered/not resolved
12 Conduction disorder	20060402				not recovered/not resolved
13 CRP increased					not recovered/not resolved
14 PO2 decreased	20060402				not recovered/not resolved
15 PTT prolonged	20060402				not recovered/not resolved
16 Pulmonary congestion					not recovered/not resolved
17 Respiratory insufficiency	20060402				not recovered/not resolved
18 Retrosternal pain	20060403				not recovered/not resolved
19 Sinus tachycardia	20060402				not recovered/not resolved
20 Speech disorder	20060402				not recovered/not resolved
21 Stenocardia					not recovered/not resolved
22 Vasodilatation					not recovered/not resolved
23 Vomiting	20060401				not recovered/not resolved
24 Weakness	20060402				not recovered/not resolved
25 Negative T wave	20060402				not recovered/not resolved
26 Base excess increased	20060402				not recovered/not resolved
27 Drug intolerance	20060402				not recovered/not resolved
28 Quick's value decreased	20060402				not recovered/not resolved
29 Stomatitis	20060401		4 Day		not recovered/not resolved
30 Diarrhoea	20060401		4 Day		not recovered/not resolved
31 Urinary infection	20060401		4 Day		not recovered/not resolved
32 Thrombocytopenia	20060401		4 Day		not recovered/not resolved
33 Pleural effusion	20060402		4 Day		not recovered/not resolved
34 Pneumonitis	20060401		4 Day		not recovered/not resolved
35 Pulmonary embolism	20060401		4 Day		not recovered/not resolved
36 Postoperative complication	20060401		4 Day		not recovered/not resolved
37 Hematoma					not recovered/not resolved
38 Hyperventilation	20060402				not recovered/not resolved
39 Nausea	20060401				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	Celebrex 200 mg Hartkapseln celecoxib	Oral	200 Mg milligram(s)/Day	20060331-20060331 Pain management 1440 Minute
2	Dulcolax bisacodyl	Rectal	2 Df dosage form/Day	20060401-20060401 Obstipation 1440 Minute
3	LAXOBERAL sodium picosulfate		/	-
4	mono-embolex certoparin sodium		/	-
5	Voltaren 50 Zaepfchen diclofenac sodium	Rectal	100 Mg milligram(s)/Day	20060401-20060401 Pain management 1440 Minute

Drugs (concomitant)

6	anaesthetika		/	-
7	HEPARIN heparin	Intravenous (not otherwis	/	-
8	Hormone		/	-
9	REFLUDAN lepirudin		/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Suprapubic catheter insertion			Unknown	
Vaginal hysterectomy			Unknown	Vaginal hysterectomy with plasty and installation of a suprapubic catheter due to hypermenorrhea
Hypermenorrhea			Unknown	
Hallux malleus			Unknown	Hallux malleus operation of both feet in 2000
Pulmonary embolism			Yes	
Platelet count decreased			Yes	
Uterine myoma			Yes	Uterus myomatosus

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 12.07.2006 DE-002147023-GXKR2006DE02293

Sender diagnosis: not reported

Narrative

Initial report received on 21 Apr 2006 from German Health Authority, originating source anesthesiologist, BfArM number 06016435. The patient was treated with Celebrex (celecoxib) on 31 Mar 2006, and on 1 Apr 2006 with Dulcolax (bisacodyl). On 1 Apr 2006, also Voltaren suppositories were started, as a postoperative analgetic. Under the section for "other comments" (not in the adverse event section), a "parallel suspicion of HIT II (clinically acute pulmonary arterial embolism and thrombocyte drop unter Mono Embolex)" was mentioned. No HIT-test findings were known so far, and no further data were given. Furthermore, on 1 Apr 2006, patient showed stomatitis and diarrhea, lasting for 4 days. Antibacterial and antimycotic local therapy was initiated. Outcome for these events was indicated as "not yet recovered".

Follow-up report received from the anesthesiologist via German HA (BfArM no. 06016435) on 18 May 2006: An ICU transfer report was provided where the following diagnoses were listed: postoperative acute pulmonary artery embolism with associated pleuropneumonia and pleural effusion (right), condition after vaginal hysterectomy due to hypermenorrhea, uterine myoma with postoperative urinary infection and secondary thrombocytopenia, diarrhea, stomatitis during drug intolerance (Dulcolax suppositories (bisacodyl), Voltaren suppositories (diclofenac), Celebrex (celecoxib), Laxoberal (sodium picosulfate)), hypokalemia. On 30 Mar 2006 the patient with a history of hallux malleus operation of both feet in 2000 was hospitalized electively for uterus extirpation due to hypermenorrhea with uterine myoma. At admission the patient did not report about any ongoing drug therapy but stated later that she received a hormonal once every three months injection (not specified). Additionally nicotine abuse had been existent until about three weeks before hospitalization. At anamnesis the patient showed no pain and no cardiopulmonary disorder. ECG was normal except a mild bradycardiac sinus rhythm. Preoperative lab data were normal except anemia. On 31 Mar 2006 vaginal hysterectomy with plasty and installation of a suprapubic catheter was performed with normal anesthesia (not specified). The intra- and postoperative course was normal except nausea and vomiting on the first postoperative day. In the evening of 01 Apr 2006 the patient complained about chills and fever of 39.8°C with

general weakness, speech disorder and showed mild dyspnea on the next morning. She was transferred to operative ICU with sinus tachycardia (HF 115bpm), mild right heart conduction delay and negative T wave (lead V4-V6) in the ECG. Blood gas analysis showed a partial respiratory insufficiency with compensating hyperventilation (pH 7.47, PCO₂ 31.0mmHg, base excess 0.1, blood bicarbonate 22.4mmol/l, PO₂ 66.1mmHg, oxygen saturation 93.9%). Further lab analysis showed the following abnormal findings: Hb 9.3g/dl, leukocytosis of 12800/Åµl, decreased thrombocyte count of 120000/Åµl (preoperative value: 170000/Åµl), hypokalemia of 3.2mmol/l, QuickâĶs value 43%, PTT 52sec, D dimer 3.6Åµg/ml. Additionally the patient complained about pain in oral cavity, lips and tongue associated with speech disorders. In parallel watery stools occurred which were associated with hypokalemia. After abdominal sonography a small pleural effusion was suspected. Abdominal spiral CT analysis confirmed mild bilateral pleural effusions and showed a mass of about 8cm near left iliac bone with mild increased abdominal fluid accumulation which was regarded as hematoma. Thoracic X-ray analysis was normal except a small discoid atelectasis (left). As the complaints were regarded as related to the diarrhea balanced infusion therapy (not specified) was performed. Additionally the patient received prophylaxis with a low molecular heparin (not specified, it remained unclear if Mono-Embolex was given at this time) and a proton pump inhibitor (not specified). Bacterial and viral stool analysis was negative. On 03 Apr 2006 diarrhea abated whereas stomatitis was still ongoing and was treated with chlorhexidine (not specified) and Candiothermal (nystatin) mouth rinses and benzocaine lozenges (not specified). At examination the patient complained about retrosternal pain during deep breathing. Blood gas analysis showed a distinct partial respiratory insufficiency (pH 7.46, PCO₂ 38.2mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 48.0mmHg, oxygen saturation 89.8%): Right heart conduction delay was ongoing whereas repolarization disturbances were no longer detectable. Lung scintigraphy indicated two small triangle-like perfusion defects of the right lung (ventrally and dorsally) and acute pulmonary embolism was suspected. Duplex sonography of leg veins showed no indication for deep vein thrombosis. Intravenous heparin (not specified) administration was performed. Due to a decrease of thrombocyte count and the fact that a heparin-induced thrombocytopenia could not be excluded the patient was switched to Refludan (lepirudin) after change of central venous catheter and infusion system on 04 Apr 2006. On 06 Apr 2006 subcutaneous injection of heparin (not specified, 7500IU three times daily) was re-initiated based on the results of heparin-induced thrombocytopenia- and ELISA-testing (therefore suspicion of HIT was deleted as event). In the further course the patient showed a central and basal pulmonary congestion with pleural effusions and possible infiltrates which was regarded as matching to a post embolic pneumonia. On 05 Apr 2006 therapy with ampicillin and sulbactam (not specified, first iv and since 07 Apr 2006 oral administration) was started as the patient showed subfebrile temperatures, a mild increase of leukocytes and a positive urotube test for coagulase-negative staphylococci. During this therapy body temperature and CRP level decreased, but leukocytosis aggravated to 16600/Åµl. Furthermore the patient received elastic stockings and was mobilized carefully. Till transfer from ICU the patient showed a clear dyspnea on exertion whereas radiological findings and oxygenation were improving. On 07 Apr 2006 the patient was transferred. On this day partial respiratory insufficiency was still ongoing (pH 7.45, PCO₂ 34.1mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 54.9mmHg, oxygen saturation 91.5%) and in the morning the patient had complained about stenocardia-like symptoms with dyspnea, retrosternal pain and anxiety. Although troponin T was normal and D dimer was decreasing ECG analysis showed a negative T wave (V2). Therefore trans-thoracic echocardiography was performed and showed dilatation of vena cava and small bilateral pleural effusions. At this time stomatitis was clearly resolving and abdominal examination was normal. As the last bowel movement had already been on 03 Apr 2006 metoclopramid (not specified) and Movicol (macrogol) were prescribed as a relationship between Dulcolax suppositories and Laxoberal and the stomatitis and diarrhea was suspected. It remained unclear when the patient received Voltaren suppositories and Mono-Embolex.

Significant follow up due to internal review on 11 Jul 2006: Novartis Comment for certoparin was changed.

NOVARTIS, 08.05.2007 DE-002147023-GXKR2006DE02293

Sender diagnosis: not reported

Narrative

Initial report received on 21 Apr 2006 from German Health Authority, originating source anesthesiologist, BfArM number 06016435. The patient was treated with Celebrex (celecoxib) on 31 Mar 2006, and on 1 Apr 2006 with Dulcolax (bisacodyl). On 1 Apr 2006, also Voltaren suppositories were started, as a postoperative analgetic. Under the section for "other comments" (not in the adverse event section), a "parallel suspicion of HIT II (clinically acute pulmonary arterial embolism and thrombocyte drop unter Mono Embolex)" was mentioned. No HIT-test findings were known so far, and no further data were given. Furthermore, on 1 Apr 2006, patient showed stomatitis and diarrhea, lasting for 4 days. Antibacterial and antimycotic local therapy was initiated. Outcome for these events was indicated as "not yet recovered".

Follow-up report received from the anesthesiologist via German HA (BfArM no. 06016435) on 18 May 2006: An ICU transfer report was provided where the following diagnoses were listed: postoperative acute pulmonary artery embolism with associated pleuropneumonia and pleural effusion (right), condition after vaginal hysterectomy due to hypermenorrhea, uterine myoma with postoperative urinary infection and secondary thrombocytopenia, diarrhea, stomatitis during drug intolerance (Dulcolax suppositories (bisacodyl), Voltaren suppositories (diclofenac), Celebrex (celecoxib), Laxoberal (sodium picosulfate)), hypokalemia. On 30 Mar 2006 the patient with a history of hallux malleus operation of both feet in 2000 was hospitalized electively for uterus extirpation due to hypermenorrhea with uterine myoma. At admission the patient did not report about any ongoing drug therapy but stated later that she received a hormonal once every three months injection (not specified). Additionally nicotine abuse had been existent until about three weeks before hospitalization. At anamnesis the patient showed no pain and no cardiopulmonary disorder. ECG was normal except a mild bradycardiac sinus rhythm. Preoperative lab data were normal except anemia. On 31 Mar 2006 vaginal hysterectomy with plasty and installation of a suprapubic catheter was performed with normal anesthesia (not specified). The intra- and postoperative course was normal except nausea and vomiting on the first postoperative day. In the evening of 01 Apr 2006 the patient complained about chills and fever of 39.8°C with general weakness, speech disorder and showed mild dyspnea on the next morning. She was transferred to operative ICU with sinus tachycardia (HF 115bpm), mild right heart conduction delay and negative T wave (lead V4-V6) in the ECG. Blood gas analysis showed a partial respiratory insufficiency with compensating hyperventilation (pH 7.47, PCO₂ 31.0mmHg, base excess 0.1, blood bicarbonate 22.4mmol/l, PO₂ 66.1mmHg, oxygen saturation 93.9%). Further lab analysis showed the following abnormal findings: Hb 9.3g/dl, leukocytosis of 12800/Åµl, decreased thrombocyte count of 120000/Åµl (preoperative value: 170000/Åµl), hypokalemia of 3.2mmol/l, QuickâĶs value 43%, PTT 52sec, D dimer 3.6Åµg/ml. Additionally the patient complained about pain in oral cavity, lips and tongue associated with speech disorders. In parallel watery stools occurred which were associated with hypokalemia. After abdominal sonography a small pleural effusion was suspected. Abdominal spiral CT analysis confirmed mild bilateral pleural effusions and showed a mass of about 8cm near left iliac bone with mild increased abdominal fluid accumulation which was regarded

as hematoma. Thoracic X-ray analysis was normal except a small discoid atelectasis (left). As the complaints were regarded as related to the diarrhea balanced infusion therapy (not specified) was performed. Additionally the patient received prophylaxis with a low molecular heparin (not specified, it remained unclear if Mono-Embolex was given at this time) and a proton pump inhibitor (not specified). Bacterial and viral stool analysis was negative. On 03 Apr 2006 diarrhea abated whereas stomatitis was still ongoing and was treated with chlorhexidine (not specified) and Candiohermal (nystatin) mouth rinses and benzocaine lozenges (not specified). At examination the patient complained about retrosternal pain during deep breathing. Blood gas analysis showed a distinct partial respiratory insufficiency (pH 7.46, PCO₂ 38.2mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 48.0mmHg, oxygen saturation 89.8%): Right heart conduction delay was ongoing whereas repolarization disturbances were no longer detectable. Lung scintigraphy indicated two small triangle-like perfusion defects of the right lung (ventrally and dorsally) and acute pulmonary embolism was suspected. Duplex sonography of leg veins showed no indication for deep vein thrombosis. Intravenous heparin (not specified) administration was performed. Due to a decrease of thrombocyte count and the fact that a heparin-induced thrombocytopenia could not be excluded the patient was switched to Refludan (lepirudin) after change of central venous catheter and infusion system on 04 Apr 2006. On 06 Apr 2006 subcutaneous injection of heparin (not specified, 7500IU three times daily) was re-initiated based on the results of heparin-induced thrombocytopenia- and ELISA-testing (therefore suspicion of HIT was deleted as event). In the further course the patient showed a central and basal pulmonary congestion with pleural effusions and possible infiltrates which was regarded as matching to a post embolic pneumonia. On 05 Apr 2006 therapy with ampicillin and sulbactam (not specified, first iv and since 07 Apr 2006 oral administration) was started as the patient showed subfebrile temperatures, a mild increase of leukocytes and a positive urotube test for coagulase-negative staphylococci. During this therapy body temperature and CRP level decreased, but leukocytosis aggravated to 16600/Åµl. Furthermore the patient received elastic stockings and was mobilized carefully. Till transfer from ICU the patient showed a clear dyspnea on exertion whereas radiological findings and oxygenation were improving. On 07 Apr 2006 the patient was transferred. On this day partial respiratory insufficiency was still ongoing (pH 7.45, PCO₂ 34.1mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 54.9mmHg, oxygen saturation 91.5%) and in the morning the patient had complained about stenocardia-like symptoms with dyspnea, retrosternal pain and anxiety. Although troponin T was normal and D dimer was decreasing ECG analysis showed a negative T wave (V2). Therefore trans-thoracic echocardiography was performed and showed dilatation of vena cava and small bilateral pleural effusions. At this time stomatitis was clearly resolving and abdominal examination was normal. As the last bowel movement had already been on 03 Apr 2006 metoclopramid (not specified) and Movicol (macrogol) were prescribed as a relationship between Dulcolax suppositories and Laxoberal and the stomatitis and diarrhea was suspected. It remained unclear when the patient received Voltaren suppositories and Mono-Embolex.

Significant follow up due to internal review on 11 Jul 2006: Novartis Comment for certoparin was changed.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06024244

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19680806	Age: 37 Year	weight (kg): 104	BMI_int: 27.92	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased	20060201		1 Week	51 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aptivus tipranavir	Oral	250 Mg milligram(s)/ Capsule, soft	20051212-20060202	HIV infection
2 DICLOFENAC SL RETARDKAPSEL diclofenac	Oral	75 Mg milligram(s)/ Prolonged-release capsule	20060118-20060202	Low back pain
			16 Day	

Drugs (concomitant)

3 fuzeon enfuvirtide	Subcutaneous	90 Mg milligram(s)/ Suspension for injection	20051212- -	HIV infection
4 TRIZIVIR abacavir sulfate; lamivudine; zidovudine	Oral	300 Mg milligram(s)/ Tablet	20051212- -	HIV infection
5 VIREAD tenofovir disoproxil fumarate	Oral	245 Mg milligram(s)/ Tablet	20051212- -	HIV infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperlipidaemia			k.A.	
Hypertriglyceridaemia			k.A.	
Hypercholesterolaemia			k.A.	
Metabolic disorder			k.A.	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 26.05.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-02585DE

Sender diagnosis: not reported

Narrative

This is a spontaneous health-care professional confirmed report from Germany.

Initial information has been received from a physician concerning a 37 year-old, male, white patient.

The patient has a history of hyperlipidemia (hypertriglyceridemia, hypercholesterolemia).

The patient was placed on Aptivus (tipranavir), 250 mg capsules 2 x bid + Norvir (ritonavir), 100 mg capsules 2 x bid for the treatment of HIV infection on 12Dec2005. He received diclofenac 75 mg SL long acting capsules for the treatment of low back pain (ischialgia) since 18Jan2006.

As concomitant drugs the patient received Trizivir (abacavir, lamivudine, zidovudine) 300mg coated tablets bid, Viread (tenofovir disoproxil fumarate), 245mg coated tablets once daily, Fuzeon (enfuvirtide) 90mg/ml suspension for injection bid sc. since 12Dec2005.

After 14 days of Aptivus+Norvir, diclofenac treatment, the patient developed a grade 4 liver function increase. Laboratory analyses showed ALT 1573 U/l).

The treatment with diclofenac was discontinued on 2Feb2006.

Information on the treatment of the events was not provided.

After 1 week the event disappeared completely, the patient fully recovered.

It is the opinion of the physician that the adverse event is related to Aptivus/Norvir, Diclofenac therapy.

Due to the medical judgement the event was judged to be serious.

Further information is not expected. Case closed.

Sender diagnosis: not reported

Narrative

This is a spontaneous health-care professional confirmed report from Germany.

Initial information has been received from a physician concerning a 37 year-old, male, white patient.

The patient has a history of hyperlipidemia (hypertriglyceridemia, hypercholesterinemia).

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Information on the treatment of the events was not provided.

After 1 week the event disappeared completely, the patient fully recovered.

It is the opinion of the physician that the adverse event is related to Aptivus/Norvir, Diclofenac therapy.

Due to the medical judgement the event was judged to be serious.

Further information is not expected. Case closed.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06070916

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: 19611108 Age: 43 Year weight (kg): 74.4 BMI_int: 24.29 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzymes	20050509				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 imurek 50 azathioprine	Oral	150 Mg milligram(s)/Day	20050509-20050510 2 Day	Crohn's
2 voltaren diclofenac sodium	Oral	50 Mg milligram(s)/	-20050510 -	Lumboischialgia

Drugs (concomitant)

3 budenofalk budesonide	Oral	9 Mg milligram(s)/Day	20050410-20050509 30 Day	Crohn's
4 salofalk klysmen	Rectal	4 Mg milligram(s)/Day	20000601-20050511 4 Year	Crohn's
5 salofalk 250	Oral	500 Mg milligram(s)/Day	20050401- -	Crohn's

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Intervertebral disc disorder	200505		Yes	Lendenwirbelsäule
Crohn's disease	19991201		Yes	
Spinal disorder	20050101		Yes	Schaden an der Lendenwirbelsäule
Acute pancreatitis	200403		No	biliäre;ERCP und Stent
Alcohol use			Yes	über Feiertage 3 Flaschen Bier und 3 Gläser Sekt /d

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin direct		erhöht/increased	µA	
Cholesterol		erhöht/increased	µA	
LDH		erhöht/increased	µA	
Gamma GT		erhöht/increased	µA	
Alanine aminotransferase		erhöht/increased	µA	
Bilirubin total		erhöht/increased	µA	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06081681

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: 19630728 Age: 42 Year weight (kg): 73 BMI_int: 25.26 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20051001	20051207	68 Day		recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxypen amoxicillin trihydrate	Oral	3 G gram(s)/Day	20051101-20051107 7 Day	Respiratory infection
2 bronchipret primula rhizome; thyme extract	Oral	3* Gtt drop(s)/Day	-20051107 -	Throat pain
3 cerazette desogestrel	Oral	0.075 Mg milligram(s)/Day	20031001-20051107 2 Year	Hypermenorrhea
4 doxycyclin doxycycline	Oral	200 Mg milligram(s)/Day	20051028-20051107 11 Day	Respiratory tract infection
5 laif hypericum extract	Oral	900 Mg milligram(s)/	20030401- -	Depression
6 paracetamol paracetamol	Oral	1500 Mg milligram(s)/Day	20051004-20051107 35 Day	Throat pain
7 syneudon amitriptyline hydrochloride	Oral	50 Mg milligram(s)/Day	20050110-20051107 9 Month	Depression
8 voltaren resinat diclofenac resinate	Oral	150 Mg milligram(s)/Day	20030923-20051107 2 Year	

Drugs (concomitant)

9 ferro sanol ferroglycine sulfate complex	Oral	100 Mg milligram(s)/Day	20021203-20051107 2 Year	Iron deficiency
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Smoker			No	
Alcohol use			Yes	gelegentlich
Contraception NOS			Yes	
Throat tightness	20050101		Yes	Kloss im Hals
Apoplexy	1999		No	
Allergy			Yes	Nickel
Adenoma adrenal			Yes	V.a.
Uterine myoma	20051101		Yes	
Haemangioma	20051001		Yes	Leber
Hernia hiatal	20051001		Yes	
Genital infection female	20050101		Yes	rezidivierende Pilzinfektion evtl. bei wiederholter Antibiotikatherapie
Liver transplant	20051207		Yes	

Primary source

Qualification: Physician observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin direct		erhöht/increased	NA	
Haematocrit		erniedrigt/decreased	NA	
Haemoglobin		erniedrigt/decreased	NA	
Red blood cell count		erniedrigt/decreased	NA	
Albumin		erniedrigt/decreased	NA	
Quick's test		Abfall/decreased	NA	
Lactate dehydrogenase		erhöht/increased	NA	
Alkaline phosphatase		erhöht/increased	NA	
Gamma GT		erhöht/increased	NA	
Glutamate dehydrogenase		erhöht/increased	NA	
Alanine aminotransferase		erhöht/increased	NA	
Aspartate aminotransferase		erhöht/increased	NA	
Bilirubin total		erhöht/increased	NA	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06030505

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		No

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20060324	deathcause: Gastric perforation
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gastric perforation					fatal
2 Gastric operation					fatal
3 Gastrointestinal hemorrhage					Unknown
4 Nausea					Unknown
5 Vomiting					Unknown
6 Diarrhea					Unknown
7 Flatulence					Unknown
8 Abdominal pain					Unknown
9 Gastric ulcer					Unknown
10 Vomiting blood					Unknown
11 Oesophageal disorder					Unknown
12 Appetite absent					Unknown
13 Hepatitis	20060305				Unknown
14 Liver disorder	20060305				Unknown
15 Heart insufficiency	20060305				Unknown
16 Sleeplessness	20060305				Unknown
17 Irritability	20060305				Unknown
18 Tiredness	20060305				Unknown
19 Dizziness	20060305				Unknown
20 Trembling	20060305				Unknown
21 Depression	20060305				Unknown
22 Hearing impaired	20060305				Unknown
23 Water retention	20060305				Unknown
24 Asthma	20060305				Unknown
25 Hair loss	20060305				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Voltaren Dolo 12,5 mg diclofenac potassium	Oral	0.5 Df dosage form/Day	-	Back pain
			-	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 11.07.2006 DE-002147023-PHNU2006DE02475

Sender diagnosis: not reported

Narrative

Initial report received on 3 Jul 2006 at Novartis Consumer Health Munich. Report from patient's daughter (letter). This patient was treated with Voltaren Dolo (diclofenac) half a tablet per day, for back pain, according to patient's daughter. The patient was hospitalized on 5 Mar 2006. Reportedly, the relative noticed that patient suffered from adverse events after reading the package leaflet. The daughter listed the following events which she considered to be due to Voltaren Dolo: "gastrointestinal hemorrhages, nausea, vomiting, diarrhea, flatulence, abdominal pain, no appetite,

gastric ulcers, stomach penetration, bloody vomiting, hepatitis, cardiac muscle weakness, damage of oesophagus, irritability, sleeplessness, tiredness, dizziness, trembling, depressions, hearing impaired, hair loss, liver inflammation, water accumulation in the whole body, asthma attack". Due to 2 stomach penetrations, patient underwent a "very severe operation" at her stomach, and she died on 24 Mar 2006 as a consequence, according to reporter.

Serious spontaneous report (hospitalization) "hepatitis", "cardiac failure", "insomnia", "depression", "hearing impaired", "fluid retention", "asthma", and "alopecia" already listed in the Basic Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 28.07.2006 DE-002147023-PHNU2006DE02475

Sender diagnosis: not reported

Narrative

Initial report received on 3 Jul 2006 at Novartis Consumer Health Munich. Report from patient's daughter (letter). This patient was treated with Voltaren Dolo (diclofenac) half a tablet per day, for back pain, according to patient's daughter. The patient was hospitalized on 5 Mar 2006. Reportedly, the relative noticed that patient suffered from adverse events after reading the package leaflet. The daughter listed the following events which she considered to be due to Voltaren Dolo: "gastrointestinal hemorrhages, nausea, vomiting, diarrhea, flatulence, abdominal pain, no appetite, gastric ulcers, stomach penetration, bloody vomiting, hepatitis, cardiac muscle weakness, damage of oesophagus, irritability, sleeplessness, tiredness, dizziness, trembling, depressions, hearing impaired, hair loss, liver inflammation, water accumulation in the whole body, asthma attack". Due to 2 stomach penetrations, patient underwent a "very severe operation" at her stomach, and she died on 24 Mar 2006 as a consequence, according to reporter.

Follow up report received on 25 July 2006. In patient's daughter letter Novartis was pleaded to send the hospital report to the daughter. She can't understand the death of her mother within only 19 days and requested explanations. Additional documents will be prepared by the physician after he returns from his holiday in 3 weeks.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06034449

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes			Yes		

Patient

Initials:	Sex: Female	Birth: 19261218	Age: 79 Year	weight (kg): 60	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Proteinuria	200604				recovered/resolved
2 Weight gain	200604				recovered/resolved
3 Nephrotic syndrome	200604				recovered/resolved
4 Hypoalbuminaemia	200604				recovered/resolved
5 Edema lower limb	200604				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	200602-	Lumbago
diclofenac			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Spinal cord degeneration			Yes	
Hypertension			Yes	
Spondylosis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06062830

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 26 Year	weight (kg): 149	BMI_int: 58.2	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Beta haemolytic streptococcal infection					fatal
2 Injection site cellulitis					fatal
3 Pneumonia					fatal
4 Liver failure					fatal
5 Injection site abscess					fatal
6 Inflammation at site of injection					fatal
7 Fever				2 Day	fatal
8 Multi organ failure					fatal
9 Streptococcal sepsis					fatal
10 Application site infection					fatal
11 Septicemia streptococcal					fatal
12 Necrotizing fasciitis					fatal
13 Colpitis					fatal
14 Sepsis					fatal
15 Subcutaneous abscess					fatal
16 Kidney failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	/	-	Cervicobrachialgia
diclofenac			48 Hour	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Morbid obesity			Yes	
Smoker's cough			Unknown	
Smoker			Unknown	
Shoulder pain			Unknown	Neck pain.
Body mass index high			Unknown	BMI 54.7
Bronchitis			Unknown	smoker's bronchitis

Primary source

Qualification:	Physician	observestudytype:
Literature:	Grass, Schuff, Bertram, Rothschild Intramuscular injection of diclo-case report and critical discussion Z orthop Ihre Grenzgeb 2004; 142:489-92	
Qualification:	Other Health Professional	observestudytype:
Literature:	Rectsmedizin; Grass H, Bertram C, Scuff A, Dettmeyer R; 16 (161); 2006; 161-164; Die intramuskulaere injection: Risikoabwaegung and Patientenaufklaerung am Beispiel diclofenac	
Qualification:	Other Health Professional	observestudytype:
Literature:	Zeitschrift fuer Orthopaedie und Ihre Grenzgebiete; Grass H, Schuff A, Bertram C, Rothschild MA; 142; 2004; 489-492; Intramuscular injection of diclofenac - Case report and critical discussion	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 05.03.2007 DE-002147023-PHBS2004DE13654

Sender diagnosis: not reported

Narrative

Literature report received on 11 Oct 2004: This patient received a first intramuscular injection in left upper arm of diclofenac (manufacturer unknown) for persisting neck and shoulder pain. 14 hours later, a second injection was given. 48 hours after the first injection, the patient developed high fever (over 39C) accompanied by first phlegmon development and then abscess formation at the injection site of the left upper arm. The patient was treated with high doses of cephalozin (3x2 g i.v.) and the abscess was opened surgically two days later. A microbiological analysis of a wound smear showed beta-haemolytic streptococci of sero-group A. Postoperatively, the patient developed severe sepsis in association with pneumonia and progressive kidney and liver failure. In addition, the patient developed colpitis. Despite of intensive medical care including treatment with catecholamines, ventilation and hemodialysis the patient developed multiorgan failure and died 13 days after the first i.m. injection. Full English translation was requested.

Full English translation was received on 4 Nov 2004. This 26-year-old woman represented at the on-call medical service with persistent neck and shoulder pain (cervicobrachialgia). She received 2 intramuscular injections in the left upper arm with an interval of 14 hours between them for the acute treatment of the pain. The active substance administered was diclofenac. Owing to gross obesity (BW 149 kg, height 160 cm, BMI 54.7), it was decided not to administer an intragluteal injection. The known particulars of the case did not give yield any evidence of incorrect disinfection of the skin or incorrect injection technique. 48 hours after the first injection the woman developed a high fever with temperatures above 39C on axillary measurement. This rise in temperature was accompanied by an initially phlegmonous change at the site of injection on the left upper arm, which was opened surgically after 2 days, during which an abscess was formed under in-patient conditions and antibiotic treatment (initially antibiotic therapy with cephalozin 3 x 2 g I.V. high-dose treatment). The subcutaneous fat was described as normal in the surgical report. After opening of the muscle fascia, copious purulent secretion was removed. The microbiological analysis of a wound swab identified sero-group A (3-haemolytic streptococci). The patient developed severe septicemia postoperatively. Pneumonia and increasing kidney and liver failure predominated in this septic course. Colpitis also occurred. Despite intensive medical endeavours with administration of catecholamines, artificial ventilation, hemodialysis, repeated debriding of the wound and antibiotic treatment adjusted to repeated antibiotic sensitivity tests, fatal multiple organ failure occurred 13 days after the last I.M. injection. A coroner's post-mortem examination was ordered to clarify liability in terms of medical negligence. Macroscopically there were non-specific organ changes consistent with the clinical picture of multiple organ failure. At the left upper arm the conditions were largely the normal postoperative findings after opening of an abscess. In the deep musculature of the upper arm (extensor muscles) small necrotic foci were also identified. Histological, examination of the muscles of the upper arm yielded the characteristic picture of necrotizing fasciitis in the area surrounding the site of surgery. No evidence of pre-existing disease distinguishable from the signs of the multiple organ failure was found; a history of so-called "smoker's" bronchitis was known. A causal connection was first of all confirmed between the intramuscular injection of diclofenac and the muscular inflammation. Even when the required disinfection procedures are followed before an injection, there is a residual risk of the spread of microorganisms in the area of injection. The reported case fits with the number of already published severe complications after administration of diclofenac as an i.m. injection.

Follow-up information received on 01 Mar 2007. This report was published again on Rechtsmedizin, 2006, 16 (161) p161-164, Die intramuskuläre Injektion: Risikoabwägung und Patientenaufklärung am Beispiel Diclofenac, Grass H, Bertram C, Schuff A, Dettmeyer R. The full English translation of this article was requested.

NOVARTIS, 27.03.2007 DE-002147023-PHBS2004DE13654

Sender diagnosis: not reported

Narrative

Literature report received on 11 Oct 2004: This patient received a first intramuscular injection in left upper arm of diclofenac (manufacturer unknown) for persisting neck and shoulder pain. 14 hours later, a second injection was given. 48 hours after the first injection, the patient developed high fever (over 39C) accompanied by first phlegmon development and then abscess formation at the injection site of the left upper arm. The patient was treated with high doses of cephalozin (3x2 g i.v.) and the abscess was opened surgically two days later. A microbiological analysis of a wound smear showed beta-haemolytic streptococci of sero-group A. Postoperatively, the patient developed severe sepsis in association with pneumonia and progressive kidney and liver failure. In addition, the patient developed colpitis. Despite of intensive medical care including treatment with catecholamines, ventilation and hemodialysis the patient developed multiorgan failure and died 13 days after the first i.m. injection. Full English translation was requested.

Full English translation was received on 4 Nov 2004. This 26-year-old woman represented at the on-call medical service with persistent neck and shoulder pain (cervicobrachialgia). She received 2 intramuscular injections in the left upper arm with an interval of 14 hours between them for the acute treatment of the pain. The active substance administered was diclofenac. Owing to gross obesity (BW 149 kg, height 160 cm, BMI 54.7), it was decided not to administer an intragluteal injection. The known particulars of the case did not give yield any evidence of incorrect disinfection of the skin or incorrect injection technique. 48 hours after the first injection the woman developed a high fever with temperatures above 39C on axillary measurement. This rise in temperature was accompanied by an initially phlegmonous change at the site of injection on the left upper arm, which was opened surgically after 2 days, during which an abscess was formed under in-patient conditions and antibiotic treatment (initially antibiotic therapy with cephalozin 3 x 2 g I.V. high-dose treatment). The subcutaneous fat was described as normal in the surgical report. After opening of the muscle fascia, copious purulent secretion was removed. The microbiological analysis of a wound swab identified sero-group A (3-haemolytic streptococci). The patient developed severe septicemia postoperatively. Pneumonia and increasing kidney and liver failure predominated in this septic course. Colpitis also occurred. Despite intensive medical endeavours with administration of catecholamines, artificial ventilation, hemodialysis, repeated debriding of the wound and antibiotic treatment adjusted to repeated antibiotic sensitivity tests, fatal multiple organ failure occurred 13 days after the last I.M. injection. A coroner's post-mortem examination was ordered to clarify liability in terms of medical negligence. Macroscopically there were non-specific organ

changes consistent with the clinical picture of multiple organ failure. At the left upper arm the conditions were largely the normal postoperative findings after opening of an abscess. In the deep musculature of the upper arm (extensor muscles) small necrotic foci were also identified. Histological examination of the muscles of the upper arm yielded the characteristic picture of necrotizing fasciitis in the area surrounding the site of surgery. No evidence of pre-existing disease distinguishable from the signs of the multiple organ failure was found; a history of so-called "smoker's" bronchitis was known. A causal connection was first of all confirmed between the intramuscular injection of diclofenac and the muscular inflammation. Even when the required disinfection procedures are followed before an injection, there is a residual risk of the spread of microorganisms in the area of injection. The reported case fits with the number of already published severe complications after administration of diclofenac as an i.m. injection.

Follow-up information received on 01 Mar 2007. This report was published again on Rechtsmedizin, 2006, 16 (161) p161-164, Die intramuskuläre Injektion: Risikoabwägung und Patientenaufklärung am Beispiel Diclofenac, Grass H, Bertram C, Schuff A, Dettmeyer R. The full English translation of this article was requested.

Follow-up information received on 21 Mar 2007. The full English translation of this article was received. No new information was added.

Generics [UK] Ltd, 06.05.2008 DE-MYLANLABS-2008S1007051

Sender diagnosis: not reported

Narrative

GRAB H, SCHUFF A, BERTRAM C, ROTHSCCHILD MA. INTRAMUSCULAR ADMINISTRATION OF DICLOFENAC-CASE REPORT AND CRITICAL DISCUSSION OF A THERAPEUTIC MEASURE. Z ORTHOP. 2004. 142:489-492.

A 26-year-old woman went to the emergency medical service on account of persistent neck and shoulder pain (cervicobrachialgia). For acute treatment of the pain symptoms, she was given 2 intramuscular injections in the left upper arm spaced 14 hours apart. The drug administered was diclofenac. Medical personnel refrained from giving an intragluteal injection on account of morbid obesity (body weight 149 kg, height 160 cm, BMI 54.7); descriptions of the circumstances of the case gave no indications of improper skin disinfection or injection technique. Forty-eight hours after the first injection, the woman had a high fever with axillary temperatures measured at over 39 °C. This rise in temperature was accompanied by an initially phlegmonous change at the injection site on the left upper arm; after 2 days with abscess formation in the interim under hospital conditions and antibiotic treatment (initially, high-dose antibiotic therapy with 3 x 2 g IV cefazolin), this was surgically incised. In the operation report, the subcutaneous fatty tissue was described as unremarkable. Copious purulent secretion drained out following incision of the fascia. Microbiological testing of a wound swab found serogroup A beta-hemolytic streptococci. The patient developed severe sepsis postoperatively. The predominant features of the sepsis were pneumonia and worsening renal and hepatic failure. In addition, colitis occurred. Despite intensive medical efforts involving the administration of catecholamines, artificial respiration, hemodialysis, repeated wound debridement, and antibiotics adjusted in accordance with repeated antibiotic sensitivity tests, fatal multiorgan failure occurred 13 days after the first IM injection.

A forensic autopsy was ordered to clarify the issue of imputed negligence in the sense of medical malpractice. From the macroscopic perspective, non-specific organ changes consistent with the clinical picture of multiorgan failure were found at the autopsy. Largely unremarkable postoperative conditions subsequent to abscess incision were found at the left upper arm. In the deeper upper arm musculature (extensor muscles), it was possible to distinguish fairly small necrotic foci. In terms of histology, the examination of the upper arm musculature presented the typical picture of necrotizing fasciitis in the area of the surgical margins. There were no indications of preexisting diseases that could be differentiated from the signs of multiorgan failure; a so-called smoker's cough was known from the patient's history.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06041445

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes			Yes		

Patient

Initials:	Sex: Male	Birth: 19720208	Age: 33 Year	weight (kg): 67.3	BMI_int: 21.73	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Haemolytic anaemia	2006				not recovered/not resolved
2 Reticulocyte count increased	2006				not recovered/not resolved
3 Haptoglobin decreased	2006				not recovered/not resolved
4 LDH increased	2006				not recovered/not resolved
5 Blood bilirubin indirect increased	2006				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac dispers diclofenac sodium	Oral	150 Mg milligram(s)/Day	20060701-20060703 3 Day	Cephalgia

Drugs (concomitant)

2 ibu-ratiopharm 800 ibuprofen	Oral	/	- 1 Year	Cephalgia
3 symbicort budesonide; formoterol	Respiratory (inhalation)	/	200603- -	Allergic asthma

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pollen allergy			Unknown	
House dust allergy			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06045127

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 63 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nephritis					Unknown
2 Kidney enlargement					recovered/resolved
3 Urine volume decreased					recovered/resolved
4 Urea increased					recovered/resolved
5 Serum creatinine increased					recovered/resolved
6 Renal tubular disorder					recovered/resolved
7 Melena					recovered/resolved
8 Hepatic enzymes increased					recovered/resolved
9 Haematocrit decreased					recovered/resolved
10 Fever					recovered/resolved
11 Ferritin increased					recovered/resolved
12 C-reactive protein increased					recovered/resolved
13 Anuria					recovered/resolved
14 Acute renal failure					recovered/resolved
15 Abdominal pain					recovered/resolved
16 Elevated liver enzymes					Unknown
17 Urinary infection					Unknown
18 Nephritis interstitial					recovered/resolved
19 Uraemia					recovered/resolved
20 Anuria					recovered/resolved
21 Abdominal pain localised					Unknown
22 Acute renal failure					recovered/resolved
23 Fever					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylsalicylic acid	Oral	/	-	Drug use for unknown
acetylsalicylic acid		245	-	indication
2 diclofenac	Oral	/	-	Back pain
diclofenac		245	-	
3 PARACETAMOL+ASA	Oral	/	-	Back pain
acetylsalicylic acid; paracetamol		245	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Milk allergy			Yes	
Ankylosing spondylitis	1998		Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Nephrology Dialysis Transplantation; Ruester M, Kiehntop M, Groene H-J, Wolf G; 21; 2006; 2328-2330; A friday afternoon case of apparent anti-glomerular basement nephritis	
Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:

Literature:

Qualification: Other Health Professional observestudytype:

Literature: Ruester M, Kiehnopf M, Groene H, Wolf G. A Friday afternoon case of apparent anti-glomerular basement nephritis. Nephrol Dial Transplant 2006 Aug;21:8:2328-2330.

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
C-reactive protein		365.4mg/l	365.4mg/l	
Proteinuria		<300 mg/l	<300 mg/l	
Proteinuria		204 mg/24 h	204 mg/24	
Serum creatinine		647 micromol/l	647 micron	
Urea		26.2 mmol/l	26.2 mmol/l	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 26.09.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-05096GD

Sender diagnosis: not reported

Narrative

The case of a patient is reported who developed analgesic-induced acute renal failure after the use of paracetamol + ASA and diclofenac for back pain.

A 63-year-old man was first admitted to the hospital with sudden fever > 39 degC lasting for 5 days, accompanied by breath-dependent right-sided abdominal pain and melena. Furthermore, the patient had recently noticed a declining volume of urine. The patient had suffered from ankylosing spondylitis since 1998. No other accompanying diseases were known. The patient was transferred to a clinic, with the working diagnosis of acute renal failure on a Friday morning. There was no arthritis and haemoptysis. The patient admitted contact with mice in the past, as he was an avid gardener. He reported the intake of 2 tablets of a combination of acetyl salicylic acid and acetaminophen in the last days and of circa 40 tablets of diclofenac in the previous 3 months for back pain. He had taken no other medication. On admission, the patient appeared sickly, with fever of 38.9 degC and pain in the right epigastric region on palpation. Blood pressure was 120/80 mmHg. The patient remained anuric (< 80 ml/day) despite volume infusion (1.5 l of 0.9 % NaCl). There were no skin signs of vasculitis and the neurological examination was within normal limits. Initial serum laboratory investigations revealed serum creatinine of 647 µmol/l (normal range 72 -124 µmol/l) and urea of 26.2 mmol/l (normal range 1.7 - 8.3 mmol/l), which corresponds to a calculated clearance of 13.5 ml/min (Cockcroft-Gault formula). Inflammatory markers were increased with a C-reactive protein of 365.4 mg/l (normal range 0 - 5 mg/l) and a ferritin as acute phase protein of 3107 mcg/l (normal range 30 - 300 mcg/l). A modest elevation of liver enzymes was also observed. An urinary sediment showed < 5 % dysmorphic erythrocytes, a few hyaline casts, but proteinuria was > 300 mg/l. Electrocardiogram, X-ray of the chest and a gastroscopy, performed due to melena, were normal. The ultrasound revealed enlarged kidneys up to 14 x 7 cm, suggesting acute renal failure and not chronic renal disease.

An acute Hantavirus infection was excluded. The titres of different auto-antibodies were negative (ANA, ds-DNA, c-ANA, p-ANCA), but an enzyme-linked immunosorbent assay (ELISA) (quantitative anti-GBM from Imtec, Berlin, Germany) for anti-GBM antibodies was positive, with high titres (> 200 U/ml). Although anti-GBM disease was unlikely because of the clinical presentation (no active urinary sediment, no proteinuria), the high anti-GBM titre, received on Friday afternoon in the presence of deteriorating renal function, prompted us to consider rapid-progressive glomerulonephritis. Treatment with methylprednisolone pulse therapy (1 g/day) and plasmapheresis was promptly initiated. Due to the anuria and uraemia, this regimen was supported by an intermittent haemodialysis therapy (a total of 3 dialysis sessions). A renal biopsy was performed the following Monday. By light microscopy, a diffuse, moderate acute and potentially reversible tubular damage, in combination with a focal mild-to-moderate interstitial nephritis, was seen (data not shown). However, glomeruli were normal and revealed no signs of rapid progressive glomerulonephritis. The indirect immunofluorescence for IgG was also negative. Immunohistochemistry for IgA, IgG, IgM, C1q, C3, fibrin and C4d were negative. Plasmapheresis was stopped and prednisolone was tapered. The serum was tested for the presence of anti-GBM antibodies again and remained positive with the ELISA kit from Imtec. However, using monkey renal slides as target, no binding to glomerular basement membranes was not seen by indirect immunofluorescence. Furthermore, an anti-GBM ELISA from another manufacturer (Orgentec Diagnostika, Main, Germany) and a western blot test (Prof. Selig, Karlsruhe, Germany) did not reveal anti-GBM antibodies. Further analysis of the serum revealed antibodies against bovine albumin and after questioning the patient, an allergy to milk was revealed. The blocking reagent of the ELISA kit from Imtec contained bovine serum albumin, suggesting the reason for the false-positive result. The likely diagnosis in this case was analgesic and volume depletion-induced acute renal failure complicated by urinary infection, but not anti-GBM disease. After 4 months, renal function was completely recovered, and proteinuria was 204 mg/24 h. No immunosuppressive treatment was given.

Boehringer Ingelheim GmbH, Germany, 28.09.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-05096GD

Sender diagnosis: not reported

Narrative

The case of a patient is reported who developed analgesic-induced acute renal failure after the use of paracetamol + ASA and diclofenac for back pain.

A 63-year-old man was first admitted to the hospital with sudden fever > 39 degC lasting for 5 days, accompanied by breath-dependent right-sided

abdominal pain and melena. Furthermore, the patient had recently noticed a declining volume of urine. The patient had suffered from ankylosing spondylitis since 1998. No other accompanying diseases were known. The patient was transferred to a clinic, with the working diagnosis of acute renal failure on a Friday morning. There was no arthritis and haemoptysis. The patient admitted contact with mice in the past, as he was an avid gardener. He reported the intake of 2 tablets of a combination of acetyl salicylic acid and acetaminophen in the last days and of circa 40 tablets of diclofenac in the previous 3 months for back pain. He had taken no other medication. On admission, the patient appeared sickly, with fever of 38.9 degC and pain in the right epigastric region on palpation. Blood pressure was 120/80 mmHg. The patient remained anuric (< 80 ml/day) despite volume infusion (1.5 l of 0.9 % NaCl). There were no skin signs of vasculitis and the neurological examination was within normal limits. Initial serum laboratory investigations revealed serum creatinine of 647 µmol/l (normal range 72 -124 µmol/l) and urea of 26.2 mmol/l (normal range 1.7 - 8.3 mmol/l), which corresponds to a calculated clearance of 13.5 ml/min (Cockcroft-Gault formula). Inflammatory markers were increased with a C-reactive protein of 365.4 mg/l (normal range 0 - 5 mg/l) and a ferritin as acute phase protein of 3107 µg/l (normal range 30 - 300 µg/l). A modest elevation of liver enzymes was also observed. An urinary sediment showed < 5 % dysmorphic erythrocytes, a few hyaline casts, but proteinuria was > 300 mg/l. Electrocardiogram, X-ray of the chest and a gastroscopy, performed due to melena, were normal. The ultrasound revealed enlarged kidneys up to 14 x 7 cm, suggesting acute renal failure and not chronic renal disease.

An acute Hantavirus infection was excluded. The titres of different auto-antibodies were negative (ANA, ds-DNA, c-ANCA, p-ANCA), but an enzyme-linked immunosorbent assay (ELISA) (quantitative anti-GBM from Imtec, Berlin, Germany) for anti-GBM antibodies was positive, with high titres (> 200 U/ml). Although anti-GBM disease was unlikely because of the clinical presentation (no active urinary sediment, no proteinuria), the high anti-GBM titre, received on Friday afternoon in the presence of deteriorating renal function, prompted us to consider rapid-progressive glomerulonephritis. Treatment with methylprednisolone pulse therapy (1 g/day) and plasmapheresis was promptly initiated. Due to the anuria and uraemia, this regimen was supported by an intermittent haemodialysis therapy (a total of 3 dialysis sessions). A renal biopsy was performed the following Monday. By light microscopy, a diffuse, moderate acute and potentially reversible tubular damage, in combination with a focal mild-to-moderate interstitial nephritis, was seen (data not shown). However, glomeruli were normal and revealed no signs of rapid progressive glomerulonephritis. The indirect immunofluorescence for IgG was also negative. Immunohistochemistry for IgA, IgG, IgM, C1q, C3, fibrin and C4d were negative. Plasmapheresis was stopped and prednisolone was tapered. The serum was tested for the presence of anti-GBM antibodies again and remained positive with the ELISA kit from Imtec. However, using monkey renal slides as target, no binding to glomerular basement membranes was not seen by indirect immunofluorescence. Furthermore, an anti-GBM ELISA from another manufacturer (Orgentec Diagnostika, Main, Germany) and a western blot test (Prof. Selig, Karlsruhe, Germany) did not reveal anti-GBM antibodies. Further analysis of the serum revealed antibodies against bovine albumin and after questioning the patient, an allergy to milk was revealed. The blocking reagent of the ELISA kit from Imtec contained bovine serum albumin, suggesting the reason for the false-positive result. The likely diagnosis in this case was analgesic and volume depletion-induced acute renal failure complicated by urinary infection, but not anti-GBM disease. After 4 months, renal function was completely recovered, and proteinuria was 204 mg/24 h. No immunosuppressive treatment was given.

NOVARTIS, 02.11.2006 DE-002147023-PHBS2006DE16212

Sender diagnosis: not reported

Narrative

Literature report received on 18 Oct 2006: The patient was first admitted to hospital with sudden fever of over 39.0°C lasting for 5 days, accompanied by breath-dependent right-sided abdominal pain and melena. Furthermore, the patient had recently noticed a declining volume of urine. The patient had suffered from ankylosing spondylitis since 1998. No other accompanying diseases were known. The patient was transferred to the clinic, with the working diagnosis of acute renal failure. There was no arthritis and haemoptysis. The patient admitted contact with mice in the past, as he was an avid gardener. He reported the intake of two tablets of a combination of acetyl salicylic acid and acetaminophen in the last days and of around 40 tablets of diclofenac (manufacturer unknown) in the previous 3 months for back pain. He had taken no other medication. On admission, the patient appeared sickly, with fever of 38.0°C and pain in the right epigastric region on palpation. Blood pressure was 120/80 mmHg. The patient remained anuric (<80 ml/day) despite volume infusion (1.5 l of 0.9% NaCl). There were no skin signs of vasculitis and the neurological examination was within normal limits. Initial serum laboratory investigations revealed serum creatinine of 647 µmol/l (normal range 72-124 µmol/l) and urea of 26.2 mmol/l (normal range 1.7-8.3 mmol/l), which corresponds to a calculated clearance of 13.5 ml/min (Cockcroft-Gault formula). Inflammatory markers were increased with a C-reactive protein of 365.4 mg/l (normal range 0-5mg/l) and a ferritin as acute phase protein of 3107 µg/l (normal range 30-300 µg/l). A modest elevation of liver enzymes was also observed. A urinary sediment showed <5% dysmorphic erythrocytes, a few hyaline casts, but proteinuria was <300 mg/l. Electrocardiogram, X-ray of the chest and a gastroscopy, performed due to melena, were normal. The ultrasound revealed enlarged kidneys up to 14 x 7 cm, suggesting acute renal failure and not chronic renal disease. An acute Hantavirus infection was excluded. The titres of different auto-antibodies were negative (ANA, ds-DNA, c-ANCA, p-ANCA), but an enzyme-linked immunosorbent assay for anti-GBM antibodies was positive, with high titres (>200 U/ml). Although anti-GBM disease was unlikely because of the clinical presentation (no active urinary sediment, no proteinuria), the high anti-GBM titre, in the presence of deteriorating renal function, prompted to consider rapid-progressive glomerulonephritis. Treatment with methylprednisolone pulse therapy and plasmapheresis was promptly initiated. Due to anuria and uraemia, this regimen was supported by an intermittent haemodialysis therapy (a total of three dialysis sessions). A renal biopsy was performed. By light microscopy, a diffuse, moderate, acute and potentially reversible tubular damage, in combination with a focal mild-to-moderate interstitial nephritis, was seen. However, glomeruli were normal and revealed no signs of rapid progressive glomerulonephritis. The indirect immunofluorescence for IgG was also negative. Immunohistochemistry for IgA, IgG, IgM, C1q, C3, fibrin and C4d were negative. Plasmapheresis was stopped and prednisolone was tapered. The serum was tested for the presence of anti-GBM antibodies again and remained positive with the ELISA kit from Imtec. Further analysis of the serum revealed antibodies against bovine albumin and after questioning the patient, an allergy to milk was revealed. The blocking reagent of the ELISA kit from Imtec contained bovine serum albumin, suggesting the reason for the false-positive result. The likely diagnosis in this case was analgesic and volume depletion-induced acute renal failure complicated by urinary infection, but not anti-GBM disease. After 4 months, renal function was completely recovered, and proteinuria was 204 mg/24 h. No immunosuppressive treatment was given.

PLIVA Research and Development Ltd., 02.05.2007 DE-PLIVA Research and Development Ltd.-107#08#2007-01241

Sender diagnosis: not reported

Narrative

This case was found in scientific literature.

A 63-year-old man was first admitted to the hospital with sudden fever $>39^{\circ}\text{C}$ lasting for 5 days, accompanied by breath-dependent right-sided abdominal pain and melena. Furthermore, the patient had recently noticed a declining volume of urine. The patient had suffered from ankylosing spondylitis since 1998. No other accompanying diseases were known. The patient was transferred to our clinic, with the working diagnosis of acute renal failure on a Friday morning.

There was no arthritis and haemoptysis. The patient admitted contact with mice in the past, as he was an avid gardener. He reported the intake of two tablets of a combination of acetyl salicylic acid and acetaminophen in the last days and of circa 40 tablets of diclofenac in the previous 3 months for back pain. He had taken no other medication. On admission, the patient appeared sickly, with fever of 38.0°C and pain in the right epigastric region on palpation. Blood pressure was 120/80 mmHg. The patient remained anuric ($<80\text{ml/day}$) despite volume infusion (1.5l of 0.9% NaCl). There were no skin signs of vasculitis and the neurological examination was within normal limits. Initial serum laboratory investigations revealed serum creatinine of 647 micromol/l (normal range 72-124 micromol/l) and urea of 26.2 mmol/l (normal range 1.7-8.3 mmol/l), which corresponds to a calculated clearance of 13.5 ml/min (Cockcroft-Gault formula). Inflammatory markers were increased with a C-reactive protein of 365.4mg/l (normal range 0-5mg/l) and a ferritin as acute phase protein of 3107 micromol/l (normal range 30-300 micromol/l). A modest elevation of liver enzymes was also observed. A urinary sediment showed $<5\%$ dysmorphic erythrocytes, a few hyaline casts, but proteinuria was $<300\text{ mg/l}$. Electrocardiogram, X-ray of the chest and a gastroscopy, performed due to melena, were normal. The ultrasound revealed enlarged kidneys up to $14 \times 7\text{ cm}$, suggesting acute renal failure and not chronic renal disease.

An acute Hantavirus infection was excluded. The titres of different auto-antibodies were negative (ANA, ds-DNA, c-ANCA, p-ANCA), but an enzyme-linked immunosorbent assay (ELISA) (quantitative anti-GBM from Imtec, Berlin, Germany) for anti-GBM antibodies was positive, with high titres ($>200\text{ U/ml}$). Although anti-GBM disease was unlikely because of the clinical presentation (no active urinary sediment, no proteinuria), the high anti-GBM titre, received on Friday afternoon in the presence of deteriorating renal function, prompted us to consider rapid-progressive glomerulonephritis. Treatment with methylprednisolone pulse therapy (1 g/day) and plasmapheresis was promptly initiated. Due to the anuria and uraemia, this regimen was supported by an intermittent haemodialysis therapy (a total of three dialysis sessions). A renal biopsy was performed the following Monday. By light microscopy, a diffuse, moderate, acute and potentially reversible tubular damage, in combination with a focal mild-to-moderate interstitial nephritis, was seen (data not shown). However, glomeruli were normal and revealed no signs of rapid progressive glomerulonephritis. The indirect immunofluorescence for IgG was also negative, Immunohistochemistry for IgA, IgG, IgM, C1q, C3, fibrin and C4d were negative. Plasmapheresis was stopped and prednisolone was tapered. The serum was tested for the presence of anti-GBM antibodies again and remained positive with the ELISA kit from Imtec. However, using monkey renal slides as target, no binding to glomerular basement membranes was not seen by indirect immunofluorescence. Furthermore, an anti-GBM ELISA from another manufacturer (Orgentec Diagnostika, Mainz, Germany) and a western blot Lest (Prof. Selig, Karlsruhe, Germany) did not reveal anti-GBM antibodies. Further analysis of the serum revealed antibodies against bovine albumin and after questioning the patient, an allergy to milk was revealed. The blocking reagent of the ELISA kit from Imtec contained bovine serum albumin, suggesting the reason for the false-positive result. The likely diagnosis in this case was analgesic and volume depletion-induced acute renal failure complicated by urinary infection, but not anti-GBM disease. After 4 months, renal function was completely recovered, and proteinuria was 204 mg/24 h . No immunosuppressive treatment was given.

Dr. R. Pflieger Chemische Fabrik GmbH, 17.09.2009 DE-PFLEGER-000070

Sender diagnosis: not reported

Narrative

A 63-year-old man was admitted to the hospital with sudden fever more than 39°C lasting for 5 days, accompanied by breath-dependent right-sided abdominal pain and melena. The patient had recently noticed a declining volume of urine. The patient had suffered from ankylosing spondylitis since 1998. The diagnosis was acute renal failure. He reported the intake of two tablets of a combination of acetylsalicylic acid and acetaminophen in the last days and of circa 40 tablets of diclofenac in the previous 3 months for back pain.

After 4 months, renal function was completely recovered.<dbtext>To_Char_NARRATIVE_2</dbtext>

Reporter's comment:

The likely diagnosis in this case was analgesic and volume depletion-induced acute renal failure complicated by urinary infection.

Heumann Pharma GmbH & Co. Generika KG, 01.10.2009 DE-HEUMANN1-000050

Sender diagnosis: not reported

Narrative

The following information was received from a scientific literature article via literature order on 25 Sep 2009 (date of notice).

A 63-year-old male patient (DOB unknown) was admitted to hospital with sudden fever $>39^{\circ}\text{C}$ lasting for 5 days, accompanied by breath-dependent, right-sided abdominal pain and melena. Furthermore, the patient had noticed a decline in volume of urine recently. Medical history included ankylosing spondylitis since 1998.

The patient had taken approximately 40 tablets of diclofenac for back pain in the previous 3 months. Additionally, he had taken two tablets of a combination of acetyl salicylic acid and acetaminophen in the last days.

The patient was transferred to the reporter's clinic and diagnosed with a working diagnosis of acute renal failure which was confirmed by several subsequently performed tests. On admission, no arthritis or haematoptysis were present. Due to the deterioration of the renal function, the patient was treated with methylprednisolone pulse therapy (1g/day) and plasmapheresis was initiated. He was supported by three dialysis sessions due to anuria and uraemia. A renal biopsy was performed the next Monday which revealed potentially reversible tubular damage and a focal mild-to-moderate interstitial nephritis. Glomeruli were normal and revealed no signs of rapid progressive glomerulonephritis. During the course of several serum investigations, an allergy to milk was detected.

After 4 months, renal function was completely recovered and proteinuria was 204 mg/24h. No immunosuppressive treatment was given.

By time of this report renal failure acute was recovered as well as the fever & 39°C occurring at first admittance to hospital. All other reactions' outcomes are unknown.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06052929

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19560506	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver cirrhosis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/Day	1990-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.11.2006 DE-002147023-PHNU2006DE03597

Sender diagnosis: not reported

Narrative

Initial report received on 31 Oct 2006. Preliminary scarce information from physician who was asked to write an expert opinion on an individual case. The patient had been treated with Voltaren coated tablets (diclofenac) for a long period of time, but intermittently. Reportedly, the patient had taken Voltaren 4 to 5 times in the 1990s, when required, thereby using N1 packages (smallest package sizes available in Germany). The physician stated that a pathologist had assessed the event as liver cirrhosis "drug-toxic", while he rated a causal relationship with Voltaren as remote, due to the weak temporal relationship. The reporting physician was suspecting some grade of ethanol toxicity in the case.

Serious spontaneous report (medically significantly assessed as unlikely according to the Basic Prescribing Information due to greater severity. The following term (liver damage including hepatitis) is already included in the Basic Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 02.01.2007 DE-002147023-PHNU2006DE03597

Sender diagnosis: not reported

Narrative

Initial report received on 31 Oct 2006. Preliminary scarce information from physician who was asked to write an expert opinion on an individual case. The patient had been treated with Voltaren coated tablets (diclofenac) for a long period of time, but intermittently. Reportedly, the patient had taken Voltaren 4 to 5 times in the 1990s, when required, thereby using N1 packages (smallest package sizes available in Germany). The physician stated that a pathologist had assessed the event as liver cirrhosis "drug-toxic", while he rated a causal relationship with Voltaren as remote, due to the weak temporal relationship. The reporting physician was suspecting some grade of ethanol toxicity in the case.

Follow up received on 20 Dec 2006. In a short letter, the reporter stated that a causal relationship with respect to Voltaren was no longer suspected. No further data were included.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058484

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency			8 Day		recovered/resolved
2 Drug abuse					Unknown
3 Suicide attempt					Unknown
4 Overdose					Unknown
5 Hepatic damage			8 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC		/	-	
diclofenac sodium			-	
2 IBUPROFEN		/	-	
ibuprofen			-	
3 PARACETAMOL	Oral	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal insufficiency			Unknown	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06545GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 27;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 8000;

Possibly influencing factor: renal insufficiency, 1000 mg diclofenac (overdose), 3600 mg ibuprofen (overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 2;

Duration of renal insufficiency: 8 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity. ... An additional predisposing condition seemed to be a pre-existing renal insufficiency with acetaminophen causing an acute-on-chronic renal failure that was observed in 3 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

Boehringer Ingelheim GmbH, Germany, 13.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06545GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D:

creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 27;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 8000;

Possibly influencing factor: renal insufficiency, 1000 mg diclofenac (overdose), 3600 mg ibuprofen (overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 2;

Duration of renal insufficiency: 8 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity. ... An additional predisposing condition seemed to be a pre-existing renal insufficiency with acetaminophen causing an acute-on-chronic renal failure that was observed in 3 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07000457

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No				Yes		

Patient

Initials:	Sex: Female	Birth: 19450517	Age: 59 Year	weight (kg): 64	BMI_int: 24.09	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20050209			8 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac diclofenac sodium	Oral	25 Mg milligram(s)/Day	20050202-20050206 5 Day	Upper abdominal pain

Drugs (concomitant)

2 johanniskraut hypericum extract	Oral	1 Df dosage form/Day	200101- -
3 magastron magaldrate	Oral	800 Mg milligram(s)/	198606- -
4 mcp-ratiopharm metoclopramide hydrochloride	Oral	/	200407-200502 7 Month
5 Multivitamine ascorbic acid; ergocalciferol; folic acid; nicotinamide; panthenol; retinol; riboflavin; thiamine hydrochloride	Oral	1 Df dosage form/Day	200101- -
6 novaminsulfon-ratiopharm metamizole	Oral	500 Mg milligram(s)/	1990-20050202 -
7 paracetamol paracetamol	Oral	500 Mg milligram(s)/Day	20050207-20050209 3 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Upper respiratory infection			Unknown	
Achalasia			Unknown	
Esophageal dilatation	1995		Unknown	
Hepatitis B	1997		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07000458

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19570520	Age: 48 Year	weight (kg): 82	BMI_int: 25.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose accidental	20050721	20050729	9 Day	31 Day	recovered/resolved
2 Lipase increased	20050721	20050729	9 Day	31 Day	recovered/resolved
3 Pancreatitis acute	20050721	20050729	9 Day	31 Day	recovered/resolved
4 Gamma-GT increased	20050721	20050729	9 Day	31 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo dispers diclofenac sodium	Oral	150 Mg milligram(s)/Day	20050621-20050725	Pain 35 Day
2 diclofenac retard diclofenac sodium	Oral	200 Mg milligram(s)/Day	20050621-20050725	Pain 35 Day
3 voltaren resinat diclofenac resinate	Oral	150 Mg milligram(s)/Day	20050621-20050725	Arthrosis 35 Day

Drugs (concomitant)

4 beofenac aceclofenac	Oral	100 Mg milligram(s)/	20050621-20050725	35 Day
5 heparin-natrium heparin sodium	Subcutaneous	5000 Kiu iu (1000s)/Day	20050621-20050725	35 Day
6 klinomycin minocycline	Oral	150 Mg milligram(s)/Day	20050621-20050725	35 Day
7 lexotanil bromazepam	Oral	3 Mg milligram(s)/Day	20050621-20050725	35 Day
8 novalgin tropfen metamizole sodium	Oral	25 Gtt drop(s)/Day	20050621-20050725	35 Day
9 oxygesic oxycodone hydrochloride	Oral	20 Mg milligram(s)/Day	20050621-20050725	35 Day
10 sobelin solubile clindamycin phosphate	Intravenous (not otherwis	1200 Mg milligram(s)/Day	20050621-20050725	35 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Upper abdominal pain			Unknown	
Knee operation			Unknown	
Smoker	1975		Unknown	
Penicillin allergy			Unknown	
Drug allergy			Unknown	Tramal

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 31.01.2007 DE-002147023-PHNU2007DE00606

Sender diagnosis: not reported

Narrative

Initial report received on 18 Jan 2007 (report received from BfArM, BfArM number 07000458). This patient was on Voltaren Resinat (150mg/d) + Diclofenac Retard (200mg/d) + Diclo Dispers (150mg/d) from 21 Jun 2005 until 25 Jul 2005, for arthrosis pain and pain unspecified, respectively. Apparently, all diclofenac drugs were combined (diclofenac overdose). Manufacturers of Diclofenac Retard and Diclo Dispers (both diclofenac) were not explicitly indicated. Also several other medications were given during the indicated time period. On 21 Jul 2005, patient suffered from acute pancreatitis. Lipase was increased (3490U/l) and gamma GT was elevated (166U/l). The creatinine level was 1.03mg/dl (no normal range given). Other lab was apparently normal. All diclofenac preparations were stopped on 25 Jul 2005. Pancreatitis improved after 29 Jul 2005, and patient fully recovered. Causality was rated as possible.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07003210

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19691012	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests abnormal					Unknown
2 Hepatitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Rectal	/Day	-	
diclofenac			-	
2 diclofenac	Unknown	/	-	Back pain
diclofenac			30240 Minute	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 08.02.2007 DE-002147023-PHNU2007DE00737

Sender diagnosis: not reported

Narrative

Initial physician report received on 31 Jan 2007. This patient who was administered Diclofenac (formulation and manufacturer unknown) due to back pain. After 3 weeks administration the patient developed hepatitis with increase of liver enzymes. Preliminary identical hepatitis episode due to Diclofenac was 7 and 3 years ago. The physician asked Novartis about further NSAID therapy and the mechanism of NSAID induced Hepatitis.

Sender's comment:

Serious spontaneous report (medically significant),Hepatitis already listed in the Basis Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07015502

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19510308	Age: 56 Year	weight (kg): 60	BMI_int: 23.44	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GOT increased	20070312			12 Day	not recovered/not resolved
2 Gamma-GT increased	20070312			12 Day	not recovered/not resolved
3 Liver function tests raised	20070312			12 Day	not recovered/not resolved
4 Alkaline phosphatase increased	20070312			12 Day	not recovered/not resolved
5 GPT increased	20070312			12 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat	Oral	/	20070301-20070305	Back pain
diclofenac resinate			5 Day	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 04.06.2007 DE-002147023-PHNU2007DE01813

Sender diagnosis: not reported

Narrative

Initial physician report received on 10 May 2007. A patient had been treated with Voltaren (diclofenac) resinat for 4 days and developed liver enzyme increase. No further information available.

Follow up report received on 25 May 2007. Events started on 12 Mar 2007 until today. Voltaren was started on 01 Mar 2007 to 05 Mar 2007. Lab values on 12 Mar 2007 were increased GGT (normal value <38 U/l) 263 U/l, baseline 31 Jul 2006 119 U/l, GOT (normal range <31 U/l) 55 U/l, GPT (normal range <34 U/l) 114 U/l, baseline 31 Jul 2007.

26 U/l, AP 167 U/l (normal range 53-141 U/l). Condition improved after discontinuation. Patient was not yet recovered. Two months after therapy GGT was still increased 130 U/l and GPT still on a small increased level 37 U/l. The relationship to the drug was assessed as probable.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07025847

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19890831	Age: 17 Year	weight (kg): 45	BMI_int: 17.58	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hematuria	20070510		14 Day	3 Day	recovered/resolved
2 Proteinuria	20070511			4 Day	recovered/resolved
3 Renal disorder	200705		10 Day		recovered/resolved
4 Glomerular filtration rate increased	20070513			6 Day	recovered/resolved
5 Gamma-GT increased	20070513			6 Day	recovered/resolved
6 Flank pain	20070510		10 Day	3 Day	recovered/resolved
7 Transaminases increased	20070513			6 Day	recovered/resolved
8 Albuminuria	20070510		14 Day	3 Day	recovered/resolved
9 Creatinine increased	20070513			6 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	/	20070509-20070509 1440 Minute	Skeletal pain
2 voltaren resinat diclofenac resinate	Oral	75 Mg milligram(s)/Day	20070508-20070508 1440 Minute	Skeletal pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fall			Unknown	Fall from bicycle

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.06.2007 DE-002147023-PHNU2007DE02055

Sender diagnosis: not reported

Narrative

Initial report received on 6 Jun 2007. Preliminary scarce information from physician. This patient was treated with Voltaren Resinat (diclofenac, colestyramin) for unstated reasons. On 16 May 2007, the patient suffered from acute renal failure and gamma GT and transaminases were found increased. The patient was admitted to hospital.

Sender's comment:

Serious spontaneous report (hospitalisation),Renal failure acute,Transaminases increased already listed in the Basis Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 06.07.2007 DE-002147023-PHNU2007DE02055

Sender diagnosis: not reported

Narrative

Initial report received on 6 Jun 2007. Preliminary scarce information from physician. This patient was treated with Voltaren Resinat (diclofenac, colestyramin) for unstated reasons. On 16 May 2007, the patient suffered from acute renal failure and gamma GT and transaminases were found increased. The patient was admitted to hospital.

Follow up received on 2 Jul 2007 (written documentation from physician). Further patient details were given. The patient was treated with Voltaren Resinat since 8 May 2007 for skeletal pain after a previous fall from a bicycle. As adverse events, flank pain, microhematuria, albuminuria, creatinine increase, and renal swelling (sonographically verified) were mentioned. The term renal failure was not confirmed / repeated. Also an increase of gamma GT was documented. The attached laboratory sheet referred to a highest creatinine level of 1.7mg/dl, and GFR (glomerular filtration rate) exceeded 60ml/min. Spurs of blood were found in the urine, but no leukocytes. On 11 May 2007, protein level was 1911.2mg/l, and albumin was 1425.7mg/l (upper limit of normal 20mg/l). Voltaren Resinat was stopped. The patient remained in the hospital for several days, but she could be discharged (no date given) and she made a full recovery. Causality was rated as probable for Voltaren Resinat, and other causations were excluded.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07066389

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19350910	Age: 71 Year	weight (kg): 84	BMI_int: 28.73	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Antibody result abnormal					Unknown
2 Hematocrit low	20070307			2 Day	recovering/resolving
3 Chills	20070308			1 Day	recovered/resolved
4 Creatinine increased	20070308			1 Day	recovered/resolved
5 CRP increased	20070312			5 Day	Unknown
6 Dizziness	20070308			1 Day	recovered/resolved
7 Hemoglobin decreased	20070307			2 Day	recovering/resolving
8 Hemolysis	20070308			1 Day	Unknown
9 Hypotension	20070308			1 Day	recovered/resolved
10 Lactate dehydrogenase increased	20070308			1 Day	Unknown
11 Thrombocyte count decreased	20070308			1 Day	recovered/resolved
12 Leukocyte count decreased	20070308			1 Day	recovered/resolved
13 Erythrocytes decreased	20070307			2 Day	recovering/resolving
14 Immune hemolytic anemia drug-induced	20070308			1 Day	recovered/resolved
15 Plasma protein abnormal					Unknown
16 Bilirubin increased	20070308			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN RESINAT diclofenac resinate	Oral	75 Mg milligram(s)/Day	20070308-20070308 1440 Minute	Pain management

Drugs (concomitant)

2 ALLOPURINOL allopurinol	Oral	1 Df dosage form/Day	-	
3 ARCOXIA etoricoxib	Oral	120 Mg milligram(s)/Day	20070307-	Pain management
4 BISOPROLOL bisoprolol	Oral	1 Df dosage form/Day	-	
5 clexane enoxaparin	Subcutaneous	/	-	
6 NEXIUM esomeprazole magnesium	Oral	1 Df dosage form/Day	-	
7 NORVASC /DEN/ amlodipine besilate	Oral	1 Df dosage form/Day	-	
8 prednisolon prednisolone	Oral	/	-	
9 RAMIPRIL ramipril	Oral	1 Df dosage form/Day	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Coxarthrosis			Yes	Right-sided coxarthrosis
Hypertension arterial			Yes	
Reflux oesophagitis			Yes	
Hemolytic anemia			Unknown	...nach intra- und postoperativer Blutung
Hematocrit low			Unknown	
Hemoglobin decreased			Unknown	
Intervertebral disc prolapse			Unknown	Intervertebral disc prolapse C6/C7
Erythrocytes decreased			Unknown	
Postoperative hemorrhage			Unknown	Anemia after intra- and postoperative hemorrhage
Hip prosthesis insertion	20070503		No	rechts

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Haemoglobin		7.4	mg/dl	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 24.08.2007 DE-002147023-PHNU2007DE02908

Sender diagnosis: not reported

Narrative

Initial report received on 21 Aug 2007 from German Health Authority (BfArM number 07066389), originating source hospital orthopedic unit. This patient was treated with Voltaren Resinat (diclofenac, colestyramin) and Arcoxia (etoricoxib, as a study medication) as pain medications. Arcoxia was given on 7 Mar 2007 only, and Voltaren Resinat was used on 8 Mar 2007 only. On 8 Mar 2007, patient showed a drop of hemoglobin from 9.9 to 7.4mg/dl, and the diagnosis of autoimmune hemolytic anemia was made. According to patient's anamnestic information, possible hemolytic anemia under a already previously used diclofenac drug (not further specified) was suspected. The patient was treated with prednisolon, two sets of erythrocyte concentrates, and parenteral fluid substitution. The patient fully recovered. Voltaren Resinat was not restarted. Later testing revealed diclofenac positive antibodies. Voltaren Resinat was assessed as causally related.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 27.08.2007 DE-002147023-PHNU2007DE02908

Sender diagnosis: not reported

Narrative

Initial report received on 21 Aug 2007 from German Health Authority (BfArM number 07066389), originating source hospital orthopedic unit. This patient was treated with Voltaren Resinat (diclofenac, colestyramin) and Arcoxia (etoricoxib, as a study medication) as pain medications. Arcoxia was given on 7 Mar 2007 only, and Voltaren Resinat was used on 8 Mar 2007 only. On 8 Mar 2007, patient showed a drop of hemoglobin from 9.9 to 7.4mg/dl, and the diagnosis of autoimmune hemolytic anemia was made. According to patient's anamnestic information, possible hemolytic anemia under a already previously used diclofenac drug (not further specified) was suspected. The patient was treated with prednisolon, two sets of erythrocyte concentrates, and parenteral fluid substitution. The patient fully recovered. Voltaren Resinat was not restarted. Later testing revealed diclofenac positive antibodies. Voltaren Resinat was assessed as causally related.

Internal data correction from 24 Aug 2007 of the data of 21 Aug 2007. Diclofenac was recoded with Novartis code (initially accidentally entered with WHO-code).

NOVARTIS, 08.10.2007 DE-002147023-PHNU2007DE03302

Sender diagnosis: not reported

Narrative

Initial report received on 1 Oct 2007 from German Health Authority, originating source physician (BfArM number 07066389, doctor letter and attached laboratory outprints). This patient was hospitalized on 4 Mar 2007 for elective endoprosthetic hip joint replacement (on 5 Mar 2007), under routine perioperative antibiotic and thrombosis prophylaxis. After the operation, physical therapy was initiated. Postoperative course was uneventful except for anemia due to intra- and postoperative hemorrhage. In the morning of 8 Mar 2007, after oral intake of one dose of Voltaren Resinat (diclofenac, colestyramin), patient suffered from dizziness, chills, and hypotension. Hemoglobin dropped from 9.9mg/dl to 7.4mg/dl, and a massive increase of hemolysis parameters was seen. The attached laboratory sheets also included evidence of increased CRP, increased creatinine, decreased thrombocyte and leukocyte counts, increased bilirubin, and increased LDH (lactate dehydrogenase), but these lab abnormalities were not further mentioned or indicated as adverse events. Under suspicion of drug-induced autoimmune hemolytic anemia, high dose i.v. prednisolon was initiated. After 2 erythrocyte concentrates and under enteral and parenteral fluid restitution, the circulatory situation stabilized. Hemolysis parameters and renal function parameters normalized. Further hematological diagnostics confirmed a significant increase of antibodies against diclofenac, and the event was assessed as diclofenac-induced autoimmune hemolytic anemia. The further course was without complication. The patient was discharged on 16 Mar 2007.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07072644

Serious Yes	Death No	Lifethr. Yes	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: 19490316	Age: 58 Year	weight (kg): 60	BMI_int: 21.26	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Creatinine increased					recovering/resolving
2 SGOT increased	20070618				recovered/resolved
3 Alkaline phosphatase increased	20070618				recovering/resolving
4 Gamma GT increased	20070618				recovering/resolving
5 SGPT increased	20070618				recovered/resolved
6 Leukocytosis	20070618				recovering/resolving
7 Quick's value decreased	20070618				recovering/resolving
8 Hypokalaemia	20070618				recovering/resolving
9 C-reactive protein increased	20070618				recovering/resolving
10 Hepatic failure	20070618		15 Day		not recovered/not resolved
11 Impaired renal function	20070618		15 Day		recovering/resolving
12 Diarrhoea	20070618				recovering/resolving
13 Hepatic failure	20070618				recovering/resolving
14 Hepatitis toxic drug-induced					recovering/resolving
15 Nausea	20070618				recovering/resolving
16 Pruritus	20070618				recovering/resolving
17 Vomiting					recovering/resolving
18 Cholestatic icterus	20070618				recovering/resolving
19 Liver function tests abnormal					Unknown
20 Faeces discoloured	20070618				recovered/resolved
21 Asthenia	20070618				recovering/resolving
22 Bilirubinaemia	20070618				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	/	-20070617 -	Ache
2 HERCEPTIN trastuzumab	Intravenous (not otherwis	330 Mg milligram(s)/Day	200402-20070605 -	Breast cancer metastatic
3 ibuhexal ibuprofen	Oral	/	-20070617 -	Pain NOS
4 ZOMETA zoledronic acid	Intravenous (not otherwis	4 Mg milligram(s)/Day	200507-20070605 -	Bone metastases

Drugs (concomitant)

5	aluminium hydroxide gel	Oral	124.9 Mg milligram(s)/	-
	aluminium hydroxide; aluminium hydroxide gel			-
6	magnesium carbonate	Oral	83.8 Mg milligram(s)/	-
	magnesium carbonate			-
7	METAMIZOLE	Oral	/	-
	metamizole sodium			-
8	METOCLOPRAMID	Oral	/	-
	metoclopramide hydrochloride			-
9	OCTACAINE	Oral	5 Mg milligram(s)/	-
	octacaine			-
10	tepilta	Oral	/	-
	aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide; oxetacaine			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Anaemia			Unknown	
Lymph node metastases	200703		Unknown	
Radiation therapy	200507		Unknown	
Breast operation			Unknown	
Chemotherapy			Unknown	
Metastatic carcinoma			Unknown	
Drug dependence			Unknown	
Pruritus			Unknown	
Blood transfusion	20070612		Unknown	
Breast cancer	200211		Yes	breast cancer left diagnosed in Nov 2002 with ablatio in Sep 2003; breast cancer right dignosed in M
Lymph node excision	200703		Unknown	partial lymphadenectomy right clavicular and left cervical
Radiation therapy	200703		Unknown	Postoperative, with 50.4 Gy
Bone metastases	200507		Yes	Thoracal Spine Th12 and left foot

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.08.2007 DE-NOVNCHHQ-GXKR2007DE06957

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 17 Aug 2007 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of metastatic breast cancer with breast ablation, polychemotherapy and radiation developed hepatic failure with renal impairment during treatment with IbuHEXAL (ibuprofen) and diclofenac (MAH unknown) for pain. Both drugs were taken irregularly at an unknown dosage for several months until 17 Jun 2007. Concomitantly the patient received several other drugs, of which Trastuzumab (herceptin) and zoledronic acid were also considered causative. The reporter stated that the patient took ibuprofen and diclofenac pain-dependent on her own initiative. The adverse events icterus with nausea and vomiting, pruritus and diarrhoea occurred on 18 Jun 2007. The patient was hospitalised on 20 Jun 2007 with abnormal laboratory values so as leucocytosis, increased CRP, decreased QuickÂ's value, hypokaliaemia and abnormal liver values. A liver biopsy was performed and revealed a drug-induced toxic hepatic failure. The patient was treated with amoxicillin/clavulanic acid, ondansetrone, prednisolon, pantoprazol and received substitution of glucose, vitamin K, iron, and sodiumchloride. When the patient was released from hospital on 05 Jul 2007 the condition was improving. The reporter assessed the causality of the adverse events and administration of ibuprofen and diclofenac as possible.

NOVARTIS, 23.08.2007 DE-NOVNCHHQ-GXKR2007DE06957

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 17 Aug 2007 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of metastatic breast cancer with breast ablation, polychemotherapy and radiation developed hepatic failure with renal impairment during treatment with IbuHEXAL (ibuprofen) and diclofenac (MAH unknown) for pain. Both drugs were taken irregularly at an unknown dosage for several months until 17 Jun 2007. Concomitantly the patient received several other drugs, of which Trastuzumab (herceptin) and zoledronic acid were also considered causative. The reporter stated that the patient took ibuprofen and diclofenac pain-dependent on her own initiative. The adverse events icterus with nausea and vomiting, pruritus and diarrhoea occurred on 18 Jun 2007. The patient was hospitalised on 20 Jun 2007 with abnormal laboratory values so as leucocytosis, increased CRP, decreased Quick's value, hypokalaemia and abnormal liver values. A liver biopsy was performed and revealed a drug-induced toxic hepatic failure. The patient was treated with amoxicillin/clavulanic acid, ondansetrone, prednisolon, pantoprazol and received substitution of glucose, vitamin K, iron, and sodiumchloride. When the patient was released from hospital on 05 Jul 2007 the condition was improving. The reporter assessed the causality of the adverse events and administration of ibuprofen and diclofenac as possible.

NOVARTIS, 23.08.2007 DE-SZ09-GXKR2007DE06957

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 17 Aug 2007 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of metastatic breast cancer with breast ablation, polychemotherapy and radiation developed hepatic failure with renal impairment during treatment with IbuHEXAL (ibuprofen) and diclofenac (MAH unknown) for pain. Both drugs were taken irregularly at an unknown dosage for several months until 17 Jun 2007. Concomitantly the patient received several other drugs, of which Trastuzumab (herceptin) and zoledronic acid were also considered causative. The reporter stated that the patient took ibuprofen and diclofenac pain-dependent on her own initiative. The adverse events icterus with nausea and vomiting, pruritus and diarrhoea occurred on 18 Jun 2007. The patient was hospitalised on 20 Jun 2007 with abnormal laboratory values so as leucocytosis, increased CRP, decreased Quick's value, hypokalaemia and abnormal liver values. A liver biopsy was performed and revealed a drug-induced toxic hepatic failure. The patient was treated with amoxicillin/clavulanic acid, ondansetrone, prednisolon, pantoprazol and received substitution of glucose, vitamin K, iron, and sodiumchloride. When the patient was released from hospital on 05 Jul 2007 the condition was improving. The reporter assessed the causality of the adverse events and administration of ibuprofen and diclofenac as possible.

NOVARTIS, 09.01.2008 DE-NOVNCHHQ-GXKR2008DE00464

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 03 Jan 2008 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of breast cancer on the left side diagnosed in Nov 2002 and on the right side diagnosed in Mar 2004 with bilateral ablation, chemotherapeutic combination and radiotherapy for bone metastases in Jul 2005, lymph node metastases with partial metastectomy and radiation in Mar 2007, and repeated blood transfusion for anaemia developed hepatocellular damage with hepatic failure, icterus, discoloured stools, nausea, vomiting, diarrhoea, pruritus, asthenia, hyperbilirubinaemia, transaminases increased, gamma GT increased, alkaline phosphatase increased, prothrombin level decreased, blood potassium decreased, leucocytosis, CRP increased, creatinine increased and renal function abnormal during treatment with IbuHEXAL (ibuprofen) for pain taken at an unknown dosage occasionally for several months. Concomitantly the patient received several other drugs, of which trastuzumab, zoledronate and diclofenac were considered causative as well. The adverse event occurred on 18 Jun 2007. Two days later the patient was admitted to hospital, 13 days after she had received the last infusion of trastuzumab and zoledronate. The patient was released from hospital 2 ½ weeks later with markedly improved condition. The reporter provided no causality assessment.

NOVARTIS, 09.01.2008 DE-SZ09-GXKR2008DE00464

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 03 Jan 2008 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of breast cancer on the left side diagnosed in Nov 2002 and on the right side diagnosed in Mar 2004 with bilateral ablation, chemotherapeutic combination and radiotherapy for bone metastases in Jul 2005, lymph node metastases with partial metastasectomy and radiation in Mar 2007, and repeated blood transfusion for anaemia developed hepatocellular damage with hepatic failure, icterus, discoloured stools, nausea, vomiting, diarrhoea, pruritus, asthenia, hyperbilirubinaemia, transaminases increased, gamma GT increased, alkaline phosphatase increased, prothrombin level decreased, blood potassium decreased, leucocytosis, CRP increased, creatinine increased and renal function abnormal during treatment with IbuHEXAL (ibuprofen) for pain taken at an unknown dosage occasionally for several months. Concomitantly the patient received several other drugs, of which trastuzumab, zoledronate and diclofenac were considered causative as well. The adverse event occurred on 18 Jun 2007. Two days later the patient was admitted to hospital, 13 days after she had received the last infusion of trastuzumab and zoledronate. The patient was released from hospital 2 ½ weeks later with markedly improved condition. The reporter provided no causality assessment.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07082707

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19330609	Age: 72 Year	weight (kg): 74.6	BMI_int: 32.29	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests raised	20060310				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clarithromycin clarithromycin	Oral	500 Mg milligram(s)/Day	20051215-20051227 13 Day	Borrelia infection
2 doxycycline doxycycline	Oral	400 Mg milligram(s)/Day	20051111-20051205 25 Day	Borrelia infection
3 voltaren dispers diclofenac sodium	Oral	50 Mg milligram(s)/	20040101-20060321 2 Year	Joint ache

Drugs (concomitant)

4 atacand candesartan cilexetil	Oral	4 Mg milligram(s)/Day	19950101-20060321 11 Year	Hypertension arterial
5 cibacen benazepril hydrochloride	Oral	20 Mg milligram(s)/Day	19900101- -	Hypertension arterial
6 fosamax alendronate sodium	Oral	70 Mg milligram(s)/Week	20020101-20060321 4 Year	Osteoporosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hallux Valgus correction			No	
Hypertension arterial	19940101		Yes	
Osteoporosis	20020101		Yes	
Dysuria	20020101		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine urine	20060322	Abfall/decreased	68 umol/l	
Aspartate aminotransferase	20060322	Anstieg/increased	192 3umol	
Alanine aminotransferase	20060322	Anstieg/increased	185 3umol	
Glutamate dehydrogenase	20060322	Anstieg/increased	109 3umol/	
Gamma GT	20060322	Anstieg/increased	87 3umol	
Iron	20060322	Anstieg/increased	46.4 umol/l	
Serum ferritin	20060322	Anstieg/increased	31.7 ug/l	
Transferrin saturation	20060322	Anstieg/increased	72 %	
Aspartate aminotransferase	20060328	verbessert/improved	169 3umol	
Glutamate dehydrogenase	20060328	verbessert/improved	50 3umol/	
Gamma GT	20060328	verbessert/improved	80 3umol	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07083159

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19320208	Age: 73 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	20051231	20060206	38 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Katadolon flupirtine maleate	Oral	150 Mg milligram(s)/Day	20051004-20051225	Pain 83 Day
2 Tavanic 500 mg Tabletten levofloxacin	Oral	500 Mg milligram(s)/Day	20051217-20051221	Bronchitis 5 Day
3 Voltaren 50 diclofenac sodium	Oral	50 Mg milligram(s)/Day	20051005-20051225	Pain 82 Day

Drugs (concomitant)

4 Novalgin Tropfen metamizole	Oral	1500 Mg milligram(s)/Day	20050930-20060116	Pain 3 Month
5 tramador tramadol hydrochloride	Oral	50 Mg milligram(s)/Day	20050930-20051225	Pain 87 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Yes	
Arthrosis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGOT	20060117	21.94	umol/s.l	
SGPT	20060117	22.16	umol/s.l	
Gamma GT	20060117	6.34	umol/s.l	
Bilirubin total	20060117	179.7	umol/s.l	

case: DE-BFARM-07084659

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19630530 Age: 43 Year weight (kg): 76 BMI int: 30.44 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20060807				not recovered/not resolved
2 Hypoglycaemia	20060807	20060807			recovered/resolved
3 Renal failure acute	20060807				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Lorano loratadine	Oral	10 Mg milligram(s)/Day	-20060807	Rhinitis
2 Pantozol 40mg pantoprazole sodium	Oral	40 Mg milligram(s)/2 Day	20060729-	Prophylaxis
3 Tavanic 500 mg Tabletten levofloxacin	Oral	500 Mg milligram(s)/Day	20060729-20060807 10 Day	Angina tonsillaris
4 Voltaren Dispers diclofenac sodium	Oral	150 Mg milligram(s)/Day	20060729-20060807 10 Day	Arthritis reactive

Drugs (concomitant)

5	Berlthyrox levothyroxine sodium	Oral	25 µg microgram(s)/Day	20060729- -	Hashimoto's thyroiditis
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthritis infective	20060725	20060807	No	
Angina tonsillaris	20060725	20060803	No	
Thyroiditis NOS			Yes	
Rhinitis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatine	20060807	337	umol/l	
SGPT	20060807	24.82	umol/s.l	
SGOT	20060807	63.96	umol/s.l	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07076472

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19290817	Age: 77 Year	weight (kg): 61	BMI_int: 26.06	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GGT increased					recovered/resolved
2 Hepatitis acute	20070509				recovered/resolved
3 Alkaline phosphatase increased					recovered/resolved
4 Transaminases increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac	Oral	150 Mg milligram(s)/	1997-20070509	Arthrosis
diclofenac sodium			-	
2 diclofenac	Oral	50 Mg milligram(s)/Day	200704-20070509	Arthrosis
diclofenac			-	

Drugs (concomitant)

3 acemetacin	Oral	60 Mg milligram(s)/Day	2002-	
acemetacin			-	
4 centrum	Oral	/	-	
minerals nos; vitamins nos			-	
5 Enalagamma HCT 10mg/25mg Tabletten	Oral	0.5 Df dosage form/Day	200703-20070517	Hypertension
enalapril maleate; hydrochlorothiazide			-	
6 lansoprazol	Oral	60 Mg milligram(s)/Day	20070509-	
lansoprazole			-	
7 simvahexal	Oral	/	1997-20070516	
simvastatin			-	
8 timolol-pos0.5%	Ophthalmic	/	2004-	
timolol maleate			-	
9 vasomotal	Oral	32 Mg milligram(s)/Day	200507-	
betahistine hydrochloride			-	
10 vertigo-vomex s suppositorien	Rectal	/Day	-	
dimenhydrinate			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Radiation therapy			Unknown	knee radiation before surgery
Lipid metabolism disorder			Unknown	
Hypersensitivity			Yes	Allergy for patches and diclofenac
Allergy multiple			Unknown	Pflaster, Diclofenac
Knee prosthesis insertion	1995	1995	No	
Degeneration of intervertebral disc, site unspecified			Unknown	
Degenerative joint disease			Unknown	
Meniere's disease			Yes	
Reflux oesophagitis			Yes	
Osteoporosis			Yes	
Hypertension arterial			Unknown	
Goiter nodular			Yes	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 06.09.2007 DE-SZ09-GXKR2007DE07342

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 30 Aug 2007 from the German health authority (BfArM case number: DE-BFARM-07076472), reported by a physician:

This patient with a medical history of knee joint prosthesis in 1995 with radiation, Meniere's disease, peptic esophagitis, osteoporosis, nodular goiter, hypertension, lipid metabolism disorder, and allergy for patches and diclofenac developed acute hepatitis with increased transaminases, GGT, and alkaline phosphatase during treatment with Diclac (diclofenac) and diclofenac of an unknown MAH for arthrosis. Diclofenac had been taken at a dosage of 150 mg twice monthly since 1997 and on a regular basis from Apr 2007 until 09 May 2007 at a dosage of 50 mg daily. Concomitantly the patient received several other drugs. The adverse event was observed on 09 May 2007 when transaminases, GGT, and alkaline phosphatase were highly increased. Furthermore, hepatitis B antigen and antibodies were tested positive. Diclofenac was discontinued and the laboratory parameters were already decreasing on the next day. The condition resolved completely. The reporter assessed the causality of the adverse event and administration of diclofenac as probable.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07076814

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials: Sex: Female Birth: 19320307 Age: 73 Year weight (kg): 65 BMI_int: 24.46 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 CRP increased	20060719				recovering/resolving
2 Haemoglobin decreased	20060719				recovering/resolving
3 Haptoglobin decreased	20060719				recovering/resolving
4 Hyperbilirubinemia	20060720				recovering/resolving
5 Icterus	2006				recovering/resolving
6 LDH increased	20060719				recovering/resolving
7 Reticulocytosis	20060720				recovering/resolving
8 Splenomegaly	20060720				recovering/resolving
9 Erythrocytes decreased	20060720				recovering/resolving
10 Transaminases increased	20060719				recovering/resolving
11 Coombs test positive	20060803				recovering/resolving
12 Antibody test positive	2006				recovering/resolving
13 Urine colour abnormal	200606				recovering/resolving
14 Anemia hemolytic autoimmune (NOS)	20070607				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL diclofenac sodium	Topical	/Day 95	1997-20060804 -	Joint pain

Drugs (concomitant)					
2	BLOPRESS candesartan cilexetil	Oral	0.5 Df dosage form/Day	1992-20060908	Hypertension
3	ciprobeta ciprofloxacin hydrochloride	Oral	500 Mg milligram(s)/Day	20060610-20060619	Helicobacter infection
4	DICLOFENAC diclofenac	Unknown	2*1 Df dosage form/Week 245	1997-200510	
5	DIGITOXIN digitoxin	Oral	1 Df dosage form/Day	1978-2006	Helicobacter gastritis
6	DIGITOXIN digitoxin	Oral	500 Mg milligram(s)/	20060610-20060619	Helicobacter gastritis
7	ENDOXAN cyclophosphamide	Unknown	1 Df dosage form/Day	-	
8	falithrom phenprocoumon	Oral	/	200507-20060704	
9	IBUPROFEN ibuprofen	Unknown	1 Df dosage form/Week 245	200510-200604	
10	Influenza Vaccine influenza vaccine		1 Df dosage form/Day 120	200510-200510	
11	LEGALON silybum marianum	Unknown	1 Df dosage form/Day 5	20060704-20060719	
12	NEXIUM esomeprazole magnesium	Unknown	1 Df dosage form/Day	-	
13	PHENPROCUMON phenprocoumon	Unknown	/	200507-20060704	
14	prednisolon prednisolone	Unknown	1 Df dosage form/	-	
15	SICCAPOS GEL carbomer	Unknown	/	20060908-20060908	
16	Vitamin B calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Oral	1 Df dosage form/Day 245	200503-20060908	Trigeminal neuralgia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pulmonary embolism			Yes	
Helicobacter gastritis			Yes	
Varicose veins NOS			Yes	
Hypertension			Yes	
Hip arthroplasty			Yes	
Trigeminal neuralgia	2005		Yes	
Hip prosthesis insertion			Unknown	
Deep vein thrombosis	2005		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 31.08.2007 DE-002147023-PHNU2007DE02979

Sender diagnosis: not reported

Narrative

Initial physician report received on 28 Aug 2007 via German Health Authorities: This patient used Voltaren gel (diclofenac) 1-2 application/week from 1997 to 04 Aug 2006 due to joint pain and developed life threatening anemia haemolytic autoimmune with decrease of Hb 19 Jul 2006 7.7 g/dl,

(baseline 10.3 g/dl on 07 Jun 2006) 20 Jul 2006 7.3 g/dl, 07 Aug 2006 8.8 g/dl. Erythrocytes decreased 20 Jul 2006 1.86 Tpt/l (normal value 4.1-5.1). Reticulocytosis was 146 (normal value 2-20) Leucocytosis was measured 6.6 Gpt/l (normal value 4.4-11.3). Haptoglobin decreased <0.0 g/l (normal value 0.3-2.0). LDH increase 19 Jul 2006 1046 U/l (normal value <215). Hyperbilirubinemia was measured on 20 Jul 2006 73.8 Åµmol/l. Direct coombs test positive IgA on 03 Aug 2006. Diclofenac antibody test positive. After discontinuation of diclofenac gel icterus, hemolysis parameters and reticulocytes decreased, Hb increased. Condition improved. Patient was released from hospital. Ambulatory controls were planned. The relationship to Voltaren gel was definite.

Reporter's comment:

NOVARTIS, 03.03.2010 DE-002147023-PHNU2007DE02979

Sender diagnosis: not reported

Narrative

Initial physician report received on 28 Aug 2007 via German Health Authorities: This patient used Voltaren gel (diclofenac) 1-2 application/week from 1997 to 04 Aug 2006 due to joint pain and developed life threatening anemia haemolytic autoimmune with decrease of Hb 19 Jul 2006 7.7 g/dl, (baseline 10.3 g/dl on 07 Jun 2006) 20 Jul 2006 7.3 g/dl, 07 Aug 2006 8.8 g/dl. Erythrocytes decreased 20 Jul 2006 1.86 Tpt/l (normal value 4.1-5.1). Reticulocytosis was 146 (normal value 2-20) Leucocytosis was measured 6.6 Gpt/l (normal value 4.4-11.3). Haptoglobin decreased <0.0 g/l (normal value 0.3-2.0). LDH increase 19 Jul 2006 1046 U/l (normal value <215). Hyperbilirubinemia was measured on 20 Jul 2006 73.8 Åµmol/l. Direct coombs test positive IgA on 03 Aug 2006. Diclofenac antibody test positive. After discontinuation of diclofenac gel icterus, hemolysis parameters and reticulocytes decreased, Hb increased. Condition improved. Patient was released from hospital. Ambulatory controls were planned. The relationship to Voltaren gel was definite.

Following internal review done on 01 Mar 2010 of the report received on 28 Aug 2007. Significant case correction done.

NOVARTIS, 03.03.2010 DE-002147023-PHNU2007DE02979

Sender diagnosis: not reported

Narrative

Initial physician report received on 28 Aug 2007 via German Health Authorities: This patient used Voltaren gel (diclofenac) 1-2 application/week from 1997 to 04 Aug 2006 due to joint pain and developed life threatening anemia haemolytic autoimmune with decrease of Hb 19 Jul 2006 7.7 g/dl, (baseline 10.3 g/dl on 07 Jun 2006) 20 Jul 2006 7.3 g/dl, 07 Aug 2006 8.8 g/dl. Erythrocytes decreased 20 Jul 2006 1.86 Tpt/l (normal value 4.1-5.1). Reticulocytosis was 146 (normal value 2-20) Leucocytosis was measured 6.6 Gpt/l (normal value 4.4-11.3). Haptoglobin decreased <0.0 g/l (normal value 0.3-2.0). LDH increase 19 Jul 2006 1046 U/l (normal value <215). Hyperbilirubinemia was measured on 20 Jul 2006 73.8 Åµmol/l. Direct coombs test positive IgA on 03 Aug 2006. Diclofenac antibody test positive. After discontinuation of diclofenac gel icterus, hemolysis parameters and reticulocytes decreased, Hb increased. Condition improved. Patient was released from hospital. Ambulatory controls were planned. The relationship to Voltaren gel was definite.

Following internal review done on 01 Mar 2010 of the report received on 28 Aug 2007. Significant case correction done.

NOVARTIS, 03.03.2010 DE-NOVNCHHQ-PHNU2007DE02979

Sender diagnosis: not reported

Narrative

Initial physician report received on 28 Aug 2007 via German Health Authorities: This patient used Voltaren gel (diclofenac) 1-2 application/week from 1997 to 04 Aug 2006 due to joint pain and developed life threatening anemia haemolytic autoimmune with decrease of Hb 19 Jul 2006 7.7 g/dl, (baseline 10.3 g/dl on 07 Jun 2006) 20 Jul 2006 7.3 g/dl, 07 Aug 2006 8.8 g/dl. Erythrocytes decreased 20 Jul 2006 1.86 Tpt/l (normal value 4.1-5.1).

Reticulocytosis was 146 (normal value 2-20) Leucocytosis was measured 6.6 Gpt/l (normal value 4.4-11.3). Haptoglobin decreased <0.0 g/l (normal value 0.3-2.0). LDH increase 19 Jul 2006 1046 U/l (normal value <215). Hyperbilirubinemia was measured on 20 Jul 2006 73.8 $\mu\text{mol/l}$. Direct coombs test positive IgA on 03 Aug 2006. Diclofenac antibody test positive. After discontinuation of diclofenac gel icterus, hemolysis parameters and reticulocytes decreased, Hb increased. Condition improved. Patient was released from hospital. Ambulatory controls were planned. The relationship to Voltaren gel was definite.

Following internal review done on 01 Mar 2010 of the report received on 28 Aug 2007. Significant case correction done.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07076815

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: 19270829 Age: 79 Year weight (kg): 58 BMI_int: 20.8 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased	20070502			20 Month	Unknown
2 Hyperbilirubinaemia	20070429			20 Month	Unknown
3 Haptoglobin decreased	20070502			20 Month	Unknown
4 Haemoglobin urine positive	20070429			20 Month	Unknown
5 Coombs test positive	20070429			20 Month	Unknown
6 Immune haemolytic anaemia drug-induced	20070409			19 Month	Unknown
7 Leukocytosis	20070429			20 Month	Unknown
8 Erythrocytes decreased	20070429			20 Month	Unknown
9 Haemoglobin decreased	20070409			19 Month	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	75 Mg milligram(s)/Day	20050823-20070429	Pain 20 Month

Drugs (concomitant)

2 falithrom phenprocoumon	Oral	/	200503-20070429	-
3 l-thyroxin levothyroxine sodium	Oral	25 µg microgram(s)/Day	1997-20070429	-
4 merimono 1mg estradiol valerate	Oral	1 Mg milligram(s)/Day	-20070429	-
5 metoprolol metoprolol	Oral	0.5 Df dosage form/Day	2005-20070429	Hypertension arterial
6 provas valsartan	Oral	80 Mg milligram(s)/Day	20030716-20070429	Hypertension arterial 3 Year
7 simagel-tabletten almasilate	Oral	/	-20070429	-
8 vitamin b complex calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Oral	/	-20070429	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Smoker			Unknown	
Paresis			Unknown	Tetraparesis
Ex-smoker	1998		Unknown	
Renal insufficiency			Unknown	praerenal
Hyperthyroidism			Unknown	latent
Goiter nodular			Unknown	
Pulmonary empyema			Unknown	
Tetraplegia			Unknown	incomplet
Atrial fibrillation			Unknown	chronic
Microangiopathy			Unknown	
Single vessel disease			Unknown	
Lacunar infarction			Unknown	old

Primary source

Qualification: Other Health Professional observestudytype:

Literature:

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 30.08.2007 DE-002147023-PHNU2007DE02970

Sender diagnosis: not reported

Narrative

Initial physician report received on 28 Aug 2007 via german health authorities. This patient used Voltaren Resinat (diclofenac, colestyramin) 75 mg from 23 Aug 2005 to 29 April 2007 due to pain and developed life threatening anemia haemolytic autoimmune with decrease of Hb 09 April 2007 7.4 mmol/l, (baseline 12.0 mmol/l on 21 Mar 2007) 29 April 2007 5.5 mmol/l, 07 May 2007 6.5 mmol/l, 14 May 2007 7.6 mmol/l. Hb was found additionally in urine. Erythrocytes decreased 29 Mar 2007 2.83 Tpt/l (normal value 4.0-5.4). Leucocytosis was measured 29 Apr 2007 14.0 Gpt/l (normal value 3.8-12.0). Haptoglobin decreased <7.5 mg/dl (normal value 36-195). Small LDH increase 02May 2007 4.88 Åµkat/l (normal value <4.12). Hyperbiliruinemia was measured on 29 Apr 2007 25.4 Åµmol/l. Direct coombs test positive on 29 Apr 2007. Under therapy with prednisolon Hb value increased again. Clinical outcome not known. The relationship to Voltaren was assessed as possible.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07089231

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19661023	Age: 40 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia	20070917				recovering/resolving
2 LDH increased	20070917				recovering/resolving
3 Gamma-GT increased	20070917				recovering/resolving
4 GOT increased	20070917				recovering/resolving
5 GPT increased	20070917				recovering/resolving
6 Transaminases increased	20070917				recovering/resolving
7 Anemia hemolytic	20070917				recovering/resolving
8 Iron deficiency anemia					Unknown
9 Haemoglobin decreased	20070917				recovering/resolving
10 Haematocrit decreased	20070917				recovering/resolving
11 Erythrocytes abnormal	20070917				Unknown
12 Erythrocytes decreased	20070917				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren		/Day	-	
diclofenac sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Viral infection			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.09.2007 DE-002147023-PHNU2007DE03197

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 19 Sep 2007. Patient has been treated with Voltaren (diclofenac) only once and developed suspicion of hemolytic anemia with decrease of erythrocytes $3.71 \times 10^{12} / \mu\text{l}$ (normal range 4.1-5.2), Hb decrease 9.3 g/dl (normal range 12.0-16.0), Hematocrit decrease 30.5 % (normal range 36-45 %) and further erythrocytes abnormalities. Additionally transaminases were increased: GPT 332 U/l (normal range <34), GOT 400 U/l (normal range <31). Also the following parameters were abnormal: LDH increase 507 U/l (normal range <247), Gamma GT increase 131 U/l (normal range <38), leucopenia $3.34 \times 10^9 / \mu\text{l}$ (normal range 4.4-11.3). Pharmacist asked Novartis about discontinuation of Voltaren. Serious spontaneous report (medically significant) hemolytic anemia, transaminases increased and leucopenia already listed in the Basic Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 15.10.2007 DE-002147023-PHNU2007DE03197

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 19 Sep 2007. Patient has been treated with Voltaren (diclofenac) only once and developed suspicion of hemolytic anemia with decrease of erythrocytes $3.71 \times 10^{12} / \mu\text{l}$ (normal range 4.1-5.2), Hb decrease 9.3 g/dl (normal range 12.0-16.0), Hematocrit decrease 30.5 % (normal range 36-45 %) and further erythrocytes abnormalities. Additionally transaminases were increased: GPT 332 U/l (normal range <34), GOT 400 U/l (normal range <31). Also the following parameters were abnormal: LDH increase 507 U/l (normal range <247), Gamma GT increase 131 U/l (normal range <38), leucopenia $3.34 \times 10^9 / \mu\text{l}$ (normal range 4.4-11.3). Pharmacist asked Novartis about discontinuation of Voltaren.

Follow up report received on 05 Oct 2007. Physician reported that he had no further information available and mentioned that the suspicious haemolytic anemia could be a iron deficiency anemia, too. A viral infection as other possible cause for the symptoms was mentioned. The reporter ticked "no" concerning the relationship between the symptoms and Voltaren.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07098864

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19571024	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	200605				Unknown
2 Hepatitis toxic	200605				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ATORVASTATIN		/	-	
atorvastatin			-	
2 voltaren dispers	Oral	100 Mg milligram(s)/ Dispersible tablet	-	
diclofenac sodium			10080 Minute	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 17.10.2007 DE-002147023-PHNU2007DE03435

Sender diagnosis: not reported

Narrative

Initial report received on 12 Oct 2007. Preliminary information from physician. This patient was treated with Voltaren Dispers (diclofenac) for one week, due to unstated reasons. She showed increased transaminases and the diagnosis of toxic hepatitis was made. Event onset date was reported as May 2006. No outcome information was given.

Sender's comment:

Serious spontaneous report [medically significant] 'hepatitis toxic' already listed in the Basic Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:NOVARTIS, 22.01.2008 DE-002147023-PHNU2007DE03435

Sender diagnosis: not reported

Narrative

Initial report received on 12 Oct 2007. Preliminary information from physician. This patient was treated with Voltaren Dispers (diclofenac) for one week, due to unstated reasons. She showed increased transaminases and the diagnosis of toxic hepatitis was made. Event onset date was reported as May 2006. No outcome information was given.

Follow up received on 16 Jan 2008. The physician reported that the reported case of toxic hepatitis due to Voltaren Dispers was an error. The drug reaction was definitely only related to Atorvastatin and not Voltaren Dispers.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07116004

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19451207	Age: 61 Year	weight (kg): 63	BMI_int: 22.32	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20070927			7 Day	recovered/resolved
2 Tiredness	20070926		14 Day	6 Day	recovered/resolved
3 GPT increased					Unknown
4 GOT increased					Unknown
5 Cholesterol blood increased	20070927			7 Day	recovered/resolved
6 Alpha 2 globulin decreased					Unknown
7 LDH increased	20070927			7 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/ Capsule, hard	20070922-20070922 1440 Minute	Meniscus operation
2 voltaren diclofenac sodium	Oral	/ Capsule, hard	20070921-20070921 1440 Minute	Meniscus operation
3 voltaren diclofenac sodium	Oral	/ Capsule, hard	20070923-20070923 1440 Minute	Meniscus operation

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 03.12.2007 DE-002147023-PHNU2007DE03290

Sender diagnosis: not reported

Narrative

Initial report received on 27 Sep 2007. Physician reported about patient who was treated with Voltaren (diclofenac) "capsules" (but reportedly not Voltaren Resinat) after a meniscus operation. The patient complained about tiredness, and the drug was stopped. Laboratory showed an increase of transaminases (GOT three times increased, GPT 111U/l) and of lactate dehydrogenase (255U/l, normal range <214U/l). Alpha-2-globulin was slightly decreased. All other laboratory, including CK (creatin phosphokinase), gamma GT and hepatitis serology, were normal. It was added that patient had had normal laboratory values before operation / before therapy initiation.

Follow up received on 28 Nov 2007 (written documentation from physician). Further patient details were given. The patient suffered from significant tiredness since 26 Sep 2007; this event lasted for about 14 days. Also, on 27 Sep 2007, the following lab abnormalities were recorded: GOT 97U/l, GPT 111U/l, LDH 252U/l, cholesterol 284mg/dl (newly reported). Medication was stopped on 23 Sep 2007. The patient fully recovered after withdrawal (also lab was normal by 8 Oct 2007). The reporter considered the drug as probably related, and other causations were excluded.

All other events were assessed as non-serious.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07126387

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic function abnormal	20050113	20050214	33 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 remergil mirtazapine	Unknown	45 Mg milligram(s)/Day	20050112-20050220 40 Day	
2 voltaren resinat diclofenac resinate	Unknown	150 Mg milligram(s)/Day	20050107-20050112 6 Day	

Drugs (concomitant)

3	cipralex escitalopram oxalate	Unknown	25 Mg milligram(s)/Day	20050214-20050223 10 Day
4	cipralex escitalopram oxalate	Unknown	Mg milligram(s)/Day	20050301- -
5	cipralex escitalopram oxalate	Unknown	15 Mg milligram(s)/Day	20050202-20050207 6 Day
6	cipralex escitalopram oxalate	Unknown	30 Mg milligram(s)/Day	20050224-20050228 5 Day
7	cipralex escitalopram oxalate	Unknown	10 Mg milligram(s)/Day	20050121-20050201 12 Day
8	cipralex escitalopram oxalate	Unknown	20 Mg milligram(s)/Day	20050208-20050213 6 Day
9	remergil mirtazapine	Unknown	30 Mg milligram(s)/Day	20050116- -
10	saroten retard amitriptyline hydrochloride	Unknown	50 Mg milligram(s)/Day	20050214- -
11	saroten retard amitriptyline hydrochloride	Unknown	25 Mg milligram(s)/Day	20050202-20050207 6 Day
12	saroten retard amitriptyline hydrochloride	Unknown	50 Mg milligram(s)/Day	20050208-20050213 6 Day
13	solian amisulpride	Unknown	200 Mg milligram(s)/Day	20050324-20050325 2 Day
14	solian amisulpride	Unknown	100 Mg milligram(s)/Day	20050326-20050327 2 Day
15	solian amisulpride	Unknown	300 Mg milligram(s)/Day	20050201-20050323 51 Day
16	solian amisulpride	Unknown	400 Mg milligram(s)/Day	20050121-20050131 11 Day
17	tavor lorazepam	Unknown	0.5 Mg milligram(s)/Day	20050221-20050223 3 Day
18	tavor lorazepam	Unknown	1 Mg milligram(s)/Day	20050107-20050107 -
19	tavor lorazepam	Unknown	0.25 Mg milligram(s)/Day	20050224-20050228 5 Day
20	tavor lorazepam	Unknown	0.75 Mg milligram(s)/Day	20050216-20050220 5 Day
21	tavor lorazepam	Unknown	1.0 Mg milligram(s)/Day	20050214-20050215 2 Day
22	tavor lorazepam	Unknown	1.5 Mg milligram(s)/Day	20050202-20050203 2 Day
23	tavor lorazepam	Unknown	1.5 Mg milligram(s)/Day	20050108-20050111 4 Day
24	tavor lorazepam	Unknown	2 Mg milligram(s)/Day	20050112-20050113 2 Day
25	tavor lorazepam	Unknown	3 Mg milligram(s)/Day	20050114-20050114 -
26	tavor lorazepam	Unknown	4 Mg milligram(s)/Day	20050115-20050119 5 Day
27	tavor lorazepam	Unknown	2.5 Mg milligram(s)/Day	20050120-20050120 -
28	tavor lorazepam	Unknown	3.5 Mg milligram(s)/Day	20050121-20050123 3 Day
29	tavor lorazepam	Unknown	3.0 Mg milligram(s)/Day	20050124-20050124 -
30	tavor lorazepam	Unknown	2.5 Mg milligram(s)/Day	20050125-20050127

Drugs (concomitant)

31	tavor lorazepam	Unknown	2.0 Mg milligram(s)/Day	3 Day 20050128-20050201
32	tilidalor loesung naloxone hydrochloride; tilidine hydrochloride	Unknown	30 Gtt drop(s)/Day	5 Day 20050119-20050119
33	tilidalor loesung naloxone hydrochloride; tilidine hydrochloride	Unknown	30 Gtt drop(s)/Day	- 20050128-20050131
34	tilidalor loesung naloxone hydrochloride; tilidine hydrochloride	Unknown	15 Gtt drop(s)/Day	4 Day 20050201-
35	tilidalor loesung naloxone hydrochloride; tilidine hydrochloride	Unknown	45 Gtt drop(s)/Day	- 20050120-20050127
36	voltaren diclofenac sodium	Unknown	75 Mg milligram(s)/Day	8 Day 20050106-
37	voltaren resinat diclofenac resinate	Unknown	150 Mg milligram(s)/Day	- 20050201-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Depressive disorder			Unknown	rezidevirend, gegenwärtig schwere Episode ohne psychotische Symptome
Coxarthrosis			Unknown	sonstige posttraumatische Koxarthrose
Hepatic cirrhosis			Unknown	vor Jahren rez. erhöhter C2-Konsum-im Abdomen Sono beg. kleinknotige Leberzirrhose

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Lab test		vorhanden/present	NA	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08003819

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	2007				Unknown
2 GPT increased	2007				Unknown
3 Liver function tests abnormal	2007				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Unknown	/ Dispersible tablet	2007- -	Pain

Drugs (concomitant)

2 ENDOXAN cyclophosphamide	Unknown	/	-	-
3 prednisone prednisone	Unknown	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Vasculitis			Unknown	autoimmune vasculitis
Leg amputation			Unknown	forefoot amputation due to necroses
Liver function tests abnormal			Unknown	liver function test increase some time ago

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 04.01.2008 DE-002147023-PHNU2007DE04282

Sender diagnosis: not reported

Narrative

Initial report received on 27 Dec 2007. Physician reported about patient who was treated with Voltaren Dispers (diclofenac) for about 3 to 4 months, due to pain in context with underlying autoimmune vasculitis. The patient anamnesis included increased liver function tests due to suspected ethanol use (red wine, 1.5 bottles/day, according to report) or with suspected implication of the autoimmune disorder. Later, the patient stopped wine use, and in Jul 2007, liver function tests were normal. The patient's comedication was unchanged. Voltaren Dispers was initiated, in Aug or Sep 2007. In Nov 2007, liver function tests were found elevated (gamma GT around 300U/l, GPT around 80U/l), and just before Christmas 2007, a further increase was noticed (gamma GT 630U/l, GPT 165U/l). The patient did not show a clinical symptomatology.

are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08011729

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					Yes

Patient

Initials:	Sex: Female	Birth: 19880108	Age: 19 Year	weight (kg): 53	BMI_int: 19.23	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fulminant hepatic failure	200712				recovered/resolved
2 Hepatitis toxic drug-induced	20071221	20080109	20 Day	11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm	Oral	/	20071211-20071221	Arthritis
diclofenac sodium			11 Day	

Drugs (concomitant)

2 ibuprofen 800 mg (Tabletten)	Oral	/	-	Arthritis
ibuprofen		Tablet	4 Month	
3 Kontrazeptiva		/	-	Contraception
			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 22.02.2008 DE-RATIOPH-2008R021366

Sender diagnosis: not reported

Narrative

Reported terms: Hepatitis toxic drug induced(Hepatitis toxic)

Case reference number 2008R021366 is a spontaneous case report sent by a physician which refers to a female aged 20 years.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Diclofenac-ratiopharm (DICLOFENAC), indication Arthritis on 11-Dec-2007. The patient has been exposed to the following concomitant drug(s): Ibuprofen 800 mg (Tabletten) (IBUPROFEN), indication Arthritis and Contraceptive agents (HORMONAL CONTRACEPTIVES FOR SYSTEMIC USE), indication Contraception. The patient developed Hepatitis toxic drug induced (Hepatitis toxic)(start date 21-Dec-2007).

The following action was taken: for Diclofenac-ratiopharm (DICLOFENAC) - Drug withdrawn

The outcome for Hepatitis toxic drug induced (Hepatitis toxic) is Recovered / Resolved.

Rechallenge information for Diclofenac-ratiopharm is not applicable. Dechallenge information for Diclofenac-ratiopharm is not applicable.

The reporter's assessment for drug Diclofenac-ratiopharm related with Hepatitis toxic drug induced is as follows: Probable. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08061313

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19591109	Age: 48 Year	weight (kg): 61	BMI_int: 23.83	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GPT increased	200712				recovering/resolving
2 Haptoglobin decreased					Unknown
3 Hepatic enzymes increased	200712				recovering/resolving
4 Hepatitis	200712				recovering/resolving
5 Icterus					Unknown
6 Nausea					Unknown
7 Shivering					Unknown
8 Tiredness					Unknown
9 GGT increased	200712				recovering/resolving
10 Discolored stools					Unknown
11 Ferritin increased					Unknown
12 Bilirubin increased					recovered/resolved with sequel
13 Alkaline phosphatase increased	200712				recovering/resolving
14 Abdominal pain					recovering/resolving
15 GOT increased	200712				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 OMEPRAZOLE omeprazole	Oral	20 Mg milligram(s)/Day	20071221- -	Gastric disorder
2 VOLTAREN diclofenac sodium	Oral	75 Mg milligram(s)/Day	200710- -	Polytraumatism

Drugs (concomitant)

3 SOBELIN clindamycin hydrochloride	/	-	-	Infection
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Polytraumatism	200710		Unknown	mit rez.Frakturen
Hypothyroidism			Unknown	
Hypokalemia			Unknown	
Nicotine addiction			No	chronic

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		27.9	mg/dl	
Glutamic-oxaloacetic transferase		1439	U/l	
GPT		2526	U/l	
Gamma GT		1200	U/l	
Alkaline phosphatase		377	U/l	

Haptoglobin	< 10	mg/dl
Serum ferritin	11460	µg/l
Quick	104	%
INR	1.3	NA

Narratives (nur bei elektronischem Eingang)NOVARTIS, 01.07.2008 DE-002147023-PHHY2008DE11305

Sender diagnosis: not reported

Narrative

Initial report received via German Health Authorities on 17 June 2008. Patient had been treated with Voltaren (diclofenac sodium) due to condition after Polytrauma with fractures and necrosis and Omeprazole for stomach protection. Patient developed toxic hepatitis with liver enzyme increase GOT 1439 U/l, GPT 2526 U/l, GGT 1200 U/l, AP 377 U/l. Bilirubin was increased to 27.9 mg/dl. Ferritin increased to 11460 µg/l. Haptoglobin < 10mg/dl. Further symptoms were nausea, tiredness, shivering, icterus. Therapy was discontinued. Patient not yet fully recovered. Liver values showed improvement. The relationship was assessed as suspected for Voltaren and omeprazol.

Serious spontaneous report [medically significant] hepatitis already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:NOVARTIS, 01.07.2008 DE-002147023-PHHY2008DE11305

Sender diagnosis: not reported

Narrative

Initial report received via German Health Authorities on 17 June 2008. Patient had been treated with Voltaren (diclofenac sodium) due to condition after Polytrauma with fractures and necrosis and Omeprazole for stomach protection. Patient developed toxic hepatitis with liver enzyme increase GOT 1439 U/l, GPT 2526 U/l, GGT 1200 U/l, AP 377 U/l. Bilirubin was increased to 27.9 mg/dl. Ferritin increased to 11460 µg/l. Haptoglobin < 10mg/dl. Further symptoms were nausea, tiredness, shivering, icterus. Therapy was discontinued. Patient not yet fully recovered. Liver values showed improvement. The relationship was assessed as suspected for Voltaren and omeprazol.

Serious spontaneous report [medically significant] hepatitis already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:NOVARTIS, 01.07.2008 DE-002147023-PHHY2008DE11305

Sender diagnosis: not reported

Narrative

Initial report received via German Health Authorities on 17 June 2008. Patient had been treated with Voltaren (diclofenac sodium) due to condition after Polytrauma with fractures and necrosis and Omeprazole for stomach protection. Patient developed toxic hepatitis with liver enzyme increase GOT 1439 U/l, GPT 2526 U/l, GGT 1200 U/l, AP 377 U/l. Bilirubin was increased to 27.9 mg/dl. Ferritin increased to 11460 µg/l. Haptoglobin < 10mg/dl. Further symptoms were nausea, tiredness, shivering, icterus. Therapy was discontinued. Patient not yet fully recovered. Liver values showed improvement. The relationship was assessed as suspected for Voltaren and omeprazol.

Serious spontaneous report [medically significant] hepatitis already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 01.07.2008 DE-002147023-PHHY2008DE11305

Sender diagnosis: not reported

Narrative

Initial report received via German Health Authorities on 17 June 2008. Patient had been treated with Voltaren (diclofenac sodium) due to condition after Polytrauma with fractures and necrosis and Omeprazole for stomach protection. Patient developed toxic hepatitis with liver enzyme increase GOT 1439 U/l, GPT 2526 U/l, GGT 1200 U/l, AP 377 U/l. Bilirubin was increased to 27.9 mg/dl. Ferritin increased to 11460 µg/l. Haptoglobin < 10mg/dl. Further symptoms were nausea, tiredness, shivering, icterus. Therapy was discontinued. Patient not yet fully recovered. Liver values showed improvement. The relationship was assessed as suspected for Voltaren and omeprazol.

Serious spontaneous report (medically significant) hepatitis already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08094581

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19611225	Age: 46 Year	weight (kg): 80	BMI_int: 27.68	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug interaction	20080421	20080525	35 Day	178 Day	recovered/resolved
2 Ammonia increased	20080525	20080528	4 Day	212 Day	recovered/resolved
3 Hepatic encephalopathy	20080525	20080528	4 Day	212 Day	recovered/resolved
4 Liver function tests raised	200804	20080528	4 Day	212 Day	recovered/resolved
5 Hepatitis toxic	20080525	20080528	4 Day	212 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	100 Mg milligram(s)/Day	20080421-20080524	Arthropathy
diclofenac		Capsule	34 Day	
2 EXJADE	Oral	1000 Mg milligram(s)/Day	20071027-20080411	Ferritin high
deferasirox			6 Month	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute lymphocytic leukemia			Unknown	Acute lymphocytic leukemia in "maintenance therapy"
Arthropathy			Unknown	
Arthritis			Unknown	Oligoarthritis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 07.08.2008 DE-002147023-PHHY2008DE15676

Sender diagnosis: not reported

Narrative

Initial physician report received on 27 Jul 2008. This patient with oligoarthritis and arthropathy started treatment with Exjade (deferasirox), on 27 Oct 2007 at a dose of 1000 mg daily, 500 mg BID for elevation of ferritin. Exjade was temporarily discontinued in the further treatment course. Concomitant medication included diclofenac. Patient's malignant underlying disease was an ALL in stage of "maintenance therapy". The patient experienced "progressive elevation of liver values" accompanied by "clinical hepatic encephalopathy". In addition, the reporter described the events as "toxic hepatitis" with elevated liver values and increase of ammonium. Both diclofenac and Exjade were discontinued on 24 May 2008. All events lasted for 3 days only. The reporter suspected a causal relationship to Exjade but not to diclofenac. The outcome was indicated as recovery.

Serious spontaneous report clinical hepatic encephalopathy, ammonia increased [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

The following term toxic hepatitis [medically significant], already included in the Core Data Sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 02.10.2008 DE-002147023-PHHY2008DE15676

Sender diagnosis: not reported

Narrative

Initial physician report received on 27 Jul 2008. This patient with oligoarthritis and arthropathy started treatment with Exjade (deferasirox), on 27 Oct 2007 at a dose of 1000 mg daily, 500 mg BID for elevation of ferritin. Exjade was temporarily discontinued in the further treatment course. Concomitant medication included diclofenac. Patient's malignant underlying disease was an ALL in stage of "maintenance therapy". The patient experienced "progressive elevation of liver values" accompanied by "clinical hepatic encephalopathy". In addition, the reporter described the events as "toxic hepatitis" with elevated liver values and increase of ammonium. Both diclofenac and Exjade were discontinued on 24 May 2008. All events lasted for 3 days only. The reporter suspected a causal relationship to Exjade but not to diclofenac. The outcome was indicated as recovery.

Follow up report received on 24 Sep 2008 from a physician: The physician documented an interruption of Exjade treatment (unspecified date). The hepatic encephalopathy recovered completely approximately after 14 days. The elevation of liver values occurred already in Apr 2008 and recovered completely after 6 weeks. The physician mentioned a possible drug interaction between diclofenac and Exjade as an alternative cause for the events. Toxic hepatitis [medically significant], already included in the Core Data Sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 12.05.2009 DE-002147023-PHHY2008DE15676

Sender diagnosis: not reported

Narrative

Initial physician report received on 27 Jul 2008. This patient with oligoarthritis and arthropathy started treatment with Exjade (deferasirox), on 27 Oct 2007 at a dose of 1000 mg daily, 500 mg BID for elevation of ferritin. Exjade was temporarily discontinued in the further treatment course. Concomitant medication included diclofenac. Patient's malignant underlying disease was an ALL in stage of "maintenance therapy". The patient experienced "progressive elevation of liver values" accompanied by "clinical hepatic encephalopathy". In addition, the reporter described the events as "toxic hepatitis" with elevated liver values and increase of ammonium. Both diclofenac and Exjade were discontinued on 24 May 2008. All events lasted for 3 days only. The reporter suspected a causal relationship to Exjade but not to diclofenac. The outcome was indicated as recovery.

Follow up report received on 24 Sep 2008 from a physician: The physician documented an interruption of Exjade treatment (unspecified date). The hepatic encephalopathy recovered completely approximately after 14 days. The elevation of liver values occurred already in Apr 2008 and recovered completely after 6 weeks. The physician mentioned a possible drug interaction between diclofenac and Exjade as an alternative cause for the events.

Follow-up information (written follow-up report; response on a request in form of answers to specific questions concerning treatment periods of the drugs Exjade and diclofenac, the possible mechanism of interaction and the causal relationship to both drugs) received from the treating physician on 01 May 2009. The patient was treated with Exjade (1000 mg daily) from 27 Oct 2007 to 11 Apr 2008. In this period Exjade was disrupted only short-time when the patient received the "maintenance therapy of leukaemia". Those short-term discontinuation phases were described as retrospectively not reconstructable. Exjade (1000 mg daily) was administered without any disruptions from 12 Apr 2008 to 24 May 2008. The patient did not take diclofenac prior to 21 Apr 2008. The treating physician stated that "due to the potential hepatotoxic effect of diclofenac" the mechanism of interaction could have been a "potentiation of the hepatotoxic effect of Exjade". The physician finally assessed that the "hepatotoxicity was more probably or at least primarily caused by Exjade".

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08091779

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19671229	Age: 40 Year	weight (kg): 106	BMI_int: 32.72	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hemoglobin decreased					Unknown
2 Bilirubin increased					Unknown
3 Autoimmune hemolytic anemia	200805				not recovered/not resolved
4 LDH increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	2 Df dosage form/Day	20080529-20080605	Pain in jaw
diclofenac			8 Day	

Drugs (concomitant)

2 Voltaren	Oral	3*1 Df dosage form/Week	20080529-20080605	
diclofenac sodium			8 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		11.2	mg/dl	
LDH		402	U/l	
Hb		7.8	mg/dl	
Autoantibody		positiv/positive	NA	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.08.2008 DE-002147023-PHHY2008DE17887

Sender diagnosis: not reported

Narrative

Initial report received from the German Health Authority (HA Ref DE-BFARM-08091779) on 12 Aug 2008. The patient with a medical history of arterial hypertension was on therapy with diclofenac (manufacturer unknown) for pain in jaw from 29 May 2008 to 05 Jun 2008. Oral Voltaren (3 dosage form in a week) was reported as concomitant medication. The patient was hospitalized for autoimmune hemolytic anaemia following therapy in May 2008. Laboratory examination done on an unknown date showed bilirubin 11.2 mg/dl, LDH 402 U/L, hemoglobin 7.8 mg/dl, and positive test for Autoantibody. Outcome was reported as condition unchanged. The causality was reported as possible.

Serious spontaneous report Autoimmune hemolytic anaemia [hospitalization], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

CT-Arzneimittel GmbH, 01.09.2008 DE-CT-2008CT086408

Sender diagnosis: not reported

Narrative

Reported terms: Autoimmune hemolytic anemia(Anaemia haemolytic autoimmune)

Case reference number 2008CT086408 is a spontaneous case report sent by a physician via AKdÄ, which refers to a male aged 40 years. The patient's past medical history is unknown. There is no information about risk factors. Concomitant diseases include Hypertension arterial (Hypertension).

The patient started taking Diclofenac (DICLOFENAC), indication Pain in jaw on 29-May-2008. The patient has been exposed to the following concomitant drug(s): Voltaren (DICLOFENAC), indication unknown. The patient developed Autoimmune hemolytic anemia (Anaemia haemolytic autoimmune)(start date ??-May-2008). The patient was hospitalized.

The following action was taken: for Diclofenac (DICLOFENAC) - Drug withdrawn

The outcome for Autoimmune hemolytic anemia (Anaemia haemolytic autoimmune) is Not Recovered / Not Resolved.

Rechallenge information for Diclofenac is not applicable. Dechallenge information for Diclofenac is not applicable.

The reporter's assessment for drug Diclofenac related with Autoimmune hemolytic anemia is as follows: Possible. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08097989

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: 19500202	Age:	weight (kg): 101.2	BMI_int: 44.38	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic lesion					recovered/resolved
2 Icterus					recovered/resolved
3 Nausea					recovered/resolved
4 Vomiting					recovered/resolved
5 Liver damage					recovered/resolved
6 Hepatic lesion					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clindamycin		3 Df dosage form/Day	-	Injury
clindamycin			-	
2 voltaren		75 Mg milligram(s)/Day	-	Operation NOS
diclofenac sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Unknown	
Type 2 diabetes mellitus			Unknown	
Hyperthyroidism			Unknown	
Penicillin allergy			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Blood bilirubin indirect	20080815	µmol/l	74.79	
Blood bilirubin indirect	20080816	µmol/l	33.51	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 09.09.2008 DE-002147023-PHHY2008DE19803

Sender diagnosis: not reported

Narrative

Initial physician report received via health authorities on 28 Aug 2008: The patient with penicillin allergy, hyperthyroidism, diabetes mellitus type 2, and hypertension has been treated with Voltaren 75 mg /day (diclofenac) for post operative (pain), and clindamicin 900 mg for prophylaxis since 1 week and and both drugs were suspected to cause icterus and nausea and hepatic lesion. Toxic liver damage and vomiting were also reported. Patient recovered. The relationship was assessed as possible for both drugs.

The available information was considered inadequate to fully assess the case.

Serious spontaneous report "hepatic lesion" [medically significant], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 09.09.2008 DE-NOVNCHHQ-PHHY2008DE19803

Sender diagnosis: not reported

Narrative

Initial physician report received via health authorities on 28 Aug 2008: The patient with penicillin allergy, hyperthyroidism, diabetes mellitus type 2, and hypertension has been treated with Voltaren 75 mg /day (diclofenac) for post operative (pain), and clindamicin 900 mg for prophylaxis since 1 week and and both drugs were suspected to cause icterus and nausea and hepatic lesion. Toxic liver damage and vomiting were also reported. Patient recovered. The relationship was assessed as possible for both drugs.

The available information was considered inadequate to fully assess the case.

Serious spontaneous report "hepatic lesion" [medically significant], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08112522

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19360610	Age: 71 Year	weight (kg): 90	BMI_int: 27.78	Age fetus:
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Patientdeath

deathdate: 20080517	deathcause: Circulatory collapse
deathdate: 20080517	deathcause: Kidney failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20080505				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclac 75 ID diclofenac	Oral	150 Mg milligram(s)/Day 212	20080414-20080511 28 Day	Arm fracture
2 Siofor 1000 mg metformin	Oral	2000 Mg milligram(s)/Day 82	2007-20080511 -	Type 2 diabetes mellitus

Drugs (concomitant)

3 Berodual fenoterol hydrobromide; ipratropium bromide	Respiratory (inhalation)	2 Df dosage form/Day 109	-20080513 -	COPD
4 Calcivit D calcium carbonate; colecalciferol	Oral	2 Df dosage form/Day 59	-20080511 -	
5 Concor Cor 2.5 mg bisoprolol	Oral	5 Mg milligram(s)/Day 82	-20080511 -	
6 Domperidon 10 mg domperidone	Oral	/	-20080511 -	
7 Flutide 125 µg fluticasone propionate	Respiratory (inhalation)	250 µg microgram(s)/Day 109	-20080513 -	COPD
8 Furosemid 40 mg furosemide	Oral	60 Mg milligram(s)/Day 245	2007- -	
9 Opipramol opipramol	Oral	1 Df dosage form/Day	2007-20080511 -	
10 Pantozol pantoprazole	Oral	2* /Day 94	2007- -	Reflux oesophagitis
11 Promethazin promethazine	Oral	1 Df dosage form/Day	-20080511 -	
12 Spironolacton 100 mg spironolactone	Oral	100 Mg milligram(s)/Day 245	-20080511 -	
13 Vitamin B 12 cyanocobalamin	Oral	2 Df dosage form/Day	2007-20080511 -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
COPD			Unknown	
Type 2 diabetes mellitus			Unknown	
Hypertension arterial			Unknown	
Cardiac failure			Unknown	
Resection of rectum	1996		Unknown	
Osteoporosis			Unknown	
Reflux oesophagitis	200701		Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20080512	991	U/l	
Alanine aminotransferase increased	20080514	2315	U/l	
Alanine aminotransferase increased	20080517	370	U/l	
Aspartate aminotransferase increased	20080512	5397	U/l	
Aspartate aminotransferase increased	20080514	13846	U/l	
Aspartate aminotransferase increased	20080517	2562	U/l	
Bilirubin increased	20080514	2.55	mg/dl	
Bilirubin increased	20080517	3.88	mg/dl	
Bilirubin normal	20080512	0.74	mg/dl	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 25.09.2008 DE-FAKOS-170199

Sender diagnosis: not reported

Narrative

Die UAW hat sich ambulant entwickelt. Ende der Einnahme von Diclac und Siofor am 11.05.2008. Im weiteren Verlauf weiterhin erhöhte Transaminasen und Anstieg des Bilirubins. Exitus letalis am 17.05.2008 durch akutes Leberversagen (Todesursache laut Autopsie: periphere Kreislaufinsuffizienz bei fortgeschrittener Leberdystrophie).

Diclac Tabl, Hexal, ist spezifisch gelabelt: "akute Hepatitis mit oder ohne Ikterus", Stand: 29.08.2007.

Siofor Tabl, Berlin-Chemie, ist nicht gelabelt, Stand: 30.08.2005.

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 29.09.2008 DE-FAKOS-170199

Sender diagnosis: not reported

Narrative

Die UAW hat sich ambulant entwickelt. Ende der Einnahme von Diclac und Siofor am 11.05.2008. Im weiteren Verlauf weiterhin erhöhte Transaminasen und Anstieg des Bilirubins. Exitus letalis am 17.05.2008 durch akutes Leberversagen (Todesursache laut Autopsie: periphere Kreislaufinsuffizienz bei fortgeschrittener Leberdystrophie).

Diclac Tabl, Hexal, ist spezifisch gelabelt: "akute Hepatitis mit oder ohne Ikterus", Stand: 29.08.2007.

Siofor Tabl, Berlin-Chemie, ist nicht gelabelt, Stand: 30.08.2005.

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 03.07.2009 DE-FAKOS-170199

Sender diagnosis: not reported

Narrative

Aufnahme des Patienten in ein Krankenhaus am 12.05.2008. Gründe waren unklare abdominale Beschwerden, Übelkeit und Erbrechen seit 4 Tagen. Die Transaminasen zeigten sich bei Aufnahme massiv erhöht: ALAT: 991 U/l (Normbereich: <45 U/l), ASAT: 5397 U/l (Normbereich: <50 U/l). Die Maximalwerte wurden erreicht am 14.05.2008: ALAT: 2315 U/l und ASAT: 13846 U/l. Im Verlauf traten Gerinnungsstörungen infolge des Leberversagens auf (Quick <7 %, INR >6,7). Der Ammoniakspiegel betrug 83,3 µmol/l (16-60 µmol/l).

Exitus letalis am 17.05.2008 durch akutes Leberversagen (Todesursache laut Autopsie: periphere Kreislaufinsuffizienz auf dem Boden einer ausgedehnten Leberdystrophie).

Als Auslöser der Hepatitis werden verdächtigt: Diclac Tabl. (Einnahme: 14.04.-11.05.2008), Siofor Tabl. (Einnahme: 2007-11.05.2008). Diclac wird als "wahrscheinlich", Siofor als "möglich" eingestuft.

Nach Absetzen der Arzneimittel zeigte sich zunächst noch ein Anstieg der Transaminasen über 3 Tage, wonach sich eine spontane Erholung zeigte. Dies ist als Dechallenge interpretierbar. Das Leberversagen ist als assoziiert mit der UAW zu betrachten.

Das Siofor wird als „möglich“ aufgrund der bereits langen Einnahmeperiode bewertet. Das Diclac war neu angesetzt.<dbtext>To_Char_NARRATIVE_2</dbtext>

PVZ-FAKOS, Klinische Pharmakologie, 09.07.2009 DE-FAKOS-170199

Sender diagnosis: not reported

Narrative

Aufnahme des Patienten in ein Krankenhaus am 12.05.2008. Gründe waren unklare abdominale Beschwerden, Übelkeit und Erbrechen seit 4 Tagen. Die Transaminasen zeigten sich bei Aufnahme massiv erhöht: ALAT: 991 U/l (Normbereich: <45 U/l), ASAT: 5397 U/l (Normbereich: <50 U/l). Die Maximalwerte wurden erreicht am 14.05.2008: ALAT: 2315 U/l und ASAT: 13846 U/l. Im Verlauf traten Gerinnungsstörungen infolge des Leberversagens auf (Quick <7 %, INR >6,7). Der Ammoniakspiegel betrug 83,3 µmol/l (16-60 µmol/l).

Exitus letalis am 17.05.2008 durch akutes Leberversagen (Todesursache laut Autopsie: periphere Kreislaufinsuffizienz auf dem Boden einer ausgedehnten Leberdystrophie).

Als Auslöser der Hepatitis werden verdächtigt: Diclac Tabl. (Einnahme: 14.04.-11.05.2008), Siofor Tabl. (Einnahme: 2007-11.05.2008). Diclac wird als "wahrscheinlich", Siofor als "möglich" eingestuft.

Nach Absetzen der Arzneimittel zeigte sich zunächst noch ein Anstieg der Transaminasen über 3 Tage, wonach sich eine spontane Erholung zeigte. Dies ist als Dechallenge interpretierbar. Das Leberversagen ist als assoziiert mit der UAW zu betrachten.

Das Siofor wird als „möglich“ aufgrund der bereits langen Einnahmeperiode bewertet. Das Diclac war neu angesetzt.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08127397

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19570712	Age: 51 Year	weight (kg): 78	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaemia haemolytic	20080805			5 Day	recovered/resolved with sequel
2 Icterus	20080802			2 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	20080801-20080802 2 Day	Lumboischialgia

Drugs (concomitant)

2 ibuprofen ibuprofen	Oral	/	20080803- -	Intervertebral disc prolapse
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hodgkin's disease	2001		Unknown	
Chemotherapy	200103	200105	No	
Radiation therapy	200107		No	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hb		2.6	g/dl	
LDM		753	U/l	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08130089

Serious Yes	Death No	Lifethr. Yes	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice	20080802			2 Day	Unknown
2 Anemia hemolytic	20080805			5 Day	recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	20080801-20080802	Lumboischialgia
diclofenac			2 Day	

Drugs (concomitant)

2 ibuprofen	/	20080803-	Intervertebral disc prolapse
ibuprofen		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Radiation therapy	200107		Unknown	
Chemotherapy	200103	200105	No	
Intervertebral disc prolapse			Unknown	
Hodgkin's disease			Yes	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
LDH		753 U/L		
Hemoglobin		2.6g/dL		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 05.11.2008 DE-002147023-PHHY2008DE26883

Sender diagnosis: not reported

Narrative

Initial report received on 31 Oct 2008: The patient had concurrent condition of hodgkin's disease and intervertebral disc prolapse and had history of chemotherapy and radiation therapy. The patient was on Diclofenac (manufacturer unknown) for lumboischialgia at a daily dose of 150mg per day from 01 Aug 2008 to 02 Aug 2008. The patient developed haemolytic anaemia on 05 Aug 2008, the outcome of which was reported to be recovery with sequelae. The patient also had icterus from 02 Aug 2008. The events were considered life-threatening and required hospitalization. Outcome for icterus was not reported. The reporter did suspect a relationship between diclofenac and the reported events. The available information was considered inadequate to fully assess the case.

Serious spontaneous report, hemolytic anemia and jaundice (life threatening) already listed according to the Core Data Sheet.

All spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08130998

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19700818	Age: 38 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Back pain	20081005			3 Day	not recovered/not resolved
2 Gastroenteritis	200810				Unknown
3 Diarrhea	200810				Unknown
4 Nausea	200810				Unknown
5 Steatosis hepatic	20081006	200810		4 Day	recovered/resolved
6 Hepatitis viral	20081005	200810		3 Day	recovered/resolved
7 Abdominal pain upper	20081005			3 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/	20081003-	Pain
			-	

Drugs (concomitant)

2 cotrim sulfamethoxazole; trimethoprim	Oral	/	-	Bladder infection
			-	
3 Cranberry drug ascorbic acid; vaccinium oxycoccus		/	-	Bladder infection
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Gastroenteritis			Unknown	
Prophylactic vaccination			Unknown	Hepatitis A & B vaccination in 1999 (only 2 cycles)
Hepatitis			Unknown	
Migraine			Unknown	
Bladder infection NOS			Unknown	
Headache			Unknown	partially unilaterally.

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	20081005	238	NA	
GPT	20081006	416	NA	
GPT	20081008	279	NA	
Aspartate aminotransferase	20081005	342	NA	
Aspartate aminotransferase	20081006	211	NA	
Aspartate aminotransferase	20081008	80	NA	
Chest X-ray	20081008	nicht vorhanden	NAabsent	
ECG	20081008	nicht vorhanden	NAabsent	

GGT	20081005	74	NA
GGT	20081006	104	NA
GGT	20081008	90	NA
Transaminase NOS		>500	NA
Sonogram abnormal	20081006	nicht vorhanden	absent

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 07.11.2008 DE-NOVNCHHQ-PHHY2008DE26919

Sender diagnosis: not reported

Narrative

Initial report received on 31 Oct 2008 (phone call from physician and doctor letter, dated 17 Oct 2008, received via fax on 31 Oct 2008). This patient was treated for an unknown time period with Voltaren 100mg/d, due to vertebrogenic pains. As a treatment of recurrent bladder infections, the patient was occasionally set on Cotrim (cotrimoxazole) medication, but it was unclear whether patient received this drug in Oct 2008. The patient history included mention of a pregnancy hepatitis (some 6 years ago) and of migraine. Also, the patient's family had witnessed a gastroenteritis with diarrhoea and nausea, more than one week ago. On 5 Oct 2008, probably while on Voltaren 100mg/d treatment, patient appeared with vertebrogenic pains, and she reported significant pressure pain above the liver. Laboratory revealed transaminases increased more than ten-fold (GOT 342U/l, GPT 238U/l, gamma GT 74U/l). Patient was admitted on 6 Oct 2008, in good overall condition, no nausea, no vomiting, no diarrhoeas, no pruritus, no abdominal pain, no bad mouth odour. No stay outside of her home country Germany during the last 3 months. Findings upon admission were mostly normal (status, ECG, chest x-ray). Abdomen: smooth and atonic abdominal walls, only mild pressure pain above right upper abdomen. Abnormal laboratory upon admission as follows: GOT 211U/l, GPT 416U/l, gamma GT 104U/l, all other lab was normal. Virus serology ruled out acute hepatitis A, B and C. Abdominal sonography showed liver with only mildly accentuated intrahepatic bile ducts in right upper abdomen, ductus hepaticus communis with normal calibre intrahepatically and extrahepatically, mild steatosis, no concretions or signs of inflammation, pancreas normal, no signs of ileus, no free fluid, normal findings with respect to parenchymatous upper abdominal organs. On 8 Oct 2008, liver function tests were found significantly improved (GOT 80U/l, GPT 279U/l, gamma GT 90U/l). Since patient became and remained without complaints during the hospital stay, no liver biopsy was undertaken. Based on the previous gastroenteritis episode in the family, an accompanying viral hepatitis was suspected at the time of hospital discharge on 8 Oct 2008. Patient was in good general condition and she was feeling well. As per phone call on 31 Oct 2008, a physician stated that maximal transaminase levels had even exceeded 500U/l, but no date and no further comment was given. The laboratory completely normalised within 3 weeks after discharge. The reporter now speculated about an implication of Voltaren intake just prior to hospital admission, but he was not totally certain with respect to causality.

NOVARTIS, 11.11.2008 DE-002147023-PHHY2008DE26919

Sender diagnosis: not reported

Narrative

Initial report received on 31 Oct 2008 (phone call from physician and doctor letter, dated 17 Oct 2008, received via fax on 31 Oct 2008). This patient was treated for an unknown time period with Voltaren 100mg/d, due to vertebrogenic pains. As a treatment of recurrent bladder infections, the patient was occasionally set on Cotrim (cotrimoxazole) medication, but it was unclear whether patient received this drug in Oct 2008. The patient history included mention of a pregnancy hepatitis (some 6 years ago) and of migraine. Also, the patient's family had witnessed a gastroenteritis with diarrhoea and nausea, more than one week ago. On 5 Oct 2008, probably while on Voltaren 100mg/d treatment, patient appeared with vertebrogenic pains, and she reported significant pressure pain above the liver. Laboratory revealed transaminases increased more than ten-fold (GOT 342U/l, GPT 238U/l, gamma GT 74U/l). Patient was admitted on 6 Oct 2008, in good overall condition, no nausea, no vomiting, no diarrhoeas, no pruritus, no abdominal pain, no bad mouth odour. No stay outside of her home country Germany during the last 3 months. Findings upon admission were mostly normal (status, ECG, chest x-ray). Abdomen: smooth and atonic abdominal walls, only mild pressure pain above right upper abdomen. Abnormal laboratory upon admission as follows: GOT 211U/l, GPT 416U/l, gamma GT 104U/l, all other lab was normal. Virus serology ruled out acute hepatitis A, B and C. Abdominal sonography showed liver with only mildly accentuated intrahepatic bile ducts in right upper abdomen, ductus hepaticus communis with normal calibre intrahepatically and extrahepatically, mild steatosis, no concretions or signs of inflammation, pancreas normal, no signs of ileus, no free fluid, normal findings with respect to parenchymatous upper abdominal organs. On 8 Oct 2008, liver function tests were found significantly improved (GOT 80U/l, GPT 279U/l, gamma GT 90U/l). Since patient became and remained without complaints during the hospital stay, no liver biopsy was undertaken. Based on the previous gastroenteritis episode in the family, an accompanying viral hepatitis was suspected at the time of hospital discharge on 8 Oct 2008. Patient was in good general condition and she was feeling well. As per phone call on 31 Oct 2008, a physician stated that maximal transaminase levels had even exceeded 500U/l, but no date and no further comment was given. The laboratory completely normalised within 3 weeks after discharge. The reporter now speculated about an implication of Voltaren intake just prior to hospital admission, but he was not totally certain with respect to causality.

NOVARTIS, 19.03.2009 DE-002147023-PHHY2008DE26919

Sender diagnosis: not reported

Narrative

Initial report received on 31 Oct 2008 (phone call from physician and doctor letter, dated 17 Oct 2008, received via fax on 31 Oct 2008). This patient was treated for an unknown time period with Voltaren 100mg/d, due to vertebrogenic pains. As a treatment of recurrent bladder infections, the patient was occasionally set on Cotrim (cotrimoxazole) medication, but it was unclear whether patient received this drug in Oct 2008. The patient history included mention of a pregnancy hepatitis (some 6 years ago) and of migraine. Also, the patient's family had witnessed a gastroenteritis with diarrhoea

and nausea, more than one week ago. On 5 Oct 2008, probably while on Voltaren 100mg/d treatment, patient appeared with vertebragenic pains, and she reported significant pressure pain above the liver. Laboratory revealed transaminases increased more than ten-fold (GOT 342U/l, GPT 238U/l, gamma GT 74U/l). Patient was admitted on 6 Oct 2008, in good overall condition, no nausea, no vomiting, no diarrhoeas, no pruritus, no abdominal pain, no bad mouth odour. No stay outside of her home country Germany during the last 3 months. Findings upon admission were mostly normal (status, ECG, chest x-ray). Abdomen: smooth and atonic abdominal walls, only mild pressure pain above right upper abdomen. Abnormal laboratory upon admission as follows: GOT 211U/l, GPT 416U/l, gamma GT 104U/l, all other lab was normal. Virus serology ruled out acute hepatitis A, B and C. Abdominal sonography showed liver with only mildly accentuated intrahepatic bile ducts in right upper abdomen, ductus hepaticus communis with normal calibre intrahepatically and extrahepatically, mild steatosis, no concrements or signs of inflammation, pancreas normal, no signs of ileus, no free fluid, normal findings with respect to parenchymatous upper abdominal organs. On 8 Oct 2008, liver function tests were found significantly improved (GOT 80U/l, GPT 279U/l, gamma GT 90U/l). Since patient became and remained without complaints during the hospital stay, no liver biopsy was undertaken. Based on the previous gastroenteritis episode in the family, an accompanying viral hepatitis was suspected at the time of hospital discharge on 8 Oct 2008. Patient was in good general condition and she was feeling well. As per phone call on 31 Oct 2008, a physician stated that maximal transaminase levels had even exceeded 500U/l, but no date and no further comment was given. The laboratory completely normalised within 3 weeks after discharge. The reporter now speculated about an implication of Voltaren intake just prior to hospital admission, but he was not totally certain with respect to causality.

Follow up received on 10 Mar 2009 (AE documentation sheet filled out by hospital internal specialist): Therapy with Voltaren was given since 03 Oct 2008 (date marked with a question mark) in unclear dosage ("25mg", with question mark), for headache with migraine. The patient appeared at the hospital with back pain. Laboratory revealed 10-times increased transaminases (no levels given). Therapy with Voltaren was stopped and not restarted (no rechallenge). The outcome was reported as "not yet recovered". The reporter considered Voltaren as possibly related, but added that also an accompanying viral hepatitis was to be discussed, based on the previous viral gastroenteritic infection with diarrhoea and nausea, one week ago (patient and family).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08146347

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19661219	Age: 41 Year	weight (kg): 56	BMI_int: 20.08	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Reflux esophagitis					Unknown
2 Mucosal discoloration					Unknown
3 C-reactive protein increased	20080708			28 Day	Unknown
4 Calcium decreased	20080708			28 Day	Unknown
5 Gastrointestinal haemorrhage	20080702	20080715	14 Day	22 Day	recovered/resolved
6 GGTP increase	20080708			28 Day	Unknown
7 Overdose					Unknown
8 Potassium decreased	20080708			28 Day	Unknown
9 Sodium decreased	20080708			28 Day	Unknown
10 Urea increased	20080708			28 Day	Unknown
11 White blood cell count increased	20080708			28 Day	Unknown
12 Duodenal ulcer					Unknown
13 Helicobacter pylori infection					Unknown
14 Hematemesis					Unknown
15 Hiatus hernia					Unknown
16 Laparotomy	20080714				Unknown
17 Melena					Unknown
18 Pain worsened					Unknown
19 Pyloroplasty	20080714				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN DOLO diclofenac potassium	Oral	37.5 Mg milligram(s)/Day 82	20080611-20080708 28 Day	Back pain
2 VOLTAREN DOLO diclofenac potassium	Oral	/ 82	- -	
3 VOLTAREN DOLO diclofenac potassium	Oral	37.5 Mg milligram(s)/Day 82	-20080708 -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol abuse	19930101		Yes	
Pain NOS	20080611		Yes	Rückenschmerzen

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20080708	0.29	µmol/s.l	
Alkaline phosphatase	20080708	1.79	µmol/l	
Aspartate aminotransferase	20080708	0.53	µmol/s.l	
C-reactive protein	20080708	29.3	mg/l	

Calcium	20080708	2.10	mmol/l
Creatinine	20080708	65.0	NA
Fibrinogen	20080708	3.99	g/l
Gamma GT	20080708	1.08	mmol/s.l
Haematocrit	20080711	0.35	NA
Haematocrit	20080714	0.14	NA
Haematocrit	20080708	0.4	NA
Haemoglobin	20080711	7.14	mmol/l
Haemoglobin	20080714	3.09	mmol/l
Haemoglobin	20080708	8.81	mmol/l
INR	20080708	0.98	NA
MCV	20080711	103.0	fl
MCV	20080708	96.5	fl
Partial prothrombin time	20080708	31.7	sec
Potassium	20080708	3.32	mmol/l
Red blood cell count	20080711	3.4	Tpt/l
Red blood cell count	20080708	4.18	Tpt/l
Sodium	20080708	131	mmol/l
Thrombocyte count	20080708	285	Gpt/l
Urea	20080708	8.9	mmol/l
Quick's test	20080708	101	Prozent
Total white blood cell count	20080708	14.9	Gpt/l

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 12.12.2008 DE-PVZIBET-WEIMAR-10002

Sender diagnosis: not reported

Narrative

Sender's comment:

Initialmeldung - Zusatzinformationen werden erwartet

Reporter's comment:

NOVARTIS, 19.12.2008 DE-002147023-PHHY2008DE32084

Sender diagnosis: not reported

Narrative

Initial report received from the Health Authority (DE-BFARM-08146347) on 15 Dec 2008: The patient medical history included a concurrent condition of alcohol abuse since 01 Jan 1993. This patient was taking Voltaren (diclofenac) 25 mg for an unknown indication and experienced gastrointestinal bleeding on 02 Jul 2008. The patient was taking Voltaren 3 x1 tablet/day in the first week and up to 20 x 1 tablet/day in the second week. The outcome of the event was reported as not recovered. The lab data revealed that on 08 Jul 2008, the MCV, total white blood cell count, urea, Gamma GT and C-Reactive protein were increased (values not provided) and sodium, potassium, calcium and red blood cell count were decreased. On 11 Jul 2008, the lab data showed that the hemoglobin and hematocrit were decreased (values not provided).

Serious spontaneous report, gastrointestinal bleeding and overdose (life threatening) already listed according to the Core Data Sheet.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements.

Reporter's comment:

HELIOS Klinikum Wuppertal, 10.11.2009 DE-PVZIBE-WEIMAR-10002

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
gastrointestinale Blutung

KURZZUSAMMENFASSUNG:

obere gastrointestinale Blutung unter Diclofenac

BESCHREIBUNG:

Patient kam über die Rettungsstelle zur stationären Aufnahme wegen Hämatemesis seit einer Woche und Teerstuhl seit 3 Tagen. Patient hat seit 4 Wochen Voltaren als Selbstmedikation gegen Rückenschmerzen eingenommen. Da die Schmerzen schlimmer wurden und vor 2 Wochen ins Epigastrium ausstrahlten, nahm er bis zu 20 Tbl. am Tag und seit einer Woche dazu 0,7 ml Schnaps, bekannter Alkoholabusus. seit 2 Jahren trocken

THERAPIE:

Rettungsstelle: 1 Amp. Pantozol 40 mg i.v.

stationär: - Voltaren abgesetzt, kalte flüssige Kost

- Sterofundin tgl. 2000 ml als Infusion

- Pantozol 40 mg 2xd i.v., ab 09.07.08 über Perfusor

200 mg Pantozol/ 50 ml NaCl 2,1 ml/h

- ab 09.07.08: Klacid 500 mg 2x1/d; Amoxycillin 1 g 2x1/d

- bei Bedarf Buscopan Supp

14.07.08: - notfallmässige Relaparatomie mit Umstechung des Gefäßstumpfes des Ulcus duodeni, Pylorotomie und Pyloroplastik

Intensivstation: mehrfach Erythrozytenkonzentrate und Gerinnungsfaktoren

VERLAUF:

- 09.07.08 Gastroskopie:

Großes, florides Ulcus im Bulbus duodeni; Forrest II c (Hämatin);

große axiale Hernie; Refluxösophagitis; Schleimhauterythem im

Corpus und Antrum; mehrere Ulcera im Antrum ventriculi; Blutung

Forrest III (keine Blutungsstigmata); Schleimhauterythem im Bulbus duodeni distal.

Aufgrund einer endoskopisch nicht ausreichend versorgbaren oberen gastrointestinalen Blutung im Bereich des Bulbus duodeni am 14.07.08

Verlegung auf chirurgische Station. Am selben Tag erneut Hämatemesis-Blutung aus floriden Ulcus im Bulbus duodeni, Forrest I a; postoperative Betreuung auf Intensivstation ab 14.07.08.

17.07.08: Rückverlegung auf chirurgische Station

ERLÄUTERUNGEN ZUR UAW:

Die Chronologie spricht stark für einen Zusammenhang. Als alternative Ursache käme der positive Helicobacter pylori Befall und der Alkoholabusus in Frage, da aber das Medikament so stark überdosiert über einen längeren Zeitraum (3 Wochen) eingenommen wurde und der Alkohol wurde erst genommen, als die UAW schon begonnen hatte und die Schmerzen unerträglich wurden, S3.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 17.11.2009 DE-002147023-PHHY2008DE32084

Sender diagnosis: not reported

Narrative

Initial report received from the Health Authority (DE-BFARM-08146347) on 15 Dec 2008: The patient medical history included a concurrent condition of alcohol abuse since 01 Jan 1993. This patient was taking Voltaren (diclofenac) 25 mg for an unknown indication and experienced gastrointestinal bleeding on 02 Jul 2008. The patient was taking Voltaren 3 x1 tablet/day in the first week and up to 20 x 1 tablet/day in the second week. The outcome of the event was reported as not recovered. The lab data revealed that on 08 Jul 2008, the MCV, total white blood cell count, urea, Gamma GT and C-Reactive protein were increased (values not provided) and sodium, potassium, calcium and red blood cell count were decreased. On 11 Jul 2008, the lab data showed that the hemoglobin and hematocrit were decreased (values not provided).

Follow up report received from a regional pharmacovigilance center (intensive monitoring project) via HA on 10 Nov 2009: It was reported that this patient was treated with Voltaren dolo (diclofenac potassium) since 11 Jun 2008 for back pain. The patient started experiencing upper gastrointestinal bleeding under Voltaren since 02 Jul 2008. He was hospitalised due to hematemesis (unknown date) and melena. As the back pain had been worsening and had radiated to epigastrium he took up to 20 tablets per day and also had liquor (0.7 ml) since an unknown date. The patient had a concurrent condition of alcohol abuse however was recovering from alcoholism since 2 years. He was treated with 1 amp Pantozol 40 mg iv in hospital and Voltaren was stopped on 08 Jul 2008. Sterofundin 2000 ml daily as infusion, Pantozol 40 mg 2xd iv and since 09 Jul 2008 via perfusor 200 mg Pantozol/50 ml NaCl 2.1 ml/h were started alongwith Klacid 500 mg 2x1/d, Amoxycillin 1 g 2x1/d as needed and Buscopan supp. On 14 Jul 2008 a emergency relaparotomy, pylorotomy and pyloroplasty were performed. The patient was also treated for several times with erythrocyte concentrates and coagulation factors at the intensive care unit. Gastroscopy on 09 Jul 2008 showed large florid ulcer in bulbus duodeni, Forrest IIc (hematin), large axial hernia, reflux esophagitis, mucosa erythema in corpus and antrum, several ulcera in antrum ventriculi, bleeding Forrest III (no bleeding stigma) and mucosal erythema in bulbus duodeni distal. On 14 Jul 2008 hematemesis bleeding from florid ulcer in bulbus duodeni was observed. It was also reported that postoperative care was given in the intensive care unit. On 17 Jul 2008 the patient was re-transferred to surgical station. Chronology supported a relationship between the event and Voltaren. Alternative cause could be also be positive Helicobacter pylori infection and alcohol abuse. Voltaren was overdosed for a longer time (3 weeks), alcohol intake started after onset of adverse event. The event outcome for gastrointestinal bleeding was reported as complete recovery. No further information was provided. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event "overdose, reflux oesophagitis, white blood cell count increased and pain" and "Voltaren dolo" as suspected and hiatus hernia as not suspected based on the current available information.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08146902

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatocellular injury					recovered/resolved
2 Hepatotoxicity				1 Week	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	Drug use for unknown indication
diclofenac			-	indication
2 extractum kava kava		/	-	Drug use for unknown indication
kava-kava rhizoma			1 Week	indication
3 hyoscine butylbromide		/	-	Drug use for unknown indication
hyoscine butylbromide			-	indication
4 omeprazole		/	-	Drug use for unknown indication
omeprazole			-	indication
5 progesterone		/	-	Drug use for unknown indication
progesterone			-	indication
6 sulfasalazine		/	-	Drug use for unknown indication
sulfasalazine			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	Teschke R, Schwarzenboeck A, Hennermann KH. Kava hepatotoxicity: a clinical survey and critical analysis of 26 suspected cases. Eur J Gastroenterol Hepatol 2008;20:1182-1193	

Narratives (nur bei elektronischem Eingang)

biomo pharma GmbH, 15.12.2008 DE-BIOMO-001607

Sender diagnosis: not reported

Narrative

Internal case number: 100722 biomo D_081210 omeprazole

Coded term: hepatotoxicity

This case report from Germany referring to a 26-year old female was reported in the literature (based on information from BfArM).

A 26-year old female (patient 16) experienced hepatotoxicity during treatment with ethanolic kava kava extract in combination with sulfasalazine, diclofenac, progesterone, omeprazole and hyoscine butylbromide.

The patient took ethanolic kava kava extract (kavapyrones 50 mg/d) for 0.25 months resulting in cumulative kavapyrones of 0.4 g. Kava and concomitant medication were discontinued and hepatotoxicity showed restoration with good clinical outcome. The reporters assessed the causality of the hepatotoxicity to kava kava as possible, to the concomitant medication as unlikely. <dbtext>To_Char_NARRATIVE_2</dbtext> kava hepatotoxicity had characteristics primarily of an idiosyncratic liver injury based on a metabolic aberration (presumably cytochrome P450 enzymes) in unusually susceptible humans, rather than of a predictable liver disease (Teschke et al., 2008).

The patient in this case took also omeprazole. Dosage, treatment duration and indication are unknown. According to the SPC of omeprazol-biomo, changes in the liver enzymes (uncommon) and hepatitis with and without icterus as well as liver failure in patients with pre-existing severe liver disease are very rare adverse reactions to omeprazole treatment. Due to the lack of information the causality is not assessable in this case. It is recommended to observe the literature for further cases of possible omeprazole induced hepatotoxicity. An amendment of the current SPC of omeprazol-biomo is not necessary at the moment.

Reporter's comment:

The reporters assessed the causality of the hepatotoxicity to kava kava as possible, to the concomitant medication as unlikely.

SANOFI-SYNTHELABO, 29.12.2008 DE-SANOFI-SYNTHELABO-A01200816245

Sender diagnosis: not reported

Narrative

Reaction as reported: hepatocellular liver injury
LLT: HEPATOCELLULAR INJURY / 10019837
PT: HEPATOCELLULAR INJURY / 10019837

Narrative:

Initial serious case received from a full article via Health Authority in Germany and Switzerland on 15-Dec-2008:

Our case concerns the patient 16:

A 26 year-old female patient who had been treated with diclofenac experienced hepatocellular liver injury. The medical history was not clearly reported for this patient. As concomitant medication, he had been treated with kava extracts, sulfasalazine, progesterone, omeprazole and butylscopolaminium which were all suspected.

Action taken with diclofenac was not clearly reported as well as for other concomitant drugs, sulfasalazine, progesterone, omeprazole and butylscopolaminium. Only kava extracts data were reported with 0.25 month duration treatment with dose of 50 mg daily

On an unknown date, some investigations were performed and revealed the following results:

The activities of the aminotransferases (ALT; aspartate aminotransferase, AST) are commonly greater than 5 N (upper limit of normal range) and often fairly high, reaching values of up to 3121 IU/L. The initial ratio of ALT: AST is usually greater than 1. gamma -Glutamyl transpeptidase and ALP activities are commonly normal or slightly elevated. In all evaluable patients there was an increased ratio of ALT: ALP greater than or equal to 5 when serum activities were expressed as a multiple of N. These findings are compatible with hepatocellular liver disease and not with a cholestatic or a mixed type. Neither cytopenia nor eosinophilia in the differential blood count was commonly observed.

At the time of the report, the 'patient' outcome was favourable (recovered) and the reporter's causality was reported as excluded.

The article was, in part, as follows:

Hepatotoxicity has been previously suspected by national regulatory agencies in 26 patients in causal relationship with the treatment by kava extracts commonly used as herbal anxiolytic drugs.

Methods A quantitative causality assessment was undertaken using the system of the Council for International Organizations of Medical Sciences, scale of objective probability scoring.

Results Causality was unassessable, unrelated, or excluded in 16 patients owing to lack of temporal association and causes independent of kava or comedicated drugs.

Low Council for International Organizations of Medical Sciences scores additionally

resulted in excluded or unlikely causality assessments (n= 2), leaving a total of eight patients with various degrees of causality for kava and or less comedicated drugs.

Only one out of these eight patients adhered to the regulatory recommendations regarding both daily dose (inferior or equal to 120mg kavapyrones) and duration of therapy (inferior or equal to 3 months) and experienced toxic liver injury with a probable causality for kava. In six cases with kava overdose and/or increased duration of kava treatment causality for kava was possible (n=3) and for kava together with the comedicated drug(s) possible (n= 2) or probable (n=1).

Conclusion Kava taken as recommended is associated with rare hepatotoxicity, whereas overdose, prolonged treatment, and comedication may carry an increased risk.

An other case has been handled in the ex-aventis database concerning diclofenac with number 20016747EU.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002836

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19431227	Age: 64 Year	weight (kg): 104.2	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Glucose high	20080725				Unknown
2 Coffee ground vomiting	20080725				Unknown
3 AST increased	20080728				Unknown
4 Acute gastric ulcer with haemorrhage	20080725	20080730	6 Day		recovered/resolved
5 Melena	20080725				Unknown
6 Thrombocyte count decreased	20080725				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	3*50 Mg milligram(s)/ Wee 181	-20080724 -	Chronic pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Intervertebral disc prolapse			No	Z.n. OP; Beginn und Ende unbekannt
Chronic pain			Yes	chronisches Schmerzsyndrom; Beginn unbekannt

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20080728	0.41	µkat/l	
Amylase	20080728	0.59	µkat/l	
Aspartate aminotransferase	20080728	1.1	µkat/l	
Aspartate aminotransferase	20080728	0.38	µkat/l	
Bilirubin total	20080728	11.7	µmol/l	
Blood potassium	20080725	3.9	mmol/l	
C-reactive protein	20080725	5.4	mg/l	
Creatinine	20080725	86	µmol/l	
Gamma GT	20080728	0.58	µkat/l	
Glucose	20080726	6.6	mmol/l	
Glucose	20080726	7.5	mmol/l	
Glucose	20080726	6.7	mmol/l	
Glucose	20080725	11.4	mmol/l	
Haematocrit	20080726	0.25	Prozent	
Haematocrit	20080728	0.29	Prozent	
Haematocrit	20080725	0.27	Prozent	
Haemoglobin	20080726	5.2	mmol/l	

Haemoglobin	20080725	5.5	mmol/l
Haemoglobin	20080728	6.21	mmol/l
HbA1C	20080728	5.7	Prozent
INR	20080725	1.0	Physiolog.
Partial prothrombin time	20080725	24.7	sec
Sodium	20080725	143	mmol/l
Thrombin time	20080725	13.4	sec
Thrombocyte count	20080725	105	gpt/l
Antithrombin III	20080725	77	Prozent
Quick's test	20080725	96	Prozent
Total white blood cell count	20080725	8.4	10E9/l
Antithrombin III	20080725	77 prozent	
Aspartate aminotransferase	20080728	1.1 ukat/l	
C-reactive protein	20080725	5.4	mg/l
Endoscopy	20090725		
Endoscopy	20090730		
Glucose	20080726	6.6, 6.7, 7.5	mmol/l
Glucose	20080725	11.4	mmol/l
Haematocrit	20080726	0.25 prozent	
Haematocrit	20080728	0.29 prozent	
Haematocrit	20080725	0.27 prozent	
Haemoglobin	20080725	5.5	mmol/l
Haemoglobin	20080726	5.2	mmol/l
Haemoglobin	20080728	6.21	mmol/l
Thrombin time	20080725	13.4 sec	
Thrombocyte count	20080725	105 gpt/l	

Narratives (nur bei elektronischem Eingang)

Medizinische Fakultät der Universität Rostock, 09.01.2009 DE-PVZIBET-ROSTOCK-10018

Sender diagnosis: not reported

Narrative

Sender's comment:

Initialmeldung - Zusatzinformationen werden erwartet

Reporter's comment:

NOVARTIS, 22.01.2009 DE-002147023-PHHY2009DE01249

Sender diagnosis: not reported

Narrative

Initial Health Authority (HA no. DE-BFARM-09002836) report received on 12 Jan 2009. The patient with medical history of intervertebral disc prolapse

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and chronic pain was on therapy with Voltaren (diclofenac sodium) for chronic pain. Voltaren was discontinued on 24 Jul 2008. The patient had acute gastric ulcer with hemorrhage following therapy on 25 Jul 2008. Outcome was recovery as of 30 Jul 2008.

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious spontaneous, gastric ulcer with hemorrhage (hospitalization), assessed as listed according to the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements.

Reporter's comment:

HELIOS Klinikum Wuppertal, 10.11.2009 DE-PVZIBE-ROSTOCK-10018

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Ulcus ventriculi Forrest IIb

KURZZUSAMMENFASSUNG:

Ulcus ventriculi Blutung unter Diclofenac

BESCHREIBUNG:

Patient setzte am Aufnahmetag erstmalig Teerstuhl ab und erbrach einmal kaffeesatzartig, Rektale Untersuchung: Teerstuhl positiv am Aufnahmetag
THERAPIE:

letzte Einnahme Diclofenac 24.07.2008

Pantozol 2x40mg i.v. bis 28.07.2008, dann Oralisierung 2x40mg, Transfusion von 2 EKKL am 25. und 26.07.2008

VERLAUF:

25.07.2008 ÖGD: Ulcus ventriculi Forrest IIb, 30.07.2008 Kontroll-ÖGD: Regredientes Ulcus mit sauberen Fibringrund; während des stationären Aufenthaltes wurde nur am Aufnahmetag nochmalig Teerstuhl abgesetzt, ansonsten unauffälliger Verlauf, Kontroll-Gastroskopie in 6 Wochen empfohlen

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: VOLTAREN

BETREFFENDE UAW: akutes Magengeschwuer mit Blutung

ERLÄUTERUNG: Die Langzeiteinnahme von 2-3 Dragee Diclofenac/Woche aufgrund chronischer Schmerzen kann zur Ulcera im Gastrointestinal-Trakt führen. Diclofenac ist möglich. Risikofaktoren: fraglicher Alkoholkonsum

Helicobacter pylori als Alternativursache wurde nicht untersucht.<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 18.11.2009 DE-002147023-PHHY2009DE01249

Sender diagnosis: not reported

Narrative

Initial Health Authority (HA no. DE-BFARM-09002836) report received on 12 Jan 2009. The patient with medical history of intervertebral disc prolapse and chronic pain was on therapy with Voltaren (diclofenac sodium) for chronic pain. Voltaren was discontinued on 24 Jul 2008. The patient had acute gastric ulcer with hemorrhage following therapy on 25 Jul 2008. Outcome was recovery as of 30 Jul 2008.

Follow up report received from a regional pharmacovigilance center (intensive monitoring project) via HA on 10 Nov 2009: On 24 Jul 2008, the patient had the last intake of Voltaren. On the day of hospitalization (date unknown), the patient developed melena for the first time and vomiting (like "coffee grounds"). On 25 Jul 2008, the patient's thrombocyte count was 105 gpt/l (150-200) and glucose was 11.4 mmol/l (normal range 5.3-6.1) and on 28 Jul 2008 the AST was 1.1ukat/l (-0.83) . On an unspecified date, rectal examination revealed melena positive. On 25 Jul 2008, esophagogastroduodenoscopy revealed ulcer ventriculi forrest IIb. On 30 Jul 2008, esophagogastroduodenoscopy revealed regredient ulcer. The causality of the events with Voltaren was assessed as possible. In the absence of a health care professional's causality, the Novartis Medical Safety

Physician has provisionally assessed a relationship between the event "blood glucose increased" and "Voltaren" as not suspected and platelet count decreased as suspected based on the current available information.

Serious adverse drug reaction report, platelet count decreased[hospitalization], assessed as listed according to the Core Data Sheet.

In the absence of the reporting healthcare professional causality, the Novartis Medical Safety Physician has provisionally assessed the event platelet count decreased as suspected based on the current available information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003331

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19660612	Age: 42 Year	weight (kg): 86	BMI_int: 27.14	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea					recovering/resolving
2 Lassitude					recovering/resolving
3 Drug interaction					recovering/resolving
4 Hepatic enzyme increased	20081003	20081105	34 Day	4 Month	recovering/resolving
5 Vomiting					recovering/resolving
6 Jaundice	20081003				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 50 diclofenac sodium	Oral	100 Mg milligram(s)/Day 245	20081017-20081018 2 Day	Ache
2 ibuprofen ibuprofen	Oral	800 Mg milligram(s)/ 245	20081019-20081021 3 Day	Ache
3 IBUPROFEN ibuprofen	Oral	800 Mg milligram(s)/ 245	20081021-20081021 3 Day	Pain in spine
4 IBUPROFEN ibuprofen	Oral	800 Mg milligram(s)/ 245	20081019-20081019 3 Day	Pain in spine
5 JANUVIA sitagliptin phosphate	Oral	100 Mg milligram(s)/Day 82	20070101-20081023 21 Month	Type 2 diabetes mellitus
6 KATADOLON flupirtine maleate	Oral	200 Mg milligram(s)/Day 245	20080717-20081016 2 Month	Pain
7 SIMVASTATIN simvastatin	Oral	20 Mg milligram(s)/Day 82	20080701-20081023 3 Month	Hypercholesterolaemia

Drugs (concomitant)

8 METFORMIN metformin	Oral	200 Mg milligram(s)/Day 82	2003-20081023 -	Type 2 diabetes mellitus
9 PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/ 245	20081022-20081023 2 Day	Nausea
10 VIAGRA sildenafil citrate	Oral	25 Mg milligram(s)/ 82	2005- -	Potency disturbance

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypercholesterolaemia	20080701		Yes	
Low back pain			Yes	Stop Date: Continuing
Nausea			Yes	Stop Date: Continuing
Bone pain	20060101		Yes	
Ache			Yes	Stop Date: Continuing
Pain			Yes	
Spinal pain	2006		Yes	
Type 2 diabetes mellitus	20030101		Yes	
Erectile dysfunction			Yes	
Potency disturbance			Yes	Stop Date: Continuing

Primary source

Qualification: Physician

observestudytype:

Literature:

Qualification: Other Health Professional

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20081024	38.72	µmol/s.l	
Alanine aminotransferase	20081105	18.3	µmol/s.l	
Alkaline phosphatase	20081105	2.76	µmol/l	
Alkaline phosphatase	20081024	3.14	µmol/l	
Aspartate aminotransferase	20081105	5.25	µmol/s.l	
Aspartate aminotransferase	20081024	15.07	µmol/s.l	
Bilirubin direct	20081024	31.4	µmol/l	
Bilirubin direct	20081105	17.9	µmol/l	
Bilirubin total	20081024	49.7	µmol/l	
Bilirubin total	20081105	33.3	µmol/l	
Calcium	20081024	2.39	mmol/l	
Creatinine	20081024	73.0	µmol/l	
Fibrinogen	20081024	2.21	g/l	
Gamma GT	20081024	12.21	mmol/s.l	
Gamma GT	20081105	9.97	mmol/s.l	
Haematocrit	20081024	0.47		
Haemoglobin	20081024	10.30	mmol/l	
HbA1C	20081030	9.20	Prozent	
INR	20081105	0.98		
INR	20081024	1.23		
Lactate dehydrogenase	20081105	4.16	µmol/l	
Lactate dehydrogenase	20081024	8.13	µmol/l	
Mean corpuscular haemoglobin	20081024	88.9	fl	
Partial prothrombin time	20081024	33.6	sec	
Potassium	20081024	3.85	mmol/l	
Sodium	20081024	134	mmol/l	
Thrombin time	20081025	23.5	sec	
Thrombocyte count	20081024	163	Gpt/l	
Urea	20081024	3.3	mmol/l	
Ammonia	20081025	58.9	µmol/l	
Antithrombin III	20081025	59	Prozent	
Quick's test	20081105	102	Prozent	
Quick's test	20081024	63.0	Prozent	
Urine microalbumin	20081030	151	mg/l	
Total white blood cell count	20081024	9.1	Gpt/l	

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 12.01.2009 DE-PVZIBET-WEIMAR-10104

Sender diagnosis: not reported

Narrative

Sender's comment:

Initialmeldung - Zusatzinformationen werden erwartet

Reporter's comment:

NOVARTIS, 20.01.2009 DE-NOVNCHHQ-GXKR2009DE00717

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 14 Jan 2009 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-09003331), provided by a regional pharmacovigilance center, reported by a physician: This patient with a medical history of type II diabetes mellitus and pain in spine developed hepatic enzyme increased during treatment with simvastatin (MAH unknown) for hypercholesterolaemia, Diclac (diclofenac) for ache and ibuprofen (MAH unknown) for ache. Katadolon (flupirtine maleate) was also suspected. Several other drugs were taken concomitantly. The outcome of the adverse events was reported as not resolved. The reporter appraised the causality of the intake of simvastatine and the adverse event as possible, the intake of Diclac and ibuprofen and the adverse event as dubious.

information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

009507513, 21.01.2009 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance and pain in spine who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hypercholesterolaemia (duration not reported).

Concomitant therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of pain (started on 17-JUL-2008).

Other prior and concomitant therapy included sitagliptin phosphate (JANUVIA) (MDS), metformin, pantoprazole sodium (PANTOZOL), sildenafil citrate (VIAGRA), diclofenac Na (DICLAC) and ibuprofen.

On 16-OCT-2008, therapy with flupirtine was stopped.

On 24-OCT-2008, therapy with simvastatin was discontinued (last dose taken on 23-OCT-2008) because that day laboratory analysis revealed increased values for AST, ALT, GGT, AP, direct and total bilirubin and LDH (values not provided). However, onset date for "increase of liver enzymes" was explicitly reported as 03-NOV-2008. The patient was hospitalized (date not reported).

On 05-NOV-2008, AST, ALT, GGT, AP, direct and total bilirubin and LDH were still increased. The patient was reported as not recovered from increase of liver enzymes.

The regional pharmacovigilance center felt that increase of liver enzymes was possibly related to therapy with simvastatin and very likely related to therapy with flupirtine maleate.

Additional information has been requested.

PLIVA Research and Development Ltd., 04.02.2009 DE-PLIVA Research and Development Ltd.-107#03#2009-00296

Sender diagnosis: not reported

Narrative

This case was received from German Health Authority-the BFARM (DE-BFARM-09003331) on 27-Jan-2009.

A 42-year-old male patient with pain in spine, hypercholesterolaemia and type 2 diabetes mellitus developed hepatic enzyme increased during treatment with Katadolon (flupirtine maleate-200 mg daily, oral) for ache from 17-Jul-2008 till 16-Oct-2008 and simvastatin (20 mg daily, film-coated tablet, oral) for hypercholesterolaemia from Jul-2008 till 23-Oct-2008.

Concomitant medications included Januvia (sitagliptin-100 mg daily, film-coated tablet, oral) for type 2 diabetes mellitus from 2007 till 23-Oct-2008, metformin (100 mg twice daily, film-coated tablet, oral) for type 2 diabetes mellitus from 2003 till 23-Oct-2008, Pantozol (pantoprazole sodium-40 mg daily, oral) for nausea from 22-Oct-2008 till 23-Oct-2008 and Viagra (sildenafil citrate-25 mg daily, film-coated tablet, oral) for potency disturbance since 2005.

Interacting drugs included Diclac (diclofenac sodium-50 mg twice daily, oral) for ache from 17-Oct-2008 till 18-Oct-2008 and ibuprofen (800 mg, oral) for ache on 19th and 21th Oct 2008.

Patient past drug therapy: Actos for type 2 diabetes mellitus from 2003 till 2007.

Laboratory investigations revealed decreased thrombocyte count, quick's test and sodium level on 24-Oct-2008. International normalised ratio, ASAT, ALAT, AP, GGT, Bilirubin total, Bilirubin direct and lactate dehydrogenase were increased on 24-Oct-2008.

On 25-Oct-2008 thrombin time and ammonia were increased and antithrombin III was decreased.

HbA1C and Urine microalbumin were increased on 30-Oct-2008.

On 05-Nov-2008 ASAT, ALAT, AP, GGT, Bilirubin total, Bilirubin direct and lactate dehydrogenase were increased.

The reaction onset was on 03-Nov-2008.

The outcome was stated as not recovered and the causality was reported as highly probable for Katadolon and possible for simvastatin.

HELIOS Klinikum Wuppertal, 04.10.2010 DE-PVZIBE-WEIMAR-10104

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberenzymanstieg

KURZZUSAMMENFASSUNG:

Zunehmende Abgeschlagenheit bei Leberenzymerrhöhung nach Katadoloneinnahme und unter Simvastatin

BESCHREIBUNG:

Patient wurde wegen einer seit 3 Wochen zunehmenden Abgeschlagenheit stationär aufgenommen. Als führendes Symptom zeigte sich laborchemisch ein beginnendes Leberversagen mit deutlicher Erhöhung von Bilirubin und Transaminasen, Abfall der Gerinnungsfunktion und Anstieg des Ammoniakspiegels. Patient hatte aufgrund von Kreuzschmerzen vom 17.07 - 16.10.08 Flupirtin eingenommen. Da die schmerzensenkende Wirkung aussetzte, wurde es abgesetzt und durch Diclofenac ersetzt. Aufgrund einer möglichen Fahrtauglichkeitseinschränkung wurde dieses nach 2 Tagen von Ibuprofen ersetzt. Seit dem 22.10.08 traten zusätzlich Übelkeit und ein einmaliges Erbrechen auf.

THERAPIE:

- vom 24.10 - 27.10.08: Sterofundin 1000 tgl. als Infusion

- vom 24.10. - 03.11.08: Konaktion 10 Trf. tgl.

- ab 24.10.08: Absetzen aller Medikamente

VERLAUF:

- Hepatitis-Serologie:

Anti-HAV-IgM, HBs-AG, Anti-HBc, Anti-HBc-IgM, Anti-HCV negativ

- Infektionsdiagnostik:

EBV- IgM negativ, EBV-EA-AK negativ, EBV-IgG 1:960, Rotaviren und Adenoviren negativ

- Autoimmundiagnostik:

ANA, Doppelstrang-DNA, AMA, p-, c-, ANCA, LKM, SLA sämtliche negativ

- 24.10.08 Oberbauchsonographie:

Leber grenzwertig groß, gering strukturverdichtet und konturabgerundet im Sinne einer Steatosis hepatis Grad 1.

- 30.10.08 Leberpunktion:

Das histomorphologische Bild zeigt Veränderungen wie sie im Rahmen einer medikamentös-toxischen Genese zu sehen sein können. Darüber hinaus zeigen sich stoffwechselbedingte Veränderungen bei Diabetes mellitus.

Nach Absetzen aller Medikamente langsame Besserung der pathologischen Laborparameter.

CDT vom 24.10.08: 2,08% (Der Wertebereich von 1,8-2,2% ist als Grauzone definiert. Die Interpretation der Meßergebnisse in diesem Bereich sollte mit der entsprechenden Vorsicht erfolgen.) Bei dem Patienten wurde Alkohol als toxisches Agens ausgeschlossen.

ERLÄUTERUNGEN ZUR UAW:

Lt. Fachinformation sind unter Simvastatin und Katadolon Leberenzym erhöhungen möglich. Lt. Entlassungsbrief liegt ein medikamentös-toxisches Lebersversagen nach Katadolon-/Simvastatin-Einnahme vor.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: KATADOLON

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: - Challenge: very suggestive

- Dechallenge: suggestive

Katadolon wird aufgrund des längeren Einnahmezeitraumes am stärksten verdächtigt

MEDIKAMENT: diclac 50

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Lt. Fachinformation kann Diclofenac zur Erhöhung der Serumtransaminasen führen und bei Langzeittherapie zu Leberschäden. Da die (als Symptom der Leberschädigung interpretierte) Abgeschlagenheit schon vor Tabletteneinnahme eintrat, und eine sehr kurze Einnahmedauer vorliegt, wird es als ursächliches Medikament nicht verdächtigt, nur im Sinne einer Verstärkung der Symptome.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Lt. Fachinformation kann Ibuprofen selten zu Leberschäden und Leberfunktionsstörungen führen, insbesondere bei Langzeitbehandlung. Da die (als Symptom der Leberschädigung interpretierte) Abgeschlagenheit schon vor Tabletteneinnahme eintrat, wird es nicht verdächtigt als ursächliches Medikament, sondern nur im Sinne einer Symptomverstärkung.

MEDIKAMENT: SIMVASTATIN

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Lt. Fachinformation ist eine Transaminasenerhöhung bei Einnahme von 40 mg dokumentiert als seltenes Ereignis. Hier lag die Tagesdosis bei 20 mg, daher unwahrscheinlicher, aber nicht ausgeschlossen. Ebenfalls sind selten Hepatitis und Ikterus beschrieben.

MEDIKAMENT: JANUVIA

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Januvia wird verdächtigt besonders hinsichtlich Chronologie.<dbtext>To_Char_NARRATIVE_2</dbtext>

009507513, 07.10.2010 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance, low back pain, pain (unspecified) and pain in spine who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hypercholesterolaemia (duration not reported).

Suspect therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of back pain (started on 17-JUL-2008) and sitagliptin phosphate (MSD), 100 mg tablet, 100 mg once a day for the treatment of type II diabetes mellitus.

Concomitant therapy included metformin, pantoprazole sodium (PANTOZOL) and sildenafil citrate (VIAGRA).

In the beginning of October 2008, the patient developed fatigue. On 16-OCT-2008, therapy with flupirtine was stopped due to missing analgesic effect. On 17-OCT-2008, suspect therapy with diclofenac Na (DICALAC), 50 mg tablet, 50 mg twice a day for the treatment of ache was initiated. On 18-OCT-2008, therapy with diclofenac Na (DICALAC) was discontinued and replaced with suspect therapy ibuprofen, 800 mg tablet, 800 mg once a day for the treatment of ache (taken on 19-OCT-2008 and 21-OCT-2008) due to possible reduced fitness to drive. On 21-OCT-2010, therapy with ibuprofen was discontinued.

On approximately 24-OCT-2008, the patient was hospitalized due to persisting fatigue. On 24-OCT-2008, therapy with simvastatin was discontinued (last dose taken on 23-OCT-2008) because that day laboratory analysis revealed increased liver enzymes. However, the patient was diagnosed with "increase of liver enzymes" and onset date was reported as 03-OCT-2008 in follow-up (the only by the agency coded adverse event). Blood analysis revealed INR 1.23/ units unknown, direct serum bilirubin 31.4 mcml/l, blood lactate dehydrogenase 8.13 mcml/l, serum alanine aminotransferase 38.72 mcml/s.l, total serum bilirubin 49.7 mcml/l, serum gamma-glutamyl transferase 12.21 mmol/s.l, serum aspartate aminotransferase 15.07 mcml/s.l and serum alkaline phosphatase 3.14 mcml/s.l. That same day, abdominal ultrasound revealed grade 1 hepatic steatosis and serum transferrin test (CDT) showed 2.08 %, however, alcohol was excluded as toxic agent.

As of 24-OCT-2008, the patient was treated with electrolytes (unspecified) (+) sodium lactate (STEROFUNDIN) i. v. and phytonadione (KONAKION), 10 drops daily.

On approximately 24-OCT-2008, laboratory analysis revealed negative deoxyribonucleic acid antibodies test, negative rotavirus and adenovirus test, negative serum Epstein-Barr viral capsid antigen immunoglobulin G antibody test (1:960/ units unknown), negative serum Epstein-Barr viral capsid antigen immunoglobulin M antibody test, negative serum Epstein-Barr virus early antigen immunoglobulin M antibody test, negative serum soluble liver antigen autoantibody test, negative serum perinuclear antineutrophil cytoplasmic antibody test (p-ANCA), negative serum cytoplasmic antineutrophil cytoplasmic antibody test (c-ANCA), negative serum liver and kidney microsomal antibody test, negative serum hepatitis C antibody test, negative serum hepatitis B surface antigen test, negative serum hepatitis B core immunoglobulin M antibody test, negative serum hepatitis B core immunoglobulin G antibody test, negative serum hepatitis A immunoglobulin M antibody test, negative serum antinuclear antibodies test (ANA) and negative serum antimitochondrial antibody test (AMA).

On 30-Oct-2008, hepatic biopsy showed medicamentous-toxic liver changes as well as metabolism-induced changes in the context of the patient's diabetes mellitus. That same day, whole blood glycosylated hemoglobin was 9.20 %.

In the further course, pathological laboratory parameters slightly decreased.

On 05-Nov-2008, blood analysis revealed INR 0.98/ units unknown, serum alkaline phosphatase 2.76 mcmol/s.l, serum alanine aminotransferase 18.3 mcmol/s.l, direct serum bilirubin 17.9 mcmol/l, total serum bilirubin 33.3 mcmol/l, serum gamma-glutamyl transferase 9.97 mmol/s.l, serum aspartate aminotransferase 5.25 mcmol/s.l, prothrombin time 102 % and blood lactate dehydrogenase 4.16 mcmol/l.

As of 05-NOV-2008, the patient was recovering from increase of liver enzymes, however the agency indicated 05-NOV-2008 as stopp date. According to the discharge letter, a medicamentous-toxic liver failure after flupirtine maleate (KATADOLON)/ simvastatin intake has taken place (not coded as AE by the agency).

The regional pharmacovigilance center felt that increase of liver enzymes were possibly related to therapy with simvastatin and/or sitagliptin phosphate and probably related to therapy with flupirtine maleate. Diclofenac Na (DICALAC) and ibuprofen were listed as possible interacting drugs.

Additional information is not expected.

sanofi-aventis, 18.10.2010 DE-SANOFI-AVENTIS-2010SA060395

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09003331) on 06-Oct-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

It's involved a 42-year-old male patient suffered from diabetes mellitus type II treated with metformin hydrochloride 100 mg/day starting in 2003 as well as with sitagliptin phosphate starting in 2007. The hypercholesterolemia was treated with simvastatin 20 mg/day from July 2008. For potency disturbance the patient took sildenafil citrate 25 mg as needed since 2005.

Due to pain in spine the patient following drugs were used: flupirtine maleate 200 mg/day was started on 17-Jul-2008. After decreasing of drug effect the patient switched to diclofenac, starting on 17-Oct-2008. Two days later the medication was again switched to ibuprofen (due to possible disability to drive a car), which was given until 21-Oct-2008. Simvastatin and sitagliptin phosphate were discontinued on 23-Oct-2008.

Since 03-Oct-2008 the patient noticed an increasing lassitude. Additionally he complained about nausea and one-time vomiting since 22-Oct-2008.

Two days later the patient was hospitalised and increased hepatic enzymes were revealed by lab tests.

Corrective treatment with pantoprazole 40 mg/day against nausea was given from 22-Oct-2008 to the next day, sterofundin 1000 ml from 24-Oct-2008 to 27-Oct-2008, phytomenadione 10 drops/day from 24-Oct-2008 to 03-Nov-2008. On 05-Nov-2008 the patient was recovering.

Reporter causality:

Possible for ibuprofen as interactive drug (aggravation of the existing symptoms)

Possible for simvastatin

sanofi-aventis, 14.01.2011 DE-SANOFI-AVENTIS-2010SA060395

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09003331) on 06-Oct-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

It's involved a 42-year-old male patient suffered from diabetes mellitus type II treated with metformin hydrochloride 100 mg/day starting in 2003 as well

as with sitagliptin phosphate starting in 2007. The hypercholesterolemia was treated with simvastatin 20 mg/day from July 2008. For potency disturbance the patient took sildenafil citrate 25 mg as needed since 2005.

Due to pain in spine the patient following drugs were used: flupirtine maleate 200 mg/day was started on 17-Jul-2008. After decreasing of drug effect the patient switched to diclofenac, starting on 17-Oct-2008. Two days later the medication was again switched to ibuprofen (due to possible disability to drive a car), which was given until 21-Oct-2008. Simvastatin and sitagliptin phosphate were discontinued on 23-Oct-2008.

Since 03-Oct-2008 the patient noticed an increasing lassitude. Additionally he complained about nausea and one-time vomiting since 22-Oct-2008. Two days later the patient was hospitalised and increased hepatic enzymes were revealed by lab tests.

Corrective treatment with pantoprazole 40 mg/day against nausea was given from 22-Oct-2008 to the next day, sterofundin 1000 ml from 24-Oct-2008 to 27-Oct-2008, phytomenadione 10 drops/day from 24-Oct-2008 to 03-Nov-2008. On 05-Nov-2008 the patient was recovering.

Reporter causality:

Possible for ibuprofen as interactive drug (aggravation of the existing symptoms)

Possible for simvastatin

Significant correction was performed on 12-JAN-2011: hepatocellular damage in context of a drug interaction between ibuprofen, simvastatin, flupirtine, diclofenac and sitagliptin is now considered unlisted for ibuprofen and simvastatin.

009507513, 17.12.2012 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency via a regional Pharmacovigilance center, concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance, low back pain, pain (unspecified) and pain in spine who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day, orally for the treatment of hypercholesterolemia (duration not reported).

Suspect therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of back pain (started on 17-JUL-2008) and sitagliptin phosphate (JANUVIA), 100 mg tablet, 100 mg once a day for the treatment of type II diabetes mellitus. Concomitant therapy included metformin, pantoprazole sodium (PANTOZOL) and sildenafil citrate (VIAGRA).

In the beginning of October 2008, the patient developed fatigue. On 16-OCT-2008, therapy with flupirtine was stopped due to missing analgesic effect. On 17-OCT-2008, suspect therapy with diclofenac Na (DICAL), 50 mg tablet, 50 mg twice a day for the treatment of ache was initiated. On 18-OCT-2008, therapy with diclofenac Na (DICAL) was discontinued and replaced with suspect therapy ibuprofen, 800 mg tablet, 800 mg once a day for the treatment of ache (taken on 19-OCT-2008 and 21-OCT-2008) due to possible reduced fitness to drive. On 21-OCT-2010, therapy with ibuprofen was discontinued.

On approximately 24-OCT-2008, the patient was hospitalized due to persisting fatigue. On 24-OCT-2008, therapy with simvastatin was discontinued (last dose taken on 23-OCT-2008) because that day laboratory analysis revealed increased liver enzymes. However, the patient was diagnosed with "increase of liver enzymes" and onset date was reported as 03-OCT-2008 in follow-up (the only by the agency coded adverse event). Blood analysis revealed INR 1.23/ units unknown, direct serum bilirubin 31.4 mcml/l, blood lactate dehydrogenase 8.13 mcml/l, serum alanine aminotransferase 38.72 mcml/s.l, total serum bilirubin 49.7 mcml/l, serum gamma-glutamyl transferase 12.21 mmol/s.l, serum aspartate aminotransferase 15.07 mcml/s.l and serum alkaline phosphatase 3.14 mcml/s.l. That same day, abdominal ultrasound revealed grade 1 hepatic steatosis and serum transferrin test (CDT) showed 2.08 %, however, alcohol was excluded as toxic agent.

As of 24-OCT-2008, the patient was treated with electrolytes (unspecified) (+) sodium lactate (STEROFUNDIN) i. v. and phytionadione (KONAKION), 10 drops daily.

On approximately 24-OCT-2008, laboratory analysis revealed negative deoxyribonucleic acid antibodies test, negative rotavirus and adenovirus test, negative serum Epstein-Barr viral capsid antigen immunoglobulin G antibody test (1:960/ units unknown), negative serum Epstein-Barr viral capsid antigen immunoglobulin M antibody test, negative serum Epstein-Barr virus early antigen immunoglobulin M antibody test, negative serum soluble liver antigen autoantibody test, negative serum perinuclear antineutrophil cytoplasmic antibody test (p-ANCA), negative serum cytoplasmic antineutrophil cytoplasmic antibody test (c-ANCA), negative serum liver and kidney microsomal antibody test, negative serum hepatitis C antibody test, negative serum hepatitis B surface antigen test, negative serum hepatitis B core immunoglobulin M antibody test, negative serum hepatitis B core immunoglobulin G antibody test, negative serum hepatitis A immunoglobulin M antibody test, negative serum antinuclear antibodies test (ANA) and negative serum antimitochondrial antibody test (AMA).

On 30-Oct-2008, hepatic biopsy showed medicamentous-toxic liver changes as well as metabolism-induced changes in the context of the patient's diabetes mellitus. That same day, whole blood glycosylated hemoglobin was 9.20 %.

In the further course, pathological laboratory parameters slightly decreased.

On 05-Nov-2008, blood analysis revealed INR 0.98/ units unknown, serum alkaline phosphatase 2.76 mcml/s.l, serum alanine aminotransferase 18.3 mcml/s.l, direct serum bilirubin 17.9 mcml/l, total serum bilirubin 33.3 mcml/l, serum gamma-glutamyl transferase 9.97 mmol/s.l, serum aspartate aminotransferase 5.25 mcml/s.l, prothrombin time 102 % and blood lactate dehydrogenase 4.16 mcml/l.

As of 05-NOV-2008, the patient was recovering from increase of liver enzymes, however the agency indicated 05-NOV-2008 as stop date. According to the discharge letter, a medicamentous-toxic liver failure after flupirtine maleate (KATADOLON)/ simvastatin intake has taken place (not coded as AE by the agency).

The regional Pharmacovigilance center felt that increase of liver enzymes were possibly related to therapy with simvastatin and/or sitagliptin phosphate and probably related to therapy with flupirtine maleate. Diclofenac Na (DICALAC) and ibuprofen were listed as possible interacting drugs.

This report was identified from a line listing obtained on request by the company from the FDA under the freedom of information act on 06-OCT-2010. It was reported that the patient's concurrent conditions included bone pain. It was reported that the patient's onset date for hepatic enzyme increased was 03-OCT-2008 (previously reported as 24-oct-2008), and it was reported that on the same date, the patient experienced Drug interaction. It was also stated that in 2007, the patient started therapy with sitagliptin phosphate (JANUVIA), 100 mg once a day, orally for type 2 diabetes mellitus. It was reported that the start date for flupirtine maleate (KATADOLON) was 17-JUL-2008 (also reported as 14-JUL-2008). On unspecified dates laboratory tests such as coagulating function, ammonia level, retrovirus test and adenovirus test were performed with results of: decreased, increased, negative and negative, respectively.

At the time of the report, the patient's outcome for both events was updated from recovering to not recovered.

Additional information is not expected.

009507513, 27.12.2012 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency via a regional Pharmacovigilance center, concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance, low back pain, pain (unspecified) and pain in spine since 2006 who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day, orally for the treatment of hypercholesterolemia (duration not reported).

Suspect therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of back pain (started on 17-JUL-2008) and sitagliptin phosphate (JANUVIA), 100 mg tablet, 100 mg once a day for the treatment of type II diabetes mellitus. Concomitant therapy included metformin, pantoprazole sodium (PANTOZOL) and sildenafil citrate (VIAGRA).

In the beginning of October 2008, the patient developed fatigue. On 16-OCT-2008, therapy with flupirtine was stopped due to missing analgesic effect. On 17-OCT-2008, suspect therapy with diclofenac Na (DICALAC), 50 mg tablet, 50 mg twice a day for the treatment of ache was initiated. On 18-OCT-2008, therapy with diclofenac Na (DICALAC) was discontinued and replaced with suspect therapy ibuprofen, 800 mg tablet, 800 mg once a day for the treatment of ache (taken on 19-OCT-2008 and 21-OCT-2008) due to possible reduced fitness to drive. On 21-OCT-2010, therapy with ibuprofen was discontinued.

On approximately 24-OCT-2008, the patient was hospitalized due to persisting fatigue. On 24-OCT-2008, therapy with simvastatin was discontinued (last dose taken on 23-OCT-2008) because that day laboratory analysis revealed increased liver enzymes. However, the patient was diagnosed with "increase of liver enzymes" and onset date was reported as 03-OCT-2008 in follow-up (the only by the agency coded adverse event). Blood analysis revealed INR 1.23/ units unknown, direct serum bilirubin 31.4 mcml/l, blood lactate dehydrogenase 8.13 mcml/l, serum alanine aminotransferase 38.72 mcml/s.l, total serum bilirubin 49.7 mcml/l, serum gamma-glutamyl transferase 12.21 mmol/s.l, serum aspartate aminotransferase 15.07 mcml/s.l and serum alkaline phosphatase 3.14 mcml/s.l. That same day, abdominal ultrasound revealed grade 1 hepatic steatosis and serum transferrin test (CDT) showed 2.08 %, however, alcohol was excluded as toxic agent.

As of 24-OCT-2008, the patient was treated with electrolytes (unspecified) (+) sodium lactate (STEROFUNDIN) i. v. and phytonadione (KONAKION), 10 drops daily.

On approximately 24-OCT-2008, laboratory analysis revealed negative deoxyribonucleic acid antibodies test, negative rotavirus and adenovirus test, negative serum Epstein-Barr viral capsid antigen immunoglobulin G antibody test (1:960/ units unknown), negative serum Epstein-Barr viral capsid antigen immunoglobulin M antibody test, negative serum Epstein-Barr virus early antigen immunoglobulin M antibody test, negative serum soluble liver antigen autoantibody test, negative serum perinuclear antineutrophil cytoplasmic antibody test (p-ANCA), negative serum cytoplasmic antineutrophil cytoplasmic antibody test (c-ANCA), negative serum liver and kidney microsomal antibody test, negative serum hepatitis C antibody test, negative serum hepatitis B surface antigen test, negative serum hepatitis B core immunoglobulin M antibody test, negative serum hepatitis B core immunoglobulin G antibody test, negative serum hepatitis A immunoglobulin M antibody test, negative serum antinuclear antibodies test (ANA) and negative serum antimitochondrial antibody test (AMA).

On 30-OCT-2008, hepatic biopsy showed medicamentous-toxic liver changes as well as metabolism-induced changes in the context of the patient's diabetes mellitus. That same day, whole blood glycosylated hemoglobin was 9.20 %.

In the further course, pathological laboratory parameters slightly decreased.

On 05-NOV-2008, blood analysis revealed INR 0.98/ units unknown, serum alkaline phosphatase 2.76 mcml/s.l, serum alanine aminotransferase 18.3 mcml/s.l, direct serum bilirubin 17.9 mcml/l, total serum bilirubin 33.3 mcml/l, serum gamma-glutamyl transferase 9.97 mmol/s.l, serum aspartate aminotransferase 5.25 mcml/s.l, prothrombin time 102 % and blood lactate dehydrogenase 4.16 mcml/l.

As of 05-NOV-2008, the patient was recovering from increase of liver enzymes, however the agency indicated 05-NOV-2008 as stop date. According to the discharge letter, a medicamentous-toxic liver failure after flupirtine maleate (KATADOLON)/ simvastatin intake has taken place (not coded as AE by the agency).

The regional Pharmacovigilance center felt that increase of liver enzymes were possibly related to therapy with simvastatin and/or sitagliptin phosphate and probably related to therapy with flupirtine maleate. Diclofenac Na (DICALAC) and ibuprofen were listed as possible interacting drugs.

This report was identified from a line listing obtained on request by the company from the FDA under the freedom of information act on 06-OCT-2010. It was reported that the patient's concurrent conditions included bone pain. It was reported that the patient's onset date for hepatic enzyme increased was 03-OCT-2008 (previously reported as 24-OCT-2008), and it was reported that on the same date, the patient experienced Drug interaction. It was also stated that in 2007, the patient started therapy with sitagliptin phosphate (JANUVIA), 100 mg once a day, orally for type 2 diabetes mellitus. It was reported that the start date for flupirtine maleate (KATADOLON) was 17-JUL-2008 (also reported as 14-JUL-2008). On unspecified dates laboratory tests such as coagulating function, ammonia level, retrovirus test and adenovirus test were performed with results of: decreased, increased, negative and negative, respectively.

At the time of the report, the patient's outcome for both events was updated from recovering to not recovered.

Follow up information has been received from a line listing obtained on request by the company from the FDA under the freedom of information act on 19-AUG-2011. It was reported that on 05-NOV-2008, the patient was recovering from hepatic enzymes increased and that drug interaction was not an adverse reaction.

Additional information is not expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003370

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19240216	Age: 84 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sense of oppression	20080922	20080923	2 Day	1 Day	recovered/resolved
2 Allergy	20080922	20080923	2 Day	1 Day	recovered/resolved
3 Alanine aminotransferase increased					Unknown
4 Alkaline phosphatase increased					Unknown
5 White blood cell count increased					Unknown
6 Fibrinogen increased					Unknown
7 Pruritis	20080922	20080923	2 Day	1 Day	recovered/resolved
8 Sweating attack	20080922	20080923	2 Day	1 Day	recovered/resolved
9 Aspartate aminotransferase increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN diclofenac potassium	Oral	50 Mg milligram(s)/Day 245	20080922-20080922 -	Ache across chest

Drugs (concomitant)

2 ACTRAPID HUMAN insulin human	Subcutaneous	48 lu international unit(s)/l - 120	-	Type 2 diabetes mellitus
3 ass "hexal" acetylsalicylic acid	Oral	100 Mg milligram(s)/Day 245	-	Cardiovascular event prophylaxis
4 isoket 20 isosorbide dinitrate	Oral	40 Mg milligram(s)/Day 245	-	Angina pectoris
5 METOHEXAL metoprolol tartrate	Oral	47.5 Mg milligram(s)/Day 245	-	Hypertension arterial
6 RAMIPRIL ramipril	Oral	5 Mg milligram(s)/Day 245	-	Hypertension arterial
7 SIMVA simvastatin	Oral	20 Mg milligram(s)/Day 245	-	Hypercholesterolaemia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypercholesterolaemia			Yes	Beginn unbekannt
Angina pectoris			Yes	Beginn unbekannt
Hypertension arterial			Yes	Beginn unbekannt
Type 2 diabetes mellitus			Yes	Beginn unbekannt

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Activated partial thromboplastin time	20080922	26.4	sec	
Alanine aminotransferase	20080922	1.65	µmol/s.l	
Alkaline phosphatase	20080922	2.81	µmol/l	

Aspartate aminotransferase	20080922	1.29	μmol/s.l
Calcium	20080922	2.24	mmol/l
Creatinine	20080922	87	μmol/l
Fibrinogen	20080922	8.9	g/l
Gamma GT	20080922	0.72	mmol/s.l
Haematocrit	20080922	0.43	
Haemoglobin	20080922	8.87	mmol/l
INR	20080922	1	
Mean corpuscular haemoglobin	20080922	87	fl
Potassium	20080922	4.2	mmol/l
Sodium	20080922	139	mmol/l
Thrombocyte count	20080922	327	Gpt/l
Urea	20080922	6.3	mmol/l
Quick's test	20080922	96.7	Prozent
Total white blood cell count	20080922	13.3	Gpt/l

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 12.01.2009 DE-PVZIBET-WEIMAR-10064

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initialmeldung - Zusatzinformationen werden erwartet

Reporter's comment:

NOVARTIS, 23.01.2009 DE-002147023-PHHY2009DE01344

Sender diagnosis: not reported

Narrative

Initial report received from BfArM (DE-BFARM-09003370): This patient initiated Voltaren (diclofenac sodium) 50 mg, oral therapy on 22 Sep 2008 and took once only. On 22 Sep 2008, the patient experienced sense of oppression, pruritus and sweating. The event recovered on 23 Sep 2008. Event causality was possible.

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious spontaneous report, sense of oppression (hospitalization), assessed as unlisted according to the Core Data Sheet.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements.

Reporter's comment:

HELIOS Klinikum Wuppertal, 23.04.2010 DE-PVZIBE-WEIMAR-10064

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Allergie

KURZZUSAMMENFASSUNG:

V.a. Allergie nach Einnahme einer Tabl. Voltaren.

BESCHREIBUNG:

Einweisung erfolgte wegen Beklemmungsgefühl, Schweißausbruch und Juckreiz. Seit 19.09.08 Schmerzen im Rücken-/Brustbereich. Einnahme einer Tbl. Voltaren. Eine Stunde später: Beklemmungsgefühl, Schweißausbruch und Juckreiz.

THERAPIE:

- Voltaren abgesetzt

VERLAUF:

Unauffälliger Verlauf, da bereits wenige Stunden nach stationärer Aufnahme die Symptome der Allergie verschwunden waren. Oberbauch-Sono, CT-Thorax (Ausschluß Lungenembolie), EKG unauffällig.

ERLÄUTERUNGEN ZUR UAW:

Die Einstufung erfolgte als UAW, da die klinischen Symptome nach Ausschluss anderer Ursachen am ehesten i.S. einer Allergie zu deuten sind. Zitat aus Entlassungsbrief: "Der Pat. hatte wegen Schmerzen eine Tbl. Voltaren eingenommen und kurz danach bekam er Beklemmungsgefühle und Juckreiz." Der zeitliche Faktor untermauert hier das Vorliegen einer UAW - bedingt durch die Einnahme von Voltaren.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: VOLTAREN

BETREFFENDE UAW: Allergie

ERLÄUTERUNG: Allergien unter Voltaren sind gut dokumentierte Nebenwirkung. Chronologischer Zusammenhang macht eine UAW plausibel.

NOVARTIS, 06.05.2010 DE-002147023-PHHY2009DE01344

Sender diagnosis: not reported

Narrative

Initial report received from BfArM (DE-BFARM-09003370): This patient initiated Voltaren (diclofenac sodium) 50 mg, oral therapy on 22 Sep 2008 and took once only. On 22 Sep 2008, the patient experienced sense of oppression, pruritus and sweating. The event recovered on 23 Sep 2008. Event causality was possible.

Follow-up information received from regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE-BFARM-09003370) on 23 Apr 2010. This patient had concurrent condition of type 2 diabetes mellitus, hypercholesterolemia, hypertension arterial and angina pectoris. The patient started treatment with Voltaren 50 mg once a day for pain across chest on 22 Sep 2008. On 22 Sep 2008 the patient experienced allergy, with sense of oppression, pruritus and sweating attack. The patient also had increased total white blood cell count (13.3 Gpt/l, Normal 3.8-10 Gpt/l), fibrinogen (8.9 g/l, Normal 1.5-4.5 g/l), aspartate aminotransferase (1.29 umol/l, Normal less than 0.62 umol/l), alanine aminotransferase (1.65 umol/l, Normal less than 0.68 umol/l) and alkaline phosphatase (2.81 umol/l, Normal 0.67- 2.50 umol/l). The outcome of allergy, with sense of oppression, pruritus and sweating attack was reported to be complete recovery. Outcome for the other events not reported. In the absence of the reporting healthcare professional causality, the Novartis Medical Safety Physician has provisionally assessed the event as suspected based on the current available information.

In the absence of the reporting healthcare professional causality, the Novartis Medical Safety Physician has provisionally assessed the event as suspected based on the current available information.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09013015

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Medication error					Unknown
3 Black stools					Unknown
4 Gastric bleeding					Unknown
5 INR increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 pradaxa		220 Mg milligram(s)/Day	-	
dabigatran etexilate			1 Week	
2 voltaren		/	-	
diclofenac sodium			1 Week	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal insufficiency			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 02.02.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-01020DE

Sender diagnosis: not reported

Narrative

This is a spontaneous health-care professional confirmed report from Germany.

On 29Jan2009 information has been received from a physician via direct contact with a company sales representative.

A <75-year-old female patient suffered from acute gastric bleeding, INR 6.1 and for two days black stool one week after start of therapy with Paradaxa (dabigatran) 2x110 mg/day and Voltaren (diclofenac) for an unknown indication. Patient was transferred in an emergency hospital due to acute gastric bleeding. During transferring INR 6.1 was diagnosed, genesis unclear. Renal function during entry in hospital was normal.

Information on the treatment of the events, on the action taken regarding the suspect drugs, on the outcome of the events, on co-medication, on co-diseases, on (relevant) medical history was not provided.

The reporting physician rated the causal relationship between the event acute gastric bleeding and Pradaxa and Voltaren as definite.

29Jan2009 - initial via e-mail from sales representative and via phone call from physician

No change in the company core safety information or other measures are recommended at this time.

GEIERH

Case assessment created by GEIERH on 29 Jan 2009

Reporter's comment:

Sender diagnosis: not reported

Narrative

This is a spontaneous health-care professional confirmed report from Germany.

On 09Feb2009 follow-up information has been received from a physician via direct contact with a company sales representative.

A female patient (born 1927) suffered from acute gastric bleeding, INR 6.1 and for two days black stool one week after start of therapy with Paradaxa (dabigatran etexilate) 2x110 mg/day and Voltaren (diclofenac) for an unknown indication. Patient was transferred in an emergency hospital due to acute gastric bleeding. During transferring INR 6.1 was diagnosed, genesis unclear. Renal function during entry in hospital was normal. Co-disease included renal insufficiency. The reporting physician stated that she has made an error concerning the Pradaxa medication (medication error). The physician stated that the patient received Pradaxa in overdose.

Information on the treatment of the events, on the action taken regarding the suspect drugs, on the outcome of the events, on co-medication, on (relevant) medical history was not provided.

The reporting physician rated the causal relationship between the event acute gastric bleeding and Pradaxa and Voltaren as definite.

29Jan2009 - initial via e-mail from sales representative and via phone call from physician

09Feb2009 - Follow-up via e-mail from sales representative

Reporter's comment:

Sender diagnosis: not reported

Narrative

This is a spontaneous health-care professional confirmed report from Germany.

On 09Feb2009 follow-up information has been received from a physician via direct contact with a company sales representative.

A female patient (born 1927) suffered from acute gastric bleeding, INR 6.1 and for two days black stool one week after start of therapy with Paradaxa (dabigatran etexilate) 2x110 mg/day and Voltaren (diclofenac) for an unknown indication. Patient was transferred in an emergency hospital due to acute gastric bleeding. During transferring INR 6.1 was diagnosed, genesis unclear. Renal function during entry in hospital was normal. Co-disease included renal insufficiency. The reporting physician stated that she has made an error concerning the Pradaxa medication (medication error). The physician stated that the patient received Pradaxa in overdose.

Information on the treatment of the events, on the action taken regarding the suspect drugs, on the outcome of the events, on co-medication, on (relevant) medical history was not provided.

The reporting physician rated the causal relationship between the event acute gastric bleeding and Pradaxa and Voltaren as definite.

29Jan2009 - initial via e-mail from sales representative and via phone call from physician

09Feb2009 - Follow-up via e-mail from sales representative

13Feb2009 Follow-up due to request - MedDRA changed

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09016278

Serious Yes	Death	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19321001	Age: 76 Year	weight (kg): 81	BMI_int: 26.15	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea					Unknown
2 Yellow ocular coloring					Unknown
3 Faeces pale					Unknown
4 Urine discoloration					Unknown
5 Malaise					Unknown
6 Dizziness					Unknown
7 Fever					Unknown
8 Cholestasis	20081015				recovering/resolving
9 Sepsis NOS	20081015				recovering/resolving
10 Hepatitis toxic drug-induced	20081015				recovering/resolving
11 Icterus					Unknown
12 Necrosis					Unknown
13 Sepsis					recovering/resolving
14 Hepatitis toxic drug-induced	20081015	200811	1 Month		recovering/resolving
15 Itching					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure 100 mg acetylsalicylic acid	Oral	/	-	Coronary artery disease
2 METHOTREXATE methotrexate		/	-	Seronegative rheumatoid arthritis
3 Rewodina dual 75 Kaps diclofenac sodium	Oral	/	-	Seronegative rheumatoid arthritis

Drugs (concomitant)

4 berlinsulin h insulin; insulin human injection, isophane	Subcutaneous	/	-	Type 2 diabetes mellitus
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ulcus cruris			Unknown	
Metabolic disorder			Unknown	Risk factor
Diabetic nephropathy			Unknown	
Diabetes mellitus			Unknown	Type II. Current condition.
Skin ulcer			Unknown	Current condition
Erythropoiesis abnormal			Unknown	
Type 2 diabetes mellitus			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Haemoglobin	20081008	7.60 mmol/l	7.60 mmol/l
Haemoglobin	20081013	7.40 mmol/l	7.40 mmol/l
Haemoglobin	20081017	7.00 mmol/l	7.00 mmol/l
Haematocrit	20081008	0.36 l/l	0.36 l/l
Haematocrit	20081013	0.32 l/l	0.32 l/l
Haematocrit	20081017	0.31 l/l	0.31 l/l
Lipase	20081008	1.89 µmol/sl	1.89 µmol/s
Lipase	20081013	1.68 µmol/sl	1.68 µmol/s
Bilirubin total	20081008	510.90 µmol/l	510.90 µmol/l
Bilirubin total	20081013	494.30 µmol/l	494.30 µmol/l
Bilirubin total	20081017	436.90 µmol/l	436.90 µmol/l
Bilirubin total	20081021	305.30 µmol/l	305.30 µmol/l
GGT	20081008	12.93 µmol/sl	12.93 µmol/s
GGT	20081013	6.08 µmol/sl	6.08 µmol/s
GGT	20081017	3.28 µmol/sl	3.28 µmol/s
GGT	20081021	2.97 µmol/sl	2.97 µmol/s
Creatinine	20081008	169 µmol/l	169 µmol/l
C-reactive protein	20081008	72.5 mg/l	72.5 mg/l
C-reactive protein	20081013	35.6 mg/l	35.6 mg/l
C-reactive protein	20081017	62.7 mg/l	62.7 mg/l
Activated partial thromboplastin time	20081021	46.0	NA
Alanine aminotransferase	20081013	7.12	umol/sl
Alanine aminotransferase	20081017	5.18	umol/sl
Alanine aminotransferase	20081021	4.85	umol/sl
Alanine aminotransferase	20081008	13.20	umol/sl
Aspartate aminotransferase	20081013	6.86	umol/sl
Aspartate aminotransferase	20081017	5.07	umol/sl
Aspartate aminotransferase	20081021	4.54	umol/sl
Aspartate aminotransferase	20081008	16.60	umol/sl
Blood alkaline phosphatase	20081021	4.08	umol/sl
Blood alkaline phosphatase	20081013	3.21	umol/sl
Blood alkaline phosphatase	20081017	2.98	umol/sl
Blood alkaline phosphatase	20081008	3.93	umol/sl
Blood bilirubin	20081021	305.30	umol/l
Blood bilirubin	20081008	510.90	umol/l
Blood bilirubin	20081017	436.90	umol/l
Blood bilirubin	20081013	494.30	umol/l
Blood creatine	20081008	169	umol/l
Blood urea	20081017	9.1	mmol/l
Blood urea	20081013	9.4	mmol/l
Blood urea	20081008	12.0	mmol/l
Hepatitis A antibody	20081021	Increased	NA
Liver biopsy		Drug induced toxic liver damage was confirmed	
Platelet count	20081013	444	Gpt/l

Platelet count	20081008	426	Gpt/l
Red blood cell count	20081013	4.02	Tpt/l
Red blood cell count	20081017	3.63	Tpt/l
Red blood cell count	20081008	4.18	Tpt/l
Serum ferritin	20081015	748	ug/l
White blood cell count	20081013	12.9	Gpt/l
White blood cell count	20081008	15.1	Gpt/l
Ultrasonography	Abdominal sonography did not reveal any findings		

Narratives (nur bei elektronischem Eingang)

PLIVA Research and Development Ltd., 09.02.2009 DE-PLIVA Research and Development Ltd.-107#01#2009-00392

Sender diagnosis: not reported

Narrative

This case was received from a health professional via hospital report on 29-JAN-2009.

A 76-year-old male patient was hospitalized from 08-OCT-2008 till 24-OCT-2008 for drug-induced toxic hepatitis with icterus, cholestasis with beginning of deformation, necrosis, nausea, itching, malaise, pale faeces, dizziness and discolored urine. The patient was treated with Rewodina dual 75 (diclofenac) and methotrexate (brand/trade name unknown) both for seronegative rheumatoid arthritis and with acetylsalicylic acid 100 (brand/trade name unknown) for coronary artery disease. All drugs were discontinued. The patient did not drink alcohol. Concomitantly the patient was treated with Berlinsulin (insulin human) for type 2 diabetes mellitus. Concomitant disease included toxic damage of erythropoiesis, ulcus cruris and diabetic nephropathy.

Viral hepatitis (A,B,C) was excluded. Abdominal sonography did not reveal any findings. Drug induced toxic liver damage was confirmed by liver biopsy. Treatment with colestyramine for itching, Rocephin (ceftriaxone sodium) for fever resulting from sepsis, Tavanic (levofloxacin), prednisolon for arthritis, Hepa-Merz (ornithine aspartate) and novaminsulfon as necessary was started. The patient was discharged with recommendation to avoid methotrexate and NSAR.

The outcome was stated as recovering. The causality was reported as probable for Rewodina dual 75 and as possible for methotrexate and acetylsalicylic acid.

The result of severe toxic liver damage. Sepsis is assumed to be the result of patient's debilitated condition due to the liver function disorder as well as underlying diseases.

Literature search in PubMed did not reveal further specific cases on the suspected drugs and sepsis. One further case on cholestasis and diclofenac was found in PubMed. No case reports were found in drug safety database on suspected drugs and unlisted ADRs.

Risk minimizing measures are not necessary.

Reporter's comment:

ACTAVISPR, 18.03.2009 DE-ACTAVISPR-ACTA009071

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a license partner and originated from Germany.

A 76-year-old male patient with a risk factor significant for metabolic disorder and a medical history significant for skin ulcer, diabetic nephropathy and diabetes mellitus type II, ulcus cruris, toxic damage of erythropoiesis, experienced drug-induced toxic hepatitis, cholestasis and sepsis with acinetobacter whilst being treated with acetylsalicylic acid 100 (unit not specified) at a dose of one DF (frequency unspecified) for coronary artery disease, together with diclofenac at a dose of one DF (strength 75 mg, frequency unspecified) and methotrexate at an unknown daily dosage, both for rheumatoid arthritis. Concomitant medications included insulin human/isophane (Berlinsulin). All suspect drugs had been commenced on an unknown date and on 8 October 2008, after an unspecified period of therapy, the patient was hospitalized for drug-induced toxic hepatitis with icterus, cholestasis with beginning of deformation, necrosis, nausea, itching, malaise, pale faeces, dizziness, discoloured urine and sepsis with acinetobacter. Viral hepatitis (A, B, C) was excluded. Abdominal sonography did not reveal any findings. The drug induced liver damage was confirmed by a liver biopsy. All suspect drugs were discontinued. The patient was then treated with ceftriaxone, levofloxacin, prednisolone, colestyramine, ornithine and dipyrone. He was discharged on 24 October 2008. At the time of reporting, the patient was recovering. No further information is available.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09031352

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19320210	Age: 76 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Subileus	20090217	20090319	31 Day		recovering/resolving
2 Petechiae	20090219				recovering/resolving
3 Glomerulonephritis					recovering/resolving
4 Vasculitis	200902				recovering/resolving
5 Skin biopsy	200902				Unknown
6 Ascites	20090217				recovering/resolving
7 Melena	20090217				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	

Drugs (concomitant)

2 calcilac	Oral	/	20090212-	Osteoporosis prophylaxis
folic acid			-	
3 DIGIMERCK		/	-	
digitoxin			-	
4 FENTANYL		/	-	
fentanyl			-	
5 Hydrochlorothiazid		/	-	
hydrochlorothiazide			-	
6 PANTOZOL	Oral	/	20090212-	
pantoprazole sodium			-	
7 RAMIPRIL		/	-	
ramipril			-	
8 TETRAZEPAM		/	-	
tetrazepam			-	
9 TORASEMIDE	Oral	/	-	
torasemide			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cardiac insufficiency			Unknown	
Infection MRSA			Unknown	
Polymyositis			Unknown	
Ulcer skin			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
CT scan		NA	NA	

Narratives (nur bei elektronischem Eingang)NOVARTIS, 06.03.2009 DE-002147023-PHHY2009DE06534

Sender diagnosis: not reported

Narrative

Initial report received from the clinician on 20 Feb 2009: This patient was treated with a diclofenac-containing drug (manufacturer unstated) for an unknown time period. On 19 Feb 2009, the patient was transferred from one hospital to the reporting hospital, after the diagnosis of CT-proven ileus (large and small intestines) with melena and ascites. Diagnostics were still ongoing, but any mechanical reason for ileus was ruled out. The reporter added that the diagnosis of vasculitis was also made some days ago (Feb 2009), via skin biopsy. The patient was set on corticoid therapy. No causality assessment with respect to diclofenac was given. The outcome of the event was not reported.

The available information was considered inadequate to fully assess the case.

Serious spontaneous report, [hospitalisation], Vasculitis, Ileus, Ascites assessed as unlisted according to the Core Data Sheet.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Serious spontaneous report, [hospitalisation], Melena already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 23.04.2009 DE-002147023-PHHY2009DE06534

Sender diagnosis: not reported

Narrative

Initial report received from the clinician on 20 Feb 2009: This patient was treated with a diclofenac-containing drug (manufacturer unstated) for an unknown time period. On 19 Feb 2009, the patient was transferred from one hospital to the reporting hospital, after the diagnosis of CT-proven ileus (large and small intestines) with melena and ascites. Diagnostics were still ongoing, but any mechanical reason for ileus was ruled out. The reporter added that the diagnosis of vasculitis was also made some days ago (Feb 2009), via skin biopsy. The patient was set on corticoid therapy. No causality assessment with respect to diclofenac was given. The outcome of the event was not reported.

Follow-up information received on 15 Apr 2009: The physician could not clearly remember the individual case, but she will further investigate on it. She stated that there may have been a case of [suspicion of vasculitis], from her memory, but she was uncertain on that.

provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Serious spontaneous report, [hospitalisation], Melena already listed in the Core Data Sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 30.04.2009 DE-002147023-PHHY2009DE06534

Sender diagnosis: not reported

Narrative

Initial report received from the clinician on 20 Feb 2009: This patient was treated with a diclofenac-containing drug (manufacturer unstated) for an unknown time period. On 19 Feb 2009, the patient was transferred from one hospital to the reporting hospital, after the diagnosis of CT-proven ileus (large and small intestines) with melena and ascites. Diagnostics were still ongoing, but any mechanical reason for ileus was ruled out. The reporter added that the diagnosis of vasculitis was also made some days ago (Feb 2009), via skin biopsy. The patient was set on corticoid therapy. No causality assessment with respect to diclofenac was given. The outcome of the event was not reported.

Follow-up information received on 15 Apr 2009: The physician could not clearly remember the individual case, but she will further investigate on it. She stated that there may have been a case of [suspicion of vasculitis], from her memory, but she was uncertain on that.

Follow-up information received from physician on 20 Apr 2009: The event ileus was deleted based upon follow-up information received. The patient had concurrent condition of polymyositis and large toe ulcer with evidence of MRSA and cardiac insufficiency. On 19 Feb 2009, patient presented petechiae, predominating on the extremities. Subileus (not ileus) and glomerulonephritis, lasting until 19 Mar 2009, were reported. The conditions improved after stopping therapy. The reporter noted that there was no certain evidence of drug-induced vasculitis, but treatment with from 19 Feb 2009 Prednisolon (prednisolone) led to improvement. Outcome was reported as condition improving. Causality was reported as suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09030684

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19240327	Age: 84 Year	weight (kg): 100	BMI_int: 30.86	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time decreased					Unknown
2 Pyloric ulcer					recovered/resolved
3 Stool black	20080921			5 Day	recovered/resolved
4 Weakness	20080925			9 Day	recovered/resolved
5 Stool odor abnormal	20080921			5 Day	recovered/resolved
6 International normalised ratio increased	20080925	20080926	2 Day	9 Day	recovered/resolved
7 Dizzy	20080925			9 Day	recovered/resolved
8 Creatinine high	20080925	20080930	6 Day	9 Day	recovered/resolved
9 Ulcer haemorrhage	20080921	20080925	5 Day	5 Day	recovered/resolved
10 Red blood cell count decreased	20080926	20081006	11 Day	10 Day	recovered/resolved
11 Helicobacter pylori identification test positive					Unknown
12 Blood urea increased	20080925	20080930	6 Day	9 Day	recovered/resolved
13 Activated partial thromboplastin time increased	20080925	20080926	2 Day	9 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 FALITHROM phenprocoumon	Oral	7* / Week 82	1998-20080925 -	Thrombosis prophylaxis
2 VOLTAREN RESINAT "NOVARTIS" diclofenac resinate	Oral	2*75 Mg milligram(s)/ Day 5	20080917-20080925 9 Day	Pain NOS

Drugs (concomitant)

3 DETRUSITOL tolterodine l-tartrate	Oral	2 Mg milligram(s)/ Day 82	2006- -	Incontinence
4 DILTAHEXAL diltiazem hydrochloride	Oral	2*60 Mg milligram(s)/ Day 82	2006- -	Hypertension
5 FOSAMAX alendronate sodium	Oral	70 Mg milligram(s)/Week 245	2007- -	Osteoporosis
6 LORZAAR PLUS hydrochlorothiazide; losartan potassium	Oral	2*1 Df dosage form/ Day 82	2006- -	Hypertension
7 PENTALONG 50 pentaerithrityl tetranitrate	Oral	2*25 Mg milligram(s)/ Day 245	2006-20080925 -	Angina pectoris

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hip replacement			Yes	Z.n. Hüft -TEP beidseitig
Incontinence			Unknown	
Hypertension	1998		Yes	
Pulmonary embolism	1998	1998	No	Z.n.
Thrombosis leg	1998	1998	No	Z.n. Beinvenenthrombose
Osteoporosis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20080925	129	µmol/l	
Red blood cell count	20080925	2.6	Tpt/l	
Haemoglobin	20080925	4.4	mmol/l	
Haematocrit	20080925	0.22	NA	
Thrombocyte count	20080925	333 Gpt/l	Gpt/l	
Quick's test	20080925	8	Prozent	
INR	20080925	5.5	NA	
Activated partial thromboplastin time	20080925	94	mmol/l	
Red blood cell count	20080926	3.1	Tpt/l	
Haemoglobin	20080926	5.5	mmol/l	
Haematocrit	20080926	0.26	NA	
Thrombocyte count	20080926	339 Gpt/l	Gpt/l	
Quick's test	20080926	80	Prozent	
INR	20080926	1.1	NA	
Creatinine	20080930	75	µmol/l	
INR	20080929	1.2	NA	
Quick's test	20080927	72	Prozent	
Red blood cell count	20081006	4.1	Tpt/l	
Haemoglobin	20081006	7	mmol/l	
Haematocrit	20081006	0.35	NA	
Thrombocyte count	20081006	350 Gpt/l	Gpt/l	
Aspartate aminotransferase	20081010	0.48	µmol/l*s	
Alanine aminotransferase	20081010	0.44	µmol/l*s	
Gamma GT	20081010	0.25	µmol/l*s	
Activated partial thromboplastin time	20080925	94	mmol/l	
Activated partial thromboplastin time	20080926	32	mmol/l	
Urea	20080925	23.7	mmol/l	
Urea	20080926	#	mmol/l	
Urea	20080930	4.9	mmol/l	
Activated partial thromboplastin time	20080927	29	mmol/l	
Creatinine	20080926	#	umol/l	
Gamma GT	20081010	0.25	umol/l	

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 11.03.2009 DE-PVZIBE-JENA-10024

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
 Ulkusblutung bei Falithromüberdosierung

KURZZUSAMMENFASSUNG:

Ulkusblutung mit Teerstuhl bei Quickerniedrigung <8% unter Falithrom, Voltaren und Hp-Nachweis (Alternativursache).

BESCHREIBUNG:

Seit 21.09.2008 auffällig dunkler Stuhlgang, übelriechend. Am 24.09.2008 erstmals Blut dabei. Am 25.09.2008 (Aufnahmetag) sehr schwindelig und kraftlos gefühlt, daraufhin Notarzt geholt (10:00 Uhr).

THERAPIE:

ZNA: PPSB (2000 i.E.). 25. und 26.09.2008 O-Diät; Pantozol 40 2xd (zunächst i.v., dann oral). 25.09.08 2 Eks; Falithrom pausiert; Voltaren abgesetzt. 26.09.2008 1 Amp Konakion. 28.09.2008 2 Eks. Ab 05.10.2008 Eradikationstherapie mit Clarithromycin 250 1-0-1 und Metronidazol 400 1-0-1 (im Arztbrief kein Procedere enthalten). Zur Entlassung Falithrom pausiert, Kontroll-Gastroskopie in 4 Wochen empfohlen, Clexane zu Antikoagulation.

VERLAUF:

RR: 25.09. 140/60. 26.09. 135/75. Ab 27.09. 170/80. 25.09.08 Gastroskopie: Petechien im Magen, Ulcus im Antrum ventriculi Forrest III und Ulcus am Pylorus Forrest II b.. Ab 27.09. 170/80. 27.09.08 Teerstuhl (flüssig). 29.09.08 Gastroskopie: Intra- und präpylorisch fibrinbelegtes Ulcus mit Abheilungs-Tendenz. An der Angulusfalte zweites Ulcus mit stattgehabter Blutung. 30.09.08 Clo-Test neg. (Hp: negativ). Koloskopie 30.09.08: kein Hinweis auf entzündlichen oder malignen Prozess. Keine Blutungsquelle im unteren GIT. ab 30.09.08 kein Teerstuhl mehr. Histologie (05.10.08): In beiden Präparaten lässt sich eine Besiedelung der Magenschleimhaut durch Helicobacter pylori nachweisen. Keine Malignität.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: VOLTAREN RESINAT "NOVARTIS"

BETREFFENDE UAW: Ulcus mit Blutung

ERLÄUTERUNG: C3, da sehr starker zeitlicher Zusammenhang ==> Blutungsbeginn wenige Tage nach Therapiebeginn. S2, da Hp-Nachweis als Alternativursache gilt.

MEDIKAMENT: FOSAMAX

BETREFFENDE UAW: Ulcus mit Blutung

ERLÄUTERUNG: NW: GIT-Beschwerden im oberen Bereich (lokale Irritation) --> in diesem Fall Ulcera im unteren Magen. Auch zeitlich Zusammenhang nicht zwingend. Einnahme Mittwoch, Blutung Sonntag ==> Präparat nicht verdächtig.

MEDIKAMENT: FALITHROM

BETREFFENDE UAW: Ulcus mit Blutung

ERLÄUTERUNG: S3 trotz Alternativursache Ulcera bei Hp da positives Labor. C3, da nach Wiederherstellung der Gerinnung keine GIT-Blutung mehr (trotz noch vorhandener Ulcera).

NOVARTIS, 20.03.2009 DE-002147023-PHHY2009DE09335

Sender diagnosis: not reported

Narrative

Initial report received from Health Authority (HA reference number : DE-BFARM-09030684) on 11 Mar 2009: The patient had concurrent condition of hypertension, hip replacement, osteoporosis and incontinence and historical condition of thrombosis leg and pulmonary hypertension. The patient was on Voltaren Resinat (diclofenac resinate) capsule from 17 Sep 2008 for pain. On 21 Sep 2008 the patient had noticeably dark stool which was foul-smelling. On 24 Sep 2008 there was first appearance of blood and on 25 Sep 2008 the patient felt very dizzy and weak. The patient was admitted to hospital on 25 Sep 2009. On 25 Sep 2008, gastroscopy revealed petechiae in stomach, ulcer in antrum ventriculi Forrest III and ulcer at the pylorus Forrest IIb. A diagnosis of ulcer haemorrhage was made on 21 Sep 2009. The outcome of which was reported to be complete recovery. The patient was treated with PPSB (blood product containing prothrombin, proconvertin, Stuart-Power factor and antihemophilic factor B) (2000 I.U.). On 25 Sep 2008 patient was prescribed Pantozol (pantoprazole) 40mg, 2 times daily first IV the through oral route. On 25 Sep 2008 2 erythrocyte concentrates was given and Falithrombin was temporarily discontinued. Therapy with Voltaren Resinat was discontinued on 26 Sep 2008 and 1 ampoule Konakion was administered. On 28 Sep 2008 2 erythrocyte concentrations was again administered. On 05 Oct 2008, eradication therapy with clarithromycin 250 BID and metronidazole 400 was started. From 30 Sep 2008 there was no more tarry stool. Histology on 05 Oct 2008: revealed colonies of Helicobacter pylori. The patient also had increased blood creatinine (129 umol/l), blood urea (23.7 mmol/L), activated partial thromboplastin time (94 mmol/l) and international normalized ratio (5.5) and decreased prothrombin time (8) the outcome of which was not reported.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements

Reporter's comment:

HELIOS Klinikum Wuppertal, 12.11.2010 DE-PVZIBE-JENA-10024

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
 Ulkusblutung bei Falithromüberdosierung

KURZZUSAMMENFASSUNG:

Ulkusblutung mit Teerstuhl bei Quickeniedrigung <8% unter Falithrom, Voltaren und Hp-Nachweis (Alternativursache).

BESCHREIBUNG:

Seit 21.09.2008 auffällig dunkler Stuhlgang, übelriechend. Am 24.09.2008 erstmals Blut dabei. Am 25.09.2008 (Aufnahmetag) sehr schwindelig und kraftlos gefühlt, daraufhin Notarzt geholt (10:00 Uhr).

THERAPIE:

ZNA: PPSB (2000 i.E.). 25. und 26.09.2008 O-Diät; Pantozol 40 2xd (zunächst i.v., dann oral). 25.09.08 2 Eks; Falithrom pausiert; Voltaren abgesetzt. 26.09.2008 1 Amp Konaktion. 28.09.2008 2 Eks. Ab 05.10.2008 Eradikationstherapie mit Clarithromycin 250 1-0-1 und Metronidazol 400 1-0-1 (im Arztbrief kein Procedere enthalten). Zur Entlassung Falithrom pausiert, Kontroll-Gastroskopie in 4 Wochen empfohlen, Clexane zu Antikoagulation.

VERLAUF:

RR: 25.09. 140/60. 26.09. 135/75.. Ab 27.09. 170/80. 25.09.08 Gastroskopie: Petechien im Magen, Ulcus im Antrum ventriculi Forrest III und Ulcus am Pylorus Forrest II b.. Ab 27.09. 170/80. 27.09.08 Teerstuhl (flüssig). 29.09.08 Gastroskopie: Intra- und präpylorisch fibrinbelegtes Ulcus mit Abheilungs-Tendenz. An der Angulusfalte zweites Ulcus mit stattgehabter Blutung. 30.09.08 Clo-Test neg. (Hp: negativ). Koloskopie 30.09.08: kein Hinweis auf entzündlichen oder malignen Prozess. Keine Blutungsquelle im unteren GIT. ab 30.09.08 kein Teerstuhl mehr. Histologie (05.10.08): In beiden Präparaten lässt sich eine Besiedelung der Magenschleimhaut durch Helicobacter pylori nachweisen. Keine Malignität.

ERLÄUTERUNGEN ZUR UAW:

Obere GIT-Blutung mit Kreislaufinstabilität, was schließlich zur KH-Aufnahme führte. Blutung a.e. unter Falithrom bei Ulcus unter Voltaren, wobei H.p.Gastritis als Risikofaktor bzw. Alternativursache infrage kommt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: voltaren resinat

BETREFFENDE UAW: Ulcus mit Blutung

ERLÄUTERUNG: C3, da sehr starker zeitlicher Zusammenhang ==> Blutungsbeginn wenige Tage nach Therapiebeginn. S2, da Hp-Nachweis als Alternativursache gilt.

MEDIKAMENT: FALITHROM

BETREFFENDE UAW: Ulcus mit Blutung

ERLÄUTERUNG: S3 da Blutung a.e. auf Falithrom zurückzuführen ist. Ulcus bei H.p bzw. unter Voltaren zwar Quelle, aber nicht Ursache der unkontrollierten Blutung. C3, da nach Wiederherstellung der Gerinnung keine GIT-Blutung mehr (trotz noch vorhandener Ulcera).

NOVARTIS, 24.11.2010 DE-002147023-PHHY2009DE09335

Sender diagnosis: not reported

Narrative

Initial report received from Health Authority (HA reference number : DE-BFARM-09030684) on 11 Mar 2009: The patient had concurrent condition of hypertension, hip replacement, osteoporosis and incontinence and historical condition of thrombosis leg and pulmonary hypertension. The patient was on Voltaren Resinat (diclofenac resinate) capsule from 17 Sep 2008 for pain. On 21 Sep 2008 the patient had noticeably dark stool which was foul-smelling. On 24 Sep 2008 there was first appearance of blood and on 25 Sep 2008 the patient felt very dizzy and weak. The patient was admitted to hospital on 25 Sep 2009. On 25 Sep 2008, gastroscopy revealed petechiae in stomach, ulcer in antrum ventriculi Forrest III and ulcer at the pylorus Forrest IIb. A diagnosis of ulcer haemorrhage was made on 21 Sep 2009. The outcome of which was reported to be complete recovery. The patient was treated with PPSB (blood product containing prothrombin, proconvertin, Stuart-Power factor and antihemophilic factor B) (2000 I.U.). On 25 Sep 2008 patient was prescribed Pantozol (pantoprazole) 40mg, 2 times daily first IV the through oral route. On 25 Sep 2008 2 erythrocyte concentrates was given and Falithrombin was temporarily discontinued. Therapy with Voltaren Resinat was discontinued on 26 Sep 2008 and 1 ampoule Konikion was administered. On 28 Sep 2008 2 erythrocyte concentrations was again administered. On 05 Oct 2008, eradication therapy with clarithromycin 250 BID and metronidazole 400 was started. From 30 Sep 2008 there was no more tarry stool. Histology on 05 Oct 2008: revealed colonies of Helicobacter pylori. The patient also had increased blood creatinine (129 umol/l), blood urea (23.7 mmol/L), activated partial thromboplastin time (94 mmol/l) and international normalized ratio (5.5) and decreased prothrombin time (8) the outcome of which was not reported.

Follow up report received from Health Authority (HA reference number: DE-BFARM-09030684) on 12 Nov 2010: Treatment with Voltaren Resinate was discontinued on 25 Sep 2008. On 26 Sep 2008 red blood cell count was 3.1 Tpt/l. The outcome of the red blood cell count decreased was reported as complete recovery on 06 Oct 2008.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements.

Based on Review of available reported data does not allow for a proper causality assessment. Reported event(s) is (are) unassessable

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09031387

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19380906	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Activated partial thromboplastin time increased					Unknown
2 Diarrhea	20081103	20081124	22 Day	22 Day	recovered/resolved
3 Gastro intestinal bleed	20081123	20081124	2 Day		recovered/resolved
4 Colitis	20081103	20081125	23 Day		recovered/resolved
5 Urea increased					Unknown
6 Prothrombin time decreased					Unknown
7 International normalised ratio increased		20081125			recovered/resolved
8 Gamma GT increased					Unknown
9 Fibrinogen increased					Unknown
10 Alanine aminotransferase increased					Unknown
11 Colitis					Unknown
12 Creatinine increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 FALITHROM phenprocoumon	Oral	1.5 Mg milligram(s)/Day 82	1994-20081123 -	Atrial fibrillation
2 MOBEC meloxicam	Oral	7.5 Mg milligram(s)/Day 245	1984- -	Uveitis anterior
3 voltaren resinat diclofenac resinate	Oral	75 Mg milligram(s)/Day 5	20081013-20081123 42 Day	Fracture

Drugs (concomitant)

4 LEVOPAR benserazide hydrochloride; levodopa	Oral	125 Mg milligram(s)/Day 5	2000- -	Neuropathy
5 l-thyroxin levothyroxine sodium	Oral	50 µg microgram(s)/Day 245	1991- -	Strumectomy
6 PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/Day 245	1994- -	Reflux oesophagitis
7 PROVAS buflomedil hydrochloride	Oral	320 Mg milligram(s)/Day 82	200805- -	Hypertension arterial
8 SIMVASTATIN simvastatin	Oral	10 Mg milligram(s)/Day 82	- -	Hyperlipidemia
9 XIPAMIDE xipamide	Oral	10 Mg milligram(s)/Day 245	2000- -	Edema lower limb

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arrhythmia absoluta	1994		Yes	bei Vorhofflimmern
Duodenal ulcer	1985	1985	No	
Neuropathy	2000		Yes	der Beine
Fracture	200810		Yes	Brustwirbelkörper
Uveitis anterior	1984		Yes	chronisch
Hypertension arterial	1968		Yes	
Strumectomy			No	
Arthritis rheumatoid	1984		Yes	Z.n. Ellenbogenbestrahlung 2006
Hyperlipidemia			Yes	Erkrankungsbeginn unbekannt
Hypothyroidism	1955		No	Strumaresektion 1991
Reflux oesophagitis	1994		No	
Edema lower limb			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Haemoglobin	20081124	8.81	mmol/l	
Haematocrit	20081124	0.42	NA	
Mean corpuscular haemoglobin	20081124	89.1	fl	
Total white blood cell count	20081124	7.0	Gpt/l	
Thrombocyte count	20081124	328	Gpt/l	
Quick's test	20081124	32.1	Prozent	
International normalised ratio	20081124	2.14	NA	
Partial prothrombin time	20081124	45.2	sec	
Fibrinogen	20081124	5.04	g/l	
Sodium	20081124	142	mmol/l	
Potassium	20081124	3.75	mmol/l	
Chloride	20081124	106	mmol/l	
Calcium	20081124	2.53	mmol/l	
Creatinine	20081124	102	µmol/l	
Urea	20081124	10.8	mmol/l	
Aspartate aminotransferase	20081124	0.58	µmol/s.l	
Alanine aminotransferase	20081124	0.62	µmol/s.l	
Alkaline phosphatase	20081124	0.79	µmol/l	
Gamma GT	20081124	0.63	µmol/s.l	
Quick's test	20081125	58.9	Prozent	
International normalised ratio	20081125	1.29	NA	
Partial prothrombin time	20081125	37.3	sec	

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 11.03.2009 DE-PVZIBE-WEIMAR-10142

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

untere gastrointestinale Blutung

KURZZUSAMMENFASSUNG:

Untere gastrointestinale Blutung unter Phenprocoumon und Diclofenac.

BESCHREIBUNG:

Patientin wurde mit seit dem 03.11.08 rezidivierend auftretenden Diarrhoen (bis zu 5xtgl.), die seit dem 23.11.08 Blutbeimengungen enthielten, aufgenommen. Sie steht unter Phenprocoumontherapie und erhielt aufgrund einer Brustwirbelfraktur seit dem 13.10.08 Diclofenac.

THERAPIE:

- ab 24.11.08: Diclofenac abgesetzt; Phenprocoumon pausiert, ab 27.11.08 erneute Gabe
- 24./25.11.08: Konakion tgl. 1 Amp. i.v.
- ab 26.11.08: Salofalk Klysma 4 mg tgl.

VERLAUF:

- ab 25.11.08 sistieren der Diarrhoe

- 26.11.08 Koloskopie:

Eingesehen bis Zökum, rechte Hemikolon unsauber, daher eingeschränkt beurteilbar, gering ausgeprägte unspezifische Kolitis im Sigma und Colon descendens.

Histologie: Reizzustand mit erhöhter Vulnerabilität, ohne Anhalt für eine chronisch-entzündliche Darmerkrankung oder Malignität.

- Kein Hinweis auf darmpathogene Keime in der mikrobiologischen Stuhluntersuchung.
- 28.11.08: Clostridium difficile Toxin A, B, Noroviren, Rotaviren und Adenovirus AG negativ

ANMERKUNGEN ZU FOLGENDER UAW:

Diarrhoe

KURZZUSAMMENFASSUNG:

Diarrhoe bei Kolitis unter Diclofenac.

BESCHREIBUNG:

Patientin wurde mit seit dem 03.11.08 rezidivierend auftretenden Diarrhoen (bis zu 5xtgl.), die seit dem 23.11.08 Blutbeimengungen enthielten, aufgenommen. Sie steht unter Phenprocoumontherapie und erhielt aufgrund einer Brustwirbelfraktur seit dem 13.10.08 Diclofenac.

THERAPIE:

- ab 24.11.08: Diclofenac abgesetzt; Phenprocoumon pausiert, ab 27.11.08 erneute Gabe
- 24./25.11.08: Konakion tgl. 1 Amp. i.v.
- ab 26.11.08: Salofalk Klysma 4 mg tgl.

VERLAUF:

- ab 25.11.08 sistieren der Diarrhoe

- 26.11.08 Koloskopie:

Eingesehen bis Zökum, rechte Hemikolon unsauber, daher eingeschränkt beurteilbar, gering ausgeprägte unspezifische Kolitis im Sigma und Colon descendens.

Histologie: Reizzustand mit erhöhter Vulnerabilität, ohne Anhalt für eine chronisch-entzündliche Darmerkrankung oder Malignität.

- Kein Hinweis auf darmpathogene Keime in der mikrobiologischen Stuhluntersuchung.
- 28.11.08: Clostridium difficile Toxin A, B, Noroviren, Rotaviren und Adenovirus AG negativ

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: Verdächtig, da es auch zu blutigen Diarrhoen führen kann, außerdem führt es zur Wirkungsverstärkung des Antikoagulanz.

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: Diarrhoe

ERLÄUTERUNG: - Challenge: very suggestive

- Dechallenge: suggestive
- S2, da bakterielle und virale Genese ausgeschlossen

MEDIKAMENT: FALITHROM

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: - Challenge: very suggestive

- Dechallenge: suggestive
- S2, da Rechallenge nicht beurteilbar, da Einnahmezeitraum zu kurz für Erreichung des therapeutischen Bereiches

MEDIKAMENT: MOBEC

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: Aufgrund des sehr langen und beschwerdefreien Einnahmezeitraumes nicht verdächtig, aber trotzdem synergistischer Effekt auf Diclofenac und Phenprocoumon möglich.

MEDIKAMENT: MOBEC

BETREFFENDE UAW: Diarrhoe

ERLÄUTERUNG: Aufgrund des sehr langen und beschwerdefreien Einnahmezeitraumes nicht verdächtig, aber trotzdem synergistischer Effekt auf Diclofenac und Phenprocoumon möglich.<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 23.03.2009 DE-002147023-PHHY2009DE09337

Sender diagnosis: not reported

Narrative

Initial report received from Health Authority (HA reference number: DE-BFARM-09031387) on 12 Mar 2009: The patient had concurrent condition of hypothyroidism, arrhythmia absoluta, hypertension arterial, arthritis rheumatoid, neuropathy, hyperlipidemia, uveitis anterior and edema lower limb and historical condition of urinary infection, ache across chest, strumectomy and reflux esophagitis. The patient was on Voltaren Resinat (diclofenac resinate) for thoracic vertebral fracture from 13 Oct 2008, and was also receiving Falithrom (phenprocoumon) for atrial fibrillation. The patient developed diarrhea from 03 Nov 2008 and lower gastrointestinal bleeding from 23 Nov 2008, and was admitted. Therapy with Voltaren Resinat was stopped on 24 Nov 2008, and Falithrom on 27 Nov 2008. Treatment was with Konakion (phytomenadione) on 24 and 25 Nov 2008, and Salofalk Klysma (mesalazine) since 26 Nov 2008. The outcome of both the event was reported to be complete recovery. Colonoscopy done on 26 Nov 2008 revealed unspecific colitis in sigma and colon descendans, the outcome of which was not reported. Tests done on 24 Nov 2008 revealed increased activated partial prothrombin time (45.2 sec; normal range 26 to 36 sec), international normalized ratio (2.14; normal up to 1.15), fibrinogen (5.04g/l; normal range 1.5 to 4.5g/l), creatinine (102 umol/l; normal up to 97 umol/l), urea (10.8 mmol/l; normal range 1.7 to 8.3 mmol/l), alanine aminotransferase (0.62umol/s.l; normal up to 0.52 umol/s.l) and gamma GT (0.63 umol/s.l; normal up to 0.53 umol/s.l). On the same day, quick's test revealed low prothrombin time (32.1%; normal more than 70%). INR repeated on 25 Nov 2008 was normal (1.29), the outcome of other investigations were not reported.

HELIOS Klinikum Wuppertal, 12.10.2010 DE-PVZIBE-WEIMAR-10142

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Gastrointestinale Blutung

KURZZUSAMMENFASSUNG:

Gastrointestinale Blutung unter Phenprocoumon und Diclofenac sowie Mobec.

BESCHREIBUNG:

Patientin wurde mit seit dem 03.11.08 rezidivierend auftretenden Diarrhoen (bis zu 5xtgl.), die seit dem 23.11.08 Blutbeimengungen enthielten, aufgenommen. Sie steht unter Phenprocoumontherapie und erhielt aufgrund einer Brustwirbelfraktur seit dem 13.10.08 Diclofenac.

THERAPIE:

- ab 24.11.08: Diclofenac abgesetzt; Phenprocoumon pausiert, ab 27.11.08 erneute Gabe
- 24./25.11.08: Konakion tgl. 1 Amp. i.v.
- ab 26.11.08: Salofalk Klysma 4 mg tgl.

VERLAUF:

- ab 25.11.08 sistieren der Diarrhoe
- 26.11.08 Koloskopie:
Eingesehen bis Zökum, rechte Hemikolon unsauber, daher eingeschränkt beurteilbar, gering ausgeprägte unspezifische Kolitis im Sigma und Colon descendans.
Histologie: Reizzustand mit erhöhter Vulnerabilität, ohne Anhalt für eine chronisch-entzündliche Darmerkrankung oder Malignität.
- Kein Hinweis auf darmpathogene Keime in der mikrobiologischen Stuhluntersuchung.
- 28.11.08: Clostridium difficile Toxin A, B, Noroviren, Rotaviren und Adenovirus AG negativ

ERLÄUTERUNGEN ZUR UAW:

Die Ursache der gastrointestinalen Blutung wird in Phenprocoumon und Diclofenac sowie Mobec gesehen.

ANMERKUNGEN ZU FOLGENDER UAW:

Kolitis

KURZZUSAMMENFASSUNG:

Kolitis unter Diclofenac sowie Mobec.

BESCHREIBUNG:

Patientin wurde mit seit dem 03.11.08 rezidivierend auftretenden Diarrhoen (bis zu 5xtgl.), die seit dem 23.11.08 Blutbeimengungen enthielten, aufgenommen. Sie steht unter Phenprocoumontherapie und erhielt aufgrund einer Brustwirbelfraktur seit dem 13.10.08 Diclofenac.

THERAPIE:

- ab 24.11.08: Diclofenac abgesetzt; Phenprocoumon pausiert, ab 27.11.08 erneute Gabe
- 24./25.11.08: Konakion tgl. 1 Amp. i.v.
- ab 26.11.08: Salofalk Klysma 4 mg tgl.

VERLAUF:

- ab 25.11.08 sistieren der Diarrhoe
- 26.11.08 Koloskopie:

Eingesehen bis Zökum, rechte Hemikolon unsauber, daher eingeschränkt beurteilbar, gering ausgeprägte unspezifische Kolitis im Sigma und Colon descendens.

Histologie: Reizzustand mit erhöhter Vulnerabilität, ohne Anhalt für eine chronisch-entzündliche Darmerkrankung oder Malignität.

- Kein Hinweis auf darmpathogene Keime in der mikrobiologischen Stuhluntersuchung.
- 28.11.08: Clostridium difficile Toxin A, B, Noroviren, Rotaviren und Adenovirus AG negativ

ERLÄUTERUNGEN ZUR UAW:

Lt. Entlassungsbefehl wird als Ursache der Kolitis die Einnahme von Diclofenac erwogen.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: voltaren resinat

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: Verdächtig, da es auch zu blutigen Diarrhoen führen kann, außerdem führt es zur Risikoerhöhung für gastrointestinale Blutungen bei Gabe von Antikoaganzien.

MEDIKAMENT: voltaren resinat

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: - Challenge: very suggestive

- Dechallenge: suggestive
- S2, da bakterielle und virale Genese ausgeschlossen

MEDIKAMENT: FALITHROM

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: - Challenge: very suggestive

- Dechallenge: suggestive
- S2, da Rechallenge nicht beurteilbar, da Einnahmezeitraum zu kurz für Erreichung des therapeutischen Bereiches

MEDIKAMENT: MOBEC

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: Aufgrund des sehr langen und beschwerdefreien Einnahmezeitraumes nicht verdächtig, aber trotzdem synergistischer Effekt auf Diclofenac und Phenprocoumon möglich.

MEDIKAMENT: MOBEC

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Aufgrund des sehr langen und beschwerdefreien Einnahmezeitraumes nicht verdächtig, aber trotzdem synergistischer Effekt auf Diclofenac und Phenprocoumon möglich.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09038676

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19300103	Age: 78 Year	weight (kg): 65	BMI_int: 27.06	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gastrointestinal bleeding	20080924	20080929	6 Day		recovered/resolved
2 Blood stool	200809				Unknown
3 Colonoscopy abnormal	20080925				Unknown
4 Gastric inflammation	20080925				Unknown
5 GGTP increase	20080924				Unknown
6 PTT prolonged	20080924				Unknown
7 Perianal bleeding	200809				Unknown
8 Diverticulosis of colon (without mention of haemorrhage)					Unknown
9 Quick's value decreased	20080924				Unknown
10 Gastroscopy abnormal	20080925				Unknown
11 Drug interaction					recovered/resolved
12 Anal bleeding	20080924	20080929	6 Day	3 Year	recovered/resolved
13 Gastrointestinal bleeding	200809				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC diclofenac	Oral	75 Mg milligram(s)/ 5	2005- -	Gonarthrosis
2 FALITHROM "HEXAL" phenprocoumon	Oral	6*1.5 Mg milligram(s)/ We 82	2002-20080924 -	Atrial flutter

Drugs (concomitant)

3 AMIODARONE amiodarone	Oral	200 Mg milligram(s)/ Day 245	2002- -	Atrial flutter
4 ARELIX "AVENTIS PHARMA" piretanide sodium	Oral	2*3 Mg milligram(s)/ Day 245	- -	Arterial hypotension
5 ATACAND candesartan cilexetil	Oral	16 Mg milligram(s)/ Day 245	- -	Arterial hypotension
6 DIGITOXIN digitoxin	Oral	0.05 Mg milligram(s)/ Day 245	2002- -	Tachycardia
7 EFEROX "EFEKA" levothyroxine sodium		125 Mg milligram(s)/ Day 245	- -	Hypothyroidism
8 OMEPRAZOLE omeprazole	Oral	40 Mg milligram(s)/ Day 5	2005- -	Prophylaxis against drug-induced ulcer
9 OMEPRAZOLE omeprazole		/ 5	- -	Ulcer
10 RAMIPRIL ramipril	Oral	10 Mg milligram(s)/ Day 245	- -	Arterial hypotension
11 VERAPAMIL verapamil	Oral	2*120 Mg milligram(s)/ Da 82	2002- -	Tachycardia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Atrial flutter	2002		Yes	rechtsatrial
Gonarthrosis			Yes	Beginn unbekannt; Z.n. Knie-TEP
Hemorrhoids			Yes	Beginn unbekannt
Mitral valve incompetence			Yes	Beginn unbekannt
Sick sinus syndrome	2002	2002	No	Z.n. DDD-SM-Implantation
Hypothyroidism			Yes	Beginn unbekannt; bei Z.n. Radiojodtherapie
Prophylaxis against drug-induced ulcer			Yes	
Ulcer			Unknown	on prophylaxis for drug induced ulcer
Hypertension arterial			Yes	Beginn unbekannt
Cardiac pacemaker insertion	2002	2002	No	
Diverticulosis			Unknown	
Hemorrhoids			Yes	
Radioactive iodine therapy			No	
Total knee replacement			No	
Tachycardia	2002		Yes	atrial
Arterial hypotension			k.A.	

Primary source

Qualification: Pharmacist observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20080924	70	µmol/l	
Creatinine	20080926	76	µmol/l	
Aspartate aminotransferase	20080924	0.21	µmol/sl	
Alanine aminotransferase	20080924	0.19	µmol/sl	
Gamma-glutamyltransferase	20080924	1.3 umol/sl	µmol/sl	
Quick's test	20080924	25	%	
Quick's test	20080925	48	%	
Quick's test	20080926	81	%	
APTT	20080924	37	s	
APTT	20080925	35	s	
APTT	20080926	23	s	
Haemoglobin	20080924	9.3	mmol/l	
Haemoglobin	20080926	8.3	mmol/l	
Haematocrit	20080924	0.446	NA	
Haematocrit	20080926	0.399	NA	
Potassium	20080924	3.7	mmol/l	
Potassium	20080926	4.4	mmol/l	
Sodium	20080924	140	mmol/l	
Sodium	20080926	145	mmol/l	
INR	20080924	2.5	NA	
INR	20080925	1.6	NA	
INR	20080926	1.2	NA	

Narratives (nur bei elektronischem Eingang)

Universität Greifswald, 26.03.2009 DE-PVZIBE-GREIFSWALD-10168

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
perianale Blutung bei ausgeprägter Divertikulose

KURZZUSAMMENFASSUNG:

perianale Blutung bei ausgeprägter Divertikulose unter Phenprocoumon-Medikation

BESCHREIBUNG:

Die stationäre Aufnahme der Patientin erfolgte durch den Notarzt nachdem er am Nachmittag des 24.09.08 beim Toilettengang Stuhl mit hellrotem Blut durchmischt sowie aufgelagerten Koagel absetzte. Anschließend 3-4x massiver Blutabgang, Stuhlkonsistenz normal, keine abdominellen Schmerzen, keine Übelkeit, kein Erbrechen. Erstmaliges Ereignis. RR 156/71 HF 82/min

THERAPIE:

Gabe von 1l E153 ab dem 24.09.08;

Pausieren der Falithrom-Medikation vom 24.09.-30.09.08;

Gabe von 2x40mg Pantozol ab dem 24.09.08;

VERLAUF:

Gastroskopie vom 25.09.2008: im oberen, mittleren und unteren Ösophagusdrittel zeigt sich eine unauffällige Schleimhaut, im Magenfundus und Magencorpus zeigt sich eine unauffällige Schleimhaut, im Magenantrum zeigt sich eine feinfleckige Schleimhautrötung mit geringer ödematöser Schwellung, im eingesehenen Duodenum zeigt sich eine unauffällige Schleimhaut = Antrumgastritis, kein sicherer Nachweis einer akuten oder stattgehabten Blutung;

Koloskopie vom 25.09.08: von der linken Flexur bis zum distalen Sigma finden sich massiv viele Divertikel, im gesamten eingesehenen Bereich findet sich blutig tingierter Stuhl;

Koloskopie vom 29.09.08: kein Nachweis von Blut oder Teerstuhl im eingesehenen Bereich, von der linken Flexur bis in das distale Sigma viele reizlos imponierende teilweise große Divertikel, in allen Abschnitten unauffällige Schleimhaut;

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: FALITHROM "HEXAL"

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: als Ursache der unteren gastrointestinalen Blutung ist eine Blutung aus Divertikeln des Sigma unter Phenprocoumon-Medikation anzusehen<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 06.04.2009 DE-002147023-PHHY2009DE11589

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via HA on 26 Mar 2009: The patient with a relevant history and concurrent conditions of Sick sinus syndrome, Mitral valve incompetence, Arterial hypotension, Hemorrhoids, Gonarthrosis, Atrial flutter, Tachycardia, Hypothyroidism and Prophylaxis against drug-induced ulcer was on treatment with Diclofenac (diclofenac) on an unknown date in 2005 for Gonarthrosis. Falithrom (Hexal) was a co-suspect drug which the patient was taking on an unknown date in 2002 for atrial flutter. However temporary discontinuation of Falithrom medication happened from 24 Sep 2008 to 30 Sep 2008. Concomittant medications included amiodarone, arelix, atacand, digitoxin, eferox, omeprazole, ramipril and verapamil. The patient experienced Gastrointestinal bleeding on an unknown date along with perianal bleeding with marked diverticulosis. The patient was hospitalised on 24 Sep 2008 by an emergency physician following a visit of the patient at her home. In the afternoon of that day, the patient had defecated and showed stool mixed with bright red blood as well as coagulation. Following this 3 to 4 severe defecations with blood happened however stool consistency was normal and no abdominal pain, no nausea, no vomiting occurred. Blood pressure was 156/71 and heart rate was 82 beats/min. Gastroscopy on 25 Sep 2008 showed that in the upper, middle and lower esophagus sections, the mucous membrane was without pathological findings; in the stomach fundus and stomach corpus the mucous membrane was without pathological findings; in the stomach antrum, the mucous membrane showed a finely spotted mucous membrane erythema with minimal edemous swelling, in the visible portion of the duodenum the mucous membrane showed no pathological findings, antrum gastritis and no certain evidence of acute or past bleeding. Colonoscopy on 25 Sep 2008 showed from the left flexure to the distal sigma, several major diverticula in the entire area viewed, blood-tinged stool could be seen. Colonoscopy on 29 Sep 2008 showed no evidence of blood or tarry stool in the entire area viewed, from the left flexure to the distal sigma, several not-inflamed impressive diverticula (in part large), in all areas mucous membrane without pathological findings. Administration of 1 liter E153 starting on 24 Sep 2008, temporary discontinuation of Falithrom medication from 24 Sep 2008 to 30 Sep 2008 and administration of 2 x 40mg pantozol starting on 24 Sep 2008 were used as treatments. The event outcome was not reported. In the absence of a reporting healthcare professional causality, the Novartis medical safety physician has provisionally assessed the event (s) of "gastrointestinal hemorrhage, perianal bleeding and gastritis" as suspected and "diverticulosis of colon" as not suspected to be related to [Diclofenac], based on the current available information.

HELIOS Klinikum Wuppertal, 02.12.2010 DE-PVZIBE-GREIFSWALD-10168

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

perianale Blutung bei ausgeprägter Divertikulose

KURZZUSAMMENFASSUNG:

perianale Blutung bei ausgeprägter Divertikulose unter Phenprocoumon-Medikation

BESCHREIBUNG:

Die Patientin befand sich vom 24.09. bis 30.09.08 in stationärer Behandlung.

Die stationäre Aufnahme der Patientin erfolgte durch den Notarzt nachdem er am Nachmittag des 24.09.08 beim Toilettengang Stuhl mit hellrotem Blut durchmischt sowie aufgelagerten Koagel absetzte. Anschließend 3-4x massiver Blutabgang, Stuhlkonsistenz normal, keine abdominellen Schmerzen, keine Übelkeit, kein Erbrechen. Erstmaliges Ereignis. RR 156/71 HF 82/min

THERAPIE:

Gabe von 1l E153 ab dem 24.09.08;

Pausieren der Falithrom-Medikation vom 24.09.-30.09.08;

Gabe von 2x40mg Pantozol ab dem 24.09.08;

VERLAUF:

Gastroskopie vom 25.09.2008: im oberen, mittleren und unteren Ösophagusdrittel zeigt sich eine unauffällige Schleimhaut, im Magenfundus und Magencorpus zeigt sich eine unauffällige Schleimhaut, im Magenantrum zeigt sich eine feinfleckige Schleimhautrötung mit geringer ödematöser Schwellung, im eingesehenen Duodenum zeigt sich eine unauffällige Schleimhaut = Antrumgastritis, kein sicherer Nachweis einer akuten oder stattgehabten Blutung;

Koloskopie vom 25.09.08: von der linken Flexur bis zum distalen Sigma finden sich massiv viele Divertikel, im gesamten eingesehenen Bereich findet sich blutig tingierter Stuhl;

Koloskopie vom 29.09.08: kein Nachweis von Blut oder Teerstuhl im eingesehenen Bereich, von der linken Flexur bis in das distale Sigma viele reizlos imponierende teilweise große Divertikel, in allen Abschnitten unauffällige Schleimhaut;

ERLÄUTERUNGEN ZUR UAW:

Als Ursache der Krankenhausaufnahme ist als sehr wahrscheinlich die perianale Blutung bei ausgeprägter Divertikulose zu werten. Keine Alternativursachen eruierbar.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: FALITHROM "HEXAL"

BETREFFENDE UAW: gastrointestinale Blutung

ERLÄUTERUNG: als Ursache der unteren gastrointestinalen Blutung ist eine Blutung aus Divertikeln des Sigma unter Phenprocoumon-Medikation anzusehen

sanofi-aventis, 16.12.2010 DE-SANOFI-AVENTIS-2010SA073494

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09038676) on 06-Dec-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This case concerns a 78-year-old female patient, whose medical history included radioactive iodine therapy and total knee replacement.

Concomitantly the patient suffered from sick sinus syndrome, mitral valve incompetence, hemorrhoids, gonarthrosis, and diverticulosis under phenprocoumon medication.

Concomitant medication included piretanide sodium, ramipril 10 mg/day and candesartan cilexetil 16 mg/day for arterial hypertension, levothyroxine sodium 125 mcg/day for hypothyroidism, amiodarone hydrochloride 200 mg/day for right atrial flutter, verapamil hydrochloride 120 mg twice daily and digitoxin 0.05 mg/day for atrial tachycardia as well as omeprazole 40 mg/day for prophylaxis against drug-induced ulcer. On 24-Sep-2008, the patient detected light-red bloody stool with coagulum and reported about massive anal bleeding 3- 4 times later. Subsequently the patient was hospitalised by the emergent physician on the same day. The stool consistency was normal. Phenprocoumon was stopped; corrective treatment with electrolyte solution 1 litre and pantoprazole 40 mg twice daily was started.

On 25-Sep-2008 the performed coloscopy displayed bloody stool. Four days later the bloody or tarry stool could not be revealed by coloscopy and the patient was discharged on the next day.

No further information will be expected because the case was closed by PV-center.

Reporter causality: probable for phenprocoumon, possible for diclofenac

NOVARTIS, 17.12.2010 DE-002147023-PHHY2009DE11589

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via HA on 26 Mar 2009: The patient with a relevant history and concurrent conditions of Sick sinus syndrome, Mitral valve incompetence, Arterial hypotension, Hemorrhoids, Gonarthrosis, Atrial flutter, Tachycardia, Hypothyroidism and Prophylaxis against drug-induced ulcer was on treatment with Diclofenac (diclofenac) on an unknown date in 2005 for Gonarthrosis. Falithrom (Hexal) was a co-suspect drug which the patient was taking on an unknown date in 2002 for atrial flutter. However temporary discontinuation of Falithrom medication happened from 24 Sep 2008 to 30 Sep 2008. Concomittant medications included amiodarone, arelix, atacand, digitoxin, eferox, omeprazole, ramipril and verapamil. The patient experienced Gastrointestinal bleeding on an unknown date along with perianal bleeding with marked diverticulosis. The patient was hospitalised on 24 Sep 2008 by an emergency physician following a visit of the patient at her home. In the afternoon of that day, the patient had defecated and showed stool mixed with bright red blood as well as coagulation. Following this 3 to 4 severe defecations with blood happened however stool consistency was normal and no abdominal pain, no nausea, no vomiting occurred. Blood pressure was 156/71 and heart rate was 82 beats/min. Gastroscopy on 25 Sep 2008 showed that in the upper, middle and lower esophagus sections, the mucous membrane was without pathological findings; in the stomach fundus and stomach corpus the mucous membrane was without pathological findings; in the stomach antrum, the mucous membrane showed a finely spotted mucous membrane erythema with minimal edemous swelling, in the visible portion of the duodenum the mucous membrane showed no pathological findings, antrum gastritis and no certain evidence of acute or past bleeding. Colonoscopy on 25 Sep 2008 showed from the left flexure to the distal sigma, several major diverticula in the entire area viewed, blood-tinged stool could be seen. Colonoscopy on 29 Sep 2008 showed no evidence of blood or tarry stool in the entire area viewed, from the left flexure to the distal sigma, several not-inflamed impressive diverticula (in part large), in all areas mucous membrane without pathological findings. Administration of 1 liter E153 starting on 24 Sep 2008, temporary discontinuation of Falithrom medication from 24 Sep 2008 to 30 Sep 2008 and administration of 2 x 40mg pantozol starting on 24 Sep 2008 were used as treatments. The event outcome was not reported. In the absence of a reporting healthcare professional causality, the Novartis medical safety physician has provisionally assessed the event (s) of "gastrointestinal hemorrhage, perianal bleeding and gastritis" as suspected and "diverticulosis of colon" as not suspected to be related to [Diclofenac], based on the current available information.

Follow up information received from a Regional Pharmacovigilance Center (intensive monitoring project) on 03 Dec 2010. The cause of the hospitalization was most probably the perianal bleeding with marked diverticulosis. No other alternative reason could be found.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09039416

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: 19300207 Age: 78 Year weight (kg): 53 BMI_int: 22.64 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia	20081227	20081230	4 Day		recovered/resolved
2 Inflammation	20090106				Unknown
3 AST increased					Unknown
4 Blood creatinine increased					Unknown
5 Blood pressure increased					Unknown
6 Blood sodium decreased					Unknown
7 C-reactive protein increased					Unknown
8 Gastric mucosal lesion NOS					Unknown
9 Nausea					Unknown
10 Pale					Unknown
11 Stress					Unknown
12 Tarry stools		20090107			recovered/resolved
13 Vomiting					Unknown
14 GGT increased					Unknown
15 Reduced general condition					Unknown
16 Leucopenia	20081230	20090107	9 Day		recovered/resolved
17 Upper gastrointestinal haemorrhage	20081227	20081230	4 Day		recovered/resolved
18 Drug interaction					recovered/resolved
19 Off label use					recovered/resolved
20 Upper gastrointestinal hemorrhage	20081227	200901	4.3 Week	26.1 Week	recovered/resolved
21 Diverticula of colon					Unknown
22 Red blood cell count decreased	20081230				Unknown
23 Abdominal pain					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-09039416					
1	AMITRIPTYLINE amitriptyline	Oral	25 Mg milligram(s)/Day 6	200806- -	Sleep disorder NOS
2	AMLODIPIN RATIOPHARM amlodipine	Oral	10 Mg milligram(s)/Day 245	2006- -	Hypertension arterial
3	CLEXANE heparin-fraction, sodium salt	Subcutaneous	0.5 ml millilitre(s)/Day	20081230- -	Thrombosis prophylaxis
4	DEKRISTOL colecalfiferol	Oral	20000 lu international unit 245	200811-20081228 1 Month	Osteoporosis prophylaxis
5	EBIXA memantine hydrochloride	Oral	10 Mg milligram(s)/Day 245	200806- -	Dementia Alzheimer's type
6	FENTANYL fentanyl	Transdermal	12 µg microgram(s)/3 Day 250	200811- -	Postoperative pain
7	ISCOVER clopidogrel sulfate	Oral	75 Mg milligram(s)/Day 245	200806-20090101 7 Month	Apoplexy
8	MONO-EMBOLEX heparin-fraction, sodium salt	Subcutaneous	3000 lu international unit(s) 120	200811-20081230 1 Month	Thrombosis prophylaxis
9	PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/Day 245	200811- -	Gastric ulcer prophylaxis
10	SUTENT sunitinib	Oral	50 Mg milligram(s)/Day 5	20081103-20081230 58 Day	Renal cell carcinoma NOS
11	SUTENT sunitinib	Oral	50 Mg milligram(s)/Day 5	20081203-20081230 28 Day	Renal cell carcinoma NOS
12	SUTENT sunitinib	Oral	50 Mg milligram(s)/Day 5	20081022-20081119 29 Day	Renal cell carcinoma NOS
13	VOLTAREN RESINAT "NOVARTIS" diclofenac resinate	Oral	75 Mg milligram(s)/Day 245	200811-20081230 1 Month	Postoperative pain

Drugs (concomitant)					
14	novalglin metamizole	Oral	3*500 Mg milligram(s)/ Da 160	200811- -	Postoperative pain

Medical History and concurrent conditions					
MedDRA-term	Begin	End	Cont.	Comment	
Renal cell carcinoma NOS	2008	20080715	No	07/2008 Nephrektomie links	
Device implant NOS	20081113	20081113	No		
Nephrectomy	20080715	20080715	No		
Colitis			Unknown	FOR WHICH RECEIVED METRONIDAZOLE FROM 30-DEC-2008 TO 09-JAN-2009	
Depression			Yes	Beginn unbekannt	
Sleep disorder NOS			Yes	Beginn unbekannt	
Postoperative pain	2008		Yes		
Dementia Alzheimer's type	200806		Yes		
Apoplexy	200806	200806	No		
Bone metastases	2008	20081113	No	totale Endoprothesen-OP 12/2008	
Hypertension arterial	2006		Yes		
Cataract			Yes	Beginn unbekannt	

Primary source		
Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik					
Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung	
Creatinine	20081230	115	µmol/l		
Aspartate aminotransferase	20081230	0.73	µkat/l		
Alanine aminotransferase	20081230	0.37	µkat/l		

Bilirubin total	20081230	4.7	µmol/l
Alkaline phosphatase	20081230	2.31	µkat/l
Gamma GT	20081230	1.45	µkat/l
Quick's test	20081230	122	Prozent
Partial prothrombin time	20081230	31.0	sec
INR	20081230	0.9	Physiologis
Thrombocyte count	20081230	216	Gpt/l
Thrombocyte count	20081231	207	Gpt/l
Thrombocyte count	20090107	339	Gpt/l
Thrombocyte count	20081022	414	Gpt/l
Thrombin time	20081230	17.6	sec
Haemoglobin	20081230	6.3	mmol/l
Haemoglobin	20081231	5.6	mmol/l
Haemoglobin	20090107	5.3	mmol/l
Haemoglobin	20081022	6.1	mmol/l
Haematocrit	20081230	0.30	SI-Einheit
Haematocrit	20081231	0.28	SI-Einheit
Haematocrit	20090107	0.27	SI-Einheit
Haematocrit	20081022	0.31	SI-Einheit
Total white blood cell count	20081230	3.3	10E9/l
Total white blood cell count	20081231	3.0	10E9/l
Total white blood cell count	20090107	4.3	10E9/l
Total white blood cell count	20081022	9.09	10E9/l
C-reactive protein	20081230	136	mg/l
C-reactive protein	20081231	111	mg/l
C-reactive protein	20090102	42.9	mg/l
Amylase	20081230	0.66	µkat/l
Lipase	20081230	0.36	µkat/l
Blood potassium	20081230	5.1	mmol/l
Blood potassium	20081231	4.7	mmol/l
Sodium	20081230	128	mmol/l
Sodium	20081231	132	mmol/l
Blood calcium	20081231	2.1	mmol/l
Red blood cell count	20081230	3.36	Tpt/l
MCV	20081230	91.4	fl
MCH	20081230	1875	amol
MCHC	20081230	20.5	mmol/l

Narratives (nur bei elektronischem Eingang)

Medizinische Fakultät der Universität Rostock, 26.03.2009 DE-PVZIBE-ROSTOCK-10119

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Blutung im oberen gastrointestinalen Bereich

KURZZUSAMMENFASSUNG:

abdominale Schmerzen, Erbrechen, Teerstuhl unter Diclofenac, Sunitinib, Clopidogrel, Certoparin

BESCHREIBUNG:

Einweisungsdiagnose: Teerstuhl, Verdacht auf gastrointestinale Blutung oder Kolitis;

Krankenhausaufenthalt: 30.12.2008-09.01.2009;

seit 3 Tagen Teerstühle abgesetzt, seit 3-4 Wochen abdominale Beschwerden (gesamter Bauch), vor 5 Tagen 1xErbrechen;

(anamnestisch: ambulanter Versuch einer Koloskopie, laut Patientin Abbruch wegen Verwachsungen);

Übelkeit tritt vorwiegend nach Sutent-Einnahme auf;

bei Aufnahme: blasse Patientin im reduzierten Allgemeinzustand, Blutdruck 145/75mmHg;

Zustand nach totaler Endoprothesen-OP am 13.11.2008 wegen Knochenmetastasen

THERAPIE:

30.12.08-06.01.09 Spizef;

30.12.08-09.01.09 Clont bei Verdacht auf Colitis (Symptomatik, Entzündungsparameter);

30.12.08-06.01.09 Protonenpumpeninhibitor hochdosiert i.v., dann oral;

Clexane 1x0.5ml weiter;

07.01.2009 Transfusion von 2 Erythrozytenkonzentraten

VERLAUF:

im Verlauf rückläufige abdominale Symptomatik und Paraklinik, kreislaufstabil, Teerstuhl bis 07.01.2009;

30.12.2009 Gastroskopie: Ösophagusschleimhaut unauffällig, in der Kardia fibrinbedeckte Läsionen, z.Z. keine akute Blutung, entzündlich veränderte Schleimhaut der Kardia, Restmagen und Duodenum unauffällig, Zusammenfassend: Verdacht auf stattgehabte Blutung an der Kardia;

06.01.2009 Rektoskopie: leichtgradige Entzündung im distalen Sigma ohne Stenose, keine Divertikel;

06.01.2009 Durchleuchtung nach Kontrastmittel-Einlauf (nur Beurteilung des distalen Kolons): Schleifenbildung in der Sigmaregion, einzelne Divertikel, keine sicheren Entzündungszeichen, keine Passagenhindernisse

ERLÄUTERUNGEN ZUR UAW:

Die Einweisungssymptomatik Teerstuhl, niedriger Hb-Wert und der Gastroskopiebefund lassen eine gastrointestinale Blutung vermuten, die durch die Einnahme von Diclofenac, Sunitinib, Clopidogrel und Certoparin bedingt oder verstärkt wurde. Alternativursache: stressbedingte Läsionen bei Schmerzen

ANMERKUNGEN ZU FOLGENDER UAW:

Leukopenie

KURZZUSAMMENFASSUNG:

Leukopenie unter Sutent

BESCHREIBUNG:

Im Aufnahmeblutbild war eine Leukopenie von 3,0 Gpt/ml auffällig. Bei Entlassung und nach Absetzen von Sutent wurden 9,09 Gpt/ml Leukozyten (Normalbefund) gemessen.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ISCOVER

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Blutungsverstärkung wegen Thrombozytenaggregationshemmung

MEDIKAMENT: NOVALGIN

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Fachinfo: Leukopenie

MEDIKAMENT: NOVALGIN

BETREFFENDE UAW: Leukopenie

ERLÄUTERUNG: Leukopenien sind prinzipiell bekannte UAW unter Metamizol. In diesem Fall erfolgte eine Normalisierung der Leukozytenzahl trotz Weitergabe von metamizol, so dass ein Zusammenhang unwahrscheinlich ist.

MEDIKAMENT: AMITRIPTYLINE

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Fachinfo: Leukopenie

MEDIKAMENT: AMITRIPTYLINE

BETREFFENDE UAW: Leukopenie

ERLÄUTERUNG: Leukopenien sind prinzipiell unter Amitriptylin bekannte UAW. In diesem Fall trat trotz Weitergabe eine Normalisierung der Leukozytenwerte ein, so dass ein Zusammenhang hier unwahrscheinlich ist.

MEDIKAMENT: FENTANYL

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Fachinfo: Erbrechen

MEDIKAMENT: MONO-EMBOLEX

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Blutungsverstärkung durch Gerinnungshemmung

MEDIKAMENT: VOLTAREN RESINAT "NOVARTIS"

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Schleimhautschädigung der Kardia und Thrombozytenaggregationshemmung liegen im UAW-Spektrum.

Alternativursache: stressbedingte Schleimhautschädigung bei Schmerzen

MEDIKAMENT: SUTENT

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bere<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 08.04.2009 DE-002147023-PHHY2009DE11802

Sender diagnosis: not reported

Narrative

Initial report received from regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-09039416) on 27 Mar 2009: The patient's medical history included renal cell carcinoma, bone metastases (total endoprosthesis operation done on 13 Nov 2008), apoplexy and concurrent conditions of hypertension arterial, cataract, dementia Alzheimer's type, and sleep disorder. This patient was on multitherapy, including Voltaren resinat (diclofenac) for postoperative pain (start date Nov 2008), Sutent (sunitinib) for renal cell carcinoma, Iscover (clopidogrel sulfate) for apoplexy and Mono-embolex (certoparin; heparin fraction, sodium salt) for thrombosis prophylaxis. The patient was hospitalized on 30 Dec 2008 with complaints of abdominal pain since three to four weeks, vomiting since five days and tarry stools since 27 Dec 2008. Also nausea was predominant after intake of Sutent. At hospitalization, the patient was pale with reduced general condition, blood pressure was 145/75 mm Hg. The lab tests done on 30 Dec 2008 revealed increased creatinine 115 umol/l (normal less than 88), aspartate aminotransferase 0.73 ukat/l (normal less than 0.58), gamma GT 1.45 ukat/l (normal less than 0.65), hemoglobin 6.3 mmol/l (normal range 6.9 to 9.5), hematocrit 0.30 SI-Einheit (normal range 0.34 to 0.45), Total White blood cell count 3.3 10E9/l (normal range 4 to 10), C reactive protein 136 mg/l (normal less than 5) and sodium 128 mmol/l (normal range 136 to 152). Leukopenia was also reported. Gastrointestinal hemorrhage or colitis was suspected. The patient was treated with Spizef (cefotiam hydrochloride), Clexane (enoxaparin sodium, heparin-fraction, sodium salt), Clont (metronidazole) and proton pump inhibitors, and was also transfused with 2 units of erythrocyte concentrates. The abdominal symptoms decreased, and tarry stools also recovered on 07 Jan 2009. The tarry stools, low hemoglobin and results of gastroscopy (inflammatory changes in mucosa of cardiac stomach, suspicion of state after hemorrhage of cardiac stomach) led to the diagnosis of upper gastrointestinal hemorrhage, which was suspected to be caused or worsened by intake of diclofenac, sunitinib, clopidogrel and certoparin. All the suspect drugs were discontinued on 30 Dec 2008 (Iscover discontinued on 01 Jan 2009). The alternative cause was supposed to be the lesion caused by stress. The outcome for gastrointestinal hemorrhage and leucopenia was reported as complete recovery on 30 Dec 2008. The patient was discharged on 09 Jan 2009.

SANOFI-SYNTHELABO, 09.04.2009 DE-SANOFI-SYNTHELABO-A01200903319

Sender diagnosis: not reported

Narrative

Reaction as reported: gastroscopy: suspected upper GI haemorrhage, no acute bleeding on time (with tarry stools, vomiting nos, total abdominal pain/disorder, nausea after taking sunitinib)

LLT: UPPER GASTROINTESTINAL HAEMORRHAGE / 10046274

PT: UPPER GASTROINTESTINAL HAEMORRHAGE / 10046274

Reaction as reported: leucopenia (WBC at 3.0 10E9/L)

LLT: LEUCOPENIA / 10024283

PT: LEUKOPENIA / 10024384

Reaction as reported: interacting drugs (clopidogrel, certoparin, enoxaparin)

LLT: DRUG INTERACTION / 10013710

PT: DRUG INTERACTION / 10013710

Narrative:

Initial information received from a physician via the Health Authorities in Germany through Bristol-Myers Squibb (BMS ref.: 2009032701) on 27-Mar-2009:

A 78-year-old female patient had a history of renal cell carcinoma from 2008 to 15-Jul-2008 requiring left nephrectomy, bone metastases from 2008 to 13-Nov-2008, requiring total endoprosthesis surgery on 13-Nov-2008, arterial hypertension, dementia of the Alzheimer's type, postoperative pain, and sleep disorder.

The patient has been started on clopidogrel (Iscover) 75 mg daily for apoplexy in June 2008. She had been also treated with amitriptyline 25 mg daily for sleep disorder since June 2008, with amlodipine 5 mg twice daily for arterial hypertension since 2006, colecalciferol 20,000 IU for prophylaxis of osteoporosis since November 2008, with memantine 10 mg daily for dementia of the Alzheimer's type since June 2008, with fentanyl patch 12 mcg every 3 days and metamizole 30 drops three times daily, both for postoperative pain since November 2008, and with pantoprazole 40 mg daily for prophylaxis of gastric ulcer since November 2008.

She also started sunitinib 50 mg daily for renal cell carcinoma: first cycle from 22-Oct-2008 until 19-Nov-2009, and second cycle from 02-Dec-2008 to 30-Dec-2008. Diclofenac resinate 75 mg daily for postoperative pain had been initiated in November 2008. Heparin sodium (certoparin) with enoxaparin had been initiated for prophylaxis of thrombosis in November 2008.

On 30-Dec-2008, the patient was hospitalised with 4 to 5 week history of abdominal pain, vomiting, and 3-day history of tarry stools. Colonoscopy was interrupted due to accretions. On admission, she was pale with reduced general condition. Blood pressure was 145/75 mmHg.

Gastroscopy revealed normal esophagus mucosa, in the cardia fibrin covered lesions, no acute bleeding on time, inflammatory changed mucosa in the cardia, remain stomach and duodenum normal. In summary: suspicion of occurred bleeding on the cardia. Blood work-up was also performed, and revealed leukopenia at 3.0 Gpt/mL.

From 30-Dec-2008 to 06-Jan-2009, she received cefotiam (Spizef).

From 30-Dec-2008 to 09-Jan-2009, she received metronidazole (Clont) with suspicion of colitis (inflammation).

From 30-Dec-2008 to 06-Jan-2009, proton pump inhibitor, at high dose intravenous followed by oral therapy were given.

On 06-Jan-2009, rectoscopy showed mild inflammation in distal sigma without stenosis, no diverticel. X-ray after contrast agent enema (only assessment of distal colon): loop formation in the sigma region, individual diverticel, no certain signs of inflammation and no passages barriers.

On 07-Jan-2009, transfusion of one unit of red blood cells was given.

In the course of therapy, regressive abdominal and paraclinical symptoms were noticed. Circulatory condition became stable, and tarry stools stopped on 07-Jan-2009.

Explanation to the adverse drug reaction:

The symptoms on admission tarry stools, decreased haemoglobin value and gastroscopy results assumed a gastrointestinal haemorrhage which was induced or increased through the intake of diclofenac, sunitinib, clopidogrel and certoparin. Alternate cause: stress-related lesions of pains.

Explanation to leucopenia:

Leucopenia under therapy with sunitinib

Description: The haemogram at admission showed leucopenia (3.0 Gpt/ml). After discharge and after discontinuation of sunitinib leucocytes were 9.09 Gpt/ml (normal findings).

Explanation to the causalities:

Medication: clopidogrel

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: Increased bleedings because of inhibition of thrombocyte aggregation.

Medication: diclofenac

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: leucopenia.

Medication: diclofenac

Adverse drug reaction: leucopenia

Explanation: Leucopenia is principally known as an adverse event under metamizole. In this case there was a normalization of the number of leucocytes in spite of further administration of metamizole; a relation is not likely.

Medication: amitriptyline

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: leucopenia.

Medication: amitriptyline

Adverse drug reaction: leucopenia

Explanation: Leucopenia is principally known as an adverse event under amitriptyline. In this case there was a normalization of the number of leucocytes in spite of further administration of amitriptyline; a relation is not likely.

Medication: fentanyl

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting.

Medication: heparin sodium

Adverse drug reaction: upper gastrointestinal haemorrhage
 Explanation: Increased bleedings through anticoagulation.

Medication: Voltaren resinate

Adverse drug reaction: upper gastrointestinal haemorrhage
 Explanation: mucosa damage of the cardia and inhibition of thrombocyte aggregation are expected in the adverse events spectrum. Alternate cause: stress-induced damage of the mucosa with pains.

Medication: sunitinib

Adverse drug reaction: upper gastrointestinal haemorrhage
 Explanation: SmPC: vomiting, upper abdominal disorder, neutropenia, gastrointestinal bleeding, CYP3A4 substrate.

Medication: sunitinib

Adverse drug reaction: leucopenia
 Explanation: Neutropenia is a known adverse event under sunitinib. After discontinuation normalization of the number of leucocytes.

Medication: colecalciferol

Adverse drug reaction: upper gastrointestinal haemorrhage
 Explanation: SmPC: vomiting

Medication: memantine

Adverse drug reaction: upper gastrointestinal haemorrhage
 Explanation: SmPC: vomiting

The reporter considered upper gastrointestinal haemorrhage possibly related to sunitinib, likely related to voltaren, possibly related to clopidogrel, possibly related to heparin, dubiously related to amitriptyline, dubiously related to amlodipine, dubiously related to colecalciferol, dubiously related to memantine, dubiously related to fentanyl, dubiously related to metamizol and dubiously related to pantoprazole.

The reporter considered leucopenia possibly related to sunitinib, dubiously related to voltaren, dubiously related to clopidogrel, dubiously related to heparin, dubiously related to amitriptyline, dubiously related to amlodipine, dubiously related to colecalciferol, dubiously related to memantine, dubiously related to fentanyl, dubiously related to metamizole and dubiously related to pantoprazole.

Associate case Id for enoxaparine is pending

Additional information received on 06-Apr-2009:

Sunitinib was administered from 22-Oct-2008 to 19-Nov-2008 (first cycle), and from 03-Dec-2008 to 30-Dec-2009 (second cycle).
 No further information provided.

SANOFI-SYNTHELABO, 15.04.2009 DE-SANOFI-SYNTHELABO-A01200903319

Sender diagnosis: not reported

Narrative

Reaction as reported: gastroscopy: suspected upper GI haemorrhage, no acute bleeding on time (with tarry stools, vomiting nos, total abdominal pain/disorder, nausea after taking sunitinib)

LLT: UPPER GASTROINTESTINAL HAEMORRHAGE / 10046274

PT: UPPER GASTROINTESTINAL HAEMORRHAGE / 10046274

Reaction as reported: leucopenia (WBC at $3.0 \cdot 10^9/L$)

LLT: LEUCOPENIA / 10024283

PT: LEUKOPENIA / 10024384

Reaction as reported: interacting drugs (clopidogrel, certoparin, enoxaparin)

LLT: DRUG INTERACTION / 10013710

PT: DRUG INTERACTION / 10013710

Narrative:

Initial information received from a physician via the Health Authorities in Germany through Bristol-Myers Squibb (BMS ref.: 2009032701) on 27-Mar-2009:

A 78-year-old female patient had a history of renal cell carcinoma from 2008 to 15-Jul-2008 requiring left nephrectomy, bone metastases from 2008 to 13-Nov-2008, requiring total endoprosthesis surgery on 13-Nov-2008, arterial hypertension, dementia of the Alzheimer's type, postoperative pain, and sleep disorder.

The patient has been started on clopidogrel (Iscover) 75 mg daily for apoplexy in June 2008. She had been also treated with amitriptyline 25 mg daily for sleep disorder since June 2008, with amlodipine 5 mg twice daily for arterial hypertension since 2006, colecalciferol 20,000 IU for prophylaxis of osteoporosis since November 2008, with memantine 10 mg daily for dementia of the Alzheimer's type since June 2008, with fentanyl patch 12 mcg every 3 days and metamizole 30 drops three times daily, both for postoperative pain since November 2008, and with pantoprazole 40 mg daily for prophylaxis of gastric ulcer since November 2008.

She also started sunitinib 50 mg daily for renal cell carcinoma: first cycle from 22-Oct-2008 until 19-Nov-2009, and second cycle from 02-Dec-2008 to 30-Dec-2008. Diclofenac resinate 75 mg daily for postoperative pain had been initiated in November 2008. Heparin sodium (certoparin) with enoxaparin had been initiated for prophylaxis of thrombosis in November 2008.

On 30-Dec-2008, the patient was hospitalised with 4 to 5 week history of abdominal pain, vomiting, and 3-day history of tarry stools. Colonoscopy was interrupted due to accretions. On admission, she was pale with reduced general condition. Blood pressure was 145/75 mmHg.

Gastroscopy revealed normal esophagus mucosa, in the cardia fibrin covered lesions, no acute bleeding on time, inflammatory changed mucosa in the cardia, remain stomach and duodenum normal. In summary: suspicion of occurred bleeding on the cardia. Blood work-up was also performed, and revealed leukoepnia at 3.0 Gpt/mL.

From 30-Dec-2008 to 06-Jan-2009, she received cefotiam (Spizef).

From 30-Dec-2008 to 09-Jan-2009, she received metronidazole (Clont) with suspicion of colitis (inflammation).

From 30-Dec-2008 to 06-Jan-2009, proton pump inhibitor, at high dose intravenous followed by oral therapy were given.

On 06-Jan-2009, rectoscopy showed mild inflammation in distal sigma without stenosis, no diverticel. X-ray after contrast agent enema (only assessment of distal colon): loop formation in the sigma region, individual diverticel, no certain signs of inflammation and no passages barriers.

On 07-Jan-2009, transfusion of one unit of red blood cells was given.

In the course of therapy, regressive abdominal and paraclinical symptoms were noticed. Circulatory condition became stable, and tarry stools stopped on 07-Jan-2009.

Explanation to the adverse drug reaction:

The symptoms on admission tarry stools, decreased haemoglobin value and gastroscopy results assumed a gastrointestinal haemorrhage which was induced or increased through the intake of diclofenac, sunitinib, clopidogrel and certoparin. Alternate cause: stress-related lesions of pains.

Explanation to leucopenia:

Leucopenia under therapy with sunitinib

Description: The haemogram at admission showed leucopenia (3.0 Gpt/ml). After discharge and after discontinuation of sunitinib leucocytes were 9.09 Gpt/ml (normal findings).

Explanation to the causalities:

Medication: clopidogrel

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: Increased bleedings because of inhibition of thrombocyte aggregation.

Medication: diclofenac

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: leucopenia.

Medication: diclofenac

Adverse drug reaction: leucopenia

Explanation: Leucopenia is principally known as an adverse event under metamizole. In this case there was a normalization of the number of leucocytes in spite of further administration of metamizole; a relation is not likely.

Medication: amitriptyline

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: leucopenia.

Medication: amitriptyline

Adverse drug reaction: leucopenia

Explanation: Leucopenia is principally known as an adverse event under amitriptyline. In this case there was a normalization of the number of leucocytes in spite of further administration of amitriptyline; a relation is not likely.

Medication: fentanyl

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting.

Medication: heparin sodium

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: Increased bleedings through anticoagulation.

Medication: Voltaren resinate

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: mucosa damage of the cardia and inhibition of thrombocyte aggregation are expected in the adverse events spectrum. Alternate cause: stress-induced damage of the mucosa with pains.

Medication: sunitinib

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting, upper abdominal disorder, neutropenia, gastrointestinal bleeding, CYP3A4 substrate.

Medication: sunitinib

Adverse drug reaction: leucopenia

Explanation: Neutropenia is a known adverse event under sunitinib. After discontinuation normalization of the number of leucocytes.

Medication: colecalciferol

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting

Medication: memantine

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting

The reporter considered upper gastrointestinal haemorrhage possibly related to sunitinib, likely related to voltaren, possibly related to clopidogrel, possibly related to heparin, dubiously related to amitriptyline, dubiously related to amlodipine, dubiously related to colecalciferol, dubiously related to memantine, dubiously related to fentanyl, dubiously related to metamizol and dubiously related to pantoprazole.

The reporter considered leucopenia possibly related to sunitinib, dubiously related to voltaren, dubiously related to clopidogrel, dubiously related to heparin, dubiously related to amitriptyline, dubiously related to amlodipine, dubiously related to colecalciferol, dubiously related to memantine, dubiously related to fentanyl, dubiously related to metamizole and dubiously related to pantoprazole.

Associate case Id for enoxaparine is pending

Additional information received on 06-Apr-2009:

Sunitinib was administered from 22-Oct-2008 to 19-Nov-2008 (first cycle), and from 03-Dec-2008 to 30-Dec-2009 (second cycle).

No further information provided.

HELIOS Klinikum Wuppertal, 04.06.2010 DE-PVZIBE-ROSTOCK-10119

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Blutung im oberen gastrointestinalen Bereich

KURZZUSAMMENFASSUNG:

abdominale Schmerzen, Erbrechen, Teerstuhl unter Diclofenac, Sunitinib, Clopidogrel, Certoparin

BESCHREIBUNG:

Einweisungsdiagnose: Teerstuhl, Verdacht auf gastrointestinale Blutung oder Kolitis;

Krankenhausaufenthalt: 30.12.2008-09.01.2009;

seit 3 Tagen Teerstühle abgesetzt, seit 3-4 Wochen abdominale Beschwerden (gesamter Bauch), vor 5 Tagen 1xErbrechen;

(anamnestisch: ambulanter Versuch einer Koloskopie, laut Patientin Abbruch wegen Verwachsungen);

Übelkeit tritt vorwiegend nach Sutent-Einnahme auf;

bei Aufnahme: blasse Patientin im reduzierten Allgemeinzustand, Blutdruck 145/75mmHg;

Zustand nach totaler Endoprothesen-OP am 13.11.2008 wegen Knochenmetastasen

THERAPIE:

30.12.08-06.01.09 Spizef;

30.12.08-09.01.09 Clont bei Verdacht auf Colitis (Symptomatik, Entzündungsparameter);

30.12.08-06.01.09 Protonenpumpeninhibitor hochdosiert i.v., dann oral;

Clexane 1x0.5ml weiter;

07.01.2009 Transfusion von 2 Erythrozytenkonzentraten

VERLAUF:

im Verlauf rückläufige abdominale Symptomatik und Paraklinik, kreislaufstabil, Teerstuhl bis 07.01.2009;

30.12.2009 Gastroskopie: Ösophagusschleimhaut unauffällig, in der Kardia fibrinbedeckte Läsionen, z.Z. keine akute Blutung, entzündlich veränderte Schleimhaut der Kardia, Restmagen und Duodenum unauffällig, Zusammenfassend: Verdacht auf stattgehabte Blutung an der Kardia;

06.01.2009 Rektoskopie: leichtgradige Entzündung im distalen Sigma ohne Stenose, keine Divertikel;

06.01.2009 Durchleuchtung nach Kontrastmittel-Einlauf (nur Beurteilung des distalen Kolons): Schleifenbildung in der Sigmaregion, einzelne Divertikel, keine sicheren Entzündungszeichen, keine Passagenhindernisse

ERLÄUTERUNGEN ZUR UAW:

Die Einweisungssymptomatik Teerstuhl, niedriger Hb-Wert und der Gastroskopiebefund lassen eine gastrointestinale Blutung vermuten, die durch die Einnahme von Diclofenac, Sunitinib, Clopidogrel und Certoparin bedingt oder verstärkt wurde. Alternativursache: stressbedingte Läsionen bei Schmerzen

ANMERKUNGEN ZU FOLGENDER UAW:

Leukopenie

KURZZUSAMMENFASSUNG:

Leukopenie unter Sutent

BESCHREIBUNG:

Im Aufnahmeblutbild war eine Leukopenie von 3,0 Gpt/ml auffällig. Bei Entlassung und nach Absetzen von Sutent wurden 9,09 Gpt/ml Leukozyten (Normalbefund) gemessen.

THERAPIE:

30.12.08 Absetzen von Sutent

VERLAUF:

im Verlauf rückläufige abdominale Symptomatik und Paraklinik, kreislaufstabil, Teerstuhl bis 07.01.2009;

30.12.2009 Gastroskopie: Ösophagusschleimhaut unauffällig, in der Kardie fibrinbedeckte Läsionen, z.Z. keine akute Blutung, entzündlich veränderte Schleimhaut der Kardie, Restmagen und Duodenum unauffällig, Zusammenfassend: Verdacht auf stattgehabte Blutung an der Kardie;

06.01.2009 Rektoskopie: leichtgradige Entzündung im distalen Sigma ohne Stenose, keine Divertikel;

06.01.2009 Durchleuchtung nach Kontrastmittel-Einlauf (nur Beurteilung des distalen Kolons): Schleifenbildung in der Sigmaregion, einzelne Divertikel, keine sicheren Entzündungszeichen, keine Passagenhindernisse

ERLÄUTERUNGEN ZUR UAW:

Unter der Therapie mit Sutent treten Leukopenien häufig auf. Nach Absetzen von Sutent normalisierte sich die Leukozytenzahl.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:**MEDIKAMENT: ISCOVER**

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Blutungsverstärkung wegen Thrombozytenaggregationshemmung

MEDIKAMENT: MONO-EMBOLEX

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Blutungsverstärkung durch Gerinnungshemmung

MEDIKAMENT: VOLTAREN RESINAT "NOVARTIS"

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Schleimhautschädigung der Kardie und Thrombozytenaggregationshemmung liegen im UAW-Spektrum.

Alternativursache: stressbedingte Schleimhautschädigung bei Schmerzen

MEDIKAMENT: SUTENT

BETREFFENDE UAW: Blutung im oberen gastrointestinalen Bereich

ERLÄUTERUNG: Toxische Schleimhautschädigung

sanofi-aventis, 18.06.2010 DE-SANOFI-AVENTIS-A01200903319

Sender diagnosis: not reported

Narrative

Initial information received from a physician via the Health Authorities in Germany through Bristol-Myers Squibb (BMS ref.: 2009032701) on 27-Mar-2009:

A 78-year-old female patient had a history of renal cell carcinoma from 2008 to 15-Jul-2008 requiring left nephrectomy, bone metastases from 2008 to 13-Nov-2008, requiring total endoprosthesis surgery on 13-Nov-2008, arterial hypertension, dementia of the Alzheimer's type, postoperative pain, and sleep disorder.

The patient has been started on clopidogrel (Iscover) 75 mg daily for apoplexy in June 2008. She had been also treated with amitriptyline 25 mg daily for sleep disorder since June 2008, with amlodipine 5 mg twice daily for arterial hypertension since 2006, colecalciferol 20,000 IU for prophylaxis of osteoporosis since November 2008, with memantine 10 mg daily for dementia of the Alzheimer's type since June 2008, with fentanyl patch 12 mcg every 3 days and metamizole 30 drops three times daily, both for postoperative pain since November 2008, and with pantoprazole 40 mg daily for prophylaxis of gastric ulcer since November 2008.

She also started sunitinib 50 mg daily for renal cell carcinoma: first cycle from 22-Oct-2008 until 19-Nov-2009, and second cycle from 02-Dec-2008 to 30-Dec-2008. Diclofenac resinate 75 mg daily for postoperative pain had been initiated in November 2008. Heparin sodium (certoparin) with enoxaparin had been initiated for prophylaxis of thrombosis in November 2008.

On 30-Dec-2008, the patient was hospitalised with 4 to 5 week history of abdominal pain, vomiting, and 3-day history of tarry stools. Colonoscopy was interrupted due to accretions. On admission, she was pale with reduced general condition. Blood pressure was 145/75 mmHg.

Gastroscopy revealed normal esophagus mucosa, in the cardia fibrin covered lesions, no acute bleeding on time, inflammatory changed mucosa in the cardia, remain stomach and duodenum normal. In summary: suspicion of occurred bleeding on the cardia. Blood work-up was also performed, and revealed leukopenia at 3.0 Gpt/mL.

From 30-Dec-2008 to 06-Jan-2009, she received cefotiam (Spizef).

From 30-Dec-2008 to 09-Jan-2009, she received metronidazole (Clont) with suspicion of colitis (inflammation).

From 30-Dec-2008 to 06-Jan-2009, proton pump inhibitor, at high dose intravenous followed by oral therapy were given.

On 06-Jan-2009, rectoscopy showed mild inflammation in distal sigma without stenosis, no diverticel. X-ray after contrast agent enema (only assessment of distal colon): loop formation in the sigma region, individual diverticel, no certain signs of inflammation and no passages barriers.

On 07-Jan-2009, transfusion of one unit of red blood cells was given.

In the course of therapy, regressive abdominal and paraclinical symptoms were noticed. Circulatory condition became stable, and tarry stools stopped on 07-Jan-2009.

Explanation to the adverse drug reaction:

The symptoms on admission tarry stools, decreased haemoglobin value and gastroscopy results assumed a gastrointestinal haemorrhage which was induced or increased through the intake of diclofenac, sunitinib, clopidogrel and certoparin. Alternate cause: stress-related lesions of pains.

Explanation to leucopenia:

Leucopenia under therapy with sunitinib

Description: The haemogram at admission showed leucopenia (3.0 Gpt/ml). After discharge and after discontinuation of sunitinib leucocytes were 9.09 Gpt/ml (normal findings).

Explanation to the causalities:

Medication: clopidogrel

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: Increased bleedings because of inhibition of thrombocyte aggregation.

Medication: diclofenac

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: leucopenia.

Medication: diclofenac

Adverse drug reaction: leucopenia

Explanation: Leucopenia is principally known as an adverse event under metamizole. In this case there was a normalization of the number of leucocytes in spite of further administration of metamizole; a relation is not likely.

Medication: amitriptyline

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: leucopenia.

Medication: amitriptyline

Adverse drug reaction: leucopenia

Explanation: Leucopenia is principally known as an adverse event under amitriptyline. In this case there was a normalization of the number of leucocytes in spite of further administration of amitriptyline; a relation is not likely.

Medication: fentanyl

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting.

Medication: heparin sodium

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: Increased bleedings through anticoagulation.

Medication: Voltaren resinate

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: mucosa damage of the cardia and inhibition of thrombocyte aggregation are expected in the adverse events spectrum. Alternate cause: stress-induced damage of the mucosa with pains.

Medication: sunitinib

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting, upper abdominal disorder, neutropenia, gastrointestinal bleeding, CYP3A4 substrate.

Medication: sunitinib

Adverse drug reaction: leucopenia

Explanation: Neutropenia is a known adverse event under sunitinib. After discontinuation normalization of the number of leucocytes.

Medication: colecalciferol

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting

Medication: memantine

Adverse drug reaction: upper gastrointestinal haemorrhage

Explanation: SmPC: vomiting

The reporter considered upper gastrointestinal haemorrhage possibly related to sunitinib, likely related to voltaren, possibly related to clopidogrel, possibly related to heparin, dubiously related to amitriptyline, dubiously related to amlodipine, dubiously related to colecalciferol, dubiously related to memantine, dubiously related to fentanyl, dubiously related to metamizol and dubiously related to pantoprazole.

The reporter considered leucopenia possibly related to sunitinib, dubiously related to voltaren, dubiously related to clopidogrel, dubiously related to heparin, dubiously related to amitriptyline, dubiously related to amlodipine, dubiously related to colecalciferol, dubiously related to memantine, dubiously related to fentanyl, dubiously related to metamizole and dubiously related to pantoprazole.

Additional information received on 06-Apr-2009:

Sunitinib was administered from 22-Oct-2008 to 19-Nov-2008 (first cycle), and from 03-Dec-2008 to 30-Dec-2009 (second cycle).

No further information provided.

Additional information from a physician received on 07-Jun-2010 via BMS (Local Market Number 2009032701, no cares number) through Regional Pharmacovigilance Center Helios (DE-PVZIBE-ROSTOCK-10119) via German Health Authority BfArM (DE-BFARM-09039416):

The patient's medical history also included ongoing depression.

The dose of fentanyl was updated to 25 mcg/h.

After stopping sunitinib, the leucopenia normalized (i.e. resolved).

Gastrointestinal haemorrhage possibly related to diclofenac resinate (method WHO score).

Company off note:

-heparin was suspect drug whereas enoxaprine (ongoing) was not suspected

-drug interaction only between clopidogrel and heparin both stopped

-hospitalization until 09-Jan-2009, complete resolution of events

No further information provided. Case closed.

NOVARTIS, 22.06.2010 DE-002147023-PHHY2009DE11802

Sender diagnosis: not reported

Narrative

Initial report received from regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-09039416) on 27 Mar 2009: The patient's medical history included renal cell carcinoma, bone metastases (total endoprosthesis operation done on 13 Nov 2008), apoplexy and concurrent conditions of hypertension arterial, cataract, dementia Alzheimer's type, and sleep disorder. This patient was on multitherapy, including Voltaren resinat (diclofenac) for postoperative pain (start date Nov 2008), Sutent (sunitinib) for renal cell carcinoma, Iscover (clopidogrel sulfate) for apoplexy and Mono-embolex (certoparin; heparin fraction, sodium salt) for thrombosis prophylaxis. The patient was hospitalized on 30 Dec 2008 with complaints of abdominal pain since three to four weeks, vomiting since five days and tarry stools since 27 Dec 2008. Also nausea was predominant after intake of Sutent. At hospitalization, the patient was pale with reduced general condition, blood pressure was 145/75 mm Hg. The lab tests done on 30 Dec 2008 revealed increased creatinine 115 µmol/l (normal less than 88), aspartate aminotransferase 0.73 Ukat/l (normal less than 0.58), gamma GT 1.45 Ukat/l (normal less than 0.65), hemoglobin 6.3 mmol/l (normal range 6.9 to 9.5), hematocrit 0.30 SI-Einheit (normal range 0.34 to 0.45), Total White blood cell count 3.3 10E9/l (normal range 4 to 10), C reactive protein 136 mg/l (normal less than 5) and sodium 128 mmol/l (normal range 136 to 152). Leukopenia was also reported. Gastrointestinal hemorrhage or colitis was suspected. The patient was treated with Spizef (cefotiam hydrochloride), Clexane (enoxaparin sodium, heparin-fraction, sodium salt), Clont (metronidazole) and proton pump inhibitors, and was also transfused with 2 units of erythrocyte concentrates. The abdominal symptoms decreased, and tarry stools also recovered on 07 Jan 2009. The tarry stools, low hemoglobin and results of gastroscopy (inflammatory changes in mucosa of cardiac stomach, suspicion of state after hemorrhage of cardiac stomach) led to the diagnosis of upper gastrointestinal hemorrhage, which was suspected to be caused or worsened by intake of diclofenac, sunitinib, clopidogrel and certoparin. All the suspect drugs were discontinued on 30 Dec 2008 (Iscover discontinued on 01 Jan 2009). The alternative cause was supposed to be the lesion caused by stress. The outcome for gastrointestinal hemorrhage and leucopenia was reported as complete recovery on 30 Dec 2008. The patient was discharged on 09 Jan 2009.

Follow-up information was received from the regional pharmacovigilance center (intensive monitoring project) via Health Authority on 07 Jun 2010: Treatment with Sutent (sunitinib) was discontinued on 30 Dec 2008. On 30 Dec 2008, patient's red blood cell count was 3.36 Tpt/l (normal range 4.0-5.5 Tpt/l). In the further course the abdominal symptoms and paraclinic were regressive, the circulatory system was stable. On 06 Jan 2009 during a

rectoscopy a mild inflammation of the distal sigmoid colon without stenosis was found. There were no diverticula. After giving a contrast agent the distal colon was found to have lemnisci in the sigmoid region and some diverticula. The tarry stool lasted until 07 Jan 2009. In a gastroscopy done on 30 Dec 2009, there were fibrin-covered lesions in the cardia found and the mucosa was changed due to inflammation. There was no acute bleeding, the oesophageal mucosa, the duodenum and the rest of the gaster were without findings. It was suspected that there had been a bleeding in the cardia in the past. As explanation it was added that leucopenia was a common event when taking Sutuent. After stopping of Sutuent the leucocytes were normalising. The causality was reported as suspected for upper gastrointestinal hemorrhage. There was no more information expected, therefore the case was closed. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event "blood sodium decreased, leucopenia" and "Voltaren Resinat, Mono-Embolex" as suspected based on the current available information; the other events were assessed as non-serious.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09042861

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19870122	Age: 22 Year	weight (kg): 57	BMI_int: 20.2	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute fulminant hepatitis	20090207			9 Day	recovering/resolving
2 Weakness					not recovered/not resolved
3 Ascites					recovering/resolving
4 Body temperature increased	20090207			9 Day	recovering/resolving
5 Chills	20090207			9 Day	recovering/resolving
6 Headache					recovering/resolving
7 Hepatic function decreased					recovering/resolving
8 Hepatosplenomegaly					recovering/resolving
9 Liver failure					recovering/resolving
10 Vomiting					recovering/resolving
11 Liver damage	20090207				recovering/resolving
12 Hepatic lesion	20090207				recovering/resolving
13 Urine color abnormal		20090218			recovered/resolved
14 Explorative laparotomy	20090211			13 Day	Unknown
15 Abdominal pain	20090207			9 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CLEXANE enoxaparin sodium	Subcutaneous	0.4 Mg milligram(s)/Day 120	20090121- -	Thrombosis prophylaxis
2 ibuprofen ibuprofen	Oral	400 Mg milligram(s)/Day 245	20090121-20090130 10 Day	Pain NOS
3 OXYGESIC oxycodone hydrochloride	Oral	30 Mg milligram(s)/Day 212	20090130-20090210 12 Day	Pain NOS
4 PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/ Day 245	20090121-20090210 21 Day	Gastric ulcer prophylaxis
5 voltaren diclofenac sodium	Rectal	100 Mg milligram(s)/Day 242	20090130-20090210 12 Day	Pain NOS

Drugs (concomitant)

6 BERLOSIN metamizole	Oral	500 Mg milligram(s)/Day 245	20090121-20090130 10 Day	Pain NOS
7 sex hormones and modulators of the geni. sys.		/	2002-20090121 7 Year	Contraception

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Intervertebral disc replacement	20090127		Unknown	
Abdominal injury			Unknown	blunt abdominal trauma
Pain NOS	20090121		Yes	
Lumbar vertebral fracture	20090121		Yes	lumbar vertebral fracture
Road traffic accident	20090121	20090121	No	Z.n.

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Qualification: Other Health Professional

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20090223	0.62	µmol/l*s	
Aspartate aminotransferase	20090226	0.23	µmol/l*s	
Alanine aminotransferase	20090223	0.87	µmol/l*s	
Alanine aminotransferase	20090226	0.72	µmol/l*s	
Bilirubin total	20090223	24	µmol/l	
Bilirubin direct	20090226	19.9	µmol/l	
Alkaline phosphatase	20090226	1.62	µmol/l*s	
Gamma GT	20090223	2.05	µmol/l*s	
Gamma GT	20090226	1.76	µmol/l*s	
Quick's test	20090210	64	Prozent	
Quick's test	20090223	80	Prozent	
Quick's test	20090226	99	Prozent	
Albumin	20090223	60	g/l	
C-reactive protein	20090210	192	mg/l	
C-reactive protein	20090223	3.3	mg/l	
C-reactive protein	20090226	2	mg/l	
Creatinine	20090223	72	µmol/l	
Gamma GT	20090226	1.76	µmol/l*s	

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 03.04.2009 DE-PVZIBE-JENA-10072

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

fulminante Hepatitis unter Diclofenac, Pantoprazol, Oxycodon

KURZZUSAMMENFASSUNG:

fulminante Hepatitis mit Synthesestörung

BESCHREIBUNG:

Überweisung der jungen Patientin aus Heimatkrankenhaus zur Exploration Lebertransplantation bei Leberversagen infolge fulminanter Hepatitis. Ab 07.02.09 intermittierende Temperaturanstiege, Schüttelfrost, Bauch- und Kopfschmerzen. Aufnahme im Heimatkrankenhaus am 10.02.2009. Dort am 11.02.2009 explorative Laparotomie bei V.a. Milzruptur bei freier Flüssigkeit im Abdomen und Z.n. Verkehrsunfall 21.01.09 mit Fraktur eines Lendenwirbels und operativer Versorgung (27.01.09 Wirbelkörper-Ersatz), sowie Z.n. stumpfem Bauchtrauma. Bei freier Flüssigkeit handelte es sich um Aszites. Erbrechen bis 18.02.09, Urin dunkel (braun). Bei Eintreffen in unserer Klinik geht es Patientin schon besser, subjektiv noch geschwächt, aber sonst beschwerdefrei.

THERAPIE:

Heimatkrankenhaus: 10.02.-23.02.09 Intensivstation, Diagnostik. 10.02.09 Pantozol, Voltaren, Oxygesic abgesetzt. 23.02.-27.02.09 Clexane 0.4 mg/d, Diagnostik.

VERLAUF:

Überweisungsdiagnose: extreme Hepatosplenomegalie und Lebersynthesestörung unklarer Genese. In Heimatkrankenhaus bereits autoimmune Hepatitis, primäre biliäre Zirrhose, frische virale Infektion (CMV, HSV, EBV, Adenovirus, Parvo-Virus, Paramyxovirus, Mumps-, Masern-, RSV, Coxsackie-Virus, HIV, Hepatitis A,B,C, etc) ausgeschlossen. 13.02.09 MEGX-Test (Leberfunktionstest) < 20 µg/ml --> stark erniedrigt. 20.02.09 MEGX-Test nach 15 min 81,5 µg/ml, 30 min 71,5 µg/ml --> immer noch stark erniedrigt. Leider sind aus dem Aufenthalt im Heimatkrankenhaus keine Leberwerte übermittelt. Temperatur 23.02. 37,6°C; 24.02. 36,8°C. 24.02.09 SLA-AAK, LKM-AAK negativ --> keine autoimmune Hepatitis, AMA-AAK negativ --> keine primäre biliäre Zirrhose, ANCA - Titer negativ --> keine primäre sklerosierende Cholangitis.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PANTOZOL**BETREFFENDE UAW:** akute fulminante Hepatitis

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: OXYGESIC**BETREFFENDE UAW:** akute fulminante Hepatitis

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: VOLTAREN**BETREFFENDE UAW:** akute fulminante Hepatitis

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

NOVARTIS, 14.04.2009 DE-002147023-PHHY2009DE12954

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (HA reference number: DE-BFARM-09042861), and also via Nycomed (0204946) on 03 Apr 2009: The patient had road traffic accident on 21 Jan 2009 with blunt abdominal trauma, which resulted in lumbar vertebral fracture and the patient underwent lumbar vertebral replacement on 27 Jan 2009. The polymedicated patient was on Voltaren (diclofenac sodium) suppository from 30 Jan 2009 for pain. The patient was also on Oxygesic (oxycodone hydrochloride) for pain and Pantozol (pantoprazole) for gastric ulcer prophylaxis, which were also considered suspect. Since 07 Feb 2009 the patient had intermittent temperature increases, chills, abdominal pain and headache. The patient was admitted to hospital on 10 Feb 2009. On 11 Feb 2009 explorative laparotomy was done for the suspicion of spleen rupture with free fluid in abdomen (due to blunt abdominal trauma suffered during the road traffic accident). The free fluid was assessed as ascitis, possible spleen rupture was excluded. Extreme hepatosplenomegaly and liver synthesis impairment was found. Laboratory tests done on 10 Feb 2009 revealed increased C-reactive protein 192 mg/l (normal 0-7.5). On 13 Feb 2009, revealed severely decreased Monoethyl glycine xylidide (MEGX) test (liver function deterioration; Liver function test values were not reported), on 23 Feb 2009 revealed aspartate aminotransferase 0.62 umol/l (normal less than 0.52), Alanine aminotransferase 0.87 umol/l (normal less than 0.56), bilirubin total 24 umol/l (normal less than 21), gamma GT 2.05 umol/l (normal 0-0.63), albumin 60 g/l (normal 35-53), normal C reactive protein (3.3), and on 26 Feb 2009, bilirubin direct 19.9 umol/l (normal 0-3), normal aspartate aminotransferase (0.23). A diagnosis of acute fulminant hepatitis with liver failure was made. This event was considered life threatening. The onset for acute fulminant hepatitis was reported to be 07 Feb 2009. The patient complained of vomiting and dark urine (brown) until 18 Feb 2009. The clinicians regarded causal relationship as most probable for diclofenac, Oxygesic and Pantozol were also suspected. The therapy with all three drugs was discontinued on 10 Feb 2009, and the outcome of the patient was reported to be condition improving for all the symptoms except for weakness.

HELIOS Klinikum Wuppertal, 21.04.2010 DE-PVZIBE-JENA-10072

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschädigung

KURZZUSAMMENFASSUNG:

Leberschädigung mit Synthesestörung unter Diclofenac, Pantoprazol, Oxycodon und Clexane

BESCHREIBUNG:

Überweisung der jungen Patientin aus Heimatkrankenhaus zur Exploration Lebertransplantation bei Leberversagen infolge fulminanter Hepatitis. Ab 07.02.09 intermittierende Temperaturanstiege, Schüttelfrost, Bauch- und Kopfschmerzen. Aufnahme im Heimatkrankenhaus am 10.02.2009. Dort am 11.02.2009 explorative Laparotomie bei V.a. zweizeitige Milzruptur bei freier Flüssigkeit im Abdomen und Z.n. Verkehrsunfall 21.01.09 mit Fraktur eines Lendenwirbels und operativer Versorgung (27.01.09 Wirbelkörper-Ersatz), sowie Z.n. stumpfem Bauchtrauma. Bei freier Flüssigkeit handelte es sich um Aszites. Erbrechen bis 18.02.09, Urin dunkel (braun). Bei Eintreffen in unserer Klinik geht es Patientin schon besser, subjektiv noch

geschwächt, aber sonst beschwerdefrei.

THERAPIE:

Heimatkrankenhaus: 10.02.-23.02.09 Intensivstation, Diagnostik. 10.02.09 Pantozol, Voltaren, Oxygesic abgesetzt. 23.02.-27.02.09 Clexane 0.4 mg/d, Diagnostik.

VERLAUF:

Überweisungsdiagnose: extreme Hepatosplenomegalie und Lebersynthesestörung unklarer Genese. In Heimatkrankenhaus bereits autoimmune Hepatitis, primäre biliäre Zirrhose, frische virale Infektion (CMV, HSV, EBV, Adenovirus, Parvo-Virus, Paramyxovirus, Mumps-, Masern-, RSV, Coxsackie-Virus, HIV, Hepatitis A,B,C, etc) ausgeschlossen. 13.02.09 MEGX-Test (Leberfunktionstest) < 20 µg/ml --> stark erniedrigt. 20.02.09 MEGX-Test nach 15 min 81,5 µg/ml, 30 min 71,5 µg/ml --> immer noch stark erniedrigt. Leider sind aus dem Aufenthalt im Heimatkrankenhaus keine Leberwerte übermittelt. Temperatur 23.02. 37,6°C; 24.02. 36,8°C. 24.02.09 SLA-AAK, LKM-AAK negativ -->keine autoimmune Hepatitis, AMA-AAK negativ --> keine primäre biliäre Zirrhose, ANCA - Titer negativ --> keine primäre sklerosierende Cholangitis.

ERLÄUTERUNGEN ZUR UAW:

10.2. - 23.2 intensivmedizinische Betreuung in Heimat-KH wg fulminanter Hepatitis (dortige Diagnose). Schließlich bei Unsicherheit über den zukünftigen Verlauf und evtl Notwendigkeit einer Leber-Transplantation Verlegung zu uns. Hier bereits bei Vorstellung wesentlich besserer Zustand als erwartet mit weiterer Stabilisierung im Verlauf.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PANTOZOL

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: OXYGESIC

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: VOLTAREN

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: CLEXANE

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: It Fachinfo wird ein Anstieg der Leberenzyme häufig beschrieben. In diesem Fall negatives Dechallenge, da trotz Weitergabe des Medikaments Rückbildung der hepatischen Dysfunktion. Zusammenhang unwahrscheinlich.

NOVARTIS, 27.04.2010 DE-002147023-PHHY2009DE12954

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (HA reference number: DE-BFARM-09042861), and also via Nycomed (0204946) on 03 Apr 2009: The patient had road traffic accident on 21 Jan 2009 with blunt abdominal trauma, which resulted in lumbar vertebral fracture and the patient underwent lumbar vertebral replacement on 27 Jan 2009. The polymedicated patient was on Voltaren (diclofenac sodium) suppository from 30 Jan 2009 for pain. The patient was also on Oxygesic (oxycodone hydrochloride) for pain and Pantozol (pantoprazole) for gastric ulcer prophylaxis, which were also considered suspect. Since 07 Feb 2009 the patient had intermittent temperature increases, chills, abdominal pain and headache. The patient was admitted to hospital on 10 Feb 2009. On 11 Feb 2009 explorative laparotomy was done for the suspicion of spleen rupture with free fluid in abdomen (due to blunt abdominal trauma suffered during the road traffic accident). The free fluid was assessed as ascitis, possible spleen rupture was excluded. Extreme hepatosplenomegaly and liver synthesis impairment was found. Laboratory tests done on 10 Feb 2009 revealed increased C-reactive protein 192 mg/l (normal 0-7.5). On 13 Feb 2009, revealed severely decreased Monoethyl glycine xylylidide (MEGX) test (liver function deterioration; Liver function test values were not reported), on 23 Feb 2009 revealed aspartate

aminotransferase 0.62 umol/l (normal less than 0.52), Alanine aminotransferase 0.87 umol/l (normal less than 0.56), bilirubin total 24 umol/l (normal less than 21), gamma GT 2.05 umol/l (normal 0-0.63), albumin 60 g/l (normal 35-53), normal C reactive protein (3.3), and on 26 Feb 2009, bilirubin direct 19.9 umol/l (normal 0-3), normal aspartate aminotransferase (0.23). A diagnosis of acute fulminant hepatitis with liver failure was made. This event was considered life threatening. The onset for acute fulminant hepatitis was reported to be 07 Feb 2009. The patient complained of vomiting and dark urine (brown) until 18 Feb 2009. The clinicians regarded causal relationship as most probable for diclofenac, Oxygesic and Pantozol were also suspected. The therapy with all three drugs was discontinued on 10 Feb 2009, and the outcome of the patient was reported to be condition improving for all the symptoms except for weakness.

Follow-up report received from a regional Pharmacovigilance center (intensive monitoring project) via Health Authority on 22 Apr 2010: The patient was in intensive care unit due to fulminant hepatitis from 10 Feb 2010 to 23 Feb 2010 (diagnosis of home hospital) and transferred to Helios clinical. Patient's condition was stabilized. Clexane was continued, but the hepatic dysfunction was dissolved. The reporter assessed the causality as unlikely for Clexane and Ibuprofen, possible for Pantozol, Oxycodone and Diclofenac sodium.

sanofi-aventis, 27.04.2010 DE-SANOFI-AVENTIS-2010SA023580

Sender diagnosis: not reported

Narrative

Initial Report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-JENA-10072) and was extracted from the website of the German health authority (authority number DE-BFARM-09042861) on 23-Apr-2010

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This serious case involves a 22-year-old female patient. On 21-Jan-2009 she experienced a road traffic accident with fracture of a lumbar vertebrae and a blunt abdominal trauma. Six days later a vertebral body replacement was performed. Since 07-Feb-2009 she suffered from intermittent increases in temperature, chill, abdominal pain and headache. Three days later the patient was hospitalised. From 10-Feb-2009 until 23-Feb-2009 she was in an intensive-care unit due to a fulminant hepatitis. Pantoprazole sodium, diclofenac potassium and oxycodone hydrochloride were withdrawn on 10-Feb-2009. On 11-Feb-2009 exploratory laparotomy was performed with suspicion of a staged splenic rupture at free fluid in the abdomen. Ascites was diagnosed. On 13-Feb-2009 a liver function test was performed with the following finding: monoethylglycine xylidide was greatly decreased (<20 µg/ml). The patient suffered from vomiting until 18-Feb-2009, urine was dark (brown). On 20-Feb-2009 a further liver function test was performed with the following finding: monoethylglycine xylidide was still greatly decreased with 81.5 µg/ml after 15 min and 71.5 µg/ml after 30 min.

The patient was transferred to university hospital for evaluation of a liver transplantation because of hepatic failure due to fulminant hepatitis. Referral diagnosis: extreme hepatosplenomegaly and hepatic synthesis disorder of unknown pathogenesis. On admission the patient was nearly free of symptoms, but subjectively weakened. Primary biliary cirrhosis, fresh viral infections, autoimmune hepatitis and primary sclerosing cholangitis were excluded. The patient was recovering.

The reporter assessed the causal relationship according to WHO score for pantoprazole sodium and oxycodone hydrochloride as "possible", for diclofenac potassium as "probable" and for enoxaparin sodium and ibuprofen as "unlikely".

Explanatory notes on causality:

For pantoprazole sodium, oxycodone hydrochloride and diclofenac potassium: Liver damage was most likely due to drug-related toxicity by the intake of diclofenac. Pantoprazole sodium and oxycodone hydrochloride were also suspected. In addition, viral or operation related causes could not be ruled out with certainty. The liver function spontaneously recovered after withdrawal of the suspected medication.

For enoxaparin sodium: Increase in liver enzymes is frequently described in the Physician's information leaflet. In this case, however, dechallenge was negative. Despite continuation of treatment with the drug hepatic dysfunction regressed. Therefore, a causal relationship is unlikely.

Causality explanation concerning ibuprofen was not provided.

Bayer Vital GmbH, 28.04.2010 DE-BAYER-201023009GPV

Sender diagnosis: not reported

Narrative

This solicited case report was identified on 22-Apr-2010 in an internet-based pool of cases of BfArM, the regulatory health authority in Germany, reported from a German physician via local pharmacovigilance centre, which systematically screens for adverse drug reactions in selected hospitals. The report refers to a 22 year-old female (height: 168 cm, weight: 57 kg), who experienced hepatic lesion with synthesis disorder upon treatment with ibuprofen, diclofenac, pantoprazol, oxycodon, and enoxaparin.

Patient's concomitant conditions/risk factors included a road traffic accident on 21-Jan-2009, with lumbar vertebral fracture and pain ongoing since 21-Jan-2009. Concomitant drugs were given for pain. Contraceptives have been ceased on 21-Jan-2009 (see '*Concomitant Drugs' section).

Patient received the following drugs, reported as 'suspect drugs':

- oral Ibuprofen 400 mg tablet was given once daily from 21-Jan-2009 until 30-Jan-2009 for pain;
- oral Pantozol (pantoprazole) 40 mg tablet was given once daily from 21-Jan-2009 until 10-Feb-2009 for gastric ulcer prophylaxis;
- rectal Voltaren (diclofenac) 50 mg suppository was given twice daily from 30-Jan-2009 until 10-Feb-2009 for pain;
- oral Oxygesic (oxycodone) 10 mg prolonged-release tablet was given thrice daily from 30-Jan-2009 until 10-Feb-2009 for pain;
- subcutaneous Clexane (enoxaparin) 0.4 mg injection was given once daily from 21-Jan-2009 from thrombosis prophylaxis.

Onset, progression and outcome of the adverse reaction:

From 07-Feb-2009 onward, the 22 year-old female patient experienced intermittent rise in temperature, shivering, abdominal pain and headache.

On 10-Feb-2009, patient was admitted to hospital in her hometown.

On 11-Feb-2009, explorative laparotomy was performed for suspected two-timed splenic rupture with free abdominal fluid. Patient had experienced a traffic accident on 21-Jan-2009 with lumbar vertebra fracture which had to be treated surgically (replacement of vertebral body on 27-Jan-2009), and blunt abdominal trauma. Free fluid was identified as ascites.

Until 18-Feb-2009, the patient suffered from vomiting, her urine was found to be dark brown.

Patient was transferred to reporting clinic for the exploration of a liver transplantation for liver failure due to fulminant hepatitis.

On admission at this hospital, patient's condition had improved, patient felt that she was still weak, but was otherwise free from complaints.

Therapy:

Hospital in the patient's hometown: From 10-Feb-2009 to 23-Feb-2009, treatment at intensive care unit, diagnostics. On 10-Feb-2009: pantoprazol, diclofenac, oxycodon

withdrawn. From 23-Feb-2009 to 27-Feb-2009, enoxaparin 0.4 mg daily, diagnostics.

Course:

Diagnosis provided by the admitting hospital: extreme hepatosplenomegaly and liver synthesis disorder of unknown origin. In the hospital of patient's hometown already autoimmune hepatitis, primary biliary cirrhosis. Fresh viral infection was excluded (no cytomegaly virus, herpes simplex virus, Epstein-Barr virus, adenovirus, parvovirus, mumps, measles, respiratory syncytial virus, Coxsackie virus, human immunodeficiency virus, hepatitis A, B, C etc.). Monoethylglycine xylidide testing (liver function test) < 20 mcg/ml severely decreased. On 20-Feb-2009, the results of the MEGX test were still severely decreased: 81.5 mcg/ml after 15 m, 71.5 mcg/ml after 30 m. Unfortunately, no liver values were reported by the hospital in patient's hometown.

Temperature: 23-Feb-2009: 37.6°C, 24 Feb-2009: 36.8 °C. On 24-Feb-2009: soluble liver antigen auto-antibodies and liver kidney microsome auto-antibodies: negative, that meant (i.e.) there was no auto-immune hepatitis, anti-mitochondrial antibodies: negative, i.e. no primary biliary cirrhosis, anti-neutrophil cytoplasmic antibodies titre: negative, i.e. no primary sclerotic cholangitis.

Comments on the serious adverse event:

From 10-Feb-2009 to 23-Feb-2009, intensive care at hospital of patient's hometown due to fulminant hepatitis (diagnosis made there). Finally, due to uncertainty as to future course and possible necessity of a liver transplantation, patient was transferred to the reporting hospital. There, patient was found to be in an essentially better condition as had been expected. In the course, her condition stabilised further. At the time of the report, patient was recovering.

Event was considered serious due to its life-threatening nature and due to requiring hospitalization/prolongation of existing hospitalization.

Causality assessment:

Reporter assessed the diagnosis HEPATIC LESION to be probably related to Voltaren (diclofenac), to be possibly related to Pantozol (pantoprazole) and Oxygesic (oxycodone), and to be unlikely related to Ibuprofen and Clexane (enoxaparin) using method 'WHO score' for evaluation.

HELIOS Klinikum Wuppertal, 01.06.2010 DE-PVZIBE-JENA-10072

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschädigung

KURZZUSAMMENFASSUNG:

Leberschädigung mit Synthesestörung unter Diclofenac, Pantoprazol, Oxycodon und Clexane

BESCHREIBUNG:

Überweisung der jungen Patientin aus Heimatkrankenhaus zur Exploration Lebertransplantation bei Leberversagen infolge fulminanter Hepatitis. Ab 07.02.09 intermittierende Temperaturanstiege, Schüttelfrost, Bauch- und Kopfschmerzen. Aufnahme im Heimatkrankenhaus am 10.02.2009. Dort am 11.02.2009 explorative Laparotomie bei V.a. zweizeitige Milzruptur bei freier Flüssigkeit im Abdomen und Z.n. Verkehrsunfall 21.01.09 mit Fraktur eines Lendenwirbels und operativer Versorgung (27.01.09 Wirbelkörper-Ersatz), sowie Z.n. stumpfem Bauchtrauma. Bei freier Flüssigkeit handelte es sich um Aszites. Erbrechen bis 18.02.09, Urin dunkel (braun). Bei Eintreffen in unserer Klinik geht es Patientin schon besser, subjektiv noch geschwächt, aber sonst beschwerdefrei.

THERAPIE:

Heimatkrankenhaus: 10.02.-23.02.09 Intensivstation, Diagnostik. 10.02.09 Pantozol, Voltaren, Oxygesic abgesetzt. 23.02.-27.02.09 Clexane 0.4 mg/d, Diagnostik.

VERLAUF:

Überweisungsdiagnose: extreme Hepatosplenomegalie und Lebersynthesestörung unklarer Genese. In Heimatkrankenhaus bereits autoimmune Hepatitis, primäre biliäre Zirrhose, frische virale Infektion (CMV, HSV, EBV, Adenovirus, Parvo-Virus, Paramyxovirus, Mumps-, Masern-, RSV, Coxsackie-Virus, HIV, Hepatitis A,B,C, etc) ausgeschlossen. 13.02.09 MEGX-Test (Leberfunktionstest) < 20 µg/ml --> stark erniedrigt. 20.02.09 MEGX-Test nach 15 min 81,5 µg/ml, 30 min 71,5 µg/ml --> immer noch stark erniedrigt. Leider sind aus dem Aufenthalt im Heimatkrankenhaus keine Leberwerte übermittelt. Temperatur 23.02. 37,6°C; 24.02. 36,8°C. 24.02.09 SLA-AAK, LKM-AAK negativ -->keine autoimmune Hepatitis, AMA-AAK negativ --> keine primäre biliäre Zirrhose, ANCA - Titer negativ --> keine primäre sklerosierende Cholangitis.

ERLÄUTERUNGEN ZUR UAW:

10.2. - 23.2 intensivmedizinische Betreuung in Heimat-KH wg fulminanter Hepatitis (dortige Diagnose). Schließlich bei Unsicherheit über den zukünftigen Verlauf und evtl Notwendigkeit einer Leber-Transplantation Verlegung zu uns. Hier bereits bei Vorstellung wesentlich besserer Zustand als erwartet mit weiterer Stabilisierung im Verlauf.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PANTOZOL

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: OXYGESIC

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: VOLTAREN

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: CLEXANE

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: lt Fachinfo wird ein Anstieg der Leberenzyme häufig beschrieben. In diesem Fall negatives Dechallenge, da trotz Weitergabe des Medikaments Rückbildung der hepatischen Dysfunktion. Zusammenhang unwahrscheinlich.

sanofi-aventis, 18.06.2010 DE-SANOFI-AVENTIS-2010SA023580

Sender diagnosis: not reported

Narrative

Initial Report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-JENA-10072) and was extracted from the website of the German health authority (authority number DE-BFARM-09042861) on 23-Apr-2010

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This serious case involves a 22-year-old female patient. On 21-Jan-2009 she experienced a road traffic accident with fracture of a lumbar vertebrae and a blunt abdominal trauma. Six days later a vertebral body replacement was performed. Since 07-Feb-2009 she suffered from intermittent increases in temperature, chill, abdominal pain and headache. Three days later the patient was hospitalised. From 10-Feb-2009 until 23-Feb-2009 she was in an intensive-care unit due to a fulminant hepatitis. Pantoprazole sodium, diclofenac potassium and oxycodone hydrochloride were withdrawn on 10-Feb-2009. On 11-Feb-2009 exploratory laparotomy was performed with suspicion of a staged splenic rupture at free fluid in the abdomen. Ascites was diagnosed. On 13-Feb-2009 a liver function test was performed with the following finding: monoethylglycine xylidide was greatly decreased (<20 µg/ml). The patient suffered from vomiting until 18-Feb-2009, urine was dark (brown). On 20-Feb-2009 a further liver function test was performed with the following finding: monoethylglycine xylidide was still greatly decreased with 81.5 µg/ml after 15 min and 71.5 µg/ml after 30 min.

The patient was transferred to university hospital for evaluation of a liver transplantation because of hepatic failure due to fulminant hepatitis. Referral diagnosis: extreme hepatosplenomegaly and hepatic synthesis disorder of unknown pathogenesis. On admission the patient was nearly free of symptoms, but subjectively weakened. Primary biliary cirrhosis, fresh viral infections, autoimmune hepatitis and primary sclerosing cholangitis were excluded. The patient was recovering.

The reporter assessed the causal relationship according to WHO score for pantoprazole sodium and oxycodone hydrochloride as "possible", for diclofenac potassium as "probable" and for enoxaparin sodium and ibuprofen as "unlikely".

Explanatory notes on causality:

For pantoprazole sodium, oxycodone hydrochloride and diclofenac potassium: Liver damage was most likely due to drug-related toxicity by the intake of diclofenac. Pantoprazole sodium and oxycodone hydrochloride were also suspected. In addition, viral or operation related causes could not be ruled out with certainty. The liver function spontaneously recovered after withdrawal of the suspected medication.

For enoxaparin sodium: Increase in liver enzymes is frequently described in the Physician's information leaflet. In this case, however, dechallenge was

negative (?). Despite continuation of treatment with the drug hepatic dysfunction regressed. Therefore, a causal relationship is unlikely.

Causality explanation concerning ibuprofen was not provided.

Additional Information received on 07-Jun-2010:

We received additional information from the regional pharmacovigilance centre via the German Health Authority on 07-Jun-2010.

The ibuprofen drug was not further suspected to cause the liver damage and was reclassified to concomitant drug. Therefore, no causality assessment was necessary for ibuprofen.

The causal relationship to enoxaparin sodium was assessed as unlikely.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09043574

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 52 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GPT increased					Unknown
2 Sore throat					recovered/resolved
3 Common cold					recovered/resolved
4 Scales					Unknown
5 Pustule					recovering/resolving
6 Leucocytosis NOS					recovering/resolving
7 CRP increased					recovered/resolved
8 Fever					recovering/resolving
9 Swelling					recovered/resolved
10 Eyelid oedema					recovered/resolved
11 Oedema					recovering/resolving
12 Redness					recovered/resolved
13 General malaise					recovered/resolved
14 Acute generalised exanthematous pustulosis					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	Joint disorder
diclofenac			-	
2 penicillin		/	-	
benzylpenicillin			-	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Prange B., Marini A., Kalke A., Hodzic-Avdagic N., Ruzicka T., Hengge U.R. Acute localized exanthematous pustulosis. Journal der Deutschen Dermatologischen Gesellschaft. 2005;3(3):210-212	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bacterial culture		propionibacterium	MA	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 06.04.2009 DE-002147023-PHHY2009DE12265

Sender diagnosis: not reported

Narrative

Initial literature report received on 26 Mar 2009: This patient developed acute localized exanthematous pustulosis after intake of penicillin (manufacturer unknown) and diclofenac (manufacturer unknown) for joints complaints on unknown dates. She presented with a common cold with sore throat, and one day after penicillin therapy, she complained of generalized malaise, redness and swelling in front of the right ear and on the right cheeks as well as in the area of the right lower eyelid, and fever. At the time of the presentation, she a large painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localized exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with penicillin and diclofenac was negative. The reporter stated that the event was probably caused by penicillin or diclofenac.

The remaining reported leading event was assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

NOVARTIS, 14.04.2009 DE-002147023-PHHY2009DE12265

Sender diagnosis: not reported

Narrative

Initial literature report received on 26 Mar 2009: This patient developed acute localized exanthematous pustulosis after intake of penicillin (manufacturer unknown) and diclofenac (manufacturer unknown) for joints complaints on unknown dates. She presented with a common cold with sore throat, and one day after penicillin therapy, she complained of generalized malaise, redness and swelling in front of the right ear and on the right cheeks as well as in the area of the right lower eyelid, and fever. At the time of the presentation, she a large painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localized exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with penicillin and diclofenac was negative. The reporter stated that the event was probably caused by penicillin or diclofenac.

Follow-up information received on 01 Apr 2009: Full English translation received. Bacterial culture of the affected area showed propionibacteria. The treatment of the patient consisted on discontinuation of the suspect drug and saline-moistened compress.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09044899

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19500904	Age: 58 Year	weight (kg): 82	BMI_int: 30.12	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vaginal bleeding	20090323	20090326	4 Day	3 Week	recovering/resolving
2 Liver function tests raised					Unknown
3 GGT increased					Unknown
4 Raised liver function tests	20090325			23 Day	Unknown
5 Drug interaction					Unknown
6 Hemoglobin decreased					recovering/resolving
7 Hepatic steatosis					Unknown
8 Lower abdominal pain	20090321			19 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Unknown	150 Mg milligram(s)/Day	20090303-20090322 20 Day	Drug use for unknown indication
2 Xarelto rivaroxaban	Oral	10 Mg milligram(s)/Day 245	20090303-20090324 22 Day	Hip arthroplasty

Drugs (concomitant)

3 Arixtra fondaparinux sodium		/	-	Thrombosis prophylaxis
4 Lyrika / pregabalin pregabalin	Unknown	150 Mg milligram(s)/Day	20090303-20090322 20 Day	Drug use for unknown indication
5 MST morphine sulfate	Unknown	30 Mg milligram(s)/Day	-	Pain NOS
6 Novalgine metamizole	Unknown	60 Gtt drop(s)/Day 158	20090303-20090322 20 Day	Drug use for unknown indication
7 Pantozol pantoprazole sodium	Unknown	40 Mg milligram(s)/Day	20090303-20090322 20 Day	Drug use for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Haemorrhoids			Unknown	haemorrhoids
Hip arthroplasty	200311		Unknown	hip total endoprosthesis left
Spondylolisthesis			Unknown	spondylolisthesis
Basophil percentage increased	20060210		Unknown	
Eosinophil percentage increased	20060210		Unknown	
Drug hypersensitivity			Unknown	hypersensitivity to heparin
Diverticulitis intestinal	200602		No	sigma diverticulitis
Monocyte percentage increased	20060210		Unknown	
Sigmoidectomy	200602		Unknown	sigma resection
Coxarthrosis			Unknown	coxarthrosis
Colonic diverticulosis			Unknown	sigma diverticulosis

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit weitere Erläuterung
Alkaline phosphatase	20090402	179	U/L
Alkaline phosphatase	20090330	189	U/L
Alkaline phosphatase	20090327	225	U/L
Alkaline phosphatase	20090325	208	U/L
Bilirubin total	20090325	0.43	MG/DL
Creatinine	20090317	0.9	MG/DL
Gamma-glutamyltransferase	20090324	235	U/L
Gamma-glutamyltransferase	20090317	365	U/L
Gamma-glutamyltransferase	20090330	318	U/L
Gamma-glutamyltransferase	20090325	327	U/L
Gamma-glutamyltransferase	20090327	362	U/L
Gamma-glutamyltransferase	20090402	288	U/L
Hematocrit	20090324	36	%
Hematocrit	20090317	39	%
Hematocrit	20090325	42	%
Hematocrit	20090327	40	%
Hematocrit	20090402	42	%
Hemoglobin	20090317	11.9	G/DL
Hemoglobin	20090324	10.8	G/DL
Hemoglobin	20090327	12.5	G/DL
Hemoglobin	20090325	12.6	G/DL
Hemoglobin	20090402	12.7	G/DL
Platelet count	20090402	332	GIGA/L
Platelet count	20090325	483	GIGA/L
Platelet count	20090327	409	GIGA/L
Platelet count	20090324	360	GIGA/L
Platelet count	20090317	465	GIGA/L
Red blood cell count	20090325	4.8	T/L
Red blood cell count	20090324	4.0	T/L
Red blood cell count	20090317	4.4	T/L
Red blood cell count	20090402	4.7	T/L
Red blood cell count	20090327	4.6	T/L
SGOT	20090402	21	U/L
SGOT	20090330	21	U/L
SGOT	20090327	40	U/L
SGOT	20090327	31	U/L
SGPT	20090327	77	U/L
SGPT	20090402	42	U/L
SGPT	20090330	49	U/L
SGPT	20090325	45	U/L

White blood cells	20090317	9.4	GIGA/L
White blood cells	20090324	7.1	GIGA/L
White blood cells	20090325	8.4	GIGA/L
White blood cells	20090327	8.0	GIGA/L
White blood cells	20090402	8.2	GIGA/L
Alanine aminotransferase	20090327	77U/L	NA
Alanine aminotransferase	20090402	42U/L	NA
Alanine aminotransferase	20090325	45U/L	NA
Sigmoidoscopy	20090324	nicht vorhanden	NAabsent
Ultrasound examination		nicht vorhanden	NAabsent
Activated partial thromboplastin time	20060210	30	S
Alkaline phosphatase	20060210	122	U/L
Bilirubin total	20060210	0.65	MG/DL
C-reactive protein	20060210	0.43	MG/DL
Cholesterol total	20060210	231	MG/DL
Creatinine	20060210	0.8	MG/DL
Gamma-glutamyltransferase	20060210	269	U/L
HDL	20060210	49	MG/DL
Hematocrit	20060210	43	%
Hemoglobin	20060210	13.5	G/DL
Low density lipoprotein cholesterol	20060210	150	MG/DL
Lymphocytes	20060210	43	%
Mean cell haemoglobin concentration	20060210	32	G/DL
Mean cell volume	20060210	88	FL
Platelet count	20060210	384000	/MCL
Potassium	20060210	4.7	MMOL/L
Prothrombin time ratio	20060210	1.0	/1
Red blood cell count	20060210	4.8	T/L
Serum glucose	20060210	100	MG/DL
SGOT	20060210	36	U/L
SGPT	20060210	55	U/L
Sodium	20060210	140	MMOL/L
Triglycerides	20060210	143	MG/DL
Urea	20060210	4.8	MG/DL
White blood cells	20060210	6300	/MCL
Lipase	20060210	40.2	U/L

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 07.04.2009 DE-BAYER-200917657GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 25-Mar-2009 from a health professional in Germany, via company's sales representative, referring to a 58 year-old Caucasian female.

This case includes follow up information from 26-Mar-2009.

Medical and drug history:

Patient's relevant medical history includes cox arthrosis, hip total endoprosthesis and sigma resection. Concomitant drugs have been given.

Suspect drug, timing and conditions surrounding the onset of the reaction:

Patient received Xarelto (rivaroxaban) for hip tep, anticoagulation. Administration began on 03-Mar-2009 until 22-Mar-2009 at a total daily dose of 10 MG oral.

The progression of the event and its outcome in the patient:

On 23-Mar-2009, patient experienced VAGINAL BLEEDING (lower abdominal pain, hemoglobin decreased) considered serious due to hospitalization. Xarelto was withdrawn and the patient improved.

The patient underwent hip total endoprosthesis surgery on 03-Mar-2009, and was treated with Xarelto (rivaroxaban). From 21-Mar-2009 onward, patient experienced unclear lower abdominal pain. Additionally, on 23-Mar-2009, vaginal bleeding set in. The patient's hemoglobin value decreased by 1.1 g/dl. The reporter considered the adverse events to be associated with the suspected drug. He gave the following reasons for his assessment: 1st a rectoscopy was performed without revealing any findings, 2nd lower abdominal sonography had proved suspicious (no further explanation provided), and 3rd anticoagulation.

According to the reporter, the adverse events had ended on 26-Mar-2009, on the other hand, the patient was reported having only partially recovered.

The patient received Xarelto for prophylaxis postoperative after hip TEP. During following rehabilitation procedure the patient developed vaginal bleeding. After clarifying examinations like ultrasound etc. anticoagulation therapy was changed to Heparin.

Currently no further information available.

The original reporter's clinical assessment:

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related.

Bayer Vital GmbH, 25.05.2009 DE-BAYER-200917657GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 25-Mar-2009 from a health professional in Germany, via company's sales representative, referring to a 58 year-old Caucasian female.

This case includes follow up information from 26-Mar-2009.

Medical and drug history:

Patient's relevant medical history includes cox arthrosis, hip total endoprosthesis and sigma resection. Concomitant drugs have been given.

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The patient received Xarelto for prophylaxis postoperative after hip TEP. During following rehabilitation procedure the patient developed vaginal bleeding. After clarifying examinations like ultrasound etc. anticoagulation therapy was changed to Heparin.

Currently no further information available.

The original reporter's clinical assessment:

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related.

****FU 11-May-2009:**

The patient was hospitalized for rehabilitation from 16-Mar-2009 to 06-Apr-2009 after hip joint total endoprosthesis (right).

Additional concomitant conditions have been reported: spondylolisthesis, hypersensitivity to heparin, and intolerance of Xarelto. Concomitant drugs have been added.

Information on suspect drugs has been updated.

The patient received:

- Xarelto (rivaroxaban) as anticoagulation treatment in context of the hip total endoprosthesis operation (right) from 03-Mar-2009 until 24-Mar-2009 (at

a total daily dose of 10 MG, orally

- Voltaren (diclofenac) from 03-Mar-2009 until 22-Mar-2009 (at a total daily dose of 150 MG, unknown route of administration)

Therapy with Xarelto was stopped due to lower abdominal pain and vaginal bleeding on 24-Mar-2009. For anticoagulation the patient was treated with Arixtra (fondaparinux sodium) at that point of time.

Ultrasound: no visible extraluminal fluid in abdominal area. Menorrhagia in the region of uterus/utero-vaginal could not be excluded for sure.

Control of liver values and alkaline phosphatase was recommended due to steatosis hepatis. Further advises included withdrawal of Arixtra and non-steroidal antirheumatics.

Sigmoidoscopy: no identifiable bleeding source, some evidence for remaining diverticula, bland anastomosis after sigma resection, interior haemorrhoids without signs of acute or previous bleeding on 24-Mar-2009.

Lab tests revealed increased GGT (on 17-Mar-2009: 365 U/l; on 02-Apr-2009: 288 U/l) and increased alkaline phosphatase. Also slightly increased SGOT and SGPT were observed. Event term: INCREASED LIVER FUNCTION TEST.

Patient's family physician was advised to check laboratory values and to perform gastroenterological controls for ruling out rectal bleeding from diverticulosis.

Lab tests have been added; see "Lab Data" section.

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related to Xarelto, and not related to Voltaren. The reporter disclosed that INCREASED LIVER VALUES were attributed to therapy with Xarelto.

Bayer Vital GmbH, 26.05.2009 DE-BAYER-200917657GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 25-Mar-2009 from a health professional in Germany, via company's sales representative, referring to a 58 year-old Caucasian female.

This case includes follow up information from 26-Mar-2009.

Medical and drug history:

Patient's relevant medical history includes cox arthrosis, hip total endoprosthesis and sigma resection. Concomitant drugs have been given.

Suspect drug, timing and conditions surrounding the onset of the reaction:

Patient received Xarelto (rivaroxaban) for hip tep, anticoagulation. Administration began on 03-Mar-2009 until 22-Mar-2009 at a total daily dose of 10 MG oral.

The progression of the event and its outcome in the patient:

On 23-Mar-2009, patient experienced VAGINAL BLEEDING (lower abdominal pain, hemoglobin decreased) considered serious due to hospitalization. Xarelto was withdrawn and the patient improved.

The patient underwent hip total endoprosthesis surgery on 03-Mar-2009, and was treated with Xarelto (rivaroxaban). From 21-Mar-2009 onward, patient experienced unclear lower abdominal pain. Additionally, on 23-Mar-2009, vaginal bleeding set in. The patient's hemoglobin value decreased by 1.1 g/dl. The reporter considered the adverse events to be associated with the suspected drug. He gave the following reasons for his assessment: 1st a rectoscopy was performed without revealing any findings, 2nd lower abdominal sonography had proved suspicious (no further explanation provided), and 3rd anticoagulation.

According to the reporter, the adverse events had ended on 26-Mar-2009, on the other hand, the patient was reported having only partially recovered.

The patient received Xarelto for prophylaxis postoperative after hip TEP. During following rehabilitation procedure the patient developed vaginal bleeding. After clarifying examinations like ultrasound etc. anticoagulation therapy was changed to Heparin.

Currently no further information available.

The original reporter's clinical assessment:

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related.

****FU 11-May-2009:**

The patient was hospitalized for rehabilitation from 16-Mar-2009 to 06-Apr-2009 after hip joint total endoprosthesis (right).

Additional concomitant conditions have been reported: spondylolisthesis, hypersensitivity to heparin, and intolerance of Xarelto. Concomitant drugs have been added.

Information on suspect drugs has been updated.

The patient received:

- Xarelto (rivaroxaban) as anticoagulation treatment in context of the hip total endoprosthesis operation (right) from 03-Mar-2009 until 24-Mar-2009 (at a total daily dose of 10 MG, orally

- Voltaren (diclofenac) from 03-Mar-2009 until 22-Mar-2009 (at a total daily dose of 150 MG, unknown route of administration)

Therapy with Xarelto was stopped due to lower abdominal pain and vaginal bleeding on 24-Mar-2009. For anticoagulation the patient was treated with

Arixtra (fondaparinux sodium) at that point of time.

Ultrasound: no visible extraluminal fluid in abdominal area. Menorrhagia in the region of uterus/utero-vaginal could not be excluded for sure.

Control of liver values and alkaline phosphatase was recommended due to steatosis hepatis. Further advises included withdrawal of Arixtra and non-steroidal antirheumatics.

Sigmoidoscopy: no identifiable bleeding source, some evidence for remaining diverticula, bland anastomosis after sigma resection, interior haemorrhoids without signs of acute or previous bleeding on 24-Mar-2009.

Lab tests revealed increased GGT (on 17-Mar-2009: 365 U/l; on 02-Apr-2009: 288 U/l) and increased alkaline phosphatase. Also slightly increased SGOT and SGPT were observed. Event term: INCREASED LIVER FUNCTION TEST.

Patient's family physician was advised to check laboratory values and to perform gastroenterological controls for ruling out rectal bleeding from diverticulosis.

Lab tests have been added; see "Lab Data" section.

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related to Xarelto, and not related to Voltaren. The reporter disclosed that INCREASED LIVER VALUES were attributed to therapy with Xarelto.

NOVARTIS, 27.05.2009 DE-002147023-PHHY2009DE19970

Sender diagnosis: not reported

Narrative

Combined report of both the initial and follow up reports received from health care professional on 11 May 2009 and forwarded by License partner (Bayer Schering reference number: 200917657GPV) on 22 May 2009: The patient had historical condition of, hip total endoprosthesis and sigma resection. The concurrent condition of the patient included cox arthrosis, spondylolisthesis, sigma diverticulitis, hypersensitivity to heparin and haemorrhoids. She was on Voltaren (diclofenac sodium) for unknown indication from 03 Mar 2009 and Xarelto (rivaroxaban) for hip total endoprosthesis right and anticoagulation at a daily dose of 10mg from 03 Mar 2009 which was considered as co-suspect in this case. On 21 Mar 2009 the patient experienced lower abdominal pain and on 23 Mar 2009 vaginal bleeding started and hemoglobin (11.9 g/dL) decreased which were considered serious due to hospitalization. Xarelto was withdrawn on 24 Mar 2009 and the patient improved. The reporter considered the diagnosis vaginal bleeding to be probably related Xarelto, and not related to Voltaren. The patient was hospitalized for rehabilitation from 16 Mar 2009 to 06 Apr 2009 after hip joint total endoprosthesis (right). Laboratory tests revealed increased liver function test with increased GGT (365 U/l) and ALP (288 U/l) on 17 Mar 2009 and 02 Apr 2009 respectively. The reporter considered increased liver values were attributed to therapy with Xarelto. Control of liver values and alkaline phosphatase was recommended due to steatosis hepatitis. The outcome of increased liver function test was not reported. A drug interaction was also suspected by the reporter the outcome for which was not reported. The patient was hospitalised for rehabilitation from 16 Mar 2009 to 06 Apr 2009 after hip joint total endoprosthesis. The patient had received Xarelto as an anticoagulation treatment in context of the hip total endoprosthesis operation. Therapy with Xarelto was stopped due to lower abdominal pain and vaginal bleeding on 24 Mar 2009. For anticoagulation the patient was treated with Arixtra. The reporter considered the diagnosis of vaginal bleeding to be probably related to Xarelto and not related to Voltaren. The reporter disclosed that increased live values were attributed to therapy with Xarelto.

Bayer Vital GmbH, 27.05.2009 DE-BAYER-200917657GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 25-Mar-2009 from a health professional in Germany, via company's sales representative, referring to a 58 year-old Caucasian female.

This case includes follow up information from 26-Mar-2009.

Medical and drug history:

Patient's relevant medical history includes cox arthrosis, hip total endoprosthesis and sigma resection. Concomitant drugs have been given.

Suspect drug, timing and conditions surrounding the onset of the reaction:

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On 23-Mar-2009, patient experienced VAGINAL BLEEDING (lower abdominal pain, hemoglobin decreased) considered serious due to hospitalization. Xarelto was withdrawn and the patient improved.

The patient underwent hip total endoprosthesis surgery on 03-Mar-2009, and was treated with Xarelto (rivaroxaban). From 21-Mar-2009 onward, patient experienced unclear lower abdominal pain. Additionally, on 23-Mar-2009, vaginal bleeding set in. The patient's hemoglobin value decreased by 1.1 g/dl. The reporter considered the adverse events to be associated with the suspected drug. He gave the following reasons for his assessment: 1st a rectoscopy was performed without revealing any findings, 2nd lower abdominal sonography had proved suspicious (no further explanation provided), and 3rd anticoagulation.

According to the reporter, the adverse events had ended on 26-Mar-2009, on the other hand, the patient was reported having only partially recovered.

The patient received Xarelto for prophylaxis postoperative after hip TEP. During following rehabilitation procedure the patient developed vaginal

bleeding. After clarifying examinations like ultrasound etc. anticoagulation therapy was changed to Heparin.

Currently no further information available.

The original reporter's clinical assessment:

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related.

****FU 11-May-2009:**

The patient was hospitalized for rehabilitation from 16-Mar-2009 to 06-Apr-2009 after hip joint total endoprosthesis (right).

Additional concomitant conditions have been reported: spondylolisthesis, hypersensitivity to heparin, and intolerance of Xarelto.

Concomitant drugs have been added.

Information on suspect drugs has been updated.

The patient received:

- Xarelto (rivaroxaban) as anticoagulation treatment in context of the hip total endoprosthesis operation (right) from 03-Mar-2009 until 24-Mar-2009 (at a total daily dose of 10 MG, orally)
- Voltaren (diclofenac) from 03-Mar-2009 until 22-Mar-2009 (at a total daily dose of 150 MG, unknown route of administration)

Therapy with Xarelto was stopped due to lower abdominal pain and vaginal bleeding on 24-Mar-2009. For anticoagulation the patient was treated with Arixtra (fondaparinux sodium) at that point of time.

Ultrasound: no visible extraluminal fluid in abdominal area. Menorrhagia in the region of uterus/utero-vaginal could not be excluded for sure.

Control of liver values and alkaline phosphatase was recommended due to steatosis hepatis. Further advises included withdrawal of Arixtra and non-steroidal antirheumatics.

Sigmoidoscopy: no identifiable bleeding source, some evidence for remaining diverticula, bland anastomosis after sigma resection, interior haemorrhoids without signs of acute or previous bleeding on 24-Mar-2009.

Lab tests revealed increased GGT (on 17-Mar-2009: 365 U/l; on 02-Apr-2009: 288 U/l) and increased alkaline phosphatase. Also slightly increased SGOT and SGPT were observed. Event term: INCREASED LIVER FUNCTION TEST.

Patient's family physician was advised to check laboratory values and to perform gastroenterological controls for ruling out rectal bleeding from diverticulosis.

Lab tests have been added; see "Lab Data" section.

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related to Xarelto, and not related to Voltaren. The reporter disclosed that INCREASED LIVER VALUES were attributed to therapy with Xarelto.

NOVARTIS, 10.06.2009 DE-002147023-PHHY2009DE19970

Sender diagnosis: not reported

Narrative

Combined report of both the initial and follow up reports received from health care professional on 11 May 2009 and forwarded by License partner (Bayer Schering reference number: 200917657GPV) on 22 May 2009: The patient had historical condition of, hip total endoprosthesis and sigma resection. The concurrent condition of the patient included cox arthrosis, spondylolisthesis, sigma diverticulitis, hypersensitivity to heparin and haemorrhoids. She was on Voltaren (diclofenac sodium) for unknown indication from 03 Mar 2009 and Xarelto (rivaroxaban) for hip total endoprosthesis right and anticoagulation at a daily dose of 10mg from 03 Mar 2009 which was considered as co-suspect in this case. On 21 Mar 2009 the patient experienced lower abdominal pain and on 23 Mar 2009 vaginal bleeding started and hemoglobin (11.9 g/dL) decreased which were considered serious due to hospitalization. Xarelto was withdrawn on 24 Mar 2009 and the patient improved. The reporter considered the diagnosis vaginal bleeding to be probably related Xarelto, and not related to Voltaren. The patient was hospitalized for rehabilitation from 16 Mar 2009 to 06 Apr 2009 after hip joint total endoprosthesis (right). Laboratory tests revealed increased liver function test with increased GGT (365 U/l) and ALP (288 U/l) on 17 Mar 2009 and 02 Apr 2009 respectively. The reporter considered increased liver values were attributed to therapy with Xarelto. Control of liver values and alkaline phosphatase was recommended due to steatosis hepatitis. The outcome of increased liver function test was not reported. A drug interaction was also suspected by the reporter the outcome for which was not reported. The patient was hospitalized for rehabilitation from 16 Mar 2009 to 06 Apr 2009 after hip joint total endoprosthesis. The patient had received Xarelto as an anticoagulation treatment in context of the hip total endoprosthesis operation. Therapy with Xarelto was stopped due to lower abdominal pain and vaginal bleeding on 24 Mar 2009. For anticoagulation the patient was treated with Arixtra. The reporter considered the diagnosis of vaginal bleeding to be probably related to Xarelto and not related to Voltaren. The reporter disclosed that increased liver values were attributed to therapy with Xarelto.

Bayer Vital GmbH, 10.06.2009 DE-BAYER-200917657GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 25-Mar-2009 from a health professional in Germany, via company's sales representative, referring to a 58 year-old Caucasian female.

This case includes follow up information from 26-Mar-2009.

Medical and drug history:

Patient's relevant medical history includes cox arthrosis, hip total endoprosthesis and sigma resection. Concomitant drugs have been given.

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The progression of the event and its outcome in the patient:

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The patient underwent hip total endoprosthesis surgery on 03-Mar-2009, and was treated with Xarelto (rivaroxaban). From 21-Mar-2009 onward, patient experienced unclear lower abdominal pain. Additionally, on 23-Mar-2009, vaginal bleeding set in. The patient's hemoglobin value decreased by 1.1 g/dl. The reporter considered the adverse events to be associated with the suspected drug. He gave the following reasons for his assessment: 1st a rectoscopy was performed without revealing any findings, 2nd lower abdominal sonography had proved suspicious (no further explanation provided), and 3rd anticoagulation.

According to the reporter, the adverse events had ended on 26-Mar-2009, on the other hand, the patient was reported having only partially recovered.

The patient received Xarelto for prophylaxis postoperative after hip TEP. During following rehabilitation procedure the patient developed vaginal bleeding. After clarifying examinations like ultrasound etc. anticoagulation therapy was changed to Heparin.

Currently no further information available.

The original reporter's clinical assessment:

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related.

****FU 11-May-2009:**

The patient was hospitalized for rehabilitation from 16-Mar-2009 to 06-Apr-2009 after hip joint total endoprosthesis (right).

Additional concomitant conditions have been reported: spondylolisthesis, hypersensitivity to heparin, and intolerance of Xarelto.

Concomitant drugs have been added.

Information on suspect drugs has been updated.

The patient received:

- Xarelto (rivaroxaban) as anticoagulation treatment in context of the hip total endoprosthesis operation (right) from 03-Mar-2009 until 24-Mar-2009 (at a total daily dose of 10 MG, orally)
- Voltaren (diclofenac) from 03-Mar-2009 until 22-Mar-2009 (at a total daily dose of 150 MG, unknown route of administration)

Therapy with Xarelto was stopped due to lower abdominal pain and vaginal bleeding on 24-Mar-2009. For anticoagulation the patient was treated with Arixtra (fondaparinux sodium) at that point of time.

Ultrasound: no visible extraluminal fluid in abdominal area. Menorrhagia in the region of uterus/utero-vaginal could not be excluded for sure.

Control of liver values and alkaline phosphatase was recommended due to steatosis hepatis. Further advises included withdrawal of Arixtra and non-steroidal antirheumatics.

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Lab tests revealed increased GGT (on 17-Mar-2009: 365 U/l; on 02-Apr-2009: 288 U/l) and increased alkaline phosphatase. Also slightly increased SGOT and SGPT were observed. Event term: INCREASED LIVER FUNCTION TEST.

Patient's family physician was advised to check laboratory values and to perform gastroenterological controls for ruling out rectal bleeding from diverticulosis.

Lab tests have been added; see "Lab Data" section.

The reporter considered the diagnosis VAGINAL BLEEDING to be probably related to Xarelto, and not related to Voltaren. The reporter disclosed that INCREASED LIVER VALUES were attributed to therapy with Xarelto.

****FU 28-May-2009 (submitted by the attending orthopedic surgeon):**

Medical history: Patient had previous hospitalization in Feb/Mar-2006 due to diverticulitis and sigma resection.

Lab tests from Feb-2006 have been added; see "Lab Data" and "Other Relevant Tests" sections.

No cholelithiasis in medical history known.

*****BAYER Schering global comment (part one)*****

Initial (25-Mar-2009) and follow-up (26-Mar-2009) report: VAGINAL BLEEDING (lower abdominal pain, hemoglobin decreased), considered serious due to hospitalization, is listed in the Company Core Data Sheet (CCDS) for Xarelto (rivaroxaban). In the present spontaneous case the event was observed in a patient receiving Xarelto for three weeks after a total hip endoprosthesis. Due to a compatible temporal relationship and the increased risk of bleeding during anticoagulant therapy, a causal association can not be excluded. However, it is unclear whether sonography findings might indicate an alternative explanation for the event. In addition, concomitant use of diclofenac might have contributed to the bleeding. A warning statement regarding an increased risk of bleeding when rivaroxaban is co-administered with NSAIDs is provided in the CCDS for Xarelto. Further information has been requested (e.g. clarification of abdominal sonography results).

NOVARTIS, 12.06.2009 DE-002147023-PHHY2009DE19970

Sender diagnosis: not reported

Narrative

Combined report of both the initial and follow up reports received from health care professional on 11 May 2009 and forwarded by License partner (Bayer Schering reference number: 200917657GPV) on 22 May 2009: The patient had historical condition of, hip total endoprosthesis and sigma resection. The concurrent condition of the patient included cox arthrosis, spondylolisthesis, sigma diverticulitis, hypersensitivity to heparin and haemorrhoids. She was on Voltaren (diclofenac sodium) for unknown indication from 03 Mar 2009 and Xarelto (rivaroxaban) for hip total endoprosthesis right and anticoagulation at a daily dose of 10mg from 03 Mar 2009 which was considered as co-suspect in this case. On 21 Mar 2009 the patient experienced lower abdominal pain and on 23 Mar 2009 vaginal bleeding started and hemoglobin (11.9 g/dL) decreased which were considered serious due to hospitalization. Xarelto was withdrawn on 24 Mar 2009 and the patient improved. The reporter considered the diagnosis vaginal bleeding to be probably related Xarelto, and not related to Voltaren. The patient was hospitalized for rehabilitation from 16 Mar 2009 to 06 Apr 2009 after hip joint total endoprosthesis (right). Laboratory tests revealed increased liver function test with increased GGT (365 U/l) and ALP (288 U/l) on 17 Mar 2009 and 02 Apr 2009 respectively. The reporter considered increased liver values were attributed to therapy with Xarelto. Control of liver values and alkaline phosphatase was recommended due to steatosis hepatitis. The outcome of increased liver function test was not reported. A drug interaction was also suspected by the reporter the outcome for which was not reported. The patient was hospitalised for rehabilitation from 16 Mar 2009 to 06 Apr 2009 after hip joint total endoprosthesis. The patient had received Xarelto as an anticoagulation treatment in context of the hip total endoprosthesis operation. Therapy with Xarelto was stopped due to lower abdominal pain and vaginal bleeding on 24 Mar 2009. For anticoagulation the patient was treated with Arixtra. The reporter considered the diagnosis of vaginal bleeding to be probably related to Xarelto and not related to Voltaren. The reporter disclosed that increased live values were attributed to therapy with Xarelto.

Follow-up information received on 28 May 2009 and forwarded by License partner on 10 Jun 2009: The patient had historical condition of increased monocyte (50%), eosinophil (50%) and basophil (50%) as of 10 Feb 2006.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09052398

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19290730	Age: 79 Year	weight (kg): 98	BMI_int: 29.59	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased	20090407				recovering/resolving
2 ALP increased					recovering/resolving
3 Hepatic enzymes increased	20081001	20090409	6 Month		recovering/resolving
4 Diarrhea	20090402	20090408	7 Day		recovered/resolved
5 Hepatic enzymes increased	20081001				recovering/resolving
6 Gamma GT increased	20081001				recovering/resolving
7 Urinary infection	20090318	20090328	11 Day		recovered/resolved
8 Appendicitis					Unknown
9 AST increased	20090407				recovering/resolving
10 Cardiac failure	200809				Unknown
11 Peritonitis					Unknown
12 Peritoneal abscess					Unknown
13 Paralytic ileus					Unknown
14 Gamma glutamyl transpeptidase increased	20090407				recovering/resolving
15 Antibiotic-associated diarrhea	20090402	20090408	7 Day		recovered/resolved
16 Neuropathy					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXI CLAVULAN AL 875/125 /ALIUD PHAR amoxicillin trihydrate; clavulanate potassium	Oral	2 Df dosage form/Day 245	20090318-20090328 11 Day	Urinary infection
2 INSPRA eplerenone	Oral	25 Mg milligram(s)/Day 82	200809- -	Cardiac failure
3 INSUMAN COMB 25/75 insulin human	Subcutaneous	2* /Day 120	200709- -	Type 2 diabetes mellitus
4 KALINOR potassium chloride	Oral	2 Df dosage form/Day 59	200809- -	Potassium replacement
5 MONO-EMBOLEX heparin-fraction, sodium salt	Subcutaneous	3000 Iu international unit(s) 120	200809- -	Bedridden
6 OMEPRAZOLE omeprazole	Oral	40 Mg milligram(s)/Day 245	200809- -	Gastric ulcer prophylaxis
7 SIMVASTATIN simvastatin	Oral	40 Mg milligram(s)/Day 82	200709-20090407 19 Month	Hyperlipidaemia
8 TORASEMIDE HEXAL 200 mg torasemide	Oral	100 Mg milligram(s)/Day 245	200809- -	Renal impairment NOS
9 VOLTAREN DISPERS diclofenac	Oral	50 Mg milligram(s)/ 245	2007-200904 2 Year	Coxarthrosis

Drugs (concomitant)

10 STILNOX zolpidem	Oral	10 Mg milligram(s)/Day 82	200901- -	Disorder sleep
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Appendicitis	20080910	20090115	No	Z.n. Perforation mit Peritonealabzeß, Peritonitis und Z.n. paralytischen Ileus
Cardiac failure	200809		Yes	
Disorder sleep			Yes	Beginn unbekannt
Hesitancy			Yes	Beginn unbekannt; unklarer Genese mit Blasendauerkatheter
Hypertension arterial			Yes	Beginn unbekannt; normotensiv derzeit
Prostatic hyperplasia			Yes	Beginn unbekannt
Pulmonary embolus	2004	2004	No	Falithromtherapie bis 01-2009
Renal insufficiency	200709		Yes	chronisch, Stadium 2
Thrombophlebitis	1972		No	und weitere 2x unklaren Zeitpunktes; Ende unbekannt
Urinary infection	20090318	20090328	No	
Coxarthrosis			Yes	Beginn unbekannt; bds.
Gonarthrosis			Yes	Beginn unbekannt; bds.
Hyperlipidaemia			Yes	Beginn unbekannt
Peripheral arterial occlusive disease			Yes	Beginn unbekannt
Type 2 diabetes mellitus			Yes	Beginn unbekannt; insulinpflichtig mit Neuropathie

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20090407	164	µmol/l	
Creatinine	20090409	143.8	µmol/l	
Urea	20090407	11.7	mmol/l	
Urea	20090409	7.5	mmol/l	
Aspartate aminotransferase	20090407	1.62	µmol/s.l	
Aspartate aminotransferase	20090409	1.17	µmol/s.l	
Alanine aminotransferase	20090407	2.41	µmol/s.l	
Alanine aminotransferase	20090409	1.94	µmol/s.l	
Alkaline phosphatase	20090407	8.05	µmol/l	
Alkaline phosphatase	20090409	7.28	µmol/l	
Gamma GT	20090407	19.97	µmol/l.s	
Gamma GT	20090409	16.69	µmol/l.s	
Haemoglobin	20090407	7.57	mmol/l	
Haematocrit	20090407	0.38	-	
Mean corpuscular haemoglobin	20090407	90	fl	
Total white blood cell count	20090407	9.1	Gpt/l	
Thrombocyte count	20090407	242	Gpt/l	
Quick's test	20090407	65	Prozent	
INR	20090407	1.26	-	
Sodium	20090407	138	mmol/l	
Potassium	20090407	3.88	mmol/l	

Calcium	20090407	2.41	mmol/l
C-reactive protein	20090409	29.1	mg/l
Haematocrit	20090407	0.38	NA
INR	20090407	1.26	NA
GGTP	20090407	19.97 umol/s.l	umol/s.l
GGTP	20090409	16.69 umol/s.l	umol/s.l

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 24.04.2009 DE-PVZIBE-WEIMAR-10225

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Diarrhoe

KURZZUSAMMENFASSUNG:

Antibiotikaassoziierte Diarrhoe nach Amoxicillin und Clavulansäure-Therapie und erhöhte Leberwerte unter Simvastatin

BESCHREIBUNG:

Die stationäre Aufnahme erfolgte am 07.04.09 über den Hausarzt wegen Diarrhoe seit dem 02.04.09 und erhöhten Leberwerten seit 10/2008. Der Patient nahm auf Grund eines Harnwegsinfektes von 18. - 28.03.09 Amoxicillin und Clavulansäure ein, außerdem seit mindestens 09/2007 Simvastatin (genauer Behandlungsbeginn nicht mehr eruierbar).

THERAPIE:

- ab 07.04.09: Simvastatin abgesetzt
- vom 07.04 - 09.04.09: Sterofundin 1000 ml tgl. als Infusion

VERLAUF:

- 07./08.04.09: Noro-, Adeno- und Rotavirusnachweis und Clostridium difficile Toxin A/B negativ.
- ab 09.04.09 kein Durchfall mehr
- die erhöhten Leberwerte zeigten sich am 09.04.09 deutlich regredient,
- cytos. Leber-AG Typ 1 vom 08.04.09 negativ
- Infektionsserologie (anti-HAV-IgM, HBsAg, Anti-HBc, Anti-HBc-IgM, Anti-HCV) negativ

ERLÄUTERUNGEN ZUR UAW:

Die stationäre Aufnahme erfolgte sowohl wegen der Diarrhoe, die nach Antibiotikaeinnahme auftrat und wegen erhöhter Leberwerte, die durch Simvastatin bedingt sein konnte.

ANMERKUNGEN ZU FOLGENDER UAW:

Leberenzyme erhöht

KURZZUSAMMENFASSUNG:

Antibiotikaassoziierte Diarrhoe nach Amoxicillin und Clavulansäure-Therapie und erhöhte Leberwerte unter Simvastatin

BESCHREIBUNG:

Die stationäre Aufnahme erfolgte am 07.04.09 über den Hausarzt wegen Diarrhoe seit dem 02.04.09 und erhöhten Leberwerten seit 10/2008. Der Patient nahm auf Grund eines Harnwegsinfektes von 18. - 28.03.09 Amoxicillin und Clavulansäure ein, außerdem seit mindestens 09/2007 Simvastatin (genauer Behandlungsbeginn nicht mehr eruierbar).

THERAPIE:

- ab 07.04.09: Simvastatin abgesetzt
- vom 07.04 - 09.04.09: Sterofundin 1000 ml tgl. als Infusion

VERLAUF:

- 07./08.04.09: Noro-, Adeno- und Rotavirusnachweis und Clostridium difficile Toxin A/B negativ.
- ab 09.04.09 kein Durchfall mehr
- die erhöhten Leberwerte zeigten sich am 09.04.09 deutlich regredient,
- cytos. Leber-AG Typ 1 vom 08.04.09 negativ
- Infektionsserologie (anti-HAV-IgM, HBsAg, Anti-HBc, Anti-HBc-IgM, Anti-HCV) negativ

ERLÄUTERUNGEN ZUR UAW:

Die stationäre Aufnahme erfolgte sowohl wegen der Diarrhoe, die nach Antibiotikaeinnahme auftrat und wegen erhöhter Leberwerte, die durch Simvastatin bedingt sein konnte.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: AMOXI CLAVULAN AL 875/125 /ALIUD PHARMA

BETREFFENDE UAW: Diarrhoe

ERLÄUTERUNG: Die Chronologie spricht stark für einen Zusammenhang mit Auftreten der Diarrhoe nach Therapie und relativ schnellem Sistieren. Gängige Diarrhoeerreger waren nicht nachweisbar. Salmonellen, Shigellen, Campylobacter Species und Yersinien nicht getestet, daher S2.

MEDIKAMENT: SIMVASTATIN

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: Chronologisch ist ein Zusammenhang möglich mit positiver Dechallenge, daher C2. Ein akute bzw. eine autoimmune Hepatitis könnte ausgeschlossen werden. Da eine alkoholbedingte Leberschädigung nicht völlig ausschließbar ist, S2.

MEDIKAMENT: VOLTAREN DISPERS

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: lt. Fachinformation kann es häufig zu Transaminasenerhöhung kommen und gelegentlich zu Leberschäden, akute Hepatitis mit und ohne Ikterus bei Langzeitanwendung. Eine Langzeitanwendung liegt vor, allerdings mit geringer Dosis (nur 50-100 mg/Wo.), daher ist ein Zusammenhang möglich. Während des stationären Aufenthaltes bekam der Patient keine weitere Gabe, somit kann dies auch als positive Dechallenge gewertet werden. Da eine ethyltoxische Leberschädigung (lt. Patienten fast täglich 1 Glas Bier) nicht völlig auszuschließen ist, S2.

<dbtext>To_Char_NARRATIVE_2</dbtext>

009507513, 30.04.2009 DE-MERCK-0904DEU00109

Sender diagnosis: not reported

Narrative

Information has been received from a physician and the German agency concerning a 79-year-old bedridden male patient with a daily use of alcohol (one glass of beer daily), coxarthrosis, cardiac failure, type 2 diabetes mellitus, potassium replacement, gastric ulcer prophylaxis, sleep disorder, renal impairment, hypertension arterial, hesitancy, gonarthrosis, peripheral arterial occlusive disease, prostatic hyperplasia and a history of pulmonary embolism, appendicitis and thrombophlebitis who, in September 2007, was placed on therapy with simvastatin (manufacturer unknown), 40 mg tablet, 40 mg, once a day for the treatment of hyperlipidaemia (exact duration not reported). Concomitant suspect therapy included amoxicillin/clavulanate potassium (AMOXI CLAVULAN), 875/125 mg tablet, 875/125 mg twice a day for the treatment of urinary tract infection (not reported as an adverse event by the agency/physician; treatment from 18-MAR-2008 until 28-MAR-2009) and diclofenac Na (VOLTAREN DISPERS), 50 mg tablet, 50 mg as needed (about one to two tablets a week) for the treatment of coxarthrosis (therapy initiated in 2007; exact duration not reported).

Other concomitant therapy included eplerenone (INSPRA), insulin, biphasic isophane [injection] (INSUMAN COMB 25), KHCO₃/K citrate (KALINOR), certoparin sodium (MONO EMBOLIX), omeprazole, zolpidem tartrate (STILNOX) and torsemide.

In October 2008, the patient experienced increased liver enzymes (values not reported). On 02-APR-2009, the patient developed diarrhoea. On 07-APR-2009, he was hospitalized due to diarrhoea and increased liver enzymes. That same day, therapy with simvastatin was discontinued (last intake was on 06-APR-2009). On 07-APR-2009, laboratory analysis revealed AST 1.62 mcml/s.l; ALT 2.41 mcml/s.l; AP 8.05 mcmol/l; GGT 19.97 mcmol/l.s. No evidence of Norwalk-like viruses, adenoviruses or rotaviruses were found; Clostridium difficile A/B negative. On 08-APR-2009, serum hepatitis B core immunoglobulin M antibody test, serum hepatitis C antibody test, serum hepatitis B surface antigen test, serum hepatitis B core immunoglobulin G antibody test and serum hepatitis A immunoglobulin M antibody test were negative. From 07-APR-2009 until 09-APR-2009, he received treatment with dextrose (+) electrolytes (unspecified) (STEROFUNDIN) infusions, 1000 ml daily. On 09-APR-2009, the patient recovered from diarrhoea. On 09-APR-2009, laboratory analysis revealed AST 1.17 mcml/s.l; ALT 1.94 mcml/s.l; AP 7.28 mcmol/l; GGT 16.69 mcmol/l.s, i.e. the patient was recovering from increased liver enzymes.

The reporting physician felt that increased liver enzymes was possibly related to therapy with simvastatin and/or therapy with diclofenac Na (VOLTAREN DISPERS). He felt that the diarrhoea was probably not related to therapy with simvastatin but was likely related to therapy with amoxicillin/clavulanate potassium (AMOXI CLAVULAN)

Additional information has been requested.

NOVARTIS, 05.05.2009 DE-002147023-PHHY2009DE15920

Sender diagnosis: not reported

Narrative

Initial physician report was received from a regional pharmacovigilance center (intensive monitoring project) via HA (DE-BFARM-09052398) on 24 Apr 2009: The patient with a relevant history of type 2 diabetes mellitus, pulmonary embolus (2004), hyperlipidemia, type 2 diabetes mellitus, thrombophlebitis (1972), renal insufficiency (Sep 2007), hypertension arterial, hesitancy, gonarthrosis, peripheral arterial occlusive, prostatic hyperplasia, and sleep disorder was on treatment with Voltaren dispers (diclofenac) since an unknown date in 2007 for coxarthrosis. Co-suspect drugs included Amoxi clavulan al (amoxicillin, clavulanate potassium) for urinary infection from 18 Mar 2009 to 28 Mar 2009, and simvastatin since Sep 2007 for hyperlipidemia. The patient was on multiple concomitant medications. The patient experienced antibiotic associated diarrhea after amoxicillin and clavulanic acid therapy and hepatic enzymes increased on simvastatin. The patient was admitted on 07 Apr 2009 by his primary care physician with diarrhea since 02 Apr 2009 and increased liver values since Oct 2008. Laboratory examination on 07 Apr 2009 revealed increased Alanine transferase (2.41 umol/s.l, normal less than 0.68), Aspartate aminotransferase (1.62 umol/s.l, normal less than 0.62), Alkaline phosphatase (8.05 umol/l, normal 0.67 to 2.5), Gamma glutamyl transferase (19.97 umol/s.l, normal less than 0.82). Simvastatin was discontinued on 07 Apr 2009. From 07 Apr 2009 to 09 Apr 2009 Sterofundin (electrolytes) 1000 ml daily infusion was used as a treatment drug. It was also reported that on 07 Apr 2009 and 08 Apr 2009 norovirus, adenovirus, rotavirus and Clostridium difficile toxin were A/B negative. From 09 Apr 2009 onwards, no more diarrhea was

observed. The increased liver values were observed to be markedly regressive on 09 Apr 2009 (ALT 1.94 umol/s.l, AST 1.17 umol/s.l, ALP 7.28 umol/l, GGTP 16.69 umol/s.l) and cytosolic liver antigen type 1 on 08 Apr 2009 was also negative. Infectious serology (anti-HAV-IgM, HBsAg, anti-HBc, anti-HBc-IgM, anti-HCV) was negative. The event outcome was reported as recovery for diarrhea and condition improving for hepatic enzymes increased. The causality was assessed as dubious for diarrhea and possible for hepatic enzymes increased. The patient also suffered cardiac failure since Sep 2008, appendicitis on 10 Sep 2008 and urinary infection from 18 Mar 2009 to 28 Mar 2009.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

SANOFI-SYNTHELABO, 07.05.2009 DE-SANOFI-SYNTHELABO-A04200900562

Sender diagnosis: not reported

Narrative

Reaction as reported: elevated ASAT(2.6*ULN),ALAT(3.5*ULN),PAL(11.8*ULN)
LLT: HEPATIC ENZYMES INCREASED / 10019662
PT: HEPATIC ENZYME INCREASED / 10060795

Reaction as reported: elevated Gamma GT(24.3*ULN)
LLT: GAMMA GT INCREASED / 10017670
PT: GAMMA-GLUTAMYLTRANSFERASE INCREASED / 10017693

Reaction as reported: diarrhea
LLT: DIARRHEA / 10012727
PT: DIARRHOEA / 10012735

Narrative:

Initial report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-WEIMAR-10225) and was extracted from the website of the German health authority (authority number: DE-BFARM- 09052398). The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

The case involves a 79-year-old male patient with history of thrombophlebitis in 1972, pulmonary embolism in 2004, perforated appendicitis associated with peritoneal abscess, peritonitis, and paralytic ileus on 10-Sep-2008. At time of event onset, the patient was suffering from hyperlipidaemia, insulin-dependent type 2 diabetes mellitus with neuropathy, chronic renal insufficiency since Sep-2007, arterial hypertension, cardiac failure since Sep-2008, sleep disorder, hesitancy leading to permanent bladder catheterization, bilateral gon- and coxarthrosis, peripheral arterial occlusive disease, and prostatic hyperplasia.

The following drugs were reported to have been taken concomitantly:

since Sep-2007: isophane insulin+insulin human

since Sep-2008 or earlier: eplerenone, potassium chloride, heparin-fraction, omeprazole

since Sep-2008: torademedide

since Jan-2009: zolpidem

from 18-Mar-2009 to 28-Mar-2009: amoxicillin trihydrate+clavulanate potassium

The patient initiated treatment with simvastatin (no trade name provided)40 mg/day for hyperlipidaemia in Sep-2007 or earlier. On unspecified date in 2007, he started to take 1-2 of 50-mg diclofenac sodium tablets weekly for coxarthrosis as needed. On 07-Apr-2009 the patient was admitted to a hospital due to increased hepatic enzymes (aspartate aminotransferase 1.62 umol/l*s, alanine aminotransferase 2.41 umol/l*s, alkaline phosphatase 8.05 umol/l, gamma-GT 19.97 umol/l*s) that had existed since Oct-2008. Additionally, the patient had suffered from diarrhea since 02-Apr-2009. On 07-Apr-2009 simvastatin was discontinued. From 07-Apr-2009 to 09-Apr-2009 electrolyte infusions 1000 ml daily were administered. Diclofenac sodium was not given during the hospitalization. Treatments with isophane insulin+insulin human, eplerenone, potassium chloride, heparin-fraction, omeprazole, torademedide, and zolpidem were continued. There were negative results for anti-HAV-IgM, HBsAg, Anti-HBc, Anti-HBc-IgM, Anti-HCV, noro-, adeno- and rota-virus, Clostridium difficile toxin A/B and liver cytosol antigen type 1. Laboratory test on 09-Apr-2009 showed obvious regressive change in increased hepatic enzymes (outcome: "recovering/resolving after 09-Apr-2009") and diarrhea had resolved.

The PV centre reported "increased hepatic enzyme" as adverse reaction with start date of "01-Oct-2008". Simvastatin and diclofenac were suspected for the reaction.

Causality determined by PV centre concerning "increased hepatic enzymes":

"possible" for simvastatin and diclofenac

Explanation of the assessment:

Simvastatin: the causal relationship with simvastatin is possible due to positive dechallenge. An acute or autoimmune hepatitis and an alcohol-induced liver damage couldn't be ruled out.

Diclofenac: frequent occurrence of increased transaminases and occasional incidences of liver damage and acute hepatitis with and without jaundice under long-term administration were described in the product information. The patient had received diclofenac for a long term, but in the low dose (only 50-100 mg/week), therefore the causal relationship is possible. During the hospitalization diclofenac was not given and hepatic enzyme levels decreased, that could be assessed as positive dechallenge. An ethyl-toxic liver damage (patient reported to have taken 1 glass of beer near-daily) could not be ruled out.

All other diagnoses and symptoms were only mentioned by the PV centre in its unformatted text.

HELIOS Klinikum Wuppertal, 24.11.2009 DE-PVZIBE-WEIMAR-10225

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Diarrhoe

KURZZUSAMMENFASSUNG:

Antibiotikaassoziierte Diarrhoe nach Amoxicillin und Clavulansäure-Therapie

BESCHREIBUNG:

Die stationäre Aufnahme erfolgte am 07.04.09 über den Hausarzt wegen Diarrhoe seit dem 02.04.09 und erhöhten Leberwerten seit 10/2008. Der Patient nahm auf Grund eines Harnwegsinfektes von 18. - 28.03.09 Amoxicillin und Clavulansäure ein, außerdem seit mindestens 09/2007

Simvastatin (genauer Behandlungsbeginn nicht mehr eruierbar).

THERAPIE: - vom 07.04 - 09.04.09: Sterofundin 1000 ml tgl. als Infusion

VERLAUF:

- 07./08.04.09: Noro-, Adeno- und Rotavirusnachweis und Clostridium difficile Toxin A/B negativ.

- ab 09.04.09 kein Durchfall mehr

ERLÄUTERUNGEN ZUR UAW:

Die stationäre Aufnahme erfolgte sowohl wegen der Diarrhoe, die nach Antibiotikaeinnahme auftrat und wegen erhöhter Leberwerte, die durch Simvastatin bedingt sein konnte.

ANMERKUNGEN ZU FOLGENDER UAW:

Leberenzyme erhöht

KURZZUSAMMENFASSUNG:

erhöhte Leberenzyme unter Simvastatin und Diclofenac

BESCHREIBUNG:

Die stationäre Aufnahme erfolgte am 07.04.09 über den Hausarzt wegen Diarrhoe seit dem 02.04.09 und erhöhten Leberwerten seit 10/2008. Der Patient nahm auf Grund eines Harnwegsinfektes von 18. - 28.03.09 Amoxicillin und Clavulansäure ein, außerdem seit mindestens 09/2007

Simvastatin (genauer Behandlungsbeginn nicht mehr eruierbar).

THERAPIE:

am 07.04.09 Simvastatin abgesetzt - stationär erfolgte keine weitere Voltarengabe

VERLAUF:

- die erhöhten Leberwerte zeigten sich am 09.04.09 deutlich regredient,

- cytos. Leber-AG Typ 1 vom 08.04.09 negativ

- Infektionsserologie (anti-HAV-IgM, HBsAg, Anti-HBc, Anti-HBc-IgM, Anti-HCV) negativ

ERLÄUTERUNGEN ZUR UAW:

Die stationäre Aufnahme erfolgte sowohl wegen der Diarrhoe, die nach Antibiotikaeinnahme auftrat und wegen erhöhter Leberwerte, die durch Simvastatin bedingt sein konnte.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: AMOXI CLAVULAN AL 875/125 /ALIUD PHARMA

BETREFFENDE UAW: Diarrhoe

ERLÄUTERUNG: Die Chronologie spricht stark für einen Zusammenhang mit Auftreten der Diarrhoe nach Therapie und relativ schnellem Sistieren. Gängige Diarrhoeerreger waren nicht nachweisbar. Salmonellen, Shigellen, Campylobacter Species und Yersinien nicht getestet, daher S2.

MEDIKAMENT: SIMVASTATIN

BETREFFENDE UAW: Leberenzyme erhöht

ERLÄUTERUNG: Chronologisch ist ein Zusammenhang möglich mit positiver Dechallenge, daher C2. Eine autoimmune Hepatitis konnte ausgeschlossen werden, eine virale Genese erscheint unwahrscheinlich(nicht alles getestet). Da eine alkoholbedingte Leberschädigung nicht völlig ausschließbar ist, S2.

MEDIKAMENT: VOLTAREN DISPERS

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: lt. Fachinformation kann es häufig zu Transaminasenerhöhung kommen und gelegentlich zu Leberschäden, akute Hepatitis mit und ohne Ikterus bei Langzeitanwendung. Eine Langzeitanwendung liegt vor, allerdings mit geringer Dosis (nur 50-100 mg/Wo.), daher ist ein Zusammenhang möglich. Während des stationären Aufenthaltes bekam der Patient keine weitere Gabe, somit kann dies auch als positive Dechallenge gewertet werden. Eine autoimmune Hepatitis konnte ausgeschlossen werden, eine virale Genese erscheint unwahrscheinlich (nicht alles getestet). Da eine alkoholbedingte Leberschädigung nicht völlig ausschließbar ist, S2.

MEDIKAMENT: STILNOX

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: lt. Fachinformation sind erhöhte Leberenzymwerte beschrieben ohne Angaben der Häufigkeit, da die Werte bereits erhöht waren vor Einnahmebeginn, wird das Medikament nicht verdächtigt.

MEDIKAMENT: MONO-EMBOLEX

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: In der Fachinformation ist ein leichter Anstieg der Leberenzymwerte beschrieben. C1, da das Medikament nicht abgesetzt wurde. Eine autoimmune Hepatitis konnte ausgeschlossen werden, eine virale Genese erscheint unwahrscheinlich. <dbtext>To_Char_NARRATIVE_2</dbtext>

009507513, 30.11.2009 DE-MERCK-0904DEU00109

Sender diagnosis: not reported

Narrative

Information has been received from a physician via the German agency and a regional pharmacovigilance center concerning a 79-year-old bedridden male patient with a daily use of alcohol (one glass of beer daily), coxarthrosis, cardiac failure, type 2 diabetes mellitus, potassium replacement, gastric ulcer prophylaxis, sleep disorder, renal impairment, hypertension arterial, hesitancy, gonarthrosis, peripheral arterial occlusive disease, prostatic hyperplasia and a history of pulmonary embolism, appendicitis and thrombophlebitis who, in September 2007, was placed on therapy with simvastatin (manufacturer unknown), 40 mg tablet, 40 mg, once a day for the treatment of hyperlipidaemia (exact duration not reported). Concomitant suspect therapy included amoxicillin/clavulanate potassium (AMOXI CLAVULAN), 875/125 mg tablet, 875/125 mg twice a day for the treatment of urinary tract infection (not reported as an adverse event by the agency/physician; treatment from 18-MAR-2008 until 28-MAR-2009), diclofenac Na (VOLTAREN DISPERS), 50 mg tablet, 50 mg as needed (about one to two tablets a week) for the treatment of coxarthrosis (therapy initiated in 2007; exact duration not reported), eplerenone (INSPIRA), 25 mg tablet, 25 mg once a day taken from September 2008 for the treatment of cardiac failure (exact dose and duration not reported), insulin, biphasic isophane [injection] (INSUMAN COMB 25/75), 28/0/8 IU administered from September 2007 daily for the treatment of type II diabetes mellitus, KHCO₃/K citrate (KALINOR), effervescent tablet, 1 tablet taken from September 2008 twice a day as potassium replacement (exact dose and duration not reported), certoparin sodium (MONO EMBOLEX), injection form, 3000 IU daily administered subcutaneously for bedriddenness (duration not reported), omeprazole, 40 mg tablet, 40 mg once a day taken from September 2008 as gastric ulcer prophylaxis (exact duration not reported) and torsemide (TORASEMIDE HEXAL), 100 mg tablet, 100 mg taken daily from September 2008 for the treatment of renal impairment (exact duration not reported).

Other concomitant therapy included zolpidem tartrate (STILNOX).

In October 2008, the patient experienced increased liver enzymes (values not reported). On 02-APR-2009, the patient developed diarrhoea. On 07-APR-2009, he was hospitalized due to diarrhoea and increased liver enzymes. That same day, therapy with simvastatin was discontinued (last intake was on 06-APR-2009). On 07-APR-2009, laboratory analysis revealed AST 1.62 mcml/s.l; ALT 2.41 mcml/s.l; AP 8.05 mcml/l; GGT 19.97 mcml/l.s. No evidence of Norwalk-like viruses, adenoviruses or rotaviruses were found; Clostridium difficile A/B negative. On 08-APR-2009, serum hepatitis B core immunoglobulin M antibody test, serum hepatitis C antibody test, serum hepatitis B surface antigen test, serum hepatitis B core immunoglobulin G antibody test and serum hepatitis A immunoglobulin M antibody test were negative. From 07-APR-2009 until 09-APR-2009, he received treatment with dextrose (+) electrolytes (unspecified) (STEROFUNDIN) infusions, 1000 ml daily. On 09-APR-2009, the patient recovered from diarrhoea. On 09-APR-2009, laboratory analysis revealed AST 1.17 mcml/s.l; ALT 1.94 mcml/s.l; AP 7.28 mcml/l; GGT 16.69 mcml/l.s, i.e. the patient was recovering from increased liver enzymes.

The reporting physician/agency/regional pharmacovigilance center felt that increased liver enzymes was possibly related to therapy with simvastatin and/or therapy with diclofenac Na (VOLTAREN DISPERS). The physician considered a viral origin as unlikely. Drug relationship regarding increased liver enzymes was rated as "dubious" for eplerenone (INSPIRA), insulin, biphasic isophane [injection] (INSUMAN COMB 25/75), KHCO₃/K citrate (KALINOR), certoparin sodium (MONO EMBOLEX), omeprazole and torsemide (TORASEMIDE HEXAL).

The reporting physician/agency/regional pharmacovigilance center felt that diarrhoea was likely related to therapy with amoxicillin/clavulanate potassium (AMOXI CLAVULAN) and possibly related to diclofenac Na (VOLTAREN DISPERS).

Drug relationship regarding diarrhoea was rated as "dubious" for simvastatin, eplerenone (INSPIRA), insulin, biphasic isophane [injection] (INSUMAN COMB 25/75), KHCO₃/K citrate (KALINOR), certoparin sodium (MONO EMBOLEX), omeprazole and torsemide (TORASEMIDE HEXAL).

Additional information has been requested.

Bayer Vital GmbH, 04.12.2009 DE-BAYER-200940731GPV

Sender diagnosis: not reported

Narrative

Source of report and patient demography:

This solicited case report was received on 25-Nov-2009 from the regulatory authority of Germany by a pharmacovigilance center, referring to a 79 year-old male.

Medical and drug history:

Patient had concomitant conditions of arterial hypertension, hesitancy, gonarthrosis, peripheral arterial occlusive and prostatic hyperplasia. Patient's relevant medical history includes pulmonary embolus, appendicitis and thrombophlebitis. It is unknown whether any concomitant drugs have been given.

Suspect drugs, timing and conditions surrounding the onset of the reactions:

Patient received Omeprazol for gastric ulcer prophylaxis. Drug was administered since Sep-2008; Amoxi Clavulan for urinary tract infection from 18-Mar-2009 to 28-Mar-2009, Simvastatin for hyperlipidaemia since Sep-2007 at a total daily dose of 40 mg orally; Voltaren (diclofenac) for coxarthrosis since 2007; Inspra (eplerenone) for cardiac failure since Sep-2008 at a total daily dose of 25 mg orally; Insuman Comb for diabetes mellitus type 2 since Sep-2009; Kalinor for potassium replacement since Sep-2008; Mono-Embolex (certoparin sodium) for bedridden also since Sep-2008 at a total daily dose of 3000 IU subcutaneously; Torasemide Hexal for renal impairment nos at a total daily dose of 100 mg orally and Stilnox (zolpidem) for disorder sleep at a total daily dose of 10 mg orally.

The progression of the events and their outcome in the patient:

On 01-Oct-2008, the patient experienced INCREASED HEPATIC ENZYMES. The condition improved on 08-Apr-2009. On 02-Apr-2009, the patient experienced DIARRHEA. The condition recovered on 08-Apr-2009. Both events were considered serious as it led to hospitalization. The dose of Omeprazole was not changed, Amoxi Clavulan and Simvastatin were withdrawn.

The original reporter's clinical assessment:

The reporter considered the DIARRHEA to be probably related to Amoxi Clavulan and INCREASED HEPATIC ENZYMES to be probably related to Simvastatin.

NOVARTIS, 09.12.2009 DE-002147023-PHHY2009DE15920

Sender diagnosis: not reported

Narrative

Initial physician report was received from a regional pharmacovigilance center (intensive monitoring project) via HA (DE-BFARM-09052398) on 24 Apr 2009: The patient with a relevant history of type 2 diabetes mellitus, pulmonary embolus (2004), hyperlipidemia, type 2 diabetes mellitus, thrombophlebitis (1972), renal insufficiency (Sep 2007), hypertension arterial, hesitancy, gonarthrosis, peripheral arterial occlusive, prostatic hyperplasia, and sleep disorder was on treatment with Voltaren dispers (diclofenac) since an unknown date in 2007 for coxarthrosis. Co-suspect drugs included Amoxi clavulan al (amoxicillin, clavulanate potassium) for urinary infection from 18 Mar 2009 to 28 Mar 2009, and simvastatin since Sep 2007 for hyperlipidemia. The patient was on multiple concomitant medications. The patient experienced antibiotic associated diarrhea after amoxicillin and clavulanic acid therapy and hepatic enzymes increased on simvastatin. The patient was admitted on 07 Apr 2009 by his primary care physician with diarrhea since 02 Apr 2009 and increased liver values since Oct 2008. Laboratory examination on 07 Apr 2009 revealed increased Alanine transferase (2.41umol/s.l, normal less than 0.68), Aspartate aminotransferase (1.62 umol/s.l, normal less than 0.62), Alkaline phosphatase (8.05 umol/l, normal 0.67 to 2.5), Gamma glutamyl transferase (19.97 umol/s.l, normal less than 0.82). Simvastatin was discontinued on 07 Apr 2009. From 07 Apr 2009 to 09 Apr 2009 Sterofundin (electrolytes) 1000 ml daily infusion was used as a treatment drug. It was also reported that on 07 Apr 2009 and 08 Apr 2009 norovirus, adenovirus, rotavirus and Clostridium difficile toxin were A/B negative. From 09 Apr 2009 onwards, no more diarrhea was observed. The increased liver values were observed to be markedly regressive on 09 Apr 2009 (ALT 1.94 umol/s.l, AST 1.17 umol/s.l, ALP 7.28 umol/l, GGTP 16.69 umol/s.l) and cytosolic liver antigen type 1 on 08 Apr 2009 was also negative. Infectious serology (anti-HAV-IgM, HBsAg, anti-HBc, anti-HBc-IgM, anti-HCV) was negative. The event outcome was reported as recovery for diarrhea and condition improving for hepatic enzymes increased. The causality was assessed as dubious for diarrhea and possible for hepatic enzymes increased. The patient also suffered cardiac failure since Sep 2008, appendicitis on 10 Sep 2008 and urinary infection from 18 Mar 2009 to 28 Mar 2009.

Follow-up of this case was received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority on 25 Nov 2009: The patient was also on therapy with co-suspect drugs like Mono-Embolex (certoparin sodium) 3000 IU once a day since the patient was bedridden from unknown date, Inspra (eplerenone) for cardiac failure from Sep 2008, Insuman Comb (insulin human, insulin human injection, isophane) for type 2 diabetes mellitus from Sep 2007, Kalinor (potassium bicarbonate, potassium citrate) for potassium replacement from Sep 2008, omeprazole (manufacturer unknown) 40 mg once a day for gastric ulcer prophylaxis from Sep 2008, simvastatin (manufacturer unknown) 40 mg once a day for hyperlipidemia from Sep 2007 to 07 Apr 2009 and torasemide (manufacturer unknown) for renal impairment from unknown date. On unknown date the patient had status post perforation with peritoneal abscess, peritonitis and status post paralytic ileus the outcome of which was not reported. The patient also developed neuropathy and had an indwelling bladder catheter (cause unknown). The outcome of the events was not reported. The causality was reported as dubious for diarrhoea and possible for hepatic enzymes increased with Voltaren and dubious for diarrhoea and hepatic enzymes increased with Mono-embolex. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event " appendicitis, peritoneal abscess, peritonitis, ileus paralytic and cardiac failure " and "Voltaren dispers and Mono-embolex" as suspected based on the current available information. The event "urinary tract infection and neuropathy" were assessed as non-serious.

- Hepatic enzymes increased [hospitalization], already listed in the core data sheet. Reporting healthcare professional causality is suspected.

In the absence of the reporting healthcare professional causality, the Novartis Medical Safety Physician has provisionally assessed the event (appendicitis, peritoneal abscess, peritonitis, ile

Reporter's comment:

sanofi-aventis, 10.12.2009 DE-SANOFI-AVENTIS-A04200900562

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-WEIMAR-10225) and was extracted from the website of the German health authority (authority number: DE-BFARM- 09052398). The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

The case involves a 79-year-old male patient with history of thrombophlebitis in 1972, pulmonary embolism in 2004, perforated appendicitis associated with peritoneal abscess, peritonitis, and paralytic ileus on 10-Sep-2008. At time of event onset, the patient was suffering from hyperlipidaemia, insulin-dependent type 2 diabetes mellitus with neuropathy, chronic renal insufficiency since Sep-2007, arterial hypertension, cardiac failure since Sep-2008, sleep disorder, hesitancy leading to permanent bladder catheterization, bilateral gon- and coxarthrosis, peripheral arterial occlusive disease, and prostatic hyperplasia.

The following drugs were reported to have been taken concomitantly:

since Sep-2007: isophane insulin+insulin human

since Sep-2008 or earlier: eplerenone, potassium chloride, heparin-fraction, omeprazole

since Sep-2008: torademide

since Jan-2009: zolpidem

from 18-Mar-2009 to 28-Mar-2009: amoxicillin trihydrate+clavulanate potassium.

The patient initiated treatment with simvastatin (no trade name provided) 40 mg/day for hyperlipidaemia in Sep-2007 or earlier. On unspecified date in 2007, he started to take 1-2 of 50-mg diclofenac sodium tablets weekly for coxarthrosis as needed. On 07-Apr-2009 the patient was admitted to a hospital due to increased hepatic enzymes (aspartate aminotransferase 1.62 umol/l*s, alanine aminotransferase 2.41 umol/l*s, alkaline phosphatase 8.05 umol/l, gamma-GT 19.97 umol/l*s) that had existed since Oct-2008. Additionally, the patient had suffered from diarrhea since 02-Apr-2009. On 07-Apr-2009 simvastatin was discontinued. From 07-Apr-2009 to 09-Apr-2009 electrolyte infusions 1000 ml daily were administered. Diclofenac sodium was not given during the hospitalization. Treatments with isophane insulin+insulin human, eplerenone, potassium chloride, heparin-fraction, omeprazole, torademide, and zolpidem were continued. There were negative results for anti-HAV-IgM, HBsAg, Anti-HBc, Anti-HBc-IgM, Anti-HCV, noro-, adeno- and rota-virus, Clostridium difficile toxin A/B and liver cytosol antigen type 1. Laboratory test on 09-Apr-2009 showed obvious regressive change in increased hepatic enzymes (outcome: "recovering/resolving after 09-Apr-2009") and diarrhea had resolved.

The PV centre reported "increased hepatic enzyme" as adverse reaction with start date of "01-Oct-2008". Simvastatin and diclofenac were suspected for the reaction.

Causality determined by PV centre concerning "increased hepatic enzymes":

"possible" for simvastatin and diclofenac

Explanation of the assessment:

Simvastatin: the causal relationship with simvastatin is possible due to positive dechallenge. An acute or autoimmune hepatitis and an alcohol-induced liver damage couldn't be ruled out.

Diclofenac: frequent occurrence of increased transaminases and occasional incidences of liver damage and acute hepatitis with and without jaundice under long-term administration were described in the product information. The patient had received diclofenac for a long term, but in the low dose (only 50-100 mg/week), therefore the causal relationship is possible. During the hospitalization diclofenac was not given and hepatic enzyme levels decreased, that could be assessed as positive dechallenge. An ethyl-toxic liver damage (patient reported to have taken 1 glass of beer near-daily) could not be ruled out.

All other diagnoses and symptoms were only mentioned by the PV centre in its unformatted text.

Additional information received on 03-Dec-2009:

The following follow-up information has been obtained from the website of the German regulatory authority (BfArM), initially provided via the PV-centre:

All the medications initially reported as concomitant drugs (isophane insulin+insulin human, eplerenone, potassium chloride, heparin-fraction, omeprazole, torademide and zolpidem) were recorded in the fields of suspected drugs in the follow-up report of the BfArM. However, the PV centre stated in their case narrative that the increased liver values occurred under simvastatin and diclofenac. Therefore, only the both drugs (simvastatin and diclofenac) remained as suspected drugs as initially reported. Further new information was not provided.

Sender diagnosis: not reported

Narrative

Information has been received from a physician via the German agency and a regional pharmacovigilance center concerning a 79-year-old bedridden male patient with a daily use of alcohol (one glass of beer daily), coxarthrosis, cardiac failure, type 2 diabetes mellitus, potassium replacement, gastric ulcer prophylaxis, sleep disorder, renal impairment, hypertension arterial, hesitancy, gonarthrosis, peripheral arterial occlusive disease, prostatic hyperplasia and a history of pulmonary embolism, appendicitis and thrombophlebitis who, in September 2007, was placed on therapy with simvastatin (manufacturer unknown), 40 mg tablet, 40 mg, once a day for the treatment of hyperlipidaemia (exact duration not reported). Concomitant suspect therapy included amoxicillin/clavulanate potassium (AMOXI CLAVULAN), 875/125 mg tablet, 875/125 mg twice a day for the treatment of urinary tract infection (not reported as an adverse event by the agency/physician; treatment from 18-MAR-2008 until 28-MAR-2009), diclofenac Na (VOLTAREN DISPERS), 50 mg tablet, 50 mg as needed (about one to two tablets a week) for the treatment of coxarthrosis (therapy initiated in 2007; exact duration not reported), eplerenone (INSPIRA), 25 mg tablet, 25 mg once a day taken from September 2008 for the treatment of cardiac failure (exact dose and duration not reported), insulin, biphasic isophane [injection] (INSUMAN COMB 25/75), 28/0/8 I administered from September 2007 daily for the treatment of type II diabetes mellitus, KHCO₃/K citrate (KALINOR), effervescent tablet, 1 tablet taken from September 2008 twice a day as potassium replacement (exact dose and duration not reported), certoparin sodium (MONO EMBOLEX), injection form, 3000 I daily administered subcutaneously for bedriddenness (duration not reported), omeprazole, 40 mg tablet, 40 mg once a day taken from September 2008 as gastric ulcer prophylaxis (exact duration not reported) and torsemide (TORASEMIDE HEXAL), 100 mg tablet, 100 mg taken daily from September 2008 for the treatment of renal impairment (exact duration not reported) .

Other concomitant therapy included zolpidem tartrate (STILNOX).

In October 2008, the patient experienced increased liver enzymes (values not reported). On 02-APR-2009, the patient developed diarrhoea. On 07-APR-2009, he was hospitalized due to diarrhoea and increased liver enzymes. That same day, therapy with simvastatin was discontinued (last intake was on 06-APR-2009). On 07-APR-2009, laboratory analysis revealed AST 1.62 mcml/s.l; ALT 2.41 mcml/s.l; AP 8.05 mcml/l; GGT 19.97 mcml/l.s. No evidence of Norwalk-like viruses, adenoviruses or rotaviruses were found; Clostridium difficile A/B negative. On 08-APR-2009, serum hepatitis B core immunoglobulin M antibody test, serum hepatitis C antibody test, serum hepatitis B surface antigen test, serum hepatitis B core immunoglobulin G antibody test and serum hepatitis A immunoglobulin M antibody test were negative. From 07-APR-2009 until 09-APR-2009, he received treatment with dextrose (+) electrolytes (unspecified) (STEROFUNDIN) infusions, 1000 ml daily. On 09-APR-2009, the patient recovered from diarrhoea. On 09-APR-2009, laboratory analysis revealed AST 1.17 mcml/s.l; ALT 1.94 mcml/s.l; AP 7.28 mcml/l; GGT 16.69 mcml/l.s, i.e. the patient was recovering from increased liver enzymes.

The reporting physician/agency/regional pharmacovigilance center felt that increased liver enzymes was possibly related to therapy with simvastatin and/or therapy with diclofenac Na (VOLTAREN DISPERS). The physician considered a viral origin as unlikely. Drug relationship regarding increased liver enzymes was rated as "dubious" for eplerenone (INSPIRA), insulin, biphasic isophane [injection] (INSUMAN COMB 25/75), KHCO₃/K citrate (KALINOR), certoparin sodium (MONO EMBOLEX), omeprazole and torsemide (TORASEMIDE HEXAL).

The reporting physician/agency/regional pharmacovigilance center felt that diarrhoea was likely related to therapy with amoxicillin/clavulanate potassium (AMOXI CLAVULAN) and possibly related to diclofenac Na (VOLTAREN DISPERS).

Drug relationship regarding diarrhoea was rated as "dubious" for simvastatin, eplerenone (INSPIRA), insulin, biphasic isophane [injection] (INSUMAN COMB 25/75), KHCO₃/K citrate (KALINOR), certoparin sodium (MONO EMBOLEX), omeprazole and torsemide (TORASEMIDE HEXAL).

Follow-up information received on 21-FEB-2010: The manufacturer of simvastatin could not be determined.

Additional information is not expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09059911

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GGT increased	200904				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/ 39	-200904	Back pain

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GGT	200904	600 U/L	U/L	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 11.05.2009 DE-002147023-PHHY2009DE16809

Sender diagnosis: not reported

Narrative

Initial report received from a consumer on 29 Apr 2009. Patient has been treated with Voltaren Dispers (diclofenac free) tablet as needed due to back pain for several years and developed an increase of gamma-glutamyltransferase (gamma-GT) 600 U/l (reference range not provided) in Apr 2009. Therapy was discontinued in Apr 2009. The reporter asked Novartis about a possible relationship to Voltaren and the reversibility of the symptoms. Event was ongoing at the time of the report.

Serious spontaneous report, Gamma glutamyl transferase increased [medically significant], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09085204

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19400519	Age: 69 Year	weight (kg): 85	BMI_int: 31.6	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	20090615			5 Month	recovered/resolved
2 Damage liver	20090615			5 Month	recovered/resolved
3 Conjunctiva colouring yellow	20090615			5 Month	recovered/resolved
4 Liver function test abnormal	20090615			5 Month	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC SANDOZ	Oral	150 Mg milligram(s)/Day	20090115-20090618	Arthrosis
diclofenac		94	5 Month	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cholecystectomy			k.A.	
Gastroesophageal reflux			k.A.	
Hypertonia			k.A.	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase increased	20090629	162 U/L		
Alkaline phosphatase increased	20090617	281 U/L		
Alpha globulin increased	20090617	3.9	%	
Bilirubin total	20090617	5.20 mg/dl		
Cholesterol	20091014	212 mg/dl		
CRP increased	20090617			
CRP increased	20090629	12.4 mg/dl		
Gamma-GT increased	20090629	98 U/L		
Gamma-GT increased	20090617	283 U/L		
GOT increased	20091014			
GOT increased	20090617	394 U/L		
GPT increased	20090617	874 U/L		
GPT increased	20090629	70 U/L		
Sedimentation rate decreased	20090629	32/64		
Sedimentation rate decreased	20090722	20/30		
Sedimentation rate decreased	20091014	18/28		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 03.07.2009 DE-SZ09-GXKR2009DE07458

Sender diagnosis: not reported

Narrative

Initial spontaneous consumer case report received from a patient's husband on 25 Jun 2009. On an unreported date in 2009, the patient with an unknown medical history, commenced a treatment with Diclofenac Sandoz 50mg gastro-resistant tablets (diclofenac) orally for an unknown indication. The patient developed icterus (yellow eyes, elevated liver function tests) on an unreported date. The treatment with suspect drug was discontinued and treatment received for the adverse event was unknown. Outcome: condition improving.

Local case ID: 1219

The available information was considered inadequate to fully assess the case.

Serious spontaneous report (hospitalisation) already listed in the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 03.11.2009 DE-SZ09-GXKR2009DE07458

Sender diagnosis: not reported

Narrative

Initial spontaneous consumer case report received from a patient's husband on 25 Jun 2009. On an unreported date in 2009, the patient with an unknown medical history, commenced a treatment with Diclofenac Sandoz 50mg gastro-resistant tablets (diclofenac) orally for an unknown indication. The patient developed icterus (yellow eyes, elevated liver function tests) on an unreported date. The treatment with suspect drug was discontinued and treatment received for the adverse event was unknown. Outcome: condition improving.

Local case ID: 1219

Significant follow up information received from physician on 23 Oct 2009. On 15 Jan 2009, the patient with a medical history of arterial hypertonia, gastro-oesophageal reflux, cholecystectomy, subclinical hypothyreosis commenced the treatment with treatment with Diclofenac Sandoz 50 mg magensaftresistente Tabletten (diclofenac sodium) for arthrosis. On 15 Jun 2009, the patient developed medical -toxic damage of liver parenchyma. The suspected drug was discontinued on 18 Jun 2009. The patient went to the hospital and has been treated stationary from 18 Jun 2009 to 24 Jun 2009 (withdrawal of all drugs). Patient's weight, height and lab data were added.

Outcome: recovered

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09090323

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials: Sex: Male Birth: 19400222 Age: 69 Year weight (kg): 84 BMI_int: 27.12 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased					Unknown
2 Abdominal pain NOS	20090301			5 Day	recovering/resolving
3 Pancreatitis	20090227	20090304	6 Day	2 Day	recovered/resolved
4 Amylase increased	20090302			6 Day	Unknown
5 Edematous pancreatitis	20090227	20090304	6 Day	6 Day	recovered/resolved
6 AST increased	20090302			6 Day	Unknown
7 Epigastric pain	20090227			3 Day	recovering/resolving
8 Lipase increased	20090302			6 Day	Unknown
9 Abdominal pain					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 buprenorphine buprenorphine	Transdermal	/	20090224-20090302 7 Day	Postoperative pain
2 IBUPROFEN ibuprofen	Oral	600 Mg milligram(s)/Day 82	2007-20090220 -	Coxarthrosis
3 OMEP omeprazole	Oral	40 Mg milligram(s)/Day 245	20090225- -	Gastric ulcer prophylaxis
4 PARACETAMOL paracetamol	Oral	1500 Mg milligram(s)/Day 245	20090226-20090227 2 Day	Postoperative pain
5 RAMIPRIL ramipril	Oral	2* /Day 245	2000- -	Hypertension arterial
6 VOLTAREN RESINAT diclofenac resinate	Oral	150 Mg milligram(s)/Day 6	20090225-20090227 3 Day	Postoperative pain

Drugs (concomitant)

7 BERLOSIN metamizole	Oral	1500 Mg milligram(s)/Day 245	20090226-20090227 2 Day	Postoperative pain
8 CEFUROXIME cefuroxime	Intravenous drip	1.5 Mg milligram(s)/Day 293	20090224-20090224 1 Day	Infection prophylaxis
9 FLOTRIN terazosin hydrochloride	Oral	2 Mg milligram(s)/Day 245	2005- -	Benign prostatic hyperplasia
10 norspan buprenorphine	Transdermal	/Day 250	20090224-20090302 7 Day	Postoperative pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Total hip replacement	200807		No	
Total hip replacement	20090224		No	
Prosthesis implantation	200807		Unknown	
Prosthesis implantation	20090224		Unknown	
Hypertension arterial	1990		Yes	
Alcohol abuse, unspecified drinking behavior			Yes	
Ex-smoker	1980		Yes	1955-1980 1 Packung/Woche
Alcohol use			Yes	Beginn unbekannt, 3 Bier+1 Flasche Rotwein in der Woche
Coxarthrosis			Yes	Beginn unbekannt, TEP links 07/2008, rechts 24.02.2009
Pancreatitis	200807	200807	No	
Benign prostatic hyperplasia	200807		Yes	Beginn unbekannt

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20090302	89	µmol/l	
Creatinine clearance	20090302	>1.0	ml/s	
Aspartate aminotransferase	20090223	26.0	U/l	
Aspartate aminotransferase	20090302	52.4	U/l	
Alanine aminotransferase	20090223	21.0	U/l	
Alanine aminotransferase	20090302	55.1	U/l	
Alkaline phosphatase	20090223	57.0	U/l	
Gamma GT	20090223	14.0	U/l	
Gamma GT	20090302	32.0	U/l	
C-reactive protein	20090302	24.9	mg/l	
Amylase	20090223	57.1	U/l	
Amylase	20090302	1495	U/l	
Lipase	20090302	1429	U/l	
Potassium	20090302	4.1	mmol/l	
Sodium	20090302	138	mmol/l	
Alanine aminotransferase	20090223	21	NA	
Alanine aminotransferase	20090302	55.1	NA	
Amylase	20090223	57.1	NA	
Amylase	20090302	1495	NA	
Aspartate aminotransferase	20090223	26	NA	
Aspartate aminotransferase	20090302	52.4	NA	
C-reactive protein	20090302	24.9	mg/l	
Lipase	20090302	1429	NA	
Ultrasonography	20090223	nicht vorhanden	NA	Absent

Ultrasonography	20090302	nicht vorhanden	absent
Calcium	20090223	2.31	mmol/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal / Universität Witten, 14.07.2009 DE-PVZIBE-02-10154

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Pankreatitis

KURZZUSAMMENFASSUNG:

akute Pankreatitis infolge von Diclofenac (Paracetamol)

BESCHREIBUNG:

seit 27.02.2009 leichte epigastrale Schmerzen;

ab 01.03.2009 heftig stechende Bauchschmerzen, in den Unterbauch ausstrahlend, Zustand nach Hüft-TEP-OP am 24.02.2009;

bei Aufnahme 12-fach erhöhter Lipase-Wert;

anamnestisch: 07/2008 identisches Ereignis nach 1. Hüft-TEP-OP (Arzneimittelanamnese nicht bekannt, OP war in Sachsen)

THERAPIE:

initiale Nahrungskarenz, Flüssigkeitssubstitution (Sterofundin);

Schmerzmittel abgesetzt;

Würzburger Perfusor oder Haldol;

Clexane 40mg s.c.

VERLAUF:

kein typisches Vorliegen von Risikofaktoren für eine Pankreatitis;

schnelle Besserung der Symptomatik (keine weitere Laborkontrolle erfolgt);

02.03.2009 Sonographie Abdomen: ödematöse Pankreatitis, Pankreasgang nicht erweitert, Splenoportales Gefäßsystem unauffällig, Leber, Gallenblase und Gallenwege unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-04.03.2009;

Einweisungsgrund: akute Pankreatitis, Zustand nach Totaler Endoprothese des Hüftgelenks am 24.02.2009;

Im zeitlichen Verlauf wären Diclofenac, Paracetamol, Buprenorphin und Cefuroxim verdächtig. Für NSAR sind Pankreatiden beschrieben. Für Paracetamol, Buprenorphin und Cefuroxim gibt es Hinweise in der Literatur.

Nach der ersten Hüft-TEP im Juli 2008 trat ebenfalls eine Pankreatitis auf. Die damalige Arzneimittelanamnese ist leider nicht ermittelbar. Als Alternativursache für die Pankreatitis könnte eine Reaktion auf die Narkose diskutiert werden, ebenso auf den chirurgischen Eingriff. Allerdings sind bisher in der Literatur nur abdominalchirurgische Eingriffe mit Pankreatitiden assoziiert worden. Die Gallenwege waren sonographisch unauffällig. Gegen akuten Alkoholkonsum als Ursache spricht die normalwertige Gamma-GT. Triglyzeride wurden nicht bestimmt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:**MEDIKAMENT: RAMIPRIL**

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist in der Fachinfo als sehr selten beschrieben. Medikament wird schon seit mehreren Jahren eingenommen und wurde nicht abgesetzt. Trotz Weitergabe erfolgte eine Symptombesserung, daher wird Ramipril hier nicht verdächtig.

MEDIKAMENT: NORSPAN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Nur geringe bis gar kein Einfluss auf Sphincter Oddi (Staritz et al, 1986, Effect of modern analgesic on the bile duct sphincter in man. Gut 27:567);

In der Fachinfo sind keine Pankreatiden beschrieben, aber abdominale Schmerzen.

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Der Arzneimittelbrief (Jahrgang 36 Nr.7, Juli 2002): Paracetamol-assoziierte Pankreatiden beschrieben; in der Fachinfo sind Pankreatiden bei Überdosierung beschrieben.

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist eine bekannte UAW, ein zeitlicher Zusammenhang besteht.

MEDIKAMENT: OMEP

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist für Omeprazol beschrieben, Medikament wurde weitergegeben, es besteht daher kein zeitlicher Zusammenhang.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: In der Fachinfo sind Pankreatiden als sehr selten beschrieben.<dbtext>To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 22.07.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04909DE

Sender diagnosis: not reported

Narrative

This is a spontaneous (health professional confirmed) report from the German Health Authority via Web-Tool.

On 16Jul09 information has been received concerning a 69-years old male patient who suffered from pancreatitis from 27Feb09 to 04Mar09 while on therapy with paracetamol 3 tablets per day from 26Feb09 to 27Feb09. The reporting physician assumed the relationship between paracetamol and the event as likely.

Relevant medical history included coxarthrosis, benign prostatic hyperplasia and arterial hypertension. The patient is alcohol user (3 bottles of beer and 1 bottle of red wine in the week) and is ex-smoker.

The patient suffered from pancreatitis in Jul08. Concomitant medication included Berlosin 500mg (metamizole sodium), 3 tablets daily for postoperative pain; Cefuroxime (cefuroxime) 1,5 mg daily for infection prophylaxis; Voltaren Resinat 75 mg (diclofenac resinate), daily 2 capsules for postoperative pain; Diclofenac Resinate (diclofenac resinate) for an unknown indication; Flotrin 2 mg (terazosin hydrochloride), daily 1 tablet for benign prostatic hyperplasia; Ibuprofen 600 mg (ibuprofen) daily 1 film-coated tablet for coxarthrosis; Norspan (buprenorphine) daily 1 transdermal patch for postoperative pain; Omeprazol 40 mg (omeprazole) daily 1 tablet for gastric ulcer prophylaxis and Ramipril (ramipril) daily 2 tablets (2,5-0-5 mg) for hypertension arterial.

Tracking:

16Jul09 - initial information via Web-Tool

SANOFI-SYNTHELABO, 28.07.2009 DE-SANOFI-SYNTHELABO-A04200901023

Sender diagnosis: not reported

Narrative

Reaction as reported: pancreatitis (lipase 25 xULN, amylase 14 xULN)

LLT: PANCREATITIS / 10033645

PT: PANCREATITIS / 10033645

Narrative:

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

NOVARTIS, 29.07.2009 DE-002147023-PHHY2009DE29503

Sender diagnosis: not reported

Narrative

Initial report was received from a physician via regional Pharmacovigilance Center (intensive monitoring project) via Health Authority (DE- BFARM - 09090323) on 15 Jul 2009: This patient with the history of pancreatitis in Jul 2008, smoking, drinking, coxarthrosis, benign prostatic hyperplasia and hypertension was on multi drug treatment including Voltaren resinat (diclofenac resinate) capsule, 75 mg bid for postoperative pain from 25 Feb 2009 to 27 Feb 2009. The co-suspects included Paracetamol, Norspan (buprenorphine) and Cefuroxim. The patient had hip total endoprosthesis (TEP) surgery on 24 Feb 2009. The patient experienced acute pancreatitis, mild epigastric pain 27 Feb 2009 and severe prickling abdominal pain on 01 Mar 2009. The patient was hospitalized from 02 Mar 2009 to 04 Mar 2009. On 02 Mar 2009, lipase increased to 1429 U/L (normal high range 57 U/L) and amylase increased to 1495 U/L (normal range 28-100 U/L). There was a mild increase in the AST (52.4 U/L) and ALT (55.1 U/L) ON 02 Mar 2009. The patient had experienced similar event after first hip TEP surgery in Jul 2008. The pain medications were stopped. The events were treated with initial food abstention, fluid substitution (Sterofundin); Wuerzburger perfusor or Haldol and Clexane 40 mg subcutaneous. On 02 Mar 2009, sonography abdomen showed edematous pancreatitis and pancreas duct was not enlarged with no other findings. The symptoms improved fast and pancreatitis resolved on 04 Mar 2009. Due to temporal relationship, Voltaren Resinat, Paracetamol, Norspan and Cefuroxim were suspected drugs.

HELIOS Klinikum Wuppertal, 26.01.2010 DE-PVZIBE-02-10154

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Pankreatitis

KURZZUSAMMENFASSUNG:

akute Pankreatitis infolge von Diclofenac (Paracetamol, Buprenorphin, Cefuroxim)

BESCHREIBUNG:

seit 27.02.2009 leichte epigastrale Schmerzen;

ab 01.03.2009 heftig stechende Bauchschmerzen, in den Unterbauch ausstrahlend, Zustand nach Hüft-TEP-OP am 24.02.2009;

bei Aufnahme 12-fach erhöhter Lipase-Wert;

anamnestisch: 07/2008 identisches Ereignis nach 1. Hüft-TEP-OP (Arzneimittelanamnese nicht bekannt, OP war in Sachsen)

THERAPIE:

initiale Nahrungskarenz, Flüssigkeitssubstitution (Sterofundin);

Schmerzmittel abgesetzt;

Würzburger Perfusor (Tramadol, Metamizol) und Haldol;

Clexane 40mg s.c.

VERLAUF:

kein typisches Vorliegen von Risikofaktoren für eine Pankreatitis (es wurde ein regelmäßiger Alkoholkonsum angegeben, allerdings zur Zeit wegen OP kein Alkohol);

schnelle Besserung der Symptomatik (keine weitere Laborkontrolle erfolgt);

02.03.2009 Sonographie Abdomen: ödematöse Pankreatitis, Pankreasgang nicht erweitert, Splenoportales Gefäßsystem unauffällig, Leber,

Gallenblase und Gallenwege unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-04.03.2009;

Einweisungsgrund: akute Pankreatitis, Zustand nach Totaler Endoprothese des Hüftgelenks am 24.02.2009;

Im zeitlichen Verlauf wären Diclofenac, Paracetamol, Buprenorphin und Cefuroxim verdächtig. Für NSAR sind Pankreatiden auch in der Fachinfo beschrieben. Für Paracetamol, Buprenorphin und Cefuroxim gibt es Hinweise in der Literatur (aber nicht eindeutig in der Fachinfo).

Nach der ersten Hüft-TEP im Juli 2008 trat ebenfalls eine Pankreatitis auf. Die damalige Arzneimittelanamnese ist leider nicht ermittelbar. Als Alternativursache für die Pankreatitis könnte eine Reaktion auf die Narkose diskutiert werden, ebenso auf den chirurgischen Eingriff. Allerdings sind bisher in der Literatur nur abdominalchirurgische Eingriffe mit Pankreatitiden assoziiert worden. Die Gallenwege waren sonographisch unauffällig. Gegen akuten Alkoholkonsum als Ursache spricht die normalwertige Gamma-GT. Triglyzeride wurden nicht bestimmt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: RAMIPRIL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist in der Fachinfo als sehr selten beschrieben. Medikament wird schon seit mehreren Jahren eingenommen und wurde nicht abgesetzt. Trotz Weitergabe erfolgte eine Symptombesserung, daher ist Ramipril hier unwahrscheinlich.

MEDIKAMENT: NORSPAN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Nur geringe bis gar kein Einfluss auf Sphincter Oddi (Staritz et al, 1986, Effect of modern analgesic on the bile duct sphincter in man. Gut 27:567);

In der Fachinfo sind keine Pankreatiden beschrieben, aber abdominale Schmerzen.

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Der Arzneimittelbrief (Jahrgang 36 Nr.7, Juli 2002): Paracetamol-assoziierte Pankreatiden beschrieben; in der Fachinfo sind Pankreatiden bei Überdosierung beschrieben.

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist eine bekannte UAW, ein zeitlicher Zusammenhang besteht.

MEDIKAMENT: OMEP

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist für Omeprazol beschrieben, Medikament wurde weitergegeben, darunter Symptombesserung - es besteht daher kein zeitlicher Zusammenhang.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: In der Fachinfo sind Pankreatiden als sehr selten beschrieben. Die Einnahme wurde 1 Woche vor Auftreten der Symptome beendet und über einen mehrjährigen Zeitraum gut vertragen.

NOVARTIS, 03.02.2010 DE-NOVNCHHQ-GXKR2009DE08058

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 15 Jul 2009 from the German Health Authority (DE-BfArM-09090323), provided by a pharmacovigilance center, reported by a physician: This multimorbid patient developed pancreatitis during treatment with paracetamol tablets (MAH unknown) for postoperative pain. Voltaren resinate (diclofenac resinate) was also suspected. Several other drugs were taken concomitantly. The outcome of the adverse event was reported as resolved. For paracetamol causality was assessed as likely.

Significant follow-up received from health authority (BfArM homepage, DE-BFARM-09090323) on 27 Jan 2010 changing several concomitant drugs into suspected drugs and changing causality assessment: Ramipril (MAH unknown), Omeprazole (omeprazole) and ibuprofen (MAH unknown) were also suspected. Causality for ramipril, Omeprazole and ibuprofen was assessed as unlikely. For paracetamol causality was assessed as possible. Spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 04.02.2010 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04909DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report health care professional confirmed report from the German Health Authority BfArM.

18.03.2013

BfArM - Pharmacovigilance

377 / 594

On 28Jan2010 follow-up information was received from a physician in Germany via the German Health Authority BfArM concerning a 69-year-old male patient, who was on paracetamol 3x500 mg/day from 26Feb2009 to 27Feb2009 for postoperative pain, on ramipril 2,5-0-5 mg/day since 200 and continuing for arterial hypertension, on Voltaren Resinat (diclofenac resinate) 2x75 mg/day from 25Feb2009 to 27Feb2009 for postoperative pain, Omep (omeprazole) 1x40 mg/day since 25Feb2009 and continuing for gastric ulcer prophylaxis and on ibuprofen 1x600 mg/day from 2007 to 20Feb2009 for coxarthrosis. The event was considered as serious due to the statement of the reporter. Ramipril and Omep were continued. Paracetamol, Voltaren Resinat and ibuprofen were discontinued. The patient has recovered.

From 27Feb2009 to 04Mar2009 the patient suffered from pancreatitis.

Comments to the following event:

Pancreatitis

Short summary:

acute pancreatitis due to diclofenac (Paracetamol, Buprenorphin, Cefuroxim)

Description:

since 27Feb2009 mild epigastric pain,

since 01Mar2009 severe stabbing stomach pain, emissive into the lower abdomen, condition after hip-TEP-operation on 24Feb2009,

during accommodation 12-point increased lipase value,

anamnesic: Jul2008 identical event after first hip-TEP-operation (medicament anamnesis not known, operation was in Saxony)

Therapy:

initial abstinence from food, liquid substitution (Sterofundin)

analgesics discontinued

Würzburger perfusion (tramadol, metamizol) and Haldol,

Clexane 40 mg sc

Course:

no typical existent of risk factors for a pancreatitis (regular intake of alcohol was stated, but at the time of the operation no alcohol),

fast improvement of the symptomatology (no further laboratory test has been carried out)

02Mar2009 sonography abdomen: oedematous pancreatitis, pancreatic duct not expanded, splenoportal vascular system without pathological findings,

liver, gallbladder and bile duct without pathological findings

explanation of the event:

hospitalisation from 02Mar2009 - 04Mar2009

reason for hospitalisation: acute pancreatitis, condition after total endoprosthesis of hip joint on 24Feb2009,

in chronological sequence Diclofenac, Paracetamol, Buprenorphin and Cefuroxim are suspected. For NSAR are pancreatitis also described in the

product information. For Paracetamol, Buprenorphin and Cefuroxim there are references in the literature (but not definite in the product information)

After the first hip-TEP in Jul2008 a pancreatitis also occurred. At that time a medicament anamnesis is not ascertainable. As alternative reason for the pancreatitis could be discussed a reaction due to the anaesthesia, as well due to the surgical intervention. However until now only abdominal surgical interventions are associated with pancreatitis in literature.

The bile duct were sonographic without pathological findings. Against acute intake of alcohol as reason speaks the normal value of Gamma-GT (glutamyl transpeptidase).

Triglycerides were not calculated.

Concomitant medications included Flotrin (terazosin hydrochloride) 1x2 mg/day since 2005 and continuing for benign prostatic hyperplasia, Cefuroxime (cefuroximaxetil) 1x1,5 mg/day on 24Feb2009 for infection prophylaxis, Norspan (buprenorphine) 1x1 plaster/day from 24Feb2009 to 02Mar2009 for postoperative pain and Berlosin (metamizole sodium) 3x500 mg/day from 26Feb2009 to 27Feb2009 for postoperative pain.

Concomitant diseases included ex-smoker since -1980 (1955-1980 1 package/week) and continuing, alcohol use (beginning unknown, 3 beer + 1 bottle wine a week) and continuing, coxarthrosis (beginning unknown) and continuing, benign prostatic hyperplasia (beginning unknown) and continuing and arterial hypertension sine 1990 and continuing.

Past diseases included TEP (total endoprosthesis) left in Jul2008, TEP (total endoprosthesis) right on 24Feb2009 and pancreatitis in Jul2008.

The reporter rated the causal relationship between the event and Paracetamol and Voltaren Resinat as possible and rated the causal relationship between the event and Ramipril, Omep and Ibuprofen as unlikely.

Tracking of Changes:

16Jul2009 - initial information from physician via the German Health Authority BfArM

28Jan2010 - follow-up information from physician via the German Health Authority BfArM

sanofi-aventis, 04.02.2010 DE-SANOFI-AVENTIS-A04200901023

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority

(authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

Additional information extracted from the website of the German health authority on 28-Jan-2010. Information by PV centre: The patient had reported regular alcohol consumption, but around the time of operation the patient did not consume alcohol.

The following drugs were switched from concomitant to suspect medication: ramipril (trade name not provided) 7.5mg/day from 2000 onwards for arterial hypertension, ibuprofen (trade name not provided) 600mg/day from 2007 to 20-Feb-2009 for coxarthrosis, and omeprazole 1x40mg/day from 25-Feb-2009 onwards as gastric ulcer prophylaxis.

Causality assessment was changed by reporter to WHO score:

"possible" for paracetamol, diclofenac,

"unlikely" for ramipril, ibuprofen, omeprazole.

Comments regarding causality of new suspect drugs:

Pancreatitis is described in physicians' information for ramipril as very rare; the drug had been taken for several years, the symptoms improved despite continuation of treatment, therefore ramipril is rather unlikely in this case. Pancreatitis is described in physicians' information for ibuprofen as very rare; the drug was stopped one week before onset of symptoms and had been tolerated over a longer period of time. Pancreatitis is described for omeprazole; symptoms improved despite continuation of treatment with omeprazole, therefore no temporal relationship exists in this case.

NOVARTIS, 05.02.2010 DE-002147023-PHHY2009DE29503

Sender diagnosis: not reported

Narrative

Initial report was received from a physician via regional Pharmacovigilance Center (intensive monitoring project) via Health Authority (DE- BFARM - 09090323) on 15 Jul 2009: This patient with the history of pancreatitis in Jul 2008, smoking, drinking, coxarthrosis, benign prostatic hyperplasia and hypertension was on multi drug treatment including Voltaren resinat (diclofenac resinate) capsule, 75 mg bid for postoperative pain from 25 Feb 2009 to 27 Feb 2009. The co-suspects included Paracetamol, Norspan (buprenorphine) and Cefuroxim. The patient had hip total endoprosthesis (TEP) surgery on 24 Feb 2009. The patient experienced acute pancreatitis, mild epigastric pain 27 Feb 2009 and severe prickling abdominal pain on 01 Mar 2009. The patient was hospitalized from 02 Mar 2009 to 04 Mar 2009. On 02 Mar 2009, lipase increased to 1429 U/L (normal high range 57 U/L) and amylase increased to 1495 U/L (normal range 28-100 U/L). There was a mild increase in the AST (52.4 U/L) and ALT (55.1 U/L) ON 02 Mar 2009. The patient had experienced similar event after first hip TEP surgery in Jul 2008. The pain medications were stopped. The events were treated with initial food abstention, fluid substitution (Sterofundin); Wuerzburger perfusor or Haldol and Clexane 40 mg subcutaneous. On 02 Mar 2009, sonography abdomen showed edematous pancreatitis and pancreas duct was not enlarged with no other findings. The symptoms improved fast and pancreatitis resolved on 04 Mar 2009. Due to temporal relationship, Voltaren Resinat, Paracetamol, Norspan and Cefuroxim were suspected drugs.

Follow-up information received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority on 27 Jan 2010: Ramipril (manufacturer unknown) 2.5 mg and 5 mg two times a day for hypertension arterial from 2000 and Ibuprofen (manufacturer unknown) 600 mg per day for coxarthrosis from 2007 to 20 Feb 2009 were added as co-suspect in this case. Norspan (buprenorphine) and Cefuroxim was changed from suspect drug to concomitant medications. The onset date for pancreatitis was reported as 27 Feb 2009.

sanofi-aventis, 07.04.2010 DE-SANOFI-AVENTIS-A04200901023

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority 18.03.2013

(authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

Additional information extracted from the website of the German health authority on 28-Jan-2010. Information by PV centre: The patient had reported regular alcohol consummation, but around the time of operation the patient did not consume alcohol.

The following drugs were switched from concomitant to suspect medication: ramipril (trade name not provided) 7.5mg/day from 2000 onwards for arterial hypertension, ibuprofen (trade name not provided) 600mg/day from 2007 to 20-Feb-2009 for coxarthrosis, and omeprazole 1x40mg/day from 25-Feb-2009 onwards as gastric ulcer prophylaxis.

Causality assessment was changed by reporter to WHO score:

"possible" for paracetamol, diclofenac,

"unlikely" for ramipril, ibuprofen, omeprazole.

Comments regarding causality of new suspect drugs:

Pancreatitis is described in physicians' information for ramipril as very rare; the drug had been taken for several years, the symptoms improved despite continuation of treatment, therefore ramipril is rather unlikely in this case. Pancreatitis is described in physicians' information for ibuprofen as very rare; the drug was stopped one week before onset of symptoms and had been tolerated over a longer period of time. Pancreatitis is described for omeprazole; symptoms improved despite continuation of treatment with omeprazole, therefore no temporal relationship exists in this case.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09097281

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19430517	Age:	weight (kg): 61	BMI_int: 21.87	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac sodium	Oral	75 Mg milligram(s)/	20090123-20090621	
diclofenac sodium			4 Month	
2 FLUPIRTINE	Oral	400 Mg milligram(s)/Day	20090526-20090621	
flupirtine			27 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Coxarthrosis			Unknown	left side, history of physical therapy

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20090621	3.23 mmol/l	NA	
Alanine aminotransferase	20090621	6.40 mmol/l	NA	
Alanine aminotransferase	20090703	3.47 mmol/l	NA	
Aspartate aminotransferase	20090703	1.57 mmol/l	NA	

Narratives (nur bei elektronischem Eingang)

TEVA, 23.07.2009 DE-TEVA-202656ISR

Sender diagnosis: not reported

Narrative

15-Jul-2009, Spontaneous Serious Report, Forwarded by Germany (Local #FLPDS-09-52).

A physician reported the case of a 66 year old female patient being on Katadolon S long to the company. Because of a severe coxarthrosis (with need for TEP) the patient started Diclofenac (Brand: Diclo 75 SL, not a Teva product) in a dose of 1-2 capsules daily on 23-JAN-2009. On 26-MAY-2009 Katadolon S long in a dose of 1 tablet daily had been started additionally. On 21-JUN-2009 increased values of ASAT (3,23 mmol/l) and ALAT (6,40 mmol/l) had been noticed. Both drugs had been discontinued (21-JUN-2009). Transaminases improved with values of ALAT = 3,47 mmol/l and ASAT = 1,57 mmol/l on 03-JUL-2009, although the patient has not yet fully recovered.

The physician assessed the causality as probably related to the drug combination of Diclofenac and Flupirtin.

This case was considered serious based on the following criteria: Medically serious

Teva Comment: Possibly related to both suspected drugs, based on positive dechallenge.

reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09099438

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth: 19790831	Age: 29 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests raised	20090715			3 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ANTIALLERGIC AGENTS		/	-	Pollinosis
2 VOLTAREN DISPERS	Oral	2 Df dosage form/Day	20090713-20090715	Back pain
diclofenac sodium			3 Day	

Drugs (concomitant)

3 Kontrazeptiva	/	-
		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pollinosis			k.A.	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function test	20090715	600		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 31.07.2009 DE-002147023-PHHY2009DE29840

Sender diagnosis: not reported

Narrative

Initial consumer report received on 17 Jul 2009: This patient had been treated with Voltaren Dispers (diclofenac free acid) from 13 Jul 2009 until 15 Jul 2009 for back pain. Also other medications had been given, but those have not been specified. A routine laboratory check on 15 Jul 2009 had revealed an increase of a liver function test to around 600. The individual liver value and/or unit had not been further specified. Voltaren Dispers had been stopped on 15 Jul 2009. The event outcome was not reported.

Serious spontaneous report, liver function test abnormal [medically significant], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 03.09.2009 DE-002147023-PHHY2009DE29840

Sender diagnosis: not reported

Narrative

Initial consumer report received on 17 Jul 2009: This patient had been treated with Voltaren Dispers (diclofenac free acid) from 13 Jul 2009 until 15 Jul 2009 for back pain. Also other medications had been given, but those have not been specified. A routine laboratory check on 15 Jul 2009 had revealed an increase of a liver function test to around 600. The individual liver value and/or unit had not been further specified. Voltaren Dispers had been stopped on 15 Jul 2009. The event outcome was not reported.

Follow-up information received from consumer on 31 Aug 2009: The therapy with Voltaren Dispers was discontinued on 15 Jul 2009 and the liver function test had significantly improved. Ultrasound diagnostics did not reveal any pathology. The father had stated that the underlying cause could not be established, adding that the use of an antiallergic drug (against pollinosis) might have also caused the event. The patient had denied to visit the treating physician again, and the father mentioned that, therefore, no further details were to be expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09105492

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19261129	Age: 82 Year	weight (kg): 98	BMI_int: 30.25	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20090629				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclac 75 ID, 75 mg, Retardtabl, Hexal diclofenac	Oral	/ 212	2007-20090628	-
2 Unacid PD oral, 375 mg, Filmtabl, Pfizer sultamicillin	Oral	/Day 82	20090626-20090629 4 Day	

Drugs (concomitant)

3 Aricept, 5 bzw. 10 mg, Filmtabl, Eisai GmbH donepezil	Oral	1 Df dosage form/Day 82	2003-20090628	Dementia Alzheimer's type
4 ASS ratiopharm TAH, 100 mg, Tabl acetylsalicylic acid	Oral	100 Mg milligram(s)/Day 245	1999-20090628	-
5 Fenistil, 4 mg, Amp, Novartis dimetindene	Intravenous (not otherwis	4 Mg milligram(s)/Day 120	20090626-20090626	Bug bite
6 Gabapentin, 600 mg, Filmtabl, Hexal gabapentin	Oral	1200 Mg milligram(s)/Day 82	2006-	-
7 Lorano, 10 mg, Tabl, Hexal loratadine	Oral	10 Mg milligram(s)/Day 245	20090626-20090626	-
8 Metohexal-Succ bzw. Beloc-Zok mite, 95 bzw. metoprolol	Oral	95 Mg milligram(s)/Day 212	2007-	-
9 Solu-Decortin H, 250 mg, Amp, Merck Pharma prednisolone	Intravenous (not otherwis	250 Mg milligram(s)/Day 120	20060626-20060626	Bug bite
10 Tramadol retard, 100 mg, Tabl, NN tramadol	Oral	200 Mg milligram(s)/Day	20090623-20090624 2 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cardiac pacemaker insertion	1996		k.A.	Z.n. DDD-Schrittmacher-Implantation
Hypertension arterial			k.A.	
Hypertensive heart disease			k.A.	
Dementia Alzheimer's type			k.A.	
Phlegmon	20090626		k.A.	Unterarmphlegmone rechts
Hyperthyroidism			k.A.	manifeste Hyperthyreose

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20090629	80	U/l	
Alanine aminotransferase increased	20090630	512	U/l	
Alanine aminotransferase increased	20090701	750	U/l	

Alanine aminotransferase increased	20090702	574	U/l
Alanine aminotransferase increased	20090706	223	U/l
Alanine aminotransferase increased	20090710	97	U/l
Aspartate aminotransferase increased	20090629	45	U/l
Aspartate aminotransferase increased	20090630	533	U/l
Aspartate aminotransferase increased	20090701	548	U/l
Aspartate aminotransferase increased	20090702	210	U/l
Aspartate aminotransferase increased	20090706	56	U/l
Aspartate aminotransferase normal	20090710	35	U/l
Bilirubin increased	20090629	12.8	mg/dl
Bilirubin increased	20090630	8.5	mg/dl
Bilirubin increased	20090701	6.1	mg/dl
Bilirubin increased	20090710	3.7	mg/dl
Alkaline phosphatase normal	20090629	101	U/l
Alkaline phosphatase normal	20090630	92	U/l
Alkaline phosphatase normal	20090701	112	U/l

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 12.08.2009 DE-FAKOS-170266

Sender diagnosis: not reported

Narrative

Der Patient war vom 20.-28.06.2009 in der Kurzzeitpflege. Am 25.06.2009 bestand Verdacht auf einen Insektenstich (entzündliches Lymphödem des Handrückens). Es erfolgte die Gabe von einer Tablette Lorano, 1 Amp. Fenistil, 250 mg Solu-Decortin und der Beginn einer Unacid-Therapie am 26.06.2009.

Am 29.06.2009 wurde der Patient mit der Feuerwehr ins Krankenhaus gebracht wegen eines generalisierten Ikterus und Schluckbeschwerden. Labor vom 29.06.2009: ALAT 80 U/l (Normbereich: 0-45 U/l), ASAT 45 U/l (Normbereich: 0-35 U/l), Bilirubin gesamt 12,8 mg/dl (Normbereich: 0,1-1,2 mg/dl), Alkalische Phosphatase (AP) 101 U/l (Normbereich: 40-140 U/l). Unacid wurde am 29.06.09 abgesetzt.

Labor vom 30.06.2009: ALAT 512 U/l, ASAT 533 U/l, Bilirubin gesamt 8,5 mg/dl, Alkalische Phosphatase (AP) 92 U/l. Die Maximalwerte der Aminotransferasen wurden am 01.07.2009 erreicht: ALAT 750 U/l, ASAT 548 U/l (weiterhin AP 112 U/l). Außerdem wurde eine immunhämolytische Anämie mit Kälteautoantikörpern diagnostiziert, woraufhin der Patient 2 Erythrozytenkonzentrate am 10.07.2009 erhielt. Nach Besserung der Leberwerte am 10.07.2009 (ALAT 97 U/l, ASAT 35 U/l, Bilirubin gesamt 3,7 mg/dl), wurde der Patient am 16.07.2009 entlassen.

Die UAW hat sich ambulant entwickelt.

Als Auslöser der Hepatitis werden verdächtigt:

Unacid Tabl. wird als "wahrscheinliche" UAW-Ursache angenommen (Einnahme: 26.-29.06.2009).

Diclac Hexal wird mit "möglich" bewertet (Einnahme: 2007-20.06.2009 und 25.-28.06.2009)<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09120709

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice NOS					Unknown
2 Metastases NOS					Unknown
3 Liver disorder	2009				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac Sandoz 50mg diclofenac sodium	Oral	50 Mg milligram(s)/	-	-

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09119440

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19581124	Age: 50 Year	weight (kg): 72	BMI_int: 21.74	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20090828				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	50 Mg milligram(s)/	20090615-	Arthralgia
diclofenac			-	
2 peg-interferon alfa 2a	Subcutaneous	180 µg microgram(s)/ We	20081219-	Chronic hepatitis C
peginterferon alfa-2a			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthralgia			Yes	
Chronic hepatitis C			Yes	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Transaminases	20090828	INCREASED	UNITLESS	

Narratives (nur bei elektronischem Eingang)

[ROCHE, 10.09.2009 DE-ROCHE-655266](#)

Sender diagnosis: not reported

Narrative

INITIAL INFORMATION FOR THIS SOLICITED REPORT WAS RECEIVED ON 2 SEP 2009 FROM A PHYSICIAN AND CONCERNS A 50YEAR-OLD MALE PATIENT WHO EXPERIENCED INCREASED TRANSAMINASES WHILE RECEIVING PEG-INTERFERON ALFA 2A FOR CHRONIC HEPATITIS C AND DICLOFENAC FOR ARTHRALGIA. THE PATIENT (#2815/241158) IS ENROLLED INTO NON INTERVENTIONAL STUDY ML21645.

THE PATIENT'S MEDICAL HISTORY AND ONGOING MEDICATIONS WERE NOT REPORTED.

THE PATIENT STARTED SUBCUTANEOUS PEG-INTERFERON ALFA 2A AT 180 UM PER WEEK ON 19 DEC 2008 AND ORAL DICLOFENAC AT 50 MG (UNTIL 1-0-1) ON 15 JUN 2009. ON 28 AUG 2009 PATIENT'S TRANSAMINASES WERE INCREASED AND HAVE BEEN PERSISTING SINCE THEN, WHICH REQUIRES INTENSIVE LAB-COURSE CONTROL. PEG-INTERFERON ALFA 2A AND DICLOFENAC WERE ONGOING.

THE REPORTER ASSESSED THE EVENT AS MEDICALLY SIGNIFICANT.
THE DRUG/EVENT RELATIONSHIP WAS ASSESSED AS UNCLEAR.

reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09125095

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Shock
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pericardial effusion					fatal
2 Myoglobin blood increased					Unknown
3 Shock					fatal
4 Bruise					Unknown
5 Somnolence					Unknown
6 Thrombocytopenia					Unknown
7 Allergic reaction					fatal
8 Anaphylactic shock					fatal
9 Bruise					Unknown
10 Muscle cramps					Unknown
11 Pericardial effusion					Unknown
12 Perioral numbness					Unknown
13 Ascites					fatal
14 CRP increased					Unknown
15 Multiple organ failure					fatal
16 Ferritin increased					Unknown
17 Hemoglobin decreased					Unknown
18 Leucocytosis NOS					Unknown
19 Liver enlargement					fatal
20 Cramp muscle					Unknown
21 Pleural effusion					fatal
22 Blood creatinine increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexamethasone		/	-	
dexamethasone			-	
2 diclofenac		150 Mg milligram(s)/Day	-	
diclofenac			-	
3 piroxicam	Intramuscular	/	-	
piroxicam			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Unknown	
Spinal disorder			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Stammschult T., Arnold R., Brune K., Gundert-Remy U., Fatal multiple organ failure in association with a single intramuscular injection of piroxicam and dexamethasone and oral intake of diclofenac. Br J Clin Pharmacol. 2009;68 (1):6-59	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		1.87 mg/dl	NA	
Serum creatinine		1.8 g/dl	NA	
CT scan		NA	NA	
C-reactive protein		NA	NA	
Hemoglobin		7.9 g/dl	NA	
Heart rate		111/min	NA	
Leucocytosis NOS		17300/ul	NA	
Myoglobinemia		357 u/l	NA	
Procalcitonin		0.56 ug/l	NA	
Thrombocytopenia		62,000/ul	NA	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.09.2009 DE-002147023-PHHY2009DE39740

Sender diagnosis: not reported

Narrative

Initial literature report received via Drug Commission of the German Medical Association (DCGMA) on 14 Sep 2009: This patient with lumbar spine syndrome was treated with i.m. injection of 20 mg piroxicam with dexamethasone (dose not stated). The patient reported perioral numbness lasting a few minutes. He was discharged with a prescription of diclofenac (manufacturer unknown) 75 mg twice daily. Ten minutes after taking the first dose muscular cramps occurred, the patient became somnolent. At re-admission to the hospital the findings were: BP 127/80, heart rate 111/min. The skin showed multiple bruises. Laboratory: hemoglobin 7.9 g/dl, thrombocytopenia (62000/ul), leucocytosis (17300/ul), CRP tenfold upper normal range, serum myoglobin 357 u/l (<90), serum creatinine 1.8 g/dl, bilirubin 1.87 mg/dl, thromboplastin time 67%, procalcitonin 0.56 ug/l, ferritin 15137 ug/l. A CT scan showed liver enlargement, ascites, pleural and pericardial effusions. In spite of intensive care, the patient died from multiple organ failure the next morning. A post mortem confirmed shock as cause. No evidence of infectious diseases, use of illicit drugs or life-threatening diseases was found. After having ruled out alternative explanations, the concomitant administration of dexamethasone, piroxicam and diclofenac was judged the most probable cause for the fatal organ failure. Despite extensive diagnostic measures and a post mortem, the causality assessment remains possible or probable.

Leukocytosis, thrombocytopenia, somnolence, haemoglobin decreased, [hospitalisation], already listed in the Core Data Sheet.

All literature reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

The remaining events were assessed as non-serious by the reporting health care professional/Novartis Medical Safety Physician.

Reporter's comment:

sanofi-aventis, 22.01.2010 DE-SANOFI-AVENTIS-2010SA001908

Sender diagnosis: not reported

Narrative

Initial Report:

This literature serious case was received on 11-Jan-2010.

A case reported hinting at a serious, acute allergic reaction is presented here. A 27-year-old man presented to an emergency unit with back pain lasting for 2 days. A lumbar spine syndrome was diagnosed. After i.m. injection of 20 mg piroxicam with dexamethasone (dose not stated) the patient reported perioral numbness lasting a few minutes. He was discharged with a prescription of 75 mg diclofenac twice daily. Ten minutes after taking the first dose muscular cramps occurred, the patient became somnolent. At re-admission to the hospital the findings were: RR 127/80, heart rate 111/min. The skin showed multiple bruises. Laboratory: hemoglobin 7.9 g/dl, thrombocytopenia (62 000/ μ l), leucocytosis (17 300/ μ l), CRP tenfold upper normal range, serum myoglobine 357 μ l (<90), serum creatinine 1.8 g/dl, bilirubin 1.87 mg/dl, thromboplastin time 67%, procalcitonin 0.56 μ g/l, ferritin 15 137 μ g/l. A CT scan showed liver enlargement, ascites, pleural and pericardial effusions. In spite of intensive care, the patient died from multiple organ failure the next morning. A post mortem confirmed shock as cause. No evidence of infectious diseases, use of illicit drugs or life-threatening diseases was found. After having ruled out alternative explanations, the concomitant administration of

dexamethasone, piroxicam and diclofenac was judged the most probable cause for the fatal organ failure. Despite extensive diagnostic measures and a post mortem, the causality assessment remains 'possible' or 'probable'.

Reporter's comment:

sanofi-aventis, 10.02.2010 DE-SANOFI-AVENTIS-2010SA001908

Sender diagnosis: not reported

Narrative

Initial Report:

This literature serious case was received on 11-Jan-2010.

A case reported hinting at a serious, acute allergic reaction is presented here. A 27-year-old man presented to an emergency unit with back pain lasting for 2 days. A lumbar spine syndrome was diagnosed. After i.m. injection of 20 mg piroxicam with dexamethasone (dose not stated) the patient reported perioral numbness lasting a few minutes. He was discharged with a prescription of 75 mg diclofenac twice daily. Ten minutes after taking the first dose muscular cramps occurred, the patient became somnolent. At re-admission to the hospital the findings were: RR 127/80, heart rate 111/min. The skin showed multiple bruises. Laboratory: hemoglobin 7.9 g/dl, thrombocytopenia (62 000/ μ l), leucocytosis (17 300/ μ l), CRP tenfold upper normal range, serum myoglobine 357 μ l (<90), serum creatinine 1.8 g/dl, bilirubin 1.87 mg/dl, thromboplastin time 67%, procalcitonin 0.56 μ g/l, ferritin 15 137 μ g/l. A CT scan showed liver enlargement, ascites, pleural and pericardial effusions. In spite of intensive care, the patient died from multiple organ failure the next morning. A post mortem confirmed shock as cause. No evidence of infectious diseases, use of illicit drugs or life-threatening diseases was found. After having ruled out alternative explanations, the concomitant administration of dexamethasone, piroxicam and diclofenac was judged the most probable cause for the fatal organ failure. Despite extensive diagnostic measures and a post mortem, the causality assessment remains 'possible' or 'probable'.

09-FEB-2010: after internal review piroxicam was considered as non-company product

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09136417

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		No

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels					recovering/resolving
2 Icterus					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLO 1A PHARMA diclofenac	Oral	/	- -	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)NOVARTIS, 15.10.2009 DE-SZ09-GXKR2009DE11045

Sender diagnosis: not reported

Narrative

Initial spontaneous consumer case report received on 06 Oct 2009. The patient with an unknown medical history developed increase in liver enzyme count and icterus after intake of Diclo 50-1A Pharma (diclofenac sodium). An inpatient healthcare of several days was necessary. The patient took a total of approximately 10 tablets of diclofenac sodium. On unreported date, treatment with the suspected drug was withdrawn. Treatment received for the adverse event was unknown.

Outcome: condition resolving

Local case ID: 2009-469

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09138268

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multi-organ failure
deathdate:	deathcause: Shock

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Perioral numbness				10 Minute	Unknown
2 Muscle cramp				10 Minute	Unknown
3 Somnolence				10 Minute	Unknown
4 Heart rate increased				10 Minute	Unknown
5 Contusion				10 Minute	Unknown
6 Thrombocytopenia				10 Minute	Unknown
7 Acute allergic reaction				10 Minute	Unknown
8 Multi-organ failure				10 Minute	fatal
9 Leukocytosis				10 Minute	Unknown
10 Shock				10 Minute	fatal
11 Pericardial effusion				10 Minute	Unknown
12 Pleural effusion				10 Minute	Unknown
13 C-reactive protein increased				10 Minute	Unknown
14 Blood myoglobin increased				10 Minute	Unknown
15 Liver enlargement				10 Minute	Unknown
16 Ascites				10 Minute	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexamethasone		/	-	Lumbar syndrome
dexamethasone			-	
2 diclofenac	Oral	150 Mg milligram(s)/Day	-	Lumbar syndrome
diclofenac			-	
3 piroxicam	Intramuscular	/	-	Lumbar syndrome
piroxicam			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Stammsschulte T et al. Fatal Multiple Organ Failure in Association with A Single Intramuscular Injection of Piroxicam and Dexamethasone and Oral Intake of Diclofenac. Abstract presented at the 11th Annual Congress of Clinical Pharmacology;2009, Heidelberg, Germany.	

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 19.10.2009 DE-GlaxoSmithKline-B0598108A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of multiple organ failure in a 27-year-old male patient who received Diclofenac (brand name / trade name unknown) for lumbar spine syndrome.

Co-suspect medication included Piroxicam and Dexamethasone.

On an unknown date, the patient presented to an emergency unit with back pain lasting for 2 days and a lumbar spine syndrome was diagnosed. The patient was treated with intramuscular injection of 20 mg piroxicam with dexamethasone (dose not stated). It was reported that the patient reported perioral numbness lasting a few minutes and he was discharged with a prescription of 75 mg diclofenac twice daily. Ten minutes after taking the first dose the patient developed muscular cramps and the patient became somnolent. At re-admission his RR was 127/80, heart rate 111/min and the skin showed multiple bruises. Laboratory examinations revealed haemoglobin 7.9 g/dl, thrombocytopenia (62000/mcII), leucocytosis (17300/mcII), CRP tenfold upper normal range, serum myoglobine 357 (less than 90), serum creatinine 1.8 g/dl, bilirubin 1.87 mg/dl, thromboplastin time 67%, procalcitonin 0.56 mcg/l, ferritin 15137 mcg/l. A CT scan revealed liver enlargement, ascites, pleural and pericardial effusions. In spite of intensive care, the patient died from multiple organ failure the next morning and a post mortem confirmed shock as cause. It was reported that there was no evidence of infectious diseases, use of illicit drugs or life-threatening diseases. After having ruled out alternative explanations, the concomitant administration of dexamethasone, piroxicam and diclofenac was judged the most probable cause for the fatal organ failure. The authors considered that the event was a serious, acute allergic reaction.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09138470

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 52 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20091006	201001			recovered/resolved
2 Bilirubin increased	20091006	201001			recovered/resolved
3 Icterus					recovered/resolved
4 Transaminases increased	20091006	201001			recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	/	-20091007	Back pain

Drugs (concomitant)

2 KATADOLON flupirtine maleate	/	-	-
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Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20091007	41.2		
Aspartate aminotransferase	20091007	19.8		
Bilirubin	20091007	31.5		
Gamma GT	20091007	7.5		
Laparoscopy				
Laparoscopy	20091007			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 20.10.2009 DE-002147023-PHHY2009DE43662

Sender diagnosis: not reported

Narrative

Initial report received from physician on 08 Oct 2009: The patient was on treatment with Voltaren (diclofenac sodium) 50mg per day for about 3 weeks due to back pain. On 06 Oct 2009, a massive increase of liver values was detected. Voltaren therapy was stopped on 07 Oct 2009. Pre-treatment liver values had been normal, and no explanatory pathology was known from the anamnesis. Liver parameters from 07 Oct 2009 was reported as ALAT 41.2 (units and normal range not provided), ASAT 19.8, gamma-GT 7.5 and Bilirubin 31.5 (units unspecified) but corresponding to ALAT around 5000U/l and bilirubin of around 2.3mg/dl, in standard terms. The outcome of the event was reported to be condition worsened.

Serious spontaneous report, hepatic enzyme increased [medically significant], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 18.06.2010 DE-002147023-PHHY2009DE43662

Sender diagnosis: not reported

Narrative

Initial report received from physician on 08 Oct 2009: The patient was on treatment with Voltaren (diclofenac sodium) 50mg per day for about 3 weeks due to back pain. On 06 Oct 2009, a massive increase of liver values was detected. Voltaren therapy was stopped on 07 Oct 2009. Pre-treatment liver values had been normal, and no explanatory pathology was known from the anamnesis. Liver parameters from 07 Oct 2009 was reported as ALAT 41.2 (units and normal range not provided), ASAT 19.8, gamma-GT 7.5 and Bilirubin 31.5 (units unspecified) but corresponding to ALAT around 5000U/l and bilirubin of around 2.3mg/dl, in standard terms. The outcome of the event was reported to be condition worsened.

Follow up report received from a physician on 07 Jun 2010: This patient received Voltaren for about 8 weeks (not 3 weeks as previously reported) due to lumbalgia. The patient experienced severe icterus and was hospitalized on an unknown date. Laparoscopy was done on an unknown date, liver puncture was undertaken and transplantation was discussed. The patient completely recovered after the treatment with Corticoid. Voltaren has been assessed as definitely related.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09149817

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 21 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased	200909	200910	31 Day		recovered/resolved
2 Icterus	200909	200910	31 Day		recovered/resolved
3 Transaminases increased	200909	200910	31 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN diclofenac sodium	Oral	/ 39	200907-200907	Pain

Drugs (concomitant)

2 Antibiotika	/	-
		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tonsillectomy	200907		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin	200909	20 mg/dl		
Transaminases	200909	700 to 800 U/l		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 10.11.2009 DE-002147023-PHHY2009DE47145

Sender diagnosis: not reported

Narrative

This is a combined report received from the physician wherein the initial report was received on 27 Oct 2009 and follow up on 29 Oct 2009: This patient started treatment with Voltaren dispers (diclofenac free acid) in Jul 2009 for pain after tonsillectomy for 10 days. In Sep 2009, patient had developed massive icterus associated with bilirubin levels of up to 20mg/dl and transaminases of around 700 to 800/UI, without explainable reason. The patient was using stomach protection medications, narcotic drugs and antibiotics. The reporter confirmed that the events occurred after stopping Voltaren Dispers in Jul 2009. The laboratory reports are completely normalised. The reporter was not totally certain with respect to causality but did not see a clear causal relation with Voltaren dispers. The reporter reported that no other causation has been established so far with the events. The outcome of the events was completely recovery in Oct 2009.

Reporter's comment:

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-09150676

Serious No Death No Lifethr. No Hospital. No Disabling No Congenital No Other No Pregnancy Med. conf. Yes

Patient

Initials: Sex: Female Birth: 19521203 Age: 56 Year weight (kg): 70 BMI_int: 24.22 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hip surgery	200909				Unknown
2 Hepatic enzyme increased	20090416			47 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophenac	Oral	/	20090301-20090709	Necrosis
diclofenac sodium		245	4 Month	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Necrosis			Yes	
Abstains from alcohol			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:
Qualification: Other Health Professional observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
unknown	20090724	Max GOT 66 GPT 135 GGT410		
Gamma GT	20090416	47	U/l	
unknown	20090528	GPT 56 GGT 75	U/L	
unknown	20090703	GGT 170 Hepatitis A neg		
Hep A neg Hepatit B Z N Hepatitis B Imn	20090703	nicht vorhanden	MAbsent	
EBV CMV Z n Infektion keिनew akute Inf	20090729	nicht vorhanden	MAbsent	
GOT 42 GPT 75 GGT 396	20090805	nicht vorhanden	MAbsent	
GOT 59 GPT 90 GGT 319 LDH 186	20090824	nicht vorhanden	MAbsent	
GOT 38 GPT 59 GGT 193	20090916	nicht vorhanden	MAbsent	
GOT 49 GPT 120 GGT 486	20091105	nicht vorhanden	MAbsent	
GGT	20091105	486 U/L	U/L	
GGT	20090824	319 U/L	U/L	
GGT	20090805	396 U/L	U/L	
GGT	20090724	410 U/L	U/L	
GPT	20090724	135 U/L	U/L	
GPT	20091105	120 U/L	U/L	
GPT	20090824	90 U/L	U/L	
GPT	20090805	75 U/L	U/L	
SGOT	20090724	56 U/L	U/L	

SGOT	20090824	59 U/L	U/L
SGOT	20091105	49 U/L	U/L
SGOT	20090805	42 U/L	U/L

Narratives (nur bei elektronischem Eingang)

Paul-Ehrlich-Institut, 11.11.2009 DE-PEI-WEBHUMAN_UAW_2009024224

Sender diagnosis: not reported

Narrative

s obiger Text Evtl erneute Gabe von NSAR in der Klinik? weitere Kontrollen laufen Weiter Alkoholkarenz

Sender's comment:

Reporter's comment:

NOVARTIS, 30.11.2009 DE-002147023-PHHY2009DE50794

Sender diagnosis: not reported

Narrative

Initial report received from physician via Health Authority (reference number: DE-BFARM-09150676) on 16 Nov 2009: The patient had concurrent condition of alcohol abstention. The patient was on treatment with diclofenac (manufacturer unknown) for necrosis from 01 Mar 2009. The patient had hepatic enzyme increased including increased GOT (56 U/L), GPT (135 U/L) and GGT (410 U/L) on 24 Jul 2009 and the values were GOT (49 U/L), GPT (120 U/L) and GGT (486 U/L) on 05 Nov 2009. The patient also had hip surgery on Sep 2009. Therapy with Voltaren was discontinued on 09 Jul 2009. The outcome for hepatic enzyme increased was reported to be condition unchanged whereas outcome for hip surgery was not reported. The causality was reported as likely.

- hip surgery [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

-hepatic enzyme increased [medically significant], already listed in the core data sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09168134

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		No

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatosis					Unknown
2 Yellow skin					Unknown
3 Hepatic enzyme increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac diclofenac sodium	Oral	/ 94	- 7 Day	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 09.12.2009 DE-SZ09-GXKR2009DE12990

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 01 Dec 2009 from a consumer (husband of patient): This patient with an unknown medical history developed hepatitis with increased hepatic enzymes and yellow skin while treated with Diclac (diclofenac) for about 1 week. Dosage and exact therapy dates were not stated. No information on concomitant treatment was given. The patient was hospitalized with yellow skin and a hepatitis with increased hepatic enzymes was diagnosed. The outcome of the adverse events was not reported. The reporter provided no causality assessment.

Serious spontaneous report (hospitalisation) of liver disorder, already listed in the Core Data Sheet Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10010926

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19381109	Age: 70 Year	weight (kg): 70	BMI_int: 24.51	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute renal failure	20090803	20090806	4 Day		recovered/resolved
2 Acute renal failure	20090803	20090806	4 Day		recovered/resolved
3 Acute prerenal failure	20090803	20090806	4 Day		recovered/resolved
4 Erythropenia					Unknown
5 Hyperkalaemia					Unknown
6 Thrombopenia					Unknown
7 Gastroenteritis					recovered/resolved
8 Glycosylated haemoglobin increased					Unknown
9 LDH increased					Unknown
10 Nausea					recovered/resolved
11 Platelet count decreased					Unknown
12 Bilirubin conjugated increased					Unknown
13 Diarrhoea					recovered/resolved
14 Vomiting					recovered/resolved
15 Exsiccosis					recovered/resolved
16 Acute renal failure	20090803				recovered/resolved
17 Bicytopenia	20090803				not recovered/not resolved
18 Drug interaction	20090803	20090806	4 Day		recovered/resolved
19 Bicytopenia	20090803				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	BISOPROLOL FUMARATE W/HYDROCHLOR	Oral	1 Df dosage form/Day	-20090803	Hypertension arterial
	bisoprolol; hydrochlorothiazide		82	-	
2	BISOPROLOL FUMARATE/HYDROCHLOROT	Oral	/	-20090803	Hypertension
	bisoprolol fumarate; hydrochlorothiazide			-	
3	diclofenac	Oral	50 Mg milligram(s)/Day	-20090803	Pain
	diclofenac			-	
4	DICLOFENAC	Oral	50 Mg milligram(s)/Day	-20090803	Pain NOS
	diclofenac		245	-	
5	DIGITOXIN	Oral	0.07 Mg milligram(s)/ Day	-	
	digitoxin		245	-	
6	DOXAZOSIN	Oral	4 Mg milligram(s)/Day	-	Hypertension arterial
	doxazosin		212	-	
7	ENALAPRIL	Oral	10 Mg milligram(s)/Day	-20090803	Hypertension arterial
	enalapril		245	-	
8	ENEAS	Oral	1 Df dosage form/Day	-20090803	Hypertension arterial
	enalapril maleate; nitrendipine		245	-	
9	SPIRONOLACTONE	Oral	25 Mg milligram(s)/Day	-20090803	Hypertension arterial
	spironolactone		245	-	
10	TORASEMIDE	Oral	10 Mg milligram(s)/Day	-20090804	Hypertension arterial
	torasemide		245	-	
11	Trevilor Retard	Oral	75 Mg milligram(s)/Day	-20090803	
			208	-	
12	VENLAFAXINE	Oral	75 Mg milligram(s)/Day	-20090803	
	venlafaxine		245	-	

Drugs (concomitant)

13	Acetylsalizylsaeure	Oral	100 Mg milligram(s)/Day	-20090805	Myocardial infarction prophylaxis
	acetylsalicylic acid		245	-	
14	ACTRAPID INSULIN NOVO	Subcutaneous	3* / Day	-	Type 2 diabetes mellitus
	insulin		120	-	
15	LANTUS	Subcutaneous	64 lu international unit(s)/	-	Type 2 diabetes mellitus
	insulin glargine		120	-	
16	METFORMIN	Oral	2000 Mg milligram(s)/Day	-20090802	Type 2 diabetes mellitus
	metformin		82	-	
17	NIASPAN	Oral	1500 Mg milligram(s)/Day	-	Hyperlipoproteinaemia
	nicotinic acid		212	-	
18	XALATAN	Sunconjunctival	1.5 µg microgram(s)/Day	-	Glaucoma
	latanoprost		71	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ex-smoker	1994		Yes	1979-1994 5 Zigaretten/Tag
Glaucoma		2003	No	Beginn unbekannt, Operation beidseits 2003
Renal colic	1963	1963	No	Nierensteine
Type 2 diabetes mellitus	1975		Yes	insulinpflichtig seit 1994
Cholecystitis	1965	1965	No	bei Cholezystolithiasis, Cholecystektomie 1965
Hypertension arterial	1988		Yes	
Renal calculus	1963	1963	No	
Pain NOS			Yes	Beginn unbekannt
Cholecystectomy	1965	1965	No	
Gallbladder stone	1965	1965	No	
Kidney stone	1963	1963	No	
Glaucoma surgery	2003		No	
Hyperlipoproteinaemia			Yes	Beginn unbekannt

Primary source

Qualification: Pharmacist observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20090803	746	µmol/l	
Creatinine	20090804	510	µmol/l	
Creatinine	20090805	219	µmol/l	
Creatinine	20090811	106	µmol/l	
Creatinine	20090815	107	µmol/l	
Creatinine clearance	20090803	0.08	ml/s	
Creatinine clearance	20090804	0.13	ml/s	
Creatinine clearance	20090805	0.91	ml/s	
Creatinine clearance	20090811	0.79	ml/s	
Creatinine clearance	20090815	0.78	ml/s	
Urea	20090804	54.0	mmol/l	
Urea	20090805	30.1	mmol/l	
Urea	20090811	5.76	mmol/l	
Urea	20090815	5.49	mmol/l	
Aspartate aminotransferase	20090803	11.1	U/l	
Aspartate aminotransferase	20090805	16.1	U/l	
Alanine aminotransferase	20090803	16.7	U/l	
Alanine aminotransferase	20090805	18.4	U/l	
Bilirubin total	20090805	12.9	µmol/l	
Bilirubin total	20090807	20.0	µmol/l	
Bilirubin direct	20090807	3.30	µmol/l	
Blood bilirubin indirect	20090807	16.7	µmol/l	
Quick's test	20090803	84	Prozent	
Quick's test	20090805	83	Prozent	
Partial prothrombin time	20090803	28.2	sec	
Partial prothrombin time	20090805	27.2	sec	
INR	20090803	1.10	physiologis	
INR	20090805	1.11	physiologis	
Thrombocyte count	20090803	22	Gpt/l	
Thrombocyte count	20090804	23	Gpt/l	
Thrombocyte count	20090805	16	Gpt/l	
Thrombocyte count	20090811	16	Gpt/l	
Thrombocyte count	20090815	40	Gpt/l	
Thrombocyte count	20090817	32	Gpt/l	
LDH	20090803	181	U/l	
LDH	20090807	361	U/l	
Haemoglobin	20090803	6.4	mmol/l	
Haemoglobin	20090804	5.8	mmol/l	
Haemoglobin	20090805	5.8	mmol/l	
Haemoglobin	20090811	5.1	mmol/l	
Haemoglobin	20090815	6.8	mmol/l	

Haemoglobin	20090817	6.1	mmol/l
Haematocrit	20090803	0.30	SI-Einheit
Haematocrit	20090804	0.27	SI-Einheit
Haematocrit	20090805	0.28	SI-Einheit
Haematocrit	20090811	0.24	SI-Einheit
Haematocrit	20090815	0.33	SI-Einheit
Haematocrit	20090817	0.30	SI-Einheit
Total white blood cell count	20090803	6.88	Gpt/l
Total white blood cell count	20090805	6.41	Gpt/l
Total white blood cell count	20090815	5.48	Gpt/l
Glucose	20090803	7.4	mmol/l
Potassium	20090803	6.3	mmol/l
Potassium	20090804	4.4	mmol/l
Potassium	20090805	4.5	mmol/l
Potassium	20090811	3.9	mmol/l
Sodium	20090803	137	mmol/l
Sodium	20090804	146	mmol/l
Sodium	20090805	145	mmol/l
Sodium	20090811	138	mmol/l
Calcium	20090803	2.30	mmol/l
Calcium	20090804	2.23	mmol/l
Calcium	20090805	2.25	mmol/l
Calcium	20090811	2.22	mmol/l
Red blood cell count	20090803	3.04	Tpt/l
Red blood cell count	20090804	2.77	Tpt/l
Red blood cell count	20090805	2.80	Tpt/l
Red blood cell count	20090811	2.43	Tpt/l
Red blood cell count	20090815	3.46	Tpt/l
Red blood cell count	20090817	3.20	Tpt/l
HbA1C	20090803	6.8	Prozent
HbA1C	20090805	6.2	Prozent
Blood pH	20090803	7.23	pH-Wert
Blood pH	20090804	7.33	pH-Wert
Blood pH	20090805	7.34	pH-Wert
Blood pH	20090806	7.36	pH-Wert
Blood pH	20090808	7.45	pH-Wert
Blood pH	20090810	7.48	pH-Wert
Blood pH	20090812	7.74	pH-Wert
PCO2	20090803	3.2	kPa
PCO2	20090804	4.2	kPa
PCO2	20090805	4.8	kPa
PCO2	20090806	5.5	kPa
PCO2	20090808	4.6	kPa
PCO2	20090810	5.4	kPa

PCO2	20090812	5.6	kPa
Bicarbonate	20090803	12.1	mmol/l
Bicarbonate	20090804	17.6	mmol/l
Bicarbonate	20090805	19.3	mmol/l
Bicarbonate	20090806	22.4	mmol/l
Bicarbonate	20090808	24.6	mmol/l
Bicarbonate	20090810	30.6	mmol/l
Bicarbonate	20090812	29.6	mmol/l
Base excess	20090803	-16.4	mmol/l
Base excess	20090804	-8.2	mmol/l
Base excess	20090805	-5.9	mmol/l
Base excess	20090806	-2.1	mmol/l
Base excess	20090808	0.2	mmol/l
Base excess	20090810	6.7	mmol/l
Base excess	20090812	5.8	mmol/l
PO2	20090803	11.7	kPa
PO2	20090804	7.5	kPa
PO2	20090805	6.28	kPa
PO2	20090806	6.5	kPa
PO2	20090808	9.39	kPa
PO2	20090810	6.78	kPa
PO2	20090812	8.34	kPa
O2 saturation	20090803	97	%
O2 saturation	20090804	87	%
O2 saturation	20090805	84	%
O2 saturation	20090806	85	%
O2 saturation	20090808	96	%
O2 saturation	20090810	99	%
O2 saturation	20090812	93	%
MCV	20090804	98.6	fl
MCV	20090805	100.0	fl
MCV	20090811	99.6	fl
MCV	20090812	93.6	fl
MCV	20090814	95.9	fl
MCV	20090817	95.3	fl
MCH	20090804	2.09	fmol
MCH	20090805	2.07	fmol
MCH	20090811	2.10	fmol
MCH	20090812	1.97	fmol
MCH	20090814	1.97	fmol
MCH	20090817	1.91	fmol
MCHC	20090804	21.2	mmol/l
MCHC	20090805	20.7	mmol/l
MCHC	20090811	21.1	mmol/l

MCHC	20090812	21.0	mmol/l
MCHC	20090814	20.5	mmol/l
MCHC	20090817	20.0	mmol/l
Reticulocyte percentage	20090805	0.61	Prozent
Reticulocyte count	20090805	1.71	10 ¹⁰ /l
Erythroblast count	20090805	0	/nl
ANA	20090811	-	Titer
Proteinase-3 antibody	20090811	-	Ratio
Myeloperoxidase antibody test	20090811	-	Ratio
Haptoglobin	20090807	2.80	g/l
Myoglobin blood	20090805	177	ng/ml
Vitamin B12	20090806	301	pg/ml
Blood folic acid	20090806	10.5	ng/ml
Digitalis level	20090805	19.6	µg/l
Base excess	20090805	-5.9	mmol/l
Base excess	20090810	6.7	mmol/l
Base excess	20090803	-16.4	mmol/l
Bicarbonate	20090803	12.1	mmol/l
Bicarbonate	20090805	19.3	mmol/l
Bicarbonate	20090810	30.6	mmol/l
Blood bilirubin indirect	20090807	16.7	umol/l
Blood pH	20090808	7.45	NA
Blood pH	20090805	7.34	NA
Blood pH	20090803	7.23	NA
Blood pH	20090810	7.48	NA
Creatinine	20090803	746	umol/l
Creatinine	20090815	107	umol/l
Creatinine	20090805	219	umol/l
Creatinine clearance	20090805	0.91 ml/sec	NA
Creatinine clearance	20090815	0.78 ml/sec	NA
Creatinine clearance	20090803	0.08 ml/sec	NA
Haematocrit	20090803	0.30	NA
Haematocrit	20090815	0.33	NA
Haematocrit	20090805	0.28	NA
Haemoglobin	20090805	5.8	mmol/l
Haemoglobin	20090815	6.8	mmol/l
Haemoglobin	20090803	6.4	mmol/l
HbA1C	20090803	6.8	NA
HbA1C	20090805	6.2	NA
LDH	20090807	361 U/L	NA
MCH	20090804	2.09 fmol	NA
MCH	20090812	1.97 fmol	NA
MCH	20090805	2.07 fmol	NA
MCV	20090805	100 fl	NA

MCV	20090812	93.6 fl	NA
MCV	20090814	95.9 fl	NA
Myoglobin blood	20090805	177 ng/ml	NA
O2 saturation	20090803	97	%
O2 saturation	20090804	87	%
O2 saturation	20090805	84	%
O2 saturation	20090810	99	%
PCO2	20090803	3.2 kPa	NA
PCO2	20090805	4.8 kPa	NA
PO2	20090810	6.78 kPa	NA
PO2	20090805	6.28 kPa	NA
PO2	20090804	7.5 kPa	NA
Potassium	20090803	6.3	mmol/l
Potassium	20090805	4.5	mmol/l
Red blood cell count	20090803	3.04 Tpt/l	NA
Red blood cell count	20090805	2.80 Tpt/l	NA
Red blood cell count	20090815	3.46 Tpt/l	NA
Thrombocyte count	20090815	40 Gpt/l	NA
Thrombocyte count	20090805	16 Gpt/l	NA
Thrombocyte count	20090803	22 Gpt/l	NA
Urea	20090804	54	mmol/l
Urea	20090805	30.1	mmol/l
Urea	20090815	5.49	mmol/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 28.01.2010 DE-PVZIBE-02-10214

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
akutes prärenales Nierenversagen

KURZZUSAMMENFASSUNG:

akutes prärenales Nierenversagen unter der Einnahme von Eneas, Enalapril mono, Torasemid, Bisoprolol HCT comp. und Spironolacton bei zusätzlichem Flüssigkeitsverlust infolge Diarrhoe und Erbrechen

BESCHREIBUNG:

seit 15.07.2009 Diarrhoe, Übelkeit und Erbrechen;

bei Einweisung: Exsikkose, Haut und sichtbare Schleimhäute blass und trocken, Blutdruck 125/50mmHg;

seit 15.07.2009 wegen der Übelkeit und dem Erbrechen kaum noch gegessen und sehr wenig getrunken

THERAPIE:

intravenöser Volumenersatz;

intravenöse Gabe von Natriumhydrogencarbonat;

11.08.2009 Transfusion von 2 Erythrozytenkonzentraten und 1 Thrombozytenkonzentrat

VERLAUF:

11.08.2009 Knochenmarkaspiration und Biopsie: fast vollständiger Verlust der Megakaryozyten, verminderte Erythropoese;

11.08.2009: keine Antikörper der heparininduzierten Thrombozytopenie nachgewiesen (ELISA), keine sicheren Hinweise auf eine heparininduzierte Thrombozytopenie, keine Hinweise auf eine Autoimmunthrombozytopenie, keine thrombozytäre Alloimmunisierung;

zügige Besserung des Allgemeinzustandes nach Volumenersatz und Gabe von Natriumhydrogencarbonat, Rückgang der Hyperkaliämie und der Nierenretentionsparameter

nach Entlassung ist die Vorstellung der Patientin in der onkologischen Tagesklinik geplant, um die Diagnostik bezüglich des Knochenmarks voranzutreiben

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 03.-17.08.2009;

Krankenhauseinweisung aufgrund eines akuten prärenal Nierenversagens, dass sich mit einem reduzierten Allgemeinzustand, Übelkeit, Erbrechen,

Exsikkose und Hyperkaliämie darstellte, unter der Einnahme von Eneas, Enalapril mono, Torasemid, Bisoprolol HCT comp. und Spironolacton; Besserung der Symptomatik trat nach Absetzen der Medikamente ein sowie Volumenersatz ein. Beigetragen zu der akuten Problematik hat sicherlich der eine Gastroenteritis (Diarrhoe, Übelkeit, Erbrechen) im Vorfeld der akuten Problematik.

ANMERKUNGEN ZU FOLGENDER UAW:

Bizytopenie (Erythropenie und Thrombopenie)

KURZZUSAMMENFASSUNG:

Erythropenie und Thrombopenie unter der Einnahme von Eneas, Enalapril mono, Torasemid, Bisoprolol HCT comp., Doxazosin, Venlafaxin und Diclofenac

BESCHREIBUNG:

seit 15.07.2009 Diarrhoe, Übelkeit und Erbrechen;

bei Einweisung: Exsikkose, Haut und sichtbare Schleimhäute blass und trocken, Blutdruck 125/50mmHg;

seit 15.07.2009 wegen der Übelkeit und dem Erbrechen kaum noch gegessen und sehr wenig getrunken

THERAPIE:

intravenöser Volumenersatz;

intravenöse Gabe von Natriumhydrogencarbonat;

11.08.2009 Transfusion von 2 Erythrozytenkonzentraten und 1 Thrombozytenkonzentrat

VERLAUF:

11.08.2009 Knochenmarkaspiration und Biopsie: fast vollständiger Verlust der Megakaryozyten, verminderte Erythropoese;

11.08.2009: keine Antikörper der heparininduzierten Thrombozytopenie nachgewiesen (ELISA), keine sicheren Hinweise auf eine heparininduzierte Thrombozytopenie, keine Hinweise auf eine Autoimmunthrombozytopenie, keine thrombozytäre Alloimmunisierung;

zügige Besserung des Allgemeinzustandes nach Volumenersatz und Gabe von Natriumhydrogencarbonat, Rückgang der Hyperkaliämie und der Nierenretentionsparameter

Aufgrund der Bizytopenie erfolgte eine Verlängerung des Krankenhausaufenthaltes

nach Entlassung ist die Vorstellung der Patientin in der onkologischen Tagesklinik geplant, um die Diagnostik bezüglich des Knochenmarks voranzutreiben

ERLÄUTERUNGEN ZUR UAW:

begleitende Erythropenie und Thrombopenie; in der Knochenmarkbiopsie fast vollständiger Verlust der Megakaryozyten und verminderte Erythropoese. Keine Hinweise für immunologisch induzierte Ursache der Thrombopenie. Bleibt auch der V.a. eine Thrombopenie und Erythropenie

unter der Einnahme von Eneas, Enalapril mono, Torasemid, Bisoprolol HCT comp., Doxazosin, Venlafaxin und Diclofenac

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ENEAS

BETREFFENDE UAW: akutes Nierenversagen

ER<dbtext>To_Char_NARRATIVE_2</dbtext>

009507513, 04.02.2010 DE-MERCK-1002DEU00002

Sender diagnosis: not reported

Narrative

Information has been received from a pharmacist and a regional pharmacovigilance center via the German agency concerning a 70-year-old female patient (ex-smoker) with pain, type 2 diabetes mellitus and hyperlipoproteinemia and a history of glaucoma, kidney stones (in 1963), renal colic (in 1963), gallbladder stones, cholecystitis, cholecystectomy and bilateral glaucoma surgery who was placed on therapy with enalapril maleate (manufacturer unknown), tablet, 10 mg, once a day for the treatment of arterial hypertension (duration not reported). Concomitant suspect therapy included enalapril maleate (+) nitrendipine (ENEAS), tablet, 10-20 mg, once a day for the treatment of arterial hypertension (duration not reported), torsemide, tablet, 10 mg, once a day for the treatment of arterial hypertension (duration not reported), bisoprolol fumarate/HCTZ, tablet, 5-12.5 mg, once a day for the treatment of arterial hypertension (duration not reported), spironolactone, tablet, 25 mg, once a day for the treatment of arterial hypertension (duration not reported), doxazosin mesylate, tablet, 4 mg, once a day for the treatment of arterial hypertension (duration not reported), venlafaxine HCl, tablet, 37.5 mg, twice a day (duration and indication not reported) and diclofenac, tablet, 50 mg, once a day for the treatment of pain (duration not reported). Other concomitant therapy included digitoxin, metformin, latanoprost (XALATAN), insulin, neutral (ACTRAPID), insulin glargine (LANTUS), aspirin and niacin (NIASPAN).

On 15-JUL-2009, the patient experienced nausea and developed vomiting and diarrhea as symptoms of gastroenteritis, due to which she hardly ate anything and drank very little.

On 03-AUG-2009, the patient was hospitalized due to acute prerenal failure with poor general condition, nausea, vomiting, exsiccosis and hyperkalemia. Laboratory analysis revealed serum creatinine 746 µmol/l, creatinine clearance 0.08 ml/s, serum potassium 6.3 mmol/l, platelet count 22 Gpt/l, RBC count 3.04 Tpt/l and hemoglobin 6.4 mmol/l. Diagnoses of acute prerenal failure and bicytopenia (erythropenia and thrombopenia) were made. The patient was treated with intravenous volume substitution and sodium hydrogen carbonate. Therapy with enalapril maleate (last dose taken on 02-AUG-2009), enalapril maleate (+) nitrendipine (last dose taken on 02-AUG-2009), bisoprolol fumarate/HCTZ (last dose taken on 02-AUG-2009), spironolactone (last dose taken on 02-AUG-2009), diclofenac (last dose taken on 02-AUG-2009) and venlafaxine HCl was discontinued. Therapy with doxazosine mesylate continued. In the further course, the patient's hyperkalemia and renal retention parameters were decreasing.

On 04-AUG-2009, laboratory analysis revealed serum creatinine 510 µmol/l, creatinine clearance 0.13 ml/s, serum blood urea 54.0 mmol/l, serum potassium 4.4 mmol/l, platelet count 23 Gpt/l, RBC count 2.77 Tpt/l and hemoglobin 5.8 mmol/l. That same day, therapy with torsemide was discontinued. On 05-AUG-2009, laboratory analysis revealed serum creatinine 219 µmol/l, creatinine clearance 0.91 ml/s, serum blood urea 30.1 mmol/l, platelet count 16 Gpt/l, RBC count 2.80 Tpt/l, hemoglobin 5.8 mmol/l, reticulocyte count 0.61% and absolute reticulocyte count 1.71 10^9 /l. On 06-AUG-2009, the patient was considered as recovered from acute prerenal failure.

On 11-AUG-2009, laboratory analysis revealed serum creatinine 106 µmol/l, creatinine clearance 0.79 ml/s, serum blood urea 5.76 mmol/l, platelet count 16 Gpt/l, RBC count 2.43 Tpt/l, hemoglobin 5.1 mmol/l, ANA negative, proteinase 3 antibody negative and myeloperoxidase antibody negative. Bone marrow biopsy showed almost complete loss of megacaryocytes, decreased erythropoiesis. There was no detection of antibodies of heparin-induced thrombocytopenia (ELISA), no certain signs of heparin-induced thrombocytopenia, no evidence of autoimmune thrombocytopenia, no thrombocytic alloimmunization. That same day, the patient received two RBC concentrates and one platelet concentrate. On 15-AUG-2009, laboratory analysis revealed serum creatinine 107 µmol/l, creatinine clearance 0.78 ml/s, serum blood urea 5.49 mmol/l, platelet count 40 Gpt/l, RBC count 3.46 Tpt/l and hemoglobin 6.8 mmol/l. On 17-AUG-2009, laboratory analysis revealed platelet count 32 Gpt/l, RBC count 3.20 Tpt/l and hemoglobin 6.1 mmol/l. The patient's bicytopenia (erythropenia and thrombopenia) persisted. On 17-AUG-2009, the patient was discharged from hospital. Further outpatient diagnostic work-up concerning bone marrow was planned.

The regional pharmacovigilance center felt that acute prerenal failure was possibly related to therapy with enalapril maleate, enalapril maleate (+) nitrendipine, torsemide, bisoprolol fumarate/HCTZ and spironolactone and probably not related to therapy with doxazosin mesylate, venlafaxine HCl and diclofenac, and that bicytopenia (erythropenia and thrombopenia) was related to therapy with enalapril maleate, enalapril maleate (+) nitrendipine, torsemide, bisoprolol fumarate/HCTZ, doxazosin mesylate, venlafaxine HCl and diclofenac and probably not related to therapy with spironolactone. As alternative cause and risk factor the additional fluid loss due to diarrhea and vomiting as well as decreased fluid intake were taken into consideration. Acute prerenal failure and bicytopenia (erythropenia and thrombopenia) were the only events coded by the agency.

Acute prerenal failure and bicytopenia (erythropenia and thrombopenia) were considered to be immediately life-threatening by the pharmacist/regional pharmacovigilance center/agency.

Additional information is not expected.

NOVARTIS, 04.02.2010 DE-002147023-PHHY2010DE05207

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE-BFARM-10010926) on 29 Jan 2010: The patient had medical history of glaucoma, ex-smoker, renal colic, type 2 diabetes mellitus, cholecystitis, hypertension arterial and hyperlipoproteinemia. She was on treatment with diclofenac (manufacturer unknown) 50 mg once a day for pain from unknown date to 02 Aug 2009. She was also on treatment with co-suspect medication like Eneas (enalapril maleate), enalapril 10 mg once a day, torasemide 10 mg once a day, bisoprolol with hydrochlorothiazide 1 dosage form once a day, spironolactone 25 mg once a day, doxazosin 4 mg once a day for hypertension arterial, and venlafaxine 37.5 mg two times a day for unknown indication. The therapy with all suspect drugs were discontinued except for doxazosin which the action taken was not reported. On 03 Aug 2009 the patient developed acute renal failure, nausea, vomiting, diarrhea, exsiccosis and gastroenteritis due to which she was hospitalized on the same date. The patient also had bicytopenia the outcome of which was reported to be condition unchanged. She was treated by intravenous volume substitution and sodium hydrogencarbonate intravenous. The outcome of the event was reported to be complete recovery. The patient also had increased conjugated bilirubin (16.7 µmol/l), LDH (361 U/L) and glycosylated haemoglobin (6.8) and decreased platelet count (16 Gpt/l), thrombopenia, erythropenia the outcome of which were not reported. Acute renal failure [life-threatening], and thrombocytopenia [medically significant] assessed as listed according to the Core Data Sheet.

All remaining reported leading events and associated symptoms were assessed as non-serious by Novartis Medical Safety Physician.

In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between events and diclofenac as suspected based on the current available information.

Reporter's comment:

sanofi-aventis, 05.02.2010 DE-SANOFI-AVENTIS-2010SA006043

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-02-10214) and was extracted from the website of the German

health authority (authority number: DE-BFARM-10010926). The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves a 70-year-old female ex-smoker patient with history of renal calculus with colic in 1963, cholelithiasis with cholecystitis treated with cholecystectomy in 1965 and bilateral glaucoma treated with surgery NOS in 2003. At the time of event onset, the patient was suffering from type 2 diabetes mellitus since 1975 (insulin-dependent since 1994), arterial hypertension since 1988, hyperlipoproteinaemia and pain NOS.

On an unspecified time point, the patient started treatments with enalapril maleate 10 mg/day, nitrendipine+enalapril maleate 10+20 mg/day, torasemide 10 mg/day, bisoprolol+hydrochlorothiazide 5+12.5 mg/day, spironolactone 25 mg/day, doxazosin 4 mg/day and venlafaxine hydrochloride 2x37.5 mg/day for arterial hypertension and diclofenac sodium 50 mg/day for pain NOS. Trade names of enalapril, bisoprolol+hydrochlorothiazide, spironolactone and diclofenac were not reported. Concomitant medications included digitoxin, metformin hydrochloride, latanoprost, insulin human, insulin glargine, acetylsalicylic acid and nicotinic acid.

From 03-Aug-2009 to 17-Aug-2009 the patient was hospitalized due to acute prerenal failure. She had been suffering from diarrhoea, nausea and vomiting since 15-Jul-2009. On admission to the hospital, the patient had exsiccosis and her skin and visible mucosa were pale and dry. Blood pressure was 125/50mmHg. She had eaten and drunk very few due to nausea and vomiting since 15-Jul-2009.

Bone marrow aspiration and biopsy on 11-Aug-2009 revealed almost complete loss of megakaryocytes and decreased erythropoiesis. Enzyme-linked immunosorbent assay (ELISA) on 11-Aug-2009 found no evidence of antibody concerning heparin-induced thrombocytopenia. There was no sure indication of heparin induced- thrombocytopenia, no indication of autoimmune thrombocytopenia and no platelet alloimmunisation. Corrective treatments included intravenous volume replacement and intravenous administration of sodium hydrogen carbonate. On 11-Aug-2009, transfusion of 2 erythrocyte concentrates and 1 platelet concentrate were performed.

Enalapril, nitrendipine+enalapril, bisoprolol+hydrochlorothiazide, spironolactone, metformin were last taken on 02-Aug-2009. Venlafaxine, torasemide and acetylsalicylic acid were discontinued after the event onset. Symptomatic improvement was observed after discontinuation of these drugs and volume replacement and administration of sodium hydrogen carbonate. Hyperkalaemia resolved and renal retention parameter improved. The PV centre stated that gastroenteritis (diarrhoea, nausea, and vomiting) surely contributed to the acute symptoms.

The PV centre reported acute prerenal failure and bicytopenia (erythropenia and thrombopenia) as adverse drug reactions with start date of 03-Aug-2009. Acute prerenal failure resolved on 06-Aug-2009. Bicytopenia was still ongoing at the time of the report. Enalapril, nitrendipine+enalapril, torasemide, bisoprolol+hydrochlorothiazide, spironolactone, doxazosin, venlafaxine and diclofenac were suspected for the events. The case was considered to be life-threatening. Concerning bicytopenia further oncological investigation was planned.

Causality assessment by PV centre concerning:

acute prerenal failure:

"possible": enalapril, nitrendipine+enalapril maleate, torasemide, bisoprolol+hydrochlorothiazide, spironolactone

"unlikely": doxazosin, venlafaxine, diclofenac

Bicytopenia:

"possible": enalapril, nitrendipine+enalapril maleate, torasemide, bisoprolol+hydrochlorothiazide, doxazosin, diclofenac, venlafaxine

"unlikely": spironolactone

Explanatory notes on the causality:

Acute renal failure:

Nitrendipine+enalapril maleate: renal dysfunction incl. acute renal failure is known ADR under enalapril. Fluid loss due to diarrhoea and vomiting as well as decreased fluid intake should be considered as alternative cause and risk factor.

Enalapril: Renal dysfunction may occur under enalapril, which could lead to acute renal failure. Fluid loss due to diarrhoea and vomiting as well as decreased fluid intake should be considered as alternative cause and risk factor.

Bisoprolol+hydrochlorothiazide: in combination with non-steroid antirheumatic agents and under hypovolaemia, the risk of prerenal failure increases.

Sironolactone, torasemide: the diuretic effect of torasemide and poor fluid intake could cause prerenal failure. Fluid loss due to diarrhoea and vomiting as well as decreased fluid intake should be considered as alternative cause and risk factor.

Bicytopenia:

Enalapril, nitrendipine+enalapril maleate: especially the patient with impaired renal function may develop cytopenia under the medication.

Bisoprolol+hydrochlorothiazide: cytopenia is listed as a seldom ADR.

Diclofenac, doxazosin, venlafaxine: cytopenia is listed as an ADR.

Torasemide: cytopenia may occur under torasemide.

HELIOS Klinikum Wuppertal, 08.04.2010 DE-PVZIBE-02-10214

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

akutes prärenales Nierenversagen

KURZZUSAMMENFASSUNG:

akutes prärenales Nierenversagen unter der Einnahme von Eneas, Enalapril mono, Torasemid, Bisoprolol HCT comp. und Spironolacton bei zusätzlichem Flüssigkeitsverlust infolge Diarrhoe und Erbrechen

BESCHREIBUNG:

seit 15.07.2009 Diarrhoe, Übelkeit und Erbrechen;

bei Einweisung: Exsikkose, Haut und sichtbare Schleimhäute blass und trocken, Blutdruck 125/50mmHg;

seit 15.07.2009 wegen der Übelkeit und dem Erbrechen kaum noch gegessen und sehr wenig getrunken

THERAPIE:

intravenöser Volumenersatz;

abgesetzt am 3.08.09: Enalapril, Eneas, Bisoprolol/HCT, Spironolacton, Diclofenac, Venlafaxin

abgesetzt am 4.08.09: Torasemid

intravenöse Gabe von Natriumhydrogencarbonat;

11.08.2009 Transfusion von 2 Erythrozytenkonzentraten und 1 Thrombozytenkonzentrat

VERLAUF:

unter Flüssigkeitssubstitution Besserung der Nierenfunktion

keine Übelkeit, Erbrechen, Diarrhoe ab 2. stationären Tag

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 03.-17.08.2009;

Krankensein aufgrund eines akuten prärenal Nierenversagens, dass sich mit einem reduzierten Allgemeinzustand, Übelkeit, Erbrechen, Exsikkose und Hyperkaliämie darstellte. Die Verschlechterung der Nierenfunktion trat vermutlich ein durch den Gastroenteritis-bedingten Volumenverlust, der durch die Weitereinnahme der Diuretika (Torasemid, Spironolacton sowie HCT) verschlechtert wurde. Darüberhinaus wird die Nierendurchblutung durch die Antihypertensiv wirkenden Nitrendipin, Enalapril und Bisoprolol weiter verschlechtert.

Besserung der Symptomatik trat nach Absetzen der Medikamente ein sowie Volumenersatz ein. Beigetragen zu der akuten Problematik hat sicherlich eine Gastroenteritis (Diarrhoe, Übelkeit, Erbrechen) im Vorfeld der akuten Problematik. Die akkumulierenden Urämetoxine können die Symptome Übelkeit und Erbrechen verstärkt haben und somit den weiteren Krankheitsprozess gegenseitig in einem Teufelskreislauf weiter unterstützt haben.

ANMERKUNGEN ZU FOLGENDER UAW:

Bizytopenie (Erythropenie und Thrombopenie)

KURZZUSAMMENFASSUNG:

Erythropenie und Thrombopenie unter der Einnahme von Eneas, Enalapril mono, Torasemid, Bisoprolol HCT comp., Doxazosin, Venlafaxin und Diclofenac

BESCHREIBUNG:

seit 15.07.2009 Diarrhoe, Übelkeit und Erbrechen;

bei Einweisung: Exsikkose, Haut und sichtbare Schleimhäute blass und trocken, Blutdruck 125/50mmHg;

seit 15.07.2009 wegen der Übelkeit und dem Erbrechen kaum noch gegessen und sehr wenig getrunken

THERAPIE:

abgesetzt am 3.08.09: Enalapril, Eneas, Bisoprolol/HCT, Spironolacton, Diclofenac, Venlafaxin

abgesetzt am 4.08.09: Torasemid

11.08.2009 Transfusion von 2 Erythrozytenkonzentraten und 1 Thrombozytenkonzentrat

VERLAUF:

11.08.2009 Knochenmarkaspiration und Biopsie: fast vollständiger Verlust der Megakaryozyten, verminderte Erythropoese;

11.08.2009: keine Antikörper der heparininduzierten Thrombozytopenie nachgewiesen (ELISA), keine sicheren Hinweise auf eine heparininduzierte Thrombozytopenie, keine Hinweise auf eine Autoimmunthrombozytopenie, keine thrombozytäre Alloimmunisierung;

zügige Besserung des Allgemeinzustandes nach Volumenersatz und Gabe von Natriumhydrogencarbonat, Rückgang der Hyperkaliämie und der Nierenretentionsparameter

Aufgrund der Bizytopenie erfolgte eine Verlängerung des Krankenhausaufenthaltes

nach Entlassung ist die Vorstellung der Patientin in der onkologischen Tagesklinik geplant, um die Diagnostik bezüglich des Knochenmarks voranzutreiben

ERLÄUTERUNGEN ZUR UAW:

begleitende Erythropenie und Thrombopenie; in der Knochenmarkbiopsie fast vollständiger Verlust der Megakaryozyten und verminderte Erythropoese. Keine Hinweise für immunologisch induzierte Ursache der Thrombopenie. Bleibt auch der V.a. eine Thrombopenie und Erythropenie

unter der Einnahme von Eneas, Enalapril mono, Torasemid, Bisoprolol HCT comp., Doxazosin, Venlafaxin und Diclofenac

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN To_Char_NARRATIVE_2

009507513, 14.04.2010 DE-MERCK-1002DEU00002

Sender diagnosis: not reported

Narrative

Information has been received from a pharmacist and a regional pharmacovigilance center via the German agency concerning a 70-year-old female patient (ex-smoker) with pain, type 2 diabetes mellitus and hyperlipoproteinemia and a history of glaucoma, kidney stones (in 1963), renal colic (in 1963), gallbladder stones, cholecystitis, cholecystectomy and bilateral glaucoma surgery who was placed on therapy with enalapril maleate (manufacturer unknown), tablet, 10 mg, once a day for the treatment of arterial hypertension (duration not reported). Concomitant suspect therapy included enalapril maleate (+) nitrendipine (ENEAS), tablet, 10-20 mg, once a day for the treatment of arterial hypertension (duration not reported), torsemide, tablet, 10 mg, once a day for the treatment of arterial hypertension (duration not reported), bisoprolol fumarate/HCTZ, tablet, 5-12.5 mg, once a day for the treatment of arterial hypertension (duration not reported), spironolactone, tablet, 25 mg, once a day for the treatment of arterial hypertension (duration not reported), doxazosin mesylate, tablet, 4 mg, once a day for the treatment of arterial hypertension (duration not reported), venlafaxine HCl, tablet, 37.5 mg, twice a day (duration and indication not reported) and diclofenac, tablet, 50 mg, once a day for the treatment of pain (duration not reported). Other concomitant therapy included digitoxin, metformin, latanoprost (XALATAN), insulin, neutral (ACTRAPID), insulin glargine (LANTUS), aspirin and niacin (NIASPAN).

On 15-JUL-2009, the patient experienced nausea and developed vomiting and diarrhea as symptoms of gastroenteritis, due to which she hardly ate

anything and drank very little.

On 03-AUG-2009, the patient was hospitalized due to acute prerenal failure with poor general condition, nausea, vomiting, exsiccosis and hyperkalemia. Laboratory analysis revealed serum creatinine 746 µmol/l, creatinine clearance 0.08 ml/s, serum potassium 6.3 mmol/l, platelet count 22 Gpt/l, RBC count 3.04 Tpt/l and hemoglobin 6.4 mmol/l. Diagnoses of acute prerenal failure and bicytopenia (erythropenia and thrombopenia) were made. The patient was treated with intravenous volume substitution and sodium hydrogen carbonate. Therapy with enalapril maleate (last dose taken on 02-AUG-2009), enalapril maleate (+) nitrendipine (last dose taken on 02-AUG-2009), bisoprolol fumarate/HCTZ (last dose taken on 02-AUG-2009), spironolactone (last dose taken on 02-AUG-2009), diclofenac (last dose taken on 02-AUG-2009) and venlafaxine HCl (last dose taken on 02-AUG-2009) was discontinued. Therapy with doxazosin mesylate continued. In the further course, the patient's hyperkalemia and renal retention parameters were decreasing.

On 04-AUG-2009, laboratory analysis revealed serum creatinine 510 µmol/l, creatinine clearance 0.13 ml/s, serum blood urea 54.0 mmol/l, serum potassium 4.4 mmol/l, platelet count 23 Gpt/l, RBC count 2.77 Tpt/l and hemoglobin 5.8 mmol/l. That same day, therapy with torsemide was discontinued. On 05-AUG-2009, laboratory analysis revealed serum creatinine 219 µmol/l, creatinine clearance 0.91 ml/s, serum blood urea 30.1 mmol/l, platelet count 16 Gpt/l, RBC count 2.80 Tpt/l, hemoglobin 5.8 mmol/l, reticulocyte count 0.61% and absolute reticulocyte count 1.71 10^9 /l.

Under fluid substitution, the patient's renal function improved and as of 04-AUG-2010, the patient recovered from nausea, vomiting and diarrhea.

On 05-AUG-2009, serum digitalis level was 19.6 mcg/l.

On 06-AUG-2009, the patient was considered as recovered from acute prerenal failure.

On 11-AUG-2009, laboratory analysis revealed serum creatinine 106 µmol/l, creatinine clearance 0.79 ml/s, serum blood urea 5.76 mmol/l, platelet count 16 Gpt/l, RBC count 2.43 Tpt/l, hemoglobin 5.1 mmol/l, ANA negative, proteinase 3 antibody negative and myeloperoxidase antibody negative. Bone marrow biopsy showed almost complete loss of megacaryocytes, decreased erythropoiesis. There was no detection of antibodies of heparin-induced thrombocytopenia (ELISA), no certain signs of heparin-induced thrombocytopenia, no evidence of autoimmune thrombocytopenia, no thrombocytic alloimmunization. That same day, the patient received two RBC concentrates and one platelet concentrate. On 15-AUG-2009, laboratory analysis revealed serum creatinine 107 µmol/l, creatinine clearance 0.78 ml/s, serum blood urea 5.49 mmol/l, platelet count 40 Gpt/l, RBC count 3.46 Tpt/l and hemoglobin 6.8 mmol/l. On 17-AUG-2009, laboratory analysis revealed platelet count 32 Gpt/l, RBC count 3.20 Tpt/l and hemoglobin 6.1 mmol/l. The patient's bicytopenia (erythropenia and thrombopenia) persisted. On 17-AUG-2009, the patient was discharged from hospital. Further outpatient diagnostic work-up concerning bone marrow was planned.

The regional pharmacovigilance center felt that acute prerenal failure was possibly related to therapy with enalapril maleate, torsemide, bisoprolol fumarate/HCTZ, spironolactone, enalapril maleate (+) nitrendipine and doxazosin mesylate and probably not related to therapy with venlafaxine HCl and diclofenac, and that bicytopenia (erythropenia and thrombopenia) was possibly related to therapy with enalapril maleate, enalapril maleate (+) nitrendipine, torsemide, bisoprolol fumarate/HCTZ, venlafaxine HCl, diclofenac and doxazosin mesylate. As alternative cause and risk factor for impaired renal function were taken into consideration gastroenteritis-induced fluid loss, and the intake of diuretics and several antihypertensive medication. Additionally, accumulation of uremic toxins was mentioned as another possible cause for the above mentioned renal impairment. The following comment was made regarding digitoxin: "In the context of increased digitalis efficacy nausea and vomiting can occur. Upon admission and a consequence of a renal failure an increased serum potassium value was present, leading to an increased digitalis efficacy. The digitoxin level was in middle reference range. It is unlikely that digitoxin caused the nausea and vomiting, which finally co-induced the prerenal renal failure. In the further course it cannot be ruled out that due to the hyperkalaemia, caused by renal failure, an increased digitalis efficacy with nausea and vomiting was present, which might have led to further worsening of the condition". The following comment was made regarding doxazosin mesylate: "The antihypertensive effect interacting with the extrarenal fluid loss, further medication (not specified) and the decreased fluid intake could have led to decreased renal perfusion. However, it is an additive effect."

Acute prerenal failure and bicytopenia (erythropenia and thrombopenia) were the only events coded by the agency.

Acute prerenal failure and bicytopenia (erythropenia and thrombopenia) were considered to be immediately life-threatening by the pharmacist/regional pharmacovigilance center/agency.

Additional information is not expected.

NOVARTIS, 15.04.2010 DE-002147023-PHHY2010DE05207

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE-BFARM-10010926) on 29 Jan 2010: The patient had medical history of glaucoma, ex-smoker, renal colic, type 2 diabetes mellitus, cholecystitis, hypertension arterial and hyperlipoproteinemia. She was on treatment with diclofenac (manufacturer unknown) 50 mg once a day for pain from unknown date to 02 Aug 2009. She was also on treatment with co-suspect medication like Eneas (enalapril maleate), enalapril 10 mg once a day, torasemide 10 mg once a day, bisoprolol with hydrochlorothiazide 1 dosage form once a day, spironolactone 25 mg once a day, doxazosin 4 mg once a day for hypertension arterial, and venlafaxine 37.5 mg two times a day for unknown indication. The therapy with all suspect drugs were discontinued except for doxazosin which the action taken was not reported. On 03 Aug 2009 the patient developed acute renal failure, nausea, vomiting, diarrhea, exsiccosis and gastroenteritis due to which she was hospitalized on the same date. The patient also had bicytopenia the outcome of which was reported to be condition unchanged. She was treated by intravenous volume substitution and sodium hydrogencarbonate intravenous. The outcome of the event

was reported to be complete recovery. The patient also had increased conjugated bilirubin (16.7 umol/l), LDH (361 U/L) and glycosylated haemoglobin (6.8) and decreased platelet count (16 Gpt/l), thrombopenia, erythropenia the outcome of which were not reported.

Follow-up report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority on 09 Apr 2010: This patient experienced diarrhoea on an unknown date. The therapy with Enalapril, Eneas, Bisoprolol/HCT, Spirolnolactone, Diclofenac, Venlafaxin was discontinued on 03 Aug 2009. The therapy with Torasamide was discontinued on 04 Aug 2009. The patient did not experienced nausea, vomiting, and diarrhoea since 04 Aug 2009. The outcome of the event diarrhoea was reported as complete recovery. The worsening of renal function was possibly caused by volume loss (caused by gastroenteritis) which was further worsened by the ongoing therapy with diuretics. The renal perfusion was decreased by antihypertensive acting drugs nitrendipin, enalapril and bisoprolol. The accumulating uremic toxins might have increased the symptoms nausea and vomiting. In the absence of the reporting healthcare professional causality, the Novartis Medical Safety Physician has provisionally assessed the event as suspected based on the current available information.

sanofi-aventis, 15.04.2010 DE-SANOFI-AVENTIS-2010SA006043

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-02-10214) and was extracted from the website of the German health authority (authority number: DE-BFARM-10010926). The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves a 70-year-old female ex-smoker patient with history of renal calculus with colic in 1963, cholezystolithiasis with cholecystitis treated with cholecystectomy in 1965 and bilateral glaucoma treated with surgery NOS in 2003. At the time of event onset, the patient was suffering from type 2 diabetes mellitus since 1975 (insulin-dependent since 1994), arterial hypertension since 1988, hyperlipoproteinaemia and pain NOS.

On an unspecified time point, the patient started treatments with enalapril maleate 10 mg/day, nitrendipine+enalapril maleate 10+20 mg/day, torasemide 10 mg/day, bisoprolol+hydrochlorothiazide 5+12.5 mg/day, spironolactone 25 mg/day, doxazosin 4 mg/day and venlafaxine hydrochloride 2x37.5 mg/day for arterial hypertension and diclofenac sodium 50 mg/day for pain NOS. Trade names of enalapril, bisoprolol+hydrochlorothiazide, spironolactone and diclofenac were not reported. Concomitant medications included digitoxin, metformin hydrochloride, latanoprost, insulin human, insulin glargine, acetylsalicylic acid and nicotinic acid.

From 03-Aug-2009 to 17-Aug-2009 the patient was hospitalized due to acute prerenal failure. She had been suffering from diarrhoea, nausea and vomiting since 15-Jul-2009. On admission to the hospital, the patient had exsiccosis and her skin and visible mucosa were pale and dry. Blood pressure was 125/50mmHg. She had eaten and drunk very few due to nausea and vomiting since 15-Jul-2009.

Bone marrow aspiration and biopsy on 11-Aug-2009 revealed almost complete loss of megakaryocytes and decreased erythropoiesis. Enzyme-linked immunosorbent assay (ELISA) on 11-Aug-2009 found no evidence of antibody concerning heparin-induced thrombocytopenia. There was no sure indication of heparin induced- thrombocytopenia, no indication of autoimmune thrombocytopenia and no platelet alloimmunisation. Corrective treatments included intravenous volume replacement and intravenous administration of sodium hydrogen carbonate. On 11-Aug-2009, transfusion of 2 erythrocyte concentrates and 1 platelet concentrate were performed.

Enalapril, nitrendipine+enalapril, bisoprolol+hydrochlorothiazide, spironolactone, metformin were last taken on 02-Aug-2009. Venlafaxine, torasemide and acetylsalicylic acid were discontinued after the event onset. Symptomatic improvement was observed after discontinuation of these drugs and volume replacement and administration of sodium hydrogen carbonate. Hyperkalaemia resolved and renal retention parameter improved. The PV centre stated that gastroenteritis (diarrhoea, nausea, and vomiting) surely contributed to the acute symptoms.

The PV centre reported acute prerenal failure and bicytopenia (erythropenia and thrombopenia) as adverse drug reactions with start date of 03-Aug-2009. Acute prerenal failure resolved on 06-Aug-2009. Bicytopenia was still ongoing at the time of the report. Enalapril, nitrendipine+enalapril, torasemide, bisoprolol+hydrochlorothiazide, spironolactone, doxazosin, venlafaxine and diclofenac were suspected for the events. The case was considered to be life-threatening. Concerning bicytopenia further oncological investigation was planned.

Causality assessment by PV centre concerning:

acute prerenal failure:

"possible": enalapril, nitrendipine+enalapril maleate, torasemide, bisoprolol+hydrochlorothiazide, spironolactone

"unlikely": doxazosin, venlafaxine, diclofenac

Bicytopenia:

"possible": enalapril, nitrendipine+enalapril maleate, torasemide, bisoprolol+hydrochlorothiazide, doxazosin, diclofenac, venlafaxine

"unlikely": spironolactone

Explanatory notes on the causality:

Acute renal failure:

Nitrendipine+enalapril maleate: renal dysfunction incl. acute renal failure is known ADR under enalapril. Fluid loss due to diarrhoea and vomiting as well as decreased fluid intake should be considered as alternative cause and risk factor.

Enalapril: Renal dysfunction may occur under enalapril, which could lead to acute renal failure. Fluid loss due to diarrhoea and vomiting as well as decreased fluid intake should be considered as alternative cause and risk factor.

Bisoprolol+hydrochlorothiazide: in combination with non-steroid antirheumatic agents and under hypovolaemia, the risk of prerenal failure increases.

Sironolactone, torasemide: the diuretic effect of torasemide and poor fluid intake could cause prerenal failure. Fluid loss due to diarrhoea and vomiting as well as decreased fluid intake should be considered as alternative cause and risk factor.

Bicytopenia:

Enalapril, nitrendipine+enalapril maleate: especially the patient with impaired renal function may develop cytopenia under the medication.

Bisoprolol+hydrochlorothiazide: cytopenia is listed as a seldom ADR.

Diclofenac, doxazosin, venlafaxine: cytopenia is listed as an ADR.

Torasemide: cytopenia may occur under torasemide.

Additional information received on 12-Apr-2010:

The PV centre provided following new information via the website of the German health authority:

Digitoxin was switched from a concomitant drug to an interacting drug. However, it was not mentioned that with which drug digitoxin interacted.

Spironolactone was excluded from the suspect drugs of bicytopenia. Venlafaxine and diclofenac were also no longer suspected of causing acute prerenal failure.

Explanatory notes on acute prerenal renal failure: hospitalization due to acute prerenal renal failure with symptoms of reduced general condition, nausea, vomiting, exsiccosis and hyperkalaemia. The deterioration of the renal function possibly occurred due to the gastroenteritis-induced volume loss that had worsened by the continuation of the diuretic therapy (torasemide, spironolactone and hydrochlorothiazide). The symptoms improved after discontinuation of the medication and the volume substitution. The gastroenteritis (diarrhoea, nausea and vomiting) had surely contributed to the acute problem. The accumulation of uraemia toxins could have increased the symptoms (nausea and vomiting) and so mutually supported further disease processes in the vicious cycle.

Causality assessment by PV centre concerning:

acute prerenal failure:

"possible": enalapril, nitrendipine+enalapril maleate, torasemide, bisoprolol+hydrochlorothiazide, spironolactone, doxazosin

"unlikely": digitoxin

Bicytopenia:

"possible": enalapril, nitrendipine+enalapril maleate, torasemide, bisoprolol+hydrochlorothiazide, doxazosin, diclofenac, venlafaxine

Explanatory notes on the causal relationship between acute renal failure and digitoxin: nausea and vomiting can occur under increased digitoxin effect. On the admission to the hospital and as a result of renal failure, potassium level was elevated, which leads to an increased digitoxin effect. The digitoxin level itself was in the middle of the reference range. It is unlikely that nausea and vomiting induced by digitoxin partly caused the prerenal renal failure. However, it can not be ruled out in the later clinical course that the hyperkalaemia induced by renal failure caused an increased digitoxin effect with nausea and vomiting, which could have worsened the health condition.

PFIZERINC, 30.05.2012 DE-PFIZER INC-DE-WYE-G05470410

Sender diagnosis: not reported

Narrative

Following the integration of the legacy safety adverse event reporting databases, this case has been migrated from the legacy company database into the current safety database for processing follow-up information. As a consequence of this migration, the follow-up CIOMS I or MedWatch report may indicate in the appropriate field that it is an initial report.

This is a non-interventional study report Information from a study entitled "0600--". An investigator reported that a 70-year-old female participant who received orally venlafaxine hcl (TREVILOR RETARD) at 37.5 mg 2x/day for an unknown indication. Additional suspect drugs included oral enalapril maleate/nitrendipine (ENEAS) 10 mg/20 mg daily for hypertension, oral torasemide 10 mg 1x/day for hypertension, oral bisoprolol/hydrochlorothiazide at 5 mg/12.5 mg mg per day for hypertension, oral spironolactone at 25 mg 1x/day for hypertension, ongoing oral doxazosin at 4 mg 1x/day for hypertension, oral diclofenac at 50 mg 1x/day for pain, ongoing oral digitoxin at 70 ug 1x/day. The medical history included ex-tobacco user (1979-1994: 5 cigarettes per day), type 2 diabetes mellitus (requiring Insulin administration since 1994), hypertension (starting from 1988), hyperlipidaemia and pain with a past history of glaucoma (2003 glaucoma operation both-sided), glaucoma surgery (2003), renal colic (1963), cholecystitis (1965), cholecystectomy (1965) and cholelithiasis (1965). Concomitant therapy included oral metformin 1000 mg, 2x/day for type 2 diabetes mellitus from unknown date to 02Aug2009, subconjunctival latanoprost (XALATAN) 1.5 ug 1x/day for glaucoma, subcutaneous insulin (ACTRAPID INSULIN NOVO) 20 IU 3x/day for type 2 diabetes mellitus, subcutaneous insulin glargine (LANTUS) 64 IU 1x/day for type 2 diabetes mellitus, oral acetylsalicylic acid at 100 mg 1x/day for myocardial infarction prophylaxis and oral nicotinic acid (NIASPAN) at 1500 mg 1x/day. Since 15Jul2009, the patient suffered from diarrhoea, nausea and vomiting. These events were considered as gastroenteritis (not reported as serious event). On 03Aug2009 she was hospitalized with acute renal failure (renal failure acute) with symptoms of reduced general condition, nausea, vomiting, exsiccosis and hyperkalaemia. Further diagnostic tests revealed bicytopenia (bicytopenia). The events were considered life-threatening. Renal failure was considered related to volume loss caused by gastroenteritis, which aggravated due to diuretics Torasemide, Spironolactone and HCT. In addition, the renal perfusion was reduced due to the antihypertensive drugs nitrendipine, enalapril and bisoprolol. Therefore ENEAS, bisoprolol/hydrochlorothiazide, spironolactone, diclofenac and venlafaxine were withdrawn on 03Aug2009, torasemide was withdrawn on 04Aug2009. For treatment she received volume substitution and sodium bicarbonate administration. Subsequently her general condition improved and the potassium levels as well as the renal parameters normalized. On 11Aug2009 she additionally received thrombocyte and erythrocyte concentrates but further diagnostic procedures could not clarify the reason for bicytopenia. She had recovered from renal failure and was discharged from hospital on 17Aug2009. On follow-up digitoxine was reported as another suspect product as digitoxine effect might have been increased due to increased potassium levels that might have contributed to the patient's condition. At the time of follow-up reporting bicytopenia was still reported as not recovered. The doxazosin and digitoxin were continued.

The investigator considered bicytopenia possibly related and renal failure acute not related to venlafaxine hcl. Additionally enalapril maleate/nitrendipine (ENEAS), torasemide, bisoprolol/hydrochlorothiazide and doxazosin were considered possibly related to both events. spironolactone and digitoxine were suspected for renal failure but not for bicytopenia. diclofenac was considered related to bicytopenia only. The investigator also indicated digitoxin as suspect medication for renal failure and doxazosin related not only to bicytopenia but also to renal failure. In

addition bicytopenia was considered primary event as it was the only event that was considered related to TREVILOR therapy, lab results were added and the causality assessments for all suspected drugs were explained. Information was received from a pharmacist participating in a registry (Regional Pharmacovigilance Center Wuppertal) via German regulatory authority (BfArM) regarding a 70-year-old female patient who received Trevilor (venlafaxine hydrochloride) therapy and experienced bicytopenia and acute renal failure.

The lab data included: On 03Aug2009 test results were: blood creatinine (results: 746 $\mu\text{mol/L}$); red blood cell count (results: $3.04 \times 10^{12}/\text{L}$); blood pressure systolic (results: 125 mmHg); oxygen saturation (results: 97 %); PO₂ (results: 11.7 kPa (normal range 9.5-13.9 kPa)); base excess (results: -16.4 mmol/L); blood bicarbonate (results: 12.1 mmol/L); PCO₂ (results: 3.2 kPa (normal range 4.3-5.7 kPa)); blood pH (results: 7.23 units); blood pressure diastolic (results: 50 mmHg); blood potassium (results: 6.3 mmol/L); blood glucose (results: 7.4 mmol/L); haematocrit (results: 30 %); haemoglobin (results: 6.4 mmol/L); blood lactate dehydrogenase (results: 181 IU/L); platelet count (results: $22 \times 10^9/\text{L}$); and creatinine renal clearance (results: 0.08 mL/sec). On 04-Aug-2009 test results were: mean cell volume (results: 98.6 fl (normal range 83-93 fl)); blood creatinine (results: 510 $\mu\text{mol/L}$); blood bicarbonate (results: 17.6 mmol/L); PO₂ (results: 7.5 kPa (normal range 9.5-13.9 kPa)); haematocrit (results: 27 %); PCO₂ (results: 4.2 kPa (normal range 4.3-5.7 kPa)); haemoglobin (results: 5.8 mmol/L); oxygen saturation (results: 87 %); blood pH (results: 7.33 units); platelet count (results: $23 \times 10^9/\text{L}$); base excess (results: -8.2 mmol/L); blood urea (results: 54.0 mmol/L); creatinine renal clearance (results: 0.13 mL/sec); red blood cell count (results: $2.77 \times 10^{12}/\text{L}$); and blood potassium (results: 4.4 mmol/L). On 05-Aug-2009 test results were: blood creatinine (results: 219 $\mu\text{mol/L}$); creatinine renal clearance (results: 0.91 mL/sec); blood urea (results: 30.1 mmol/L); platelet count (results: $16 \times 10^9/\text{L}$); haemoglobin (results: 5.8 mmol/L); haematocrit (results: 28 %); blood potassium (results: 4.5 mmol/L); red blood cell count (results: $2.80 \times 10^{12}/\text{L}$); blood pH (results: 7.34 units); PCO₂ (results: 4.8 kPa (normal range 4.3-5.7 kPa)); blood bicarbonate (results: 19.3 mmol/L); base excess (results: -5.9 mmol/L); PO₂ (results: 6.28 kPa (normal range 9.5-13.9 kPa)); oxygen saturation (results: 84 %); mean cell volume (results: 100.0 fl (normal range 83-93 fl)); reticulocyte count (results: $17.1 \times 10^9/\text{L}$); myoglobin blood (results: 177 ng/mL); and cardioactive drug level (results: 19.6 $\mu\text{g/L}$). On 06-Aug-2009 test results were: blood bicarbonate (results: 22.4 mmol/L); base excess (results: -2.1 mmol/L); PO₂ (results: 6.5 kPa (normal range 9.5-13.9 kPa)); PCO₂ (results: 5.5 kPa (normal range 4.3-5.7 kPa)); oxygen saturation (results: 85 %); and blood pH (results: 7.36 units). On 07-Aug-2009 test results were: blood lactate dehydrogenase (results: 361 IU/L); bilirubin conjugated (results: 16.7 $\mu\text{mol/L}$); and haptoglobin (results: 2.80 g/L). On 08Aug2009 test results were: oxygen saturation (results: 96 %); blood bicarbonate (results: 24.6 mmol/L); base excess (results: 0.2 mmol/L); blood pH (results: 7.45 units); PCO₂ (results: 4.6 kPa (normal range 4.3-5.7 kPa)); and PO₂ (results: 9.39 kPa (normal range 9.5-13.9 kPa)). On 10Aug2009 test results were: PCO₂ (results: 5.4 kPa (normal range 4.3-5.7 kPa)); blood pH (results: 7.48 units); base excess (results: 6.7 mmol/L); oxygen saturation (results: 99 %); blood bicarbonate (results: 30.6 mmol/L); and PO₂ (results: 6.78 kPa (normal range 9.5-13.9 kPa)). On 11Aug2009 test results were: red blood cell count (results: $2.43 \times 10^{12}/\text{L}$); blood potassium (results: 3.9 mmol/L); haematocrit (results: 24 %); haemoglobin (results: 5.1 mmol/L); platelet count (results: $16 \times 10^9/\text{L}$); blood urea (results: 5.76 mmol/L); creatinine renal clearance (results: 0.79 mL/sec); blood creatinine (results: 106 $\mu\text{mol/L}$); aspiration bone marrow (results: according to biopsy results: almost no megakaryocytes, reduced erythropoiesis); immunology test (results: no hint of heparin induced thrombocytopenia or autoimmune thrombocytopenia/alloimmune thrombocytopenia); antineutrophil cytoplasmic antibody (results: negative); antineutrophil cytoplasmic antibody (results: negative); antinuclear antibody (results: negative); and mean cell volume (results: 99.6 fl (normal range 83-93 fl)). On 12Aug2009 test results were: base excess (results: 5.8 mmol/L); blood pH (results: 7.74 units); blood bicarbonate (results: 29.6 mmol/L); mean cell volume (results: 93.6 fl (normal range 83-93 fl)); oxygen saturation (results: 93 %); PO₂ (results: 8.34 kPa (normal range 9.5-13.9 kPa)); and PCO₂ (results: 5.6 kPa (normal range 4.3-5.7 kPa)). Mean cell volume (results: 95.9 fl (normal range 83-93 fl)) was done on 14Aug2009. On 15-Aug-2009 test results were: creatinine renal clearance (results: 0.78 mL/sec); red blood cell count (results: $3.46 \times 10^{12}/\text{L}$); blood creatinine (results: 107 $\mu\text{mol/L}$); blood urea (results: 5.49 mmol/L); platelet count (results: $40 \times 10^9/\text{L}$); haemoglobin (results: 6.8 mmol/L); and haematocrit (results: 33 %). On 17Aug2009 test results were: platelet count (results: $32 \times 10^9/\text{L}$); mean cell volume (results: 95.3 fl (normal range 83-93 fl)); haematocrit (results: 30 %); haemoglobin (results: 6.1 mmol/L); and red blood cell count (results: $3.20 \times 10^{12}/\text{L}$).

Follow up (16May2012): This is a follow-up report to notify that 2010012609 and case DE-WYE-G05470410 are duplicate. All subsequent follow-up information will be reported under DE-WYE-G05470410. The cases 2010012609 are being deleted from the database.

New information from the duplicate case included: Adverse event drug interaction was added, onset date 03Aug2009 and resolved on 06Aug2009.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10016588

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19410920	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	20100111	20100129	19 Day	3 Month	recovering/resolving
2 Bilirubin increased					recovering/resolving
3 Hepatic enzyme increased					recovering/resolving
4 Icterus	20100111				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC	Oral	/	20091010-20091201	
diclofenac			53 Day	
2 Thym-Uvocal Inj.		/	20091204-20100106	
thymus extract		120	34 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Unknown	
Digestion impaired			Unknown	
Upper abdominal discomfort			Unknown	
Mental exhaustion			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		8.4	mg/dl	

Narratives (nur bei elektronischem Eingang)

Strathmann GmbH & Co. KG, 02.02.2010 DE-STRATHMANN-TU-01/01/10

Sender diagnosis: not reported

Narrative

Erstmeldung (Heilpraktikerin) am 22.01.2010: Die Angehörigen einer Patientin berichteten, dass bei der Patientin (R.F., geb. 1941) am 11.01.2010 plötzlich ein "Ikterus" aufzutreten sei. Die Patientin läge nun im Krankenhaus zur diagnostischen Abklärung. Die Patientin liegt bereits seit 10 Tagen im Krankenhaus, die Ursache sei jedoch bis heute, 22.01.2010 noch ungeklärt. Am 04.12.2009 sei die Patientin in die HP-Sprechstunde gekommen, mit der Bitte um eine "Thymus-Therapie". Sie klagte damals über: "Rückenschmerzen, psychisch schlecht drauf, Verdauungsstörungen, Oberbauchbeschwerden". Als Medikation nahm die Patientin zu dem Zeitpunkt "Volatren Resinat" ein. Die Patientin erhielt insgesamt 12 Thym-Uvocal Injektionen (2 mal / Woche, je eine Ampulle). Die erste Injektion erfolgte am 04.12.2009 und die 12. Injektion am 06.01.2010. Die Heilpraktikerin empfahl noch weitere Präparate (z.B. Teufelskralle, Glucosulfonamid, Soja-Protien). Ob die Patientin davon etwas eingenommen hat, wurde nicht berichtet. /

Bemerkung SKG: Der Kausalzusammenhang erscheint zum jetzigen Zeitpunkt unwahrscheinlich, auch wenn er nicht sicher auszuschließen ist. Voltaren könnte z.B. eine mögliche Ursache darstellen, ggf. wurden auch noch weitere Medikamente eingenommen. Zudem wird von der Patientin berichtet, dass sie bereits vor den Thym-Uvocal-Injektionen über Oberbauchbeschwerden, Rückenschmerzen und Verdauungsstörungen klagte. Symptome, die bei "hepatozellulärem Ikterus" oder bei einem "cholestatischen Ikterus" denkbar sind bzw. diesem voraus gehen können. Für ein hepatozelluläres Geschehen spräche z.B. eine Voltaren Einnahme als Ursache einer medikamentös bedingten Hepatitis.

Strathmann GmbH & Co. KG, 03.02.2010 DE-STRATHMANN-TU-01/01/10

Sender diagnosis: not reported

Narrative

Erstmeldung (Heilpraktikerin) am 22.01.2010: Die Angehörigen einer Patientin berichteten, dass bei der Patientin (R.F., geb. 1941) am 11.01.2010 plötzlich ein "Ikterus" aufzutreten sei. Die Patientin läge nun im Krankenhaus zur diagnostischen Abklärung. Die Patientin liegt bereits seit 10 Tagen im Krankenhaus, die Ursache sei jedoch bis heute, 22.01.2010 noch ungeklärt. Am 04.12.2009 sei die Patientin in die HP-Sprechstunde gekommen, mit der Bitte um eine "Thymus-Therapie". Sie klagte damals über: "Rückenschmerzen, psychisch schlecht drauf, Verdauungsstörungen, Oberbauchbeschwerden". Als Mediaktion nahm die Patientin zu dem Zeitpunkt "Volatren Resinat" ein. Die Patientin erhielt insgesamt 12 Thym-Uvocal Injektionen (2 mal / Woche, je eine Ampulle). Die erste Injektion erfolgte am 04.12.2009 und die 12. Injektion am 06.01.2010. Die Heilpraktikerin empfahl noch weitere Präparate (z.B. Teufelskralle, Glucosulfonamid, Soja-Protien). Ob die Patientin davon etwas eingenommen hat, wurde nicht berichtet. /

Bemerkung SKG: Der Kausalzusammenhang erscheint zum jetzigen Zeitpunkt unwahrscheinlich, auch wenn er nicht sicher auszuschließen ist. Voltaren könnte z.B. eine mögliche Ursache darstellen, ggf. wurden auch noch weitere Medikamente eingenommen. Zudem wird von der Patientin berichtet, dass sie bereits vor den Thym-Uvocal-Injektionen über Oberbauchbeschwerden, Rückenschmerzen und Verdauungsstörungen klagte. Symptome, die bei "hepatozellulärem Ikterus" oder bei einem "cholestatischen Ikterus" denkbar sind bzw. diesem voraus gehen können. Für ein hepatozelluläres Geschehen spräche z.B. eine Voltaren Einnahme als Ursache einer medikamentös bedingten Hepatitis.

sanofi-aventis, 17.02.2010 DE-SANOFI-AVENTIS-2010SA007929

Sender diagnosis: not reported

Narrative

Initial Report:

This serious case was extracted from the website of the German health authority (authority number DE-BFARM-10016588), initially reported by an anonymized physician to the BfArM. The case involves a 68-year-old female patient. Medical history and concomitant medication were not provided. The patient was treated with diclofenac (trade name and dosage not provided) from 10-Oct-2009 to 01-Dec-2009. On 11-Jan-2010 the patient experienced hepatitis with increase of hepatic values and bilirubin to 8.4 mg/dl. Subsequently the patient was hospitalised. The event lasted for 19 days and the patient is recovering. The reporter did not provide any information about the causality.

Based on the available information and safety profile a causal relationship to diclofenac cannot be excluded.

17-FEB-2010 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:NOVARTIS, 22.02.2010 DE-002147023-PHHY2010DE08464

Sender diagnosis: not reported

Narrative

Initial report received from a physician via Health Authority (reference number: DE-BFARM-10016588) on 10 Feb 2010: The patient was on treatment with diclofenac (manufacturer unknown) for unknown indication from 10 Oct 2009 to 01 Dec 2009. On 11 Jan 2010 the patient developed hepatitis with increased liver parameters and increase of bilirubin upto 8.4 mg/dl. The outcome of the event was reported to be condition improving.

SENDER'S COMMENT:

Serious spontaneous report, hepatitis [hospitalization], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

Strathmann GmbH & Co. KG, 03.03.2010 DE-STRATHMANN-TU-01/01/10

Sender diagnosis: not reported

Narrative

Erstmeldung (Heilpraktikerin) am 22.01.2010: Die Angehörigen einer Patientin berichteten, dass bei der Patientin (R.F., geb. 1941) am 11.01.2010 plötzlich ein "Ikterus" aufzutreten sei. Die Patientin läge nun im Krankenhaus zur diagnostischen Abklärung. Die Patientin liegt bereits seit 10 Tagen im Krankenhaus, die Ursache sei jedoch bis heute, 22.01.2010 noch ungeklärt. Am 04.12.2009 sei die Patientin in die HP-Sprechstunde gekommen, mit der Bitte um eine "Thymus-Therapie". Sie klagte damals über: "Rückenschmerzen, psychisch schlecht drauf, Verdauungsstörungen, Oberbauchbeschwerden". Als Mediaktion nahm die Patientin zu dem Zeitpunkt "Volatren Resinat" ein. Die Patientin erhielt insgesamt 12 Thym-Uvocal Injektionen (2 mal / Woche, je eine Ampulle). Die erste Injektion erfolgte am 04.12.2009 und die 12. Injektion am 06.01.2010. Die Heilpraktikerin empfahl noch weitere Präparate (z.B. Teufelskralle, Glucosulfonamid, Soja-Protien). Ob die Patientin davon etwas eingenommen hat, wurde nicht berichtet. /

Bemerkung SKG: Der Kausalzusammenhang erscheint zum jetzigen Zeitpunkt unwahrscheinlich, auch wenn er nicht sicher auszuschließen ist. Voltaren könnte z.B. eine mögliche Ursache darstellen, ggf. wurden auch noch weitere Medikamente eingenommen. Zudem wird von der Patientin berichtet, dass sie bereits vor den Thym-Uvocal-Injektionen über Oberbauchbeschwerden, Rückenschmerzen und Verdauungsstörungen klagte. Symptome, die bei "hepatozellulärem Ikterus" oder bei einem "cholestatischen Ikterus" denkbar sind bzw. diesem voraus gehen können. Für ein hepatozelluläres Geschehen spräche z.B. eine Voltaren Einnahme als Ursache einer medikamentös bedingten Hepatitis. /// Follow up (Krankenhaus) am 30.12.2009: Vom Chefarzt der Klinik wird schriftlich mitgeteilt, dass ein Zusammenhang zwischen dem Auftreten der Hepatitis und der Applikation von Thym-Uvocal ausgeschlossen werden konnte. "Es handelte sich vielmehr um eine Diclofenac-induzierte Hepatitis, die als solche auch histologisch bestätigt werden konnte. Die Patientin hatte im Vorfeld ihrer Erkrankung über einen längeren Zeitraum ein Diclofenac-Präparat eingenommen. Inzwischen ist die Hepatitis weitgehend abgeklungen."

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10027550

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 67 Year	weight (kg): 98	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestatic liver injury					Unknown
2 Liver function tests raised	20090930				recovering/resolving
3 Hepatic cirrhosis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 METFORMIN metformin	Oral	850 Mg milligram(s)/Day	-20091005	Diabetes
2 MIRTAZAPINE mirtazapine	Oral	15 Mg milligram(s)/Day	20090925-20091005	Depression
3 PANTOPRAZOL pantoprazole	Oral	20 Mg milligram(s)/Day	20090928-20091005	Drug use for unknown indication
4 voltaren diclofenac sodium	Oral	50 Mg milligram(s)/Day	20090928-20091005	Analgesia
5 XIMOVAN zopiclone	Oral	7.5 Mg milligram(s)/Day	20090916-20091005	Drug use for unknown indication

Drugs (concomitant)

6 ass-ratiopharm acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	-	Drug use for unknown indication
7 BELOC metoprolol tartrate	Oral	47.5 Mg milligram(s)/Day	-	Drug use for unknown indication
8 CITALOPRAM citalopram	Oral	40 Mg milligram(s)/Day	20090829-	Drug use for unknown indication
9 CITALOPRAM citalopram	Oral	20 Mg milligram(s)/Day	-20090828	Drug use for unknown indication
10 RAMIPRIL ramipril	Oral	2.5 Mg milligram(s)/Day	-20091212	Drug use for unknown indication
11 SPIRONOLACTON spironolactone	Oral	50 Mg milligram(s)/Day	-	Drug use for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Liver damage			k.A.	
Diabetes			k.A.	ICD 9-CODE 250
Hypertension			k.A.	ICD 9-CODE 401
Coronary heart disease			k.A.	ICD 9-CODE 414
Alcohol addiction			k.A.	ICD CODE: F10.2
Depression	20090728	20091113	k.A.	ICD CODE: F32.2

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
BILI GES (TOTAL BILIRUBIN)	20090908	0.36	MG/DL	
BILI GES (TOTAL BILIRUBIN)	20090930	2.48	MG/DL	
BILI GES (TOTAL BILIRUBIN)	20091002	5.09	MG/DL	
BILI GES (TOTAL BILIRUBIN)	20091005	1.81	MG/DL	
BILI GES (TOTAL BILIRUBIN)	20091013	1.23	MG/DL	
BILI GES (TOTAL BILIRUBIN)	20091020	0.93	MG/DL	
BILI GES (TOTAL BILIRUBIN)	20091102	1.09	MG/DL	
BILI GES (TOTAL BILIRUBIN)	20091112	0.75	MG/DL	
BILI DIR (BILIRUBIN DIRECT)	20090930	2.11	MG/DL	
BILI DIR (BILIRUBIN DIRECT)	20091002	4.72	MG/DL	
INR	20091002	1.15	UNK	
INR	20091005	1.10	UNK	
INR	20091020	1.08	UNK	
CRP	20090930	153.91	MG/L	
CRP	20091002	135.88	MG/L	
CRP	20091005	34.00	MG/L	
CRP	20091013	5.73	MG/L	
CRP	20091102	3.75	MG/L	
CRP	20091112	4.29	MG/L	

Narratives (nur bei elektronischem Eingang)

Schering-Plough, 01.03.2010 DE-01054554290-2010SP010274

Sender diagnosis: not reported

Narrative

A SPONTANEOUS CASE REPORT FROM GERMANY (LOCAL REFERENCE NUMBER DEU100391) ORIGINATING FROM A MEDICAL DOCTOR VIA THE AMSP PROJECT, REGARDING A MALE PATIENT BORN IN 1942 (S-P ESTIMATES AGE TO BE 67).

THE PATIENT HAD A MEDICAL HISTORY INCLUDING THE FOLLOWING: SEVERE DEPRESSION WITHOUT PSYCHOTIC SYMPTOMS (ICD CODE F32.2), ALCOHOL ADDICTION (ICD CODE F10.2) SINCE AN UNKNOWN DATE, A PREDAMAGED LIVER (THE USE OF ALCOHOL WAS ALSO MENTIONED IN THIS CONTEXT, BUT A CAUSALITY WAS NOT PROVIDED), CORONARY HEART DISEASE SINCE AN UNKNOWN DATE, HYPERTENSION SINCE AN UNKNOWN DATE AND DIABETES SINCE AN UNKNOWN DATE. HE USED THE FOLLOWING CONCOMITANT MEDICATIONS: CITALOPRAM (20 MG ORALLY, ONCE PER DAY) TAKEN FOR YEARS, FOR AN UNKNOWN INDICATION, UNTIL 28AUG2009; CITALOPRAM (40 MG ORALLY, ONCE PER DAY) SINCE 29AUG2009 (ONGOING); ASS (ACETYLSALICYLIC ACID), TAKEN FOR YEARS, FOR AN UNKNOWN INDICATION, AT A DOSE OF 100 MG ONCE PER DAY, ORALLY (ONGOING); BELOC MITE (METOPROLOL, 47.5 MG ONCE PER DAY, ORALLY), TAKEN FOR YEARS, FOR AN UNKNOWN INDICATION (ONGOING); RAMIPRIL (2.5 MG ONCE PER DAY, ORALLY), TAKEN FOR YEARS, FOR AN UNKNOWN INDICATION.

ON 25SEP2009, THE PATIENT STARTED USING MIRTAZAPIN (MANUFACTURER UNKNOWN, 15 MG ONCE PER DAY, ORALLY) FOR DEPRESSION. FURTHERMORE, HE HAD BEEN USING METFORMIN (850 MG ONCE PER DAY, ORALLY) FOR YEARS (SINCE AN UNKNOWN DATE) FOR DIABETES, XIMOVAN (ZOPICLONE) SINCE 16SEP2009 AT A REGIMEN OF 7.5 MG ONCE PER DAY, ORALLY, FOR AN UNKNOWN INDICATION, VOLTAREN (DICLOFENAC), SINCE 28SEP2009, AT A REGIMEN OF 50 MG ONCE PER DAY, ORALLY, AS ANALGETICUM, AND PANTOPRAZOL (20 MG ONCE PER DAY) SINCE 28SEP2009, FOR AN UNKNOWN INDICATION.

ON 28JUL2009, THE PATIENT WAS HOSPITALIZED DUE TO HIS PSYCHIATRIC DISEASE. ON 29JUL2009, THE FOLLOWING LAB-VALUES WERE MEASURED: GOT (AST; ASPARTATE TRANSAMINASE): 56 (NORMAL RANGE: 3-45 U/L), GPT (ALT; ALANINE TRANSAMINASE): 58 (NORMAL RANGE: 2-45 U/L), GGT (GAMMA GLUTAMYL TRANSPEPTIDASE): 306 U/L (NORMAL RANGE: 2-55 U/L). AP (ALKALINE PHOSPHATASE), ALSO MEASURED ON 29JUL2009, WAS 97 (NORMAL RANGE: 40-130 U/L). ON 30SEP2009, THE PATIENT EXPERIENCED ICTERUS WITH INCREASED LIVER ENZYMES. THE FOLLOWING LAB-TEST RESULTS WERE PROVIDED:
 MEAN CORPUSCULAR VOLUME (MCV): NORMAL RANGE: 79.0 - 97.0 FL; 08SEP2009, 09:47 AM: 94.9 FL; 30SEP2009, 09:46AM: 92.9 FL; 02OCT2009, 10:00 AM: 93.5 FL; 05OCT2009, 10:01AM: 94.4 FL; 13OCT2009, 09:48AM: 94.6 FL; 20OCT2009.03:37AM: 93.7 FL; 02NOV2009, 09:49AM: 93.0 FL; 12NOV2009, 09:59AM: 92.3 FL.

EOS (PERCENTAGE OF EOSINOPHILS): NORMAL RANGE: 1.0-5.0%; 08SEP2009, 09:47 AM: 2.5%; 30SEP2009, 09:46AM: 0.9% LOW; 02OCT2009, 10:00 AM: 2.6%; 05OCT2009, 10:01AM: 2.6%; 13OCT2009, 09:48AM: 1.6%; 20OCT2009.03:37AM: 2.7%; 02NOV2009, 09:49AM: 1.8%; 12NOV2009, 09:59AM: 2.8%.

EOS (NUMBER OF EOSINOPHILS): NORMAL RANGE: 0.0-0.7/NL; 08SEP2009, 09:47 AM: 0.2 /NL; 30SEP2009, 09:46AM: 0.1 /NL; 02OCT2009, 10:00 AM: 0.2 /NL; 05OCT2009, 10:01AM: 0.2 /NL; 13OCT2009, 09:48AM: 0.2 /NL; 20OCT2009.03:37AM: 0.2 /NL 02NOV2009, 09:49AM: 0.2 /NL; 12NOV2009, 09:59AM: 0.2 /NL.

PTT (PROTHROMBIN TIME): NORMAL RANGE: < 36 SEC; 02OCT2009, 10:00 AM: 38.5 SEC; 05OCT2009, 10:01AM: 38.7 SEC; 20OCT2009.03:37AM: 36.4 SEC.

GOT: 08SEP2009, 09:47 AM: 29 U/L; 30SEP2009, 09:46AM: 366 U/L (HIGH); 02OCT2009, 10:00 AM: 114 U/L (HIGH); 05OCT2009, 10:01AM: 57 U/L (HIGH); 13OCT2009, 09:48AM: 44 U/L; 20OCT2009.03:37AM: 31 U/L; 02NOV2009, 09:49AM: 28 U/L; 12NOV2009, 09:59AM: 31 U/L.

GPT: 08SEP2009, 09:47 AM: 24 U/L; 30SEP2009, 09:46AM: 224 U/L (HIGH); 02OCT2009, 10:00 AM: 157 U/L (HIGH); 05OCT2009, 10:01AM: 118 U/L (HIGH); 13OCT2009, 09:48AM: 68 U/L (HIGH); 20OCT2009.03:37AM: 35 U/L; 02NOV2009, 09:49AM: 30 U/L; 12NOV2009, 09:59AM: 31 U/L.

GGT: 08SEP2009, 09:47AM: 272 U/L (HIGH); 30SEP2009, 09:46AM: 889 U/L (HIGH); 02OCT2009, 10:00 AM: 921 U/L (HIGH); 05OCT2009, 10:01AM: 1016 U/L (HIGH); 13OCT2009, 09:48AM: 606 U/L (HIGH); 20OCT2009.03:37AM: 344 U/L (HIGH); 02NOV2009, 09:49AM: 193 U/L (HIGH); 12NOV2009, 09:59AM: 136 U/L (HIGH).

AP: 08SEP2009, 09:47 AM: 139 U/L (HIGH); 30SEP2009, 09:46AM: 528 U/L (HIGH); 02OCT2009, 10:00 AM: 489 U/L (HIGH); 05OCT2009, 10:01AM: 516 U/L (HIGH); 13OCT2009, 09:48AM: 284 U/L (HIGH); 20OCT2009.03:37AM: 182 U/L (HIGH); 02NOV2009, 09:49AM: 132 U/L (HIGH); 12NOV2009, 09:59AM: 117 U/L.

BILI GES (TOTAL BILIRUBIN): NORMAL RANGE: 0.20-1.10 MG/DL; 08SEP2009 0.36 MG/DL; 30SEP2009 2.48 MG/DL; 02OCT2009 5.09 MG/DL; 05OCT2009 1.81 MG/DL; 13OCT2009 1.23 MG/DL; 20OCT2009 0.93 MG/DL; 02NOV2009 1.09 MG/DL; 12NOV2009 0.75 MG/DL.

BILI DIR (BILIRUBIN DIRECT): NORMAL RANGE: 0.20- 1.10 MG/DL; 30SEP2009 2.11 MG/DL; 02OCT2009 4.72 MG/DL.

INR (INTERNATIONAL NORMALIZED RATIO; UNITS UNKNOWN, NORMAL RANGE UNKNOWN): 02OCT2009: 1.15; 05OCT2009 1.10; 20OCT2009: 1.08.

CRP (C-REACTIVE PROTEIN): NORMAL RANGE: 0.00-5.00 MG/L; 30SEP2009 153.91 MG/L; 02OCT2009 135.88 MG/L; 05OCT2009 34.00 MG/L; 13OCT2009 5.73 MG/L; 02NOV2009 3.75 MG/L; 12NOV2009 4.29 MG/L.

CHOLINESTERASE (CHE): NORMAL RANGE: 5.10-11.70 KU/L; 08SEP2009, 09:47 AM: 12.47 KU/L (HIGH); 30SEP2009, 09:46AM: 9.74 KU/L; 02OCT2009, 10:00 AM: 8.84 KU/L; 05OCT2009, 10:01AM: 9.72 KU/L; 13OCT2009, 09:48AM: 11.92 KU/L (HIGH); 20OCT2009, 03:37AM: 12.72 KU/L (HIGH); 02NOV2009, 09:49AM: 12.80 KU/L (HIGH); 12NOV2009, 09:59AM: 12.84 KU/L (HIGH).

FURTHERMORE, ON 02OCT2009 AT 10:00AM THE PATIENT'S MIRTAPAZINE LEVEL AND CITALOPRAM LEVEL WERE 73.2 NG/ML AND 197.8 NG/ML, RESPECTIVELY. AN ACUTE AND CHRONIC HEPATITIS (INCLUDING HEPATITIS A, B AND C) WERE EXCLUDED. A LIVER BIOPSY WAS PERFORMED ON AN UNKNOWN DATE AND REVEALED THE FOLLOWING: "SEVERE FIBROSIS (GRADE 4), CHOLESTASIS (CHOLESTASIS) WITH PORTAL ACCENTUATED INFLAMMATION (GRADE 2-3)." IT WAS CONCLUDED THAT THE PATIENT HAD A "MEDICAMENTOUS TOXIC INDUCED LIVER DAMAGE" (CHOLESTATIC LIVER DAMAGE). ON 13NOV2009 THE PATIENT WAS DISCHARGED FROM HOSPITAL. THE LABORATORY VALUES NORMALIZED AND THE OUTCOME FOR HEPATIC VALUES INCREASED WAS "RECOVERING". THE OUTCOME OF THE HEPATIC CIRRHOSIS (SEVERE FIBROSIS) WAS UNKNOWN.

MIRTAPAZIN, METFORMIN, XIMOVAN AND VOLTAREN WERE PERMANENTLY STOPPED ON 05OCT2009. ALSO ON 05OCT2009, PANTOPRAZOLE WAS DISCONTINUED. THE PATIENT WAS SWITCHED TO LYRICA (PREGABALIN) AND GLIMEPIRIDE (DATE UNKNOWN).

THE REPORTER CONSIDERED THE EVENT HEPATIC VALUES INCREASED TO BE SERIOUS SINCE THE PATIENT WAS TRANSFERRED TO ANOTHER WARD FROM 05OCT2009 TO 09OCT2009, DUE TO THIS EVENT. FURTHERMORE, ALTHOUGH NOT REPORTED AS AES, SCHERING-PLOUGH ADDED THE RESULTS FROM THE LIVER BIOPSY (SEVERE FIBROSIS GRADE 4 (HEPATIC CIRRHOSIS) AND MEDICAMENTOUS TOXIC INDUCED LIVER DAMAGE (CHOLESTATIC LIVER DAMAGE)) AS AES TO THE CASE; THESE WERE CONSIDERED TO BE MEDICALLY SIGNIFICANT BY SCHERING-PLOUGH.

THE REPORTER CONSIDERED THE EVENT HEPATIC ENZYMES INCREASED TO BE PROBABLY RELATED TO MIRTAPAZIN, XIMOVAN, VOLTAREN AND PANTOPRAZOL, AND POSSIBLY RELATED TO METFORMIN. THE REPORTER PROVIDED THE FOLLOWING RATIONALE FOR THE CAUSE OF THE EVENT: "HEPATOTOXIC EFFECT IS KNOWN FOR MIRTAPAZIN, METFORMIN, XIMOVAN. POSSIBLE ADDITIVE EFFECT. PREDAMAGED LIVER (ALCOHOL). NO RELEVANT INTERACTIONS. POSSIBLE RISK FACTORS: YES, PREDAMAGE OF CONCERNED ORGAN. MEDICAL HISTORY: ALCOHOL ADDICTION. AT ADMISSION GOT AND GPT SLIGHTLY INCREASED, GGT CLEARLY INCREASED." NO CAUSALITY WAS PROVIDED FOR FIBROSIS GRADE 4 (HEPATIC CIRRHOSIS) AND MEDICAMENTOUS TOXIC INDUCED LIVER DAMAGE (CHOLESTATIC LIVER DAMAGE). Test results: MIRTAPAZIN: 73.2 NG/ML (02OCT2009, 10:00AM) CITALOPRAM: 197.8 NG/ML (02OCT2009, 10:00AM)

MEAN CORPUSCULAR VOLUME (MCV): NORMAL RANGE: 79.0 - 97.0 FL

MCV (08SEP2009, 09:47 AM): 94.9 FL

MCV (30SEP2009, 09:46AM): 92.9 FL

MCV (02OCT2009, 10:00 AM): 93.5 FL

MCV (05OCT2009, 10:01AM): 94.4 FL

MCV (13OCT2009, 09:48AM): 94.6 FL

MCV (20OCT2009.03:37AM): 93.7 FL

MCV (02NOV2009, 09:49AM): 93.0 FL

MCV (12NOV2009, 09:59AM): 92.3 FL

EOS: NORMAL RANGE: 1.0-5.0%

EOS (08SEP2009, 09:47 AM): 2.5%

EOS (30SEP2009, 09:46AM): 0.9% (LOW)

EOS (02OCT2009, 10:00 AM): 2.6%

EOS (05OCT2009, 10:01AM): 2.6%
EOS (13OCT2009, 09:48AM): 1.6%
EOS (20OCT2009.03:37AM): 2.7%
EOS (02NOV2009, 09:49AM): 1.8%
EOS (12NOV2009, 09:59AM): 2.8%

EOS #: NORMAL RANGE: 0.0-0.7/NL
EOS (08SEP2009, 09:47 AM): 0.2 /NL
EOS (30SEP2009, 09:46AM): 0.1 /NL
EOS (02OCT2009, 10:00 AM): 0.2 /NL
EOS (05OCT2009, 10:01AM): 0.2 /NL
EOS (13OCT2009, 09:48AM): 0.2 /NL
EOS (20OCT2009.03:37AM): 0.2 /NL
EOS (02NOV2009, 09:49AM): 0.2 /NL
EOS (12NOV2009, 09:59AM): 0.2 /NL

PTT: NORMAL RANGE: < 36 SEC
PTT (02OCT2009, 10:00 AM): 38.5 SEC
PTT (05OCT2009, 10:01AM): 38.7 SEC
PTT (20OCT2009.03:37AM): 36.4 SEC

GOT: NORMAL RANGE: 3-45 U/L
GOT (AST) (29JUL2009): 56
GOT (08SEP2009, 09:47 AM): 29 U/L
GOT (30SEP2009, 09:46AM): 366 U/L (HIGH)
GOT (02OCT2009, 10:00 AM): 114 U/L (HIGH)
GOT (05OCT2009, 10:01AM): 57 U/L (HIGH)
GOT (13OCT2009, 09:48AM): 44 U/L
GOT (20OCT2009.03:37AM): 31 U/L
GOT (02NOV2009, 09:49AM): 28 U/L
GOT (12NOV2009, 09:59AM): 31 U/L

GPT: NORMAL RANGE: 2-45 U/L
GPT (ALT) (29JUL2009): 58
GPT (08SEP2009, 09:47 AM): 24 U/L
GPT (30SEP2009, 09:46AM): 224 U/L (HIGH)
GPT (02OCT2009, 10:00 AM): 157 U/L (HIGH)
GPT (05OCT2009, 10:01AM): 118 U/L (HIGH)
GPT (13OCT2009, 09:48AM): 68 U/L (HIGH)
GPT (20OCT2009.03:37AM): 35 U/L
GPT (02NOV2009, 09:49AM): 30 U/L
GPT (12NOV2009, 09:59AM): 31 U/L

GGT (GAMMA-GT): NORMAL RANGE: 2-55 U/L
GGT (29JUL2009): 306 U/L (HIGH)
GGT (08SEP2009, 09:47AM): 272 U/L (HIGH)
GGT (30SEP2009, 09:46AM): 889 U/L (HIGH)
GGT (02OCT2009, 10:00 AM): 921 U/L (HIGH)
GGT (05OCT2009, 10:01AM): 1016 U/L (HIGH)
GGT (13OCT2009, 09:48AM): 606 U/L (HIGH)
GGT (20OCT2009.03:37AM): 344 U/L (HIGH)
GGT (02NOV2009, 09:49AM): 193 U/L (HIGH)
GGT (12NOV2009, 09:59AM): 136 U/L (HIGH)

AP: NORMAL RANGE: 40-130 U/L
AP (29JUL2009): 97
AP (08SEP2009, 09:47 AM): 139 U/L (HIGH)
AP (30SEP2009, 09:46AM): 528 U/L (HIGH)
AP (02OCT2009, 10:00 AM): 489 U/L (HIGH)
AP (05OCT2009, 10:01AM): 516 U/L (HIGH)
AP (13OCT2009, 09:48AM): 284 U/L (HIGH)
AP (20OCT2009.03:37AM): 182 U/L (HIGH)
AP (02NOV2009, 09:49AM): 132 U/L (HIGH)
AP (12NOV2009, 09:59AM): 117 U/L

CHOLINESTERASE (CHE): NORMAL RANGE: 5.10-11.70 KU/L
CHE (08SEP2009, 09:47 AM): 12.47 KU/L (HIGH)

CHE (30SEP2009, 09:46AM): 9.74 KU/L
CHE (02OCT2009, 10:00 AM): 8.84 KU/L
CHE (05OCT2009, 10:01AM): 9.72 KU/L
CHE (13OCT2009, 09:48AM): 11.92 KU/L (HIGH)
CHE (20OCT2009.03:37AM): 12.72 KU/L (HIGH)
CHE (02NOV2009, 09:49AM): 12.80 KU/L (HIGH)
CHE (12NOV2009, 09:59AM): 12.84 KU/L (HIGH)

ACUTE AND CHRONIC HEPATITIS A / B / C WERE EXCLUDED

LIVER BIOPSY (UNKNOWN DATE): SEVERE FIBROSIS (GRADE 4), CHOSTASIS WITH PORTAL ACCENTUATED INFLAMMATION (GRADE 2-3); CONCLUSION: MEDICAMENTOUS TOXIC INDUCED LIVER DAMAGE (CHOLEATATIC LIVER DAMAGE).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10030897

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased	20090930			3 Day	recovering/resolving
2 Gamma-glutamyltransferase increased	20090930			3 Day	recovering/resolving
3 Transaminases increased	20090930			3 Day	recovering/resolving
4 Elevated liver enzymes	20090930			3 Day	recovering/resolving
5 Fibrosis liver					recovering/resolving
6 Icterus	20090930			3 Day	recovering/resolving
7 Cholestatic liver injury	20090930			368 Day	recovering/resolving
8 Drug-induced liver disease	20090930			3 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 METFORMIN metformin	Oral	850 Mg milligram(s)/Day	-20091005	-
2 MIRTAZAPINE mirtazapine	Oral	15 Mg milligram(s)/Day	20090925-20091005	11 Day
3 PANTOPRAZOLE pantoprazole	Oral	20 Mg milligram(s)/Day	20090928-20091005	8 Day
4 VOLTAREN diclofenac sodium	Oral	50 Mg milligram(s)/Day	20090928-20091005	8 Day
5 XIMOVAN zopiclone	Oral	7.5 Mg milligram(s)/Day	20090916-20091005	20 Day

Drugs (concomitant)

6 Acetylsalizylsaeure acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	-	-
7 BELOC MITE metoprolol tartrate	Oral	47.5 Mg milligram(s)/Day	-	-
8 CITALOPRAM citalopram	Oral	20 Mg milligram(s)/Day	-20090828	-
9 CITALOPRAM citalopram	Oral	40 Mg milligram(s)/Day	20090829-	-
10 RAMIPRIL ramipril	Oral	2.5 Mg milligram(s)/Day	-20091005	-
11 SPIRONOLACTONE spironolactone	Oral	50 Mg milligram(s)/Day	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			k.A.	
Transaminase NOS increased			k.A.	
Hypertension arterial			k.A.	
Coronary artery disease			k.A.	
Alcohol abuse			k.A.	
Gamma GT increased			k.A.	
Depression			k.A.	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGPT	20090908	24	IU/l	
SGPT	20090930	224	IU/l	
SGPT	20091005	118	IU/l	
SGPT	20091013	68	IU/l	
SGPT	20091020		IU/l	
SGPT	20091112	136	IU/l	
SGOT	20090908	29	IU/l	
SGOT	20090930	366	IU/l	
SGOT	20091005	57	IU/l	
SGOT	20091013	44	IU/l	
SGOT	20091020		IU/l	
Bilirubin direct	20090930	2.11 mg/dL		
Bilirubin direct	20091005			
Bilirubin direct	20091013			
Alkaline phosphatase	20090908	139	IU/l	
Alkaline phosphatase	20090930	528	IU/l	
Alkaline phosphatase	20091005	516	IU/l	
Alkaline phosphatase	20091013	284	IU/l	
Alkaline phosphatase	20091020	182	IU/l	
Alkaline phosphatase	20091102	132	IU/l	
Bilirubin total	20090908	0.36 mg/dL		
Bilirubin total	20090930	2.48 mg/dL		
Bilirubin total	20091005	1.81 mg/dL		
Bilirubin total	20091013	1.23 mg/dL		
Triglycerides	20090908	525 mg/dL		
Triglycerides	20090930			
Triglycerides	20091005	213 mg/dL		
Triglycerides	20091020	319 mg/dL		
Triglycerides	20091112	252 mg/dL		
C-reactive protein	20090908		mg/l	
C-reactive protein	20090930	153.91	mg/l	
C-reactive protein	20091005	3400	mg/l	
C-reactive protein	20091013	5.73	mg/l	
GGT	20090908	272	IU/l	
GGT	20090930	869	IU/l	
GGT	20091005	1016	IU/l	
GGT	20091013	606	IU/l	
GGT	20091020	344	IU/l	
GGT	20091102	193	IU/l	

Hemoglobin	20090908	12.9 g/dL	
Hemoglobin	20090930	13.4 g/dL	
Hemoglobin	20091005	13.1 g/dL	
Hemoglobin	20091013	14.0 g/dL	
Hemoglobin	20091020		
Lymphocyte count	20090908	43.8	%
Lymphocyte count	20090930	11.4	%
Lymphocyte count	20091005	31.6	%
Lymphocyte count	20091013	45.2	%
Lymphocyte count	20091102	47.1	%
Lymphocyte count	20091112	41.7	%
Monocyte count	20090908	9.0	%
Monocyte count	20090930	4.8	%
Monocyte count	20091005	7.6	%
Monocyte count	20091013	6.9	%
Neutrophil count	20090908	44	%
Neutrophil count	20090930	83	%
Neutrophil count	20091005	60	%
Neutrophil count	20091013	46	%
Neutrophil count	20091102	42	%
Neutrophil count	20091112	48	%
RBC count	20090908	3.9 /pL	
RBC count	20090930	4.15 /pL	
RBC count	20091005	4.06 /pL	
RBC count	20091013	4.37 /pL	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 03.03.2010 DE-002147023-PHHY2010DE10133

Sender diagnosis: not reported

Narrative

Initial report received from the non-Novartis German intensive monitoring project "Drug Safety in Psychiatry" (AMSP no. DGZ-18-004) on 18 Feb 2010: This poly medicated inpatient with a history of depression, chronic alcohol abuse, coronary artery disease, hypertension arterial, diabetes mellitus, mild transaminase and significant gamma GGT increase received treatment with Voltaren (diclofenac sodium) 50 mg daily, orally from 28 Sep 2009 to 05 Oct 2009 for an unspecified indication. Other co-suspect medications included Metformin 850 mg daily, Ximovan (Zopiclone) 7.5 mg daily, orally on 16 Sep 2009, Mirtazapine 15 mg daily, orally on 25 Sep 2009, and Pantoprazole 20 mg daily, orally on 28 Sep 2009. He was diagnosed with icterus on 30 Sep 2009 along with increases of gamma GT (1016), SGOT 366 U/L, SGPT 224 U/L and bilirubin (Total 5.8mg/dL, direct 4.72mg/dL). The patient was transferred to the internal ward for diagnostics and treatment. Serologically ruled out acute or chronic hepatitis. Liver biopsy findings were as follows: severe fibrosis (grade IV), cholestasis with inflammation (predominance portal), and drug-toxic cholestatic liver damage. On 05 Oct 2009, Mirtazapine, Ximovan and metformin were stopped, and psychiatric medication (names not mentioned) also been changed. Later, the liver values were decreasing, and patient could be retransferred to the previous ward on 09 Oct 2009. Causality has been assessed as possible for metformin, and as probable for Voltaren, Ximovan, Mirtazapine and Pantoprazole, based on well known data with respect to hepatotoxicity.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10082686

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxicity	2010				Unknown
2 Erythroderma	2010				Unknown
3 Skin disorder	2010				Unknown
4 Fever	2010				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclophenac diclofenac	Unknown	/	-	-
2 Pandemrix H1N1 a/h1n1 influenza pandemic vaccine	Unknown	/ Day 120	201002-201002 -	Prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Yes	
Diarrhea			Yes	
Skin disorder			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 27.05.2010 DE-GlaxoSmithKline-D0067560A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of hepatotoxic reaction in a subject of unspecified age and gender who was vaccinated with swine flu split dresden (Pandemrix H1N1, GlaxoSmithKline) for prophylaxis.

In February 2010 the subject received unspecified dose of Pandemrix H1N1 (unknown route and injection site).

In 2010, an unspecified time after vaccination with Pandemrix H1N1, the subject experienced hepatotoxic reaction with fever and skin change.

The subject was hospitalised.

At the time of reporting the outcome of the events was unspecified.

Follow up information was received on 19 May 2010.

The subject experienced erythrodermia and hepatotoxic reaction (liver values 20-fold increased).

As additional possible triggering substance the subject received Diclophenac for back pain. Apart from that the subject received medications only rarely.

At the time of reporting the outcome of the events was unspecified.

Further information has been requested.

GlaxoSmithKline, 22.06.2010 DE-GlaxoSmithKline-D0067560A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of hepatotoxic reaction in a subject of unspecified age and gender who was vaccinated with swine flu split dresden (Pandemrix H1N1, GlaxoSmithKline) for prophylaxis.

In February 2010 the subject received unspecified dose of Pandemrix H1N1 (unknown route and injection site).

In 2010, an unspecified time after vaccination with Pandemrix H1N1, the subject experienced hepatotoxic reaction with fever and skin change.

The subject was hospitalised.

At the time of reporting the outcome of the events was unspecified.

Follow up information was received on 19 May 2010.

The subject experienced erythrodermia and hepatotoxic reaction (liver values 20-fold increased).

As additional possible triggering substance the subject received Diclophenac for back pain. Apart from that the subject received medications only rarely.

At the time of reporting the outcome of the events was unspecified.

Further information has been requested.

Written Follow up information was received on 21 June 2010 from the reporting physician: "Following allergy anamnesis and diagnostics, the subject's complaints are no longer suspected to be related to the vaccination with Pandemrix. Skin changes and diarrhea had occurred prior to the vaccination, possibly as a sign of intolerance to diclophenac. Allergy testing with thiomersal and diclophanc were negative. The adverse event report should be revoked."

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10091530

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19541117	Age: 55 Year	weight (kg): 85	BMI_int: 28.08	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Discolouration urine	20100605			3 Day	Unknown
2 Vomiting	20100605			3 Day	recovering/resolving
3 Generalized pruritus	20100605			3 Day	Unknown
4 Intervertebral disc prolapse					Unknown
5 Hepatic lesion	20100605			3 Day	recovering/resolving
6 Itching					recovering/resolving
7 Cholestasis	20100605			3 Day	recovering/resolving
8 Nausea	20100605			3 Day	recovering/resolving
9 Urine discolouration					recovering/resolving
10 Hepatic lesion NOS	20100605				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 katadolon s long flupirtine maleate	Oral	400 Mg milligram(s)/Day 212	20100603-20100605 3 Day	Back pain
2 musaril tetrazepam	Oral	25 Mg milligram(s)/Day 82	20100603-20100605 3 Day	Back pain
3 pantozol pantoprazole sodium	Oral	20 Mg milligram(s)/Day 245	20100603-20100605 3 Day	Gastric ulcer prophylaxis
4 PANTOZOL pantoprazole	Intravenous (not otherwis	/ 245	20100612-20100614 3 Day	
5 voltaren diclofenac sodium	Oral	/ 245	20100611- -	Pain
6 voltaren resinat diclofenac resinate	Oral	75 Mg milligram(s)/Day 6	20100603-20100605 3 Day	Back pain

Drugs (concomitant)

7 ramipril ramipril	Oral	2.5 Mg milligram(s)/Day 245	200909- -	Hypertension arterial
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain	20100511		Yes	untere LWS mit Ausstrahlung ins linke Bein
Hypertension arterial	200909		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hepatitis A antibody	20100608	-	positiv/neg;	
Anti-HBc antibody	20100608	-	positiv/neg;	
Hepatitis C antibody	20100608	-	positiv/neg;	
Antinuclear antibody	20100610	-	positiv/neg;	

Antineutrophil cytoplasmic antibody	20100610	-	positiv/neg;
Antimitochondrial antibody	20100610	-	positiv/neg;
Haemoglobin	20100608	9.62	mmol/l
Haematocrit	20100608	0.47	NA
Mean corpuscular haemoglobin	20100608	87.6	fl
Total white blood cell count	20100608	5.51	Gpt/l
Thrombocyte count	20100608	249	Gpt/l
Quick's test	20100608	86.5	Prozent
INR	20100608	1.09	NA
Activated partial thromboplastin time	20100608	31.8	sec
Fibrinogen	20100608	2.33	g/l
Sodium	20100608	141	mmol/l
Potassium	20100608	4.74	mmol/l
Calcium	20100608	2.52	mmol/l
Creatinine	20100608	62.8	μmol/l
Aspartate aminotransferase	20100608	19.69	μmol/l.s
Aspartate aminotransferase	20100610	13.62	μmol/l.s
Aspartate aminotransferase	20100617	9	μmol/l.s
Alanine aminotransferase	20100608	42.04	μmol/s.l.
Alanine aminotransferase	20100610	32.19	μmol/s.l.
Alanine aminotransferase	20100617	21.27	μmol/s.l.
Alkaline phosphatase	20100608	4.31	μmol/l
Alkaline phosphatase	20100610	3.99	μmol/l
Alkaline phosphatase	20100617	3.83	μmol/l
Gamma GT	20100608	7.22	μmol/s.l
Gamma GT	20100610	8.09	μmol/s.l
Gamma GT	20100617	12.23	μmol/s.l
C-reactive protein	20100608	8.7	mg/l
C-reactive protein	20100610	6.9	mg/l
C-reactive protein	20100617	5.6	mg/l
Bilirubin total	20100608	68.4	μmol/l
Bilirubin total	20100610	46.9	μmol/l
Bilirubin total	20100617	46.2	μmol/l
Bilirubin direct	20100608	48.1	μmol/l
Bilirubin direct	20100610	37.1	μmol/l
Bilirubin direct	20100617	36.4	μmol/l
Aspartate aminotransferase	20100611	12.24	μmol/l.s
Aspartate aminotransferase	20100612	14.11	μmol/l.s
Aspartate aminotransferase	20100614	11.34	μmol/l.s
Aspartate aminotransferase	20100615	11.39	μmol/l.s
Alanine aminotransferase	20100611	27.35	μmol/s.l.
Alanine aminotransferase	20100612	28.94	μmol/s.l.
Alanine aminotransferase	20100614	22.29	μmol/s.l.
Alanine aminotransferase	20100615	23.18	μmol/s.l.

DE-BFARM-10091530			
Alkaline phosphatase	20100611	3.99	µmol/l
Alkaline phosphatase	20100612	4.56	µmol/l
Alkaline phosphatase	20100614	3.63	µmol/l
Alkaline phosphatase	20100615	3.87	µmol/l
Gamma GT	20100611	8.21	µmol/s.l
Gamma GT	20100612	10.42	µmol/s.l
Gamma GT	20100614	10.24	µmol/s.l
Gamma GT	20100615	11.36	µmol/s.l
Bilirubin total	20100611	44.2	µmol/l
Bilirubin total	20100612	54.2	µmol/l
Bilirubin total	20100614	51.4	µmol/l
Bilirubin total	20100615	55.1	µmol/l
Bilirubin direct	20100611	35.2	µmol/l
Bilirubin direct	20100612	44.4	µmol/l
Bilirubin direct	20100614	38.9	µmol/l
Bilirubin direct	20100615	43.5	µmol/l
Alanine aminotransferase	20100628	5.94	µmol/s.l.
Alkaline phosphatase	20100628	2.49	µmol/l
Aspartate aminotransferase	20100628	1.35	µmol/l.s
Bilirubin total	20100612	54.5	µmol/l
Bilirubin total	20100628	21.3	µmol/l
Gamma GT	20100628	10.55	µmol/s.l
Haematocrit	20100608	0.47	NA
INR	20100608	1.09	NA
Alanine aminotransferase	20100608	42.04	
Alkaline phosphatase	20100608	4.31	umol/l
Aspartate aminotransferase	20100608	19.69	
Bilirubin direct	20100608	48.1	umol/l
Bilirubin total	20100608	68.4	umol/l
C-reactive protein	20100608	8.7	mg/l
Gamma GT	20100608	7.22	
Haematocrit	20100608	0.47	
Haemoglobin	20100608	9.62	mmol/l
Quick's test	20100608	86.5 Prozent	
MRI	20100609		
Ultrasonography	20100609		
Alanine aminotransferase	20100610	32.19	
Alkaline phosphatase	20100610	3.99	umol/l
Aspartate aminotransferase	20100610	13.62	
Bilirubin direct	20100610	37.1	umol/l
Bilirubin total	20100610	48.9	umol/l
C-reactive protein	20100610	6.9	mg/l
Gamma GT	20100610	8.09	
Alanine aminotransferase	20100611	27.35	

Alkaline phosphatase	20100611	3.99	umol/l
Aspartate aminotransferase	20100611	12.24	
Bilirubin direct	20100611	35.2	umol/l
Gamma GT	20100611	8.21	
Alanine aminotransferase	20100612	28.94	
Alkaline phosphatase	20100612	4.56	umol/l
Aspartate aminotransferase	20100612	14.11	
Bilirubin direct	20100612	44.4	umol/l
Gamma GT	20100612	10.42	
Alanine aminotransferase	20100614	22.29	
Alkaline phosphatase	20100614	3.63	umol/l
Aspartate aminotransferase	20100614	11.34	
Bilirubin direct	20100614	38.9	umol/l
Gamma GT	20100614	10.24	
Alanine aminotransferase	20100615	23.15	
Alkaline phosphatase	20100615	3.87	umol/l
Aspartate aminotransferase	20100615	11.39	
Bilirubin direct	20100615	43.5	umol/l
Gamma GT	20100615	11.36	
Alanine aminotransferase	20100617	21.27	
Alkaline phosphatase	20100617	3.83	umol/l
Aspartate aminotransferase	20100617	9	
Bilirubin direct	20100617	36.4	umol/l
Bilirubin total	20100617	46.2	umol/l
C-reactive protein	20100617	5.6	mg/l
Gamma GT	20100617	12.23	
Alanine aminotransferase	20100628	5.94	
Aspartate aminotransferase	20100628	1.35	
Gamma GT	20100628	10.55	

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 22.06.2010 DE-PVZIBE-07-10370

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschädigung NNB

KURZZUSAMMENFASSUNG:

Leberschädigung unter Flupirtinmaleat, Diclofenac, Pantozol und Tetrazepam

BESCHREIBUNG:

Die Patientin wurde am 08.06.10 primär wegen Rückenschmerzen mit Ausstrahlung ins linke Bein in der Neurologie aufgenommen. Dort stellte sich laborchemisch eine Leberzellschädigung und Cholestase dar, die zunächst akut abgeklärt werden musste. Dazu wurde die Patientin auf die Innere Station verlegt. Die Patientin nahm vom 03. - 05.06.10 wegen der Rückenschmerzen Katadolon S long, Voltaren resinat, Musaril und Pantozol ein. Da am 05.06.10 Übelkeit und Erbrechen auftraten, setzte die Patientin diese Medikamente selbstständig ab. Seit 05.06.10 bemerkte die Patientin eine Braunfärbung des Urins und einen Juckreiz am ganzen Körper.

THERAPIE:

- weitere Katadoloneinnahme untersagt, gegen Schmerzen 4x40 Tropfen Novalgin
- am 08.06.10: 2x1 Beutel Colestyramin a 4 g
- 12.-14.05.10: 1000 ml Sterofundin i.v., 1x40 mg Pantozol i.v.
- 13.-14.06.10: 1000 ml Glucose 5 %

VERLAUF:

- 09.06.10 Oberbauchsonographie: Pankreas luftüberlagert, nicht einsehbar. Leber echoarm, ohne fokale Läsionen. Gallenblase steinfrei, Wand unauffällig. DHC zart. Intrahepatisch kein Anhalt für Cholestase.

- 09.06.10 MRT der LWS nativ: Links mediolateraler Bandscheibenvorfall bei LWK 4/5 mit deutlicher Kompression der Wurzel L 5.

Aufgrund des Bandscheibenvorfalles erfolgte zur eventuellen chirurgischen Therapie die Verlegung der Patientin auf eine traumatologische Station. Von einer Operation wurde wegen der Leberwerte abgesehen. Zur Schmerztherapie bekam die Patientin am 11.06.10 eine Tablette Voltaren. Daraufhin reagierte die Patientin wieder mit Übelkeit und Erbrechen. Sie wurde auf die Innere Station zurückverlegt. Hier bekam sie daher Infusionstherapie und Pantozol i.v.. Laborchemisch kam es ab 12.06.10 zu einer vorübergehenden Verschlechterung der Leberwerte, wahrscheinlich wegen der Voltareneinnahme und eventuell zusätzlich durch die Pantozolgabe (siehe Laborwert vom 14.06.10).

Die Leberwerte besserten sich im stationären Verlauf langsam. Patientin ist derzeit noch stationär. Gallenwegs- und Gallenblasenerkrankung, Hepatitis A/B/C, Borreliose und Autoimmunhepatitis wurden ausgeschlossen.

ERLÄUTERUNGEN ZUR UAW:

Die initiale Aufnahme der Patientin erfolgte in der Neurologie wegen Rückenschmerzen. Aufgrund der dort festgestellten Leberzellschädigung und Cholestase erfolgte eine Verlegung in die Innere Klinik bei V.a. Katadolonnebenwirkung. Da bei der weiteren Diagnostik eine Gallenwegs-, Gallenblasenerkrankung, eine Hepatitis A/B/C und eine Autoimmunhepatitis ausgeschlossen werden konnten, wurden der dringende V.a. auf eine UAW von Katadolon von den behandelnden Ärzten geäußert. Da zur gleichen Zeit auch Voltaren resinat, Musaril u. Pantozol eingenommen wurden, müssen diese Medikamente ebenso mitverdächtig werden.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: katadolon s long

BETREFFENDE UAW: Leberschädigung NNB

ERLÄUTERUNG: In der Fachinformation sind Transaminasenerhöhungen, arzneimittelinduzierte Hepatitis (akut oder chronisch, ikterische oder anikterisch) als sehr seltene Nebenwirkung beschrieben.

Am 3. Einnahmetag begannen die Symptome. Aufgrund von Übelkeit und Erbrechen setzte die Patientin selbstständig das Medikament ab. Die pathologischen Werte besserten sich langsam. Eine Gallenblasen-, Gallenwegserkrankung, Hepatitis A/B/C und Autoimmunhepatitis wurden ausgeschlossen, trotzdem ist eine nichtmedikamentös bedingte Lebererkrankung nicht völlig auszuschließen, EBV wurde nicht getestet.

MEDIKAMENT: voltaren resinat

BETREFFENDE UAW: Leberschädigung NNB

ERLÄUTERUNG: In der Fachinformation sind Transaminasenerhöhungen als häufige, Leberschäden, akute Hepatitis mit und ohne Ikterus als gelegentlich auftretende Nebenwirkungen beschrieben.

Am 3. Einnahmetag begannen die Symptome. Aufgrund von Übelkeit und Erbrechen setzte die Patientin das Medikament ab.

sanofi-aventis, 01.07.2010 DE-SANOFI-AVENTIS-2010SA036412

Sender diagnosis: not reported

Narrative

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10091530) on 24-Jun-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This case concerns a 55-year-old female patient with unreported medical history. Concomitantly she suffered from arterial hypertension treated with ramipril.

For back pain the patient had started a daily treatment with tetrazepam (Musaril) 25 mg, flupirtine maleate 400 mg, diclofenac potassium 75 mg daily and pantoprazole sodium 20 mg on 03-Jun-2010.

On 05-Jun-2010 the patient developed nausea and vomiting and discontinued the medication for back pain. On the same day she noticed brown discolouration of urine and itching on the whole body.

On 08-Jun-2010 the patient was hospitalised into the neurological ward due to the back pain with radiating to the left leg. The laborochemical investigation disclosed liver cell damage and cholestasis so that the patient was transferred to the internal ward.

Metamizole 40 drop, four times a day, against the pain was initiated. The treatment of hepatic lesion included colestyramine 4 g twice and intravenous sterofundin 1000 ml on 08-Jun-2010. Three days later the patient received diclofenac potassium (1 tablet) and reacted with nausea and vomiting again. Consequently infusion NOS and pantoprazole sodium 40 mg were administered from 12-Jun-2010 to 14-Jun-2010. Glucose 5% 1000 ml from 13-Jun-2010 to 14-Jun-2010. Laboratory tests revealed transient worsening of liver values (probably due to intake of diclofenac and perhaps due to intake of pantoprazole (values from 14-JUN-2010). In the further course the liver values were slowly decreasing. A disease of bile ducts and gall bladder, hepatitis A/B/C, borreliosis and auto immune hepatitis were excluded.

Reporter causality (WHO score): possible

Explanatory notes on ADR: The initial admission to the hospital (neurological ward) was due to back pain. Due to the liver cell damage and cholestasis, which were diagnosed, the patient was transferred to internal ward with the suspected ADR of flupirtine maleate (because bile duct and gallbladder disease, hepatitis A,B,C, borreliosis and immune hepatitis were excluded). Due to the fact that at the same time diclofenac, tetrazepam

and pantoprazole were taken, these drugs should be suspected too.

NOVARTIS, 07.07.2010 DE-002147023-PHHY2010DE41996

Sender diagnosis: not reported

Narrative

Initial report received from regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE-BFARM-10091530) on 23 Jun 2010: This patient had a concurrent condition of hypertension arterial since Sep 2009. This patient with back pain from 11 May 2010, was admitted to the Neurology Department on 08 Jun 2010 primarily due to back pain radiating to the left leg. This patient started treatment with Katadolon S. long (flupirtine maleate), Voltaren Resinate (diclofenac resinate), Musaril (tetrazepam) and Pantozol (pantoprazole) from 03 Jun 2010 for the back pain which are all suspects in this case. After hospitalization the laboratory tests revealed the presence of hepatocellular lesion and cholestasis which initially required acute evaluation. The patient was then transferred to the Internal Medicine Unit for this purpose. On 05 Jun 2010 the patient experienced nausea and vomiting therefore the treatment with these medications was discontinued on her own initiative. On 05 Jun 2010 the patient noticed brown discoloration of the urine and pruritus affecting the entire body. Due to the intervertebral disk prolapse, the patient was referred to a traumatology unit in preparation for possible surgical therapy. However, no surgical procedure was performed because of the liver function values. For the treatment of pain the patient was given one tablet of Voltaren (diclofenac sodium) on 11 Jun 2010. The patient again responded to this medication with nausea and vomiting and was transferred back to the Department of Internal Medicine where she received infusion therapy and Pantozol i.v., Novalgin (metamizole) 4x 40 drops and cholestyramine 4 g. Laboratory tests from 12 Jun 2010 onwards showed a temporary worsening of the liver function values, probably due to taking the Voltaren and possibly also because of the administration of Pantozol (on 14 Jun 2010 the liver function tests were: Aspartate amino transferase: 11.34 umol/l.s, Alanine aminotransferase: 22.29 umol/l.s, alkaline phosphatase: 3.63 umol/l, Gamma GT: 10.24 umol/s.l). The liver function values gradually improved during the further hospitalized course. The patient is currently still hospitalized. Bile duct disease and gallbladder disease, hepatitis A/B/C, borreliosis and autoimmune hepatitis were ruled out. The outcome of the event hepatic lesions was reported as condition unchanged where as of all other events was not reported. The causality assessment of hepatic lesions was reported to have a possible relationship with Katadolon, Voltaren resinate, Pantozol and Musaril. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event "cholestasis, intervertebral disc protrusion, nausea and vomiting" and "Voltaren Resinate, Pantozol and Voltaren tab" (e.g. Neoral) as suspected.

HELIOS Klinikum Wuppertal, 01.10.2010 DE-PVZIBE-07-10370

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschädigung NNB

KURZZUSAMMENFASSUNG:
Leberschädigung unter Flupirtinmaleat, Diclofenac, Pantozol und Tetrazepam
BESCHREIBUNG:

Die Patientin wurde am 08.06.10 primär wegen Rückenschmerzen mit Ausstrahlung ins linke Bein in der Neurologie aufgenommen. Dort stellte sich laborchemisch eine Leberzellschädigung und Cholestase dar, die zunächst akut abgeklärt werden musste. Dazu wurde die Patientin auf die Innere Station verlegt. Die Patientin nahm vom 03. - 05.06.10 wegen der Rückenschmerzen Katadolon S long, Voltaren resinat, Musaril und Pantozol ein. Da am 05.06.10 Übelkeit und Erbrechen auftraten, setzte die Patientin diese Medikamente selbstständig ab. Seit 05.06.10 bemerkte die Patientin eine Braunfärbung des Urins und einen Juckreiz am ganzen Körper.

THERAPIE:

- weitere Katadoloneinnahme untersagt, gegen Schmerzen 4x40 Tropfen Novalgin
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VERLAUF:

- 09.06.10 Oberbauchsonographie: Pankreas luftüberlagert, nicht einsehbar. Leber echoarm, ohne fokale Läsionen. Gallenblase steinfrei, Wand unauffällig. DHC zart. Intrahepatisch kein Anhalt für Cholestase.
- 09.06.10 MRT der LWS nativ: Links mediolateraler Bandscheibenvorfall bei LWK 4/5 mit deutlicher Kompression der Wurzel L 5. Aufgrund des Bandscheibenvorfalles erfolgte zur eventuellen chirurgischen Therapie die Verlegung der Patientin auf eine traumatologische Station. Von einer Operation wurde wegen der Leberwerte abgesehen. Zur Schmerztherapie bekam die Patientin am 11.06.10 eine Tablette Voltaren. Daraufhin reagierte die Patientin wieder mit Übelkeit und Erbrechen. Sie wurde auf die Innere Station zurückverlegt. Hier bekam sie daher Infusionstherapie und Pantozol i.v.. Laborchemisch kam es ab 12.06.10 zu einer vorübergehenden Verschlechterung der Leberwerte, wahrscheinlich wegen der Voltareneinnahme und eventuell zusätzlich durch die Pantozolgabe (siehe Laborwert vom 14.06.10).
- Die Leberwerte besserten sich im stationären Verlauf langsam. Patientin ist derzeit noch stationär. Gallenwegs- und Gallenblasenerkrankung, Hepatitis A/B/C, Borreliose und Autoimmunhepatitis wurden ausgeschlossen.

ERLÄUTERUNGEN ZUR UAW:

Die initiale Aufnahme der Patientin erfolgte in der Neurologie wegen Rückenschmerzen. Aufgrund der dort festgestellten Leberzellschädigung und Cholestase erfolgte eine Verlegung in die Innere Klinik bei V.a. Katadolonnebenwirkung. Da bei der weiteren Diagnostik eine Gallenwegs-, Gallenblasenerkrankung, eine Hepatitis A/B/C und eine Autoimmunhepatitis ausgeschlossen werden konnten, wurden der dringende V.a. auf eine

UAW von Katadolon von den behandelnden Ärzten geäußert. Da zur gleichen Zeit auch Voltaren resinat, Musaril u. Pantozol eingenommen wurden, müssen diese Medikamente ebenso mitverdächtig werden. Bei der Schmerztherapie handelt es sich um eine erstmalige Behandlung.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: katadolon s long

BETREFFENDE UAW: Leberschaedigung NNB

ERLÄUTERUNG: In der Fachinformation sind Transaminasenerhöhungen, arzneimittelinduzierte Hepatitis (akut oder chronisch, ikterische oder anikterisch) als sehr seltene Nebenwirkung beschrieben.

Am 3. Einnahmetag begannen die Symptome. Aufgrund von Übelkeit und Erbrechen setzte die Patientin selbstständig das Medikament ab. Die pathologischen Werte besserten sich langsam. Eine Gallenblasen-, Gallenwegserkrankung, Hepatitis A/B/C und Autoimmunhepatitis wurden ausgeschlossen, trotzdem ist eine nichtmedikamentös bedingte Lebererkrankung nicht völlig auszuschließen, EBV wurde nicht getestet.

MEDIKAMENT: voltaren resinat

BETREFFENDE UAW: Leberschaedigung NNB

ERLÄUTERUNG: In der Fachinformation sind Transaminasenerhöhungen als häufige, Leberschäden, akute Hepatitis mit und ohne Ikterus als gelegentlich auftretende Nebenwirkungen beschrieben.

Am 3. Einnahmetag bega<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 13.10.2010 DE-SANOFI-AVENTIS-2010SA036412

Sender diagnosis: not reported

Narrative

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10091530) on 24-Jun-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This case concerns a 55-year-old female patient with unreported medical history. Concomitantly she suffered from arterial hypertension treated with ramipril.

For back pain the patient had started a daily treatment with tetrazepam (Musaril) 25 mg, flupirtine maleate 400 mg, diclofenac potassium 75 mg daily and pantoprazole sodium 20 mg on 03-Jun-2010.

On 05-Jun-2010 the patient developed nausea and vomiting and discontinued the medication for back pain. On the same day she noticed brown discolouration of urine and itching on the whole body.

On 08-Jun-2010 the patient was hospitalised into the neurological ward due to the back pain with radiating to the left leg. The laborchemical investigation disclosed liver cell damage and cholestasis so that the patient was transferred to the internal ward.

Metamizole 40 drop, four times a day, against the pain was initiated. The treatment of hepatic lesion included colestyramine 4 g twice and intravenous sterofundin 1000 ml on 08-Jun-2010. Three days later the patient received diclofenac potassium (1 tablet) and reacted with nausea and vomiting again. Consequently infusion NOS and pantoprazole sodium 40 mg were administered from 12-Jun-2010 to 14-Jun-2010. Glucose 5% 1000 ml from 13-Jun-2010 to 14-Jun-2010. Laboratorytests revealed transient worsening of liver values (probably du to intake of diclofenac and perhaps due to intake of pantoprazole (values from 14-JUN-2010). In the further course the liver values were slowly decreasing. A disease of bile ducts and gall bladder, hepatitis A/B/C, borreliosis and auto immune hepatitis were excluded.

Reporter causality (WHO score): possible

Explanatory notes on ADR: The initial admission to the hospital (neurological ward) was due to back pain. Due to the liver cell damage and cholestasis, which were diagnosed, the patient was transferred to internal ward with the suspected ADR of flupirtine maleate (because bile duct and gallbladder disease, hepatitis A,B,C, borreliosis and immune hepatitis were excluded). Due to the fact that at the same time diclofenac, tetrazepam and pantoprazole were taken, these drugs should be suspected too.

Additional Information was extracted from the website of the German health authority on 05-Oct-2010:

Laboratory values were added from 28-Jun-2010.

Explanatory notes on ADR: The pain therapy was a first-time treatment. Liver injruy as company diagnosis was added due to laboratory values.

Sender's comment: No further additional information is expected.

NOVARTIS, 14.10.2010 DE-002147023-PHHY2010DE41996

Sender diagnosis: not reported

Narrative

Initial report received from regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE-BFARM-10091530) on 23 Jun 2010: This patient had a concurrent condition of hypertension arterial since Sep 2009. This patient with back pain from 11 May 2010, was admitted to the Neurology Department on 08 Jun 2010 primarily due to back pain radiating to the left leg. This patient started treatment with Katadolon S. long (flupirtine maleate), Voltaren Resinate (diclofenac resinate), Musaril (tetrazepam) and Pantozol (pantoprazole) from 03 Jun 2010 for

the back pain which are all suspects in this case. After hospitalization the laboratory tests revealed the presence of hepatocellular lesion and cholestasis which initially required acute evaluation. The patient was then transferred to the Internal Medicine Unit for this purpose. On 05 Jun 2010 the patient experienced nausea and vomiting therefore the treatment with these medications was discontinued on her own initiative. On 05 Jun 2010 the patient noticed brown discoloration of the urine and pruritus affecting the entire body. Due to the intervertebral disk prolapse, the patient was referred to a traumatology unit in preparation for possible surgical therapy. However, no surgical procedure was performed because of the liver function values. For the treatment of pain the patient was given one tablet of Voltaren (diclofenac sodium) on 11 Jun 2010. The patient again responded to this medication with nausea and vomiting and was transferred back to the Department of Internal Medicine where she received infusion therapy and Pantozol i.v., Novalgin (metamizole) 4x 40 drops and cholestyramine 4 g. Laboratory tests from 12 Jun 2010 onwards showed a temporary worsening of the liver function values, probably due to taking the Voltaren and possibly also because of the administration of Pantozol (on 14 Jun 2010 the liver function tests were: Aspartate amino transferase: 11.34 umol/l.s, Alanine aminotransferase: 22.29 umol/l.s, alkaline phosphatase: 3.63 umol/l, Gamma GT: 10.24 umol/s.l). The liver function values gradually improved during the further hospitalized course. The patient is currently still hospitalized. Bile duct disease and gallbladder disease, hepatitis A/B/C, borreliosis and autoimmune hepatitis were ruled out. The outcome of the event hepatic lesions was reported as condition unchanged where as of all other events was not reported. The causality assessment of hepatic lesions was reported to have a possible relationship with Katadolon, Voltaren resinate, Pantozol and Musaril. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event "cholestasis, intervertebral disc protrusion, nausea and vomiting" and "Voltaren Resinate, Pantozol and Voltaren tab" as suspected.

Follow-up was received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority on 04 Oct 2010: The lab data showed bilirubin direct at 43.5 umol/l on 15 Jun 2010 and aspartate aminotransferase at 1.35 umol/l.s, alanine aminotransferase at 5.94 umol/l.s, alkaline phosphatase at 2.49 umol/l, gamma GT at 10.55 umol/l.s, bilirubin total at 21.3 umol/l on 28 Jun 2010. The outcome of the event hepatic lesion was reported as recovering /resolving.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10099350

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth: 19690115	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicidal intention					recovered/resolved
2 Elevated liver enzymes					recovered/resolved
3 Drug intoxication					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Alcohol		/	-	
ethanol			-	
2 DICLOFENAC		/	-	
diclofenac diethylamine			-	
3 IBUPROFEN		/	-	
ibuprofen			-	
4 LOPERAMIDE		/	-	
loperamide hydrochloride			-	
5 pantozol 40 mg		1200 Mg milligram(s)/	-	
pantoprazole			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Nycomed, 06.07.2010 DE-Nycomed-0212173

Sender diagnosis: not reported

Narrative

The case was reported by Nycomed Germany with the ref.no. AZ2010009529.

It concerned a patient born on 15-JAN-1969 (gender not reported) who took 1.2 g at once (30 tablets of 40 mg) of Pantozol (pantoprazole) associated with loperamide, ibuprofen, diclofenac and a big amount of alcohol (mixed drug intoxication with suicidal intention) on an unspecified date. Neither medical history nor concomitant treatment was reported. The patient was admitted in stationary treatment and was monitored with ECG and respiratory monitoring. About 11 hours after the intake, the patient was awake, orientated and in good general condition. Lightly increased liver values (GOT, GPT and Gamma-GT) were detected through laboratory values (no values provided). It was specified that an alcohol abuse could not be ruled out.

Outcome of the adverse reactions was not reported.

are involved and are usually not considered to have any informative value in terms of the risk profiles of the drugs taken. Drug intoxication or combined drug intoxication is due to the simultaneous use of multiple drugs. Signs of drug intoxication include heart failure (arrhythmias and shock), respiratory failure, lack of consciousness, confusion, convulsions.

The causal relationship between pantoprazole and the adverse reaction of suicidal intention is considered not related and rather related to one of the above mentioned causes.

Gamma-glutamyl transferase levels rise in hepatobiliary disease, especially cholestasis, and correlates loosely with levels of alkaline phosphatase and 5'-nucleotidase. Levels also increase with induction of microsomal enzymes (e.g. by ingesting certain drugs, such as anticonvulsants, and particularly in alcoholics). An increase in serum GGT also predicts onset of metabolic syndrome or congestive heart failure. Aminotransferases (alanine aminotransferase and aspartate aminotransferase) increase when there is damage to the liver cell membrane resulting in increased permeability and therefore increase of these enzymes in blood. Acute hepatocellular necrosis or injury (e.g. hepatitis) typically results in high values. Modest elevations persist in chronic liver diseases. ASAT (GOT) may be elevated because of rhabdomyolysis or damage to heart or brain tissue. In most acute hepatocellular disorders, the ALAT (GPT) is higher than or e

Reporter's comment:Nycomed, 09.07.2010 DE-Nycomed-0212173

Sender diagnosis: not reported

Narrative

The case was reported by Nycomed Germany with the ref.no. AZ2010009529.

It concerned a patient born on 15-JAN-1969 (gender not reported) who took 1.2 g at once (30 tablets of 40 mg) of Pantozol (pantoprazole) associated with loperamide, ibuprofen, diclofenac and a big amount of alcohol (mixed drug intoxication with suicidal intention) on an unspecified date. Neither medical history nor concomitant treatment was reported. The patient was admitted in stationary treatment and was monitored with ECG and respiratory monitoring. About 11 hours after the intake, the patient was awake, orientated and in good general condition. Lightly increased liver values (GOT, GPT and Gamma-GT) were detected through laboratory values (no values provided). It was specified that an alcohol abuse could not be ruled out.

Outcome of the adverse reactions was not reported.

Follow-up information received on 07-JUL-2010:

The patient was discharged from hospital after 24 hours of observation without having any complications.

above mentioned causes.

Gamma-glutamyl transferase levels rise in hepatobiliary disease, especially cholestasis, and correlates loosely with levels of alkaline phosphatase and 5'-nucleotidase. Levels also increase with induction of microsomal enzymes (e.g. by ingesting certain drugs, such as anticonvulsants, and particularly in alcoholics). An increase in serum GGT also predicts onset of metabolic syndrome or congestive heart failure. Aminotransferases (alanine aminotransferase and aspartate aminotransferase) increase when there is damage to the liver cell membrane resulting in increased permeability and therefore increase of these enzymes in blood. Acute hepatocellular necrosis or injury (e.g. hepatitis) typically results in high values. Modest elevations persist in chronic liver diseases. ASAT (GOT) may be elevated because of rhabdomyolysis or damage to heart or brain tissue. In most acute hepatocellular disorders, the ALAT (GPT) is higher than or e

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10102052

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth:	Age: 78 Year	weight (kg): 76	BMI_int: 29.69	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Confusional state	2009				Unknown
2 Dementia	2009				Unknown
3 Urinary tract infection	200912				Unknown
4 GOT increased transient	20091228				recovered/resolved
5 GPT increased transient	20091228				recovered/resolved
6 Lactate dehydrogenase increased	20091224				Unknown
7 Normochromic normocytic anaemia	20091224				recovering/resolving
8 Pain ankle	2009				recovered/resolved
9 Renal failure acute	200912				recovered/resolved
10 Supraventricular tachycardia	20091228				recovering/resolving
11 Ankle swelling	2009				Unknown
12 Infection	2010				Unknown
13 Exsiccosis	200912				recovered/resolved
14 Gamma-glutamyltransferase increased	20091228				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-200912	
diclofenac			-	
2 JANUVIA	Oral	/	20091201-200912	Type 2 diabetes mellitus
sitagliptin phosphate		245	-	

Drugs (concomitant)

3 Diuretikum	Unknown	/	-	Hypertension arterial
			-	
4 metformin	Oral	/	-200912	Type 2 diabetes mellitus
metformin		245	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Depression			Yes	
Dietary control			Yes	diabetes, high cholesterol
Peripheral arterial occlusive disease			Yes	
Anaemia			Yes	
Aortic sclerosis			Yes	
Cardiomegaly			Yes	
Coronary artery sclerosis			Yes	
Gallbladder stone			Yes	
Hypertension arterial			Yes	
Overweight			Yes	
Cholesterol high			Yes	
Anxiety disorder			Yes	
Recurrent urinary tract infection			No	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit weitere Erläuterung
Serum creatinine	201003	0.76	mg/dl
Albumin urine	201006	<10	mg/l
Serum creatinine		0.86	mg/dl
Blood urea	20091224	291.0	mg/dl
Blood urea	20091227	152.0	mg/dl
Blood urea	20091226	237.0	mg/dl
Blood urea	20100112	48.0	mg/dl
Blood urea	20100106	35.0	mg/dl
Blood urea	20100105	33.0	mg/dl
Blood urea	20091228	121.0	mg/dl
Blood urea	20100107	51.0	mg/dl
Blood urea	20100114	41.0	mg/dl
C-reactive protein	20091224	1.0	mg/dl
C-reactive protein	20091227	11.1	mg/dl
C-reactive protein	20091226	5.4	mg/dl
C-reactive protein	20100106	28.3	mg/dl
C-reactive protein	20100107	25.6	mg/dl
C-reactive protein	20100112	5.6	mg/dl
C-reactive protein	20100114	3.6	mg/dl
C-reactive protein	20091228	13.9	mg/dl
Electrocardiogram	20091230	sinus rhythm, heart rate 105 bpm, reduced R wave t	
Electrocardiogram	20091228	control ECG: sinus rhythm, heart rate 108 bpm, red	
Electrocardiogram	20091224	heart rate 47 bpm, broad based ventricular complex	
Electrocardiogram	20091228	regular tachycardia, heart rate 190 bpm	
Glomerular filtration rate	20091228	43	ml/min/1.73
Glomerular filtration rate	20100105	>60	ml/min/1.73
Glomerular filtration rate	20100106	>60	ml/min/1.73
Glomerular filtration rate	20100107	>60	ml/min/1.73
Glomerular filtration rate	20091227	27	ml/min/1.73
Glomerular filtration rate	20100114	>60	ml/min/1.73
Glomerular filtration rate	20091226	13	ml/min/1.73
Glomerular filtration rate	20100112	>60	ml/min/1.73
Glomerular filtration rate	20091224	7	ml/min/1.73
Haematocrit	20100106	0.30	units NA
Haematocrit	20100107	0.29	units NA
Haematocrit	20091224	0.30	units NA
Haematocrit	20100114	0.32	units NA
Haematocrit	20091226	0.28	units NA
Haematocrit	20091228	0.33	units NA
Haematocrit	20100112	0.31	units NA

Haematocrit	20091227	0.28	units NA
Haemoglobin	20100106	9.4	g/dl
Haemoglobin	20100114	9.7	g/dl
Haemoglobin	20100112	9.4	g/dl
Haemoglobin	20100107	9.1	g/dl
Haemoglobin	20091224	9.6	g/dl
Haemoglobin	20091227	9.3	g/dl
Haemoglobin	20091228	10.4	g/dl
Haemoglobin	20091226	9.6	g/dl
Mean cell haemoglobin concentration	20100106	31.1	g/dl
Mean cell haemoglobin concentration	20091226	34.3	g/dl
Mean cell haemoglobin concentration	20091227	33.1	g/dl
Mean cell haemoglobin concentration	20100112	30.6	g/dl
Mean cell haemoglobin concentration	20091224	31.9	g/dl
Mean cell haemoglobin concentration	20100107	31.0	g/dl
Mean cell haemoglobin concentration	20100114	30.1	g/dl
Mean cell haemoglobin concentration	20091228	31.2	g/dl
Protein urine 24 hour	20091228	665.60	mg/d
Red blood cell count	20091227	3.19	/pl
Red blood cell count	20100107	3.12	/pl
Red blood cell count	20091224	3.27	/pl
Red blood cell count	20091226	3.24	/pl
Red blood cell count	20100112	3.36	/pl
Red blood cell count	20100114	3.51	/pl
Red blood cell count	20091228	3.57	/pl
Red blood cell count	20100106	3.21	/pl
Serum creatinine	20100112	0.64	mg/dl
Serum creatinine	20091226	3.57	mg/dl
Serum creatinine	20091224	5.91	mg/dl
Serum creatinine	20100114	0.72	mg/dl
Serum creatinine	20100106	0.66	mg/dl
Serum creatinine	20100107	0.72	mg/dl
Serum creatinine	20100105	0.68	mg/dl
Serum creatinine	20091228	1.27	mg/dl
Serum creatinine	20091227	1.89	mg/dl
Serum parathyroid hormone	20091228	102	pg/ml
Serum potassium	20091224	7.3	mmol/l
Serum potassium	20100114	3.8	mmol/l
Serum potassium	20091228	4.6	mmol/l
Serum potassium	20100106	3.4	mmol/l
Serum potassium	20100105	3.8	mmol/l
Serum potassium	20100107	3.8	mmol/l
Serum potassium	20100112	3.4	mmol/l
Serum potassium	20091226	4.6	mmol/l

Serum potassium	20091227	4.1	mmol/l
Ultrasound abdomen	20091229	kidneys of age-appropriate size without evidence o	
White blood cell count	20091224	11.86	/nl
White blood cell count	20091228	15.51	/nl
White blood cell count	20091227	13.52	/nl
White blood cell count	20100114	7.76	/nl
White blood cell count	20100106	13.49	/nl
White blood cell count	20100107	12.33	/nl
White blood cell count	20091226	13.88	/nl
White blood cell count	20100112	7.90	/nl

Narratives (nur bei elektronischem Eingang)

009507513, 09.07.2010 DE-MERCK-1007DEU00037

Sender diagnosis: not reported

Narrative

Information has been received from a standardized non-interventional registry for therapy of type 2 diabetes mellitus in clinical practise (SIRTA). A physician provided information concerning a 78-year-old female patient with hypertension, coronary artery disease, peripheral arterial occlusive disease, depression, anxiety disorder, high cholesterol and dietary control (diabetes and high cholesterol) who, on 01-DEC-2009, was placed on therapy with sitagliptin phosphate, 100 mg tablet for the treatment of type 2 diabetes mellitus (total daily dose and exact duration not reported). Concomitant therapy included metformin, unspecified diuretic, unspecified ACE inhibitor, unspecified beta blocker and other unspecified antihypertensive agent.

In approximately February 2010, the patient developed acute renal failure (pre-therapy serum creatinine value - date not reported: 0.86 mg/dl) in the context of an infection (not further specified). She was hospitalized due to acute renal failure. In approximately February 2010, therapy with sitagliptin phosphate and metformin was discontinued and insulin was started. In March 2010, laboratory analysis revealed serum creatinine 0.76 mg/dl. In June 2010, urine dipstick test revealed urine albumin <10 mg/l. The outcome was not reported.

The reporting physician felt that acute renal failure and infection were related to therapy with sitagliptin phosphate.

Additional information has been requested.

009507513, 20.07.2010 DE-MERCK-1007DEU00037

Sender diagnosis: not reported

Narrative

Information has been received from a standardized non-interventional registry for therapy of type 2 diabetes mellitus in clinical practise (SIRTA). A physician provided information concerning a 78-year-old female patient with arterial hypertension, coronary artery disease, peripheral arterial occlusive disease, cholecystolithiasis, cardiomegaly, anemia, coronary artery sclerosis, aortic sclerosis, overweight, depression, recurrent urinary tract infections (five episodes), anxiety disorder, high cholesterol and dietary control (diabetes and high cholesterol) who, on 01-DEC-2009, was placed on therapy with sitagliptin phosphate, 100 mg tablet for the treatment of type 2 diabetes mellitus (total daily dose and exact duration not reported). Concomitant suspect long-term therapy included diclofenac (dose, duration and indication not reported). Other concomitant therapy included metformin, unspecified diuretic, unspecified ACE inhibitor, unspecified beta blocker and other unspecified antihypertensive agent.

In approximately February 2010, the patient developed acute renal failure (pre-therapy serum creatinine value - date not reported: 0.86 mg/dl) in the context of an urinary tract infection. She was hospitalized due to acute renal failure. In approximately February 2010, therapy with sitagliptin phosphate and metformin was discontinued and insulin was started. In March 2010, laboratory analysis revealed serum creatinine 0.76 mg/dl. In June 2010, urine dipstick test revealed urine albumin <10 mg/l. Subsequently, the patient recovered from acute renal failure. The outcome of urinary tract infection was not reported.

Follow-up information received from the reporting physician (providing medical records) on 14-JUL-2010 revealed that, on 24-DEC-2009 (previously reported as "in approximately February 2010"), the patient was hospitalized by an emergency physician due to deterioration of her general condition, disorientation, restlessness and lack of micturition (according to the patient's relatives, the symptoms had started some days before). Upon admission, laboratory analysis revealed serum potassium 7.3 mmol/l, serum creatinine 5.91 mg/dl, glomerular filtration rate (GFR) 7 ml/min/1.73, serum blood urea 291.0 mg/dl, C-reactive protein 1.0 mg/dl, WBC count 11.86/nl, RBC count 3.27/pl, hemoglobin 9.6 g/dl, hematocrit 0.30, MCHC 31.9 g/dl. ECG showed heart rate 47 bpm, broad based ventricular complexes, no definitely identifiable T waves, regular rhythm. A diagnosis of acute renal failure with hyperkalemia, anuria, metabolic acidosis and arrhythmia in the context of exsiccosis was made and the patient had to be temporarily monitored in the ICU. Secondary diagnosis included normochromic normocytic anemia. Subsequently, in December 2009 (previously reported as "in approximately

February 2010"), therapy with sitagliptin phosphate, diclofenac and metformin was discontinued. The patient was treated with electrolyte infusions which led to quick improvement of her condition. In the further course, her renal retention parameters completely normalized and she recovered from acute renal failure and exsiccosis. During hospitalization (previously reported as "in approximately February 2010"), the patient developed urinary tract infection (*E. coli*, *Citrobacter brakii*) and was treated with ciprofloxacin; the outcome was not reported.

On 26-DEC-2009, laboratory analysis revealed serum potassium 4.6 mmol/l, serum creatinine 3.57 mg/dl, glomerular filtration rate (GFR) 13 ml/min/1.73, serum blood urea 237.0 mg/dl, C-reactive protein 5.4 mg/dl, WBC count 13.88/nl, RBC count 3.24/pl, hemoglobin 9.6 g/dl, hematocrit 0.28, MCHC 34.3 g/d. On 27-DEC-2009, laboratory analysis revealed serum potassium 4.1 mmol/l, serum creatinine 1.89 mg/dl, glomerular filtration rate (GFR) 27 ml/min/1.73, serum blood urea 152.0 mg/dl, C-reactive protein 11.1 mg/dl, WBC count 13.52/nl, RBC count 3.19/pl, hemoglobin 9.3 g/dl, hematocrit 0.28, MCHC 33.1 g/d. On 28-DEC-2009, ECG showed regular tachycardia, heart rate 190 bpm (diagnosed as supraventricular tachycardia with block pattern); control ECG showed sinus rhythm, heart rate 108 bpm, reduced R wave through to V6, no fresh ischemia signs. Laboratory analysis revealed serum potassium 4.6 mmol/l, serum creatinine 1.27 mg/dl, glomerular filtration rate (GFR) 43 ml/min/1.73, serum blood urea 121.0 mg/dl, C-reactive protein 13.9 mg/dl, WBC count 15.51/nl, RBC count 3.57/pl, hemoglobin 10.4 g/dl, hematocrit 0.33, MCHC 31.2 g/d, total urine protein/day 665.60 mg/d, parathyroid hormone 102 pg/ml. On 29-DEC-2009, abdominal ultrasound showed kidneys of age-appropriate size without evidence of obstruction or focal finding. On 30-DEC-2009, ECG showed sinus rhythm, heart rate 105 bpm, reduced R wave through to V5. The patient was recovering from supraventricular tachycardia.

On 05-JAN-2010, laboratory analysis revealed serum potassium 3.8 mmol/l, serum creatinine 0.68 mg/dl, glomerular filtration rate (GFR) >60 ml/min/1.73, serum blood urea 33.0 mg/dl. On 06-JAN-2010, laboratory analysis revealed serum potassium 3.4 mmol/l, serum creatinine 0.66 mg/dl, glomerular filtration rate (GFR) >60 ml/min/1.73, serum blood urea 35.0 mg/dl, C-reactive protein 28.3 mg/dl, WBC count 13.49/nl, RBC count 3.21/pl, hemoglobin 9.4 g/dl, hematocrit 0.30, MCHC 31.1 g/d. On 07-JAN-2010, laboratory analysis revealed serum potassium 3.8 mmol/l, serum creatinine 0.72 mg/dl, glomerular filtration rate (GFR) >60 ml/min/1.73, serum blood urea 51.0 mg/dl, C-reactive protein 25.6 mg/dl, WBC count 12.33/nl, RBC count 3.12/pl, hemoglobin 9.1 g/dl, hematocrit 0.29, MCHC 31.0 g/d. On 12-JAN-2010, laboratory analysis revealed serum potassium 3.4 mmol/l, serum creatinine 0.64 mg/dl, glomerular filtration rate (GFR) >60 ml/min/1.73, serum blood urea 48.0 mg/dl, C-reactive protein 5.6 mg/dl, WBC count 7.90/nl, RBC count 3.36/pl, hemoglobin 9.4 g/dl, hematocrit 0.31, MCHC 30.6 g/d. On 14-JAN-2010, laboratory analysis revealed serum potassium 3.8 mmol/l, serum creatinine 0.72 mg/dl, glomerular filtration rate (GFR) >60 ml/min/1.73, serum blood urea 41.0 mg/dl, C-reactive protein 3.6 mg/dl, WBC count 7.76/nl, RBC count 3.51/pl, hemoglobin 9.7 g/dl, hematocrit 0.32, MCHC 30.1 g/d. The patient was recovering from normochromic normocytic anemia. On 15-JAN-2010, she was discharged from hospital.

The reporting physician was unsure about the drug relationship between sitagliptin phosphate and acute renal failure and mentioned that acute renal failure was rather related to long-term therapy with diclofenac. She felt that urinary tract infection was not related to therapy with sitagliptin phosphate.

Follow-up information received on 14-JUL-2010 contained the following adverse experiences: transient increase of AST (28-DEC-2009), transient increase of ALT (28-DEC-2009), transient increase of GGT (28-DEC-2009), increase of LDH (24-DEC-2009), anemia (24-DEC-2009), transient pain in right ankle joint (approximately December 2009), swelling in right ankle joint (approximately December 2009), intermittent confusional state (approximately December 2009), suspicion of beginning dementia syndrome (approximately December 2009; upon internal review, determined to be an other important medical event).

Additional information is not expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10104735

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19920710	Age: 17 Year	weight (kg): 68	BMI_int: 20.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute agranulocytosis	20100625		4 Day	21 Day	recovered/resolved
2 Asthenia					recovered/resolved
3 C-reactive protein increased					Unknown
4 Chills					recovered/resolved
5 Fever					recovered/resolved
6 GPT increased					Unknown
7 Hepatocellular damage					Unknown
8 Leukopenia	201006			21 Day	recovered/resolved
9 Neutropenia					recovered/resolved
10 Odynophagia					recovered/resolved
11 General physical health deterioration					recovered/resolved
12 GPT increased					Unknown
13 Odynophagia					recovering/resolving
14 Shivering					recovering/resolving
15 Splenomegaly					Unknown
16 Swallowing difficult					recovering/resolving
17 GGT increased					Unknown
18 Exhaustion	20100625		4 Day	21 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Novalgin metamizole	Oral	120 Gtt drop(s)/Day 160	20100604-20100625 22 Day	Postoperative pain
2 VOLTAREN DISPERS "NOVARTIS" diclofenac sodium	Oral	3 Df dosage form/Day	20100604-20100625 22 Day	Postoperative pain

Drugs (concomitant)

3 PANTOZOL pantoprazole sodium	Unknown	20 Mg milligram(s)/Day	-	Prophylaxis
4 tilidin tilidine	Oral	60 Gtt drop(s)/Day 160	20100604- -	Postoperative pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Femoral neck fracture			Unknown	PATHOLOGICAL FRACTURE
Fibrous dysplasia of bone			Unknown	
Bone cyst			k.A.	
General physical health deterioration			Unknown	
Hematoma			Unknown	Slight accumulation of fluid.
Fall			Unknown	
Fracture internal fixation			Yes	
Solitary bone cyst			No	
Bone cyst excision			No	
Cyst aspiration			Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Neutrophil count		3.1	%	
Alkaline phosphatase	201006	NORMAL	NA	
Bilirubin total	201006	NORMAL	NA	
Blood electrolytes NOS	201006	NORMAL	NA	
Blood pressure diastolic	201006	70	MMHG	
Blood pressure systolic	201006	120	MMHG	
Body temperature	201006	38.8	DEGREE C	
Body temperature	201006	39.4	DEGREE C	
C-reactive protein	20100627	33.88	MG/DL	
C-reactive protein	20100625	10	MG/DL	
C-reactive protein	201006	6.33	MG/DL	
Creatine kinase	201006	NORMAL	NA	
Creatinine	201006	NORMAL	NA	
Gamma GT	201006	63	U/L	
Haemoglobin	201006	13.2	G/DL	
Heart rate	201006	100	BEATS/MII	
LDH	201006	NORMAL	NA	
Neutrophil count	201006	0.07	10E9/L	
SGOT	201006	NORMAL	NA	
SGPT	201006	114	U/L	
Thrombocyte count	201006	358	10E9/L	
Urea	201006	NORMAL	NA	
Leukocyte count NOS	201006	2.3	10E9/L	
Leukocyte count NOS	201006	1.8	10E9/L	
Cytomegalovirus serology NOS	2010	NEGATIVE	NA	
Epstein-Barr virus serology	2010	NEGATIVE	NA	
Lipase	201006	NORMAL	NA	
Neutrophil percentage	201006	3	%	
Coagulation test	201006	NORMAL	NA	

Narratives (nur bei elektronischem Eingang)sanofi-aventis, 20.07.2010 DE-SANOFI-AVENTIS-2010SA041231

Sender diagnosis: not reported

Narrative

Initial report extracted from the web site of the German health authority (identification number DE-BFARM-10104735) on 15-Jul-2010.

A physician reported via the Drug Commission of the German Medical Association (DCGMA) on a 17-year-old male patient with a medical history of femoral neck fracture.

On 04-Jun-2010, the patient started treatment with metamizole sodium (Novalgine) 4x30 drops/day and diclofenac sodium (Voltaren dispers) 3x1 dosage form/day for postoperative pain.

Concomitant medication included tilidine.

On 25-Jun-2010, after three weeks of treatment with metamizole and diclofenac, the patient developed agranulocytosis. Metamizole and diclofenac were withdrawn immediately.

Outcome was reported as "recovered/resolved", duration of event as "three days".

Causality determined by DCGMA (method: WHO Global Introspection): "possible" for metamizole and for diclofenac.

Further information was not provided.

NOVARTIS, 27.07.2010 DE-002147023-PHHY2010DE46311

Sender diagnosis: not reported

Narrative

Initial report received from the Drug Commission of the German Medical Association of physicians (DE-BFARM-10104735) via Health Authority on 14 Jul 2010: This poly-medicated patient with current condition of femoral neck fracture started the treatment with Voltaren dispers (diclofenac free acid) for postoperative pain along with Novalgin (metamizole sodium monohydrate) for postoperative pain on 04 Jun 2010. The patient experienced agranulocytosis on 25 Jun 2010. Patient's leukocyte and neutrophil count on an unknown date was 1.8×10^3 /ul and 3.1%. C-reactive protein was 10 dl and 33 dl on 25 Jun 2010 and 27 Jun 2010 respectively. Treatment with Voltaren dispers and Novalgin were discontinued on 25 Jun 2010. The outcome of the event was reported as complete recovery.

suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

sanofi-aventis, 01.03.2011 DE-SANOFI-AVENTIS-2010SA041231

Sender diagnosis: not reported

Narrative

Initial report extracted from the web site of the German health authority (identification number DE-BFARM-10104735) on 15-Jul-2010.

A physician reported via the Drug Commission of the German Medical Association (DCGMA) on a 17-year-old male patient with a medical history of femoral neck fracture.

On 04-Jun-2010, the patient started treatment with metamizole sodium (Novalgin) 4x30 drops/day and diclofenac sodium (Voltaren dispers) 3x1 dosage form/day for postoperative pain.

Concomitant medication included tilidine.

On 25-Jun-2010, after three weeks of treatment with metamizole and diclofenac, the patient developed agranulocytosis. Metamizole and diclofenac were withdrawn immediately.

Outcome was reported as "recovered/resolved", duration of event as "three days".

Causality determined by DCGMA (method: WHO Global Introspection): "possible" for metamizole and for diclofenac.

Further information was not provided.

Additional information received on 22-Feb-2011 (full article): Pfersdorff M, Spes J, Kraus MR. 17-year-old patient with neutropenia and fever during therapy with analgesics. Dtsch Med Wochenschr 2011; 136: 365-368.

Casualistic: The 17-year-old patient was admitted to the authors' hospital due to worsening of general health condition with fever of up to 39.4 degrees Celsius, chills and asthenia since the day before admission. Additionally, he was suffering from pronounced swallowing disorder in the sense of odynophagia without nausea or vomiting.

Three weeks prior to admission, the patient had suffered a pathologic medial femoral neck fracture in his left leg due to pre-existing benign bone cyst. The patient had undergone a closed reposition with cannulated screw fixation and aspiration of the bone cyst. Histopathologic examination of sample had revealed fibrous dysplasia.

Discomfort at operation site was not present at time of admission.

The patient had no further history of a centre of infection. The patient had no relevant previous disease. According to the patient, he had not been to a foreign country.

Long-term medical treatment since operation consisted of analgesic combination therapy with tilidine (100/8 mg, 1-0-1), metamizole (Novalgin, 4x30 drops), and diclofenac (Voltaren dispers, 1-1-1) under gastric protection with pantoprazole (Pantozol 20 mg, 0-0-1).

Physical examination showed a patient in a reduced general condition and good nutritional state, body temperature 38.8 degrees Celsius, blood pressure 120/70 mmHg, heart rate 100 beats/min. Skin was unremarkable. Pharynx was normal at inspection, no redness, no pustules, no ulcerations. Pulmological examination showed eupnoea, vesicular respiration, no rhonchi, sonorous percussion sound, expiration not prolonged. Heart sound was clear, rhythmic, without pathologic sounds; abdominal wall soft, no tenderness, liver and spleen not palpable, no renal angle tenderness, unremarkable status of lymph nodes, no tenderness on percussion concerning spinal cord. No peripheral oedema, all peripheral pulses palpable.

Neurologic examination was unremarkable, no meningism. The surgical wound at left thigh was without signs of local inflammation.

Laboratory showed a leucopenia of 2300/mcl as well as a slight normochromic, normocytic anaemia (haemoglobin 13.2 g/dl). Thrombocyte count was 358000/mcl. Differential blood count displayed a pronounced neutropenia of 3% (= about 70 neutrophils/mcl). CRP value was increased with 6.33 mg/dl (normal range < 0.5 mg/dl). GPT was increased with 114 U/l and gamma GT with 63 U/l. Other routine laboratory values such as coagulation, electrolytes, creatinine, urea, GOT, alkaline phosphatase, total bilirubin, LDH, creatine kinase, and lipase were within normal limits. Acute EBV and CMV infection was excluded by serology.

The patient underwent a profound diagnostic investigation (abdominal sonography, chest X-ray, urinary status including Uricult test), but no definite pointer to a centre of infection could be found. Worth mentioning was merely a splenomegaly with a diameter in length of 13 cm. Sonography of the area of wound at the patient's left thigh showed a slight fluid accumulation which was diagnosed as older haematoma after aspiration, no pathogens were detected. Several blood cultures remained sterile as well.

Because of pronounced neutropenia and to exclude a primarily haematological disease, the patient underwent bone marrow aspiration on admission, which displayed a normocellular bone marrow without signs of increase in blasts; noticeable was, however, a discontinuation of maturation in granulopoiesis at the stage of ripe promyelocytes, a typical finding of a drug-toxic bone marrow damage in the sense of agranulocytosis ("promyelocyte marrow").

Examinations and medical history led to the diagnosis of drug-toxic agranulocytosis due to treatment with metamizole and/or diclofenac, as well as a feverish infection without confirmed centre.

Both drugs were withdrawn after establishment of diagnosis.

Because of the pronounced neutropenia of unknown duration with accompanying high-grade immunodeficiency, the patient was subjected to reverse isolation, intravenous treatment with broad-spectrum antibiotics (piperacillin/sulbactam), as well as administration of filgrastim (granulocyte-colony stimulating factor, 30 million units daily).

On the third day of hospitalisation, C-reactive protein reached its peak with 33.88 mg/dl.

On the fourth day, neutrophil granulocyte count began to rise. Fever resolved and the patient's condition improved considerably in the further course. The patient was discharged from hospital after nine days.

He received an allergy pass and was recommended to avoid further treatment with metamizole and diclofenac. Analgesic therapy was continued exclusively with low-potential opioids.

Based on the definite temporal connection with the drug intake and the absence of alternative explanations for the bone marrow damage, the analgesics had to be assumed as most likely cause of agranulocytosis.

In this case it could not be clarified whether metamizole, diclofenac, or the combination of both drugs had caused the myelotoxicity. Nevertheless, metamizole had been discussed for years as trigger of agranulocytosis.

The patient had taken metamizole for 21 days before establishment of diagnosis, duration of neutropenia was four days under administration of filgrastim; so, in this case, bone marrow regeneration happened relatively fast.

The patient initially had a neutropenia of < 100/mcl, which counted as possible indicator for a severe process.

NOVARTIS, 17.03.2011 DE-002147023-PHHY2011DE17401

Sender diagnosis: not reported

Narrative

Initial literature report received on 02 Mar 2011: This patient had a medical history of fall, and suffered a pathological medial fracture of the neck of the femur due to a pre-existing benign bone cyst. Closed repositioning was subsequently performed with cannulated screws and the bone cyst was aspirated (date unknown). Histopathological preparation of the sample material revealed fibrous dysplasia and had no local pain at the operation site. Following surgery, the patient was put on continuous combined analgesic therapy. The patient received Voltaren dispers (diclofenac free acid), 1 DF, thrice daily, Tilidin retard 100/8 mg (Naloxone/tilidine), twice daily, Novalgin (metamizole sodium monohydrate), 4 x 30 drops and enteric-coated Pantozol (pantoprazole) 20 mg once daily to protect stomach. The patient was admitted to hospital because of a general deterioration in his condition over 24 hours, with a fever of 39.4 degree C, chills and exhaustion. In addition, he was suffering from pronounced swallowing difficulties in terms of odynophagia without nausea or vomiting. Physical examination revealed patient in reduced general and good nutritional state. Temperature was 38.8 degree C, Blood pressure was 120/70 mmHg, Heart rate was 100 bpm. Throat was not abnormal on inspection, had no redness, no blister formation, no ulceration. The general physical examination was normal and the preliminary neurological examination was uneventful and had no meningism. The operation wound on the left thigh had no signs of local inflammation. On admission, the blood count revealed a leucopenia of 2 300/ul (reference range not specified) and had a slight normochromic, normocytic anaemia with hemoglobin (Hb) of 13.2 g/dl (reference range not specified). The platelet count was 358 000/ul (reference range not specified). The differential blood count revealed in particular a pronounced neutropenia affecting 3% of the entire leucocytes (equivalent to approximately 70 neutrophils/ul). In addition, the CRP was increased to 6.33 mg/dl (reference range: less than 0.5 mg/dl). Moreover, the GPT (Alanine aminotransferase) was increased to 114 U/l (reference range not specified) and the GGT 63 U/l (reference range not specified). The remaining routine laboratory parameters, including coagulation, electrolytes, creatinine, urea, GOT (Aspartate aminotransferase), alkaline phosphatase, total bilirubin, LDH (lactate dehydrogenase), CK (creatin kinase) and lipase, were within the normal range. An acute EBV (Epstein Barr virus) and CMV (cytomegalovirus) infection were ruled out on the basis of serology. Abdominal ultrasound, X-ray of thorax, urinary status including urinary cultures showed no reliable evidence of the infection. The patient had a splenomegaly with 13 cm in length. The ultrasound in the region of the wound on the left thigh revealed a slight accumulation of fluid. This was aspirated and proved to be an old haematoma; however there was no evidence of any bacteria in this location. The blood culture pairs on several occasions remained negative. In view of the pronounced neutropenia and to exclude a primary hematological disease, bone marrow biopsy was done on the day of admission. This showed a marrow with predominantly normal cells with no evidence of an increase in blasts. However, there was impaired maturation of granulocyte precursor cells, with a predominance of mature promyelocytes and showed a typical sign of toxic, medication-induced bone marrow damage in terms of agranulocytosis (promyelocytes marrow). The patient was diagnosed a toxic, medication-induced agranulocytosis due to metamizole and/or diclofenac and a febrile infection without detection of the focus. Both medications were discontinued once the differential blood count was received. In view of the pronounced neutropenia of unknown duration with persistent severe lowered resistance, the patient was immediately put under reverse isolation to protect him from infection. The patient received broad-spectrum, intravenous antibiotics (piperacillin/combactam) and filgrastim (Granulocyte-Colony Stimulating factor), 30 million units/day. The CRP had reached a maximum of 33.88 mg/dl on day three. From day four, the patient had normalized neutrophil blood count, the fever outbreaks stopped and the patient was subsequently significantly improved clinically. The patient was rapidly free of

symptoms and a gastroscopy was not performed to diagnose the cause of the initial odynophagia. After a total of 9 days, the patient was discharged from inpatient treatment. He was given an allergy card and told to avoid metamizole and diclofenac for the rest of his life.

NOVARTIS, 18.03.2011 DE-SZ09-PHHY2011DE17401

Sender diagnosis: not reported

Narrative

Initial literature report received on 02 Mar 2011: This patient had a medical history of fall, and suffered a pathological medial fracture of the neck of the femur due to a pre-existing benign bone cyst. Closed repositioning was subsequently performed with cannulated screws and the bone cyst was aspirated (date unknown). Histopathological preparation of the sample material revealed fibrous dysplasia and had no local pain at the operation site. Following surgery, the patient was put on continuous combined analgesic therapy. The patient received Voltaren dispers (diclofenac free acid), 1 DF, thrice daily, Tilidin retard 100/8 mg (Naloxone/tilidine), twice daily, Novalgin (metamizole sodium monohydrate), 4 x 30 drops) and enteric-coated Pantozol (pantoprazole) 20 mg once daily to protect stomach. The patient was admitted to hospital because of a general deterioration in his condition over 24 hours, with a fever of 39.4 degree C, chills and exhaustion. In addition, he was suffering from pronounced swallowing difficulties in terms of odynophagia without nausea or vomiting. Physical examination revealed patient in reduced general and good nutritional state. Temperature was 38.8 degree C, Blood pressure was 120/70 mmHg, Heart rate was 100 bpm. Throat was not abnormal on inspection, had no redness, no blister formation, no ulceration. The general physical examination was normal and the preliminary neurological examination was uneventful and had no meningism. The operation wound on the left thigh had no signs of local inflammation. On admission, the blood count revealed a leucopenia of 2 300/ul (reference range not specified) and had a slight normochromic, normocytic anaemia with hemoglobin (Hb) of 13.2 g/dl (reference range not specified). The platelet count was 358 000/ ul (reference range not specified). The differential blood count revealed in particular a pronounced neutropenia affecting 3% of the entire leucocytes (equivalent to approximately. 70 neutrophils/ul). In addition, the CRP was increased to 6.33 mg/dl (reference range: less than 0.5 mg/dl). Moreover, the GPT (Alanine aminotransferase) was increased to 114 U/l (reference range not specified) and the GGT 63 U/l (reference range not specified). The remaining routine laboratory parameters, including coagulation, electrolytes, creatinine, urea, GOT (Aspartate aminotransferase), alkaline phosphatase, total bilirubin, LDH (lactate dehydrogenase), CK (creatin kinase) and lipase, were within the normal range. An acute EBV (Epstein Barr virus) and CMV (cytomegalovirus) infection were ruled out on the basis of serology. Abdominal ultrasound, X-ray of thorax, urinary status including urinary cultures showed no reliable evidence of the infection. The patient had a splenomegaly with 13 cm in length. The ultrasound in the region of the wound on the left thigh revealed a slight accumulation of fluid. This was aspirated and proved to be an old haematoma; however there was no evidence of any bacteria in this location. The blood culture pairs on several occasions remained negative. In view of the pronounced neutropenia and to exclude a primary hematological disease, bone marrow biopsy was done on the day of admission. This showed a marrow with predominantly normal cells with no evidence of an increase in blasts. However, there was impaired maturation of granulocyte precursor cells, with a predominance of mature promyelocytes and showed a typical sign of toxic, medication-induced bone marrow damage in terms of agranulocytosis (promyelocytes marrow). The patient was diagnosed a toxic, medication-induced agranulocytosis due to metamizole and/or diclofenac and a febrile infection without detection of the focus. Both medications were discontinued once the differential blood count was received. In view of the pronounced neutropenia of unknown duration with persistent severe lowered resistance, the patient was immediately put under reverse isolation to protect him from infection. The patient received broad-spectrum, intravenous antibiotics (piperacillin/combactam) and filgrastim (Granulocyte-Colony Stimulating factor), 30 million units/day. The CRP had reached a maximum of 33.88 mg/dl on day three. From day four, the patient had normalized neutrophil blood count, the fever outbreaks stopped and the patient was subsequently significantly improved clinically. The patient was rapidly free of symptoms and a gastroscopy was not performed to diagnose the cause of the initial odynophagia. After a total of 9 days, the patient was discharged from inpatient treatment. He was given an allergy card and told to avoid metamizole and diclofenac for the rest of his life.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-10109552

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicidal intention					recovered/resolved
2 Hepatic enzymes increased					recovered/resolved
3 Drug intoxication					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Alkohol	Unknown	/	-	Drug use for unknown indication
ethanol			-	indication
2 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac			-	indication
3 ibuprofen	Unknown	/	-	Drug use for unknown indication
ibuprofen			-	indication
4 Loperamide	Unknown	/	-	Drug use for unknown indication
loperamide			-	indication
5 pantozol 40 mg	Unknown	/	-	Drug use for unknown indication
pantoprazole			-	indication

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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GPT

Aspartate aminotransferase

Gamma GT

Narratives (nur bei elektronischem Eingang)

RANBAXY, 20.07.2010 DE-RANBAXY-2010US-35591

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2010US-35591 from GERMANY refers to a UNK, 41 years-old who reportedly had suicidal intention, drug toxicity and hepatic enzymes increased while on treatment with Ibuprofen unk, unk, loperamide unk, unk and Diclofenac unk, unk for unknown indication. The event required patient hospitalization.

This is a business partner case forwarded By Nycomed.
Per the report, the case was reported by Nycomed, Germany.

It concerned a patient born on 15-Jan-1969 (gender not reported) who took 1.2 gram at once (30 tablets of 40 mg) of Pantozol(Pantoprazole) associated with Loperamide, Ibuprofen, Diclofenac and a big amount of alcohol (Mixed drug intoxication with suicidal intention) on an unspecified date. Neither medical history nor concomitant treatment was reported. The patient was admitted in stationary treatment and was monitored with ECG and respiratory monitoring. About 11 hours after the intake, the patient was awake, oriented and in good general condition. Lightly increased liver values (GOT, GPT and Gamma-GT) were detected through laboratory values (no values provided). It was specified that an alcohol abuse could not be ruled out.

Outcome of the adverse reaction were reported as resolved.

Company (Nycomed) Remark:

Mixed drug intoxication and suicidal intention are unlisted in the current reference safety information of Pantoprazole.

Increased liver values are listed in the current reference safety information of Pantoprazole.

The causal relationship between Pantoprazole and the adverse reaction of suicidal intension is considered not related and rather related to one of the above mentioned causes.

This single case report is not considered to change the current knowledge of the safety profile of Pantoprazole.

Case Outcome: Resolved

F/U # 1: 12-Jul-2010 : (Significant):

Follow-up information was received on 12-Jul-2010.

Per the report, patient was discharged from hospital after 24 hours of observation without having any complications.

The event outcome was reported as resolved at the time of this report.

The case is deemed serious. The events are unexpected as per USPI and are possibly related to the suspect drugs as per WHO UMC system for standardized causality assessment.

Case Outcome: Resolved

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10130917

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19600219	Age: 41 Year	weight (kg): 147	BMI_int: 42.95	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blood pressure increased					Unknown
2 Diabetes mellitus					not recovered/not resolved
3 Chondrolysis					Unknown
4 Performance status decreased					Unknown
5 Visual impairment					Unknown
6 Unevaluable reaction					Unknown
7 Spleen disorder NOS	2002				not recovered/not resolved
8 Metabolic disorder NOS					not recovered/not resolved
9 Libido decreased					Unknown
10 Joint pain	2002				Unknown
11 Increased intraocular pressure					Unknown
12 Hepatic disorder NOS	2002				not recovered/not resolved
13 Forgetfulness					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac			-	indication
2 ibuprofen	Unknown	600 Mg milligram(s)/	-	Drug use for unknown indication
ibuprofen			-	indication
3 TELFAST	Oral	180 Mg milligram(s)/	-	Allergy NOS
fexofenadine hydrochloride		82	7 Month	

Drugs (concomitant)

4 Antihypertensiva	/	-	Drug use for unknown indication
		-	indication
5 CHOLESTEROL- AND TRIGLYCERIDE REDU	/	-	Drug use for unknown indication
		-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Spleen enlarged			Yes	
Liver enlargement			Yes	
Impaired liver function			Yes	
Adrenal disorder NOS			Yes	
Type II diabetes mellitus			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Blood pressure

UP TO 200 MMHG

Narratives (nur bei elektronischem Eingang)sanofi-aventis, 26.08.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Reporter's comment:sanofi-aventis, 20.09.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

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Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicit causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

sanofi-aventis, 09.12.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

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Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicite causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

sanofi-aventis, 16.12.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicite causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

Additional letter received from the patient on 08-Dec-2010:

The patient additionally wrote that due to the impaired adrenal function he wasn't no longer able to climb stairs. Additionally he became a diabetic and was unable to do physical work lasting longer than 1.5 hours. He stated that the company's drug had caused severe damages and he likely had no much time left to live. The patient specified the amount of compensation he claimed from the company.

sanofi-aventis, 04.01.2011 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicit causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

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Two additional letters received from the patient on 22-Dec-2010:

Besides already reported problems the patient provided some new statements: "The liver had torn off partially and therefore he often has to change his sitting position to avoid flow problems of ductus". Furthermore he mentioned "a severe metabolic disorder NOS which would become life-threatening. The liver was massive allergic to alcohol and would permanently be painful. In short, he was currently in an increasing progression. The damages without any doubt resulted from Telfast because he did not take any other drugs except ibuprofen and diclofenac sodium already mentioned previously." He again claimed for compensation and raised the amount of damage. In his view "Telfast was prescribed without complete information and without clarification of contraindication. The drug was prescribed in accordance with the state of the art of the time which was imperfect. Additionally your drug was included in a list of particularly harmful drugs."

sanofi-aventis, 11.01.2011 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the

drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicit causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

Additional letter received from the patient on 08-Dec-2010:

The patient additionally wrote that due to the impaired adrenal function he wasn't no longer able to climb stairs. Additionally he became a diabetic and was unable to do physical work lasting longer than 1.5 hours. He stated that the company's drug had caused severe damages and he likely had no much time left to live. The patient specified the amount of compensation he claimed from the company.

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Besides already reported problems the patient provided some new statements: "The liver had torn off partially and therefore he often has to change his sitting position to avoid flow problems of ductus". Furthermore he mentioned "a severe metabolic disorder NOS" which would become life-threatening. The liver was massive allergic to alcohol and would permanently be painful. In short, he was currently in an increasing progression. The damages without any doubt resulted from Telfast because he did not take any other drugs except ibuprofen and diclofenac sodium already mentioned previously." He again claimed for compensation and raised the amount of damage. In his view "Telfast was prescribed without complete information and without clarification of contraindication. The drug was prescribed in accordance with the state of the art of the time which was imperfect. Additionally your drug was included in a list of particularly harmful drugs."

Additional information received from the patient on 04-Jan-2011:

In a further letter the patient wrote: "Since 2010 both liver and spleen has been allergic at alcohol consumption. Libido is decreasing severely. Since 2008 I have been a diabetic and have to inject insulin permanently since 2009. I am disabled for the rest of my life and I am very forgetful. I suffer and suffered from increase in blood pressure up to 200 and from increase in intraocular pressure (20) while seated, even after hours with declining vision. Furthermore severe performance inefficiency is present." He again claimed for compensation.

sanofi-aventis, 28.09.2011 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today. Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicit causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

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Additional information received from the patient on 04-Jan-2011:

In a further letter the patient wrote: "Since 2010 both liver and spleen has been allergic at alcohol consumption. Libido is decreasing severely. Since 2008 I have been a diabetic and have to inject insulin permanently since 2009. I am disabled for the rest of my life and I am very forgetful. I suffer and suffered from increase in blood pressure up to 200 and from increase in intraocular pressure (20) while seated, even after hours with declining vision. Furthermore severe performance inefficiency is present." He again claimed for compensation.

Additional information received from the patient on 25-Jan-2011:

The consumer informed the company that two further physicians he released from their obligation to secrecy had only prescribed ibuprofen and diclofenac but not fexofenadine.

Additional information received on 01-Aug-2011:

No new medical information was provided. He again claimed for compensation but claim for damages was lowered.

Additional information received on 13-Sep-2011:

The Drug commission of the German Association (DE-DCGMA-10154583) reported about duplicate information in the case 2011SA 18379 (received via webpage of health authority: DE-BfArM-11049925). Therefore this second case will be deleted and the case information was adjusted into the current case as follows: diclofenac (trade name not provided) and ibuprofen (CT) 600 mg were upgraded from concomitant to suspect. Contrary to the initial reports, the case onset date was specified with 2002. Initially the consumer reported about the first intake of fexofenadine hydrochloride on 11-Oct-2004 (until 01-Jun-2005). In this follow up the therapy dates were not provided. Furthermore the event joint pain was additionally entered.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10131550

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19670326	Age: 43 Year	weight (kg): 100	BMI_int: 25.77	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	20100810	20100827	18 Day	4 Day	recovering/resolving
2 Jaundice					Unknown
3 Gallbladder edema					recovering/resolving
4 Scleral icterus					recovering/resolving
5 Gallbladder edema					Unknown
6 Hepatic siderosis					Unknown
7 Necrosis hepatocellular					Unknown
8 Hepatic steatosis					Unknown
9 Cholestasis intrahepatic					Unknown
10 Cholecystitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC diclofenac sodium	Oral	75 Mg milligram(s)/ 245	20100807-20100808 2 Day	Postoperative pain

Drugs (concomitant)

2 ALLOPURINOL allopurinol	/	-	Gout
3 CANDESARTAN candesartan	/	-	Hypertension arterial
4 IRBESARTAN irbesartan	/	-	Hypertension arterial

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperuricemic arthritis			Yes	
Hypertension arterial			Yes	
Barrett's oesophagus	201008		Yes	Long-Segment Barrett Ösophagus
Knee ligament repair			No	
Hyperlipidemia			Yes	
Hepatitis B immunization			Yes	
Hepatitis A immunization			Yes	
Gout			k.A.	
Knee operation			k.A.	
Gilbert's syndrome			Yes	KNOWN MORBUS MEULENGRACHT

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Albumin	201008	56.1	%	
Alkaline phosphatase	201008	181	U/L	

Alpha 1 globulin	201008	7.3	%
Alpha 2 globulin	201008	13.1	%
Blood immunoglobulin A	201008	122	MG/DL
Blood immunoglobulin G	201008	734	MG/DL
Blood immunoglobulin M	201008	61	MG/DL
C-reactive protein	201008	69.8	MG/L
Gamma globulin	201008	11.9	%
Gamma GT	201008	423	U/L
GPT	201008	95	U/L
Hematocrit	201008	33.2	%
Hemoglobin	201008	10.9	G/DL
MCHC	201008	32.8	G/DL
RBC count	201008	3.49	10E6/MCL
Serum ferritin	201008	295	MCG/L
Serum iron	201008	32	MCG/DL
Thrombocyte count	201008	729	10E3/MCL
Transferrin	201008	196.7	MG/DL
Protein total	201008	6	G/DL
Lipase	201008	79	U/L
Transferrin saturation	201008	11.5	%
Beta globulin	201008	6.3	%
Beta 2 globulin	201008	3	%

Narratives (nur bei elektronischem Eingang)

Klinikum Traunstein, 30.08.2010 Pat.-Init.: MK, *26.03.1967, männlich

Sender diagnosis: not reported

Narrative

Der Patient hatte in Eigenmedikation bei Z.n. Kreuzbandoperation Diclofenac als Schmerzmedikation eingenommen. Im Verlauf war es zu einem Sklerenikterus gekommen, weswegen der Patient seinen Hausarzt konsultierte, welcher ihn ins Krankenhaus Berchtesgaden einwies. Hier zeigte sich stark erhöhte Cholestase- und Leberwerte. Die Hepatitis A, B und C ergab keinen Hinweis für eine Hepatitisinfektion bei bestehendem Impfschutz gegen Hepatitis A und B. Sonographisch zeigte sich das Bild einer Cholecystitis weswegen der Patient bei V.a. Steinabgang eine ERCP erhielt, hier jedoch unauffälliger Befund. Letztendlich gehen wir hier aktuell von einem Gallenblasenwandödem im Rahmen der Hepatitis aus.

Am 19.08.2010 wurde der Patient ins Klinikum Traunstein zur weiteren Diagnostik und Therapie verlegt. Bei Übernahme waren die Leber und Cholestasewerte bereits rückläufig. Sonographisch stellte sich uns das Bild einer Steatosis hepatis Grad I dar. Die Autoimmundiagnostik war negativ. Die Hepatitis-Serologie erneut negativ. Es erfolgte die internistische Laparoskopie, wo eine Leberzirrhose ausgeschlossen werden konnte. Histologisch zeigte sich das Bild einer Mäßiggradige hepatitisartige Leberzellschädigung mit wechselnd alten Einzelzellnekrosen, Leberzellverfettung, mäßiger intrahepatischer Cholestase und Leberzell- und Kupferzellsiderose. Im kritischen Bericht spricht die Befundkostellation laut Pathologen für das Vorliegen einer chronisch-toxischen Leberzellschädigung, äthyltoxisch. Auch eine NSAR-Hepatitis kommt differentialdiagnostisch in Betracht. Eine äthyltoxische Genese kann glaubhaft anamnestisch ausgeschlossen werden. Somit besteht der Verdacht auf eine NSAR-Hepatitis nach Einnahme von Diclofenac DD: NASH<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 08.09.2010 DE-SANOFI-AVENTIS-2010SA052408

Sender diagnosis: not reported

Narrative

Initial report:

On 01-Sep-2010 this case was extracted from the website of the German health authority (authority number DE-BFARM-10131550); documenting a case, in which a physician had reported about a 43-year-old male. The past drug therapy included allopurinol for gout as well as alternated candesartan and irbesartan for arterial hypertension.

Concomitantly the patient suffered from Gilbert's syndrome, arthritis urica with gout, arterial hypertension, hyperlipidemia and Barrett's oesophagus. The vaccinal immune protection against hepatitis A and B was present.

Due to postoperative pain after cruciate ligament surgery the patient took diclofenac 75 mg from 07-Aug-2010 to the next day. Thereafter he developed scleral icterus and was hospitalised on 10-Aug-2010. The values of cholestasis and liver were severe increased. Due to suspicion of gallstone pass the patient underwent endoscopic retrograde cholangiopancreatography, however pathological findings were not disclosed. The physician diagnosed an edema of gallbladder wall due to hepatitis.

On 19-Aug-2010 the patient was transferred into another hospital. At this time point the values of cholestasis and liver were decreasing. The sonography displayed a hepatic steatosis grade I. The autoimmune diagnostic was negative and the hepatitis serology was positive again. The liver cirrhosis was excluded by laparoscopy. The histology presented a moderate liver damage like of hepatitis with changing old single cell necrosis and liver cell fatty degeneration, moderate intrahepatic cholestasis and liver cell and hepatic siderosis. An ethyl toxic genesis could be excluded anamnestically. Differential diagnosis: non-alcoholic steatohepatitis.

On 27-Aug-2010 the hepatitis was resolving.

Reporter causality: not reported

NOVARTIS, 15.09.2010 DE-002147023-PHHY2010DE58555

Sender diagnosis: not reported

Narrative

Initial report received from a physician via Health Authority (reference number: DE-BFARM-10131550) on 31 Aug 2010: This patient had medical history of Gilbert's syndrome, hyperlipidemia, gout, hypertension arterial, Barrett's oesophagus, knee operation and cruciate ligament surgery. He started treatment with diclofenac (manufacturer unknown) 75 mg per day from 07 Aug 2010 for post operative pain. On an unknown date he developed scleral jaundice. Blood test revealed increased ALP (181 U/l), GPT (95 U/L), GGT (423 U/L), lipase (79 U/L) and C-reactive protein (69.8 mg/dl) (Normal ranges not reported). The therapy with diclofenac was discontinued on 08 Aug 2010. Sonographic examination revealed findings of cholecystitis. Diagnosis of edema of gallbladder wall and hepatitis was made. Onset date of hepatitis was reported to be 10 Aug 2010. On 19 Aug 2010 he was admitted and it was found that the patient had Grade I hepatic steatosis. Hepatitis serology was negative. Laparoscopy was performed and ruled out the possibility of liver cirrhosis. Histological analysis revealed the pattern of moderate hepatitis like liver cell damage with alternatingly old single cell necrosis, fatty degeneration of liver cells, moderate intrahepatic cholestasis and liver cell and Kupffer cell siderosis. The outcome of hepatitis was reported to be condition improving. Non steroidal anti-inflammatory hepatitis and non alcoholic steatohepatitis could be considered as a differential diagnosis.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Based on review of available reported data there is a reasonable possibility of causal relationship for the events.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10131607

Serious Yes	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzymes	20100816			95 Day	recovering/resolving
2 CPK increased	20100816			95 Day	recovered/resolved
3 Myalgia	20100816			95 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC		/	-	
diclofenac			-	
2 SIRDALUD	Oral	6 Mg milligram(s)/	20100514-	Spinal disorder
tizanidine hydrochloride			-	

Drugs (concomitant)

3 PAROXETINE	/	20080514-	Depression
paroxetine		-	
4 TRIAMTERIL COMPLEX	/	-	Hypertension
hydrochlorothiazide; triamterene		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Adipositas			k.A.	
Hypertension			k.A.	
Varicose veins NOS			k.A.	
Depression			k.A.	
Alcohol use			k.A.	
Smoker			k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
CPK	20080212	67	µg/l	
CPK	20110111	55	µg/l	
CPK	20100819	994	µg/l	
CPK	20100817	6090	µg/l	
CPK		More than 8000	µg/l	
GGT	20101007	67	µg/l	
GGT	20100817	104	µg/l	
GGT	20110111	71	µg/l	
GOT increased	20110111	31	µg/l	
GOT increased	20101007	30	µg/l	
GOT increased	20100817	134	µg/l	

GPT	20110111	62	µg/l
GPT	20101007	56	µg/l
GPT	20100817	135	µg/l

Narratives (nur bei elektronischem Eingang)NOVARTIS, 30.08.2010 DE-002147023-PHHY2010DE55056

Sender diagnosis: not reported

Narrative

Initial physician report received on 16 Aug 2010: This patient with an unknown medical history had been treated with Sirdalud (tizanidine) 6mg/daily and diclofenac 100 from an unknown date and for an unspecified indication. While on Sirdalud and diclofenac (manufacture unknown) treatment, he started to experience a significant CPK-increase (currently; more than 8000) and myalgia on an unknown date. The seriousness criteria, outcome and causality of the events were not explicitly assessed.

Serious spontaneous report, For Sirdalud and Diclofenac

Blood creatinine phosphokinase increased [medically significant], assessed as unlisted according to the Core Data Sheet.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Based on Review of available reported data does not allow for a proper causality assessment.

Reporter's comment:NOVARTIS, 09.06.2011 DE-002147023-PHHY2010DE55056

Sender diagnosis: not reported

Narrative

Initial physician report received on 16 Aug 2010: This patient with an unknown medical history had been treated with Sirdalud (tizanidine) 6mg/daily and diclofenac 100 from an unknown date and for an unspecified indication. While on Sirdalud and diclofenac (manufacture unknown) treatment, he started to experience a significant CPK-increase (currently; more than 8000) and myalgia on an unknown date. The seriousness criteria, outcome and causality of the events were not explicitly assessed.

Follow up information received from physician on 25 May 2011: Patient's history included depression, adipositas, hypertension, alcohol, irregular smoking and varicosis. His concomitant medications included paroxetine, since 14 May 2008 and triamteril complex. Treatment with Sirdalud was started on 14 May 2010 for the indication of a spine syndrome. The onset date for the event "CPK increased" and "myalgia" was provided as 16 Aug 2010. The event "myalgia" was more specifically reported as "myalgia of the upper legs" and "Increased liver enzymes values" was reported as an additional event. The laboratory values for the parameter blood creatine phosphokinase (CPK) were reported as 67 U/L, 6090 U/L, 994 U/L and 55 U/L on 12 Feb 2008, 17 Aug 2010, 19 Aug 2010 and 11 Jan 2011 respectively. Lab values for GGT were 104, 67 and 71 U/L on 17 Aug 2010, 07 Oct 2010 and 11 Jan 2011, respectively. Lab values for GOT were reported as 134, 30 and 31 U/L on 17 Aug 2010, 07 Oct 2010 and 11 Jan 2011 respectively. The lab values for GPT were reported as 135, 56 and 62 U/L on 17 Aug 2010, 07 Oct 2010 and 11 Jan 2011 respectively. All events were assessed as non-serious by the reporter. Sirdalud treatment was reported to be discontinued and the patient's condition improved in the further course; the events "CPK increase" and "myalgia" were reported as recovered, whereas event "increased liver enzyme values" was reported as improved. The causality of the events was reported as probably related to Sirdalud for CPK increase and myalgia and as possibly related for the increased liver enzymes values.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10145202

Serious Yes	Death	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19540704	Age: 55 Year	weight (kg): 82	BMI_int: 25.03	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20100308				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclo Dispers, 50 mg, Tabl, Betapharm diclofenac	Oral	150 Mg milligram(s)/Day 245	200908-20100310 -	Psoriatic arthropathy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Osteomyelofibrosis	201003		Yes	primäre Osteomyelofibrose
Psoriatic arthropathy	1957		Yes	
Arthritis rheumatoid			Yes	
Gout	2004		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine increased	20051122	1.2	mg/dl	
Creatinine increased	20100308	5.09	mg/dl	
Creatinine increased	20100310	5.65	mg/dl	
Creatinine increased	20100311	4.39	mg/dl	
Creatinine increased	20100330	3.04	mg/dl	
Creatinine increased	20100428	2.45	mg/dl	
Increased urea	20100308	248	mg/dl	
Increased urea	20100310	221	mg/dl	
Increased urea	20100311	150	mg/dl	
Increased urea	20100330	151	mg/dl	
Increased urea	20100428	90	mg/dl	
GFR decreased	20100308	11.7	ml/min/1.73	
GFR decreased	20100310	10	ml/min/1.73	
GFR decreased	20100325	19	ml/min/1.73	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 22.09.2010 DE-FAKOS-160013

Sender diagnosis: not reported

Narrative

Am 10.03.2010 erfolgte die stationäre Aufnahme des Patienten mit akutem Nierenversagen (ANV) (Laborwerte: Kreatinin 5,65 mg/dl (Normbereich: <1,2 mg/dl), Harnstoff 221 mg/dl (Normbereich: 14-46 mg/dl)).

Aktuell wurde auch eine primäre Osteomyelofibrose (Knochenmarksbiopsie) diagnostiziert. Eine Dialysetherapie wurde in der Zeit vom 10.-22.03.2010 durchgeführt. Im Verlauf kam es zu einer Besserung der Nierenfunktion mit Stabilisierung des Serumkreatinins um 2,5-3,0 mg/dl. Der Kreatininwert betrug am 22.11.2005: 1,2 mg/dl.

Die Nierenbiopsie ergab folgenden histologischen Befund: Bild wie bei einer akuten Tubulusschädigung bei interstitieller Nephritis und hypoxämischer Tubulusschädigung. Die Histologie ist passend zu einer Schädigung durch NSAR (Nichtsteroidale Antirheumatika).

Arzneimittelanamnese: Einnahme von Diclofenac seit 08/2009, sonst keine anderen Arzneimittel. Sonographisch bei Nierenkonkrementen keine signifikante Harnstauung. Immunologische Parameter unauffällig (keine Hinweise für Vaskulitis bzw. Glomerulonephritis). Am 30.03.2010 wurde der Patient im deutlich gebesserten und stabilen Allgemeinzustand aus dem Krankenhaus entlassen (Laborwerte: Kreatinin 3,04 mg/dl, Harnstoff 151 mg/dl).

Eine medikamenteninduzierte Genese des akuten Nierenversagens ist wahrscheinlich. Die UAW ist ambulant entstanden. Als Auslöser wird folgendes Arzneimittel verdächtigt:

Diclo Dispers 50 mg Tabl., Betapharm, Einnahmezeitraum: 08/2009-10.03.2010.

Das ANV ist am wahrscheinlichsten auf eine Nierenschädigung durch Diclofenac zurückzuführen (siehe oben histologischen Befund der Nierenbiopsie).<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10158770

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 31 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anticoagulation drug level above therapeutic	20101004			46 Day	recovering/resolving
2 International normalised ratio increased	20101004			46 Day	recovered/resolved
3 Drug interaction	20101004			46 Day	recovering/resolving
4 Bleeding tendency	20101004			46 Day	recovering/resolving
5 Quick's value decreased	20101004			46 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 MARCUMAR phenprocoumon	Oral	/	2009-20101101 -	Thrombophilia
2 MARCUMAR phenprocoumon		/	201011- -	
3 PROCORALAN 7.5 mg ivabradine	Oral	7.5 Mg milligram(s)/Day 82	20100820-20101101 74 Day	Sinus tachycardia
4 VOLTAREN /00372301/ diclofenac	Oral	150 Mg milligram(s)/Day	20100916-20100923 8 Day	Ligament injury

Drugs (concomitant)

5 CONCOR /00802601/ bisoprolol	Oral	2.5 Mg milligram(s)/Day	- -	
6 Folsaeure folic acid	Oral	5 Mg milligram(s)/Day	- -	
7 HYDROCHLOROTHIAZIDE hydrochlorothiazide	Oral	12.5 Mg milligram(s)/Day	2009- -	
8 LYRICA pregabalin		/	- -	
9 PANTOZOL /01263202/ pantoprazole sodium		40 Mg milligram(s)/Day	- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Myocarditis	200803		Unknown	
Pacemaker insertion (cardiac)	20100114		Unknown	
Pulmonary embolism	200104		Unknown	
Tachycardia	200011		Unknown	recurrence in OCT-2002
Foramen ovale patent	200103		Unknown	Surgery was performed
Convulsions generalized	20040228		Unknown	
Cardiac failure acute	200212		Unknown	
AV block third degree			Unknown	
Atrial flutter	2008		Unknown	
Apoplexy	201008		Unknown	
Atrial fibrillation	2001		Yes	
Surgery herniated nucleus pulposus	20090622		Unknown	
Stroke			Yes	
Thrombophilia			Yes	Idiopathic thrombophilia
Coronary heart disease			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

SERVIER, 15.10.2010 DE-SERVIER-S10004368

Sender diagnosis: not reported

Narrative

This report was received by the company from a physician in GERMANY.

A 30-year-old female patient (nurse) with a medical history of idiopathic thrombophilia, stroke, coronary heart disease and atrial fibrillation, has been treated with PROCORALAN 5 mg due to coronary heart disease since JUN-2010.

Concomitant medication included ACE inhibitor and diuretic which were not specified for coronary heart disease and PHENPROCOUMON (Marcumar) due to stroke and atrial fibrillation diagnosed for a long time.

In JUN-2010 the patient also took DICLOFENAC (Voltaren) for pain due to ligament injury (high dosage, details not known).

Around end of SEP-2010 she noticed that it was impossible to adjust her coagulation values: Quick's value was very low, INR was high and she had a very high PHENPROCOUMON blood level.

At the beginning of OCT-2010, DICLOFENAC and PHENPROCOUMON were stopped. As the values did not improve and the patient showed a bleeding tendency (gingival bleeding, rectal bleeding and a long secondary bleeding after blood sampling), she was hospitalized on 04-OCT-2010.

The reporter knew the patient only since then. Apart from the coagulation values, all lab results were normal, autoimmune diseases were excluded. In spite of a special therapy (Vitamin K ...) only slow improvement.

On 12-OCT-2010, laboratory examination was done and results were pending.

Not discharged at time of reporting. Recovering.

The reporter could not exclude that it was an interaction between PROCORALAN and PHENPROCOUMON. An interaction between DICLOFENAC and PHENPROCOUMON could also be not fully excluded, but DICLOFENAC was already stopped two weeks ago, nevertheless with no improvement.

SERVIER, 27.10.2010 DE-SERVIER-S10004368

Sender diagnosis: not reported

Narrative

This report was received by the company from a physician in GERMANY.

A 31-year-old female patient (nurse) has been treated with PROCORALAN 5 mg since 20-AUG-2010, increased to 7.5 mg daily (date not specified) due to sinus tachycardia.

She has heavy medical history, including fulminant pulmonary artery embolism in 2001, intervention on persistent oval foramen occlusion in MAR-2001 and intermittent atrial fibrillation firstly diagnosed in 2001 (successful isthmus ablation), idiopathic thrombophilia with low Protein C and S due to heterozygote MTHFR (methylentetrahydrofolate reductase) mutation diagnosed in DEC-2001 on long-term anticoagulation, recurrent short complex tachycardia in 2002, cardiac decompensation 2004, strain-induced multiple generalised seizures (diff. diagnosis Konakion intolerance) in 2004, atrial flutter in 2008, surgery due to herniation of nucleus pulposus L5/S1 in JUN-2009, parvovirus B10 myocarditis in JUN-2009, condition after implantation of DDD pace-maker in JAN 2010 due to sino-atrial block and intermittent AVB 3rd degree. In AUG-2010 she had a media brain infarction with hemiparesis left after an electrophysiological examination while anticoagulation was interrupted.

Concomitant medication included since 2009 MARCUMAR (phenprocoumon), Ramipril 5 mg, Concor (bisoprolol) 2.5 mg and Esidrix

(hydrochlorothiaside) 12.5 mg for CHD, Lyrica (pregabalin) 230 mg resp. 175 mg, Folic acid 5 mg and Pantozol 40 mg (indication and start date not specified).

After the cerebral infarction in AUG-2010 the patient had restarted Marcumar and went to rehabilitation. She was given 4.5 to 4.75 mg Marcumar daily depending on the INR value.

The patient took VOLTAREN (diclofenac) 150 mg daily from 16 to 23-SEP-2010 for pain due to a ligament injury.

Since end of SEP-2010, INR was no more measurable with the patient's measuring instrument. Therefore, she interrupted Marcumar. However, already after a 10-day-pause, increased nasal and anal bleeding occurred (light red blood).

On 05-OCT-2010 the patient was hospitalized with anal bleeding and a no longer measurable Quick's and INR value. Apart from that, the patient said that she felt well, she had no pain.

Follow-up information received on 13-OCT-2010 regarding lab results which showed abnormal Quick value (<5 on 05 and 06-OCT ; NR 70-110%), INR (9.7 on 04-OCT; NR:2.0-3.0), PTT (61sec on 04-OCT; NR: 22-31 sec), Factor XIII (>120% on 04-OCT; NR:70-120), antithrombin (123% on 04-OCT, NR:78-109).

Haemoglobin and haematocrit slightly was slightly decreased on 05-OCT (11.1 g/dl and 33.5% respectively).

Autoimmune disease was excluded (no evidence of factor-specific inhibitors, or of lupus inhibitors).

On 07-OCT-2010, while Marcumar was stopped since 12 days phenprocoumon serum concentration was 2.6 mg/l , within NR (1.0-3.0) which led the reported to conclude that the patient should have had an extremely elevated Marcumar level.

Under Vitamin K administration per os, the Quick's value did not increase. Due to the increased nasal bleeding PPSB (prothrombin) was administered.

After repeated Vitamin K and PPSB administration the Quick's value increased and the bleeding almost completely stopped.

On 12-OCT-2010, laboratory examination showed Quick still low at 28%, normalized INR at 2.3, slightly elevated PTT at 32 sec; Hb was 11.4 g/dL and Ht 33.4%.

On 13-OCT-2010 the patient was discharged in improved condition with Quicks value 15% and INR 3.2.

PROCORALAN 7.5 mg one tablet in the morning was prescribed at discharge (no information regarding action taken during the hospitalisation)

According to the assessment of the reporter the reason for the event was not clear, however this might have been caused by the too high dosage of MARCUMAR (MARCUMAR intoxication). Additionally, an interaction between MARCUMAR and VOLTAREN (taken one week earlier) and PROCORALAN might be taken into consideration.

SERVIER, 10.12.2010 DE-SERVIER-S10004368

Sender diagnosis: not reported

Narrative

This report was received by the company from a physician in GERMANY.

A 31-year-old female patient (nurse) has been treated with PROCORALAN 5 mg since 20-AUG-2010, increased to 7.5 mg daily (date not specified) due to sinus tachycardia.

She has heavy medical history, including fulminant pulmonary artery embolism in 2001, intervention on persistent oval foramen occlusion in MAR-2001 and intermittent atrial fibrillation firstly diagnosed in 2001 (successful isthmus ablation), idiopathic thrombophilia with low Protein C and S due to heterozygote MTHFR (methylentetrahydrofolate reductase) mutation diagnosed in DEC-2001 on long-term anticoagulation, recurrent short complex tachycardia in 2002, cardiac decompensation 2004, strain-induced multiple generalised seizures (diff. diagnosis Konakion intolerance) in 2004, atrial flutter in 2008, surgery due to herniation of nucleus pulposus L5/S1 in JUN-2009, parvovirus B10 myocarditis in JUN-2009, DDD pace-maker in JAN 2010 due to sino-atrial block and intermittent AVB 3rd degree. In AUG-2010 she had a media brain infarction with hemiparesis left after an electrophysiological examination while anticoagulation was interrupted.

Concomitant medication included since 2009 MARCUMAR (phenprocoumon), Ramipril 5 mg, bisoprolol 2.5 mg and hydrochlorothiazide 12.5 mg, pregabalin 230 mg resp. 175 mg, folic acid 5 mg and pantoprazole 40 mg (indication and start date not specified).

After the cerebral infarction in AUG-2010 the patient had restarted Marcumar and went to rehabilitation. She was given 4.5 to 4.75 mg Marcumar daily depending on the INR value.

The patient took VOLTAREN (diclofenac) 150 mg daily from 16 to 23-SEP-2010 for pain due to a ligament injury.

Since end of SEP-2010, INR was no more measurable with the patient's measuring instrument. Therefore, she interrupted Marcumar. However, already after a 10-day-pause, increased nasal and anal bleeding occurred (light red blood).

On 05-OCT-2010 the patient was hospitalized with anal bleeding and a no longer measurable Quick's and INR value. Apart from that, the patient said that she felt well, she had no pain.

Follow-up information (received on 13-OCT-2010) regarding lab results which showed abnormal Quick value (<5 on 05 and 06-OCT ; NR 70-110%), INR (9.7 on 04-OCT; NR:2.0-3.0), PTT (61sec on 04-OCT; NR: 22-31 sec), Factor XIII (>120% on 04-OCT; NR:70-120), antithrombin (123% on 04-OCT, NR:78-109).

Haemoglobin and haematocrit was slightly decreased on 05-OCT (11.1 g/dl and 33.5% respectively).

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On 12-OCT-2010, laboratory examination showed Quick still low at 28%, normalized INR at 2.3, slightly elevated PTT at 32 sec; Hb was 11.4 g/dL and Ht 33.4%.

On 13-OCT-2010 the patient was discharged in improved condition with Quicks value 15% and INR 3.2.

PROCORALAN 7.5 mg one tablet in the morning was prescribed at discharge (no information regarding action taken during the hospitalisation)

According to the assessment of the reporter the reason for the event was not clear, however this might have been caused by the too high dosage of MARCUMAR (MARCUMAR intoxication). Additionally, an interaction between MARCUMAR and VOLTAREN (taken one week earlier) and PROCORALAN might be taken into consideration.

Follow up information was received on 25-NOV-2010.

After discharge from hospital PROCORALAN was maintained. The INR values were between 3 and 4 (unchanged high). PROCORALAN was stopped (date not known). After then INR had been normalised (currently 2). As the reporter was not the attending GP of the patient, he had no more information. The reporter thought that an interaction between MARCUMAR PROCORALAN might be taken into consideration as reason of the event. Proposed comment for the BfArM remained unchanged.

Follow up information was received on 02-DEC-2010.

The patient (nurse) informed that on 17-OCT-2010 she measured INR value over 3 (self measurement). Control measurement on 01-NOV-2010 showed further elevation to 4.2. On 01-NOV-2010, MARCUMAR was paused and PROCORALAN stopped. Beta blocker (bisoprolol) was increased. On 03-NOV-2010, INR 3 was measured. Therefore MARCUMAR was restarted with increasing dosage. Since then the patient had INR values between 2 and 3 under regularly MARCUMAR treatment (3 to 4,5 mg daily). The patient assessed that the event might be caused by an interaction between PROCORALAN and MARCUMAR.

Proposed comment for the BfArM remained unchanged.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10162949

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19280409	Age: 82 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Colitis	20100926	20101006	11 Day		recovered/resolved
2 Creatinine increased					Unknown
3 Diarrhoea	20100926	20101006	11 Day		recovered/resolved
4 Gamma GT increased					Unknown
5 Gastritis erosive					recovered/resolved
6 Haemoglobin decreased					Unknown
7 Hypotension					Unknown
8 Aspartate aminotransferase increased					Unknown
9 Activated partial thromboplastin time decreased					Unknown
10 Exsiccosis					Unknown
11 Mucous membrane disorder					Unknown
12 Tiredness					Unknown
13 Weakness					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 100 acetylsalicylic acid	Oral	100 Mg milligram(s)/Day 245	- -	Drug use for unknown indication
2 diclofenac diclofenac	Oral	100 Mg milligram(s)/Day 245	-20101003 -	Chronic pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ache			Yes	Beginn unbekannt; chronisches Schmerzsyndrom der Wirbelsäule
Gastritis erosive			Yes	
Pain in spine			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Activated partial thromboplastin time	20101003	21.5	sec	
Alanine aminotransferase	20101003	0.18	µmol/s.l.	
Alkaline phosphatase	20101003	1.96	µmol/l	
Aspartate aminotransferase	20101003	0.54	µmol/l.s	
Creatinine	20101003	118	µmol/l	
Gamma GT	20101003	0.63	µmol/s.l	
Haematocrit	20101003	0.35		
Haemoglobin	20101003	6.95	mmol/l	
INR	20101003	1.05		

Mean corpuscular haemoglobin	20101003	94.1	fl
Potassium	20101003	3.77	mmol/l
Sodium	20101003	140	mmol/l
Thrombocyte count	20101003	209	Gpt/l
Quick's test	20101003	88.5	Prozent
Total white blood cell count	20101003	13.3	Gpt/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 25.10.2010 DE-PVZIBE-07-10399

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Medikamentös-toxische Kolitis

KURZZUSAMMENFASSUNG:

Medikamentös-toxische Kolitis unter ASS und Diclofenac.

BESCHREIBUNG:

Einweisung wegen Schwäche bei Durchfall. Bei der Patientin bestanden seit 26.09.10 rezidivierende Durchfälle (4/d). Die Patientin fühlte sich entkräftet und müde.

THERAPIE:

- Am 03.10.10 (Aufnahmetag) wurde Diclofenac abgesetzt.
- 03.10-06.10.10: Flüssigkeitssubstitution
- ab 03.10.10: Salofalk 1 g (3/d)
- ab 03.10-10 Pantozol 40 (1/d)

VERLAUF:

Aufnahme Notarzt: EKG: normofrequenter Sinusrhythmus, Blutdruck: 90/45 mmHg, trockene Schleimhäute

- Röntgen-Thorax: regelrechter Befund
- Oberbauchsonographie: unauffällig
- Gastroskopie: erosive Gastritis
- Koloskopie: V.a. nicht klassifizierbare Kolitis (med.-tox.) bei Einnahme von ASS + Diclofenac.

Kein Nachweis von Rota-, Adeno- u. Noroviren bzw. Chlostridien-Toxin. Nach Absetzen von Diclofenac ging es der Patientin ab 06.10.10 wieder besser. Es traten keine Durchfälle seit dieser Zeit auf.

ERLÄUTERUNGEN ZUR UAW:

Lt. Entlassungsbrief-Diagnose: Medikamentös-toxische Kolitis unter ASS und Diclofenac. Symptomatik wird als UAW eingestuft. Entlassungsbrief: "Endoskopisch fand sich makroskopisch der Verdacht auf eine medikamentös-toxische Kolitis unter ASS und Diclofenac." Diclofenac wurde pausiert und die Durchfälle sistierten. Chronologie/Symptomatologie plausibel. Ein infektiöses Geschehen wurde ausgeschlossen. Exsikkose mit daraus resultierender Hypotonie wurden als Folge der Diarrhoe gewertet.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:**MEDIKAMENT:** ass 100**BETREFFENDE UAW:** Kolitis

ERLÄUTERUNG: In der Literatur wird für ASS ein schleimhaut-schädigendes Potenzial beschrieben für die Schleimhaut im gesamten GI-Trakt. ASS wurde nicht abgesetzt, aber es muss dennoch mitverdächtigt werden.

MEDIKAMENT: diclofenac**BETREFFENDE UAW:** Kolitis

ERLÄUTERUNG: In der Literatur wird für NSAR die Ausbildung einer medik.-tox. Kolitis beschrieben. Nach dem Absetzen sistierten die Durchfälle. Chronologie/Symptomatik plausibel.

NOVARTIS, 27.10.2010 DE-002147023-GXKR2010DE11974

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 25 Oct 2010 from the German Health Authority (DE-BfArM-10162949) provided by a regional pharmacovigilance center, reported by a physician: This patient with a medical history of ache developed colitis during treatment with diclofenac (MAH unknown) and acetylsalicylic acid (MAH unknown). The outcome of the adverse event was reported as recovered/resolved. Causality was assessed as possible for both suspected drugs.

Serious spontaneous report (hospitalisation) of colitis, already listed in the Core Data Sheet Information.
All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

sanofi-aventis, 05.11.2010 DE-SANOFI-AVENTIS-2010SA065036

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-07-10399) and was extracted from the website of the German health authority (authority number: DE-BFARM-10162949) on 26-Oct-2010. The pharmacovigilance (PV) centre is conducting a hospitalization registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves an 82-year-old female with chronic pain syndrome of vertebrae. The patient was treated with acetylsalicylic acid (no trade name provided) 100 mg/day for an unknown indication and diclofenac (no trade name provided) 100 mg/day for chronic pain. Start dates of the both medications were unknown. Concomitant medication was not reported.

On 26-Sep-2010 the patient developed recurrent diarrhea (4 times daily). She felt exhausted and tired. On 03-Oct-2010 the patient was referred to hospital by an emergency physician due to weakness under diarrhea. The same day diclofenac was discontinued and treatments with mesalazine 3x1 g daily and pantoprazole 40 mg daily were initiated. In addition, the patient received fluid substitution from 03-Oct-2010 to 06-Oct-2010. Acetylsalicylic acid therapy was continued. ECG, thorax x-ray, upper abdominal ultrasound found no abnormality. Gastroscopy disclosed erosive gastritis. Colonoscopy revealed suspicion of non-classifiable colitis (drug-induced toxic) under oral treatment with acetylsalicylic acid and diclofenac. No rotavirus, adenovirus and norovirus as well as clostridial toxins were detected. On 06-Oct-2010 the patient's condition improved. New episodes of diarrhea hadn't been observed since then.

The PV centre reported drug-induced toxic colitis as adverse drug reaction. Acetylsalicylic acid and diclofenac were suspected. Outcome of the event was reported as "recovered/resolved on 06-Oct-2010".

Explanatory notes on ADR by the PV centre:

Diagnosis in the hospital discharge report: drug-induced, toxic colitis under acetylsalicylic acid and diclofenac. Symptom is classified as ADR. Hospital discharge report: "According to endoscopy suspicion of drug-induced, toxic colitis under acetylsalicylic acid and diclofenac". Diclofenac was stopped and then diarrhea subsided. Chronology/symptomatology plausible. Infectious origin was excluded. Exsiccosis with resulting hypotension was assessed as consequence of the diarrhea.

Causality assessment by the PV centre concerning colitis:

"possible": acetylsalicylic acid, diclofenac

Explanatory notes on the causalities by the PV centre:

Acetylsalicylic acid: According to the literature, acetylsalicylic acid has a potential to damage mucous membrane in the entire gastrointestinal tract.

Acetylsalicylic acid was not discontinued, but has also to be co-suspected of causing the event.

Diclofenac: In the literature, development of drug-induced, toxic colitis under NSAR was reported. After discontinuation of the drug, diarrhea subsided. Chronology/symptomatology plausible.

NOVARTIS, 09.11.2010 DE-002147023-PHHY2010DE72635

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-10162949) on 25 Oct 2010: This patient with medical history of ache was on treatment with diclofenac (manufacturer unknown) 50 mg tablets two times per day orally for chronic pain from unspecified date. She was also on treatment with ASS 100 (acetyl salicylic acid) 100 mg tablet once a day orally for unspecified indication. She was hospitalized due to weakness associated with diarrhea. She was suffering recurrent diarrhea (four times a day) since 26 Sep 2010. She felt weakened and tired. Therapy with diclofenac was discontinued on the day of admission on 03 Oct 2010. She received Salofalk (mesalazine) three times per day and Pantozol (pantoprazole) 40 once a day on 03 Oct 2010. She was put on fluid replacement therapy 03 Oct 2010 to 6 Oct 2010. The lab data showed haemoglobin at 6.95 mmol/l (normal value 7.5 to 9.5), haematocrit at 0.35 (normal value 0.36 to 0.46), total white blood cell count at 13.3 Gpt/l (normal value 3.8 to 10), activated partial thromboplastin time at 21.5 (normal value 26 to 36), creatinine at 118 micromol/l (normal value < 97), aspartate aminotransferase 0.54 micromol/s.l (normal value < 0.52) and gamma GT at 0.63 micromol/s.l (normal value < 0.53) on 03 Oct 2010. The electrocardiogram (ECG) showed normofrequent sinus rhythm, blood pressure at 90/45 mmHg, dry mucous membranes. An infectious etiology was ruled out. The exsiccosis with resulting hypotension were interpreted as a consequence of the diarrhea. The chest radiography showed normal findings and the upper abdominal sonography was unremarkable. The gastroscopy showed erosive gastritis. The colonoscopy revealed suspected unclassifiable colitis (drug toxic) associated with intake of ASS and diclofenac. There was no evidence of rotavirus, adenovirus and norovirus or clostridia toxins. After discontinuation of diclofenac, the patient felt better again from 6 Oct 2010. No diarrhea has occurred since this time. The endoscopic gross findings raise the suspicion of drug toxic colitis during ASS and diclofenac treatment. The relation

between the drugs and event was reported as possible. It was reported that the treatment with ASS was not discontinued but must nevertheless be suspected of causal involvement. In the absence of the reporting healthcare professional's causality, the event gastritis erosive has been assessed as suspected for reporting purposes; the event " hemoglobin decreased, activated partial thromboplastin time shortened, blood creatinine increased, gamma-glutamyltransferase increased and aspartate aminotransferase increased" was assessed as non-serious.

HELIOS Klinikum Wuppertal, 25.11.2010 DE-PVZIBE-07-10399

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Medikamentös-toxische Kolitis

KURZZUSAMMENFASSUNG:

Medikamentös-toxische Kolitis unter ASS und Diclofenac.

BESCHREIBUNG:

Einweisung wegen Schwäche bei Durchfall. Bei der Patientin bestanden seit 26.09.10 rezidivierende Durchfälle (4/d). Die Patientin fühlte sich entkräftet und müde.

THERAPIE:

- Am 03.10.10 (Aufnahmetag) wurde Diclofenac abgesetzt.
- 03.10-06.10.10: Flüssigkeitssubstitution
- ab 03.10.10: Salofalk 1 g (3/d)
- ab 03.10-10 Pantozol 40 (1/d)

VERLAUF:

Aufnahme Notarzt: EKG: normofrequenter Sinusrhythmus, Blutdruck: 90/45 mmHg, trockene Schleimhäute

- Röntgen-Thoax: regelrechter Befund

- Oberbauchsonographie: unauffällig

- Gastroskopie: erosive Gastritis

- Koloskopie: V.a. nicht klassifizierbare Kolitis (med.-tox.) bei Einnahme von ASS + Diclofenac.

Kein Nachweis von Rota-, Adeno- u. Noroviren bzw. Chlostridien-Toxin. Nach Absetzen von Diclofenac ging es der Patientin ab 06.10.10 wieder besser. Es traten keine Durchfälle seit dieser Zeit auf.

ERLÄUTERUNGEN ZUR UAW:

Lt. Entlassungsbrief-Diagnose: Medikamentös-toxische Kolitis unter ASS und Diclofenac. Symptomatik wird als UAW eingestuft. Entlassungsbrief: "Endoskopisch fand sich makroskopisch der Verdacht auf eine medikamentös-toxische Kolitis unter ASS und Diclofenac." Diclofenac wurde pausiert und die Durchfälle sistierten. Chronologie/Symptomatologie plausibel. Ein infektiöses Geschehen wurde ausgeschlossen. Exsikkose mit daraus resultierender Hypotonie wurden als Folge der Diarrhoe gewertet.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ass 100

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: In der Literatur wird für ASS ein schleimhaut-schädigendes Potenzial beschrieben für die Schleimhaut im gesamten GI-Trakt. ASS wurde nicht abgesetzt, aber es muss dennoch mitverdächtigt werden.

Auch für ASS sind "gastrointestinale Entzündungen" beschrieben (s. z.B. Fachinfo Aspirin protect).

MEDIKAMENT: diclofenac

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: In der Literatur wird für NSAR die Ausbildung einer medik.-tox. Kolitis beschrieben. Nach dem Absetzen sistierten die Durchfälle. Chronologie/Symptomatologie plausibel.

(In der Fachinformation Voltaren findet sich: "Beschwerden im Unterbauch - z.B. blutende Colitiden" / Durchfälle sind als häufige UAWs beschrieben)

<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10171633

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19270829	Age: 82 Year	weight (kg): 58	BMI_int: 24.14	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Colitis	20100701			6 Month	recovering/resolving
2 Abdominal pain	20100701				recovering/resolving
3 Bleeding					recovering/resolving
4 Blood pressure decreased					Unknown
5 Circulatory collapse	20100706				recovering/resolving
6 Gastrointestinal erosion	20100701				recovering/resolving
7 Pancolitis	20100701				recovering/resolving
8 Colopathy					not recovered/not resolved
9 Thrombin time prolonged					Unknown
10 Watery diarrhea	20100701				not recovered/not resolved
11 Fluid loss					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure 100 mg acetylsalicylic acid	Oral	100 Mg milligram(s)/Day 245	201001-20100716 -	Myocardial infarction prophylaxis
2 diclofenac retard 75mg diclofenac sodium	Oral	150 Mg milligram(s)/Day 212	201003-20100706 -	Polymyalgia rheumatica
3 prednisolon 12.5mg prednisolone	Oral	12.5 Mg milligram(s)/Day 245	20100320- -	Polymyalgia rheumatica

Drugs (concomitant)

4 allopurinol 300mg allopurinol	Oral	150 Mg milligram(s)/Day 245	201003- -	Hyperuricaemia
5 atacand 8 mg candesartan cilexetil	Oral	4 Mg milligram(s)/Day 245	201001-20100706 -	Hypertension arterial
6 calcimagon d3 calcium carbonate; colecalciferol	Oral	2*1 Df dosage form/ Day 8	201003- -	Osteoporosis
7 carvedilol 25 mg carvedilol	Oral	12.5 Mg milligram(s)/Day 82	201001-20100706 -	Hypertension arterial
8 mesalazine mesalazine	Rectal	3 G gram(s)/Day 218	201003- -	Ischaemic colitis
9 pantoprazol 40mg pantoprazole	Oral	40 Mg milligram(s)/Day 94	201003- -	Helicobacter pylori gastritis
10 simvastatin 10mg simvastatin	Oral	10 Mg milligram(s)/Day 82	201001- -	Myocardial infarction prophylaxis
11 spironolacton 50 plus Heumann furosemide; spironolactone	Oral	2 Df dosage form/Day 245	201001-20100706 -	Hypertension arterial
12 thyronajod 75 henning levothyroxine sodium; potassium iodide	Oral	1 Df dosage form/Day 245	201001- -	Hypothyroidism
13 tilidin retard tilidine	Oral	1 Df dosage form/Day 212	201003- -	Polymyalgia rheumatica

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic renal impairment			Yes	Beginn unbekannt, Stadium 3
Normochromic normocytic anemia	201003		Yes	
Hyperuricaemia	201003		Yes	
Osteoporosis			Yes	Beginn unbekannt
Coronary disease			Yes	Beginn unbekannt, 04/2003 PTCA und Stent-RCX
Myocardial infarction			Unknown	
Hypertension arterial			Yes	Beginn unbekannt
Polymyalgia rheumatica	201003		Yes	mit Arteriitis temporalis
Ischaemic colitis	201003	201003	No	
Helicobacter pylori gastritis	201003	201003	No	Eradikationstherapie 03/2010
Hyperuricaemia	201003		Yes	hyperuricaemia
Hypothyroidism	1988		Yes	nach Schilddrüsenteilresektion 1988, aktuell euthyreot

Primary source

Qualification: Pharmacist observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20100706	0.21	µkat/l	
Aspartate aminotransferase	20100706	0.24	µkat/l	
Blood potassium	20100716	4.8	mmol/l	
Blood potassium	20100706	6.5	mmol/l	
Creatinine	20100706	245	µmol/l	
Creatinine	20100708	107	µmol/l	
Fibrinogen	20100716	7.0	g/l	
Haematocrit	20100716	0.33	SI-Einheit	SI-Einheit
Haematocrit	20100706	0.34	SI-Einheit	
Haemoglobin	20100706	6.70	mmol/l	
Haemoglobin	20100716	6.30	mmol/l	
Immunoglobulin A	20100708	1.02	g/l	
INR	20100716	0.9	Physiologis	
Partial prothrombin time	20100716	52.4	sec	
Sodium	20100706	126	mmol/l	
Sodium	20100716	139	mmol/l	
Thrombin time	20100716	71.2 Sec	sec	
Urea	20100706	20.3	mmol/l	
Antithrombin III	20100716	94	Prozent	
Quick's test	20100716	116	Prozent	
Total white blood cell count	20100706	13.20 10E9/l	10E9/l	
Colonoscopy	20100713	#	NA	

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 05.11.2010 DE-PVZIBE-02-10306

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Kolitis

KURZZUSAMMENFASSUNG:

Kolitis nach Einnahme von Prednisolon, Diclofenac und ASS

BESCHREIBUNG:

seit 01.07.2010 anhaltender, unkontrollierbarer wässriger Durchfall, bis 10x pro Tag, mit krampfartigen Bauchschmerzen;

seit 04.07.2010 mit hellroten Blutbeimengungen;

06.07.2010 Vorstellung beim Hausarzt wegen des Durchfalls, dort hat Patientin einen Kreislaufkollaps erlitten, Einweisung ins Krankenhaus;

bei Einweisung: deutliche klinische Zeichen der Exsikkose, keine krampfartigen Bauchschmerzen, aber weiterhin bestehender Durchfall

THERAPIE:

intravenöser Flüssigkeits- und Elektrolytersatz;

06.07.2010 Absetzen von Diclofenac und Umstellung auf Metamizol 4x30ggt p.o.;

06.-13.07.2010 Metronidazol 3x500mg p.o.;

06.-23.07.2010 Benefiber 1x1EL;

08.-23.07.2010 Perenterol forte 2x1 Kapsel p.o.;

14.07.2010 Absetzen von ASS und Umstellung auf Plavix 1x75mg p.o.;

16.07.2010 Transfusion von 1 EKKL;

Reduktion der antihypertensiven Medikation bei Flüssigkeitsverlust wegen der Durchfälle

VERLAUF:

07.07.2010 Stuhlprobe: negativ: Toxin A/B von Clostridium difficile, Enteritis coli, Salmonellen, Shigellen, Yersinien, Campylobacter, Adenoviren, Rotaviren, Cryptosporidien, Lamblien, Astro- und Noroviren;

08.07.2010 Ösophagogastroduodenoskopie: Ösophagus mit unauffälliger Schleimhaut, Magenschleimhaut leicht gerötet ohne Läsionen, Faltenrelief unauffällig, ÖGD insgesamt unauffällig;

08.07.2010 Labor: negativ: Anti-Transglutaminase IgA, Anti-Transglutaminase IgG, Gliadin IgA-Ak, Gliadin IgG-Ak;

13.07.2010 Koloskopie: Verdacht auf NSAR-Kolopathie mit Pankolitis und Erosionen im Colon ascendens und auf der Bauhinschen Klappe;

13.07.2010 Histo der Proben aus der ÖGD vom 08.07.10: keine sicheren Kriterien einer Sprue, keine Hinweise auf Whipple disease;

16.07.2010 Histo der Proben aus der Koloskopie vom 13.07.10: Ileum-Schleimhaut-Bioptate: ohne morphologische Hinweise auf eine chronisch entzündliche Darmerkrankung,

Biopate aus der Ileozökalclappe und dem C.ascendens: mit einer chronischen und aktiven Kolitis, möglicherweise Kombination einer NSAR-Kolopathie mit jetzt infektiöser Kolitis-Komponente, klinisch laufende Immunsuppression,

Colon transversum, descendens und Sigma: mäßig aktive Kolitis,

insgesamt: in allen Abschnitten auffällige Atrophie der Schleimhaut;

Blutdruck während des stationären Aufenthaltes 90-100/50mmHg;

Durchfall bis zur Entlassung am 23.07.2010, allgemeine Kreislaufsituation stabilisiert, Entlassung in die Häuslichkeit möglich

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 06.-23.07.2010;

Krankenseinweisung aufgrund einer Kolitis nach Einnahme von Prednisolon, Diclofenac und ASS;

Besserung nach Absetzen von Diclofenac und ASS, sowie nach symptomatischer Therapie

Infektiöse Diarrhoe-Erreger wurden nicht nachgewiesen. Eine ischämische Genese wurde von den behandelnden Ärzten differentialdiagnostisch nicht in Erwägung gezogen.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: acetylsalicylic acid 100mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Schädigungen der Darmschleimhaut unter ASS sind bekannt und in der Fachinfo beschrieben.

MEDIKAMENT: prednisolon 12.5mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Ein schädigender Einfluss von Prednisolon auf die Darmschleimhaut ist bekannt und beschrieben. Dieser kann bei gleichzeitiger Einnahme von NSAR (hier ASS und Diclofenac) noch verstärkt werden.

MEDIKAMENT: diclofenac retard 75mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Eine Schädigung der Darmschleimhaut durch Diclofenac ist bekannt und in der Fachinfo beschrieben.<dbtext>

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NOVARTIS, 10.11.2010 DE-002147023-GXKR2010DE12515

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 08 Nov 2010 from the German Health Authority (DE-BfArM-10171633) provided by a regional pharmacovigilance center, reported by a pharmacist: This multimorbid patient developed colitis during treatment with acetylsalicylic acid (MAH unknown), prednisolone (MAH unknown) and diclofenac (MAH unknown). Several other drugs were taken concomitantly. The outcome of the adverse event was reported as recovering/resolving. Causality was assessed as possible for all suspected drugs.

Serious spontaneous report (hospitalisation) colitis, already listed in the Core Data Sheet Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Bayer Vital GmbH, 16.11.2010 DE-BAYER-201045703GPV

Sender diagnosis: not reported

Narrative

This solicited report derived from a pharmacist in Germany and was forwarded to the German Health Authority by a pharmacovigilance center, which systematically screens for adverse drug reactions in selected hospitals. The report was identified on the ADR- internet page of the German Health Authority (BfArM) on 08-NOV-2010, referring to an 82 year-old female.

Medical and drug history:

Patient had concomitant conditions of Chronic renal impairment, Normochromic normocytic anemia, Hyperuricaemia, chronic renal impairment, Normochromic normocytic anemia, hyperuricaemia, osteoporosis, coronary disease, hypothyroidism, hypertension arterial, and polymyalgia rheumatica. Patient's relevant medical history includes ischaemic colitis, and Helicobacter pylori gastritis. Concomitant drugs have been given for hyperuricaemia, osteoporosis, hypertension arterial, helicobacter pylori gastritis, myocardial infarction prophylaxis, hypothyroidism, and polymyalgia rheumatica. For details see section "Concomitant Drugs" at the end of this report.

Suspect drugs, timing and conditions surrounding the onset of the reaction:**Patient received:**

- acetylsalicylic acid (acetylsalicylic acid) for myocardial infarction prophylaxis. Administration began in Jan-2010 at a total daily dose of 100 MG oral.
- prednisolon (prednisolone) for polymyalgia rheumatica. Drug was administered on 20-Mar-2010 and continuing at time of event at a total daily dose of 12.5 MG oral.
- diclofenac retard (diclofenac) for polymyalgia rheumatica. Administration began in May-2010 at a total daily dose of 150 MG oral.

The progression of the event and its outcome in the patient:

On 01-Jul-2010, patient experienced COLITIS considered serious due to hospitalization. acetylsalicylic acid was withdrawn The dose of prednisolon was not changed diclofenac retard was withdrawn, and the patient improved.

Therapy for COLITIS: Intravenous fluid and electrolyte replacement

06-JUL-2010: Diclofenac was discontinued and switched to metamizole (4x30 gtts per day, oral intake)

06-JUL-2010 to 13-JUL-2010: Metronidazole (4x500 mg, oral intake)

06-JUL-2010 to 23-JUL-2010: Benefiber (fiber supplement, 1x1 tablespoon)

08-JUL-2010 to 23-JUL-2010: Perenterol forte (yeast, 2x1 capsules)

14-JUL-2010: ASS was discontinued and switched to Plavix (clopidogrel 1x75 mg, oral intake)

16-JUL-2010: Transfusion of one erythrocyte concentrate

With diarrhea-related fluid loss the antihypertensive medication was reduced.

The original reporter's clinical assessment: For the all suspect drugs acetylsalicylic acid, prednisolon and diclofenac retard the reporter considered the diagnosis COLITIS to be possibly related.

Galenpharma GmbH, 16.11.2010 DE-HRB2237KIP-000086

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Kolitis

KURZZUSAMMENFASSUNG:

Kolitis nach Einnahme von Prednisolon, Diclofenac und ASS

BESCHREIBUNG:

seit 01.07.2010 anhaltender, unkontrollierbarer wässriger Durchfall, bis 10x pro Tag, mit krampfartigen Bauchschmerzen;
 seit 04.07.2010 mit hellroten Blutbeimengungen;
 06.07.2010 Vorstellung beim Hausarzt wegen des Durchfalls, dort hat Patientin einen Kreislaufkollaps erlitten, Einweisung ins Krankenhaus;
 bei Einweisung: deutliche klinische Zeichen der Exsikkose, keine krampfartigen Bauchschmerzen, aber weiterhin bestehender Durchfall

THERAPIE:

intravenöser Flüssigkeits- und Elektrolytersatz;
 06.07.2010 Absetzen von Diclofenac und Umstellung auf Metamizol 4x30ggt p.o.;
 06.-13.07.2010 Metronidazol 3x500mg p.o.;
 06.-23.07.2010 Benefiber 1x1EL;
 08.-23.07.2010 Perenterol forte 2x1 Kapsel p.o.;
 14.07.2010 Absetzen von ASS und Umstellung auf Plavix 1x75mg p.o.;
 16.07.2010 Transfusion von 1 EKKL;
 Reduktion der antihypertensiven Medikation bei Flüssigkeitsverlust wegen der Durchfälle

VERLAUF:

07.07.2010 Stuhlprobe: negativ: Toxin A/B von Clostridium difficile, Enteritis coli, Salmonellen, Shigellen, Yersinien, Campylobacter, Adenoviren, Rotaviren, Cryptosporidien, Lamblien, Astro- und Noroviren;
 08.07.2010 Ösophagogastroduodenoskopie: Ösophagus mit unauffälliger Schleimhaut, Magenschleimhaut leicht gerötet ohne Läsionen, Faltenrelief unauffällig, ÖGD insgesamt unauffällig;
 08.07.2010 Labor: negativ: Anti-Transglutaminase IgA, Anti-Transglutaminase IgG, Gliadin IgA-Ak, Gliadin IgG-Ak;
 13.07.2010 Koloskopie: Verdacht auf NSAR-Kolopathie mit Pankolitis und Erosionen im Colon ascendens und auf der Bauhinschen Klappe;
 13.07.2010 Histo der Proben aus der ÖGD vom 08.07.10: keine sicheren Kriterien einer Sprue, keine Hinweise auf Whipple disease;
 16.07.2010 Histo der Proben aus der Koloskopie vom 13.07.10: Ileum-Schleimhaut-Bioptate: ohne morphologische Hinweise auf eine chronisch entzündliche Darmerkrankung,
 Biopate aus der Ileozökalclappe und dem C.ascendens: mit einer chronischen und aktiven Kolitis, möglicherweise Kombination einer NSAR-Kolopathie mit jetzt infektiöser Kolitis-Komponente, klinisch laufende Immunsuppression,
 Colon transversum, descendens und Sigma: mäßig aktive Kolitis,
 insgesamt: in allen Abschnitten auffällige Atrophie der Schleimhaut;
 Blutdruck während des stationären Aufenthaltes 90-100/50mmHg;
 Durchfall bis zur Entlassung am 23.07.2010, allgemeine Kreislaufsituation stabilisiert, Entlassung in die Häuslichkeit möglich

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 06.-23.07.2010;
 Krankenhauseinweisung aufgrund einer Kolitis nach Einnahme von Prednisolon, Diclofenac und ASS;
 Besserung nach Absetzen von Diclofenac und ASS, sowie nach symptomatischer Therapie
 Infektiöse Diarrhoe-Erreger wurden nicht nachgewiesen. Eine ischämische Genese wurde von den behandelnden Ärzten differentialdiagnostisch nicht in Erwägung gezogen.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: acetylsalicylic acid 100mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Schädigungen der Darmschleimhaut unter ASS sind bekannt und in der Fachinfo beschrieben.

MEDIKAMENT: prednisolon 12.5mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Ein schädigender Einfluss von Prednisolon auf die Darmschleimhaut ist bekannt und beschrieben. Dieser kann bei gleichzeitiger Einnahme von NSAR (hier ASS und Diclofenac) noch verstärkt werden.

MEDIKAMENT: diclofenac retard 75mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Eine Schädigung der Darmschleimhaut durch Diclofenac ist bekannt und in der Fachinfo beschrieben.<dbtext>
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sanofi-aventis, 18.11.2010 DE-SANOFI-AVENTIS-2010SA067964

Sender diagnosis: not reported

Narrative

Initial report:

This spontaneous case report had been created by a regional pharmacovigilance centre and was extracted from the website of the German Health Authority (BfArM, case number DE-BFARM-10171633).

The pharmacovigilance centre (PVZ) is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this

information the centre establishes diagnoses that are assessed by the centre using the WHO score.

The case involves an 82-year-old female patient with a medical history of chronic renal impairment, normochromic normocytic anemia, hyperuricemia, osteoporosis, coronary disease, hypothyroidism, arterial hypertension, polymyalgia rheumatica, ischemic colitis and helicobacter pylori gastritis. Concomitant medication included allopurinol, colecalciferol + calcium carbonate, carvedilol, mesalazine, pantoprazole, simvastatin, spironolactone + furosemide, levothyroxine sodium + potassium iodide, tilidine and candesartane cilexetil.

Suspected medication included acetylsalicylic acid (ASA, manufacturer not reported) from Jan-2010 to 16-Jul-2010 at a dose of 1x100 mg per day orally for myocardial infarction prophylaxis, prednisolone (manufacturer not reported) since 20-Mar-2010 at a dose of 1x12.5 mg per day orally for polymyalgia rheumatica and diclofenac from Mar-2010 to 06-Jul-2010 at a dose of 2x75 mg per day orally for polymyalgia rheumatica.

Since 01-Jul-2010 the patient suffered from persisting, uncontrollable watery diarrheas, up to 10 times per day, with cramp-like abdominal pain; since 04-Jul with light red admixtures of blood.

On 06-Jul-2010 she presented at the family doctor and there she suffered from circulatory collapse and was hospitalised. On admission to hospital a marked exsiccrosis was present, no cramp-like abdominal pain but continuing diarrhea. A colitis was diagnosed.

Intravenous fluid and electrolyte replacement was performed. Diclofenac was switched to metamizole and treatment with metronidazole, guar and saccharomyces boulardii was initiated. On 14-Jul ASA was switched to clopidogrel and on 16-Jul the patient received one erythrocyte concentrate.

The antihypertensive medication was reduced due to loss of fluid.

Diarrhea persisted until discharge home on 23-Jul-2010 but general circulatory condition was stable.

Causality assessment according to the PVZ:

ASA: possible

prednisolone: possible

diclofenac: possible

Causality explanations:

ASA: damage of mucous membrane under ASA is known and described in the product information.

prednisolone: damage of mucous membrane under prednisolone is known and described. This effect can be increased due to simultaneous intake of ASA and diclofenac.

diclofenac: damage of mucous membrane under diclofenac is known and described in the product information.

NOVARTIS, 23.11.2010 DE-002147023-PHHY2010DE76943

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (HA reference number DE BFARM 1017133) on 08 Nov 2010: This poly-medicated patient had medical history of chronic renal impairment, normochromic normocytic from Mar 2010, hyperuricaemia from Mar 2010, osteoporosis, coronary disease, hypothyroidism from 1988, hypertension arterial, Ischaemic colitis and helicobacter pylori gastritis in Mar 2010. She was on treatment with diclofenac retard (manufacturer unknown) 75 mg prolonged release tablet at 75 mg BID, orally for polymyalgia rheumatica form Mar 2010. She was also taking acetyl salicylic acid (manufacturer unknown) tablet at 100 mg, QD orally for myocardial infarction prophylaxis from Jan 2010 and prednisolon (manufacturer unknown) tablet at 12.5 mg QD, orally for polymyalgia rheumatica form 20 Mar 2010. She experienced persistent uncontrollable watery diarrhea up to ten times a day with spasmodic abdominal pain since 01 Jul 2010. She also experienced admixtures of bright red blood since 04 Jul 2010. She attended a primary healthcare provider due to diarrhoea on which occasion she suffered a circulatory collapse and was admitted to hospital on 06 Jul 2010. The lab data showed creatinine at 245 micromol/l (normal value <88), urea at 20.3 mmol/l (normal value 7.2), haemoglobin at 6.70 mmol/l (normal value 6.9 to 9.8), total white blood cell count at 13.20 10E9/l (normal value 4 to 10), blood potassium at 6.5 mmol/l (normal value 3.5 to 5.3) and sodium at 126 mmol/l (normal value 136 to 152) on 06 Jul 2010. It was reported that there was clear clinical signs of exsiccrosis and no spasmodic abdominal pain but continuing diarrhoea. She was treated with intravenous fluid and electrolyte replacement. The therapy with diclofenac was discontinued on 06 Jul 2010 and switched to metamizole at 30 drops four times orally from 06 Jul 2010. She was also treated with metronidazole 500 mg TID orally from 06 Jul 2010 to 13 Jul 2010, Benefiber (cyamopsis tetragonolobus gum, partially hydrolyzed gaur gum) one tablespoon at once from 06 Jul 2010 to 23 Jul 2010 and Perenterol forte (antibiotic resistant yeast, saccharomyces boulardii) at one capsule BID orally from 08 Jul 2010 to 23 Jul 2010. The lab data showed partial prothrombin time at 52.4 (normal value 26 to 36 sec), thrombin time at 71.2 sec (normal value 15 to 22), fibrinogen at 7.0 g/l (normal value 1.8 to 3.5), haematocrit at 0.33 SI Einheit (normal value 0.34 to 0.45). The treatment with acetyl salicylic acid was discontinued on 14 Jul 2010 and switched to Plavix (clopidogrel) 75 mg QD orally. She was transfused with one red blood cell concentrate in additive solution reduction of hypertensive medication due to fluid loss resulting from the diarrhoea on 16 Jul 2010. The stool sample showed no toxin A/B of clostridium difficile, enteritis coli, salmonella, shigella, yersinia, campylobacter, adenoviruses, rotaviruses, cryptosporidiae, lamblia, astro and noroviruses on 07 Jul 2010. The esophagogastroduodenoscopy (EGD) showed esophagus with normal mucosa, gastric mucosa slightly reddened, without lesions. The mucosal plication was normal and EGD overall without normal abnormalities. The lab data showed no antitransglutaminase IgA, antitransglutaminase IgG, gliadin IgA antibodies, gliadin IgG antibodies on 08 Jul 2010. The colonoscopy revealed colopathy with pancolitis and erosions on the ascending colon and on Bauhins valve on 13 Jul 2010. The histological analysis of the EGD samples from 08 Jul 2010 showed no crutera for sprue, no evidence of whipples disease on 13 Jul 2010. The biopsy samples of ileal mucosa shoed no morphological signs of chronic inflammatory bowel disease on 16 Jul 2010. The biopsy samples from the ileocecal valve and the ascending colon showed chronic and active colitis, possibly combination of NSAID colopathy with now infectious colitis component, clinically ongoing immunosuppression, transverse, descending and sigmoid colon showed moderately active colitis. It was reported as overall mucosal atrophy in all segments. The blood pressure during hospitalization was reported as 90 to 100/50 mmHg. It was reported that she had diarrhea until discharge from hospital on 23 Jul 2010. The general cardiovascular situation was stabilized, so discharge home was possible on 23 Jul 2010. The outcome of the event was reported as recovering/resolving. The causality was reported as possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event "circulatory collapse, thrombin time prolonged" and "diclofenac" as suspected based on the current available information.

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Kolitis

KURZZUSAMMENFASSUNG:

Kolitis nach Einnahme von Prednisolon, Diclofenac und ASS

BESCHREIBUNG:

seit 01.07.2010 anhaltender, unkontrollierbarer wässriger Durchfall, bis 10x pro Tag, mit krampfartigen Bauchschmerzen;

seit 04.07.2010 mit hellroten Blutbeimengungen;

06.07.2010 Vorstellung beim Hausarzt wegen des Durchfalls, dort hat Patientin einen Kreislaufkollaps erlitten, Einweisung ins Krankenhaus;

bei Einweisung: deutliche klinische Zeichen der Exsikkose, keine krampfartigen Bauchschmerzen, aber weiterhin bestehender Durchfall

THERAPIE:

intravenöser Flüssigkeits- und Elektrolytersatz;

06.07.2010 Absetzen von Diclofenac und Umstellung auf Metamizol 4x30ggt p.o.;

06.-13.07.2010 Metronidazol 3x500mg p.o.;

06.-23.07.2010 Benefiber 1x1EL;

08.-23.07.2010 Perenterol forte 2x1 Kapsel p.o.;

14.07.2010 Absetzen von ASS und Umstellung auf Plavix 1x75mg p.o.;

16.07.2010 Transfusion von 1 EKKL;

Reduktion der antihypertensiven Medikation bei Flüssigkeitsverlust wegen der Durchfälle

VERLAUF:

07.07.2010 Stuhlprobe: negativ: Toxin A/B von Clostridium difficile, Enteritis coli, Salmonellen, Shigellen, Yersinien, Campylobacter, Adenoviren, Rotaviren, Cryptosporidien, Lamblien, Astro- und Noroviren;

08.07.2010 Ösophagogastroduodenoskopie: Ösophagus mit unauffälliger Schleimhaut, Magenschleimhaut leicht gerötet ohne Läsionen, Faltenrelief unauffällig, ÖGD insgesamt unauffällig;

08.07.2010 Labor: negativ: Anti-Transglutaminase IgA, Anti-Transglutaminase IgG, Gliadin IgA-Ak, Gliadin IgG-Ak;

13.07.2010 Koloskopie: Verdacht auf NSAR-Kolopathie mit Pankolitis und Erosionen im Colon ascendens und auf der Bauhinschen Klappe;

13.07.2010 Histologie der Proben aus der ÖGD vom 08.07.10: keine sicheren Kriterien einer Sprue, keine Hinweise auf Whipple Disease;

16.07.2010 Histologie der Proben aus der Koloskopie vom 13.07.10: Ileum-Schleimhaut-Bioptate: ohne morphologische Hinweise auf eine chronisch entzündliche Darmerkrankung,

Bioptate aus der Ileozökalklappe und dem C.ascendens: mit einer chronischen und aktiven Kolitis, möglicherweise Kombination einer NSAR-

Kolopathie mit jetzt infektiöser Kolitis-Komponente, klinisch laufende Immunsuppression,

Colon transversum, descendens und Sigma: mäßig aktive Kolitis,

insgesamt: in allen Abschnitten auffällige Atrophie der Schleimhaut;

Blutdruck während des stationären Aufenthaltes 90-100/50mmHg;

Durchfall bis zur Entlassung am 23.07.2010, allgemeine Kreislaufsituation stabilisiert, Entlassung in die Häuslichkeit möglich

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 06.-23.07.2010;

Krankensein aufgrund einer Kolitis nach Einnahme von Prednisolon, Diclofenac und ASS;

Besserung nach Absetzen von Diclofenac und ASS, sowie nach symptomatischer Therapie

Infektiöse Diarrhoe-Erreger wurden nicht nachgewiesen. Eine ischämische Genese wurde von den behandelnden Ärzten differentialdiagnostisch nicht in Erwägung gezogen.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: acetylsalicylic acid 100mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Schädigungen der Darmschleimhaut unter ASS sind bekannt und in der Fachinfo beschrieben.

MEDIKAMENT: prednisolon 12.5mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Ein schädigender Einfluss von Prednisolon auf die Darmschleimhaut ist bekannt und beschrieben. Dieser kann bei gleichzeitiger Einnahme von NSAR (hier ASS und Diclofenac) noch verstärkt werden.

MEDIKAMENT: diclofenac retard 75mg

BETREFFENDE UAW: Kolitis

ERLÄUTERUNG: Eine Schädigung der Darmschleimhaut durch Diclofenac ist bekannt und in der Fachinfo beschrieben.<dbtext>

To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10180409

Serious Yes	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 69 Year	weight (kg): 74	BMI_int: 29.64	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzyme increased	201006			3 Month	Unknown
2 Gamma-glutamyltransferase increased	201006			3 Month	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclo SL 75 diclofenac	Oral	150 Mg milligram(s)/Day	20100320-20100627 3 Month	Back pain
2 FLUPIRTINE (Katadolon S long) flupirtine	Oral	/	-	Back pain
3 Natrilix SR (INDAPAMIDE) indapamide	Oral	1.5 Mg milligram(s)/Day	-	
4 ramipril ramipril	Oral	10 Mg milligram(s)/Day	-	

Drugs (concomitant)

5 metamizole metamizole sodium	/	-	Back pain
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			Yes	
Hepatitis C			Unknown	
Hepatic steatosis			Unknown	
Lipid metabolism disorder			Unknown	
Sleep disorder			Yes	
Difficult digestion			Unknown	
Hypertension			Unknown	
Listlessness			No	

Primary source

Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Blood alkaline phosphatase	20100624	1.9	mmol/L	
Blood pressure	201006	125/90	mmHg	
Blood pressure	20100320	130/80	mmHg	
Blood pressure	201004	135/60	mmHg	
Blood pressure	201005	130/85	mmHg	
Creatinine	20100624	76	mmol/L	
Creatinine		63	mmol/L	
Haemoglobin	20100318	8.9	mmol/L	

Hemoglobin	20100624	8.5	mmol/L
MCHC	20100318	20.75	mmol/L
MCHC	20100624	20.86	mmol/L
MCV	20100318	85.7	fL
MCV	20100624	85.3	fL
Potassium	20100624	4.7	mmol/L
Urea		5.2	mmol/L

Narratives (nur bei elektronischem Eingang)TEVA, 18.11.2010 DE-TEVA-255260GER

Sender diagnosis: not reported

Narrative

10-Nov-2010, Non-Interventional Post Marketing Teva study:

A Other health professional reported the case of a Female patient of unknown age who experienced HEPATIC VALUE INCREASED while on FLUPIRTINE (Katadolon S long), DICLOFENAC enrolled in a Non-Interventional PM teva study. The therapy was ended and the drugs were withdrawn.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment:

An increase in hepatic enzyme values is possibly related to Flupirtine and Diclofenac treatment.

present.

Reporter's comment:TEVA, 22.11.2010 DE-TEVA-255260GER

Sender diagnosis: not reported

Narrative

10-Nov-2010, Non-Interventional Post Marketing Teva study,

A Other health professional reported the case of a Female patient 69 years of age who experienced HEPATIC VALUE INCREASED while on DICLOFENAC (Teva drug) and Ramipril, Natrilix SR (INDAPAMIDE) (Non-Teva drugs) enrolled in a Non-Interventional PM teva study. Diclofenac was therapy was withdrawn due to Hepatic value increased.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment:

An increase in hepatic enzyme values is possibly related to Diclofenac treatment.

present.

Reporter's comment:TEVA, 27.07.2011 DE-TEVA-255260GER

Sender diagnosis: not reported

Narrative

10-Nov-2010, Non-Interventional Post Marketing Teva study,

A Other health professional reported the case of a Female patient 69 years of age who experienced HEPATIC VALUE INCREASED while on DICLOFENAC (Teva drug) and Ramipril, Natrilix SR (INDAPAMIDE) (Non-Teva drugs) enrolled in a Non-Interventional PM Teva study. Diclofenac was therapy was withdrawn due to Hepatic value increased.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment:

An increase in hepatic enzyme values is possibly related to Diclofenac treatment.

20-Jul-2011,

Additional informations received from a Other health professional. She reported the case of a Female patient 69 Years of age who experienced GAMMA-GLUTAMYLTRANSFERASE INCREASED while on Diclo SL 75 (Teva product cannot be excluded), Ramipril (Non-Teva product), Natrilix SR (Non-Teva product, INDAPAMIDE) enrolled in a Non-Interventional PM Teva study. According to Mrs. Dr. Mueller no ADR, the cause for the hepatic value increase is related to a histological steatosis hepatis and a mild Hepatitis C.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment: Considering the follow-up information received from the physician, steatosis hepatis and hepatitis C are causative for increased hepatic value. Based on this new information, this case is re-assessed to "hepatic value increased" not related to Diclofenac treatment.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10185935

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19690223	Age: 40 Year	weight (kg): 79	BMI_int: 24.66	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug-induced hepatitis	2010				not recovered/not resolved
2 Icterus	2010	21.10.2010			not recovered/not resolved
3 Liver necrosis	2010				not recovered/not resolved
4 Prothrombin time decreased	2010				not recovered/not resolved
5 Discolouration stool	2010				not recovered/not resolved
6 Heartburn					Unknown
7 Nausea					Unknown
8 Vomiting					Unknown
9 Jaundice	2010				Unknown
10 Hepatic enzymes increased					Unknown
11 Discolouration urine	2010				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 TRANCOPAL DOLO flupirtine maleate		/	201008- -	
2 VALDOXAN 25 mg agomelatine	Oral	/ 82	201008- -	Depressed mood
3 VOLTAREN DOLO diclofenac potassium		/	- -	

Drugs (concomitant)

4 st. john's wort hypericum perforatum	Oral	3*1 Df dosage form/3 Day - -
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Depressed mood			Yes	
Sleep disorder			Yes	
Lumbago			k.A.	
Intervertebral disc prolapse			k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Prothrombin time		Low		
Serology negative		Negative		
Sonogram		Abnormal		

Narratives (nur bei elektronischem Eingang)

SERVIER, 06.12.2010 DE-SERVIER-S10005255

Sender diagnosis: not reported

Narrative

This case was received from the Regulatory Authorities in GERMANY (ref DE-BFARM-10185935).

The patient was a 40 year-old male (BMI 24.7) with a medical history of lumbago, intervertebral disc prolapse, depressed mood and sleep disorder. He was treated since AUG-2010 with VALDOXAN (daily dosage not specified) for depressed mood. Concomitant treatment included FLUPIRTINE since AUG-2010, Hypericum and occasional intakes of DICLOFENAC.

In 2010 (exact date was not specified) the patient experienced severe heartburn, nausea and vomiting. Several days later, he noticed scleral icterus and discoloration of urine and stool.

He presented at the Emergency Room and was admitted to hospital. At admission the patient was awake, oriented and free of complaints. Clinical examination showed icterus. Liver sonography performed on the day of admission showed homogenous liver parenchyma without signs of advanced cholestasis. Negative viral serologies for hepatitis A, B and C. Lab test showed increased ASAT, ALAT, GGT, AP and bilirubin and moderately decreased prothrombin time (exact values were not specified).

An acute cholestatic toxic drug-induced liver necrosis was suspected. Action taken with VALDOXAN, FLUPIRTINE, DICLOFENAC and Hypericum was not reported. The patient was treated with Phytomenadione. Prothrombin time normalized, whereas ASAT, ALAT, GGT, AP and bilirubin remained increased on a stable level.

Not recovered at the time of reporting.

No further information was reported.

In the BfArM report, a severe toxic drug-induced hepatitis was reported as a life-threatening event.

Suspected medications (with imputability) according to the Regulatory Authority : VALDOXAN (probable), FLUPIRTINE MALEATE (probable), DICLOFENAC POTASSIUM (probable)

NOVARTIS, 09.12.2010 DE-002147023-PHHY2010DE82862

Sender diagnosis: not reported

Narrative

Initial report received from a physician via German Health Authority (reference number: DE-BFARM-10185935) on 02 Dec 2010: This patient had a medical history of intake of agomelatine, flupirtine and St. John's wort to treat an underlying depressive mood involving sleep disorders and chronic pain associated with an intervertebral disk prolapse and lumbago. This poly-medicated patient was treated with Voltaren Dolo Extra (diclofenac potassium), Valdoxan (agomelatine) 25mg from Aug 2010, Trancolong (flupirtine maleate) from Aug 2010 for an unspecified indication and were all suspects in this case. The patient was referred from the emergency unit of gastroenterology ward after presenting an emergency with scleral jaundice, discoloured stools and urine for about one day. The patient previously on an unspecified date suffered from severe heartburn, vomiting and nausea. The serological examination performed on an unspecified date was found to be negative for hepatitis A, B and C. A sonographic examination on the day of admission had shown a homogeneous liver parenchyma without signs of advanced cholestasis. On admission the patient was conscious, fully oriented without subjective symptoms but with pronounced jaundice. Due to the suspicion of acute, drug-induced cholestatic liver necrosis, the patient was placed under observation and his liver values were monitored daily (values not specified). Vitamin K (Konakion) replacement was performed due to a moderately lowered prothrombin time (Quick's value). During the further course, the coagulation values returned to the normal range while the hepatic enzymes (glutamate oxaloacetate transaminase [GOT], glutamate pyruvate transaminase [GPT]) and the cholestatic parameters (bilirubin, gammaglutamyltransferase [GGT] and alkaline phosphatase [AP]) remained stable at an elevated level (levels not specified). The patient was diagnosed to have drug-induced toxic hepatitis. The outcome of the event drug-induced toxic hepatitis was reported as condition unchanged whereas that of other events was not reported. The causality of the event drug-induced toxic hepatitis was reported as probable or likely.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10204346

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac diclofenac sodium	Unknown	/	-	
2 diclofenac diclofenac	Unknown	/	-	
3 ENBREL "IMMUNEX" etanercept	Unknown	/	-	
4 LANTAREL methotrexate sodium	Unknown	/	-	

Drugs (concomitant)

5 Acetylsalizylsaeure acetylsalicylic acid	Unknown	/	-	
6 ENALAPRIL enalapril	Unknown	/	-	
7 Folsaeure folic acid	Unknown	/	-	
8 l-thyroxin levothyroxine sodium	Unknown	/	-	
9 MELOXICAM meloxicam	Unknown	/	-	
10 TRAMAL tramadol hydrochloride	Unknown	/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bursitis			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 23.12.2010 DE-SZ09-GXKR2010DE14208

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 21 Dec 2010 from another pharmaceutical company (Pfizer). The case is a solicited report from the marketing program RHEUMA-UNTERSTUETZUNGSDIENST (RUDI), received from a contactable consumer.

Case description as received from Pfizer:

An approximately 64-year-old female patient started to receive etanercept (ENBREL FERTIGSPRITZE) 50 mg per week subcutaneously for rheumatoid arthritis and chronic Polyarthritis on 16 Oct 2006. Additionally she received diclofenac (DICLOFENAC) and methotrexate sodium LANTAREL, therapy regimen unknown, as well as folic acid (FOLIC

ACID), 100 mg acetylsalicylic acid (ASS), 7.5 mg meloxicam (MELOXCAME), levothyroxine sodium (L-THYROXIN), enalapril (ENALAPRIL) and tramadol (TRAMAL) as needed. Relevant medical history was not reported. On unknown date the patient experienced bursitis which caused hospitalisation in late Sep 2010 for surgical treatment. Etanercept therapy has been temporarily discontinued for 2 weeks and was restarted. Causality assessment regarding bursitis was not provided. The reporter further stated that the patient experienced toxic hepatitis while taking diclofenac. An attending physician was said to have considered this event related to methotrexate therapy. No details regarding this event were provided. The outcome of bursitis and toxic hepatitis was unknown.

Follow-up (15 Dec 2010):

This case was updated to include: product and therapy details regarding diclofenac (DICLAC) and methotrexate, event details and outcome for toxic hepatitis as well as causality assessments. The patient has received diclofenac for a longer period of time at moderate to high doses. Especially after bursitis surgery in late Sep 2010 she administered quite high doses diclofenac and subsequently experienced toxic hepatitis. The reporter stated that the patient has used two different diclofenac products during the course of therapy which were diclofenac (DICLAC) and another unspecified diclofenac (DICLOFENAC) product. It was unknown which one has been taken during the relevant time frame. Hospitalisation was not required. Diclofenac therapy was discontinued permanently and the lab values normalised quickly so that the patient recovered. Methotrexate therapy was ongoing. The patient considered this event not related to etanercept therapy. As she had suffered from symptoms of bursitis already in the past, the patient also considered bursitis unlikely related to etanercept therapy. Surgical treatment of bursitis has been considered necessary by the attending physicians to avoid future recurrence.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11004391

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19330706	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Application site erythema					recovered/resolved
2 Quick's value decreased					not recovered/not resolved
3 Haematuria					recovered/resolved
4 Erythema					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/ 95	- 4 Day	

Drugs (concomitant)

2 MARCUMAR phenprocoumon	/	-	-
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Primary source

Qualification: Pharmacist observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 03.01.2011 DE-002147023-CHPA2010DE19563

Sender diagnosis: not reported

Narrative

This healthcare professional report combines an initial report received on 27 Dec 2010 and follow up information received on 29 Dec 2010. This male consumer reported using an unspecified amount of VOLTAREN EMULGEL (diclofenac diethylammonium salt) along with an unspecified amount of Marcumar concomitantly. It was unknown if the VOLTAREN EMULGEL was taken under medical advice. He developed "severely decreased Quick's value (severely prolonged prothrombin time), haematuria and "bloodshot skin at and beyond the application site after using the product for four days. Upon follow up, the patient was "hospitalized (approximately on 23 Dec 2010, it was not known whether outpatient or inpatient) due to the initially reported adverse events. No further information available at the time of this report. The action taken with the product was unknown. The final outcome of all the events was unknown. Follow up information requested in written form (including consent) in week 52, 2010. labeled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

NOVARTIS, 03.01.2011 DE-NOVNCHHQ-CHPA2010DE19563

Sender diagnosis: not reported

Narrative

This healthcare professional report combines an initial report received on 27 Dec 2010 and follow up information received on 29 Dec 2010. This male consumer reported using an unspecified amount of VOLTAREN EMULGEL (diclofenac diethylammonium salt) along with an unspecified amount of Marcumar concomitantly. It was unknown if the VOLTAREN EMULGEL was taken under medical advice. He developed "severely decreased Quick's

value (severely prolonged prothrombin time), haematuria and "bloodshot skin at and beyond the application site after using the product for four days. Upon follow up, the patient was "hospitalized (approximately on 23 Dec 2010, it was not known whether outpatient or inpatient) due to the initially reported adverse events. No further information available at the time of this report. The action taken with the product was unknown. The final outcome of all the events was unknown. Follow up information requested in written form (including consent) in week 52, 2010. labeled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

NOVARTIS, 08.02.2011 DE-002147023-CHPA2010DE19563

Sender diagnosis: not reported

Narrative

This healthcare professional report combines an initial report received on 27 Dec 2010 and follow up information received on 29 Dec 2010. This male consumer reported using an unspecified amount of VOLTAREN EMULGEL (diclofenac diethylammonium salt) along with an unspecified amount of Marcumar concomitantly. It was unknown if the VOLTAREN EMULGEL was taken under medical advice. He developed "severely decreased Quick's value (severely prolonged prothrombin time), haematuria and "bloodshot skin at and beyond the application site after using the product for four days. Upon follow up, the patient was "hospitalized (approximately on 23 Dec 2010, it was not known whether outpatient or inpatient) due to the initially reported adverse events. No further information available at the time of this report. The action taken with the product was unknown. The final outcome of all the events was unknown. Follow up information requested in written form (including consent) in week 52, 2010.

Additional information received on 31 Jan 2011. The pharmacist reported the Quick's value (prothrombin time) was not measureable on an unspecified date and the patient received antidote Konakion (Vitamin K). On an unspecified date, the Quick's value was: 11 %, intensively fluctuating. VOLTAREN EMULGEL was stopped on an unspecified date. Two weeks after the stop of VOLTAREN EMULGEL, "the Quick's value was ongoing severely decreased (= severely prolonged prothrombin time), and strongly fluctuating". All other, initially reported events abated completely. The patient was under medical treatment. Causality assessment by reporting pharmacist was insufficient data. No further information expected. Outcome (in terms of other events) was recovered. Outcome (in terms of decreased Quick's value) was reaction continues.

NOVARTIS, 08.02.2011 DE-NOVNCHHQ-CHPA2010DE19563

Sender diagnosis: not reported

Narrative

This healthcare professional report combines an initial report received on 27 Dec 2010 and follow up information received on 29 Dec 2010. This male consumer reported using an unspecified amount of VOLTAREN EMULGEL (diclofenac diethylammonium salt) along with an unspecified amount of Marcumar concomitantly. It was unknown if the VOLTAREN EMULGEL was taken under medical advice. He developed "severely decreased Quick's value (severely prolonged prothrombin time), haematuria and "bloodshot skin at and beyond the application site after using the product for four days. Upon follow up, the patient was "hospitalized (approximately on 23 Dec 2010, it was not known whether outpatient or inpatient) due to the initially reported adverse events. No further information available at the time of this report. The action taken with the product was unknown. The final outcome of all the events was unknown. Follow up information requested in written form (including consent) in week 52, 2010.

Additional information received on 31 Jan 2011. The pharmacist reported the Quick's value (prothrombin time) was not measureable on an unspecified date and the patient received antidote Konakion (Vitamin K). On an unspecified date, the Quick's value was: 11 %, intensively fluctuating. VOLTAREN EMULGEL was stopped on an unspecified date. Two weeks after the stop of VOLTAREN EMULGEL, "the Quick's value was ongoing severely decreased (= severely prolonged prothrombin time), and strongly fluctuating". All other, initially reported events abated completely. The patient was under medical treatment. Causality assessment by reporting pharmacist was insufficient data. No further information expected. Outcome (in terms of other events) was recovered. Outcome (in terms of decreased Quick's value) was reaction continues.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11005762

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19330919	Age: 77 Year	weight (kg): 89	BMI_int: 31.53	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Scleral icterus					recovering/resolving
2 Gamma GT increased					recovering/resolving
3 Aspartate aminotransferase increase					recovering/resolving
4 Alkaline phosphatase increased					recovering/resolving
5 Hepatic lesion	20101204	20101220	17 Day	3 Month	recovering/resolving
6 Alanine aminotransferase increase					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	75 Mg milligram(s)/Day	201009-20101204	Pain management
diclofenac		245	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain femoral	201009		Unknown	bei Z.n. Femurfraktur
Femur fracture	201009		Unknown	
Dementia	2000		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Haemoglobin	20101205	9.12	mmol/l	
Haemoglobin	20101216	8.13	mmol/l	
Haematocrit	20101205	0.44		
Haematocrit	20101216	0.39		
Mean corpuscular haemoglobin	20101205	92.9	fl	
Mean corpuscular haemoglobin	20101216	94	fl	
Total white blood cell count	20101205	6.68	Gpt/l	
Total white blood cell count	20101216	6.24	Gpt/l	
Thrombocyte count	20101205	264	Gpt/l	
Thrombocyte count	20101216	246	Gpt/l	
Quick's test	20101205	77.2	Prozent	
Quick's test	20101216	90.7	Prozent	
INR	20101205	1.14		
INR	20101216	1.05		
Activated partial thromboplastin time	20101205	28.6	sec	
Activated partial thromboplastin time	20101216	33.5	sec	

Sodium	20101205	139	mmol/l
Sodium	20101216	139	mmol/l
Potassium	20101205	3.42	mmol/l
Potassium	20101216	3.71	mmol/l
Calcium	20101205	2.44	mmol/l
Creatinine	20101205	70.3	µmol/l
Creatinine	20101216	50.2	µmol/l
Aspartate aminotransferase	20101205	19.61	µmol/l.s
Aspartate aminotransferase	20101216	6.3	µmol/l.s
Aspartate aminotransferase	20101220	6.04	µmol/l.s
Alanine aminotransferase	20101205	31.31	µmol/s.l.
Alanine aminotransferase	20101216	7.81	µmol/s.l.
Alanine aminotransferase	20101220	6.92	µmol/s.l.
Alkaline phosphatase	20101205	10.59	µmol/l
Alkaline phosphatase	20101216	5.97	µmol/l
Alkaline phosphatase	20101220	5.32	µmol/l
Gamma GT	20101205	9.71	µmol/s.l
Gamma GT	20101216	7.35	µmol/s.l
Gamma GT	20101220	5.51	µmol/s.l
C-reactive protein	20101205	25.2	mg/l
C-reactive protein	20101220	11.9	mg/l
Bilirubin total	20101205	203.5	µmol/l
Bilirubin total	20101220	120.3	µmol/l
Anti-HBc antibody	20101205	-	positiv/neg;
Antinuclear antibody	20101205	-	positiv/neg;
SM antibody	20101205	-	
Antineutrophil cytoplasmic antibody	20101205	-	positiv/neg;
c-ANCA negative	20101205	-	
p-ANCA positive	20101205	-	

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 13.01.2011 DE-PVZIBE-07-10428

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberfunktionsstörung

KURZZUSAMMENFASSUNG:
Leberfunktionsstörung unter Diclofenac.

BESCHREIBUNG:

Stationäre Einweisung am 05.12.10, da seit 04.12.10 dem Pflegepersonal im Heim gelbe Haut und Skleren auffielen. Die Patientin nahm aufgrund einer Femurfraktur 09/2010 regelmäßig Diclofenac ein.

THERAPIE:

Diclofenac wurde abgesetzt.

VERLAUF:

Nach Absetzen von Diclofenac regrediente Leberwerte, der Ikterus war bei Entlassung noch nicht komplett verschwunden. Der relativ lange stationäre Aufenthalt erklärt sich daraus, dass die Patientin am 09.12.10 nachts aus dem Bett stürzte und danach eine entsprechende Diagnostik zum Frakturausschluss erfolgte und bei Z.n. Femurfraktur eine erneute physiotherapeutische Mobilisation vor Rückverlegung ins Pflegeheim erfolgte.

- Röntgen-Thorax: Keine Infiltrate, keine Hinweise auf Herzinsuffizienz

- Sono-Abdomen: Schlechte Schallbedingungen wegen Meteorismus; soweit beurteilbar Normalbefund.

- Hepatitis A,B,C-Serologie negativ, ebenso Autoimmundiagnostik

ERLÄUTERUNGEN ZUR UAW:

Die stationäre Einweisung erfolgte aufgrund eines schmerzlosen Ikterus', dessen Ursache als #medikamentös-toxische Hepatitis durch Diclofenac# lt. Entlassungsbrief dokumentiert wurde. Chronologie plausibel, da Diclofenac erst im Zusammenhang mit der Femurfraktur im Oktober 2010 neu verordnet wurde und die Leberwerte nach Absetzen des Diclofenacs ab 04.12.10 deutlich regredient waren. Unter Diclofenac sind lt. Literatur Leberfunktionsstörungen bekannt. Da die Patientin keine weiteren Medikamente einnimmt, wird eine medikamentöse Genese bedingt durch andere Medikamente ausgeschlossen. DD wäre eine Infektion (CMV,EBV) oder auch Kontakt mit Umwelttoxinen zu diskutieren.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: diclofenac

BETREFFENDE UAW: Leberschaedigung NNB

ERLÄUTERUNG: Challenge: compatible; Dechallenge: suggestive. Chronologie plausibel, ebenso Symptomatologie, allerdings bleiben Alternativursachen, wie CMV, EBV, Umweltgifte prinzipiell denkbar. Lt. Fachinforamtion sind unter Diclofenac Erhöhungen der Transaminasen "häufig" möglich und mit "gelegentlich" werden Leberschäden (insbesondere bei Langzeittherapie) akute Hepatitis mit/ohne Ikterus angegeben.

<dbtext>To_Char_NARRATIVE_2</dbtext>

HELIOS Klinikum Wuppertal, 17.01.2011 DE-PVZIBE-07-10428

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberfunktionsstörung

KURZZUSAMMENFASSUNG:

Leberfunktionsstörung unter Diclofenac.

BESCHREIBUNG:

Stationäre Einweisung am 05.12.10, da seit 04.12.10 dem Pflegepersonal im Heim gelbe Haut und Skleren auffielen. Die Patientin nahm aufgrund einer Femurfraktur 09/2010 regelmäßig Diclofenac ein.

THERAPIE:

Diclofenac wurde abgesetzt.

VERLAUF:

Nach Absetzen von Diclofenac regrediente Leberwerte, der Ikterus war bei Entlassung noch nicht komplett verschwunden. Der relativ lange stationäre Aufenthalt erklärt sich daraus, dass die Patientin am 09.12.10 nachts aus dem Bett stürzte und danach eine entsprechende Diagnostik zum Frakturausschluss erfolgte und bei Z.n. Femurfraktur eine erneute physiotherapeutische Mobilisation vor Rückverlegung ins Pflegeheim erfolgte.

- Röntgen-Thorax: Keine Infiltrate, keine Hinweise auf Herzinsuffizienz

- Sono-Abdomen: Schlechte Schallbedingungen wegen Meteorismus; soweit beurteilbar Normalbefund.

- Hepatitis A,B,C-Serologie negativ, ebenso Autoimmundiagnostik

ERLÄUTERUNGEN ZUR UAW:

Die stationäre Einweisung erfolgte aufgrund eines schmerzlosen Ikterus', dessen Ursache als #medikamentös-toxische Hepatitis durch Diclofenac# lt. Entlassungsbrief dokumentiert wurde. Chronologie plausibel, da Diclofenac erst im Zusammenhang mit der Femurfraktur im Oktober 2010 neu verordnet wurde und die Leberwerte nach Absetzen des Diclofenacs ab 04.12.10 deutlich regredient waren. Unter Diclofenac sind lt. Literatur Leberfunktionsstörungen bekannt. Da die Patientin keine weiteren Medikamente einnimmt, wird eine medikamentöse Genese bedingt durch andere Medikamente ausgeschlossen. DD wäre eine Infektion (CMV,EBV) oder auch Kontakt mit Umwelttoxinen zu diskutieren.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: diclofenac

BETREFFENDE UAW: Leberschaedigung NNB

ERLÄUTERUNG: Challenge: compatible; Dechallenge: suggestive. Chronologie plausibel, ebenso Symptomatologie, allerdings bleiben Alternativursachen, wie CMV, EBV, Umweltgifte prinzipiell denkbar. Lt. Fachinforamtion sind unter Diclofenac Erhöhungen der Transaminasen "häufig" möglich und mit "gelegentlich" werden Leberschäden (insbesondere bei Langzeittherapie) akute Hepatitis mit/ohne Ikterus angegeben.

<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 26.01.2011 DE-002147023-PHHY2011DE03884

Sender diagnosis: not reported

Narrative

Initial report received from pharmacovigilance center (Intensive monitoring project) via Health Authority (DE-BFARM-11005762) on 14 Jan 2011: This patient with a history of dementia, fracture and pain femoral started the treatment with diclofenac (manufacturer unknown) for pain management in Sep 2010. The patient experienced liver function impairment on 04 Dec 2010. Treatment with diclofenac was discontinued on 04 Dec 2010. On 04 Dec 2010 yellow skin and sclera was observed. The patient was admitted (date not specified) due to painless jaundice, the cause of which was

reported as drug toxic hepatitis (hepatic lesion) induced by diclofenac. The outcome of the event hepatic lesion was reported as complete recovery on 20 Dec 2010, painless jaundice was reported as condition improving and liver function impairment and hepatitis were not reported. The causality was reported as possible for the hepatic lesion. The case had been closed. No further information was expected.

Reporter's comment:

sanofi-aventis, 28.01.2011 DE-SANOFI-AVENTIS-2011SA003001

Sender diagnosis: not reported

Narrative

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-11005762) on 17-Jan-2011 and on the next day.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This case involves a 77-year-old female patient, who presented dementia and lived in a nursing home. The patient did not take any further medication. In September 2010 she suffered fracture of the right femur. Subsequently she began to complain about femoral pain and started with diclofenac 75 mg/day in the same month. On 04-Dec-2010 the icterus with sclerae was detected by the nurse and diclofenac was discontinued immediately. On the next day the patient was hospitalised. Liver lesion was diagnosed with the following symptoms: increased aspartate aminotransferase (up to 19.61 mcmol/L.s), increased alanine aminotransferase (up to 31.31 mcmol/s.l.), increased alkaline phosphatase (up to 10.59 mcmol/l) and increased gamma GT (up to 9.71 mcmol/s.l.). The thoracic x-ray did not present infiltrations or signs for myocardial infarction. The abdominal sonography was difficult to perform due to meteorism; otherwise normal results were detected. Corrective treatment was not mentioned; at date of report the patient was recovering. As stop date of the event, 20-DEC-2010 was reported. Further information was not expected due to this case was closed by PV-center.

Reporter causality: possible

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11014307

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: 19501028 Age: 60 Year weight (kg): 82 BMI_int: 32.85 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute kidney failure	20101108			16 Day	recovering/resolving
2 Glomerular filtration rate decreased	20101108			18 Day	recovering/resolving
3 Drug-induced hepatitis					Unknown
4 Blood creatinine increased	20101108			18 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Clinda-Saar, 600 mg, Amp, MIP clindamycin	Intravenous (not otherwis	1800 Mg milligram(s)/Day 293	20101023-20101023 -	Spondylodiscitis
2 Clinda-Saar, 600 mg, Filmtabl, MIP clindamycin	Oral	600 Mg milligram(s)/Day 82	20101024-20101108 16 Day	Spondylodiscitis
3 Diclofenac, 50 mg, Tabl, ABZ diclofenac	Oral	50 Mg milligram(s)/Day 245	2002-20101022 -	
4 Dotarem, Amp, Guerbet gadoteric acid	Intravenous (not otherwis	4748 Mg milligram(s)/Day 120	20101022-20101022 -	
5 Staphylex, 2 g, Amp, Actavis flucloxacillin	Intravenous (not otherwis	6000 Mg milligram(s)/Day 293	20101023-20101023 -	Spondylodiscitis
6 Staphylex, 500 mg, Kps, Actavis flucloxacillin	Oral	6000 Mg milligram(s)/Day 5	20101024-20101108 16 Day	Spondylodiscitis
7 Voltaren Resinat, 75 mg, Kps, Novartis diclofenac	Oral	150 Mg milligram(s)/Day 5	20101022- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Spondylodiscitis	201009		Yes	
Intervertebral foraminal encroachment	201008		Unknown	
Drug allergy			Unknown	Allergy to Penicillin and paracetamol.
Hypertension arterial			Yes	
Diverticulosis of colon (without mention of hemorrhage)			Yes	
Asthma bronchial			Yes	
Lumbar spinal stenosis			Unknown	
Surgery	201008		Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine normal	20101023	0.9	mg/dl	
Creatinine increased	20101108	2.0	mg/dl	
Creatinine increased	20101110	3.1	mg/dl	
Creatinine increased	20101112	2.1	mg/dl	

Creatinine increased	20101129	1.0	mg/dl
Blood urea normal	20101112	39	mg/dl
Blood urea normal	20101129	36	mg/dl
GFR normal	20101023	>60	ml/min/1.73
GFR decreased	20101108	28	ml/min/1.73
GFR decreased	20101122	45	ml/min/1.73
GFR decreased	20101129	57	ml/min/1.73
Creatinine normal	20101022	0.72	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 27.01.2011 DE-FAKOS-160070

Sender diagnosis: not reported

Narrative

Sender's comment:

Initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 05.04.2011 DE-FAKOS-160070

Sender diagnosis: not reported

Narrative

Am 22.10.2010 erfolgte die stationäre Aufnahme der Patientin wegen einer mittels MRT (22.10.2010, Dotarem i.v.) nachgewiesenen Spondylodiszitis LWK 4/5 bei Zustand nach OP einer lumbalen Spinalkanalstenose 08/2010. Die Behandlung der Spondylodiszitis erfolgte konservativ mit Bettruhe, eine Schmerzmedikation erfolgte mit Diclofenac Abz 50 mg Tabl. bzw. Voltaren Resinat 75 mg Kps. (wurde seit 2002 eingenommen und aktuell die Dosierung von 1 Tabl./die auf 2 Kps/die erhöht) und die antibiotische Therapie mit Clindasaar-Amp bzw. 600 mg Filmtabl. und Staphylex 2g Amp. bzw. 500 mg Kps.

Am 08.11.2010 wurde ein erhöhtes Serumkreatinin festgestellt (2,0 mg/dl (Normbereich: 0,5-0,9 mg/dl)). Der Kreatininwert lag bei Aufnahme noch im Normbereich (22.10.2010: 0,72 mg/dl). Im Verlauf besserte sich die Nierenfunktion schnell ohne spezifische Therapie.

Sonographie: Nieren normal groß ohne Harnstauung.

Am 03.12.2010 wurde die Patientin in deutlich gebessertem Zustand aus dem Krankenhaus entlassen (Kreatinin vom 29.11.2010: 1,0 mg/dl).

Insgesamt bestand der hochgradige Verdacht auf eine medikamentenbedingte Hepatitis (siehe DE-FAKOS-170385) und akutes Nierenversagen (ANV). Eine Arzneimittelgenese ist eher wahrscheinlich.

Als Ursache für das ANV kommen die Arzneimittel Diclofenac Abz 50 mg Tabl. bzw. Voltaren Resinat 75 mg Kps sowie Clindasaar, Staphylex und Dotarem in Frage. Eine genauere Zuordnung zu einem bestimmten Arzneimittel ist nicht möglich, da mehrere der genannten Medikamente im gleichen Zeitraum eingenommen wurden. Deshalb wird die Arzneimittelkausalität mit "möglich" bewertet.

Die UAW hat sich im Krankenhaus entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 13.04.2011 DE-002147023-PHHY2011DE29310

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via Health authority (DE-BFARM-11014307) on 06 Apr 2011. This report refers to 60- year- female patient with a medical history of arterial hypertension, intervertebral foraminal encroachment since Aug 2010, asthma bronchial, and diverticulitis of colon (without mention of hemorrhage). The patient also had allergies to penicillin, paracetamol and surgery for stenosis of the lumbar spinal canal. The patient was admitted in hospital on 22 Oct 2010 due to spondylodiscitis of lumbar vertebrae 4/5 detected by MRI, in a condition following surgery for stenosis of the lumbar spinal canal on Aug 2010. The patient received Dotarem (meglumine gadoterate) intravenously for an unknown indication since 22 Oct 2010 at a dose of 4748 mg. The treatment of the spondylodiscitis was conservative with bed rest. As a part of analgesic treatment, the patient received diclofenac (manufacturer unknown) 50 mg tablet orally at a dose of 1 tablet daily

from 2002 to 22 Oct 2010. The patient also received Voltaren Resinat (diclofenac resinate) 75mg capsules orally at a dose of 2 capsules per day since 22 Oct 2010. The patient also received antibiotic therapy with Clindasaar (clindamycin phosphate) 600mg ampoules from 23 Oct 2010 and Clindasaar (clindamycin phosphate) 600mg film coated tablets from 24 Oct 2010 at a dose of 3 DF daily for spondylodiscitis. The patient also received Staphylex (flucloxacillin sodium) 2000 mg ampoules intravenously on 23 Oct 2010 at a dose of 2000 mg three times daily (discontinued on the same day) and Staphylex (flucloxacillin sodium) 500mg capsules orally at a dose of 2000 mg three times daily from 24 Oct 2010 to 08 Nov 2011 both for spondylodiscitis. On 22 Oct 2010, the creatinine value was normal 0.72mg/dl. The lab data showed elevated serum creatinine at 2.0mg/dl (0.5-0.9mg/dl) on 08 Nov 2010. On 29 Nov 2010, the creatinine level was 1.0mg/dl and blood urea was normal. On 23 Oct 2010, the glomerular filtration rate (GFR) was more than 60 ml/min. The glomerular filtration rate decreased to 28 ml/min/ 1.73 on 08 Nov 2010. The lab data showed glomerular filtration rate at 45 ml/min/1.73 on 22 Nov 2010 and at 57 ml/min/1.73 on 29 Nov 2010. Over the course, the renal function improved quickly without specified therapy. The sonography showed normal size with no urinary retention. The treatment with Voltaren Resinat was ongoing. On 03 Dec 2010, the patient was discharged from hospital with condition improved. The reporter suspected of medication induced hepatitis and acute kidney failure. The reporter assessed the causality between the drugs and acute kidney failure as suspected stating that the diclofenac, Voltaren Resinat, Clindasaar, Staphylex and Dotarem were a possible cause for acute kidney failure. The reporter further stated that a more precise attribution to a specific medication was not possible, as all the mentioned medications were taken during the same timeframe. The final outcome of the event was reported as condition improving.

NOVARTIS, 19.10.2011 DE-002147023-PHHY2011DE29310

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via Health authority (DE-BFARM-11014307) on 06 Apr 2011. This report refers to 60- year- female patient with a medical history of arterial hypertension, intervertebral foraminal encroachment since Aug 2010, asthma bronchial, and diverticulitis of colon (without mention of hemorrhage). The patient also had allergies to penicillin, paracetamol and surgery for stenosis of the lumbar spinal canal. The patient was admitted in hospital on 22 Oct 2010 due to spondylodiscitis of lumbar vertebrae 4/5 detected by MRI, in a condition following surgery for stenosis of the lumbar spinal canal on Aug 2010. The patient received Dotarem (meglumine gadoterate) intravenously for an unknown indication since 22 Oct 2010 at a dose of 4748 mg. The treatment of the spondylodiscitis was conservative with bed rest. As a part of analgesic treatment, the patient received diclofenac (manufacturer unknown) 50 mg tablet orally at a dose of 1 tablet daily from 2002 to 22 Oct 2010. The patient also received Voltaren Resinat (diclofenac resinate) 75mg capsules orally at a dose of 2 capsules per day since 22 Oct 2010. The patient also received antibiotic therapy with Clindasaar (clindamycin phosphate) 600mg ampoules from 23 Oct 2010 and Clindasaar (clindamycin phosphate) 600mg film coated tablets from 24 Oct 2010 at a dose of 3 DF daily for spondylodiscitis. The patient also received Staphylex (flucloxacillin sodium) 2000 mg ampoules intravenously on 23 Oct 2010 at a dose of 2000 mg three times daily (discontinued on the same day) and Staphylex (flucloxacillin sodium) 500mg capsules orally at a dose of 2000 mg three times daily from 24 Oct 2010 to 08 Nov 2011 both for spondylodiscitis. On 22 Oct 2010, the creatinine value was normal 0.72mg/dl. The lab data showed elevated serum creatinine at 2.0mg/dl (0.5-0.9mg/dl) on 08 Nov 2010. On 29 Nov 2010, the creatinine level was 1.0mg/dl and blood urea was normal. On 23 Oct 2010, the glomerular filtration rate (GFR) was more than 60 ml/min. The glomerular filtration rate decreased to 28 ml/min/ 1.73 on 08 Nov 2010. The lab data showed glomerular filtration rate at 45 ml/min/1.73 on 22 Nov 2010 and at 57 ml/min/1.73 on 29 Nov 2010. Over the course, the renal function improved quickly without specified therapy. The sonography showed normal size with no urinary retention. The treatment with Voltaren Resinat was ongoing. On 03 Dec 2010, the patient was discharged from hospital with condition improved. The reporter suspected of medication induced hepatitis and acute kidney failure. The reporter assessed the causality between the drugs and acute kidney failure as suspected stating that the diclofenac, Voltaren Resinat, Clindasaar, Staphylex and Dotarem were a possible cause for acute kidney failure. The reporter further stated that a more precise attribution to a specific medication was not possible, as all the mentioned medications were taken during the same timeframe. The final outcome of the event was reported as condition improving.

Following an internal review on 17 Oct 2011 for the information received on 06 Apr 2011, the following correction was done. The Co-suspect diclofenac was coded using Novartis drug dictionary (previously coded using WHO dictionary).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11014313

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19501028	Age: 60 Year	weight (kg): 82	BMI_int: 32.85	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20101108			16 Day	recovering/resolving
2 Acute kidney failure					recovering/resolving
3 Alanine aminotransferase increased	20101108			18 Day	recovering/resolving
4 Alkaline phosphatase increased	20101112			22 Day	recovering/resolving
5 Hepatitis acute	20101108			18 Day	recovering/resolving
6 Bilirubin increased	20101108			18 Day	recovering/resolving
7 Aspartate aminotransferase increased	20101108			18 Day	recovering/resolving
8 Serum creatinine increased					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Clinda-Saar, 600 mg, Amp, MIP clindamycin	Intravenous (not otherwise)	1800 Mg milligram(s)/Day 293	20101023-20101023 -	Spondylodiscitis
2 Clinda-Saar, 600 mg, Filmtabl, MIP clindamycin	Oral	600 Mg milligram(s)/Day 82	20101024-20101108 16 Day	Spondylodiscitis
3 Diclofenac, 50 mg, Tabl, ABZ diclofenac	Oral	50 Mg milligram(s)/Day 245	2002-20101022 -	
4 Dotarem, Amp, Guerbet gadoteric acid	Intravenous (not otherwise)	4748 Mg milligram(s)/Day 120	20101022-20101022 -	
5 Staphylex, 2 g, Amp, Actavis flucloxacillin	Intravenous (not otherwise)	6000 Mg milligram(s)/Day 293	20101023-20101023 -	Spondylodiscitis
6 Staphylex, 500 mg, Kps, Actavis flucloxacillin	Oral	6000 Mg milligram(s)/Day 5	20101024-20101108 16 Day	Spondylodiscitis
7 Voltaren Resinat, 75 mg, Kps, Novartis diclofenac	Oral	150 Mg milligram(s)/Day 5	20101022- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Asthma bronchial			Yes	
Diverticulosis of colon (without mention of hemorrhage)			Yes	
Hypertension arterial			Yes	
Spondylodiscitis	201009		Yes	
Drug allergy			Unknown	Allergies to penicillin, paracetamol.
Lumbar spinal stenosis			Unknown	
Surgery	201008		Unknown	
Intervertebral foraminal encroachment	201008		Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20101023	20	U/l	

Alanine aminotransferase increased	20101108	305	U/l
Alanine aminotransferase increased	20101112	180	U/l
Alanine aminotransferase normal	20101129	21	U/l
Aspartate aminotransferase normal	20101023	20	U/l
Aspartate aminotransferase increased	20101108	139	U/l
Aspartate aminotransferase increased	20101112	66	U/l
Aspartate aminotransferase normal	20101129	19	U/l
Alkaline phosphatase increased	20101112	336	U/l
Alkaline phosphatase increased	20101129	108	U/l
Bilirubin normal	20101023	0.43	mg/dl
Bilirubin increased	20101108	3.01	mg/dl
Bilirubin normal	20101129	0.58	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 27.01.2011 DE-FAKOS-170385

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 05.04.2011 DE-FAKOS-170385

Sender diagnosis: not reported

Narrative

Am 22.10.2010 erfolgte die stationäre Aufnahme der Patientin wegen einer mittels MRT (22.10.2010, Dotarem i.v.) nachgewiesenen Spondylodiszitis LWK 4/5 bei Zustand nach OP einer lumbalen Spinalkanalstenose 08/2010. Die Behandlung der Spondylodiszitis erfolgte konservativ mit Bettruhe, eine Schmerzmedikation erfolgte mit Diclofenac Abz 50 mg Tabl. bzw. Voltaren Resinat 75 mg Kps. (wurde seit 2002 eingenommen und aktuell die Dosierung von 1 Tabl./die auf 2 Kps/die erhöht) und die antibiotische Therapie mit Clindasaar-Amp bzw. 600 mg Filmtabl. und Staphylex 2g Amp. bzw. 500 mg Kps.

Am 08.11.2010 wurden erstmalig erhöhte Transaminasen (ALAT: 305 U/l (Normbereich: <31 U/l), ASAT: 139 U/l (Normbereich: <31 U/l) und Cholestaseparameter bei Verdacht auf cholestatische Hepatitis und erhöhtes Serumkreatinin festgestellt. Kreatinin und Transaminasen lagen bei Aufnahme noch im Normbereich. Im Verlauf war die Transaminasenerhöhung nach Absetzen der o.g. Arzneimittel rasch rückläufig und auch die Nierenfunktion besserte sich schnell ohne spezifische Therapie.

Eine Hepatitisserologie wurde nicht durchgeführt, allerdings bestand wohl klinisch kein Anhalt für eine virale Hepatitis. Eine Leberbiopsie wurde bei fehlender Indikation nicht durchgeführt.

Sonographie: Leber unauffällig, Lebervenen und Gallengänge nicht erweitert. Nieren normal groß ohne Harnstauung.

Am 03.12.2010 wurde die Patientin in deutlich gebessertem Zustand aus dem Krankenhaus entlassen (Werte vom 29.11.2010: ALAT: 21 U/l, ASAT: 19 U/l).

Insgesamt bestand der hochgradige Verdacht auf eine medikamentenbedingte Hepatitis und akutes Nierenversagen (ANV) (siehe DE-FAKOS-160070). Eine Arzneimittelgenese ist eher wahrscheinlich.

Als Ursache sowohl für die vermutete Hepatitis als auch für das ANV kommen die Arzneimittel Diclofenac Abz 50 mg Tabl. bzw. Voltaren Resinat 75 mg Kps sowie Clindasaar, Staphylex und Dotarem in Frage. Eine genauere Zuordnung zu einem bestimmten Arzneimittel ist nicht möglich, da mehrere der genannten Medikamente im gleichen Zeitraum eingenommen wurden. Deshalb wird die Arzneimittelkausalität mit "möglich" bewertet.

Die UAW's haben sich im Krankenhaus entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 20.04.2011 DE-002147023-PHHY2011DE29422

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via Health authority (DE-BFARM-11014313) on 06 Apr 2011. This report refers to 60-year-old female patient with a medical history of arterial hypertension, intervertebral foraminal encroachment since Aug 2010, asthma bronchial, and diverticulitis of colon (without mention of hemorrhage). The patient also had allergies to penicillin, paracetamol and had surgery for stenosis of the lumbar spinal canal in Aug 2010. The patient was admitted in hospital on 22 Oct 2010 due to spondylodiscitis of lumbar vertebrae 4/5 detected by MRI, in a condition following surgery for stenosis of the lumbar spinal canal on Aug 2010. The patient received Dotarem (meglumine gadoterate) intravenously for an unknown indication since 22 Oct 2010 at a dose of 4748 mg. The treatment of the spondylodiscitis was conservative with bed rest. As a part of analgesic treatment, the patient received diclofenac (manufacturer unknown) 50 mg tablet orally at a dose of 1 tablet daily from 2002 to 22 Oct 2010. The patient also received Voltaren Resinat (diclofenac resinate) 75mg capsules orally at a dose of 2 capsules per day since 22 Oct 2010. The patient also received antibiotic therapy with Clindasaar (clindamycin phosphate) 600mg ampoules from 23 Oct 2010 and Clindasaar (clindamycin phosphate) 600mg film coated tablets from 24 Oct 2010 at a dose of 3 DF daily for spondylodiscitis. The patient also received Staphylex (flucloxacillin sodium) 2000 mg ampoules intravenously on 23 Oct 2010 at a dose of 2000 mg three times daily (discontinued on the same day) and Staphylex (flucloxacillin sodium) 500mg capsules orally at a dose of 2000 mg three times daily from 24 Oct 2010 to 08 Nov 2011 both for spondylodiscitis. On 23 Oct 2010, alanine aminotransferase was normal with 20 U/l (normal value: less than 31) and bilirubin was 0.43 mg/dl (normal value: less than 1.2). However on 08 Nov 2010, laboratory data revealed alanine aminotransferase at 305 U/l, aspartate aminotransferase 139 U/l (normal value: less than 31) and bilirubin at 3.01mg/dl (normal value: less than 1.2). On 12 Nov 2010, alanine aminotransferase came down to 180 U/l, aspartate aminotransferase was 66 U/l and alkaline phosphates was 336 U/l (normal value: 35- 104). On 29 Nov 2010, alanine aminotransferase was normal with 21 U/l, aspartate aminotransferase was 19 U/l, alkaline phosphate was 108 U/l and bilirubin was 0.58 mg/dl. Cholestasis parameters with suspected cholestatic hepatitis and elevated serum creatinine. Over the course the increase in transaminase rapidly regressed following discontinuation of the aforementioned medication and renal function also improved quickly without the specific therapy. On 03 Dec 2010, the patient was discharged from the hospital in a clearly improved condition. The reporter suspected of medication induced hepatitis and acute kidney failure. The reporter assessed the causality between the drugs and acute hepatitis as suspected stating that the diclofenac, Voltaren Resinat, Clindasaar, Staphylex and Dotarem were a possible cause for acute hepatitis. The reporter further stated that a more precise attribution to a specific medication was not possible, as all the mentioned medications were taken during the same timeframe. The reporter assessed the events as medically significant. The final outcome of the events was reported as condition improving. The causality was reported as possible.

NOVARTIS, 19.10.2011 DE-002147023-PHHY2011DE29422

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center (intensive monitoring project) via Health authority (DE-BFARM-11014313) on 06 Apr 2011. This report refers to 60-year-old female patient with a medical history of arterial hypertension, intervertebral foraminal encroachment since Aug 2010, asthma bronchial, and diverticulitis of colon (without mention of hemorrhage). The patient also had allergies to penicillin, paracetamol and had surgery for stenosis of the lumbar spinal canal in Aug 2010. The patient was admitted in hospital on 22 Oct 2010 due to spondylodiscitis of lumbar vertebrae 4/5 detected by MRI, in a condition following surgery for stenosis of the lumbar spinal canal on Aug 2010. The patient received Dotarem (meglumine gadoterate) intravenously for an unknown indication since 22 Oct 2010 at a dose of 4748 mg. The treatment of the spondylodiscitis was conservative with bed rest. As a part of analgesic treatment, the patient received diclofenac (manufacturer unknown) 50 mg tablet orally at a dose of 1 tablet daily from 2002 to 22 Oct 2010. The patient also received Voltaren Resinat (diclofenac resinate) 75mg capsules orally at a dose of 2 capsules per day since 22 Oct 2010. The patient also received antibiotic therapy with Clindasaar (clindamycin phosphate) 600mg ampoules from 23 Oct 2010 and Clindasaar (clindamycin phosphate) 600mg film coated tablets from 24 Oct 2010 at a dose of 3 DF daily for spondylodiscitis. The patient also received Staphylex (flucloxacillin sodium) 2000 mg ampoules intravenously on 23 Oct 2010 at a dose of 2000 mg three times daily (discontinued on the same day) and Staphylex (flucloxacillin sodium) 500mg capsules orally at a dose of 2000 mg three times daily from 24 Oct 2010 to 08 Nov 2011 both for spondylodiscitis. On 23 Oct 2010, alanine aminotransferase was normal with 20 U/l (normal value: less than 31) and bilirubin was 0.43 mg/dl (normal value: less than 1.2). However on 08 Nov 2010, laboratory data revealed alanine aminotransferase at 305 U/l, aspartate aminotransferase 139 U/l (normal value: less than 31) and bilirubin at 3.01mg/dl (normal value: less than 1.2). On 12 Nov 2010, alanine aminotransferase came down to 180 U/l, aspartate aminotransferase was 66 U/l and alkaline phosphates was 336 U/l (normal value: 35- 104). On 29 Nov 2010, alanine aminotransferase was normal with 21 U/l, aspartate aminotransferase was 19 U/l, alkaline phosphate was 108 U/l and bilirubin was 0.58 mg/dl. Cholestasis parameters with suspected cholestatic hepatitis and elevated serum creatinine. Over the course the increase in transaminase rapidly regressed following discontinuation of the aforementioned medication and renal function also improved quickly without the specific therapy. On 03 Dec 2010, the patient was discharged from the hospital in a clearly improved condition. The reporter suspected of medication induced hepatitis and acute kidney failure. The reporter assessed the causality between the drugs and acute hepatitis as suspected stating that the diclofenac, Voltaren Resinat, Clindasaar, Staphylex and Dotarem were a possible cause for acute hepatitis. The reporter further stated that a more precise attribution to a specific medication was not possible, as all the mentioned medications were taken during the same timeframe. The reporter assessed the events as medically significant. The final outcome of the events was reported as condition improving. The causality was reported as possible.

Following internal review on 17 Oct 2011 for the information received on 06 Apr 2011, the following correction was done: The Co-suspect diclofenac was coded using Novartis drug dictionary (previously coded using WHO dictionary).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11018341

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials: Sex: Female Birth: 19530602 Age: 57 Year weight (kg): 110 BMI_int: 38.97 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal discomfort	20100610			6 Day	Unknown
2 Anuria	20100609			5 Day	Unknown
3 Weakness	20100610			6 Day	Unknown
4 Vomiting	20100609			5 Day	Unknown
5 Renal failure	20100610			6 Day	Unknown
6 Nausea	20100609			5 Day	Unknown
7 Autoimmune haemolytic anaemia NOS	20100608	20100628	21 Day	4 Day	recovered/resolved
8 Back pain				4 Day	Unknown
9 Feeling unwell	20100610			6 Day	Unknown
10 Hepatic failure	20100610			6 Day	Unknown
11 Hepatorenal syndrome	20100610			6 Day	Unknown
12 Jaundice	20100609			5 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 micardis plus 40/12.5 hydrochlorothiazide; telmisartan	Oral	1 Df dosage form/Day 245	200806-20100610 -	Hypertension arterial
2 rewodina 50 diclofenac sodium	Oral	50 Mg milligram(s)/Day 245	20100608-20100610 3 Day	Back pain
3 voltaren dolo 25mg diclofenac potassium	Oral	25 Mg milligram(s)/Day 82	20100605-20100608 4 Day	Back pain

Drugs (concomitant)

4 bisohexal 5mg bisoprolol fumarate	Oral	5 Mg milligram(s)/Day 82	200806- -	Hypertension arterial
5 l-thyrox 75 levothyroxine sodium	Oral	75 µg microgram(s)/Day 245	200806- -	Hypothyroidism
6 musaril 50mg tetrazepam	Oral	100 Mg milligram(s)/Day 82	20100608-20100610 3 Day	Tightness of back muscles
7 norvasc 5 mg amlodipine besilate	Oral	5 Mg milligram(s)/Day 245	201004- -	Hypertension arterial

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Adiposis			Yes	Beginn unbekannt
Hypertension arterial			Yes	Beginn unbekannt
Hypothyroidism			Yes	Beginn unbekannt
Hemolytic anemia	20080815	20080821	No	In August 2008, an episode of Coombs-negative haemolytic anaemia with exsiccosis-induced renal failu
Schizoaffective psychosis			Yes	Beginn unbekannt
Renal failure			Unknown	In August 2008, an episode of Coombs-negative haemolytic anaemia with exsiccosis-induced renal failu
Drug allergy			Unknown	Given an allergy ID card on which was written the instruction to avoid diclofenac, ibuprofen, indome
Back pain			Yes	Beginn unbekannt
Tightness of back muscles			Yes	Beginn unbekannt, schmerzreflektorisch
Agranulocytosis	1996	1996	Yes	unter Laponex
Cholecystolithiasis			Yes	Beginn unbekannt

Primary source

Qualification: Pharmacist observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
ANA	20100610	1-100	NA	
Antithrombin III	20100610	71	NA	
Basophil percentage	20100610	1	NA	
c-ANCA positive	20100610	1-10	NA	
Coombs test		Positive	NA	
Creatinine clearance	20100628	0.51 ml/s	ml/s	
Creatinine clearance	20100623	0.43 ml/s	ml/s	
Creatinine clearance	20100618	0.11ml/s	ml/s	
Creatinine clearance	20100612	0.12 ml/s	ml/s	
Creatinine clearance	20100610	0.12 ml/s	ml/s	
Eosinophil count	20100609	0.03	NA	
Eosinophil count	20100610	0.05	NA	
Eosinophil count	20100610	6	NA	
Erythroblast count	20100610	3	NA	
Glucose-6-phosphate dehydrogenase	20100611	219	NA	
Hematocrit	20100618	0.27	NA	
Hematocrit	20100623	0.29	NA	
Hematocrit	20100612	0.16	NA	
Hematocrit	20100628	0.33	NA	
Hematocrit	20100608	0.32	NA	
Hematocrit	20100610	0.19	NA	
Hematocrit	20100610	0.22	NA	
Hematocrit	20100609	0.33	NA	
Hematocrit	20100611	0.22	NA	

MCH	20100610	2.03 fmol/l	fmol/l
MCH	20100612	1.93 fmol/l	fmol/l
MCH	20100609	2.06 fmol/l	fmol/l
MCH	20100610	2.13 fmol/l	fmol/l
MCH	20100608	2.07 fmol/l	fmol/l
MCV	20100612	84.5 fl	fl
MCV	20100610	92 fl	fl
MCV	20100610	89.2 fl	fl
MCV	20100609	93 fl	fl
MCV	20100608	95 fl	fl
Metamyelocyte count	20100610	1	NA
Myelocyte percentage	20100610	1	NA
Quick's test	20100610	73	NA
RBC count	20100608	3.35 Tpt/l	Tpt/l
RBC count	20100609	3.55 Tpt/l	Tpt/l
RBC count	20100610	2.40 Tpt/l	Tpt/l
RBC count	20100618	3.18 Tpt/l	Tpt/l
RBC count	20100611	2.64 Tpt/l	Tpt/l
RBC count	20100612	1.82 Tpt/l	Tpt/l
RBC count	20100623	3.35 Tpt/l	Tpt/l
RBC count	20100628	3.74 Tpt/l	Tpt/l
RBC count	20100610	2.12 Tpt/l	Tpt/l
Red cell distribution width	20100610	12.1	NA
Red cell distribution width	20100609	13.7	NA
Red cell distribution width	20100608	12.3	NA
Thrombocyte count	20100609	113	NA
Thrombocyte count	20100608	368 Gpt/l	Gpt/l
Thrombocyte count	20100623	243 Gpt/l	Gpt/l
Thrombocyte count	20100610	48 Gpt/l	Gpt/l
Thrombocyte count	20100611	31 Gpt/l	Gpt/l
Thrombocyte count	20100612	193 Gpt/l	Gpt/l
Thrombocyte count	20100618	318 Gpt/l	Gpt/l
Thrombocyte count	20100610	81 Gpt/l	Gpt/l
Ultrasound abdomen abnormal	20100610	Abnormal	NA
WBC	20100612	9.19 Gpt/l	Gpt/l
WBC	20100628	23.3 Gpt/l	Gpt/l
WBC	20100623	7.77 Gpt/l	Gpt/l
WBC	20100618	8.81	NA
WBC	20100608	2.9 Gpt/l	Gpt/l
WBC	20100610	11.2 Gpt/l	Gpt/l
WBC	20100610	12.4Gpt/l	Gpt/l
WBC	20100609	19.3 Gpt/l	Gpt/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 03.02.2011 DE-PVZIBE-02-10319

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
autoimmune hämolytische Anämie

KURZZUSAMMENFASSUNG:

autoimmun-hämolytische Anämie (Coombs positiv mit Wärme-Autoantikörpern) mit Leber- und Nierenversagen nach Einnahme von Voltaren Dolo Extra 25mg und Rewodina (Micardis Plus)

BESCHREIBUNG:

seit 05.06.2010 Rückenschmerzen, die sich nach Einnahme von Voltaren Dolo Extra 25mg/Tag nicht besserten und am 08.06.2010 noch zunahmen; daraufhin Einweisung in ein peripheres Krankenhaus bei Verdacht auf akute Lumbago, dort u.a. Gabe von Rewodina; am 09.06.2010 Ikterus mit Übelkeit und Erbrechen sowie Anurie; 10.06.2010 Verlegung auf die Intensivstation mit V.a. hepatorenales Syndrom 10.06.2010 Verlegung ins tertiäre Krankenhaus (Nephrologie): dort bei Aufnahme deutlich reduzierter AZ, massiv erhöhte Leber-, Nieren- und Entzündungswerte, deutlicher Ikterus, kein Exanthem, ausgeprägte hämolytische Anämie
VORGESCHICHTE: Im August 2008 Episode einer Coombs-negativen hämolytischen Anämie mit auch exsikkotisch bedingtem Nierenversagen. Die Patientin berichtete die Einnahme von Diclofenac vor der stationären Aufnahme.

THERAPIE:

08.06.2010 Absetzen von Voltaren Dolo Extra 25mg;
08.06.-10.06 Gabe von Rewodina 50, Metamizol, Piritramid, Targin
10.-15.06.2010 100mg Prednisolon i.v., ab 16.06.10 orale Gabe in stetig fallender Dosierung (16.06.10 1x80mg p.o., ab 26.06.10 bis zur Entlassung 1x20mg p.o., ambulant langsames Ausschleichen geplant);
10.-15.06.2010 1 Ampulle Tavegil i.v.;
10.-15.06.2010 1 Ampulle Cimetidin i.v.;
10.-15.06.2010 2x500mg Zienam i.v.;
insgesamt 5 Hämodialysen (10.06., 12.06., 13.06., 15.06., 18.06.);
Transfusion von Erythrozytenkonzentraten (10.06.10 4 EKKL, 12.06.10 2 EKKL, 13.06.10 1 EKKL);
12.06.2010 Plasmapherese;
Aushändigung eines Allergiepasses mit dem schriftlichen Hinweis Diclofenac, Ibuprofen, Indometacin oder ähnliche NSARs zu meiden

VERLAUF:

10.06.2010 Blutkultur peripher und zentral: kein Nachweis von Bakterien; Mittelstrahlurin: kein Wachstum von Bakterien
11.06.2010 Sonographie Abdomen: Hepatosplenomegalie, große Nieren mit eher akuten Schädigungszeichen;
11.06.2010 Röntgen-Thorax: kein Hinweis auf pneumonische Infiltrate
ab 11.06.2010 deutlich gebesserter Allgemeinzustand;
12.06.2010 Konsil Hämatologie: am ehesten medikamentös induzierte autoimmun-hämolytische Anämie, kein Hinweis auf chronische lymphatische Leukämie oder paroxysmale nächtliche Hämoglobinurie, Plasmaaustausch ist indiziert;
14.06.2010 immunhämatologischer Befund: Beim Vorliegen von klinischen und laborchemischen Zeichen einer Hämolyse sprechen die serologischen Befunde für eine autoimmune hämolytische Anämie mit Auto-Antikörpern vom Wärmetyp. Der Autoantikörper liegt frei im Serum und zellständig vor. Ein zusätzliches Vorliegen eines Allo-Antikörpers ist nicht auszuschließen. Der direkte Coombstest mit mono- und polyspezifischen Seren ist durch die Behandlung der autologen Erythrozyten für IgG, IgM und C3d positiv. IgA und C3c waren nicht nachweisbar. Mittels Elution konnte ein Auto-Antikörper von den Autologen Erythrozyten abgesprengt werden, der mit allen Testzellen des Differenzierungspanels reagierte. Eine Spezifität kann so nicht bestimmt werden. Der Kälteagglutinititer ist mit <1:8 ein Normaltiter;
15.06.2010 Die durchflusszytometrische Analyse mit monoklonalen Antikörpern gegen GPI-verankerte Oberflächenantigene ergab keinen Hinweis auf paroxysmalen nächtlichen Hämoglobinurie
28.06. Entlassung bei deutlich gebesserter Allgemeinzustand, Aushändigung eines Allergiepasses

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 08.-28.06.2010;

Die Patientin nahm wegen Rückenschmerzen (rezidivierend bekannt) beginnend 3 Tage vor der stationären Aufnahme Diclofenac (Voltaren Dolo Extra) ein. Nachdem die Schmerzen im Verlauf noch zunahmen erfolgte am 08.06.2010 die stationäre Einweisung, wo die Patientin neben anderer Schmerzmedikation ein anderes Diclofenac-haltiges Präparat erhielt (Rewodina). Am Aufnahmetag zeigte sich bereits Toxizität als wahrscheinlich ursächlich angenommen wurde. Nach Absetzen, Dialyse, Plasmapherese und Glukokortikoidtherapie bildete sich die Hämolyse zurück.

MEDIKAMENT: voltaren dolo 25mg

BETREFFENDE UAW: hämolytische Anämie autoimmun

ERLÄUTERUNG: Voltaren Dolo Extra 25mg wurde vom 05.-07.06.2010 eingenommen. Bereits am 08.06.2010 leichte Zeichen, am 09.06.2010 deutliche Zeichen einer hämolytischen Anämie. Im Jahr 2008 entwickelte die Patientin bereits zeitlich mit einer Diclofenaceinnahme assoziierten Coomb-negative hämolytische Anämie. Diagnostisch ergab sich jetzt immunhämatologisch eine im direkten und indirekten Coombstest positive autoimmunhämolytische Anämie mit Autoantikörpern vom Wärmetyp. Der Autoantikörper liegt frei im Serum und zellständig vor. Als Alternativursache zu einer Diclofenacbedingten Genese konnten ausgeschlossen werden: eine aktive CMV- oder EBV-Infektion, ein Yersinien-Befall, es ergab sich kein Hinweis auf ein Lymphom, zur Zeit kein Hinweis auf Vorliegen eines systemischen Lupus Erythematoses. Eine idiopathische Autoimmunhämolytische Anämie bleibt als Alternativursache weiter bestehen. Ein Glucose-6-Phosphat-Dehydrogenase-Mangel wurde ebenso ausgeschlossen wie eine paroxysmale nächtliche Hämoglobinurie. Es bestand kein Anhalt für ein Hämolytisch-urämisches-Syndrom (HUS). Zwischen den beobachteten

Ereignissen aus 2008 und jetzt(2010) waren keine Hinweise auf eine chronische Hämolyse, wie sie bei den chronischen Erkrankungen (Lymphom, Lupus erythematoses, idiopathisch) anzunehmen wären berichtet worden, so dass hier eine akuten Genese, z.B. aufgrund der Einnahme von Diclofenac als wahrscheinlich ursächlich angenommen wurde. Nach Absetzen, Dialyse, Plasmapherese und Glukokortikoidtherapie bildete sich die Hämolyse zurück.

HELIOS Klinikum Wuppertal, 10.02.2011 DE-PVZIBE-02-10319

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
autoimmune hämolytische Anämie

KURZZUSAMMENFASSUNG:

autoimmun-hämolytische Anämie (Coombs positiv mit Wärme-Autoantikörpern) mit Leber- und Nierenversagen nach Einnahme von Voltaren Dolo Extra 25mg und Rewodina (Micardis Plus)

BESCHREIBUNG:

seit 05.06.2010 Rückenschmerzen, die sich nach Einnahme von Voltaren Dolo Extra 25mg/Tag nicht besserten und am 08.06.2010 noch zunahmen; daraufhin Einweisung in ein peripheres Krankenhaus bei Verdacht auf akute Lumbago, dort u.a. Gabe von Rewodina;

am 09.06.2010 Ikterus mit Übelkeit und Erbrechen sowie Anurie,

10.06.2010 Verlegung auf die Intensivstation mit V.a. hepatorenales Syndrom

10.06.2010 Verlegung ins tertiäre Krankenhaus (Nephrologie): dort bei Aufnahme deutlich reduzierter AZ, massiv erhöhte Leber-, Nieren- und Entzündungswerte, deutlicher Ikterus, kein Exanthem, ausgeprägte hämolytische Anämie

VORGESCHICHTE: Im August 2008 Episode einer Coombs-negativen hämolytischen Anämie mit auch exsikkotisch bedingtem Nierenversagen. Die Patientin berichtete die Einnahme von Diclofenac vor der stationären Aufnahme.

THERAPIE:

08.06.2010 Absetzen von Voltaren Dolo Extra 25mg;

08.06.-10.06 Gabe von Rewodina 50, Metamizol, Piritramid, Targin

10.-15.06.2010 100mg Prednisolon i.v., ab 16.06.10 orale Gabe in stetig fallender Dosierung (16.06.10 1x80mg p.o., ab 26.06.10 bis zur Entlassung 1x20mg p.o., ambulant langsames Ausschleichen geplant);

10.-15.06.2010 1 Ampulle Tavegil i.v.;

10.-15.06.2010 1 Ampulle Cimetidin i.v.;

10.-15.06.2010 2x500mg Zienam i.v.;

insgesamt 5 Hämodialysen (10.06., 12.06., 13.06., 15.06., 18.06.);

Transfusion von Erythrozytenkonzentraten (10.06.10 4 EKKL, 12.06.10 2 EKKL, 13.06.10 1 EKKL);

12.06.2010 Plasmapherese;

Aushändigung eines Allergiepasses mit dem schriftlichen Hinweis Diclofenac, Ibuprofen, Indometacin oder ähnliche NSARs zu meiden

VERLAUF:

10.06.2010 Blutkultur peripher und zentral: kein Nachweis von Bakterien; Mittelstrahlurin: kein Wachstum von Bakterien

11.06.2010 Sonographie Abdomen: Hepatosplenomegalie, große Nieren mit eher akuten Schädigungszeichen;

11.06.2010 Röntgen-Thorax: kein Hinweis auf pneumonische Infiltrate

ab 11.06.2010 deutlich gebesserter Allgemeinzustand;

12.06.2010 Konsil Hämatologie: am ehesten medikamentös induzierte autoimmune-hämolytische Anämie, kein Hinweis auf chronische lymphatische Leukämie oder paroxysmale nächtliche Hämoglobinurie, Plasmaaustausch ist indiziert;

14.06.2010 immunhämatologischer Befund: Beim Vorliegen von klinischen und laborchemischen Zeichen einer Hämolyse sprechen die serologischen Befunde für eine autoimmune hämolytische Anämie mit Auto-Antikörpern vom Wärmetyp. Der Autoantikörper liegt frei im Serum und zellständig vor. Ein zusätzliches Vorliegen eines Allo-Antikörpers ist nicht auszuschließen. Der direkte Coombstest mit mono- und polyspezifischen Seren ist durch die Behandlung der autologen Erythrozyten für IgG, IgM und C3d positiv. IgA und C3c waren nicht nachweisbar. Mittels Elution konnte ein Auto-Antikörper von den autologen Erythrozyten abgesprengt werden, der mit allen Testzellen des Differenzierungspanels reagierte. Eine Spezifität kann so nicht bestimmt werden. Der Kälteagglutinititer ist mit <1:8 ein Normaltiter;

15.06.2010 Die durchflusszytometrische Analyse mit monoklonalen Antikörpern gegen GPI-verankerte Oberflächenantigene ergab keinen Hinweis auf paroxysmalen nächtlichen Hämoglobinurie

28.06. Entlassung bei deutlich gebesserter Allgemeinzustand, Aushändigung eines Allergiepasses

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 08.-28.06.2010;

Am Aufnahmetag zeigte sich bereits ein leicht erhöhtes Bilirubin sowie eine geringe Anämie. Am Folgetag traten ein deutlicher Ikterus, Anurie sowie Entzündungszeichen auf, welche schließlich unter dem klinischen Bild einer Sepsis imponierten. Daraufhin erfolgte eine Verlegung ins tertiäre Krankenhaus. Hier imponierte neben dem akuten Nierenversagen insbesondere eine hämolytische Anämie. To_Char_NARRATIVE_2

BETREFFENDE UAW: hämolytische Anaemie autoimmune

ERLÄUTERUNG: Voltaren Dolo Extra 25mg wurde vom 05.-07.06.2010 eingenommen. Bereits am 08.06.2010 leichte Zeichen, am 09.06.2010 deutliche Zeichen einer hämolytischen Anämie. Im Jahr 2008 entwickelte die Patientin bereits zeitlich mit einer Diclofenaceinnahme assoziierten Coomb-negative hämolytische Anämie. Diagnostisch ergab sich jetzt immunhämatologisch eine im direkten und indirekten Coombstest positive autoimmunhämolytische Anämie mit Autoantikörpern vom Wärmetyp. Der Autoantikörper liegt frei im Serum und zellständig vor. Als Alternativursache zu einer Diclofenacbedingten Genese konnten ausgeschlossen werden: eine aktive CMV- oder EBV-Infektion, ein Yersinien-Befall, es ergab sich kein Hinweis auf ein Lymphom, zur Zeit kein Hinweis auf Vorliegen eines systemischen Lupus Erythematoses. Eine idiopathische Autoimmunhämolytische

Anämie bleibt als Alternativursache weiter bestehen. Ein Glucose-6-Phosphat-Dehydrogenase-Mangel wurde ebenso ausgeschlossen wie eine paroxysmale nächtliche Hämoglobinurie. Es bestand kein Anhalt für ein Hämolytisch-urämisches-Syndrom (HUS). Zwischen den beobachteten Ereignissen aus 2008 und jetzt (2010) waren keine Hinweise auf eine chronische Hämolyse, wie sie bei den chronischen Erkrankungen (Lymphom, Lupus erythematoses, idiopathisch) anzunehmen wären berichtet worden, so dass hier eine akute Genese, z.B. aufgrund der Einnahme von Diclofenac als wahrscheinlich ursächlich angenommen wurde. Nach Absetzen, Dialyse, Plasmapherese und Glukokortikoidtherapie bildete sich die Hämolyse zurück.

NOVARTIS, 14.02.2011 DE-002147023-PHHY2011DE09856

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE-BFARM-11018341) on 03 Feb 2011: This patient had a history of an episode of Coombs-negative hemolytic anaemia with exsiccosis-induced renal failure, hypothyroidism, hypertension arterial, adiposis, schizoaffective psychosis, cholecystolithiasis, agranulocytosis, tightness of back muscles and allergic to diclofenac, ibuprofen, indomethacin and similar NSAIDs and a history of using Leponex started treatment with Voltaren Dolo Extra (diclofenac potassium) 25 mg per day from 05 Jun 2010 for back pain. The back pain did not improve after the ingestion of Voltaren Dolo Extra 25 mg/day and it increased on 08 Jun 2010 which led to hospitalization due to suspected acute low-back pain and received a treatment with Rewodina (diclofenac sodium) 50 mg from 08 Jun 2010 which was a co-suspect in this case. This patient was also treated with Micardis plus (telmisartan, hydrochlorothiazide) 40/12.5 mg from an unspecified date since Jun 2008 for hypertension arterial was also a co-suspect in this case. On 09 Jun 2010 this patient developed jaundice with nausea and vomiting as well as anuria and on 10 Jun 2010 was transferred to the intensive-care ward with suspected hepatorenal syndrome, further transferred to a tertiary-care hospital (nephrology unit). On admission there the patient had a clearly reduced general condition, massively elevated hepatic-function, renal-function and inflammatory values, no rash and she was known to have developed autoimmune haemolytic anaemia (positive Coombs test with warm autoantibodies) with hepatic and renal failure. On 11 Jun 2010 the abdominal ultrasound showed liver and spleen enlarged, large kidneys with signs of acute damage, chest X-ray showed no evidence of pneumonic infiltrates, hematology consultation was most likely in favor of drug-induced autoimmune hemolytic anemia with no evidence of chronic lymphatic leukemia or paroxysmal nocturnal haemoglobinuria and plasma exchange was indicated. On the day of her admission, her bilirubin level was already slightly elevated and she was exhibiting mild anemia. From day 2 of her hospital stay, the patient showed clinical signs consistent with a hemolytic crisis. A haemolytic crisis typically started with weakness and a feeling of unwellness, associated with abdominal discomfort and/or back pain. The treatment with Voltaren Dolo Extra was discontinued on 08 Jun 2010. This patient received a treatment with Metamizole (metamizole), Piritramide (piritramide), Targin (naloxone hydrochloride, oxycodone hydrochloride) from 08 Jun 2010, Prednisolone (prednisolone) 100 mg intravenously from 10 Jun 2010 and later from 16 Jun 2010 administered orally in a steadily decreasing dosage from 80 mg to 20 mg with slow tapering-off the dosage, 1 ampoule of Tavegil (clemastine) I.V., 1 ampoule of Cimetidine (cimetidine) I.V., Zienam I.V. 500 mg twice daily, 5 sessions of haemodialysis, transfusion of packed red cells and plasmapheresis. The patient was discharged from the hospital on 28 Jun 2010. The outcome of the event autoimmune haemolytic anaemia was reported as complete recovery on 28 Jun 2010 and that of other events was not reported. The causality of the event autoimmune haemolytic anaemia was reported as probable by the reporter.

NOVARTIS, 16.02.2011 DE-002147023-PHHY2011DE09856

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE-BFARM-11018341) on 03 Feb 2011: This patient had a history of an episode of Coombs-negative hemolytic anaemia with exsiccosis-induced renal failure, hypothyroidism, hypertension arterial, adiposis, schizoaffective psychosis, cholecystolithiasis, agranulocytosis, tightness of back muscles and allergic to diclofenac, ibuprofen, indomethacin and similar NSAIDs and a history of using Leponex started treatment with Voltaren Dolo Extra (diclofenac potassium) 25 mg per day from 05 Jun 2010 for back pain. The back pain did not improve after the ingestion of Voltaren Dolo Extra 25 mg/day and it increased on 08 Jun 2010 which led to hospitalization due to suspected acute low-back pain and received a treatment with Rewodina (diclofenac sodium) 50 mg from 08 Jun 2010 which was a co-suspect in this case. This patient was also treated with Micardis plus (telmisartan, hydrochlorothiazide) 40/12.5 mg from an unspecified date since Jun 2008 for hypertension arterial was also a co-suspect in this case. On 09 Jun 2010 this patient developed jaundice with nausea and vomiting as well as anuria and on 10 Jun 2010 was transferred to the intensive-care ward with suspected hepatorenal syndrome, further transferred to a tertiary-care hospital (nephrology unit). On admission there the patient had a clearly reduced general condition, massively elevated hepatic-function, renal-function and inflammatory values, no rash and she was known to have developed autoimmune haemolytic anaemia (positive Coombs test with warm autoantibodies) with hepatic and renal failure. On 11 Jun 2010 the abdominal ultrasound showed liver and spleen enlarged, large kidneys with signs of acute damage, chest X-ray showed no evidence of pneumonic infiltrates, hematology consultation was most likely in favor of drug-induced autoimmune hemolytic anemia with no evidence of chronic lymphatic leukemia or paroxysmal nocturnal haemoglobinuria and plasma exchange was indicated. On the day of her admission, her bilirubin level was already slightly elevated and she was exhibiting mild anemia. From day 2 of her hospital stay, the patient showed clinical signs consistent with a hemolytic crisis. A haemolytic crisis typically started with weakness and a feeling of unwellness, associated with abdominal discomfort and/or back pain. The treatment with Voltaren Dolo Extra was discontinued on 08 Jun 2010. This patient received a treatment with Metamizole (metamizole), Piritramide (piritramide), Targin (naloxone hydrochloride, oxycodone hydrochloride) from 08 Jun 2010, Prednisolone (prednisolone) 100 mg intravenously from 10 Jun 2010 and later from 16 Jun 2010 administered orally in a steadily decreasing dosage from 80 mg to 20 mg with slow tapering-off the dosage, 1 ampoule of Tavegil (clemastine) I.V., 1 ampoule of Cimetidine (cimetidine) I.V., Zienam I.V. 500 mg twice daily, 5 sessions of haemodialysis, transfusion of packed red cells and plasmapheresis. The patient was discharged from the hospital on 28 Jun 2010. The outcome of the event autoimmune haemolytic anaemia was reported as complete recovery on 28 Jun 2010 and that of other events was not reported. The causality of the event autoimmune haemolytic anaemia was reported as probable by the reporter.

This follow up was received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (reference number: DE

BFARM-11018341) on 11 Feb 2011: It was reported that It could be possible that the increasing back pain which led to hospitalisation was a sign of the beginning of hemolytic crisis. Alternatively, the back pain could be of a different genesis.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11021445

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg): 80	BMI_int: 28.34	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	20110124			35 Day	Unknown
2 Elevated liver enzymes	20110124			35 Day	not recovered/not resolved
3 GPT increased	20110124			35 Day	Unknown
4 GOT increased	20110124			35 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mono-embolex certoparin sodium	Subcutaneous	1 Df dosage form/Day	20101221-20110128	Anticoagulant therapy
2 VOLTAREN RESINAT diclofenac resinate	Oral	150 Mg milligram(s)/Day	20101221-20110124	Analgesia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Surgery			Unknown	
Liver enzyme abnormal			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT increased	20110124	469 U/l	U/l	
GOT increased	20110124	210 U/l	U/l	
Gamma GT increased	20110124	164 U/l	U/l	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 04.02.2011 DE-002147023-PHHY2011DE06838

Sender diagnosis: not reported

Narrative

This is combined information, received from a physician via a sales representative on 25 Jan 2011 and 27 Jan 2011: This patient had a history of recurrent liver value increases. On 21 Dec 2010 the patient started treatment with Mono-Embolex 3000 I.E. Prophylaxe 3000 I.U. (certoparin) 1 DF/d for anticoagulation. The anticoagulation was required as the patient was lying in a plaster basin. Concomitantly the patient was treated with Voltaren (diclofenac) for an unknown indication. On 24 Jan 2011 the patient experienced increased liver values. The following lab values were detected: gamma GT 164 U/l, GPT 469 U/l, and GOT 210 U/l. Due to these events Voltaren was discontinued (exact stop date not reported). At the time of reporting treatment with Mono-Embolex was ongoing. Outcome of the event was unknown.

considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 25.02.2011 DE-002147023-PHHY2011DE06838

Sender diagnosis: not reported

Narrative

This is combined information, received from a physician via a sales representative on 25 Jan 2011 and 27 Jan 2011: This patient had a history of recurrent liver value increases. On 21 Dec 2010 the patient started treatment with Mono-Embolex 3000 I.E. Prophylaxe 3000 I.U. (certoparin) 1 DF/d for anticoagulation. The anticoagulation was required as the patient was lying in a plaster basin. Concomitantly the patient was treated with Voltaren (diclofenac) for an unknown indication. On 24 Jan 2011 the patient experienced increased liver values. The following lab values were detected: gamma GT 164 U/l, GPT 469 U/l, and GOT 210 U/l. Due to these events Voltaren was discontinued (exact stop date not reported). At the time of reporting treatment with Mono-Embolex was ongoing. Outcome of the event was unknown.

Follow-up information received from the physician on 17 Feb 2011: The patient had a history of recurrent mild increases of the liver enzymes with unresolved cause. The patient started treatment with Mono- Embolex for postoperative thrombosis prophylaxis and with Voltaren Resinat (not Voltaren as previously reported) 150 mg/day, orally for analgesia on 21 Dec 2010. The patient was diagnosed with increased liver values on 24 Jan 2011 with and therapy with Voltaren Resinat was discontinued on the same day. Therapy with Mono-Embolex was discontinued on 28 Jan 2011. The physician stated that the symptoms improved after discontinuation. However, the patient had not yet recovered. The physician assessed the causal relationship to Mono-Embolex and Voltaren Resinat as likely. She also stated that the available data was not sufficient for a final evaluation of the case. As alternative explanations the physician noted comedication with Voltaren and preexisting mild increase of liver enzymes.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11025151

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes Yes Yes Yes

Patient

Initials: Sex: Female Birth: 19400123 Age: 70 Year weight (kg): 70 BMI_int: 28.04 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20101101				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac-ratiopharm SL, 75 mg, Retardkps diclofenac	Oral	75 Mg milligram(s)/ 208	200907-20101103 -	Arthrosis
2 Grippe-Impfstoff N 2010/2011, Stada influenza virus vaccine polyvalent		1 Df dosage form/Day 120	20101012-20101012 -	Flu vaccination

Drugs (concomitant)

3 Artelac, Augentrpf, Dr. Mann hypromellose	Ophthalmic	8 Gtt drop(s)/Day 71	2000- -	
4 Calcitrat, Filmtabl, Merckle calcium citrate	Oral	3 Df dosage form/Day 82	201001-20101106 -	
5 L-Thyrox, 50 µg, Tabl, Hexal levothyroxine sodium	Oral	/ 245	201004- -	Latent hypothyroidism
6 Magium K Forte, Tabl (mr), Hexal magnesium hydrogen aspartate; potassium hydrogen aspartate	Oral	1 Df dosage form/Day 94	2004-20101106 -	
7 Nebivolol, 5 mg, Tabl, Stada nebivolol	Oral	5 Mg milligram(s)/Day 245	2000-20101106 -	Hypertension arterial
8 Triarese, Tabl, Hexal hydrochlorothiazide; triamterene	Oral	0.5 Df dosage form/Day 245	2009-20101106 -	
9 Vigantolekten, 500 IE, Tabl, Merck colecalciferol	Oral	500 Iu international unit(s) 245	201007- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthrosis	2008		Yes	Arthrose linker Mittelfuß
Hypertension arterial	201011		Yes	
Migraine	1984		Yes	
Latent hypothyroidism	201004		Yes	substituierte Hypothyreose

Primary source

Qualification: Physician observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20100507	15	U/l	
Alanine aminotransferase increased	20101108	1700	U/l	
Alanine aminotransferase increased	20101109	1963	U/l	
Alanine aminotransferase increased	20101110	1404	U/l	
Alanine aminotransferase increased	20101206	525	U/l	

Aspartate aminotransferase increased	20101109	1285	U/l
Aspartate aminotransferase increased	20101206	193	U/l
Alkaline phosphatase increased	20100507	106	U/l
Alkaline phosphatase increased	20101108	478	U/l
Alkaline phosphatase increased	20101109	494	U/l
Alkaline phosphatase increased	20101206	232	U/l
Bilirubin increased	20101108	14.3	mg/dl
Bilirubin increased	20101109	19.4	mg/dl
Bilirubin increased	20101206	5.0	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 14.02.2011 DE-FAKOS-170381

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 20.04.2011 DE-FAKOS-170381

Sender diagnosis: not reported

Narrative

Die Patientin fühlte seit 01.10.2010 eine starke Abgeschlagenheit und Leistungsminderung. Zusätzlich wurde am 22.10.2010 ein dunkler Urin festgestellt, Juckreiz und entfärbter Stuhl am 01.11.2010, ein Ikterus am 07.11.2010. Weiterhin wurde ein Gewichtsverlust von etwa 4 kg in den letzten 2-3 Wochen bemerkt. Der Hausarzt stellte am 08.11.2010 stark erhöhte Leberwerte fest (ALAT 1700 U/l (Normbereich: bis 35 U/l); Bilirubin gesamt 14,3 mg/dl (Normbereich: bis 1,2 mg/dl)). Vorwerte vom 07.05.2010: ALAT 15 U/l, Gamma-GT 35 U/l (Normbereich: bis 42 U/l), alkalische Phosphatase 106 U/l (Normbereich: bis 105 U/l). Am 09.11.2010 folgte die Krankenhausaufnahme mit weiterem Anstieg der Leberwerte: ALAT 1963 U/l, ASAT 1285 U/l (Normbereich: < 35 U/l), Bilirubin gesamt 19,4 mg/dl, Gamma-GT 749 U/l, alkalische Phosphatase 494 U/l. Wegen einer Arthrose im Mittelfuß links wurde ab November 2008 eine Schmerzmedikation mit Diclofenac eingeleitet, seit Juli 2009 wurden Diclofenac retard ratiopharm SL 75 mg Retardkps. 2-3 x wöchentlich eingenommen (letzte Einnahme am 03.11.2010). Am 12.10.2010 erhielt die Patientin eine Gripeschutzimpfung (Grippe-Impfstoff Stada). Eine autoimmune oder virale Genese konnte laborchemisch und in der durchgeführten Leberbiopsie am 16.11.2010 ausgeschlossen werden. Cholestatische Hepatitis mit dichter gemischter portaler Entzündungsinfiltration, erhöhte Eosinophilenbeteiligung. Abdomen-CT: kein Anhalt für einen ischämisch bedingten Bilirubinanstieg. Sonographie: Leber unauffällig. Eine Therapie mit Steroiden wurde eingeleitet. Erst nach systemischer Steroidtherapie kam es zu einer verzögerten Besserung der Leberwerte. Am 07.12.2010 konnte die Patientin aus dem Krankenhaus entlassen werden. Leberwerte vom 06.12.2010: ALAT 525 U/l, ASAT 193 U/l, Bilirubin gesamt 5,0 mg/dl, Gamma-GT 1104 U/l, alkalische Phosphatase 232 U/l. Anschließend folgte eine Rehabilitationsmaßnahme in einer Reha-Klinik. Die UAW hat sich ambulant entwickelt. Die Arzneimittelkausalität für Diclofenac retard ratiopharm SL 75 mg Retardkps. und den Grippe-Impfstoff Stada wird mit "wahrscheinlich" bewertet.<dbtext>To_Char_NARRATIVE_2</dbtext>

PVZ-FAKOS, Klinische Pharmakologie, 20.05.2011 DE-FAKOS-170381

Sender diagnosis: not reported

Narrative

Die Patientin fühlte seit 01.10.2010 eine starke Abgeschlagenheit und Leistungsminderung. Zusätzlich wurde am 22.10.2010 ein dunkler Urin festgestellt, Juckreiz und entfärbter Stuhl am 01.11.2010, ein Ikterus am 07.11.2010. Weiterhin wurde ein Gewichtsverlust von etwa 4 kg in den letzten 2-3 Wochen bemerkt. Der Hausarzt stellte am 08.11.2010 stark erhöhte Leberwerte fest (ALAT 1700 U/l (Normbereich: bis 35 U/l); Bilirubin

gesamt 14,3 mg/dl (Normbereich: bis 1,2 mg/dl)).

Vorwerte vom 07.05.2010: ALAT 15 U/l, Gamma-GT 35 U/l (Normbereich: bis 42 U/l), alkalische Phosphatase 106 U/l (Normbereich: bis 105 U/l).

Am 09.11.2010 folgte die Krankenhausaufnahme mit weiterem Anstieg der Leberwerte: ALAT 1963 U/l, ASAT 1285 U/l (Normbereich: < 35 U/l), Bilirubin gesamt 19,4 mg/dl, Gamma-GT 749 U/l, alkalische Phosphatase 494 U/l.

Wegen einer Arthrose im Mittelfuß links wurde ab November 2008 eine Schmerzmedikation mit Diclofenac eingeleitet, seit Juli 2009 wurden Diclofenac retard ratiopharm SL 75 mg Retardkps. 2-3 x wöchentlich eingenommen (letzte Einnahme am 03.11.2010). Am 12.10.2010 erhielt die Patientin eine Gripeschutzimpfung (Grippe-Impfstoff Stada).

Abdomen-CT: kein Anhalt für einen ischämisch bedingten Bilirubinanstieg.

Sonographie: Leber unauffällig.

Es bestanden keine Hinweise für eine virale oder autoimmune Genese der akuten Hepatitis (Serologie auf Hepatitis A, B, C und E negativ, ANA und AMA negativ). Die weitere Evaluierung der Histologie ergab den Befund einer akuten bis subakuten portalen und lobulären Hepatitis mit erhöhter Eosinophilenbeteiligung am Entzündungsinfiltrat bei diskreter portaler Fibrosierungstendenz. Es bestand auch ein erhöhter Anteil von B-Lymphozyten und Plasmazellen, was für eine gewisse autoimmune Triggerung sprechen kann. Der histologische Befund wurde von Pathologen bewertet als ein Befund wie bei einer medikamenteninduzierten floriden akuten bis subakuten Hepatitis wobei eine bereits manifeste chronische Hepatitis nicht beschrieben wird. Bei Verdacht auf immunologische Genese der Hepatitis wurde eine Therapie mit Steroiden eingeleitet. Erst nach systemischer Steroidtherapie kam es zu einer verzögerten Besserung der Leberwerte. Am 07.12.2010 konnte die Patientin aus dem Krankenhaus entlassen werden. Leberwerte vom 06.12.2010: ALAT 525 U/l, ASAT 193 U/l, Bilirubin gesamt 5,0 mg/dl, Gamma-GT 1104 U/l, alkalische Phosphatase 232 U/l.

Anschließend folgte eine Rehabilitationsmaßnahme in einer Reha-Klinik.

Nach erneuter Evaluierung des Falls kommt Diclofenac als die wahrscheinlichste Ursache der Hepatitis in Frage, welches schon länger eingenommen wurde, der histologische Befund mit einer akuten bis subakuten Hepatitis unterstreicht dies. Daher werden die Arzneimittelkausalitäten jetzt neu bewertet. Die Arzneimittelkausalität für Diclofenac retard ratiopharm SL 75 mg Retardkps wird mit "wahrscheinlich" bewertet. Aufgrund des engen zeitlichen Zusammenhangs zwischen der Impfung mit Grippe-Impfstoff Stada und dem Auftreten der typischen Symptome der Hepatitis wird die Arzneimittelkausalität für Grippe-Impfstoff Stada mit "möglich" bewertet.

Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11039110

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: 19700603	Age: 40 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fatty liver	20110301			34 Day	not recovered/not resolved
2 Abdominal pain	2011	2011			recovered/resolved
3 Weight gain	2011				not recovered/not resolved
4 Flatulence	2011				recovered/resolved
5 Hair growth increased	20110301			34 Day	not recovered/not resolved
6 Leucopenia	20110201	20110330	58 Day	6 Day	recovered/resolved
7 Neutropenia	2011	2011			recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	Abdominal pain
diclofenac			-	
2 ibuprofen		/	-	Abdominal pain
ibuprofen			-	
3 NAVELBINE	Intravenous (not otherwis	25 mg/m2 milligram(s)/sq.	20110126-20110126	Breast cancer
vinorelbine tartrate		231	1 Day	
4 NAVELBINE	Intravenous (not otherwis	25 Mg milligram(s)/	20110202-20110202	
vinorelbine tartrate		231	1 Day	
5 NAVELBINE	Intravenous (not otherwis	25 Mg milligram(s)/	20110323-20110323	
vinorelbine tartrate		231	1 Day	
6 NAVELBINE	Intravenous (not otherwis	25 Mg milligram(s)/21 Day	20110330-	
vinorelbine tartrate		231	-	
7 Xeloda	Oral	2000 Mg milligram(s)/Day	201101-14 Day	Breast cancer
capecitabine				

Drugs (concomitant)

8 neurobion	/	-	Drug use for unknown indication
cyanocobalamin; pyridoxine hydrochloride; thiamine hydrochloride		-	
9 Pantozol	40 Mg milligram(s)/12 Ho	201101-	Prophylaxis
pantoprazole sodium		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hand and foot syndrome	2010	2010	No	
Abdominal pain upper			Unknown	chronic abdominal pain upper
Hypermenorrhea			Unknown	
Microscopic hematuria			Unknown	
Kidney contracted			Unknown	contracted kidney on one side

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Alanine aminotransferase	20110217	69	U/l
Alanine aminotransferase	20110228	116	U/l
Alanine aminotransferase	20110316	58	U/l
Alanine aminotransferase	20110125	33	U/l
Aspartate aminotransferase	20110228	76	U/l
Aspartate aminotransferase	20110316	41	U/l
Aspartate aminotransferase	20110217	42	U/l
Aspartate aminotransferase	20110125	24	U/l
Gamma GT	20110217	48	U/l
Gamma GT	20110228	61	U/l
Gamma GT	20110316	37	U/l
WBC	20110201	2600	Giga/l
WBC	20110217	2300	Giga/l
WBC	20110221	2620	Giga/l
WBC	20110330	3000	Giga/l

Narratives (nur bei elektronischem Eingang)

PFM, 09.03.2011 DE-PFM-DE-2011-0585

Sender diagnosis: not reported

Narrative

Case number: DE-2011-0585

DFR: 03-March-2011

INTRODUCTION:

This case was reported by a gynecologist via phone call and referred to a female patient who received Navelbine (vinorelbine) IV and developed weight gain, hairiness of face and fore arm, flatulence, moderate fatty liver, and severe abdominal pain.

EVENT:

A female patient received Navelbine IV 25 mg/m² on day first and 8th of therapy and Xeloda (capecitabine) oral 2000 mg for 14 days in Jan-2011 for the indication of breast cancer.

On an not specified date after first cycle of therapy she developed severe abdominal pain (that worsened after second cycle), moderate fatty liver, hairiness of sides of face and forearms, weight gain from 55 to 65 kg during one week, and flatulence.

She was hospitalized because of severe abdominal pain on 27-Feb-2011.

TREATMENT and ACTION TAKEN: not specified

OUTCOME: Severe abdominal pain subsided (exact date not specified) and the patient was discharged from hospital on 01-March-2011. The other mentioned events still not abated.

Reporter considered abdominal pain as serious and provided no assessment for the other mentioned events.

AFFILIATE ASSESSMENT:

Causality: according to the WHO algorithm, causality is possible for mentioned events, because of positive time correlation, and Xeloda can be another explanation.

Seriousness: hospitalization for abdominal pain and medically significant for fatty liver.

Expectedness: According to the SPC of Navelbine IV, the mentioned events are unexpected, except pain that is expected.

Further information has been requested.

PIERRE FABRE MEDICAMENT HEADQUARTER'S PRODUCT SAFETY DEPT. MEDICAL ASSESSMENT.

Company evaluation : According to the French method, the assessment of the causality is C1S1 I1, dubiously related to Navelbine for all events except abdominal pain which was assessed as C1S2I1, dubiously related to Navelbine. Xeloda is also considered as suspect.

Seriousness criteria : hospitalization for abdominal pain and medically significant for fatty liver.

Other ADRs: Non serious

Labeling : Referring to Pierre Fabre Medicament Core Data Sheet of Navelbine injectable solution, all ADRs were assessed as unlisted except abdominal pain assessed as listed.

PFM, 08.04.2011 DE-PFM-DE-2011-0585

Sender diagnosis: not reported

Narrative

Case number: DE-2011-0585

DFR: 03-March-2011

INTRODUCTION:

This case was reported by a gynecologist via phone call and referred to a female patient who received Navelbine (vinorelbine) IV and developed fatty liver, moderate hepatomegalia, considerable liver parenchymal damage, severe aggravated abdominal pain, afebrile neutropenia NCI/ CTC grade 3, leucopenia NCI/CTC NCI/ CTC grade 2 , flatulence, weight gain, and hairiness of face and fore arm.

PAST DRUG HISTORY and PAST MEDICAL HISTORY:

The patient received Taxotere and Cyclophosphamide for breast cancer since 07-Sep-2010 , then this therapy stopped before Navelbine therapy (date not specified) . She developed hand and foot syndrome during Taxotere and Cyclophosphamide therapy, that was treated with Lyrica (Pregabalin).

CONCOMITANT DISEASE:

- progression of disease under therapy with Taxotere and Cyclophosphamide
- shrank kidney on one side, chronic upper abdominal pain , microhematuria, hypermenorrhea
- no history of allergies
- no previous abnormal hairiness
- abdominal pain

CONCOMITANT MEDICATION:

Pantozol (Pantoprazol) for prophylaxis (stomach protection), Neurobion (Cyanocobalamin, Pyridoxine hydrochloride, Thiamine hydrochloride) for unknown indication. The patient received Ibuprofen (date not specified) and Diclofenac since Jan-2011 for abdominal pain.

EVENT:

A 40 year-old female patient received Taxotere and Cyclophosphamide for the indication of breast cancer since 07-Sep-2010. Because of progression of disease under this therapy stopped (date not specified) and the chemotherapy switched to Navelbine IV 25 mg/m2 from 26-Jan-2011 cyclically as well as Xeloda (capecitabine) oral 2000 mg for a duration of 14 days.

She received Navelbine IV on 26-Jan-2011 and on 02-Feb-2011.

She experienced leucopenia NCI/CTC grade 2 on 01-Feb-2011.

After first cycle of therapy she developed severe abdominal pain, flatulence, neutropenia NCI/CTC grade 3 and weight gain from 55 to 65 kg during one week (exact onset dates could not be specified).

Abdominal pain worsened after second administration of therapy and she was hospitalized because of severe aggravated abdominal pain on 27-Feb-2011.

On 01-Mar-2011 she experienced moderate fatty liver, considerable liver parenchymal damage, moderate hepatomegalia and hairiness of sides of face and forearms.

Hepatitis infection has been excluded.

TREATMENT:

Diclofenac since Jan 2011 and Ibuprofen (date not specified) continued for abdominal pain and on an not specified date were discontinued because the physician considered them as suspected drugs for events. Granulocyte colony stimulating factor for leucopenia and neutropenia (date not specified). No treatment for the other events.

RELEVANT LABORATORY VALUES:

WBC: 01.02.2011: 2600/microliter, 17.02.2011: 2300//microliter, 21.02.2011 2620/microliter, 30.03.2011 3000/microliter.

Neutrophils were below 1000/microliter during the period of leucopenia (exact dates not provided).

Reportedly the liver laboratory values on 23-Dec-2010 (about one month before Navebine therapy) were normal.

GOT: 25.01.2011: 24 U/l, 17.2.2011: 42 U/l, 28.02.2011: 76 U/l, 16.03.2011: 41 U/l

GPT: 25.01.2011: 33 U/l, 17.2.2011: 69 U/l, 28.02.2011: 116 U/l, 16.03.2011: 58 U/l

GGT: 17.2.2011: 48 U/l, 28.02.2011: 61 U/l, 16.03.2011: 37 U/l

ACTION TAKEN :

Navelbine therapy has been postponed because of abdominal pain, and neutropenia and she received again Navelbine IV on 23-Mar-2011 and on 30-Mar-2011. According to the physician suspected drugs (Ibuprofen, Diclofenac) has been discontinued.

OUTCOME:

Severe abdominal pain subsided (exact date not specified) and the patient was discharged from hospital on 01-March-2011.

Flatulence has been resolved on a not specified date.

Leucopenia NCI/ CTC grade 2 resolved on 30-Mar-2011 and afebrile neutropenia grade 3 resolved in the same period of time (exact date not specified).

The other mentioned events still not abated. However the patient's situation is clinically stable.

Reporter considered the events as serious and as possibly related to Navelbine IV. He does not know if there is a link between abdominal pain and the hepatic disorders or any other causes.

AFFILIATE ASSESSMENT:

Causality: according to the WHO algorithm, causality is possible for mentioned events, because of positive time correlation. Xeloda and other concomitant medication can be another explanation. Causality for abdominal pain is unlikely related as the patient has prior history.

Seriousness: hospitalization for abdominal pain. Medically significant for fatty liver, moderate hepatomegaly, neutropenia grade 3, considerable liver parenchymal damage. Other events are non serious.

Expectedness: According to the SPC of Navelbine IV, the mentioned events are unexpected, except pain, afebrile neutropenia and leucopenia that are expected.

PIERRE FABRE MEDICAMENT HEADQUARTER'S PRODUCT SAFETY DEPT. MEDICAL ASSESSMENT.

Company evaluation : According to the French method, the assessment of the causality is C2S2I2, possibly related to Navelbine for leucopenia and neutropenia, C1S1 I1, dubiously related to Navelbine for flatulence, fatty liver, weight gain and hairiness and C1S2I1, dubiously related to Navelbine for abdominal pain. Xeloda is also considered as suspect. Diclofenac and ibuprofen were also considered as suspect by the reporter.

Seriousness criteria : hospitalization for abdominal pain and medically significant for fatty liver and neutropenia.

Other ADRs: Non serious

Labeling : Referring to Pierre Fabre Medicament Core Data Sheet of Navelbine injectable solution, all ADRs were assessed as unlisted except abdominal pain, neutropenia and leukopenia, assessed as listed.

FU received on 01-Apr-2011 from physician:

- patients initial, birth date
- NVB dosage
- concomitant disease and medication
- past drug and medical history
- action taken outcome
- onset dates
- his assessment

Narrative and affiliate assessment modified.

Further information has been requested.

FU received on 07-Apr-2011, the physician added

- neutropenia and leucopenia as event
- laboratory values
- outcome
- concomitant medication, treatment, past medical history and past drug history
- his assessment

Narrative and affiliate assessment modified.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11045060

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials: Sex: Female Birth: 19700603 Age: 39 Year weight (kg): 65 BMI_int: 23.88 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fatty liver	201002				Unknown
2 Steatosis hepatic	20110301				recovering/resolving
3 Hair growth abnormal	20110301				recovering/resolving
4 Body hair increased	201002				Unknown
5 Upper abdominal pain	201002				Unknown
6 Flatulence	201002				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	50 Mg milligram(s)/	-20110315	Abdominal pain
diclofenac			-	
2 NAVELBINE	Intravenous (not otherwis	25 mg/m2 milligram(s)/sq.	20110126-	Breast cancer
vinorelbine			-	
3 XELODA	Oral	2000 mg/m2 milligram(s)/s	20110126-	Metastatic breast cancer
capecitabine			-	

Drugs (concomitant)

4 LYRICA	Oral	50 Mg milligram(s)/	-20110201	Hand and foot syndrome
pregabalin			-	
5 PANTOZOL	Oral	40 Mg milligram(s)/	-	Gastric disorder
pantoprazole			-	
6 PARACETAMOL	Oral	500 Mg milligram(s)/	-	Pain NOS
paracetamol			-	
7 VINORELBIN	/		-	Metastatic breast cancer
vinorelbine			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Neoplasm recurrence	201012		Unknown	
Breast operation	201006		No	
Metastatic breast cancer	201001		Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Sonogram	201002	FATTY LIVER/DIAGNOSED		
Ultrasound scan	20101223	NORMAL, TEST IN THE BASIC ULTRASOUND SCAN		
Serology test	20110305	NO INFECTION/UNTITLED		

Narratives (nur bei elektronischem Eingang)

ROCHE, 11.03.2011 DE-ROCHE-764492

Sender diagnosis: not reported

Narrative

INITIAL INFORMATION FOR THIS SPONTANEOUS CASE WAS RECEIVED ON 3-MAR-2011 FROM A PHYSICIAN AND CONCERNS A 40-YEAR-OLD FEMALE PATIENT WHO EXPERIENCED FATTY LIVER, BODY HAIR INCREASED, UPPER ABDOMINAL PAIN AND FLATULENCE WHILE BEING TREATED WITH CAPECITABINE (XELODA) FOR METASTATIC MAMMA CARCINOMA.

THE PATIENT'S CONCURRENT CONDITION INCLUDED METASTATIC BREAST CANCER FIRST DIAGNOSED APPROXIMATELY IN JAN-2010. HER PAST MEDICAL HISTORY INCLUDED BREAST-PRESERVING THERAPY APPROXIMATELY IN JUN-2010 AND TUMOUR RECURRENCE IN DEC-2010. SHE CONCOMITANTLY RECEIVED VINOELBINE AND DOCETAXEL IN THE PAST.

THE PATIENT COMMENCED CAPECITABINE AT THE END OF JAN-2010. AFTER 2 CYCLES OF THIS DRUG SHE DEVELOPED, STRONG HAIRINESS OF HER FACE AND ARMS, UPPER ABDOMINAL PAIN AND FLATULENCE. A SONOGRAPHY REVEALED A FATTY LIVER. ACTION TAKEN WITH CAPECITABINE AND THE OUTCOME OF THE EVENTS WERE NOT PROVIDED.

THE REPORTER ASSESSED THE EVENTS OF UPPER ABDOMINAL PAIN AND FLATULENCE AS RELATED TO THE USE OF CAPECITABINE AND THE EVENT/DRUG RELATIONSHIP FOR THE EVENTS OF FATTY LIVER, BODY HAIR INCREASED AS UNKNOWN.

THE REPORTER DID NOT PROVIDE ANY SERIOUSNESS CRITERIA FOR THE EVENTS OF FATTY LIVER, BODY HAIR INCREASED, UPPER ABDOMINAL PAIN AND FLATULENCE.

THE COMPANY ASSESSED THESE EVENTS AS MEDICALLY SIGNIFICANT.

ROCHE, 20.04.2011 DE-ROCHE-764492

Sender diagnosis: not reported

Narrative

INITIAL INFORMATION FOR THIS SPONTANEOUS CASE WAS RECEIVED ON 3-MAR-2011 FROM A PHYSICIAN AND CONCERNS A 40-YEAR-OLD FEMALE PATIENT WHO EXPERIENCED FATTY LIVER, BODY HAIR INCREASED, UPPER ABDOMINAL PAIN AND FLATULENCE WHILE BEING TREATED WITH CAPECITABINE (XELODA) FOR METASTATIC MAMMA CARCINOMA.

THE PATIENT'S CONCURRENT CONDITION INCLUDED METASTATIC BREAST CANCER FIRST DIAGNOSED APPROXIMATELY IN JAN-2010. HER PAST MEDICAL HISTORY INCLUDED BREAST-PRESERVING THERAPY APPROXIMATELY IN JUN-2010 AND TUMOUR RECURRENCE IN DEC-2010. SHE CONCOMITANTLY RECEIVED VINOELBINE AND DOCETAXEL IN THE PAST.

THE PATIENT COMMENCED CAPECITABINE AT THE END OF JAN-2010. AFTER 2 CYCLES OF THIS DRUG SHE DEVELOPED, STRONG HAIRINESS OF HER FACE AND ARMS, UPPER ABDOMINAL PAIN AND FLATULENCE. A SONOGRAPHY REVEALED A FATTY LIVER. ACTION TAKEN WITH CAPECITABINE AND THE OUTCOME OF THE EVENTS WERE NOT PROVIDED.

THE REPORTER ASSESSED THE EVENTS OF UPPER ABDOMINAL PAIN AND FLATULENCE AS RELATED TO THE USE OF CAPECITABINE AND THE EVENT/DRUG RELATIONSHIP FOR THE EVENTS OF FATTY LIVER, BODY HAIR INCREASED AS UNKNOWN.

THE REPORTER DID NOT PROVIDE ANY SERIOUSNESS CRITERIA FOR THE EVENTS OF FATTY LIVER, BODY HAIR INCREASED, UPPER ABDOMINAL PAIN AND FLATULENCE.

THE COMPANY ASSESSED THESE EVENTS AS MEDICALLY SIGNIFICANT.

ADDITIONAL INFORMATION WAS RECEIVED ON 05 APRIL 2011.

THE PATIENT INITIALS, BIRTH DATE, WEIGHT, AND HEIGHT WERE UPDATED.

THE CONCOMITANT MEDICATION INCLUDED PREGABALIN, PARACETAMOL AND PANTOPRAZOLE.

ON 23 DECEMBER 2010, THE PATIENT HAD NORMAL HEPATIC ULTRASOUND. PATIENT HAD HISTORY OF MICROHEMATURIA AND ONE SIDE KIDNEY DISEASE. ON 26 JANUARY 2011, THE PATIENT STARTED TREATMENT WITH ORAL CAPECITABINE (FORM: UNKNOWN) AT A DOSE OF 2000 MG/ME2 DAILY (DAY-1, DAY - 14) AND INTRAVENOUS VINOELBINE (FORM: UNKNOWN) AT A DOSE OF 25 MG/ME2 THRICE WEEKLY (DAY 1, DAY 8) EVERY 21 DAYS. ON 01 MARCH 2011, THE PATIENT EXPERIENCED MEDICALLY SIGNIFICANT EVENT OF STEATOSIS HEPATIC AND HAIR GROWTH ON FACE. ON AN UNSPECIFIED DATE, THE PATIENT STARTED TREATMENT WITH ORAL DICLOFENAC (FORM: UNKNOWN) AT A DOSE OF (2 X 50 MG) DAILY. ON 05 MARCH 2011, THE PATIENT HAD SEROLOGY TEST WHICH SHOWED NO INFECTION. ON 15 MARCH 2011, THE TREATMENT WITH DICLOFENAC WAS DISCONTINUED THE PATIENT WAS ON ADJUNCT SITUATION AND HAD A LOCAL REACTION. ON 12 NOVEMBER 2010, THE PATIENT WAS UNDER TAC CHEMOTHERAPY.

THERE WAS INSUFFICIENT INFORMATION REGARDING THE THERAPY ONGOING STATUS OF CAPECITABINE AND VINOELBINE. THE OUTCOME OF THE EVENT OF STEATOSIS HEPATIC AND HAIR GROWTH ON FACE WAS STABLE.

THE PHYSICIAN ASSESSED THE EVENT OF STEATOSIS HEPATIC AS RELATED WITH CAPECITABINE, VINORELBINE AND DICLOFENAC.

THE PHYSICIAN DID NOT PROVIDE THE CAUSAL RELATIONSHIP BETWEEN THE EVENT OF HAIR GROWTH ON FACE WITH CAPECITABINE, VINORELBINE AND DICLOFENAC.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11049925

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	Yes	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Joint pain	2002				Unknown
2 Disorder liver	2002				Unknown
3 Spleen disorder	2002				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac			-	indication
2 ibuprofen ct 600		/	2001-	Gout
ibuprofen			-	
3 telfast	Unknown	/	-	Drug use for unknown indication
fexofenadine hydrochloride			-	indication

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 31.03.2011 DE-SANOFI-AVENTIS-2011SA018379

Sender diagnosis: not reported

Narrative

Initial report:

On 28-Mar-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11049925); documenting a case, in which a consumer of unknown age had reported about himself to the Drug Committee of the German Physicians Association. Relevant medical history, concomitant disease(s) and drug(s) were not provided.

For unspecified indication the patient took diclofenac and fexofenadine hydrochloride (Telfast) during an unspecified time period. Ibuprofen (Company: CT) 600 mg for gout was received in 2001. Twelve months later (in 2002) joint pain, liver and spleen disorder were detected. No further information was received.

The seriousness criterion was specified with disability.

A causal assessment was not provided.

performed: 31-MAR-2011, Sanofi-Aventis Germany

This case report includes clinical diagnoses, which are considered locally unlabelled for one (or more) suspected product(s). The medical evaluation and the facts presented here however are not very suggestive of a causal relationship (please refer to the international company comment), and thus do not imply a change in the state of scientific knowledge for the suspected product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

NOVARTIS, 08.04.2011 DE-002147023-PHHY2011DE25017

Sender diagnosis: not reported

Narrative

18.03.2013

BfArM - Pharmacovigilance

518 / 594

Case number PHHY2011DE25017, is an initial spontaneous report from a consumer through the Drug Commission of the German Medical Association of Physicians via Health Authority (BfArM- Ref no DE-BFARM-11049925) on 25 Mar 2011: This report refers to a male patient (age not specified). This patient received diclofenac (manufacturer unknown), Telfast for an unspecified indication from an unknown dates and Ibuprofen 600 for gout from 2011. In 2002, patient experienced joint pain. On an unknown date patient experienced liver disorder and spleen disorder. The outcome and the causality were not reported.

Serious spontaneous report,

- arthralgia, spleen disorder [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

- liver disorder [medically significant], assessed as listed in the core data sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11061215

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19540423	Age: 56 Year	weight (kg): 60	BMI_int: 22.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic	20110221				recovered/resolved
2 Cholestasis intrahepatic	20110221				recovered/resolved
3 Icterus	20110221				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	200 Mg milligram(s)/Day	20110211-20110213	
diclofenac			3 Day	
2 diclofenac	Oral	100 Mg milligram(s)/Day	20110214-20110221	
diclofenac			8 Day	

Drugs (concomitant)

3 cefuroxim	Oral	500 Mg milligram(s)/Day	20110211-20110213	
cefuroxime sodium			3 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
House dust allergy			Yes	
Pollen allergy			Yes	
Hallux Valgus correction	20110211	20110211	No	bds.

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11084088

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GGT increased	201103				not recovered/not resolved
2 GPT increased	201103				not recovered/not resolved
3 Hepatic enzymes increased	201103				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLO SL 1A PHARMA diclofenac	Oral	75 Mg milligram(s)/ 94	201101- -	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	201012	42	U	
GPT	201103	115	U	
GGT	201012	24	U	
GGT	201103	53	U	
MCV	201012	88.0	fL	
MCV	201103	86.6	fL	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 05.04.2011 DE-SZ09-GXKR2011DE01365

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 31 Mar 2011 from a consumer: This patient, abstinent from alcohol, developed worsening of liver values with increase of GGT and GPT during treatment with Diclo 75 SL - 1A Pharma (diclofenac) for intense back pain. Dosage and exact therapy dates were not provided. No concomitant drug was taken. While GGT was 24 and GPT was 42 in Dec 2010, whereas the values were 53 and 115, respectively, in Mar 2011. Condition was unchanged at the report date. The reporter provided no causality assessment.

Serious spontaneous report (medically significant) of hepatic enzyme increased, already listed in the Core Data Sheet Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11094624

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	Yes	Yes	YES	Yes

Patient

Initials: Sex: Male Birth: 20090829 Age: weight (kg): 1.995 BMI_int: 10.79 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Testicle undescended	20090829				Unknown
2 Premature baby	20090829				recovered/resolved
3 Intrauterine growth retardation					recovered/resolved
4 Small for gestational age	20090829				recovered/resolved
5 Ductus arteriosus stenosis fetal					recovered/resolved
6 Drug exposure in utero	20090829				recovered/resolved
7 Hyperbilirubinaemia neonatal	20090829				recovered/resolved
8 Drug exposure during pregnancy					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 decortin-h prednisolone	Transplacental	/	- 246 Day	Arthritis rheumatoid
2 DECORTIN-H prednisolone	Transplacental	20 Mg milligram(s)/Day	-	
3 DECORTIN-H prednisolone	Transplacental	15 Mg milligram(s)/Day	-	
4 DECORTIN-H prednisolone	Transplacental	30 Mg milligram(s)/Day	-	Rheumatoid arthritis
5 DECORTIN-H prednisolone	Transplacental	5 Mg milligram(s)/Day	-	
6 DECORTIN-H prednisolone	Transplacental	10 Mg milligram(s)/Day	-	
7 DECORTIN-H prednisolone	Transplacental	15 Mg milligram(s)/Day	-	
8 ibuprofen ibuprofen	Transplacental	/	- 43 Day	Arthritis rheumatoid
9 voltaren resinat diclofenac resinate	Transplacental	/	- 190 Day	Arthritis rheumatoid

Drugs (concomitant)

10 FOLSÄURE folinic acid	Transplacental	/	20081115-20090815 9 Month	Prophylaxis of neural tube defect
11 humira adalimumab	Transplacental	/	-	Arthritis rheumatoid
12 paracetamol paracetamol	Transplacental	/	- 57 Day	Arthritis rheumatoid

Primary source

Qualification: Physician observestudytype:
Literature:
Qualification: Consumer or other non health professional observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

BBGes, 24.05.2011 DE-EMBRYOTOX-20094140-B

Sender diagnosis: not reported

Narrative

The preterm baby boy was born in gw 35+1 and was small for gestational age. He needed 24 hours of phototherapy because of hyperbilirubinemia. Otherwise he was healthy, only the testicles were not descended. If they were descended in the later controls is not noticed.

Sender's comment:

Premature birth and growth retardation are described with cortisone intake. They could also be due to the underlying disease of the mother. Hyperbilirubinemia is a common condition with preterm babies, a causality with prednisolon can not be ruled out. Non descensus of testicles is typical in preterm children, a causality to ibuprofen and diclofenac is not very likely but can not be ruled out. Newer studies with causality to paracetamol only saw correlation when paracetamol was taken in the first or second trimester. the patient took paracetamol only in the third trimester therefore causality is unlikely.

Reporter's comment:

sanofi-aventis, 31.05.2011 DE-SANOFI-AVENTIS-2011SA033122

Sender diagnosis: not reported

Narrative

Initial report received on 26-May-2011:

The case was extracted from the webpage of the German health authority (authority no: DE-BFARM-11094624); initially reported from a German physician, provided via Embryotox (id. no: DE-EMBRYOTOX-20094140-B).

The case involves a male neonate whose mother was treated with ibuprofen (no trade name provided) 600 mg daily for 43 days from the 16th to 22nd week of gestation for rheumatoid arthritis. Additional therapy for rheumatoid arthritis included diclofenac resinate 140 mg twice daily from the beginning of the pregnancy until 27th week as well as prednisolone as following: 5 mg daily from the beginning of the pregnancy until 14th week of gestation, 10 mg daily from 14th week of gestation until 16th week of gestation, 15 mg daily from 16th week of gestation until 20th week of gestation, 20 mg daily from 20th week of gestation until 22nd week of gestation and 15 mg daily from 27th week of gestation until 35th week of gestation. In the 22nd week of gestation oligohydramnios was diagnosed (see associated case id: 2011SA032848), this recovered and was normally in 34th week of gestation. In the 27th week of gestation constriction of ductus arteriosus was diagnosed, which improved in the 29th + 4 week of gestation and was recovered in the 34th +1 week of gestation. The child was controlled and intrauterine growth restriction was noticed. The preterm baby boy was born in the 35th + 1 week of gestation, after premature rupture of membranes. He was small for gestational age. He needed 24 hours of phototherapy because of hyperbilirubinemia. Otherwise he was healthy, only the testicles were not descended. If they were descended in later controls was not noticed. Concomitantly she was treated with paracetamol and folic acid. Adalimumab was taken 3 month prior to conception. Further information including significant past and present medical history was not provided.

Causality Assessment according Drug Embryotox (method: Global Introspection (WHO GI)): possible for testicle undescended; unlikely for premature baby 33 to 36 weeks, small for gestational age and hyperbilirubinaemia neonatal.

NOVARTIS, 03.06.2011 DE-002147023-CHNY2011DE00451

Sender diagnosis: not reported

Narrative

Initial spontaneous case report received on 25 May 2011 from BBGes (Regional Pharmacovigilance Center, Department of Embryonaltoxikologie) via BfArM (German Health Authority, ref number DE-BfArM-11094624). This retrospective pregnancy case described the occurrence of ductus arteriosus stenosis fetal, cryptorchism, intrauterine growth retardation, hyperbilirubinaemia neonatal and premature baby in a neonate exposed to different drugs in utero. The preterm baby boy was born in gestational week 35+1. He needed 24 hours of phototherapy because of hyperbilirubinaemia. Otherwise, he was healthy, only the testicles were not descended. Suspected drugs (taken by patient's mother during pregnancy) included: Decortin-H (prednisolone, different dosages, from 0 to 35 gestational week, total 246 days), Voltaren resinate (diclofenac resinate, from 0 to 27 gestational week, total 190 days), and ibuprofen (unknown manufacturer, taken from 16 to 22 gestational week). Concomitant medication included: paracetamol (from 27 to 35 gestational week, total 57 days), Folsaure (folic acid, from 0 to 33.5 gestational week, for prophylaxis of neural tube defect). Decortin-H, Voltaren resinate, ibuprofen, and paracetamol were taken for arthritis rheumatoid. The mother's patient took Humira (adalimumab), 40 mg (frequency unknown) in preconceptional period, for arthritis rheumatoid. The outcome of the events was reported as follows: recovery after removal of diclofenac for ductus arteriosus stenosis fetal; unknown for cryptorchism; recovered for foetal growth restriction; resolved for premature baby and recovered for hyperbilirubinaemia neonatal. The reporter commented: Premature birth and growth retardation are described with cortisone intake. They could also be due to the underlying disease of the mother (arthritis rheumatoid). Hyperbilirubinaemia is a common condition with preterm babies, a causality with

prednisolone can not be ruled out. Non descensus of testicles is typical in preterm children, a causality to ibuprofen and diclofenac is not very likely but can not be ruled out. Newer studies with causality to paracetamol only saw correlation when paracetamol was taken in the first or second trimester. The mother's patient took paracetamol only in the thisrd trimester therefore causality is unlikely. Linked mother case CHNY2011DE00450.

Reporter's comment:

MENARINIP, 08.06.2011 DE-MEN-22182

Sender diagnosis: not reported

Narrative

This case, initially reported by a regional pharmacovigilance center (ID: DE-EMBRYOTOX-20094140-B) and then published by BfArM as an ICH-E2B-compliant ICSR on its website, involved a boy who was born in 35th+1 week of pregnancy as a premature baby with neonatal hyperbilirubinaemia and undescended testicle. Further, the newborn was small for gestational age.

The mother had a medical history of rheumatoid arthritis for which she had been on treatment with 600 mg/day ibuprofen from 16th to 22th gestation week (gw), 140 mg/twice daily diclofenac resinate (Voltaren resinat) from 0th to 27th week of pregnancy and prednisolone (Decortin H), which was taken during the entire pregnancy (gw 0-14: 5 mg/day, gw 14-16: 10 mg/day, gw 16-20: 15 mg/day, gw 20-22: 20 mg/day, gw 27-35: 15 mg/day).

The mother's concomitant medication had included paracetamol between 1500 mg/die and 3000 mg/die from 27th to 35th gw for rheumatoid arthritis and folic acid for prophylaxis of neural tube defect between 15-Nov-2008 and 15-Aug-2009. Preconceptional she had received 40 mg/day adalimumab (Humira) for rheumatoid arthritis.

The boy who was born in 35th+1 week of pregnancy as a premature baby, small for gestational age, with neonatal hyperbilirubinaemia and undescended testicle. The boy needed 24 hours of phototherapy because of hyperbilirubinaemia. Outcome of undescended testicle was unknown, otherwise the baby was healthy.

The reporter assessed the events to be possibly related to prednisolone. Only the event undescended testicle was considered as possibly related to ibuprofen and diclofenac resinate (Voltaren resinat), the other events were assessed to be unlikely related to these drugs.

The case of the mother was described in case report DE-MEN-39647.

NOVARTIS, 09.06.2011 DE-002147023-PHHY2011DE45641

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE45641 is an initial PMS report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-EMBRYOTOX-20094140-B) on 25 May 2011. This report refers to a male (baby) patient. Baby's mother had current condition of rheumatoid arthritis (See the case: PHHY2011DE45626). This poly-medicated baby's mother received Voltaren resinat (diclofenac resinate) at a dose of 280 mg per day for rheumatoid arthritis. She also received Decortin-h (prednisolone) at a dose of 30 mg per day, gestation weeks (gw): 0-14, 5 mg per day, gw: 14-16, 10 mg per day, gw: 16-20, 15 mg per day, gw: 20-22, 20 mg per day, gw: 27-35 and 15 mg per day for rheumatoid arthritis and Ibuprofen at a dose of 600 mg per day for rheumatoid arthritis. She took prednisolon in rising dosages in the entire pregnancy. The patient took from gestation weeks (gw) 16-22 ibuprofen, from gw 0-27 diclofenac and from gw 27-35 paracetamol. In gw 27 constriction of ductus arteriosus was diagnosed, improvement in gw 29+4, normal in gw 34+1 after therapy with diclofenac was stopped in gw 27. The child was controlled and intrauterine growth restriction was noticed. The preterm baby was born in gw 35+1 and was small for gestational age. He needed 24 hours of phototherapy because of hyperbillirubinaemia. He was healthy only the testicles were not descended. The outcome was reported as recovered for the events premature baby, small for gestational age, hyperbilirubinaemia neonatal, foetal growth retardation, ductus arteriosus stenosis foetal and the outcome was not reported for testicle undescended. The causality was reported as not suspected for the events premature baby, small for gestational age, hyperbilirubinaemia neonatal and the causality was reported as suspected for testicle undescended, constriction of ductus arteriosus and foetal growth retardation.

Reporter's comment:

MENARINIP, 09.06.2011 DE-MEN-22182

Sender diagnosis: not reported

Narrative

This case, initially reported by a regional pharmacovigilance center (ID: DE-EMBRYOTOX-20094140-B) and then published by BfArM as an ICH-E2B-compliant ICSR on its website, involved a boy who was born in 35th+1 week of pregnancy as a premature baby with neonatal hyperbilirubinaemia and undescended testicle. Further, the newborn was small for gestational age.

The mother had a medical history of rheumatoid arthritis for which she had been on treatment with 600 mg/day ibuprofen from 16th to 22th gestation week (gw), 140 mg/twice daily diclofenac resinate (Voltaren resinat) from 0th to 27th week of pregnancy and prednisolone (Decortin H), which was taken during the entire pregnancy (gw 0-14: 5 mg/day, gw 14-16: 10 mg/day, gw 16-20: 15 mg/day, gw 20-22: 20 mg/day, gw 27-35: 15 mg/day).

The mother's concomitant medication had included paracetamol between 1500 mg/die and 3000 mg/die from 27th to 35th gw for rheumatoid arthritis and folic acid for prophylaxis of neural tube defect between 15-Nov-2008 and 15-Aug-2009. Preconceptional she had received 40 mg/day adalimumab (Humira) for rheumatoid arthritis.

The boy who was born in 35th+1 week of pregnancy as a premature baby, small for gestational age, with neonatal hyperbilirubinaemia and undescended testicle. The boy needed 24 hours of phototherapy because of hyperbilirubinaemia. Outcome of undescended testicle was unknown, otherwise the baby was healthy.

The reporter assessed the events to be possibly related to prednisolone. Only the event undescended testicle was considered as possibly related to ibuprofen and diclofenac resinate (Voltaren resinat), the other events were assessed to be unlikely related to these drugs.

The case of the mother was described in case report DE-MEN-22181.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11118787

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		No

Patient

Initials:	Sex: Male	Birth: 19420212	Age: 65 Year	weight (kg): 65	BMI_int: 24.17	Age fetus:
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Patientdeath

deathdate: 20071111	deathcause: Perforation stomach
deathdate: 20071111	deathcause: Cardiac arrest
deathdate: 20071111	deathcause: Ascites
deathdate: 20071111	deathcause: Abdominal discomfort
deathdate: 20071111	deathcause: Peritonitis
deathdate: 20071111	deathcause: Ventricular fibrillation
deathdate: 20071111	deathcause: Ulcer
deathdate: 20071111	deathcause: Sepsis
deathdate: 20071111	deathcause: Septic shock

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Death unexplained					fatal
2 Ulcer	20071111				fatal
3 Septic shock	20071111				fatal
4 Sepsis	20071111				fatal
5 Peritonitis	20071111				fatal
6 Perforation stomach	20071111				fatal
7 Cardiac arrest	20071111				fatal
8 Ascites	20071111				fatal
9 Abdominal discomfort	20071111				fatal
10 Stomach pain					Unknown
11 Ventricular fibrillation	20071111				fatal
12 Bowel discomfort					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN diclofenac sodium	Oral	1 Df dosage form/ 245	20071111- -	Cervical pain
2 voltaren resinat diclofenac resinate	Oral	1 Df dosage form/ -	-20071110 -	Cervical pain

Drugs (concomitant)

3 BELOC ZOK metoprolol succinate	Oral	.5 Df dosage form/ 245	- -	
4 CORVATON molsidomine	Oral	8 Mg milligram(s)/ 245	- -	
5 isoket isosorbide dinitrate	Oral	2 Df dosage form/Day 245	- -	
6 PLAVIX clopidogrel sulfate	Oral	1 Df dosage form/ 245	- -	
7 VALORON tilidine hydrochloride		/	- -	
8 ZOCOR simvastatin	Oral	1 Df dosage form/Day 245	- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Wheals			Unknown	
Stent insertion NOS			Unknown	
Osteophytosis			Unknown	
Spleen congestion			Unknown	
Lung disease			Unknown	
Coronary heart disease			Unknown	
Surgery			Unknown	
Bronchiectasis			Unknown	
Cardiac hypertrophy			Unknown	
Cardiac insufficiency			Unknown	
Hepatic congestion			Unknown	
Hypertension arterial			Unknown	
Kyphoscoliosis			Unknown	
Lung congestion			Unknown	
Myocardial fibrosis			Unknown	
Myocardial infarction			Unknown	About 11 years ago Myocardial infarction (ventral scar of 2.5 x 2 centimetres that includes septum)
Prostatic hyperplasia			Unknown	
Renal cyst			Unknown	
Shoulder pain			Unknown	
Thoracic pain			Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 03.06.2011 DE-002147023-PHHY2011DE46256

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE46256, is a combined initial and follow-up spontaneous report received from consumer (patient's wife) on 27 May 2011. This report refers to a 65-years-old male patient. The patient had received high dosages of Voltaren (diclofenac) tablets on prescription of his physician from an unknown date for an unspecified indication. According to his reporting wife "he had severe stomach pain before he was hospitalized in the intensive care unit. He had something with his bowel and underwent a surgery prior to his death". He died in the intensive care unit in 2009. In the hospital she was advised that "the direct descendants of her husband (grandchildren and daughter) should never take Voltaren (NOS)". The reporter herself did not use Voltaren "since it seemed to dangerous for her". Reporter took care "not to use Voltaren. It damages more instead it does something good". Reporter's mother was prescribed topical Voltaren (See the case CHPA2011DE07309). No further information could be obtained from reporter, since she had difficulties to communicate in German. Further follow-up information (incl. consent) was requested on 27 May 2011. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

The remaining reported leading events were assessed as non-serious by Novartis Medical Safety Physician.

Based on Review of available reported data does not allow for a proper causality assessment. Reported event(s) is (are) unassessable

Reporter's comment:NOVARTIS, 14.06.2011 DE-002147023-PHHY2011DE46256

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE46256, is a combined initial and follow-up spontaneous report received from consumer (patient's wife) on 27 May 2011 with follow up report (written information) received from a consumer (patient's wife) via Novartis Consumer Health on 08 Jun 2011. This report refers to a 65-years-old male patient. The patient had history of myocardial infarction about 11 years ago. The patient had received high dosages of Voltaren (diclofenac) tablets on prescription of his physician in Nov 2007 for an unspecified indication. According to his reporting wife "he had severe stomach pain before he was hospitalized in the intensive care unit. He had something with his bowel and underwent a surgery prior to his death". He died in the intensive care unit in 2009. The adverse events (not specified) started in Nov 2007. Voltaren dosage was reduced and the patient was hospitalised. In the hospital she was advised that "the direct descendants of her husband (grandchildren and daughter) should never take Voltaren (NOS)". The reporter herself did not use Voltaren "since it seemed to dangerous for her". Reporter took care "not to use Voltaren. It damages more instead it does something good". Reporter's mother was prescribed topical Voltaren (See the case CHPA2011DE07309). No further information could be obtained from reporter, since she had difficulties to communicate in German. Further follow-up information (incl. consent) was requested on 27 May 2011.

Follow up report (written information) received from a consumer (patient's wife) via Novartis Consumer Health on 08 Jun 2011. Updated medical history myocardial infarction, therapy start date for Voltaren and consumer's comment.

Reporter's comment:

NOVARTIS, 27.07.2011 DE-002147023-PHHY2011DE46256

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE46256, is a combined initial and follow-up spontaneous report received from consumer (patient's wife) on 27 May 2011 with follow up report (written information) received from a consumer (patient's wife) via Novartis Consumer Health on 08 Jun 2011 with follow up physician report received on 20 Jul 2011. This report refers to a 65-years-old male patient. The patient had history of myocardial infarction about 11 years ago, tumor resection at thoracic vertebral column, stent implantation (ramus-interventricularis-anterior), coronary heart disease with stenosis of ramus-interventricularis- anterior, diffuse myocardial fibrosis predominantly at the right half, myocardial hypertrophy, cardiac insufficiency with vascular congestion in lung, liver, spleen and kidney, arterial hypertension, high-grade left-sided kyphoscoliosis of thorax, osteophytes in thoracic and lumbar region, renal cyst (left-side, diameter 2 cm), focal anthracosis with bronchiectasis, wheals of lung apex and mild nodular prostatic hyperplasia. This poly-medicated patient received Voltaren Resinat (diclofenac resinat) dose in morning for pain of cervical and thoracic vertebral column and shoulder pain from unspecified date to 10 Nov 2007. The patient was admitted at hospital on 11 Nov 2007 due to severe pain of cervical and thoracic vertebral column. According to his reporting wife, he had severe stomach pain before he was hospitalized in the intensive care unit. In hospital the patient received Voltaren (diclofenac sodium) tablet once at 5.30 pm at a dose of 100 mg. Patient received catecholamines as a treatment drug. Patient died on 11 Nov 2007 due to cardiac arrest caused by sepsis. The patient also experienced septic shock, ulcer of front wall of antrum, perforation of front wall of antrum (2cm diameter), purulent peritonitis, and purulent ascites and had air in abdomen. The physician specified that the antrum perforation likely had been present before administration of 100 mg Voltaren in hospital. The physician stated the surgery was planned but not performed. Reporter's mother was prescribed topical Voltaren (See the case CHPA2011DE07309). Causality was assessed as suspected.

Follow up report (written information) received from a consumer (patient's wife) via Novartis Consumer Health on 08 Jun 2011. Updated medical history myocardial infarction, therapy start date for Voltaren and consumer's comment.

Follow up information received from physician on 20 Jul 2011: Added patient history, added suspected drug, concomitant and treatment drug, added events, added causality.

NOVARTIS, 27.07.2011 DE-002147023-PHHY2011DE46256

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE46256, is a combined initial and follow-up spontaneous report received from consumer (patient's wife) on 27 May 2011 with follow up report (written information) received from a consumer (patient's wife) via Novartis Consumer Health on 08 Jun 2011 with follow up physician report received on 20 Jul 2011. This report refers to a 65-years-old male patient. The patient had history of myocardial infarction about 11 years ago, tumor resection at thoracic vertebral column, stent implantation (ramus-interventricularis-anterior), coronary heart disease with stenosis of ramus-interventricularis- anterior, diffuse myocardial fibrosis predominantly at the right half, myocardial hypertrophy, cardiac insufficiency with vascular congestion in lung, liver, spleen and kidney, arterial hypertension, high-grade left-sided kyphoscoliosis of thorax, osteophytes in thoracic and lumbar region, renal cyst (left-side, diameter 2 cm), focal anthracosis with bronchiectasis, wheals of lung apex and mild nodular prostatic hyperplasia. This poly-medicated patient received Voltaren Resinat (diclofenac resinat) dose in morning for pain of cervical and thoracic vertebral column and shoulder pain from unspecified date to 10 Nov 2007. The patient was admitted at hospital on 11 Nov 2007 due to severe pain of cervical and thoracic vertebral

column. According to his reporting wife, he had severe stomach pain before he was hospitalized in the intensive care unit. In hospital the patient received Voltaren (diclofenac sodium) tablet once at 5.30 pm at a dose of 100 mg. Patient received catecholamines as a treatment drug. Patient died on 11 Nov 2007 due to cardiac arrest caused by sepsis. The patient also experienced septic shock, ulcer of front wall of antrum, perforation of front wall of antrum (2cm diameter), purulent peritonitis, and purulent ascites and had air in abdomen. The physician specified that the antrum perforation likely had been present before administration of 100 mg Voltaren in hospital. The physician stated the surgery was planned but not performed. Reporter's mother was prescribed topical Voltaren (See the case CHPA2011DE07309). Causality was assessed as suspected.

Follow up report (written information) received from a consumer (patient's wife) via Novartis Consumer Health on 08 Jun 2011. Updated medical history myocardial infarction, therapy start date for Voltaren and consumer's comment.

Follow up information received from physician on 20 Jul 2011: Added patient history, added suspected drug, concomitant and treatment drug, added events, added causality.

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-11108410

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19440301	Age: 66 Year	weight (kg): 88	BMI_int: 27.46	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Wound bleeding	20100923			3 Day	Unknown
2 Haemoglobin decreased					recovered/resolved
3 Haematoma	20100928			8 Day	Unknown
4 Haematocrit decreased					recovered/resolved
5 Wound secretion	20100923	20101025	33 Day	4 Day	recovered/resolved
6 Creatinine clearance decreased					Unknown
7 C-reactive protein increased					recovered/resolved
8 Alanine aminotransferase increase					Unknown
9 Activated partial thromboplastin time increased					Unknown
10 Gamma-glutamyltransferase increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DABIGATRAN ETEXILATE	Oral	220 Mg milligram(s)/Day	20100920-20101104	Thromboembolism prophylaxis
dabigatran etexilate		5	46 Day	
2 voltaren resinat	Oral	2*75 Mg milligram(s)/ Day	20100921-20101016	Ache
diclofenac resinate		6	26 Day	

Drugs (concomitant)

3 bifiteral sirup	Oral	/	20100921-20101016	Obstipation
lactulose		244	26 Day	
4 cefuroxim 1.5g	Intravenous (not otherwis	4.5 G gram(s)/Day	20100920-20100920	Infection prophylaxis
cefuroxime sodium		120	1 Day	
5 dipidolor	Subcutaneous	3*3 Df dosage form/	20100921-20101016	Ache
piritramide		120	26 Day	
6 oxygesic akut	Oral	/	20100921-20101016	Ache
		6	26 Day	
7 oxygesic 20 mg	Oral	40 Mg milligram(s)/Day	20100922-20101009	Ache
		212	18 Day	
8 pantozol 40 mg	Oral	40 Mg milligram(s)/Day	20100921-20101016	Gastric ulcer prophylaxis
pantoprazole		94	26 Day	
9 paracetamol 500mg	Oral	3000 Mg milligram(s)/Day	20100921-20101009	Ache
paracetamol		245	19 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Smoker			Yes	Beginn unbekannt
Coxarthrosis			No	Beginn unbekannt, Hüft-TEP links 04/2010
Hip total replacement	201004		Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Creatinine	20100917	76.9	µmol/l
Creatinine	20100921	72.7	µmol/l
Creatinine	20100927	72.7	µmol/l
Creatinine	20100930	74.3	µmol/l
Creatinine	20101011	70.6	µmol/l
Creatinine	20101019	69	µmol/l
Creatinine clearance	20100917	1	ml/s
Creatinine clearance	20100921	1	ml/s
Creatinine clearance	20100927	1	ml/s
Creatinine clearance	20100930	1	ml/s
Creatinine clearance	20101011	1	ml/s
Urea	20100917	3.40	mmol/l
Urea	20100921	4.75	mmol/l
Urea	20100927	6.06	mmol/l
Urea	20100930	3.74	mmol/l
Urea	20101011	3.68	mmol/l
Aspartate aminotransferase	20100917	17.5	U/l
Aspartate aminotransferase	20100930	42.9	U/l
Aspartate aminotransferase	20101004	22.5	U/l
Aspartate aminotransferase	20101011	17.2	U/l
Aspartate aminotransferase	20101019	0.31	µkat/l
Alanine aminotransferase	20100917	19.5	U/l
Alanine aminotransferase	20100930	62.4	U/l
Alanine aminotransferase	20101004	37.3	U/l
Alanine aminotransferase	20101011	22.5	U/l
Alanine aminotransferase	20101019	0.28	µkat/l
Bilirubin total	20100917	16.2	µmol/l
Alkaline phosphatase	20100917	119	U/l
Gamma GT	20100917	41.0	U/l
Gamma GT	20100930	85.0	U/l
Gamma GT	20101011	54.0	U/l
Gamma GT	20101019	0.76	µkat/l
Quick's test	20100917	108	Prozent
Quick's test	20100927	109	Prozent
Quick's test	20100930	104	Prozent
Quick's test	20101004	98	Prozent
Quick's test	20101011	92	Prozent
Partial prothrombin time	20100917	30.8	sec
Partial prothrombin time	20100927	35.7	sec
Partial prothrombin time	20100930	31.8	sec
Partial prothrombin time	20101004	37.4	sec
Partial prothrombin time	20101011	37.7	sec
INR	20100917	0.95	Physiologis
INR	20100927	0.95	Physiologis

INR	20100930	0.97	Physiologis
INR	20101004	1.02	Physiologis
INR	20101011	1.05	Physiologis
Thrombocyte count	20100917	183	Gpt/l
Thrombocyte count	20100920	170	Gpt/l
Thrombocyte count	20100921	166	Gpt/l
Thrombocyte count	20100927	282	Gpt/l
Thrombocyte count	20100930	428	Gpt/l
Thrombocyte count	20101004	415	Gpt/l
Thrombocyte count	20101011	349	Gpt/l
Thrombocyte count	20101019	296	10E6/ μ l
Thrombocyte count	20101104	232	10E6/ μ l
LDH	20100917	159	U/l
Haemoglobin	20100917	8.4	mmol/l
Haemoglobin	20100920	6.5	mmol/l
Haemoglobin	20100921	6.3	mmol/l
Haemoglobin	20100927	5.2	mmol/l
Haemoglobin	20100930	5.7	mmol/l
Haemoglobin	20101004	6.2	mmol/l
Haemoglobin	20101011	6.7	mmol/l
Haemoglobin	20101019	7.5	mmol/l
Haemoglobin	20101104	9.0	mmol/l
Haematocrit	20100917	0.39	SI-Einheit
Haematocrit	20100920	0.31	SI-Einheit
Haematocrit	20100921	0.30	SI-Einheit
Haematocrit	20100927	0.26	SI-Einheit
Haematocrit	20100930	0.28	SI-Einheit
Haematocrit	20101004	0.31	SI-Einheit
Haematocrit	20101011	0.34	SI-Einheit
Haematocrit	20101019	0.35	SI-Einheit
Haematocrit	20101104	0.436	SI-Einheit
Total white blood cell count	20100917	5.28	Gpt/l
Total white blood cell count	20100920	8.66	Gpt/l
Total white blood cell count	20100921	7.64	Gpt/l
Total white blood cell count	20100927	5.25	Gpt/l
Total white blood cell count	20100930	6.29	Gpt/l
Total white blood cell count	20101004	5.78	Gpt/l
Total white blood cell count	20101011	4.88	Gpt/l
Total white blood cell count	20101019	5.0	10E3/ μ l
Total white blood cell count	20101104	6.3	10E3/ μ l
C-reactive protein	20100917	3.03	mg/l
C-reactive protein	20100927	116	mg/l
C-reactive protein	20100930	55.4	mg/l
C-reactive protein	20101004	27.4	mg/l

C-reactive protein	20101011	8.64	mg/l
C-reactive protein	20101019	3.9	mg/l
C-reactive protein	20101104	4.1	mg/l
Amylase	20100917	65.0	U/l
Lipase	20100917	34.3	U/l
Glucose	20100917	5.9	mmol/l
Potassium	20100917	4.3	mmol/l
Potassium	20100921	4.0	mmol/l
Potassium	20100927	3.7	mmol/l
Potassium	20101011	4.0	mmol/l
Sodium	20100917	141	mmol/l
Sodium	20100921	134	mmol/l
Sodium	20100927	141	mmol/l
Sodium	20101004	140	mmol/l
Calcium	20100917	2.34	mmol/l
Alanine aminotransferase	20100930	62.5	NA

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 09.06.2011 DE-PVZIBE-02-10341

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Wundsekretion

KURZZUSAMMENFASSUNG:

verlängerte blutig-seröse Wundsekretion unter Pradaxa (Diclofenac interagierend)

BESCHREIBUNG:

stationärer Krankenhausaufenthalt: 19.09.-16.10.2010;

20.09.2010 Hüftgelenkersatz-Operation bei Coxarthrose rechts (OP-Dauer 1,75 Stunden);

Thromboembolieprophylaxe mit Pradaxa ab 20.09.2010 19:00 Uhr;

ab 23.09.2010 blutig-seröse Wundsekretion;

28.09.2010 weiter Wundsekretion und handgroßes Hämatom;

wegen anhaltender blutig-seröser Wundsekretion wurde die Anschlußheilbehandlung, geplant ab 04.10.2010, verschoben

THERAPIE:

03.-30.10.2010 Cefuroxim 500mg 2x1;

05.10.2010 Wundrevisions-Operation und Lavage wegen persistierender Sekretion

VERLAUF:

11.10.2010 Wundabstrich: Staphylococcus capitis (Cefuroxim sensibel);

16.10.-05.11.2010 Anschlußheilbehandlung;

ab 25.10.2010 keine Sekretion mehr

ERLÄUTERUNGEN ZUR UAW:

#stationär aufgetretene UAW#

Nach elektivem Hüftgelenkersatz trat eine verlängerte blutig-seröse Wundsekretion auf, welche am 14. postoperativen Tag zu einer operativen Wundrevision führte. Diese Komplikation verlängerte den Krankenhausaufenthalt. Es ist möglich, dass die antikoagulatorische Wirkung von Pradaxa (und Diclofenac interagierend) die anhaltende blutig-seröse Wundsekretion mit unterhalten hat.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: pradaxa

BETREFFENDE UAW: Wundsekretion

ERLÄUTERUNG: Es ist möglich, dass die gerinnungshemmende Wirkung von Pradaxa die verlängerte blutig-seröse Wundsekretion unterstützt hat.

MEDIKAMENT: voltaren resinat

BETREFFENDE UAW: Wundsekretion

ERLÄUTERUNG: Diclofenac hemmt die ADP- und kollageninduzierte Plättchenaggregation. Es ist möglich, dass Diclofenac die verlängerte blutigeröse Wundsekretion mit unterstützt hat.<dbtext>To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 21.06.2011 DE-Boehringer Ingelheim GmbH, Germany-2011-DE-03558DE

Sender diagnosis: not reported

Narrative

Initial information was received from the German Health Authority concerning a 66-year-old male patient, who was on Pradaxa (dabigatran etexilate), hardgelatin capsules, orally, 110mg daily, only on the 20Sep2010 and 2x110 mg daily, between 21Sep2010 and 04Nov2010 for the treatment of thromboembolism prophylaxis and Voltaren Resinat (diclofenac resinate), hardgelatin capsule, 2x75mg orally daily, between 21Sep2010 and 16Oct2010 for the treatment of ache.

Between 23Sep2010 and 25Oct2010, the patient experienced wound secretion. The event was considered to be serious due to prolonged hospitalisation.

The patient fully recovered.

The patient is a smoker.

Concomitant medication(s) included Pantozol 40mg (pantoprazole sodium sesquihydrate), gastric juice resistant tablets, 40mg orally daily, between 21Sep2010 and 16Oct2010, for the treatment of gastric ulcer prophylaxis; Cefuroxim 1.5g (cefuroxime sodium), suspension for injection, 3x1.5g daily intravenous, on the 20Sep2010, for the treatment of infection prophylaxis; Oxygesic Akut (oxycodone hydrochloride), hardgelatin capsule, intake if required, between 21Sep2010 and 16Oct2010, for the treatment of ache; Oxygesic (oxycodone hydrochloride), long acting tablets, 2x20 mg orally daily, between 22Sep2010 and 09Oct2010 and 2x10mg orally daily, between 09Oct2010 and 16Oct2010, for the treatment of ache; Paracetamol 500mg, tablets, 3x1000mg orally daily, between 21Sep2010 and 09Oct2010, for the treatment of ache; Dipidolor (piritramide), suspension for injection, 3times daily subcutaneous, between 21Sep2010 and 16Oct2010, for the treatment of ache; Bifiteral Sirup (lactulose), syrup, if required 1 tablespoon, between 21Sep2010 and 16Oct2010 for the treatment of obstipation.

Further concomitant diseases included Coxarthrosis, beginning unknown
Past diseases included Hip-TEP left 04/2010.

The reporter rated the causal relationship with Pradaxa and Voltaren Resinat as possible.

Reporter remarks to the event:
wound secretion

Executive Summary:
prolonged bloody-serous wound secretion under Pradaxa (Diclofenac interacting)

Description:
stationary hospitalization: 19.09.-16.10.2010
20.09.2010 Hip replacement operation in coxarthrosis right (OP-life 1.75 hours)
thromboembolism prophylaxis with Pradaxa since 20.09.2010.
Since 23.09.2010 bloody-serous wound secretion;
28.09.2010 further wound secretion and hand large hematoma;
because of persistent bloody-serous wound secretion was the follow-up treatment, planned from 04.10.2010, moved

Therapy:
03.-30.10.2010 Cefuroxim 500 mg 2x1;
05.10.2010 wound revision operation and lavage due to persistent secretion

History:
11.10.2010 swab: Staphylococcus capitis (cefuroxime sensitive)
16.10.-05.11.2010 Follow-up treatment
since 25.10.2010 no more secretion

Remarks to the event:
#event has occurred stationary#
After elective hip joint replacement occurred a prolonged bloody-serous secretion which resulted after the 14 postoperativen day at a surgical wound revision.
This complication prolonged the hospital stay.
It is possible, that the anticoagulant effect of Pradaxa (and interacting diclofenac) maintain the ongoing bloody-serous wound secretion

Explanations of the causalities:
Drug: Pradaxa
Event: wound secretion
Remarks: It is possible that supported the anti-clotting effect of Pradaxa prolonged the bloody-serous wound secretion

Drug: Voltaren Resinat

Event: wound secretion

Remarks: Diclofenac inhibits the ADP-and collagen-induced platelet aggregation. It is possible that diclofenac has supported the prolonged bloody-serous wound secretion.

Tracking of Changes:

14Jun2011 - initial information received

NOVARTIS, 24.06.2011 DE-002147023-PHHY2011DE52474

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE52474, is an initial PMS report from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-11108410) on 10 Jun 2011. This case refers to a 66-year-old male. The patients medical history included coxarthrosis, and smoking. Concomitant medications included cefuroxime, pantoprazole, oxagesic, oxagesic akut, paracetamol, dipidolor and bifiteral syrup. Lab results showed haemoglobin at 8.4 mmol/l and haematocrit at 0.39 on 17 Sep 2010. On 20 Sep 2010, the patient had endoprosthetic hip replacement surgery due to coxarthrosis on right side. The patient started treatment with Pradaxa (dabigatran etexilate) capsule for thromboembolism prophylaxis from 20 Sep 2010 at a dose of 110 mg, BID. The patient also received Voltaren resinat (diclofenac resinate) at for ache from 21 Sep 2010 at 2 DF daily. On 23 Sep 2010, the patient presented with bloody serious wound secretion. On 27 Sep 2010, haemoglobin was at 8.5.2 mmol/l, haematocrit at 0.26 C-reactive protein at 116 mg/l. The patient had hand sized haematoma on 28 Sep 2010 and the wound secretion was persistent, which resulted in a surgical revision of the wound on the 14th day after the surgery. The lab data showed alanine aminotransferase at 62.4 UL, gamma-glutamyl transferase at 85 U/L on 30 Sep 2010. This complication extended the hospitalization of the patient. The lab data showed partial prothrombin time at 37.4 sec on 04 Oct 2010 and 37.7 sec on 11 Oct 2010. The creatinine clearance was at 69 ml/s on 19 Oct 2010. The reporter stated that diclofenac inhibit the ADP collagen- induced aggregation. The outcome of the events was not reported. It was stated that it was possible that diclofenac and pradaxa facilitated the extended blood serious wound secretion. Reporting healthcare professional causality is suspected.

NOVARTIS, 27.06.2011 DE-002147023-PHHY2011DE52474

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE52474, is an initial PMS report from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-11108410) on 10 Jun 2011. This case refers to a 66-year-old male. The patients medical history included coxarthrosis, and smoking. Concomitant medications included cefuroxime, pantoprazole, oxagesic, oxagesic akut, paracetamol, dipidolor and bifiteral syrup. Lab results showed haemoglobin at 8.4 mmol/l and haematocrit at 0.39 on 17 Sep 2010. On 20 Sep 2010, the patient had endoprosthetic hip replacement surgery due to coxarthrosis on right side. The patient started treatment with Pradaxa (dabigatran etexilate) capsule for thromboembolism prophylaxis from 20 Sep 2010 at a dose of 110 mg, BID. The patient also received Voltaren resinat (diclofenac resinate) at for ache from 21 Sep 2010 at 2 DF daily. On 23 Sep 2010, the patient presented with bloody serious wound secretion. On 27 Sep 2010, haemoglobin was at 8.5.2 mmol/l, haematocrit at 0.26 C-reactive protein at 116 mg/l. The patient had hand sized haematoma on 28 Sep 2010 and the wound secretion was persistent, which resulted in a surgical revision of the wound on the 14th day after the surgery. The lab data showed alanine aminotransferase at 62.4 UL, gamma-glutamyl transferase at 85 U/L on 30 Sep 2010. This complication extended the hospitalization of the patient. The lab data showed partial prothrombin time at 37.4 sec on 04 Oct 2010 and 37.7 sec on 11 Oct 2010. The creatinine clearance was at 69 ml/s on 19 Oct 2010. The reporter stated that diclofenac inhibit the ADP collagen- induced aggregation. The outcome of the events was not reported. It was stated that it was possible that diclofenac and pradaxa facilitated the extended blood serious wound secretion. Reporting healthcare professional causality is suspected.

NOVARTIS, 27.06.2011 DE-002147023-PHHY2011DE52474

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE52474, is an initial PMS report from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-11108410) on 10 Jun 2011. This case refers to a 66-year-old male. The patients medical history included coxarthrosis, and smoking. Concomitant medications included cefuroxime, pantoprazole, oxagesic, oxagesic akut, paracetamol, dipidolor and bifiteral syrup. Lab results

showed haemoglobin at 8.4 mmol/l and haematocrit at 0.39 on 17 Sep 2010. On 20 Sep 2010, the patient had endoprosthetic hip replacement surgery due to coxarthrosis on right side. The patient started treatment with Pradaxa (dabigatran etexilate) capsule for thromboembolism prophylaxis from 20 Sep 2010 at a dose of 110 mg, BID. The patient also received Voltaren resinat (diclofenac resinate) at for ache from 21 Sep 2010 at 2 DF daily. On 23 Sep 2010, the patient presented with bloody serious wound secretion. On 27 Sep 2010, haemoglobin was at 8.5.2 mmol/l, haematocrit at 0.26 C-reactive protein at 116 mg/l. The patient had hand sized haematoma on 28 Sep 2010 and the wound secretion was persistent, which resulted in a surgical revision of the wound on the 14th day after the surgery. The lab data showed alanine aminotransferase at 62.4 UL, gamma-glutamyl transferase at 85 U/L on 30 Sep 2010. This complication extended the hospitalization of the patient. The lab data showed partial prothrombin time at 37.4 sec on 04 Oct 2010 and 37.7 sec on 11 Oct 2010. The creatinine clearance was at 69 ml/s on 19 Oct 2010. The reporter stated that diclofenac inhibit the ADP collagen- induced aggregation. The outcome of the events was not reported. It was stated that it was possible that diclofenac and pradaxa facilitated the extended blood serious wound secretion. Reporting healthcare professional causality is suspected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11133624

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Female	Birth:	Age: 45 Year	weight (kg): 98	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Quick's value decreased					recovered/resolved
2 Intentional overdose	20110619				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	1000 Mg milligram(s)/ 245	20110619- -	Suicide attempt
diclofenac				
2 MARCUMAR	Oral	225 Mg milligram(s)/ 245	20110619- -	Suicide attempt
phenprocoumon				

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Quick's test		43	%	

Narratives (nur bei elektronischem Eingang)

MEDA Pharma GmbH & Co. KG, 12.07.2011 DE-MEDA-M-EU-2011070061

Sender diagnosis: not reported

Narrative

A report was created by the GIFTINFORMATIONSZENTRALE (German Drug and Poison Information Call Center, reference no. AZ 2011013459) on 05-Jul-2011, presenting a valid case for MEDA.

On 19-Jun-2011 a hospital physician called and asked for advice. The physician reported about a 45-year-old female patient (weight: 98 kg) who ingested 75 tablets of Marcumar (phenprocoumon - total dose of 225 mg) and 20 tablets of Diclofenac 50 (diclofenac - total dose of 1000 mg) in suicidal/parasuicidal intention seven hours prior to the call. Quick's test was carried out and revealed Quick's value of 43 %. The Drug and Poison Information Call Center recommended the following diagnostic procedures: coagulation control, renal value control, activated charcoal, vitamin k administration, cortison if required, gastric protection. The patient was treated in intensive care unit. Monitoring and symptomatic therapy were carried out. On 05-Jul-2011 the patient had fully recovered.

marcumar is assessed as probable. Regarding the event term intentional overdose the causality is not assessed.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11139605

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19440917	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20110125			8 Day	not recovered/not resolved
2 Jaundice	20110125			8 Day	not recovered/not resolved
3 Liver injury	20110125			8 Day	not recovered/not resolved
4 Scleral icterus	20110125				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	50 Mg milligram(s)/ 245	-	Knee pain
diclofenac			-	
2 ibuprofen 400 mg	Oral	400 Mg milligram(s)/Day 245	20110118-20110118	Knee pain
ibuprofen			1 Day	

Drugs (concomitant)

3 ENALAPRIL	/	-
enalapril		-
4 Hydrochlorothiazid	/	-
hydrochlorothiazide		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Unknown	hypertension arterial

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase		15.69	µmol/s*l	
Alanine aminotransferase		25.53	µmol/s*l	
Bilirubin		117	µmol/l	
Alkaline phosphatase		3.29	NA	
Gamma GT		6.16	NA	

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 25.07.2011 DE-BAYER-2011-063717

Sender diagnosis: not reported

Narrative

Serious, Unlisted, Related

This initial spontaneous case was reported by a health professional in Germany via the Drug Commission of the German Medical Association and was identified on the internet page of the health authority (BFARM), was received on 20-JUL-2011 and refers to a 66 year old female patient who received IBUPROFEN and experienced jaundice and hepatic enzymes increased.

The patient's concurrent condition included hypertension arterial.

It is unknown whether the patient received any concomitant medication.

On 18-Jan-2011 the patient took IBUPROFEN at 400 mg, QD for knee pain.

On 25-JAN-2011 the patient experienced jaundice, hepatic enzymes increased and therefore the patient was hospitalized. IBUPROFEN was withdrawn. The outcome of jaundice and hepatic enzymes increased were not resolved.

Reporter causality:

relationship between jaundice and IBUPROFEN related

relationship between hepatic enzymes increased and IBUPROFEN related.

sanofi-aventis, 27.07.2011 DE-SANOFI-AVENTIS-2011SA046657

Sender diagnosis: not reported

Narrative

Initial report:

On 21-Jul-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11139605); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association about a 66-year-old female patient.

Concomitantly, the patient suffered from arterial hypertension.

On 18-Jan-2011 the patient took one tablet ibuprofen 400 mg (trade name not provided) for knee pain. On 25-Jan-2011 jaundice and increased hepatic enzymes were detected leading to hospitalization. Following lab values (without reference ranges) were revealed at unspecified time point: aspartate aminotransferase 15.69 mcmol/s*l, alanine aminotransferase 25.53 mcmol/s*l and bilirubin 117 mcmol/l as well as alkaline phosphatase 3.29 and gamma GT 6.16 (units were not reported). Corrective therapy was not mentioned; the outcome was classified with not recovered.

Causality by Drug Commission: possible

Reporter's comment:

sanofi-aventis, 26.09.2011 DE-SANOFI-AVENTIS-2011SA046657

Sender diagnosis: not reported

Narrative

Initial report:

On 21-Jul-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11139605); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association about a 66-year-old female patient.

Concomitantly, the patient suffered from arterial hypertension.

The initial report concerned intake of ibuprofen 400 mg for knee pain on 18-Jan-2011. The originally document included only diclofenac (trade name not provided) 50 mg, which was taken for knee pain at four unspecified time points. Ibuprofen was not mentioned.

On 25-Jan-2011 the jaundice of skin, scleral icterus and increased hepatic enzymes were diagnosed. Following lab values (without reference ranges) were revealed at unspecified time point: aspartate aminotransferase 15.69 mcmol/s*l, alanine aminotransferase 25.53 mcmol/s*l and bilirubin 117.3 mcmol/l as well as alkaline phosphatase 3.29 and gamma GT 6.16 (units were not reported).

Subsequently the patient was hospitalised and corrective treatment with enalapril and hydrochlorothiazide was started. At date of report the patient had not yet recovered.

Causality by Drug Commission: possible

Additional information received on 20-Sep-2011:

The originally document for the initial data collection was provided by the Drug Commission of the German Medical Association.

Patients birth date and initials were amended. The suspected product was changed and the events were adjusted. Corrective therapy was amended.

Bayer Vital GmbH, 27.09.2011 DE-BAYER-2011-063717

Sender diagnosis: not reported

Narrative

Serious, Unlisted, Related

This initial spontaneous case was reported by a health professional in Germany via the Drug Commission of the German Medical Association and was identified on the internet page of the health authority (BFARM), was received on 20-JUL-2011 and refers to a 66 year old female patient who received IBUPROFEN and experienced jaundice and hepatic enzymes increased.

The patient's concurrent condition included hypertension arterial.
It is unknown whether the patient received any concomitant medication.

On 18-Jan-2011 the patient took IBUPROFEN at 400 mg, QD for knee pain.
On 25-JAN-2011 the patient experienced jaundice, hepatic enzymes increased and therefore the patient was hospitalized.
IBUPROFEN was withdrawn. The outcome of jaundice and hepatic enzymes increased were not resolved.

Reporter causality:
relationship between jaundice and IBUPROFEN related
relationship between hepatic enzymes increased and IBUPROFEN related.

Follow-up information was received on 16-sep-2011:
Patient information was updated.
Concomitant drugs were added
Lab data at time of admission updated: bilirubin -d 108, 9 (units not recorded).

Suspected drug was amended from Ibuprofen to Diclofenac.
The reporter has not provided assessment.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11201694

Serious Yes	Death No	Lifethr. Yes	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 54 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blood cholesterol increased					Unknown
2 Blood alkaline phosphatase increased					Unknown
3 HDL cholesterol increased					Unknown
4 Oral intake reduced	20110919	20111018	30 Day		recovered/resolved
5 Blood lactate dehydrogenase increased					Unknown
6 C-reactive protein increased					Unknown
7 Glomerular filtration rate decreased					Unknown
8 Hepatic enzymes increased	20110919	20111018	30 Day		recovered/resolved
9 Hepatitis drug-induced	20110919	20111018	30 Day		recovered/resolved
10 Low density lipoprotein increased					Unknown
11 Monocyte count increased					Unknown
12 Nausea	20110919	20111018	30 Day		recovered/resolved
13 Vomiting	20110919	20111018	30 Day		recovered/resolved
14 Weakness	20110919	20111018	30 Day		recovered/resolved
15 Weight decrease	20110919	20111018	30 Day		recovered/resolved
16 General physical health deterioration	20110919	20111018	30 Day		recovered/resolved
17 Incorrect dose administered	201109				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	150 Mg milligram(s)/Day 5	- -	Sciatica
2 voltaren resinat diclofenac resinate	Oral	75 Mg milligram(s)/Day 5	20110905-20110914 10 Day	Back pain

Drugs (concomitant)

3 estradiol estradiol	Transdermal	37.5 µg microgram(s)/	- -	HRT
4 PANTOPRAZOLE pantoprazole	Oral	20 Mg milligram(s)/	20110905-20110914 10 Day	Prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Herniated disc	2007		Unknown	From 2007, herniated disc (leading to sciatic pain).

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GOT increased		About 550	NA	
Gamma-GT increased		About 200	NA	
Alkaline phosphatase	20110919	249 U/l	NA	

Aspartate aminotransferase	20110926	48 U/l	NA
Aspartate aminotransferase	20111005	36 U/l	NA
Aspartate aminotransferase	20110929	55 U/l	NA
Aspartate aminotransferase		About 550	NA
Aspartate aminotransferase	20110919	524 U/l	NA
Aspartate aminotransferase	20110923	71 U/l	NA
Cholesterol	20110929	238 mg/dl	NA
Cholesterol	20110919	201 mg/dl	NA
Cholesterol	20111005	268 mg/dl	NA
Cholesterol	20111018	247 mg/dl	NA
C-reactive protein	20110919	0.8 mg/dl	NA
Gamma-glutamyltransferase	20110926	127 U/l	NA
Gamma-glutamyltransferase		About 200	NA
Gamma-glutamyltransferase	20110919	194 U/l	NA
Gamma-glutamyltransferase	20110923	156 U/l	NA
Gamma-glutamyltransferase	20110929	103 U/l	NA
Gamma-glutamyltransferase	20111005	81 U/l	NA
Gamma-glutamyltransferase	20111018	43 U/l	NA
Glomerular filtration rate	20110919	79 ml/min	NA
Glomerular filtration rate	20111005	69	NA
Glomerular filtration rate	20111018	79	NA
GPT		About 700	NA
GPT	20111005	63 U/l	NA
GPT	20110929	121 U/l	NA
GPT	20110926	146 U/l	NA
GPT	20110923	316 U/l	NA
GPT	20110919	679 U/l	NA
HDL cholesterol increased	20111005	64 mg/dl	NA
HDL cholesterol increased	20111018	64 mg/dl	NA
HDL cholesterol increased	20110929	58 mg/dl	NA
LDH	20110919	396 U/l	NA
LDL	20110929	168 mg/dl	NA
LDL	20111005	191mg/dl	NA
LDL	20111018	169 mg/dl	NA
Monocytes	20110919	11	%

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.10.2011 DE-002147023-PHHY2011DE88648

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE88648, is an initial spontaneous report received from consumer (patient) and physician via phone call on 06 Oct 2011. This report refers to a 54-years-old female patient. The patient's medical history included discomfort (NOS) during Voltaren resinat (diclofenac resinate) treatment in 2010. The patient received Voltaren resinat (diclofenac resinate) 75 mg, in Sep 2011 for the treatment of back pain. The dose was usually on one capsule a day however at one not further specified day the patient took 2 capsules. In Sep 2011, sometimes during the course the patient developed elevated liver enzymes (GPT about 700, GOT about 550, gamma-GT about 200), nausea, vomiting, weakness and weight decrease. The patient was hospitalized due to the events from 19 Sep 2011 to 22 Sep 2011. In hospital, hepatitis tests and anti-nuclear antibodies performed were negative. The patient completely recovered from nausea and vomiting. Elevated liver enzymes and weight decrease had been

improved. The event weakness was reported as still ongoing. Liver values were slightly above upper limit of normal (no values provided) at the time of reporting. The patient's physician suspected a causal relationship to Voltaren resinat as other causes could be ruled out and because of the temporal relationship (certainly related).

Hepatic enzyme increased, nausea, vomiting [hospitalization], assessed as listed in the Core Data Sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Medical comment: Based on review of available reported data the causal relationship cannot be ruled out.

Reporter's comment:

NOVARTIS, 02.11.2011 DE-002147023-PHHY2011DE88648

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE88648, is an initial spontaneous report received from consumer (patient) and physician via phone call on 06 Oct 2011, with follow up information received from physician on 28 Oct 2011. This report refers to a 54-years-old female patient. The patient's medical history included discomfort (NOS) during Voltaren Resinat (diclofenac resinate) treatment in 2010 and herniated disc (leading to sciatic pain). Concomitant medication included pantoprazole from 05 Sep 2011 to 14 Sep 2011 at 20 mg dose for stomach protection during use of NSAID and estradiol patches at 37.5 mcg dose for hormone replacement therapy. The patient received Voltaren Resinat (diclofenac resinate) at 75 mg dose for the treatment of back pain and for sciatic pain due to herniated disc from 05 Sep 2011 to 14 Sep 2011. The dose was usually on one capsule a day however at one not further specified day the patient took 2 capsules. On 19 Sep 2011, the patient developed acute drug induced hepatitis, the patient was not able to eat and drink, patient was in a poor general condition and also the patient experienced elevated liver enzymes, nausea, vomiting, weakness and weight decrease. On 19 Sep 2011, the patient's lab data showed, blood alkaline phosphatase at 249 U/l, C-reactive protein at 0.8 mg/dl, glomerular filtration rate at 79 ml/min, gamma-glutamyltransferase at 194 U/l, alanine aminotransferase at 679 U/l, aspartate aminotransferase 524 U/l, blood lactate dehydrogenase at 396 U/l, monocyte count at 11%, blood cholesterol at 201 mg/dl. The patient was hospitalized due to the events from 19 Sep 2011 to 22 Sep 2011. In hospital, hepatitis tests and anti-nuclear antibodies performed were negative. On 23 Sep 2011, the patients lab data showed gamma-glutamyltransferase at 156 U/l, alanine aminotransferase at 316 U/l, aspartate aminotransferase 71 U/l. On 26 Sep 2011, the patients lab data showed, gamma-glutamyltransferase at 127 U/l, alanine aminotransferase at 146 U/l, aspartate aminotransferase 48 U/l. On 29 Sep 2011, the patients lab data showed, aspartate aminotransferase at 55, alanine aminotransferase at 121 U/l, glomerular filtration rate at 93 ml/min, gamma-glutamyltransferase at 103 U/l, blood glucose at 102 mg/dl, HDL cholesterol at 58 mg/dl, low density lipoprotein at 168 mg/dl, blood cholesterol at 238 mg/dl. On 05 Oct 2011, the patients lab data showed aspartate aminotransferase at 36, alanine aminotransferase at 63, glomerular filtration rate at 69 ml/min, gamma-glutamyltransferase at 81 U/l, HDL cholesterol at 64 mg/dl, low density lipoprotein at 191 mg/dl, low density lipoprotein at 169 mg/dl, blood cholesterol at 268 mg/dl. On 18 Oct 2011, the patients lab data showed, glomerular filtration rate at 79 ml/min, gamma-glutamyltransferase at 43 U/l, HDL cholesterol at 64 mg/dl, blood cholesterol at 247 mg/dl. The seriousness for the events was assessed as life-threatening and the causality was assessed as certain (suspected), outcome was specified as recovered on 18 Oct 2011, the events improved after discontinuation of Voltaren Resinat.

Follow up information received from physician on 28 Oct 2011: Updated patients history, lab data, concomitant medication, suspect information (indication, start, stop date and action taken), events.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11209748

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg): 65	BMI_int: 24.77	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt					Unknown
2 Quick's value decreased					Unknown
3 Gastrointestinal bleeding					Unknown
4 Vomiting blood					Unknown
5 APTT prolonged					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	
2 PRADAXA		110 Mg milligram(s)/	-	
dabigatran etexilate		5	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
APTT		128	sec	
Quick's value decreased		6	Percent	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 28.10.2011 DE-Boehringer Ingelheim GmbH, Germany-2011-DE-06060DE

Sender diagnosis: not reported

Narrative

Initial information has been received from a hospital physician from Germany via phone.

A female patient (born 1928) took 1x8 Pradaxa 110 mg (dabigatran) and diclofenac in suicidal intention and experienced gastrointestinal bleeding, Quick decreased to 6%, a PTT prolonged to 128 sec and she vomited haematin.

The events were considered to be serious due to hospitalisation.

There is no information available on concomitant medication, concomitant diseases and outcome of event.

Tracking

25Oct2011: initial via phone

SUICIDE ATTEMPT is an unlisted adverse drug reaction for Pradaxa. It has not been reported, if the patient took Pradaxa before or used the medication of someone else to try to commit suicide. The level of detail in the report does not permit a coherent assessment of the case. The events GASTROINTESTINAL HAEMORRHAGE and HAEMATEMESIS are listed for Pradaxa, but may as well be attributed to the co-suspect drug diclofenac. The events QUICK DECREASED and APTT PROLONGED are formally unlisted, but may be explained by mode of action of Pradaxa. Based on the information provided, no change to the company core safety information or other measures are recommended at this time.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 03.11.2011 DE-Boehringer Ingelheim GmbH, Germany-2011-DE-06060DE

Sender diagnosis: not reported

Narrative

Initial information has been received from a hospital physician from Germany via phone.

A female patient (born 1928) took 1x8 Pradaxa 110 mg (dabigatran) and diclofenac in suicidal intention and experienced gastrointestinal bleeding, Quick decreased to 6%, a PTT prolonged to 128 sec and she vomited haematin.

The events were considered to be serious due to hospitalisation.

There is no information available on concomitant medication, concomitant diseases and outcome of event.

Tracking

25Oct2011: initial via phone

26Oct2011: Internal update: Reporter adress added

medication of someone else to try to commit suicide. The level of detail in the report does not permit a coherent assessment of the case.

The events GASTROINTESTINAL HAEMORRHAGE and VOMITED HAEMATIN (HAEMATEMESIS) are listed for Pradaxa, but may as well be attributed to the co-suspect drug diclofenac. The events QUICK DECREASED and APTT PROLONGED are formally unlisted, but may be explained by mode of action of Pradaxa.

Based on the information provided, no change to the company core safety information or other measures are recommended at this time.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 11.11.2011 DE-Boehringer Ingelheim GmbH, Germany-2011-DE-06060DE

Sender diagnosis: not reported

Narrative

Initial information has been received from a hospital physician from Germany via phone.

A female patient (born 1928) took 1x8 Pradaxa 110 mg (dabigatran) and diclofenac in suicidal intention and experienced gastrointestinal bleeding, Quick decreased to 6%, a PTT prolonged to 128 sec and she vomited haematin.

The events were considered to be serious due to hospitalisation.

There is no information available on concomitant medication, concomitant diseases and outcome of event.

Tracking

25Oct2011: initial via phone

26Oct2011: Internal update: Reporter adress added

04Nov2011: internal correction; lab values added to lab screen, posology field and daily dose filled out

THE EVENTS GASTROINTESTINAL HAEMORRHAGE AND VOMITED HAEMATIN (HAEMATEMESIS) are listed for Pradaxa and expected in overdose, but may as well be attributed to the co-suspect drug diclofenac. The events QUICK DECREASED and APTT PROLONGED are formally unlisted, but may be explained by mode of action of Pradaxa.

Based on the information provided, no change to the company core safety information or other measures are recommended at this time.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11242543

Serious Yes	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ANA increased	20111130				Unknown
2 TSH increased					Unknown
3 Hepatic steatosis	201112				not recovered/not resolved
4 HDL cholesterol decreased					Unknown
5 Epigastric pain					Unknown
6 Ferritin increased					Unknown
7 Glucose increased	20111116	20111130	15 Day		recovered/resolved
8 GPT increased	20111130				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PROVAS valsartan	Oral	1 Df dosage form/Day	2005-	Hypertension NOS
2 voltaren diclofenac sodium		/	-	-

Drugs (concomitant)

3 ALLOPURINOL allopurinol	/	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Uric acid high			Unknown	
Pain			Unknown	
Hypertension			Unknown	
Alcohol abuse			Unknown	
Gout attack			Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT		69	NA	
ANA	20111130	100	NA	
Glucose	20111116	136 mg/dl	NA	
Glucose	20111130	91 mg/dl	NA	
GPT		69 U/l	NA	
GPT	20111116	64 U/l	NA	
GPT	20111130	65 U/l	NA	
HDL	20111116	35 mg/dl	NA	
Serum ferritin	20111116	606.1 ug/dl	NA	

TSH	2011111€	3.79 uIU/ml	NA
Uric acid	2011111€	9.5 mg/dl	NA

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.12.2011 DE-002147023-PHHY2011DE106350

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE106350, is an initial spontaneous report from a physician (who is patient) via licence partner UCB Pharma (case number 1211PV3245) received on 02 Dec 2011 and forwarded to Novartis on 05 Dec 2011: This report refers to a male patient (unknown age). The patient started treatment with Provas (valsartan) 160 mg orally for an unspecified indication approx 6 years ago. On an unspecified date, the patient experienced epigastric pain. For the clarification of the cause of epigastric pain the physician performed some diagnostic tests (not specified) and was diagnosed with steatosis hepatitis (GPT 69; GGT normal). The seriousness for all the events was reported as serious (medically significant). The outcome and causality of the events were not reported.

-Hepatic steatosis [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Based on Review of available reported data does not allow for a proper causality assessment. Reported event(s) is (are) unassessable

Reporter's comment:

NOVARTIS, 05.01.2012 DE-002147023-PHHY2011DE106350

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE106350, is an initial spontaneous report from a physician (who is patient) via licence partner UCB Pharma (case number 1211PV3245) received on 02 Dec 2011 and forwarded to Novartis on 05 Dec 2011, with follow up report received from physician (who is patient) via licence partner UCB Pharma on 27 Dec 2011. This case refers to an adult male patient (unknown age). The medical history included occasional attacks of gout and pain with uric acid value of 9.5, moderate alcohol abuse and hypertension. The patient started treatment with Provas (valsartan) 160 mg orally for hypertension at a dose of 1 DF once daily orally from an unknown date in 2005. The patient also started Voltaren (diclofenac resinate) occasionally from an unknown date. On an unspecified date, the patient experienced epigastric pain. For the clarification of the cause of epigastric pain, the physician performed some diagnostic tests (not specified) and was diagnosed with steatosis hepatitis (GPT 69; GGT normal) in Dec 2011. On 16 Nov 2011, lab tests also revealed GPT 64 U/l, uric acid 9.5 mg/dl, HDL cholesterol 35 mg/dl, glucose 136 mg/dl, ferritin 606.1 ug/dl and TSH 3.79 uIU/ml. On 30 Nov 2011, lab tests revealed GPT 65 U/l, glucose 91 mg/dl and ANA IgG (titer) 100. Due to the event the dosage of Provas was reduced. The steatosis hepatitis and epigastric pain were considered as serious, while other remaining events were considered as non-serious. The outcome of the steatosis hepatitis was not resolved, while the outcome of the other events was not reported. the causality of all the events was not reported. The physician did not assess the causal relationship of Provas to the steatosis hepatitis.

Follow up report received from physician (who is patient) via licence partner UCB Pharma on 27 Dec 2011: Medical history and lab data updated. Suspect drug dose details updated. Events along with outcome, seriousness and causality updated.

The remaining reported leading events were assessed as non-serious by Novartis Medical Safety Physician.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

The remaining reported leading events were assessed as non-serious by Novartis Medical Safety Physician.

Alcohol abuse is a known confounder.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-11247293

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19420927	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abnormal liver function tests	20110509	20111110	6 Month		recovered/resolved
2 ALT increased					recovered/resolved
3 AST increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC		/	20041118-	
diclofenac sodium			-	
2 MTX		/	20030615-20110526	
methotrexate sodium			7 Year	
3 MTX	Oral	/	20111011-	
methotrexate sodium			-	
4 TRACLEER	Oral	/	-20110815	
bosentan		245	-	
5 TRACLEER	Oral	125 Mg milligram(s)/Day	20090608-20100120	
bosentan		245	7 Month	
6 TRACLEER	Oral	187.5 Mg milligram(s)/Day	20100121-20110615	Skin ulcer
bosentan		245	16 Month	

Drugs (concomitant)

7 OMEPRAZOL	/	-
omeprazole		-
8 PREDNISOLON	/	-
prednisolone		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Skin ulcer			k.A.	Digital ulcer associated with limited systemic sclerosis
CREST syndrome			k.A.	
Systemic sclerosis			k.A.	Limited

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Glutamic-pyruvate transaminase	20101125	34	U/L	
Glutamic-pyruvate transaminase	20110519	124	U/L	
Glutamic-pyruvate transaminase	20110607	251	U/L	
Glutamic-pyruvate transaminase	20110929	49	U/L	
Asp transferase	20101125	30	U/L	
Asp transferase	20110519	73	U/L	

Asp transferase	20110607	120	U/L
Asp transferase	20110929	34	U/L

Narratives (nur bei elektronischem Eingang)ACTELION, 20.12.2011 DE-ACTELION-A-CH2011-58276

Sender diagnosis: not reported

Narrative

Case initial reception date: 08-DEC-2011:

This solicited Health Care Professional report received from Tracleer (bosentan) post-marketing surveillance program (DUO Registry) concerns a 68-year-old male patient (# 0176-01781) with digital ulcer associated with limited systemic sclerosis who experienced medically significant abnormal LFTs. The patient had a medical history of CREST syndrome. Concomitant medication included methotrexate, diclofenac (both were also considered as co-suspect medications), prednisolone and omeprazole. Tracleer was started at 62.5 mg b.i.d. on 08Jun09, was uptitrated to 62.5 mg t.i.d. on 21Jan10 and was permanently discontinued on 15Jun11.

On 09May11, the patient was found to have abnormal LFTs. On 19May11, laboratory values showed AST 2.4xULN (73 U/L, ULN 30 assumed) and ALT 4.1xULN (124 U/L, ULN 30 assumed). Methotrexate was suspected to be responsible for this event and was discontinued on 26May11. However, LFTs further increased and showed on 07Jun11: AST 4xULN (120 U/L, ULN 30 assumed) and ALT 8.4xULN (251 U/L, ULN 30 assumed). AP and bilirubin were not provided throughout the event. The patient did not experience any clinical signs of liver injury. On 15Jun11, Tracleer was permanently discontinued. On 29Sep11, about 3 months later, laboratory values showed AST 1.1xULN (34 U/L, ULN 30 assumed) and ALT 1.6xULN (49 U/L, ULN 30 assumed). On 11Oct11 methotrexate was re-started. On 10Nov11, abnormal LFTs resolved without sequelae (no values provided). Methotrexate was restarted on the same day. The reporter assessed abnormal LFTs as serious and related to Tracleer.

ACTELION, 01.02.2012 DE-ACTELION-A-CH2011-58276

Sender diagnosis: not reported

Narrative

Case initial reception date: 08-DEC-2011:

This solicited Health Care Professional report received from Tracleer (bosentan) post-marketing surveillance program (DUO Registry) concerns a 68-year-old male patient (# 0176-01781) with digital ulcer associated with limited systemic sclerosis who experienced medically significant abnormal LFTs. The patient had a medical history of CREST syndrome. Concomitant medication included methotrexate, diclofenac (both were also considered as co-suspect medications), prednisolone and omeprazole. Tracleer was started at 62.5 mg b.i.d. on 08Jun09, was uptitrated to 62.5 mg t.i.d. on 21Jan10 and was permanently discontinued on 15Jun11.

On 09May11, the patient was found to have abnormal LFTs. On 19May11, laboratory values showed AST 2.4xULN (73 U/L, ULN 30 assumed) and ALT 4.1xULN (124 U/L, ULN 30 assumed). Methotrexate was suspected to be responsible for this event and was discontinued on 26May11. However, LFTs further increased and showed on 07Jun11: AST 4xULN (120 U/L, ULN 30 assumed) and ALT 8.4xULN (251 U/L, ULN 30 assumed). AP and bilirubin were not provided throughout the event. The patient did not experience any clinical signs of liver injury. On 15Jun11, Tracleer was permanently discontinued. On 29Sep11, about 3 months later, laboratory values showed AST 1.1xULN (34 U/L, ULN 30 assumed) and ALT 1.6xULN (49 U/L, ULN 30 assumed). On 11Oct11 methotrexate was re-started. On 10Nov11, abnormal LFTs resolved without sequelae (no values provided). Methotrexate was restarted on the same day. The reporter assessed abnormal LFTs as serious and related to Tracleer.

Follow-up information received on 19Jan12:

On 15Jun11, Tracleer was temporarily interrupted and was restarted a few weeks later on unspecified date, dosage and regimen. The patient experienced a new LFTs increase and Tracleer was permanently discontinued on 15Aug11.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11255766

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials: Sex: Male Birth: 19280826 Age: 83 Year weight (kg): 69 BMI_int: 24.45 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 INR increased	20111120				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	100 Mg milligram(s)/Day	20111116-	Back pain
diclofenac			-	
2 FALITHROM	Oral	.25 Df dosage form/	201106-	Atrial fibrillation
phenprocoumon		82	-	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
INR		> 8		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.12.2011 DE-SZ09-GXKR2011DE003440

Sender diagnosis: not reported

Narrative

Case number GXKR2011DE003440 is a spontaneous report, initially received on 25Nov2011 from a physician with follow-up from the same reporter: This report refers to an 83-year old patient (patient 2 of 3) with an unknown medical history. Concomitant medications not reported. The patient received Falithrom (phenprocoumon) for the treatment of atrial fibrillation from Jun2011 at a dose of 0.25 DF to 0.3 mg and diclofenac (MAH unknown) for the treatment of low back pain from 16Nov2011 at a dose of 1x100 mg/day. On 20Nov2011, the patient experienced increase of INR up to >8. The event was reported serious (hospitalisation). Treatment with suspect medication was stopped on unspecified date but INR still increased and the patient received Konakion (phytomendione). The outcome of the event was reported as resolved. The reporter stated that the medication was tolerated well when taken previously. Causality was assessed as probable by the physician.

Follow-up received on 22Dec2011, leading to upgrade of the case seriousness from 'non-serious' to 'serious':

Patient's data, information to medication up-dated, indications, information to adverse event up-dated, treatment, action taken with drug, outcome and causality assessment.

are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12006449

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19490609	Age: 62 Year	weight (kg): 70	BMI_int: 27.34	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110801	20110815	15 Day	24 Day	recovered/resolved
2 Alanine aminotransferase increased					recovered/resolved
3 Alkaline phosphatase increased					recovered/resolved
4 Aspartate aminotransferase increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 75 diclofenac sodium	Oral	175 Mg milligram(s)/Day 212	20110711-20110711	Ache
2 paracetamol paracetamol	Oral	1 Df dosage form/7 Hour 245	20110708-20110710 3 Day	Lumbago

Drugs (concomitant)

3 lidocaine lidocaine	Unknown	1 Df dosage form/Day 120	20110711-20110711	Lumbago
4 PREDNISOLONE prednisolone	Unknown	/	20110711-20110711	Lumbago

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Polymyalgia rheumatica	201108		Yes	
Fatty liver	201108		Yes	
Depressive episode	201108		Yes	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20110801	128	U/l	
Alanine aminotransferase increased	20110805	87	U/l	
Alanine aminotransferase increased	20110815	40	U/l	
Aspartate aminotransferase increased	20110801	49	U/l	
Aspartate aminotransferase normal	20110809	20	U/l	
Alkaline phosphatase increased	20110802	116	U/l	
Alkaline phosphatase increased	20110805	124	U/l	
Bilirubin normal	20110802	0.48	mg/dl	
Histology NOS	201108	SINGS OF DRUG-INDUCED HEPATIC DAMAGE		
Ultrasound abdomen	201108	FATTY LIVER	NA	
Hepatitis viral test	201108	NEGATIVE	NA	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 18.01.2012 DE-FAKOS-170435

Sender diagnosis: not reported

Narrative

Die 62-jährige Patientin wurde am 01.08.2011 aufgrund unklarer Ganzkörperschmerzen mit Abgeschlagenheit stationär aufgenommen. Ca. 4 Wochen vor der Einweisung hatte sie einen Hexenschuss erlitten, daraufhin bekam sie Injektionen mit Lidocain und Prednisolon im LWS (Lendenwirbelsäulen)-Bereich sowie Diclofenac und Paracetamol per os. Im Aufnahmelabor (01.08.2011) erhöhter ALAT (Alanin-Aminotransferase)-Wert (128 U/l, Normbereich: bis 35 U/l), die Hepatitis-Serologie war unauffällig. Die Abdomensonographie zeigte eine Steatosis hepatis, in der Histologie Zeichen einer medikamentös-toxischen Leberschädigung, die Kriterien einer NASH (Nichtalkoholische Steatohepatitis) wurden hingegen nicht erfüllt. Im Verlauf kam es zu einer Normalisierung der ALAT (15.08.2011: 40 U/l). Am 18.08.2011 wurde die Patientin in gebessertem Allgemeinzustand aus dem Krankenhaus entlassen.

Die Schmerzen waren am ehesten im Rahmen einer Polymyalgia rheumatica zu diskutieren, nach Kortison-Behandlung kam es zu einem deutlichen Rückgang der Symptomatik.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Diclac 75 ID bzw. Diclac retard 100 mg Retardtbl. Hexal, Einnahmezeitraum: am 11.07.2011. (jeweils 1x1 Tabl.)

Paracetamol Tabl. NN, Einnahmezeitraum: 08.-10.07.2011.

Eine UAW durch Diclac und/ oder Paracetamol ist wahrscheinlich, die restlichen Arzneimittel wurden lokal appliziert. Die Arzneimittelkausalität wird ebenfalls mit "wahrscheinlich" bewertet. Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

PVZ-FAKOS, Klinische Pharmakologie, 18.01.2012 DE-FAKOS-170435

Sender diagnosis: not reported

Narrative

Die 62-jährige Patientin wurde am 01.08.2011 aufgrund unklarer Ganzkörperschmerzen mit Abgeschlagenheit stationär aufgenommen. Ca. 4 Wochen vor der Einweisung hatte sie einen Hexenschuss erlitten, daraufhin bekam sie Injektionen mit Lidocain und Prednisolon im LWS (Lendenwirbelsäulen)-Bereich sowie Diclofenac und Paracetamol per os. Im Aufnahmelabor (01.08.2011) erhöhter ALAT (Alanin-Aminotransferase)-Wert (128 U/l, Normbereich: bis 35 U/l), die Hepatitis-Serologie war unauffällig. Die Abdomensonographie zeigte eine Steatosis hepatis, in der Histologie Zeichen einer medikamentös-toxischen Leberschädigung, die Kriterien einer NASH (Nichtalkoholische Steatohepatitis) wurden hingegen nicht erfüllt. Im Verlauf kam es zu einer Normalisierung der ALAT (15.08.2011: 40 U/l). Am 18.08.2011 wurde die Patientin in gebessertem Allgemeinzustand aus dem Krankenhaus entlassen.

Die Schmerzen waren am ehesten im Rahmen einer Polymyalgia rheumatica zu diskutieren, nach Kortison-Behandlung kam es zu einem deutlichen Rückgang der Symptomatik.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Diclac 75 ID bzw. Diclac retard 100 mg Retardtbl. Hexal, Einnahmezeitraum: am 11.07.2011. (jeweils 1x1 Tabl.)

Paracetamol Tabl. NN, Einnahmezeitraum: 08.-10.07.2011.

Eine UAW durch Diclac und/ oder Paracetamol ist wahrscheinlich, die restlichen Arzneimittel wurden lokal appliziert. Die Arzneimittelkausalität wird ebenfalls mit "wahrscheinlich" bewertet. Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 26.01.2012 DE-SANOFI-AVENTIS-2012SA004179

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre (DE-FAKOS-170435) and was extracted from the website of the German health authority (authority number: DE-BFARM-12006449) on 20-Jan-2011. The pharmacovigilance (PV) centre is conducting a hospitalization registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves a 62-year-old female patient who was admitted to hospital due to unclear pain at entire body with exhaustion on 01-Aug-2011. About 4 weeks prior to the hospitalization, she had suffered lumbago and been treated with paracetamol (no trade name provided) 1x1 tablet every seven hours orally from 08-Jul-2011 to 10-Jul-2011 and received injections with lidocaine (1x1 syringe local) and prednisolone (no dose provided) into the lumbar spine area as well as oral diclofenac (1x75 mg and 1x100 mg) on 11-Jul-2011.

Laboratory test at the hospital admission (01-Aug-2011) detected increased alanine aminotransferase (ALAT) (128 U/l, normal high range: 35 U/l).

Serology for hepatitis was unremarkable.

Abdominal ultrasound showed fatty liver and histology revealed signs of drug-induced hepatic damage. However, the criteria of nonalcoholic steatohepatitis were not met.

Following laboratory values were additionally reported:

Alanine aminotransferase: 87 U/L (05-Aug-2011)

Aspartate aminotransferase (normal high range: 35 U/L): 49 U/L (01-Aug-2011), 20 U/L (09-Aug-2011)

Alkaline phosphatase (normal range: 35 - 105 U/L): 116 U/L (02-Aug-2011), 124 U/L (05-Aug-2011)

Bilirubin (normal high range: 1.2 mg/dl): 0.48 mg/dl (02-Aug-2011)

In the clinical course, the ALAT returned to be normal (15-Aug-2011: 40 U/l). On 18-Aug-2011 the patient was discharged from hospital in the improved general condition. The pain was considered to be most likely in the context of polymyalgia rheumatica and after the cortisone therapy the symptom significantly abated. In addition to fatty liver and polymyalgia rheumatica, depressive episode was reported as medical history (start: August 2011).

The PV centre reported acute hepatitis from 01-Aug-2011 to 15-Aug-2011 as adverse drug reaction. Diclofenac and paracetamol were suspected.

Causality assessment by the PV centre concerning acute hepatitis:

"probable/likely": diclofenac, paracetamol

Sender's comments:

Initial report, complete information. Under diclofenac (Diclac 75 ID or 100 mg retard tablets of Hexal), "liver cell necrosis" is labelled as "adverse events", as of: Aug-2009 or Apr-2011.

Under paracetamol tablets of Ratiopharm, "increase in the liver transaminases" is labelled as "adverse events", as of: Nov-2008.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12007263

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19400511	Age: 71 Year	weight (kg): 73	BMI_int: 25.26	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Free haemoglobin present	20110809			4 Day	recovered/resolved
2 Bilirubin increased	20110809			4 Day	recovered/resolved
3 Haemoglobin decreased	20110809			4 Day	recovered/resolved
4 Haemolytic reaction	20110809			4 Day	recovered/resolved
5 LDH increased	20110809			4 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC	Unknown	/	20110806-20110809	Pain in hand
diclofenac sodium			4 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Obesity			Unknown	
Hypertension arterial			Yes	
Type 2 diabetes mellitus			Yes	
Hypercholesterolaemia			Yes	
Icterus			No	
Haemorrhoids			Unknown	
Transaminases increased			Yes	CHRONIC AFTER ICTERUS IN CHILDHOOD
Cholecystectomy	1989	1989	Unknown	
Wolff-Parkinson-White syndrome	1984		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Coombs test		positiv/positive	NA	
IgG		erhöht/increased	NA	
Haemoglobin decreased		5.4	g/dl	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 24.01.2012 DE-002147023-PHHY2012DE005153

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE005153, is an initial spontaneous report received from the Drug Commission of the German Medical Association of Physicians via Health Authority (E2 B report duplicate: DE-BFARM-12007263 and DE-DCGMA-11156526) on 19 Jan 2012. This case refers to a 71-year-old female patient. The patient had current conditions of type 2 diabetes mellitus, hypertension arterial, obesity, hypercholesterolaemia, haemorrhoids, transaminases increased, Wolff-Parkinson White syndrome and historical condition of cholecystectomy. The patient received diclofenac (manufacturer unknown) on 06 Aug 2011 for 4 days for pain in hand at unknown dose. On 09 Aug 2011, the patient experienced bilirubin increased, LDH (lactate dehydrogenase) increased, haemoglobin decreased, patient had haemolytic reaction and free haemoglobin present. On 09 Aug 2011, treatment with diclofenac was discontinued. The patient was hospitalised on an unspecified date due to the events. Relevant lab data included,

Coombs test positive, IgG (Blood immunoglobulin G) increased and haemoglobin decreased to 5.4 g/dl. The events were assessed as life threatening by the reporter. The outcome was reported as recovered and the causality reported as suspected.

Blood bilirubin increased, Haemoglobin decreased [life threatening] assessed as listed according to the Core Data Sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements. Based on Review of available reported data does not allow for a proper causality assessment. Reported event(s) is (are) unassessable

Reporter's comment:

sanofi-aventis, 26.01.2012 DE-SANOFI-AVENTIS-2012SA004193

Sender diagnosis: not reported

Narrative

Initial report:

On 20-Jan-2012 this case was extracted from the website of the German health authority (authority number DE-BFARM-12007263); documenting a case, in which a physician had reported about a 71-year-old female patient.

Medical history included chronic increased transaminases after icterus in childhood and cholecystectomy 1989 as well as haemorrhoids, hypercholesterolaemia, obesity, Wolff-Parkinson-White syndrome (first diagnosed in 1984), arterial hypertension and Type 2 diabetes mellitus. For pain in hand the patient started with diclofenac sodium (reported: diclofenac) on 06-Aug-2011. Concomitant medication was not mentioned. Three days later the patient developed life-threatening haemolytic reaction with increased bilirubin, increased LDH, decreased hemoglobin and present free haemoglobin.

Diclofenac sodium was stopped on the same day. Corrective therapy was not reported; however the patient recovered at unspecified time point.

Causality by Drug Commission: possible

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12049136

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth:	Age: 51 Year	weight (kg): 64	BMI_int: 22.41	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GPT increased	20120224	20120328	34 Day	3 Day	recovered/resolved
2 GOT increased	20120224	20120328	34 Day	3 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DECORTIN prednisone		250 Mg milligram(s)/	20120224-	Joint pain
2 DICLO SL 1A PHARMA diclofenac	Oral	150 Mg milligram(s)/Day 94	20120222-20120227 6 Day	Joint pain

Drugs (concomitant)

3 OMEPRAZOLE omeprazole	Oral	20 Mg milligram(s)/Day	20120222-20120227 6 Day
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Intestinal carcinoma	2006		Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20120224	37		
Aspartate aminotransferase	20120314	35		
Aspartate aminotransferase	20120308	38		
Aspartate aminotransferase	20120301	155		
Aspartate aminotransferase	20120228	122		
Aspartate aminotransferase	20120321	38		
GPT	20120329	48		
GPT	20120321	72		
GPT	20120301	379		
GPT	20120228	233		
GPT	20120224	87		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 09.03.2012 DE-SZ09-GXKR2012DE001024

Sender diagnosis: not reported

Narrative

Case number GXKR2012DE001024, is an initial spontaneous report from a consumer received on 05Mar2012. This report refers to a 51-year-old female patient. The patient's medical history was not stated. Concomitant medications included Decortin (prednisone) infusions. The patient received Diclo 75 SL-1A Pharma (diclofenac) for treatment of joint pain at a total dose of 11 tablets over one week. Further information on treatment was not

provided. At an indefinite date the patient experienced GPT (glutamat pyruvat transaminase) of 379 (no unit or normal range provided). If further actions were taken due to adverse event was not stated. This increase of alanine aminotransferase was considered serious (medically significant) by the manufacturer. The event outcome was not reported. The reporter provided no causality assessment. spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 12.04.2012 DE-SZ09-GXKR2012DE001024

Sender diagnosis: not reported

Narrative

Case number GXKR2012DE001024, is an initial spontaneous report from a consumer received on 05Mar2012 and follow up received from consumer on 10 Apr 2012. This report refers to a 51-year-old female patient. The patient's medical history was intestinal cancer in 2006. Concomitant medications were not reported. The patient received Diclo 75 SL-1A Pharma (diclofenac) for treatment of joint pain at a total dose of 11 tablets over one week, Decortin (prednisone) infusions and Ombeta (omeprazole). On 24 Feb 2012, Alanine aminotransferase was 87 IU/l and Aspartate aminotransferase was 37 IU/l (normal value for the both was less than 35 IU/l). On 28 Feb 2012, Alanine aminotransferase was 233 IU/l and Aspartate aminotransferase was 122 IU/l. On 01 Mar 2012, Alanine aminotransferase was 379 IU/l and Aspartate aminotransferase was 155 IU/l. On 08 Mar 2012, Aspartate aminotransferase was 38 IU/l. On 14 Mar 2012, Aspartate aminotransferase was 35 IU/l. On 21 Mar 2012, Alanine aminotransferase was 72 IU/l and Aspartate aminotransferase was 38 IU/l. On 29 Mar 2012, Alanine aminotransferase was 48 IU/l. The events were considered serious (medically significant) by the manufacturer. Treatment with suspected drugs was stopped. The events outcome was reported as recovered. The consumer reported the events were certainly related to the treatment with suspected drugs.

Significant follow up received on 10 Apr 2012: Updated Adverse event, Progression of events, Outcome, Medical history, Suspect drugs, Laboratory tests and Reporter's clinical (causality) assessment.

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-12065159

Serious Yes	Death Yes	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19250308	Age: 78 Year	weight (kg): 70	BMI_int: 26.67	Age fetus:
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Patientdeath

deathdate: 20110623	deathcause: Hemorrhagic shock
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic congestion					Unknown
2 Hemorrhagic shock	20110623			15 Year	fatal
3 Depression					Unknown
4 Diabetes mellitus	200401				Unknown
5 Hypertension	200707				Unknown
6 Goiter nodular					Unknown
7 Vertebral injury					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	50 Mg milligram(s)/	19960101- -	

Drugs (concomitant)

2 Acetylsalizylsaeure acetylsalicylic acid	Oral	100 Mg milligram(s)/	19960101- -	
3 Diuretika		/	20091009- -	
4 l-thyroxin levothyroxine sodium	Oral	100 µg microgram(s)/	19960101- -	
5 NIFEDIPINE nifedipine	Oral	30 Mg milligram(s)/	19960124- -	
6 RASILEZ aliskiren fumarate		/	- -	Hypertension arterial
7 SAROTEN amitriptyline hydrochloride	Oral	75 Mg milligram(s)/	20040705- -	
8 zaneril enalapril maleate; lercanidipine hydrochloride	Oral	/	20090130- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypercholesterolaemia			Unknown	

Primary source

Qualification: Physician	observestudytype: Individual patient use
Literature:	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Blood pressure	20081010	150/85	mmHg	
Blood pressure	20091009	180/90	mmHg	
Blood pressure	20110127	170/80	mmHg	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 29.03.2012 DE-002147023-PHHY2012DE025858

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE025858, is an initial PMS study (SPP100ADE06) report received from an investigator on 23 Mar 2012: The patient was enrolled in PMS study Rasilez (3A Register regarding out-patient therapy with RAS-inhibitors in patients with arterial hypertension in Germany). This report refers to an elderly female patient (pat. no: 3A2570/24). The patient's medical history included hypercholesterolaemia/ hyperlipidaemia. The patient had been on treatment with multiple concomitant drugs. The patient was treated with diclofenac (manufacturer unknown) orally at 50 mg/day for an unspecified indication since 01 Jan 1996. In Jan 2004, the patient presented with diabetes mellitus and experienced essential hypertension in Jul 2007. On 10 Oct 2008, blood pressure was 150/85 mmHg. During the follow-up visit on 09 Oct 2009 the physician noted that blood pressure had been increased to 180/90 mmHg. During the follow-up visit on 27 Jan 2011 the physician noted that blood pressure has been 170/80 mmHg. On an unknown date, the patient developed chronic congested liver, nodular goiter, depression and vertebral body compression. On 23 Jun 2011, the patient was hospitalized and died due to hemorrhagic shock. The outcome of the events essential hypertension, chronic congested liver, nodular goiter, depression and vertebral body compression was not reported. The physician assessed the intensity of the event hemorrhagic shock as severe (fatal). The reporter assessed the causal relationship of diclofenac to the event death due to hemorrhagic shock as unlikely and for other events it was not reported.

NOVARTIS, 17.04.2012 DE-002147023-PHHY2012DE025858

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE025858, is an initial PMS study (SPP100ADE06) report received from an investigator on 23 Mar 2012 with follow up report received from an investigator on 13 Apr 2012: The patient was enrolled in PMS study Rasilez (3A Register regarding out-patient therapy with RAS-inhibitors in patients with arterial hypertension in Germany). This report refers to an elderly female patient (pat. no: 3A2570/24). The patient's medical history included hypercholesterolaemia/ hyperlipidaemia. The patient had been on treatment with multiple concomitant drugs. The patient was treated with Diclo Divo Long REK 100 (diclofenac, manufacturer: Actavis) orally at 50 mg/day for an spinal joint problem since 01 Jan 1996. In Jan 2004, the patient presented with diabetes mellitus and experienced essential hypertension in Jul 2007. On 10 Oct 2008, blood pressure was 150/85 mmHg. During the follow-up visit on 09 Oct 2009 the physician noted that blood pressure had been increased to 180/90 mmHg. During the follow-up visit on 27 Jan 2011 the physician noted that blood pressure has been 170/80 mmHg. On an unknown date, the patient developed chronic congested liver, nodular goiter, depression and vertebral body compression. On 23 Jun 2011, the patient was hospitalized and died due to hemorrhagic shock. The outcome of the events essential hypertension, chronic congested liver, nodular goiter, depression and vertebral body compression was not reported. The physician assessed the intensity of the event hemorrhagic shock as severe (fatal). The reporter assessed the causal relationship of diclofenac to the event death due to hemorrhagic shock as unlikely and for other events it was not reported.

Follow up report received from from an investigator on 13 Apr 2012: It was reported that diclofenac had been specified as Diclo Divo Long REK 100 (diclofenac, manufacturer: Actavis), thus it had been clarified that the manufacturer of diclofenac was not Novartis and that in this case no Novartis-drug has been specified. Hence the case will be deactivated.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12087508

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19430403	Age: 68 Year	weight (kg): 80	BMI_int: 29.38	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased					recovered/resolved
2 Drug-induced hepatitis	20111017			5 Month	recovered/resolved
3 SGOT increased					recovered/resolved
4 SGPT increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	150 Mg milligram(s)/	201105-201110	Ache
diclofenac			5 Month	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin	20111020	19.25	mg/dl	
SGOT	20111019	1294	U/l	
SGPT	20111019	1649	U/l	

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 04.05.2012 DE-SANOFI-AVENTIS-2012SA030853

Sender diagnosis: not reported

Narrative

Initial report:

On 02-May-2012 this case was extracted from the website of the German health authority (authority number DE-BFARM-12087508); documenting a case, in which a physician had reported to the Drug Commission of German Medical Association about a 68-year-old male patient. Medical history and concomitant medication were not provided.

For ache the patient took diclofenac (trade name not provided) 150 mg from May-2011 to 01-Oct-2011. On 17-Oct-2011 drug-induced hepatitis was detected. The patient recovered at unspecified time point.

Causality was not given.

Reporter's comment:

NOVARTIS, 07.05.2012 DE-002147023-PHHY2012DE037584

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE037584, is an initial spontaneous report received from the physician of drug commission of the German medical association (DE-DCGMA-11156901) via Health authority (DE-BFRAM-12087508) on 30 Apr 2012. This report refers to a 68-year-old male patient.

The patient received diclofenac (manufacturer unknown) on May 2011 at a dose of 150 mg for the treatment of ache. On 19 Oct 2011, the patient's laboratory data showed serum glutamic pyruvic transaminase (SGPT) level at 1649 U/l (normal range unspecified), and serum glutamic oxaloacetic transaminase (SGOT) level at 1294 (normal range unspecified). On 20 Oct 2011, bilirubin level was at 19.25 mg/dl (normal range unspecified). On 17 Oct 2011, the patient developed drug induced hepatitis. On Oct 2011, treatment with diclofenac was discontinued. The outcome of the event was reported as complete recovery. The seriousness was not reported. The causality was not reported.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12088753

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea					Unknown
2 Macroscopic haematuria					Unknown
3 Leucocytosis NOS					Unknown
4 Lactate dehydrogenase increased					Unknown
5 Jaundice					Unknown
6 Bilirubin total high					Unknown
7 Fatigue					Unknown
8 Anaemia haemolytic autoimmune	201102				recovering/resolving
9 Vomiting					Unknown
10 Erythrophagocytosis					recovered/resolved
11 Haemolysis		201107			recovered/resolved
12 Autoimmune haemolytic anaemia NOS	201102	2011			recovered/resolved
13 Acute renal failure	201102	2011			recovering/resolving
14 Free hemoglobin present					recovered/resolved
15 Creatinine increased					recovering/resolving
16 Autoimmune hemolytic anemia	2011	201107			recovered/resolved
17 Anaemia					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC diclofenac	Unknown	/	2011- 3 Week	Back pain

Drugs (concomitant)

2 Cotrimoxazole	Unknown	/	201102- -	Cystitis acute
3 levofloxacin levofloxacin		/	- -	Cystitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Basal cell carcinoma excision			No	12 months earlier
Breast cancer			Unknown	Successful breast cancer treatment at the age of 59 years
Macroscopic haematuria			Unknown	caused by acute cystitis; one of the reasons for the current admission in Feb 2011
Cystitis			Unknown	
Basal cell carcinoma			Unknown	of the forehead
Back pain			Yes	
Acute cystitis			Yes	treated with cotrimoxazole 24h before admission
Vomiting			Unknown	experienced recurrent episodes in the week before presentation
Nausea			Unknown	experienced recurrent episodes in the week before presentation
Leukocytosis			Yes	
MRI			Unknown	

Primary source

Qualification: Other Health Professional observestudytype:

Literature: Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze T, Kaminski W et. al.
An autoimmune double attack.
Lancet. 2012;379:1364

Qualification: Other Health Professional observestudytype:

Literature: Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze TJ, et al.
An autoimmune double attack.
Lancet 2012; 379(9823) 1364

Qualification: Other Health Professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Antiglobulin test direct		Detected antibodies	None	Reacts to IgG and complement C3d
Bilirubin total		75	micromol/L	
Bone marrow biopsy		No evidence of	None	Neoplasia
Creatinine		0.34	mmol/L	
Haemoglobin		56	g/L	
IgG		(diclofenac) Positive		
Lactate dehydrogenase		2500	IU/L	
WBC count		17.6 x 10e9	/L	
Free haemoglobin		8.05	g/L	
Whole body MRI		No evidence of	None	Neoplasia
Eosinophil percentage		1	%	
Neutrophil percentage		90	%	
Blood smear test		(peripheral) Neutrophil		phagocytosis (about 5%)
Serology test		Negative for a	None	Microbial aetiology

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 30.04.2012 DE-002147023-PHHY2012DE034936

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE034936, is an initial literature report received on 23 Apr 2012. The author presented a case report of unusual dual

autoimmune response triggered by the combination of repeated diclofenac intake and an acute infection (cystitis). This report refers to a 66-year-old female patient. Patient's medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead 12 months earlier. Concomitant medication included cotrimoxazole for cystitis 24 hrs earlier before admitted to the reporting hospital. The patient had repeatedly self-administered diclofenac (manufacturer unknown) during the previous 3 weeks (before hospitalization) for back pain. In Feb 2011, the patient was admitted with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Blood tests showed anaemia (haemoglobin 56 g/L) and leukocytosis ($17.6 \times 10^9/L$, 90% neutrophils, 1% eosinophils); high concentrations of total bilirubin, 75 $\mu\text{mol/L}$, and creatinine, 0.34 mmol/L. Lactate dehydrogenase was 2500 U/L and free haemoglobin 805 g/L (normal <0.05 g/L), which indicated massive intra vascular haemolysis. Our suspicion of auto immune hemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. The patient was given intravenous prednisolone 125 mg/day and a blood transfusion. The acute cystitis causing the macrohaematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patients history and the known association of autoimmune hemolytic anaemia with malignancies; however, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells.¹ Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from our patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes,^{2,3} was present in more than 80% of circulating neutrophils (normal 58%) indicating that a flexible autoimmune response had been triggered in neutrophils. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune hemolytic anaemia. Hemolytic signs and renal function had nearly returned to normal after 2 weeks of steroid treatment. When last seen in July 2011, her red cell measurements were normal, and steroids were discontinued. The author suspected that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis). Patient's with acute haemolysis should be scrutinized for drug-induced hemolytic anaemia. The most effective treatment was to stop the drug and avoid the drug in the future.

sanofi-aventis, 07.05.2012 DE-SANOFI-AVENTIS-2012SA029699

Sender diagnosis: not reported

Narrative

This unsolicited literature case was identified on 25 April 2012 via an article: Fuchs T, Puellmann K, Schneider S, et al. An autoimmune double attack. The Lancet 2012; 379(9823): 1364.

This case involves a 66 year old female patient who experienced drug-induced cold-type autoimmune haemolytic anaemia and acute renal failure due to cold-type autoimmune haemolytic anaemia while treated with diclofenac sodium taken for back pain.

On an unknown date in 2011, the patient repeatedly self-administered diclofenac sodium. In February 2011, she was hospitalized due to fatigue, jaundice, and macroscopic haematuria. It was stated that in the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Blood tests showed anaemia (haemoglobin 56 g/L) and leukocytosis ($17.6 \times 10^9/L$, 90% neutrophils, 1% eosinophils), high concentrations of total bilirubin, 75 $\mu\text{mol/L}$, and creatinine, 0.34 mmol/L. Lactate dehydrogenase was 2500 U/L and free haemoglobin 8.05 g/L (normal: less than 0.05 g/L), which indicated massive intra vascular haemolysis. Subsequently, auto immune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. Thereafter, she was administered treatment with intravenous prednisolone 125 mg daily and a blood transfusion. It was reported that cancer screening was undertaken because of the her history and the known association of autoimmune haemolytic anaemia with malignancies; however, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia. It was further stated that haemolytic signs and renal function nearly returned to normal after 2 weeks of steroid treatment. Two weeks after prednisolone therapy, no erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor normalised. Later in July, 2011, her red cell measurements were normal, and steroids (prednisolone) were discontinued.

The event of drug-induced cold-type autoimmune haemolytic anaemia had resolved while acute renal failure due to cold-type autoimmune haemolytic anaemia was improving. There was insufficient information regarding the on-going therapy of diclofenac sodium.

The patient's medical history included breast cancer which was successfully treated at the age of 59. She had undergone excision of basal cell carcinoma of forehead in February 2010 (12 months earlier). Her concurrent condition included leukocytosis. Concomitant medications included cotrimoxazole and levofloxacin both taken for cystitis. No past drug was reported.

The author stated that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis).

drreddys, 11.05.2012 DE-drreddys-GER/GER/12/0024276

Sender diagnosis: not reported

Narrative

This is a report from a literature. A 66-year-old female patient developed drug-induced cold-type autoimmune haemolytic anaemia with acute renal failure after administration of diclofenac for backpain.

The patient's medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the

forehead twelve months earlier. She was not taking regular medication but had been given cotrimoxazole for cystitis 24 hours earlier.

In February 2011, the patient was admitted to hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Blood tests on unspecified dates, showed anaemia (haemoglobin 56 g/l) and leukocytosis ($17.6 \times 10^9/l$, 90% neutrophils, 1% eosinophils). High concentrations of total bilirubin 75 $\mu\text{mol/l}$, and creatinine 0.34 mmol/l. Lactate dehydrogenase was 2500 U/l and free haemoglobin 8.05 g/l (normal $<0.05 \text{ g/l}$), which indicated massive intravascular haemolysis. On an unspecified date, the suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology.

The patient was given intravenous prednisolone 125 mg/day and a blood transfusion. The acute cystitis causing the macrohaematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the previous history of basal-cell carcinoma and the known association of autoimmune haemolytic anaemia with malignancies. However, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells. Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from the patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating neutrophils (normal 5-8%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of the patient's history revealed that she had repeatedly self-administered diclofenac during the previous three weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia.

Steroids were reported as the first-line therapy in this disorder and patients with symptomatic anaemia might need blood transfusion. In refractory cases, splenectomy and rituximab are effective. In this patient, haemolytic signs and renal function had nearly returned to normal after two weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised. During the patient's last consultation in July 2011, her red cell measurements were normal, and steroids were discontinued.

The authors stated that the pathogenesis of autoimmune haemolytic anaemia is poorly understood. IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. This case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. The authors suggested that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis). Patients with acute haemolysis should be scrutinized for drug-induced haemolytic anaemia. The most effective treatment is to stop the drug and avoid the drug in the future. At the time of reporting, the outcome of the event was unknown.

This case was considered as serious (hospitalisation).

Literature source: Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze TJ, Neumaier M, et al. An autoimmune double attack. *Lancet*. 2012; 379: 1364.

Dr. R. Pfleger Chemische Fabrik GmbH, 14.05.2012 DE-PFLEGER-001769

Sender diagnosis: not reported

Narrative

Active substance (diclofenac) related report from the scientific literature.

A 66-year-old female patient developed drug-induced cold-type autoimmune haemolytic anaemia with acute renal failure after administration of diclofenac. In February, 2011, the patient was admitted to hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead twelve months earlier. She was not taking regular medication but had been given cotrimoxazole for cystitis 24 hours earlier. Blood tests showed anaemia (haemoglobin 56 g/l) and leukocytosis ($17.6 \times 10^9/l$, 90% neutrophils, 1% eosinophils). High concentrations of total bilirubin 75 $\mu\text{mol/l}$, and creatinine 0.34 mmol/l. Lactate dehydrogenase was 2500 U/l and free haemoglobin 8.05 g/l (normal $<0.05 \text{ g/l}$), which indicated massive intravascular haemolysis. The suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. The patient was given intravenous prednisolone 125 mg/day and a blood transfusion. The acute cystitis causing the macrohaematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patient's history and the known association of autoimmune haemolytic anaemia with malignancies. However, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells. Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from the patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating neutrophils (normal 5-8%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of the patient's history revealed that she had repeatedly self-administered diclofenac during the previous three weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia. Steroids are the first-line therapy in this disorder and patients with symptomatic anaemia might need blood transfusion. In refractory cases, splenectomy and rituximab are effective. In this patient, haemolytic signs and renal function had nearly returned to normal after two weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised. When last seen in July 2011, her red cell measurements were normal, and steroids were discontinued. The authors state that the pathogenesis of autoimmune haemolytic anaemia is poorly understood. IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. This case reveals a second mechanism of autoimmunity in the

pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor.<dbtext>To_Char_NARRATIVE_2</dbtext>

MEDA Pharma GmbH & Co. KG, 14.05.2012 DE-MEDA-M-EU-2012050113

Sender diagnosis: not reported

Narrative

A literature report was received by YES Services (DE-VIGILIT-698325) - literature search project VIGILIT - on 03-May-2012.

A 66-year-old female patient developed drug-induced cold-type autoimmune haemolytic anaemia with acute renal failure after administration of diclofenac. In February, 2011, the patient was admitted to hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead twelve months earlier. She was not taking regular medication but had been given cotrimoxazole for cystitis 24 hours earlier. Blood tests showed anaemia (haemoglobin 56 g/l) and leukocytosis ($17.6 \times 10^9/l$, 90% neutrophils, 1% eosinophils). High concentrations of total bilirubin $75 \mu\text{mol/l}$, and creatinine 0.34 mmol/l . Lactate dehydrogenase was 2500 U/l and free haemoglobin 8.05 g/l (normal $<0.05 \text{ g/l}$), which indicated massive intravascular haemolysis. The suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. The patient was given intravenous prednisolone 125 mg/day and a blood transfusion. The acute cystitis causing the macrohaematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patient's history and the known association of autoimmune haemolytic anaemia with malignancies. However, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells. Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from the patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating neutrophils (normal 5-8%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of the patient's history revealed that she had repeatedly self-administered diclofenac during the previous three weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia. Steroids are the first-line therapy in this disorder and patients with symptomatic anaemia might need blood transfusion. In refractory cases, splenectomy and rituximab are effective. In this patient, haemolytic signs and renal function had nearly returned to normal after two weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised. When last seen in July 2011, her red cell measurements were normal, and steroids were discontinued.

The authors state that the pathogenesis of autoimmune haemolytic anaemia is poorly understood. IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. This case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. The authors suggest that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis). Patients with acute haemolysis should be scrutinised for drug-induced haemolytic anaemia. The most effective treatment is to stop the drug and avoid the drug in the future.

Literature source:

Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze TJ, Neumaier M, et al. An autoimmune double attack. Lancet. 2012; 379:1364.
autoimmune response was triggered by the combination of repeated diclofenac intake

Heumann Pharma GmbH & Co. Generika KG, 23.05.2012 DE-HEUMANN1-001953

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

A 66-year-old female patient developed drug-induced cold-type autoimmune haemolytic anaemia with acute renal failure after administration of diclofenac. In February, 2011, the patient was admitted to hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead twelve months earlier. She was not taking regular medication but had been given cotrimoxazole for cystitis 24 hours earlier. Blood tests showed anaemia (haemoglobin 56 g/l) and leukocytosis ($17.6 \times 10^9/l$, 90% neutrophils, 1% eosinophils). High concentrations of total bilirubin $75 \mu\text{mol/l}$, and creatinine 0.34 mmol/l . Lactate dehydrogenase was 2500 U/l and free haemoglobin 8.05 g/l (normal $<0.05 \text{ g/l}$), which indicated massive intravascular haemolysis. The suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. The patient was given intravenous prednisolone 125 mg/day and a blood transfusion. The acute cystitis causing the macrohaematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patient's history and the known association of autoimmune haemolytic anaemia with malignancies. However, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells. Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from the patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating neutrophils (normal 5-8%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of the patient's history revealed that she had repeatedly self

administered diclofenac during the previous three weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia. Steroids are the first-line therapy in this disorder and patients with symptomatic anaemia might need blood transfusion. In refractory cases, splenectomy and rituximab are effective. In this patient, haemolytic signs and renal function had nearly returned to normal after two weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised. When last seen in July 2011, her red cell measurements were normal, and steroids were discontinued.

The authors state that the pathogenesis of autoimmune haemolytic anaemia is poorly understood. IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. This case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. The authors suggest that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis). Patients with acute haemolysis should be scrutinised for drug-induced haemolytic anaemia. The most effective treatment is to stop the drug and avoid the drug in the future.

MENARINIP, 29.05.2012 DE-MEN-26658

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, involved a 66-year-old female patient, who developed drug-induced cold-type autoimmune haemolytic anaemia with acute renal failure after administration of diclofenac.

In February, 2011, the patient was admitted to hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead twelve months earlier. She was not taking regular medication but had been given sulfamethoxazole/trimethoprim for cystitis 24 hours earlier. Blood tests showed anaemia (haemoglobin 56 g/l) and leukocytosis ($17.6 \times 10^9/l$, 90% neutrophils, 1% eosinophils). High concentrations of total bilirubin $75 \mu\text{mol/l}$, and creatinine 0.34 mmol/l . Lactate dehydrogenase was 2500 U/l and free haemoglobin 8.05 g/l (normal $<0.05 \text{ g/l}$), which indicated massive intravascular haemolysis. The suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. The patient was given intravenous prednisolone 125 mg/day and a blood transfusion.

The acute cystitis causing the macrohaematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patient's history and the known association of autoimmune haemolytic anaemia with malignancies. However, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells. Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from the patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating neutrophils (normal 5-8%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of the patient's history revealed that she had repeatedly self-administered diclofenac during the previous three weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia.

Haemolytic signs and renal function had nearly returned to normal after two weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised. When last seen in July 2011, her red cell measurements were normal, and steroids were discontinued.

The authors stated that the pathogenesis of autoimmune haemolytic anaemia would be poorly understood. IgG type antibodies to red-cell/drug-antigen complexes were thought to trigger the premature elimination of erythrocytes by splenic macrophages. This case revealed a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. The authors suggested that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis). Patients with acute haemolysis should be scrutinised for drug-induced haemolytic anaemia. The most effective treatment would be to stop the drug and avoid the drug in the future.

MENARINIP, 30.05.2012 DE-MEN-26658

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, involved a 66-year-old female patient, who developed drug-induced cold-type autoimmune haemolytic anaemia with acute renal failure after administration of diclofenac.

In February, 2011, the patient was admitted to hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead twelve months earlier. She was not taking regular medication but had been given sulfamethoxazole/trimethoprim for cystitis 24 hours earlier. Blood tests showed anaemia (haemoglobin 56 g/l) and leukocytosis ($17.6 \times 10^9/l$, 90% neutrophils, 1% eosinophils). High concentrations of total bilirubin $75 \mu\text{mol/l}$, and creatinine 0.34 mmol/l . Lactate dehydrogenase was 2500 U/l and free haemoglobin 8.05 g/l (normal $<0.05 \text{ g/l}$), which indicated massive intravascular haemolysis. The suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. The patient was given intravenous prednisolone 125 mg/day and a blood transfusion.

The acute cystitis causing the macrohaematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patient's history and the known association of autoimmune haemolytic anaemia with malignancies. However, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells. Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from the patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating

neutrophils (normal 5-8%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of the patient's history revealed that she had repeatedly self-administered diclofenac during the previous three weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia. Haemolytic signs and renal function had nearly returned to normal after two weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised. When last seen in July 2011, her red cell measurements were normal, and steroids were discontinued.

The authors stated that the pathogenesis of autoimmune haemolytic anaemia would be poorly understood. IgG type antibodies to red-cell/drug-antigen complexes were thought to trigger the premature elimination of erythrocytes by splenic macrophages. This case revealed a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. The authors suggested that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis). Patients with acute haemolysis should be scrutinised for drug-induced haemolytic anaemia. The most effective treatment would be to stop the drug and avoid the drug in the future.

DEXPHARM, 20.06.2012 DE-DEXPHARM-20120716

Sender diagnosis: not reported

Narrative

Report received from literature.

Reference: Fuchs, T.; Puellmann, K.; Schneider, S.; Kruth, J.; Schulze, T. J.; Neumaier, M.; Beham, A. W.; Kaminski, W. E. An autoimmune double attack. *Lancet* 2012, V379, P1364

A 66-year-old woman developed autoimmune haemolytic anaemia during treatment with diclofenac.

The woman was admitted in February 2011 with fatigue, jaundice and macroscopic haematuria. In the previous week, recurrent nausea, vomiting and haematuria had occurred. She had received cotrimoxazole [trimethoprim/sulfamethoxazole] for cystitis 24 hours before admission. Her blood tests results were: haemoglobin 56 g/L, free haemoglobin 8.05 g/L, WBC count 17.6×10^9 /L, total bilirubin 75 micromol/L, creatinine 0.34 mmol/L and lactate dehydrogenase 2500 U/L. Autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by direct antiglobulin testing, which revealed antibodies to IgG and complement C3d. The woman received a blood transfusion and prednisolone. Her cystitis-related haematuria resolved after levofloxacin treatment. Peripheral blood smears revealed neutrophil erythrophagocytosis, and immunocytochemistry demonstrated T cell variable immunoreceptor expression in neutrophils. Her history revealed that during the previous 3 weeks, she had repeatedly self-administered diclofenac for back pain [route and dosage not stated]. After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia was diagnosed. After 2 weeks of steroids, her haemolytic signs and renal function had nearly normalised, T cell receptor expression had normalised, and erythrophagocytosis was undetectable. In July 2011, her red cell measurements were normal, and steroids were withdrawn.

Author Comment: "IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. Our patient's case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. We suspect that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis)."

Company assessment: This 66 years old female had been taking diclofenac for about three weeks for back pain and cotrimoxazole for about 24 hours (for cystitis) when she developed haemolytic anaemia (Hb 56 g/L; free Hb 8.05 g/L; LDH 2500 U/L) requiring transfusion. She also had renal failure (creatinine 0.34 mmol/L). She was diagnosed with cold-type autoimmune haemolytic anaemia due to anti-diclofenac IgG antibodies. She was treated with steroids and recovered. The temporal association of use of diclofenac and the specificity of the antibodies, as well as the known risk, suggest a definite causal relationship. The development of acute renal failure is certainly related to the severe intravascular haemolysis, so indirectly related to diclofenac which has a known risk for renal failure.

STADA Arzneimittel AG, 20.06.2012 DE-STADA-048323

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze TJ, et al. An autoimmune double attack. *Lancet* 2012; 379(9823) 1364

Summary:

A 66-year-old woman developed autoimmune haemolytic anaemia during treatment with diclofenac.

The woman was admitted in February 2011 with fatigue, jaundice and macroscopic haematuria. In the previous week, recurrent nausea, vomiting and haematuria had occurred. She had received cotrimoxazole [trimethoprim/sulfamethoxazole] for cystitis 24 hours before admission. Her blood tests results were: haemoglobin 56 g/L, free haemoglobin 8.05 g/L, WBC count 17.6×10^9 /L, total bilirubin 75 micromol/L, creatinine 0.34 mmol/L and lactate dehydrogenase 2500 U/L. Autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by direct antiglobulin testing, which revealed antibodies to IgG and complement C3d.

The woman received a blood transfusion and prednisolone. Her cystitis-related haematuria resolved after levofloxacin treatment. Peripheral blood smears revealed neutrophil erythrophagocytosis, and immunocytochemistry demonstrated T cell variable immunoreceptor expression in neutrophils. Her history revealed that, during the previous 3 weeks, she had repeatedly self-administered diclofenac for back pain [route and dosage not stated].

After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia was diagnosed. After 2 weeks of steroids, her haemolytic signs and renal function had nearly normalised, T cell receptor expression had normalised, and erythrophagocytosis was undetectable. In July 2011, her red cell measurements were normal, and steroids were withdrawn. To_Char_NARRATIVE_2</dbtext>
suspect that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis).

KRKAPR, 21.06.2012 DE-KRKA-DE2012K1748LIT

Sender diagnosis: not reported

Narrative

Diclofenac

Autoimmune haemolytic anaemia in an elderly patient: case report

A 66-year-old woman developed autoimmune haemolytic anaemia during treatment with diclofenac.

The woman was admitted in February 2011 with fatigue, jaundice and macroscopic haematuria. In the previous week, recurrent nausea, vomiting and haematuria had occurred. She had received cotrimoxazole [trimethoprim/sulfamethoxazole] for cystitis 24 hours before admission. Her blood tests results were: haemoglobin 56 g/L, free haemoglobin 8.05 g/L, WBC count $17.6 \times 10^9/L$, total bilirubin $75 \mu\text{mol/L}$, creatinine 0.34 mmol/L and lactate dehydrogenase 2500 U/L . Autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by direct antiglobulin testing, which revealed antibodies to IgG and complement C3d.

The woman received a blood transfusion and prednisolone. Her cystitis-related haematuria resolved after levofloxacin treatment. Peripheral blood smears revealed neutrophil erythrophagocytosis, and immunocytochemistry demonstrated T cell variable immunoreceptor expression in neutrophils. Her history revealed that during the previous 3 weeks, she had repeatedly self-administered diclofenac for back pain [route and dosage not stated]. After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia was diagnosed. After 2 weeks of steroids, her haemolytic signs and renal function had nearly normalised, T cell receptor expression had normalised, and erythrophagocytosis was undetectable. In July 2011, her red cell measurements were normal, and steroids were withdrawn.

Author Comment "IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. Our patient's case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. We suspect that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis)."

CITATION DETAILS

Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze TJ, Neumaier M, Beham AW, Kaminski WE. An autoimmune double attack. Lancet 379: 1364, No. 9823, 7 Apr 2012. Available from: URL: <http://dx.doi.org/10.1016/s0140-6736%2811%2961939-9> (English)
Journal Fulltext.

Krka's comment: S, UL (for haemolysis), probably related

ADRs with reasonable time relationship to diclofenac intake. After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia with acute renal failure due to haemolysis was diagnosed. Acute cystitis could also be a contributive factor to unusual dual autoimmune response.

Temmler Pharma GmbH & Co. KG, 22.06.2012 DE-TEMMLER06-005127

Sender diagnosis: not reported

Narrative

This literature case was received by Temmler Pharma on 19 June 2012 from Adis Reaktionen via literature research:

A 66-year-old woman developed autoimmune haemolytic anaemia during treatment with diclofenac. The woman was admitted in February 2011 with fatigue, jaundice and macroscopic haematuria. In the previous week, recurrent nausea, vomiting and haematuria had occurred. She had received cotrimoxazole [trimethoprim/sulfamethoxazole] for cystitis 24 hours before admission. Her blood tests results were: haemoglobin 56 g/L, free haemoglobin 8.05 g/L, WBC count $17.6 \times 10^9/L$, total bilirubin 75 micromol/L , creatinine 0.34 mmol/L and lactate dehydrogenase 2500 U/L . Autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by direct antiglobulin testing, which revealed antibodies to IgG and complement C3d. The woman received a blood transfusion and prednisolone. Her cystitis-related haematuria resolved after levofloxacin treatment. Peripheral blood smears revealed neutrophil erythrophagocytosis, and immunocytochemistry demonstrated T cell variable immunoreceptor expression in neutrophils. Her history revealed that during the previous 3 weeks, she had repeatedly self-administered diclofenac for back pain [route and dosage not stated]. After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia was diagnosed. After 2 weeks of steroids, her haemolytic signs and renal function had nearly normalised, T cell receptor expression had normalised, and erythrophagocytosis was undetectable. In July 2011, her red cell measurements were normal, and steroids were withdrawn.

Temmler ID: DIC-2012-060<dbtext>To_Char_NARRATIVE_2</dbtext>

"IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. Our patient's case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor.

We suspect that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis)."

TEVA, 22.06.2012 DE-TEVA-344014GER

Sender diagnosis: not reported

Narrative

18-Jun-2012, Serious Literature Report received from Reaction (#803072220).
(WOLTERS KLUWER HEALTH; NZ-ADIS-3072220-26233)

Case of a female patient 66 years of age who experienced DRUG-INDUCED COLD-TYPE AUTOIMMUNE HAEMOLYTIC ANAEMIA, ACUTE RENAL FAILURE DUE TO HAEMOLYSIS while on DICLOFENAC (Teva's product cannot be excluded).

The woman developed autoimmune haemolytic anaemia during treatment with diclofenac. She was admitted in February 2011 with fatigue, jaundice and macroscopic haematuria. In the previous week, recurrent nausea, vomiting and haematuria had occurred. She had received cotrimoxazole [trimethoprim/sulfamethoxazole] for cystitis 24 hours before admission. Her blood tests results were: haemoglobin 56 g/L, free haemoglobin 8.05 g/L, WBC count $17.6 \times 10^9/L$, total bilirubin 75 micromol/L, creatinine 0.34 mmol/L and lactate dehydrogenase 2500 U/L. Autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by direct antiglobulin testing, which revealed antibodies to IgG and complement C3d. The woman received a blood transfusion and prednisolone. Her cystitis-related haematuria resolved after levofloxacin treatment. Peripheral blood smears revealed neutrophil erythrophagocytosis, and immunocytochemistry demonstrated T cell variable immunoreceptor expression in neutrophils. Her history revealed that during the previous 3 weeks, she had repeatedly self-administered diclofenac for back pain [route and dosage not stated]. After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia was diagnosed. After 2 weeks of steroids, her haemolytic signs and renal function had nearly normalised, T cell receptor expression had normalised, and erythrophagocytosis was undetectable. In July 2011, her red cell measurements were normal, and steroids were withdrawn.

Author comment: "IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. Our patient's case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. We suspect that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis)."

At the time of the report the outcome of the AEs were: recovering/resolving.

Concomitant Medication:

Trimethoprim/Sulfamethoxazole(SULFAMETHOXAZOLE, TRIMETHOPRIM)

Medical History:

BREAST CANCER(Concomitant) ,BASAL CELL CARCINOMA(Concomitant) ,BASAL CELL CARCINOMA EXCISION(Past) ,ACUTE CYSTITIS (Concomitant) ,BACK PAIN(Concomitant) ,MACROSCOPIC HAEMATURIA(Concomitant) ,NAUSEA(Concomitant) ,VOMITING(Concomitant)

Action Taken with suspect Drugs: DICLOFENAC - Unknown

Literature Reference:

Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze TJ, et al. An autoimmune double attack. Lancet 2012; 379(9823) 1364

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: DRUG-INDUCED COLD-TYPE AUTOIMMUNE HAEMOLYTIC ANAEMIA is probably related to diclofenac use due to temporal plausibility and especially due to positive anti-diclofenac IgG test results. Due to data available contribution of diclofenac to ACUTE RENAL FAILURE DUE TO HAEMOLYSIS cannot be excluded as diclofenac might have also contributed to acute renal impairment and not only to haemolysis that was followed by consecutive acute renal failure. Nevertheless, it cannot be excluded that development of acute renal failure was additionally supported by other reasons and circumstances, such as antibiotic treatment and underlying cystitis.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case.

ACTAVISPR, 27.06.2012 DE-ACTAVISPR-ACTA034111

Sender diagnosis: not reported

Narrative

This literature report was received from Germany.

A 66 year old female experienced autoimmune hemolytic anemia (PT: anaemia haemolytic autoimmune), acute renal failure and erythrophagocytosis whilst being treated with diclofenac for back pain. The patient had a medical history of breast cancer, basal cell carcinoma of forehead, cystitis and back pain. Concomitant medication included Co-trimoxazole (trimethoprim, sulfamethoxazole).

The patient was admitted in February 2011 with fatigue, jaundice and macroscopic hematuria. The week before admission, she experienced recurrent episodes of nausea, vomiting and hematuria with no fever. She was not taking regular medication, but had been given Co-trimoxazole (trimethoprim, sulfamethoxazole) for cystitis 24 hours earlier. Laboratory investigations revealed anemia and leukocytosis, elevated total bilirubin and creatinine, along with increased levels of lactate dehydrogenase and free hemoglobin. The suspicion of autoimmune hemolytic anemia with acute renal failure due to hemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral etiology. The patient was given intravenous prednisolone 125 mg daily and a blood transfusion. The acute cystitis that caused macrohematuria resolved after levofloxacin administration. Cancer screening showed no evidence of neoplasia. Microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells) and immunohistochemistry revealed massive induction of the T cell receptor in the patient's purified neutrophils. This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating neutrophils, indicating that a flexible autoimmune response had been triggered in neutrophils. It was eventually discovered that the patient had repeatedly self-administered diclofenac during the previous 3 weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune hemolytic anemia. After 2 weeks of steroid treatment, the hemolytic signs and renal function had nearly returned to normal. There was no erythrophagocytosis detected and the expression of the neutrophil variable immunoreceptor had normalized. The patient was last seen in July 2011, when her red cell measurements were normal and steroids were discontinued.

Dolorgiet GmbH, 28.06.2012 DE-DOLORGIET-2012diclo03825

Sender diagnosis: not reported

Narrative

A 66-year-old woman developed autoimmune haemolytic anaemia during treatment with diclofenac.

The woman was admitted in February 2011 with fatigue, jaundice and macroscopic haematuria. In the previous week, recurrent nausea, vomiting and haematuria had occurred. She had received cotrimoxazole [trimethoprim/sulfamethoxazole] for cystitis 24 hours before admission. Her blood tests results were: haemoglobin 56 g/L, free haemoglobin 8.05 g/L, WBC count $17.6 \times 10^9/L$, total bilirubin 75 $\mu\text{mol/L}$, creatinine 0.34 mmol/L and lactate dehydrogenase 2500 U/L. Autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by direct antiglobulin testing, which revealed antibodies to IgG and complement C3d.

The woman received a blood transfusion and prednisolone. Her cystitis-related haematuria resolved after levofloxacin treatment. Peripheral blood smears revealed neutrophil erythrophagocytosis, and immunocytochemistry demonstrated T cell variable immunoreceptor expression in neutrophils. Her history revealed that during the previous 3 weeks, she had repeatedly self-administered diclofenac for back pain [route and dosage not stated]. After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia was diagnosed. After 2 weeks of steroids, her haemolytic signs and renal function had nearly normalised, T cell receptor expression had normalised, and erythrophagocytosis was undetectable. In July 2011, her red cell measurements were normal, and steroids were withdrawn.

Kappler Pharma Consult GmbH, 04.07.2012 DE-KAPPLER-000172

Sender diagnosis: not reported

Narrative

Literature case:

In February, 2011, a 66-year-old woman was admitted to our hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead 12 months earlier. She was not taking regular medication but had been given cotrimoxazole for cystitis 24 h earlier. Blood tests showed anaemia (haemoglobin 56 g/L) and leukocytosis ($17.6 \times 10^9/L$, 90% neutrophils, 1% eosinophils); high concentrations of total bilirubin, 75 $\mu\text{mol/L}$, and creatinine, 0.34 mmol/L. Lactate dehydrogenase was 2500 U/L and free haemoglobin 8.05 g/L (normal <0.05 g/L), which indicated massive intravascular haemolysis. Our suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. Our patient was given intravenous prednisolone 125 mg/day and a blood transfusion. The acute cystitis causing the macro-haematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patient's history and the known association of autoimmune haemolytic anaemia with malignancies; however, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia.

Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells.' Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from our patient. This membrane-bound variable immunoreceptor, which has been identified in phagocytes,' was present in more than 80% of circulating neutrophils (normal 5-8%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of our patient's history revealed that she had repeatedly self-administered diclofenac during the previous 3 weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia. Steroids are the first-line therapy in this disorder⁴ and patients with symptomatic anaemia might need blood transfusion. In refractory cases, splenectomy and rituximab are effective. In our patient, haemolytic signs and renal function had nearly returned to normal after 2 weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised. When last seen in July, 2011, her red cell measurements were normal, and steroids were discontinued.<dbtext>To_Char_NARRATIVE_2</dbtext>

JUTA, 09.07.2012 DE-JUTA-2012-11842

Sender diagnosis: not reported

Narrative

Date of initial report: 25-JUN-2012

A literature article from the journal Lancet, entitled "An autoimmune double attack," describes a 66 year old female patient who experienced autoimmune haemolytic anaemia after self-administering diclofenac for back pain over a 3 week period.

"In February, 2011, a 66-year old woman was admitted to our hospital with fatigue, jaundice, and macroscopic haematuria. In the week before admission she had experienced recurrent episodes of nausea, vomiting, and haematuria with no fever. Medical history included successful breast cancer treatment at the age of 59 years, and excision of a basal-cell carcinoma of the forehead 12 months earlier. She was not taking regular medication but had been given cotrimoxazole for cystitis 24 h earlier. Blood tests showed anaemia (haemoglobin 56 g/L) and leukocytosis (17.6x10⁹/L, 90% neutrophils, 1% eosinophils high concentrations of total bilirubin, 75 micromol/L, and creatinine, 0.34 mmol/L. lactate dehydrogenase was 2500 U/L and her haemoglobin 8.05 g/L (normal <0.05 g/L), which indicated massive intravascular haemolysis. Our suspicion of autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by a direct antiglobulin test that detected antibodies to IgG and complement C3d. Negative serological tests ruled out a viral aetiology. Our patient was given intravenous prednisolone 125 mg/day and a blood transfusion. The acute cystitis causing the macro-haematuria resolved quickly after levofloxacin treatment. Cancer screening was undertaken because of the patient's history and the known association of autoimmune haemolytic anaemia with malignancies; however, examination of a bone-marrow biopsy sample and whole-body MRI showed no evidence of neoplasia. Surprisingly, microscopy of peripheral blood smears showed phagocytosis in neutrophils (about 5% of cells, figure A). Neutrophil erythrophagocytosis has been reported previously only in the context of leukaemias, paroxysmal cold haemoglobinuria, and rare defects of red cells. Immunocytochemistry showed massive induction of the T cell receptor in purified neutrophils from our patient (figure B). This membrane-bound variable immunoreceptor, which has been identified in phagocytes, was present in more than 80% of circulating neutrophils (normal 54%) indicating that a flexible autoimmune response had been triggered in neutrophils. Careful scrutiny of our patient's history revealed that she had repeatedly self-administered diclofenac during the previous 3 weeks for back pain. Subsequent positive testing for anti-diclofenac IgG established the diagnosis of drug-induced cold-type autoimmune haemolytic anaemia. Steroids are the first-line therapy in this disorder and patients with symptomatic anaemia might need blood transfusion. In refractory cases, splenectomy and rituximab are effective. In our patient, haemolytic signs and renal function had nearly returned to normal after 2 weeks of steroid treatment. No erythrophagocytosis was detectable, and expression of the neutrophil variable immunoreceptor had normalised (figure C). When last seen in July 2011, her red cell measurements were normal, and steroids were discontinued."

See article for further details.

onkovis, 07.12.2012 DE-KSK-PHARMA-2012-0066

Sender diagnosis: not reported

Narrative

Report from literature:

A 66-year-old woman developed autoimmune haemolytic anaemia during treatment with diclofenac.

The woman was admitted in February 2011 with fatigue, jaundice and macroscopic haematuria. In the previous week, recurrent nausea, vomiting and haematuria had occurred. She had received cotrimoxazole [trimethoprim/sulfamethoxazole] for cystitis 24 hours before admission. Her blood tests results were: haemoglobin 56 g/L, free haemoglobin 8.05 g/L, WBC count 17.6 x 10⁹ /L, total bilirubin 75 µmol/L, creatinine 0.34 mmol/L and lactate dehydrogenase 2500 U/L. Autoimmune haemolytic anaemia with acute renal failure due to haemolysis was confirmed by direct antiglobulin testing, which revealed antibodies to IgG and complement C3d.

The woman received a blood transfusion and prednisolone. Her cystitis-related haematuria resolved after levofloxacin treatment. Peripheral blood smears revealed neutrophil erythrophagocytosis, and immunocytochemistry demonstrated T cell variable immunoreceptor expression in neutrophils. Her history revealed that during the previous 3 weeks, she had repeatedly self-administered diclofenac for back pain [route and dosage not stated]. After positive anti-diclofenac IgG test results, drug-induced cold-type autoimmune haemolytic anaemia was diagnosed. After 2 weeks of steroids, her haemolytic signs and renal function had nearly normalised, T cell receptor expression had normalised, and erythrophagocytosis was undetectable. In July 2011, her red cell measurements were normal, and steroids were withdrawn.

Author Comment "IgG type antibodies to red-cell/drug-antigen complexes are thought to trigger the premature elimination of erythrocytes by splenic macrophages. Our patient's case reveals a second mechanism of autoimmunity in the pathogenesis of the disorder involving a neutrophil adaptive immunoreceptor. We suspect that the unusual dual autoimmune response was triggered by the combination of repeated diclofenac intake and an acute infection (cystitis)."

Fuchs T, Puellmann K, Schneider S, Kruth J, Schulze TJ, Neumaier M, Beham AW, Kaminski WE. An autoimmune double attack. Lancet 379: 1364, No. 9823, 7 Apr 2012.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12170715

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epilepsy aggravated					Unknown
2 Elevated liver enzyme levels					Unknown
3 Drug interaction					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclo KD 75 akut diclofenac sodium	Oral	/	-	Pain
2 Mylepsinum primidone		/	-	Epilepsy
3 Novaminsulfon-ratiopharm 500 metamizole	Oral	/	-	Pain
4 Tegretal 400 retard carbamazepine		/	-	Epilepsy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Epilepsy			Yes	
Pain			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

TEVA, 06.08.2012 DE-TEVA-351491GER

Sender diagnosis: not reported

Narrative

31-Jul-2012, Spontaneous, Patient/Consumer Serious Report

A male consumer reported the case of his wife (age not provided) who experienced NUMBER OF EPILEPTIC FITS INCREASED, DRUG INTERACTION, ELEVATED LIVER ENZYME LEVELS while on Novaminsulfon-ratiopharm 500 (METAMIZOLE, Teva product), Mylepsinum (PRIMIDONE, Teva product), Tegretal 400 retard (CARBAMAZEPINE, non-Teva product) and Diclo KD 75 akut (DICLOFENAC SODIUM, non-Teva product).

The consumer reported that his wife is suffering from epilepsy and is treated with Tegretal 400 retard and Mylepsinum (drug dosage and therapy dates not provided). On unknown date she was prescribed Novaminsulfon-ratiopharm 500 and Diclo KD 75 akut for treatment of pain (dosage and exact therapy dates not provided) and since then she is suffering from increasing numbers of epileptic fits. In addition increased hepatic enzymes (exact values not provided) were detected.

At the time of the report the outcome of the AEs were: Unknown.

Medical History:
EPILEPSY(Concomitant) ,PAIN(Concomitant)

Action Taken with suspect Drugs: Novaminsulfon-ratiopharm 500 - Unknown ,Mylepsinum - Unknown ,Tegretal 400 retard - Unknown ,Diclo KD 75 akut - Unknown

Additional information have been requested.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment: Contribution of Metamizole to increased Number of Epileptic fits is not assessable as different drugs with possible different interaction have been used and no information about possible de- and re-challenge are available.

Contribution of Primidone to increased Number of Epileptic fits is not assessable as different drugs with possible different interaction have been used and no information about possible de- and re-challenge are available. Additionally, there is no information about other possible reasons and circumstances for event available.

Contribution of Metamizole and Primidone to drug interaction with any of the other reported drugs cannot be excluded: possibly related.

Contribution of Metamizole and Primidone to elevated liver enzyme level is not assessable, as there is no information about onset of increase and start of therapies available. Nevertheless, receipt of additional data will result in re-evaluation.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

TEVA, 04.09.2012 DE-TEVA-351491GER

Sender diagnosis: not reported

Narrative

31-Jul-2012, Spontaneous, Patient/Consumer Serious Report

A male consumer reported the case of his wife (age not provided) who experienced NUMBER OF EPILEPTIC FITS INCREASED, DRUG INTERACTION, ELEVATED LIVER ENZYME LEVELS while on Novaminsulfon-ratiopharm 500 (METAMIZOLE, Teva product), Mylepsinum (PRIMIDONE, Teva product), Tegretal 400 retard (CARBAMAZEPINE, non-Teva product) and Diclo KD 75 akut (DICLOFENAC SODIUM, non-Teva product).

The consumer reported that his wife is suffering from epilepsy and is treated with Tegretal 400 retard and Mylepsinum (drug dosage and therapy dates not provided). On unknown date she was prescribed Novaminsulfon-ratiopharm 500 and Diclo KD 75 akut for treatment of pain (dosage and exact therapy dates not provided) and since then she is suffering from increasing numbers of epileptic fits. In addition increased hepatic enzymes (exact values not provided) were detected.

At the time of the report the outcome of the AEs were: Unknown.

Medical History:

EPILEPSY(Concomitant) ,PAIN(Concomitant)

Action Taken with suspect Drugs: Novaminsulfon-ratiopharm 500 - Unknown ,Mylepsinum - Unknown ,Tegretal 400 retard - Unknown ,Diclo KD 75 akut - Unknown

Additional information have been requested.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment: Contribution of Metamizole to increased Number of Epileptic fits is not assessable as different drugs with possible different interaction have been used and no information about possible de- and re-challenge are available.

Contribution of Primidone to increased Number of Epileptic fits is not assessable as different drugs with possible different interaction have been used and no information about possible de- and re-challenge are available. Additionally, there is no information about other possible reasons and circumstances for event available.

Contribution of Metamizole and Primidone to drug interaction with any of the other reported drugs cannot be excluded: possibly related.

Contribution of Metamizole and Primidone to elevated liver enzyme level is not assessable, as there is no information about onset of increase and start of therapies available. Nevertheless, receipt of additional data will result in re-evaluation.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

30-Aug-2012

Additional information received from consumer. Consumer reported that at the same time when events occurred dose of carbamazepine was gradually reduced and then stopped and switched to levetiracetam. In his opinion there is no relationship with novaminsulfon and he is not willing to give further information.

Teva Comment: Additional information did not alter latest assessment: The causal relationship NUMBER OF EPILEPTIC FITS INCREASED, DRUG INTERACTION, ELEVATED LIVER ENZYME LEVELS in relation to Novaminsulfon-ratiopharm 500 and Mylepsinum is not assessable.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12229481

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19690320	Age: 42 Year	weight (kg): 80	BMI_int: 23.89	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	20110721		2 Week	46 Day	recovered/resolved
2 Hepatic failure	20110721		2 Week	46 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	150 Mg milligram(s)/Day	20110606- -	Surgery

Drugs (concomitant)

2 novalgin metamizole sodium	Oral	2000 Mg milligram(s)/Day	20110606- -	Clavicle fracture
3 pantozol pantoprazole sodium	Oral	1 Df dosage form/Day	20110606- -	Clavicle fracture

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Clavicle fracture			Unknown	links, offene Reposition 06.06.2011

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GOT increased		1005	U/l	
GPT increased		1774	U/l	
Gamma GT increased		437	U/l	
Bilirubin increased		11.79	mg/dl	
Aspartate aminotransferase		1005 U/l	NA	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.09.2012 DE-002147023-PHHY2012DE081332

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE081332 is an initial spontaneous report received from the physician through Drug Commission of the German Medical Association of Physicians via Health Authority (E2b report duplicate: DE-BFARM-12229481) on 18 Sep 2012. This report refers to a 42-years-old male patient. Medical history included clavicle fracture. Concomitant medications included Pantozol (pantoprazole sodium) 20 mg, once a day, Novalgin (metamizole sodium monohydrate) 500 mg four times a day both received since 06 Jun 2011 for post-op clavicle fracture. The patient received the treatment with Voltaren Resinat (diclofenac resinate) at a daily dose of 150 mg (75 mg, BID) orally since 06 Jun 2011 for surgery. On 21 Jul 2011, the patient developed drug induced hepatic failure and hepatitis. On an unknown date, the lab values revealed GOT increased to 1005 U/l, GPT increased to 1774 U/l, Gamma GT increased to 437 U/l and bilirubin increased to 11.79 mg/dl. On an unknown date, the treatment with Voltaren Resinat was discontinued and the patient was recovered. These events were assessed as serious (hospitalization). Causality of the events was assessed as possible.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12248083

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Peptic ulcer					Unknown
2 Gastro intestinal bleed					Unknown
3 Hemarthrosis					Unknown
4 Toxicity renal					Unknown
5 Liver injury					Unknown
6 Cardiovascular disorder					Unknown
7 Constipation					Unknown
8 Overdose					Unknown
9 Hepatotoxicity					Unknown
10 Sedation					Unknown
11 Addiction					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 celecoxib		/	-	Pain
celecoxib			-	
2 codeine		/	-	Pain
codeine			-	
3 diclofenac		/	-	Pain
diclofenac			-	
4 ETORICOXIB		/	-	Pain
etoricoxib			-	
5 FENTANYL	Transdermal	/	-	Pain
fentanyl			-	
6 ibuprofen		/	-	Pain
ibuprofen			-	
7 MELOXICAM		/	-	Pain
meloxicam			-	
8 METAMIZOL		/	-	Pain
metamizole magnesium			-	
9 MORPHINE		/	-	Pain
morphine			-	
10 naproxen		/	-	Pain
naproxen			-	
11 PARACETAMOL		/	-	Pain
paracetamol			-	
12 piroxicam		/	-	Pain
piroxicam			-	
13 TRAMADOL	Unknown	/	-	Pain
tramadol hydrochloride			-	
14 TRAMADOL HYDROCHLORIDE		/	-	Pain
tramadol hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Yes	
Haemophilia			Yes	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia Sep-2012;18 (5):743-752.	

Narratives (nur bei elektronischem Eingang)

Janssen-Cilag, Germany Local Safety Officers, 27.09.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported.

This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 27.09.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

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Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

GRUNENTHAL, 01.10.2012 DE-GRUNENTHAL-2012-15367

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2012-15367

This spontaneous literature case from Germany was reported by a physician via license partner and was initially received on 18-Sep-2012. It was extracted from the literature article 'Pain management in patients with haemophilia: a European survey' published by Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. in Haemophilia Sep-2012;18 (5):743-752

concerns a patient of unknown age (height and weight unknown) who experienced cardiovascular disorder, liver injury, nephropathy toxic, haemarthrosis, gastrointestinal haemorrhage, peptic ulcer, constipation, dependence, sedation and overdose under tramadol hydrochloride treatment.

Other suspected medication included fentanyl (Fentanyl, transdermal) for pain, paracetamol (Paracetamol, unknown route of administration) for pain, naproxen (Naproxen, unknown route of administration) for pain, ibuprofen (Ibuprofen, unknown route of administration) for pain, codeine (Codeine, unknown route of administration) for pain, metamizole magnesium (Metamizol, unknown route of administration) for pain, diclofenac (Diclofenac, unknown route of administration) for pain, piroxicam (Piroxicam, unknown route of administration) for pain, meloxicam (Meloxicam, unknown route of administration) for pain, morphine (Morphine, unknown route of administration) for pain, celecoxib (Celecoxib, unknown route of administration) for pain and etoricoxib (Etoricoxib, unknown route of administration) for pain.

No relevant concomitant medication was reported.

The patient's concurrent conditions included haemophilia and pain.

The patient was treated with tramadol hydrochloride (Tramadol hydrochloride,) for pain.

Reporter's notes: Report received from literature.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74

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This report was serious (medically significant).

No treatment of the AEs was reported.

Third party case numbers

e2b company number: de-jnjfoc-20120909195

grt litdb accno: 803382

Literature citation:

holstein k, klamroth r, richards m, carvalho m, perez-garrido r, gringeri a. pain management in patients with haemophilia: a european survey. haemophilia. 2012; 18(5): 743-752

Janssen-Cilag, Germany Local Safety Officers, 18.10.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

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This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 22.10.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. *Haemophilia* SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported. This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

Janssen-Cilag, Germany Local Safety Officers, 07.11.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. *Haemophilia* SEP-2012;18 (5):743-752.

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Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12258821

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 77 Year	weight (kg): 93	BMI_int: 30.02	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased	20111130				Unknown
2 Bilirubin total increased	20111130				Unknown
3 Bilirubin elevated					recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Unknown	/	-	Drug use for unknown indication
2 ARAVA leflunomide	Oral	20 Mg milligram(s)/Day	-	Drug use for unknown indication
3 diclofenac diclofenac	Oral	/	-	Drug use for unknown indication
4 TOCILIZUMAB tocilizumab	Intravenous (not otherwis	760 Mg milligram(s)/ 230	20111108-	Rheumatoid arthritis

Drugs (concomitant)

5 aspirin acetylsalicylic acid	/	-
6 ENALAPRIL MALEATE enalapril maleate	/	-
7 fenoterol fenoterol	/	-
8 FUROSEMIDE furosemide	/	-
9 Glyceroltrinitrat glyceryl trinitrate	/	-
10 kalinor potassium chloride	/	-
11 METOPROLOL metoprolol tartrate	/	-
12 miflonide budesonide	400 µg microgram(s)/	-
13 PANTOPRAZOLE pantoprazole	/	-
14 prednisolone prednisolone	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cardiac arrest			No	
Asthma bronchial			Yes	
Coronary heart disease			Yes	
Gilbert's syndrome			Yes	
Blood pressure high			Yes	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin	20111025	1.36	MG/DL	
Bilirubin	20111108	1.57	MG/DL	
Bilirubin	20111130	2.19	MG/DL	
Bilirubin	20120213	1	MG/DL	
Blood urea nitrogen	20110213	5.15	MG/DL	
Blood urea nitrogen	20120523	6.07	MG/DL	
Erythrocyte sedimentation rate	20120523	18/36	UNK	
MCH	20120523	29.40	PG	

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 08.10.2012 DE-SANOFI-AVENTIS-2012SA072828

Sender diagnosis: not reported

Narrative

This serious unsolicited case from Germany (local reference number: 1029279) was received on 27-Sep-2012 from a physician and concerns a 77-year-old male patient who experienced total bilirubin of greater than twice ULN and ALT greater than 3 times the ULN whilst receiving treatment with leflunomide (Arava), acetylsalicylic acid (ASS100) and diclofenac sodium (Diclofenac) all taken for unknown indications and tocilizumab for rheumatoid arthritis. Patient (168548002) was enrolled in Study ML22985 Efficacy and safety study of a sequential therapy of tocilizumab (TCZ) and, if initially inadequately responded to tocilizumab (TCZ), followed by rituximab (RTX) in DMARD-IR patients with rheumatoid arthritis (MIRAI).

On 13-Feb-2011, blood urea nitrogen was 5.15 mg/dl (normal range 3-6.7). On 25-Oct-2011, the patient presented with elevated bilirubin to 1.36 mg/dl (normal range: 0.3 to 1.2 mg/dl). On 25-Oct-2011, following tests were performed: Hepatitis B: negative, Hepatitis C: negative, Anti nuclear Ab was 1: 1280 nuv dots, anti-mitochondrial Ab was negative (less than 1:80) and UGTAA1A1-TA- exp homozygot. Following tests were not done: Hepatitis A, Hepatitis D, Anti CMV and Anti EBV. On 08-Nov-2011, the patient was started therapy with intravenous tocilizumab solution 760 mg every 4 weeks (lot/batch number and expiration date: unspecified). On the same day, the bilirubin level increased to 1.57 mg/dl. On an unspecified date the patient started therapy with leflunomide at the dose of 20 mg once per day orally (form, lot/batch number and expiration date: unspecified), acetyl salicylic acid (strength, dose, form, route, frequency, lot/batch number and expiration date: unspecified) and diclofenac sodium at the dose of 50 mg orally, (dose, route, form and frequency, lot/batch number and expiration date: unspecified). On 30-Nov-2011, the patient's bilirubin level was 2.19 mg/dl. The event of elevated bilirubin was resolved with sequelae. On 30-Nov-2011, the event was associated with total bilirubin of greater than twice ULN. The event was also associated with ALT greater than 3 times the ULN. Doses of blinded tocilizumab and blinded methotrexate were not modified due to lab abnormalities. The patient received no treatment for the event. On 6-Dec-2011, the patient switched to Rituximab. On 8-Dec-2011, UGT1A1- MA confirmed Gilbert Syndrome. On 19-Dec-2011, patient received another dose of Rituximab. On 13-Feb-2012, the patient's bilirubin level was 1 mg/dl. On the same day, the event of elevated bilirubin was considered resolved. On 23-May-2012, blood urea nitrogen was 6.07 mg/dl, MCH was 29.40 PG, erythrocyte sedimentation rate was 18/36.

Therapy status with leflunomide, acetylsalicylic acid, diclofenac sodium was unknown. Therapy with tocilizumab was discontinued on an unspecified date. Outcome for the events of total bilirubin of greater than twice ULN and ALT greater than 3 times the ULN was unknown.

Medical history included cardiac arrest. Concurrent conditions included bronchial asthma, high blood pressure, coronary heart disease, Gilbert's syndrome. Concomitant medications included acetylsalicylic acid (Aspirin), metoprolol tartrate (Metoprolol), enalapril maleate, glyceryl trinitrate (Nitroglycerine), furosemide, pantoprazole, potassium chloride (Kalinor), fenoterol, budesonide (Miflonide) and prednisolone. The patient had not received Biologic DMARDs, statins, acetaminophen or antibiotics concomitantly. No past drugs were reported.

Causality: Related

The investigator assessed the non-serious event of special interest of elevated bilirubin as unrelated to tocilizumab but related to leflunomide, acetyl salicylic acid and diclofenac sodium.

No further information was provided.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12309729

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: 19290509 Age: 83 Year weight (kg): 77 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20120724				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 50 diclofenac sodium	Oral	100 Mg milligram(s)/Day	201207-20120724	Pain management
			-	

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGOT	20120724	389	U/l	
SGPT	20120724	607	U/l	
GGT	20120724	998	U/l	
SGOT	20120731	18	U/l	
SGPT	20120731	69	U/l	
GGT	20120731	362	U/l	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12314941

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19410509	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Discolouration urine	20120808				Unknown
2 Nausea	20120808				Unknown
3 Hepatic enzymes increased	20120808				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		50 Mg milligram(s)/Day	-	
diclofenac			-	
2 pravastatin		20 Mg milligram(s)/Day	-	
pravastatin			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT increased	20120808	1333	U/l	
Gamma GT increased	20120808	886	U/l	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 10.12.2012 DE-Bristol-Myers Squibb Company-17165200

Sender diagnosis: not reported

Narrative

Seriousness criteria: Important Medical Events.

This case was received from the Drug Commission of the German Medical Association (Reference number: DE-DCGMA-12158821) via German Health Authority (Agency number: DE-BFARM-12314941) in BMS Germany (Local file number: 2012112801).

A physician reported that a 71-year-old female patient experienced discoloration of urine, nausea and had increased hepatic enzymes while on therapy with pravastatin sodium and diclofenac. The events were considered medically significant.

The patient had received pravastatin sodium 20 mg daily and diclofenac 50 mg daily. Onset date of the events was reported as 08-Aug-2012. On 08-Aug-12 glutamate pyruvate transaminase (GPT) increased to 1333 U/l and gamma- glutamyl transpeptidase (GT) increased to 886 U/l. Therapy with pravastatin sodium and diclofenac were withdrawn. At the time of this report the outcome of the events was unknown.

Bristol-Myers Squibb Company, 01.03.2013 DE-Bristol-Myers Squibb Company-17165200

Sender diagnosis: not reported

Narrative

Seriousness criteria: Important Medical Events.

This case was received from the Drug Commission of the German Medical Association (Reference number: DE-DCGMA-12158821) via German Health Authority (Agency number: DE-BFARM-12314941) in BMS Germany (Local file number: 2012112801).

A physician reported that a 71-year-old female patient experienced discoloration of urine, nausea and had increased hepatic enzymes while on therapy with pravastatin sodium and diclofenac. The events were considered medically significant.

The patient had received pravastatin sodium 20 mg daily and diclofenac 50 mg daily. Onset date of the events was reported as 08-Aug-2012. On 08-Aug-12 glutamate pyruvate transaminase (GPT) increased to 1333 U/l and gamma- glutamyl transpeptidase (GT) increased to 886 U/l. Therapy with pravastatin sodium and diclofenac were withdrawn. At the time of this report the outcome of the events was unknown.

Supplemental information received on 20-Feb-2013 from the physician via Drug Commission of the German Medical Association in response to the query noted that the patient had incidentally detected increase in transaminases, nausea and dark urine. Due to the events, therapy with pravastatin sodium was discontinued. The patient was under observation. The sonography did not show significant abnormalities but found perfused vessels.

The patient had medical history of gonarthrosis, meniscus lesion, hyperlipidemia and status post diverticulitis.

According to the physician, the events were related to pravastatin sodium and less related (possibly) to diclofenac sodium.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12323945

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea	20120808				Unknown
2 Discolouration urine	20120808				Unknown
3 Hepatic enzymes increased	20120808				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	50 Mg milligram(s)/Day	-	
diclofenac			-	
2 PRAVASTATIN	Oral	20 Mg milligram(s)/Day	-	
pravastatin sodium		245	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	20120808	1333	U/l	
GGT	20120808	886	U/l	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 05.12.2012 DE-002147023-GXKR2012DE003696

Sender diagnosis: not reported

Narrative

Case no. GXKR2012DE003696 is an initial spontaneous report from a physician, provided by the Drug Commission of the German Medical Association (sender, case no DE-DCGMA-12158821) via the German Health Authority (BfArM homepage, case no DE-BFARM-12314941), received on 28Nov2012: This report refers to a 71-year-old female patient with unknown medical history. The patient received pravastatin (MAH unknown) at a daily dose of 1x20mg and diclofenac (MAH unknown) at a daily dose of 1x50mg, both for unknown indication. On 08Aug2012 the patient experienced discolouration of urine, nausea and increased hepatic enzymes. Hepatic enzymes on 08Aug2012 were: GPT = 1333 U/l and GGT = 886 U/l. The events were reported non-serious but assessed as serious by the MAH. Treatment with both suspect medications was stopped on unspecified date. The outcome of the events was reported as unknown. Further information and causality assessment was not provided.

information (in the context of hepatitis). All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12332398

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 6 Decade	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	Back pain
diclofenac			-	
2 Katadolon S long	Oral	/	-	Back pain
flupirtine		212	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)TEVA, 11.12.2012 DE-TEVA-374326GER

Sender diagnosis: not reported

Narrative

05-Dec-2012, Spontaneous Serious Report

A physician reported the case of a male patient in the beginning of his fifties who experienced LIVER DAMAGE while on DICLOFENAC (Teva product cannot be excluded) and Katadolon S long (FLUPIRTINE, Teva product).
The patient who was otherwise healthy was prescribed Katadolon S long and diclofenac (dosage and therapy dates not provided) for back pain. Shortly after therapy was started patient was hospitalised due to severe liver damage.

At the time of the report the outcome of the AE was: Unknown.

Medical history: BACK PAIN(Concomitant)

Action taken with suspect drugs: Katadolon S long - Unknown ,DICLOFENAC - Unknown

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: According to information available, contribution of neither Flupirtine nor Diclofenac to the occurrence of "liver damage" can be excluded: Event is possibly related to treatment with both drugs.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-13000953

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	Yes		No

Patient

Initials: Sex: Male Birth: Age: 50 Year weight (kg): BMI_int: Age fetus:

Patientdeath

deathdate: 200302 deathcause: Hepatic cirrhosis
 deathdate: 200302 deathcause: Ascites

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Ascites					fatal
2 Edema of lower extremities					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC-ratiopharm 50 mg		50 Mg milligram(s)/	-	
diclofenac sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatitis C			Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:
 Literature:

Narratives (nur bei elektronischem Eingang)

TEVA, 09.01.2013 DE-TEVA-378595GER

Sender diagnosis: not reported

Narrative

02-Jan-2013, Spontaneous, Patient/Consumer Serious Report.

A wife reported the case of her husband with 50 Years of age who experienced ASCITES, FOOT EDEMA, LEG EDEMA while on DICLOFENAC-ratiopharm 50 mg(DICLOFENAC) (Teva's drug).

At the time of the report the outcome of the AEs were: ASCITES : fatal, FOOT EDEMA : Unknown, LEG EDEMA : Unknown.

Concomitant medication: Not reported.

Medical history: HEPATITIS C(Concomitant) which lead to HEPATIC CIRRHOSIS(Cause of death), ASCITES(Cause of death)

Action taken with suspect drugs: DICLOFENAC-ratiopharm 50 mg - Drug discontinued

This case was considered serious based on the following criteria: (Death, Other Serious (Important Medical Events)).

Furthermore, the wife of the patient stated that her husband didn't receive a liver transplantation, which in her opinion should be performed at this time. The husband died in Feb-2003.

Teva Comment: Due to insufficient information, this case is not assessable. No exact information regarding a temporal relation (time interval between drug administration and actual start of event), concomitant medication or clinical data was provided. According to information available, the patient had been suffering from hepatitis C and subsequent hepatic cirrhosis - most likely responsible for the development of ascites in this case. Receipt of additional information will result in re-evaluation.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

case: DE-BFARM-13005942

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19380410 Age: 74 Year weight (kg): 52 BMI int: 21.1 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic drug-induced	20120923			11 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	75 Mg milligram(s)/Day	20120913-20120920	Tendonitis
diclofenac			8 Day	

Drugs (concomitant)

2	clopidogrel clopidogrel	Oral	75 Mg milligram(s)/Day	20120814-20120923 41 Day	Drug-eluting coronary stent placement
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Single vessel disease			Unknown	Koronare-1-Gefäßerkrankung mit Z. n. Bypass-OP 03.2012 und PTCA
Left atrial myxoma			Unknown	Z. n. Myxomresektion linksatrial 03.2012
Pernicious anaemia			Unknown	perniziöse Anämie bei Typ - A- Gastritis
Hypertension arterial			Unknown	
Penicillin allergy			Unknown	
Hypercholesterolaemia			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGOT	20120923	1694	U/l	
SGPT	20120923	1693	U/l	
GGT	20120923	17	U/l	
Alkaline phosphatase	20120923	109	U/l	
Blood bilirubin total	20120923	3.13	mg/dl	
Blood bilirubin total	20120929	0.88	mg/dl	
Alkaline phosphatase	20120929	102	U/l	
GGT	20120929	18	U/l	
SGPT	20120929	286	U/l	
SGOT	20120929	36	U/l	
Ultrasonography	20120924	vorhanden/normal		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13036031

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Quick's value decreased	20130201			18 Day	recovered/resolved
2 Incorrect drug administration duration	20130115	20130215	32 Day	1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine		/ 95	-	Pain in extremity
2 voltaren schmerzgel diclofenac diethylamine	Topical	/Day 95	20130115-20130215	Shoulder pain
			32 Day	

Drugs (concomitant)

3 AMLODIPIN "ORIFARM" amlodipine	2.5 Mg milligram(s)/Day	-
4 BISOPROLOL bisoprolol	1 Df dosage form/Day	-
5 FALITHROM phenprocoumon	/	-
6 pantoprazol pantoprazole	/	-
7 tavor lorazepam	/	-
8 zaldiar paracetamol; tramadol hydrochloride	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Joint disorder			k.A.	Hip problems
Heart disease, unspecified			k.A.	
Spinal disorder			k.A.	LUMBAR SPINE BODY 5 WITH NARROWED SPINAL CANAL

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Quick's test		20	%	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.03.2013 DE-NOVNCHHQ-CHPA2013DE002796

Sender diagnosis: not reported

Narrative

This case combines initial report and follow-up information both received from

pharmacist on 05 Feb 2013. This male patient, born 1935, used Voltaren Schmerzgel (diclofenac diethylamine salt) from time to time probably due to pain at his shoulder. The patient developed "worsening of blood picture NOS". No further details and no blood values available at date of report. No further details regarding the dose regimen available at date of report. Reportedly, the treating physician assumed a causal relationship with the use of Voltaren Schmerzgel (diclofenac diethylamine salt). Concomitantly, the patient used Falithrom (phenprocoumon), Tavor (lorazepam), Clexane (enoxaparine), Zaldiar (tramadolhydrochloride, paracetamol), bisoprolol and pantoprazol. No further details regarding the medical history of the patient available at date of report. Outcome was unknown. Follow-up information requested in written form in week 6, 2013.

Follow-up information was received from a pharmacist on 07 Mar 2013 which reassessed the case to meet the serious criteria of medically significant. Updated: patient data, kind of reaction, duration of reaction, reaction onset, CIOMS criteria, daily dose, indication for use, therapy dates, other relevant history, concomitant drugs, measures taken, information related to suspected drug and adverse reaction, outcome, causality rating and readministration of drug. This 77 year old male patient was diagnosed with 'too thin blood (Quick's test result = 20%) (rated as significant)' after topical application of Voltaren Schmerzgel (diclofenac diethylamine salt) (batch number 'W8810') used to treat 'limb pain'. The reaction was nearer described as 'the weekly results of Quick's test rapidly deteriorated'. Therapy dates were specified as 15 Jan 2013 to 15 Feb 2013 (continuous treatment). The reporter specified the onset date as '01 Feb 2013'. The stated daily dose was 'topically applied 1x in the evening'. Voltaren Schmerzgel was stopped. Following treatment by physician with stop of Falithrom (phenprocoumon) therapy and injection of Clexane (enoxaparin) 80mg 1x per day adverse symptoms lasted all together for 3 weeks. Causality rating of reporter was probable. The reporter ticked the box 'Outcome: reaction continued'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'.

NOVARTIS, 13.03.2013 2013DE002796

Sender diagnosis: not reported

Narrative

This case combines initial report and follow-up information both received from pharmacist on 05 Feb 2013. This male patient, born 1935, used Voltaren Schmerzgel (diclofenac diethylamine salt) from time to time probably due to pain at his shoulder. The patient developed "worsening of blood picture NOS". No further details and no blood values available at date of report. No further details regarding the dose regimen available at date of report. Reportedly, the treating physician assumed a causal relationship with the use of Voltaren Schmerzgel (diclofenac diethylamine salt). Concomitantly, the patient used Falithrom (phenprocoumon), Tavor (lorazepam), Clexane (enoxaparine), Zaldiar (tramadolhydrochloride, paracetamol), bisoprolol and pantoprazol. No further details regarding the medical history of the patient available at date of report. Outcome was unknown. Follow-up information requested in written form in week 6, 2013.

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number of reports: 295